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**NO. 18-0358**

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**In the Supreme Court of Texas**
**Judicial Panel on Multidistrict Litigation**

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**IN RE TEXAS OPIOID LITIGATION**

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# DISTRIBUTOR DEFENDANTS' JOINT MOTION TO TRANSFER AND REQUEST FOR IMMEDIATE STAY AND JOINDER IN MANUFACTURERS' JOINT MOTION TO TRANSFER

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# DISTRIBUTOR DEFENDANTS' JOINT MOTION TO TRANSFER AND REQUEST FOR IMMEDIATE STAY AND JOINDER IN MANUFACTURERS' JOINT MOTION TO TRANSFER

## I. INTRODUCTION

Defendants AmerisourceBergen Drug Corporation[1], McKesson Corporation, and Cardinal Health, Inc. (collectively, "Distributor Defendants") respectfully move this Court, pursuant to Tex. Gov't Code § 74.162 and Tex. R. Jud. Admin. 13.3, to transfer all pending Texas opioid-related cases listed in Appendix A, as well as any future-filed tag-along actions, to a single court for coordinated pretrial proceedings. Distributor Defendants further move the Panel to stay all related district court proceedings in the Texas opioid actions pending consideration of this Motion to Transfer.

Transfer of the Texas opioid-related cases listed in Appendix A and any related tag-along actions to a single pretrial district court for consolidated or coordinated pretrial proceedings will be for the convenience of the parties and

[1] AmerisourceBergen Corporation ("ABC") has been improperly named as a defendant on the face of the Petition in seven of the eight opioid-related cases that have been filed and served thus far in Texas state court. ABC is not a wholesale distributor of prescription medications and is therefore not a proper defendant in these cases and should be voluntarily dismissed. The proper defendant which has been named in two of the eight opioid related cases (Hopkins and Travis Counties) is AmerisourceBergen Drug Corporation ("ABDC") which is a wholesale distributor of prescription medications. ABDC is therefore the appropriate moving defendant throughout this Motion. ABC joins with ABDC in this Motion, but does not concede that it is a proper party to any of the pending actions, and it is not.

witnesses, conserve the resources of the courts and the parties, avoid duplicative and repetitive discovery, avoid likely inconsistent or conflicting pretrial rulings, and promote the just and efficient conduct and resolution of the Texas opioid-related actions. Separate pretrial proceedings across multiple counties virtually assure repeated litigation of the same or similar issues, inconsistent and conflicting pretrial rulings on those issues, and wasteful and unnecessary duplication of effort by the parties and multiple courts. Tex. Gov't Code § 74.162 and Tex. R. Jud. Admin. 13 were intended to avoid just those results.

The Manufacturer Defendants[2] filed a Joint Motion to Transfer and Request for Immediate Stay on April 20, 2018. Distributor Defendants join in the Manufacturers' motion and incorporate the arguments and authorities set forth in that motion in accordance with Texas Rule of Appellate Procedure 9.7. As set forth below, Distributor Defendants elaborate on the arguments with respect to the claims made against the Distributor Defendants.

[2] The Manufacturer Defendants filing the Motion are Purdue Pharma L.P., Purdue Pharma Inc., The Purdue Frederick Company, Inc., Allergan Finance, LLC f/k/a Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc., Johnson & Johnson, Janssen Pharmaceuticals, Inc., Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc., Janssen Pharamceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc., AbbVie Inc., Knoll Pharmaceutical Company, a wholly-owned subsidiary of AbbVie Inc., Endo Health Solutions, Inc., Endo Pharmaceuticals Inc., Watson Laboratories, Inc., Actavis LLC and Actavis Pharma, Inc. f/k/a Watson Pharma, Inc.

## II. BACKGROUND

There are hundreds of federal actions pending nationwide in which governmental entities like the Plaintiff Counties here have sued pharmaceutical manufacturers and distributors related to opioid medications. Generally, the Plaintiff Counties allege they have sustained harm and money damages because pharmaceutical manufacturers ("Manufacturer Defendants"), including those named in this case, engaged in misleading marketing practices related to their FDA-approved opioid medications and because pharmaceutical distributors ("Distributor Defendants"), including those named in this case, unreasonably failed to restrict sales to certain customers.

### A. The Federal MDL

On December 5, 2017, the United States Joint Panel on Multidistrict Litigation ("JPML") transferred over 60 of these opioid-related actions to a nation-wide federal multidistrict litigation ("MDL") in the United States District Court for the Northern District of Ohio for coordinated proceedings. *See In re Nat'l Prescription Opiate Litig.*, MDL No. 2804, 2017 WL 6031547, at **3-5. (Doc. 328-Dec. 5, 2017 Transfer Order) (J.P.M.L. Dec. 5, 2017).

Since that time, the number of cases transferred or conditionally transferred to the federal MDL has grown substantially. To date, there have been approximately 45 opioid-related actions filed by Texas Counties in either federal or state court in

Texas. The great majority of those cases have been filed in federal court and have been transferred or conditionally transferred to the federal MDL.

## B. State Court MDLs

Recognizing the advantages of judicial economy, convenience of the parties and consistent pretrial rulings made possible by coordinated proceedings in matters such as these, state courts in New York, Connecticut, Pennsylvania, and Illinois have ordered state-wide coordinated proceedings for opioid-related cases pending in those states. *See, e.g., In re Opioid Litig.*, Index No. 400000/2017 (N.Y. Sup. Ct. Suffolk County), Order (July 17, 2017).

The federal MDL and these parallel state court coordinated proceedings were established for the same reasons that a Texas state MDL proceeding is appropriate here: to avoid conflicting rulings, preserve judicial resources, reduce unnecessary hardship on the parties, and promote a just and efficient result in these proceedings.

## C. Texas Cases to Be Transferred to Texas State MDL

Distributor Defendants seek to transfer eight opioid-related cases filed by Texas Counties and pending in Texas state courts as well as additional future cases that are anticipated for coordination in a Texas state MDL.[3] The eight cases are

[3] Those eight cases are listed on the attached Appendix A. In addition to the eight cases listed on Exhibit A, Distributor Defendants are aware of a virtually identical case brought by Harris County which was filed in the 133rd Judicial District Court of Harris County, Texas. Distributor Defendants have attached as Appendix B, a list of parties for each of the eight cases for which consolidation is sought.

virtually identical.[4] In each case, a Texas Plaintiff County makes allegations against manufacturers and distributors of prescription opioid medications. Generally, each Plaintiff County makes identical allegations against the Manufacturer Defendants claiming that they engaged in misleading marketing of prescription opioid medications by overstating the benefits and downplaying the risks of the use of the opioids they manufactured and aggressively marketed (directly and through key opinion leaders) to physicians.[5] Each Plaintiff County alleges that the Distributor Defendants failed to monitor, detect, investigate, refuse and report suspicious orders of prescription opiates.[6]

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[4] Distributor Defendants have attached the Petition for each of those eight cases as follows: *Dallas,* attached as Appendix C; *Delta*, attached at Appendix D; *Falls*, attached as Appendix E; *Hopkins*, attached as Appendix F and *Van Zandt,* attached as Appendix G (all filed by Simon Greenstone Panatier Bartlett, P.C.). The *Travis* Petition, attached as Appendix H, was filed by the Lanier Law Firm and other Plaintiffs' counsel. The Lanier Law Firm also represents Dallas County. Distributor Defendants have also attached Petitions for *Maverick*, attached as Exhibit I and *Hidalgo*, attached as Exhibit J.

[5] *See, e.g.*, *Dallas* Petition, at ¶¶ 8-12, 43-44, 49-58 and 140-141, attached as Appendix C; *Delta* Petition, at ¶¶ 8-12, 41-42, 48-56 and 144-145, attached as Appendix D; *Falls* Petition, at ¶¶ 9-12, 40-55 and 131-132, attached as Appendix E; *Hopkins* Second Amended Petition, at ¶¶ 10-15, 53-73, 117-121 and 254, attached as Appendix F; *Van Zandt* Petition, at ¶¶ 9-12, 40-55, 129 and 131, attached as Appendix G; *Travis* Petition, at ¶¶ 10-13, 41-56 and 134, attached as Appendix H; *Maverick* Petition, at ¶¶ 14-17, 179-192, 222-231 and 693 attached as Exhibit I and *Hidalgo* Petition, at ¶¶ 10-13, 50-64 and 132 attached as Exhibit J.

[6] *See, e.g.*, *Dallas* Petition, at ¶¶ 13-14, 191-192 and 201-203, attached at Appendix C; *Delta* Petition, at ¶¶ 13-14, 195-196 and 205-207, attached at Appendix D; *Falls* Petition, at ¶¶ 13-14, 182-183 and 193-194, attached at Appendix E; *Hopkins* Second Amended Petition, at ¶¶ 17-18, 275, 280-283, 301-302, 338, 344, 349-350 and 355-358, attached at Appendix F; *Van Zandt* Petition, at ¶¶ 13-14, 182-183 and 192-194, attached at Appendix G; *Travis* Petition, at ¶¶ 14-15, 185-186 and 195-196, attached at Appendix H; *Maverick* Petition, at ¶¶ 94-128 and 734-735, attached as Exhibit I and *Hidalgo* Petition, at ¶¶ 14-15, 183-184 and 193-195, attached as Exhibit J.

## III. ARGUMENT AND AUTHORITIES

The requirements for transfer of Texas civil cases to a single court for consolidated pretrial proceedings are set out in Tex. Gov't Code § 74.162 (Transfer of Cases by Panel) which provides as follows:

> Notwithstanding any other law to the contrary, the judicial panel on multidistrict litigation may transfer civil actions involving one or more common questions of fact pending in the same or different . . . district courts to any district court for consolidated or coordinated pretrial proceedings, including summary judgment or other dispositive motions, but not for trial on the merits. A transfer may be made by the judicial panel on multidistrict litigation on its determination that the transfer will:
>
> (1) be for the convenience of the parties and witnesses; and
>
> (2) promote the just and efficient conduct of the actions.

*See also* Tex. R. Jud. Admin. 13. Neither Section 74.162 nor Texas Rule of Judicial Administration 13 require any minimum number of cases before the Panel may order consolidation for pretrial coordination under Rule 13. *In re Standard Guar. Ins. Co.*, 339 S.W.3d 398, 400 (Tex. M.D.L. Panel 2009) (this Panel transferred three cases because discovery in the cases would be voluminous and virtually identical); *see also In re Vanderbilt Mortg. & Fin., Inc.*, 166 S.W.3d 12, 14 (Tex. M.D.L. Panel 2005) ("While relevant, the number of pending cases and parties is not directly determinative of the necessity for pretrial transfer."). As set out below, the Texas state court opioid cases sought to be transferred satisfy the requirements of Section 74.162 and should be transferred to a single pretrial court pursuant to Rule 13.

### A. The Texas opioid cases involve one or more common questions of fact.

Section 74.162 requires that the cases to be consolidated involve "one or more common questions of fact." *See also* Tex. R. Jud. Admin. 13.3(a)(1) (requiring the motion to transfer to "state the common question or questions of fact involved in the cases"). The JPML's December 5, 2017 Transfer Order establishing the federal MDL recognized the common questions of fact warranting the transfer of numerous opioid-related cases, which are essentially identical to those sought to be transferred here, to a single federal court:

> All of the actions can be expected to implicate common fact questions as to the allegedly improper marketing and widespread diversion of prescription opiates into states, counties and cities across the nation, and discovery likely will be voluminous. Although individualized factual issues may arise in each action, such issues do not – especially at this early stage of litigation – negate the efficiencies to be gained by centralization.

*In re Nat'l Prescription Opiate Litig.*, 2017 WL 6031547, at *2. *See also In re Digitek ® Litig.*, 387 S.W.3d 115, 116 (Tex. M.D.L. Panel 2009).

In the opioid-related cases involved here, Plaintiffs' allegations in each case are virtually identical. The claims made by Plaintiff Counties against the Manufacturer Defendants are virtually identical from one case to the next, as are the claims made by Plaintiff Counties against the Distributor Defendants. Each Plaintiff County alleges that the Manufacturer Defendants engaged in misleading marketing of prescription opioid medications, overstated the benefits of opioid medications,

and understated the risks of addiction to the opioids they manufactured and purportedly marketed through key opinion leaders. Each Plaintiff County alleges that the Distributor Defendants failed to monitor and restrict sales of opioids to certain customers and violated the Texas Controlled Substances Act ("TCSA"). There are also common questions related to the applicability of TCSA to the Distributor Defendants. Similarly, the nature and description of damages sought by each Plaintiff County, as well as their standing to seek such damages, are indistinguishable from case to case.

This Panel previously concluded that product liability claims against seven pharmaceutical defendants involved common questions of fact because they involved the same medication and the plaintiffs alleged the same injuries. *In re Digitek ® Litig.*, 387 S.W.3d at 116-17 ("[A]ssigning a pretrial judge [would] help eliminate duplicative discovery, minimize conflicting demands on witnesses, and prevent inconsistent decisions."). For the same reasons, these cases involve common questions of fact.

Although the named defendants differ slightly among the eight cases to be consolidated, those differences are minor. A cursory review of Appendix B-1 through B-8 demonstrates the near total congruence of the named defendants, both Manufacturers and Distributors, from case to case. This Panel has held that if the claims, factual allegations, and legal issues overlap, minor differences among named

defendants do not prevent pretrial consolidation under Rule 13. *In re Hurricane Rita Evacuation Bus Fire,* 216 S.W.3d 70, 72 (Tex. M.D.L. Panel 2006) (cases from same incident consolidated even though defendants, expert witnesses and damage calculations would be different); *In re Ocwen Loan Servicing, LLC Mortg. Servicing Litig.*, 286 S.W.3d 669, 672 (Tex. M.D.L. Panel 2007) (cases consolidated even though only one of the defendants was named in all of the actions).

**B. Pretrial consolidation will be for the convenience of the parties and witnesses, conserve resources of the courts and the parties, improve efficiencies and avoid inconsistent rulings.**

Pretrial consolidation is appropriate when it will "(1) eliminate duplicative and repetitive discovery, (2) minimize conflicting demands on witnesses, (3) prevent inconsistent decisions on common issues, and (4) reduce unnecessary travel." *In re State Farm Lloyds Hurricane Ike Litig.*, 392 S.W.3d 353, 355-56 (Tex. M.D.L. Panel 2012). *See also In re Digitek ® Litig.*, 387 S.W.3d at 116-17.

**1. Elimination of redundant written and document discovery.**

Because these cases raise common legal and factual questions, it is axiomatic that discovery requests between the parties will be similar, if not identical. *In re Digitek ® Litig.*, 387 S.W.3d at 117 (discovery would be repetitive because multiple suits dealt with the same medication). Pretrial discovery coordination is critical to ensure that discovery is focused on relevant issues, to avoid duplication, to prevent inconsistent discovery rulings as set out below, and conserve the resources of the

courts and the parties. *See In re Delta Lloyds Ins. Co. of Hous.*, 339 S.W.3d 384, 389 (Tex. M.D.L. Panel 2008).

**2. Coordinating the cases would ease demands on witnesses.**

The Texas opioid cases will almost certainly involve many of the same fact, expert, and non-party witnesses. Judicial efficiency demands that repetitive depositions of the same witnesses be avoided. "[I]t is undeniable that it is more convenient for witnesses and parties who find themselves involved in several related cases to litigate in one pretrial court instead of several." *In re Silica Prods. Liab. Litig.*, 166 S.W.3d 3, 5 (Tex. M.D.L. Panel 2004). Pretrial consolidation to allow coordination of deposition discovery will minimize repetition and inconvenience to the witness and reduce costs for all parties.

**3. An MDL will ensure consistent pretrial rulings.**

If these cases are not coordinated, seven different state court judges[7] and their staff will devote significant time to cases which involve substantially the same factual and legal issues. It is very likely that dispositive motions and motions regarding exclusion of experts, as well as Rule 91a motions to dismiss will be filed in these cases. These motions will involve similar legal and factual issues, and the potential for inconsistent rulings is high.

[7] While there are eight different opioid cases sought to be consolidated, the Hopkins and Delta County cases are before the same Judge who "rides circuit" between multiple Counties.

The Rule 91a motions to dismiss which have now been filed in most, if not all cases, are a good example. These motions involve important issues such as standing, preemption and the "Municipal Cost Recovery Rule" (also called the "Free Public Services Doctrine"), any one of which may be dispositive of all of these cases. Coordination of these important, and in many instances, dispositive motions will mitigate the likelihood of inconsistent rulings and promote efficiency and uniformity across the litigation.

Because many, if not most of the issues in the Texas opioid cases involve one or more common questions of fact, there will certainly be common discovery in these cases. Therefore, there is a risk of inconsistent discovery rulings in addition to the substantive legal rulings. Discovery is likely to involve confidential business information ("CBI") as well as proprietary and confidential information of both Plaintiffs and Defendants. Just as with the substantive legal issues, consistency is extremely important and consolidation into a single pretrial court is likely to allow consistency in discovery rulings, including those regarding sensitive, proprietary and/or confidential information. *See In re Deep S. Crane & Rigging Co.*, 339 S.W.3d 395, 397 (Tex. M.D.L. Panel 2008) ("[S]ingle pretrial judge would be in a better position to protect the [information's] confidentiality.").

**4. Coordinating the cases would minimize travel.**

Consolidated MDL proceedings are likely to minimize the necessity that the parties and witnesses travel to multiple counties to address the same issues. *In re State Farm Lloyds Hurricane Ike Litig.*, 392 S.W.3d at 356. Absent consolidation, counsel for all parties will be required to attend hearings on identical issues, and counsel as well as witnesses, including non-party witnesses will be required to travel to multiple counties for depositions. This expensive and inefficient result is unnecessary and can be largely avoided by pretrial consolidation.

**5. Consolidating cases currently pending in Texas state district courts will facilitate coordination with the federal opioid MDL.**

Consolidation of the cases currently pending in the district courts of various Texas Counties will facilitate coordination of these proceedings with the federal MDL proceedings in *In re Nat'l Prescription Opiate Litig.*, MDL No. 2804. Coordination of MDL pre-trial proceedings in state and federal MDL court, in addition to coordination of the various cases pending in Texas state courts, likewise furthers the purpose of Texas Rule of Judicial Administration 13 to promote efficiency and protect against inconsistent determinations. Indeed, this Panel has issued transfer orders in several cases in which a nationwide MDL proceeded on a parallel track in federal court. *E.g.*, *In re Volkswagen Clean Diesel Litig.*, 516 S.W.3d 704, 705 (Tex. M.D.L. Panel 2016); *In re Digitek ® Litig.*, 387 S.W.3d 115, 118 (Tex. M.D.L. Panel 2009); *In re Toyota Unintended Acceleration Litig.*, 398

S.W.3d 892 (Tex. M.D.L. Panel 2010). Coordination and comity between the federal and state court proceedings therefore would be furthered by transfer by this Panel of the pending cases to a Texas MDL.

All of the Tex. Gov't Code § 74.162 and Tex. R. Jud. Admin. 13 requirements for pretrial consolidation are satisfied and the eight Texas opioid-related cases as well as any subsequent "Tag Along" cases, should be transferred to an MDL pretrial court.

## IV. REQUEST FOR IMMEDIATE STAY

Pursuant to Rule 13.4(b) of the Texas Rules of Judicial Administration, Defendants further request a stay of all trial court proceedings, including responsive pleading deadlines, discovery, and hearings, in the Texas opioid actions until a ruling is made by this Panel and, if granted, until the transferee judge can hold an initial status conference. A stay will protect Plaintiffs and Defendants from duplicative discovery requests, at least until this Panel rules on the motion to transfer. In addition, without a stay, Defendants would likely have to continue to file individual responsive pleadings and the parties would likely be required to file and prosecute motions before multiple judges across the state of Texas. In fact, Texas Rule of Civil Procedure 91a motions to dismiss have been filed by the Manufacturing Defendants, the Distributor Defendants, or both in several of these pending matters, and are currently set for hearing on the following dates:

- Hopkins May 3, 2018
- Dallas May 10, 2018
- Delta May 18, 2018
- Falls May 22, 2018

Given the imminent hearing dates on these critical issues, entry of a stay bears particular importance.

## V. CONCLUSION

Distributor Defendants pray that this Panel grant their Motion to Transfer the Texas opioid-related actions listed on Appendix A and any future tag-along actions to an MDL pretrial court for coordinated pretrial proceedings and discovery and that the Panel issue an order, pending its ruling on this Motion, to stay all Texas opioid cases currently pending and any future tag-along actions added to this Motion. Defendants also pray for such other and further relief to which they may be justly entitled.

Dated: May 1, 2018

Respectfully submitted,

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## CERTIFICATE OF CONFERENCE

Counsel for Distributors has contacted counsel for Plaintiff Counties in each of the affected cases regarding this motion, both with regard to the transfer to an MDL and to a stay. Counsel for Plaintiff Counties are considering their position or positions regarding the Distributors' motion and have advised that they expect to respond to Distributors' counsel by May 4. The Distributors are willing to consider proposals from Plaintiff Counties but in light of the fact that Manufacturers' Motion to Transfer has been filed and the fact that there are hearings scheduled on a number of Rule 91a Motions to Dismiss in some of the affected cases, determined that the motion should be filed without prejudice to subsequent agreements.

*/s/ Robert Scott*
Robert Scott

## CERTIFICATE OF COMPLIANCE

Pursuant to Texas Rule of Appellate Procedure 9.4(i)(3) and Texas Rule of Judicial Administration 13.3(e), I hereby certify that the portions of this motion to transfer required by subparagraphs 13.3(a)(1)-(2) do not exceed 20 pages. This is a computer-generated document created in Microsoft Word, using 14-point typeface for all text, except for footnotes which are in 12-point typeface.

*/s/ Robert Scott*
Robert Scott

## CERTIFICATE OF SERVICE

I certify that on May 1, 2018, I e-filed this document, which the MDL administrative staff will send to the MDL Panel. I further certify that true and correct copies of Distributor Defendants' Joint Motion to Transfer and Request for Immediate Stay and Joinder in Manufacturers' Joint Motion to Transfer were electronically delivered to all counsel for parties in the cases listed in Appendix A, all as required by Rule 13.3(f) and (h), and that notice required by Rule 13.3(i) will be filed with the trial court in each related case.

*/s/ Robert Scott*
Robert Scott

# APPENDIX A

## LIST OF CASES

1. No. D-1-GN-18-00290; *County of Dallas v. Purdue Pharma, L.P., et al.*
   In the 116th District Court of Dallas County, Texas.

2. No. 11125; *County of Delta v. Purdue Pharma, L.P., et al.*
   In the 62nd District Court of Delta County, Texas.

3. No. CV40015; *County of Falls v. Purdue Pharma, L.P., et al.*
   In the 82nd District Court of Falls County, Texas.

4. No. C-1349-18-C *County of Hidalgo v. Purdue Pharma LP, et al.*
   In the 139th District Court of Hidalgo County, Texas.

5. No. CV43486; *County of Hopkins v. Purdue Pharma, L.P., et al*.
   In the 62nd District Court of Hopkins County, Texas.

6. No. 17-10-34909-MCVAJA *Maverick County Texas v. Purdue Pharma L.P., et al.*
   In the 365th District Court of Maverick County, Texas.

7. No. D-1-GN-18-000625; *County of Travis v. Purdue Pharma, L.P., et al.*
   In the 216st District Court of Travis County, Texas.

8. No. 17-00302 *County of Van Zandt v. Purdue Pharma, L.P., et al.*
   In the 294th District Court of Van Zandt County, Texas.

# APPENDIX B

## LIST OF PARTIES

### Parties in *County of Dallas v. Purdue Pharma, L.P., et al.*

*Plaintiff:*

County of Dallas

*Defendants:*

AbbVie Inc.
Actavis LLC
Actavis Pharma, Inc. f/k/a Watson Pharma, Inc.
Allergan Finance, LLC f/k/a Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc.
Allergan plc f/k/a Actavis plc
AmerisourceBergen Corporation
Dr. Richard Andrews
Cardinal Health, Inc.
Endo Health Solutions Inc.
Endo Pharmaceuticals Inc.
Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Janssen Pharmaceuticals, Inc.
Johnson & Johnson
Knoll Pharmaceutical Company, a wholly-owned subsidiary of AbbVie Inc.
McKesson Corporation
Dr. Theodore Okechuku
Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Dr. Nicolas Padron
Purdue Pharma L.P.
Purdue Pharma Inc.
The Purdue Frederick Company Inc.
Watson Laboratories, Inc.

**Parties in *County of Delta v. Purdue Pharma, L.P., et al.***

*Plaintiff:*

County of Delta

*Defendants:*

Actavis LLC
Actavis Pharma, Inc. f/k/a Watson Pharma, Inc.
Allergan Finance, LLC f/k/a Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc.
Allergan plc f/k/a Actavis plc
AmerisourceBergen Corporation
Cardinal Health, Inc.
Endo Health Solutions Inc.
Endo Pharmaceuticals Inc.
Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Janssen Pharmaceuticals, Inc.
Johnson & Johnson
McKesson Corporation
Don Martin O'Neal, M.D.
Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Purdue Pharma L.P.
Purdue Pharma Inc.
The Purdue Frederick Company Inc.
Watson Laboratories, Inc.

## **Parties in *County of Falls v. Purdue Pharma, L.P., et al.***

*Plaintiff:*

County of Falls

*Defendants:*

Actavis LLC
Actavis Pharma, Inc. f/k/a Watson Pharma, Inc.
Allergan Finance, LLC f/k/a Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc.
Allergan plc f/k/a Actavis plc
AmerisourceBergen Corporation
Cardinal Health, Inc.
Endo Health Solutions Inc.
Endo Pharmaceuticals Inc.
Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Janssen Pharmaceuticals, Inc.
Johnson & Johnson
McKesson Corporation
Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Purdue Pharma L.P.
Purdue Pharma Inc.
The Purdue Frederick Company Inc.
Watson Laboratories, Inc.

## Parties in *County of Hopkins v. Purdue Pharma, L.P., et al.*

*Plaintiff:*

County of Hopkins

*Defendants:*

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Allergan plc f/k/a Actavis plc
AmerisourceBergen Corporation
AmerisourceBergen Drug Corporation, a wholly-owned subsidiary of AmerisourceBergen Corporation
Cardinal Health, Inc.
Cardinal Health 110, LLC
Howard Gregg Diamond, M.D.
Endo Health Solutions Inc.
Endo Pharmaceuticals Inc.
Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Janssen Pharmaceuticals, Inc.
Johnson & Johnson
Knoll Pharmaceutical Company
McKesson Corporation
Don Martin O'Neal, M.D.
Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Purdue Pharma L.P.
Purdue Pharma Inc.
The Purdue Frederick Company Inc.

**Parties in *County of Van Zandt v. Purdue Pharma, L.P., et al.***

*Plaintiff:*

County of Van Zandt

*Defendants:*

AbbVie Inc.
Actavis LLC
Actavis Pharma, Inc. f/k/a Watson Pharma, Inc.
Allergan Finance, LLC f/k/a Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc.
Allergan plc f/k/a Actavis plc
AmerisourceBergen Corporation
Cardinal Health, Inc.
Endo Health Solutions Inc.
Endo Pharmaceuticals Inc.
Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Janssen Pharmaceuticals, Inc.
Johnson & Johnson
Knoll Pharmaceutical Company, a wholly-owned subsidiary of AbbVie Inc.
McKesson Corporation
Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Purdue Pharma L.P.
Purdue Pharma Inc.
The Purdue Frederick Company
Watson Laboratories, Inc.

**Parties in *County of Travis v. Purdue Pharma, L.P., et al.***

*Plaintiff:*

County of Travis

*Defendants:*

AbbVie Inc.
Actavis LLC
Actavis Pharma, Inc. f/k/a Watson Pharma, Inc.
Allergan Finance, LLC f/k/a Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc.
Allergan plc f/k/a Actavis plc
AmerisourceBergen Drug Corporation
Cardinal Health, Inc.
Endo Health Solutions Inc.
Endo Pharmaceuticals Inc.
Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Janssen Pharmaceuticals, Inc.
Johnson & Johnson
Knoll Pharmaceutical Company, a wholly-owned subsidiary of AbbVie Inc.
McKesson Corporation
Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Purdue Pharma L.P.
Purdue Pharma Inc.
The Purdue Frederick Company Inc.
Watson Laboratories, Inc.

**Parties in *Maverick County, Texas v. Purdue Pharma, L.P., et al.***

*Plaintiff:*

Maverick County, Texas

*Defendants:*

Actavis LLC
Actavis Pharma, Inc. f/k/a Watson Pharma, Inc.
Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc.
Allergan PLC f/k/a Actavis PLC
AmerisourceBergen Corporation
Cardinal Health, Inc.
Cephalon, Inc.
Endo Health Solutions Inc.
Endo Pharmaceuticals Inc.
Perry Fine
Scott Fishman
Insys Therapeutics, Inc.
Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Janssen Pharmaceuticals, Inc.
Johnson & Johnson
McKesson Corporation
Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Russell Portenoy
Purdue Pharma L.P.
Purdue Pharma Inc.
Teva Pharmaceuticals USA, Inc.
The Purdue Frederick Company, Inc.
Walsh Distribution, LLC
Watson Laboratories, Inc.
Lynn Webster

**Parties in *County of Hidalgo v. Purdue Pharma, LP, et al.***

*Plaintiff:*

County of Hidalgo

*Defendants:*

Abbott Laboratories
Actavis LLC
Actavis Pharma, Inc. f/k/a Watson Pharma, Inc.
Allergan PLC, f/k/a Actavis PLC, f/k/a Allergan Finance LLC, f/k/a Actavis, Inc., f/k/a Watson Laboratories, Inc.
AmerisourceBergen Corporation
Cardinal Health, Inc.
Endo Health Solutions Inc.
Insys Therapeutics, Inc.
Insys Manufacturing LLC, a wholly-owned Subsidiary of Insys Therapeutics, Inc.
Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Knoll Pharmaceutical Company, a wholly-owned subsidiary of Abbott Laboratories
Mallinckrodt PLC
McKesson Corporation
Purdue Pharma L.P.
Purdue Pharma Inc.
Purdue Frederick Company
TEV Pharmaceuticals USA, Inc.
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## APPENDIX B – continued

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Dr. Nicolas Padron, *pro se*

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# APPENDIX A

## LIST OF CASES

1. No. D-1-GN-18-00290; *County of Dallas v. Purdue Pharma, L.P., et al.*
   In the 116th District Court of Dallas County, Texas.

2. No. 11125; *County of Delta v. Purdue Pharma, L.P., et al.*
   In the 62nd District Court of Delta County, Texas.

3. No. CV40015; *County of Falls v. Purdue Pharma, L.P., et al.*
   In the 82nd District Court of Falls County, Texas.

4. No. C-1349-18-C *County of Hidalgo v. Purdue Pharma LP, et al.*
   In the 139th District Court of Hidalgo County, Texas.

5. No. CV43486; *County of Hopkins v. Purdue Pharma, L.P., et al*.
   In the 62nd District Court of Hopkins County, Texas.

6. No. 17-10-34909-MCVAJA *Maverick County Texas v. Purdue Pharma L.P., et al.*
   In the 365th District Court of Maverick County, Texas.

7. No. D-1-GN-18-000625; *County of Travis v. Purdue Pharma, L.P., et al.*
   In the 216st District Court of Travis County, Texas.

8. No. 17-00302 *County of Van Zandt v. Purdue Pharma, L.P., et al.*
   In the 294th District Court of Van Zandt County, Texas.

# APPENDIX B

## LIST OF PARTIES

### Parties in *County of Dallas v. Purdue Pharma, L.P., et al.*

*Plaintiff:*

County of Dallas

*Defendants:*

AbbVie Inc.
Actavis LLC
Actavis Pharma, Inc. f/k/a Watson Pharma, Inc.
Allergan Finance, LLC f/k/a Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc.
Allergan plc f/k/a Actavis plc
AmerisourceBergen Corporation
Dr. Richard Andrews
Cardinal Health, Inc.
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Endo Pharmaceuticals Inc.
Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Janssen Pharmaceuticals, Inc.
Johnson & Johnson
Knoll Pharmaceutical Company, a wholly-owned subsidiary of AbbVie Inc.
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Dr. Nicolas Padron
Purdue Pharma L.P.
Purdue Pharma Inc.
The Purdue Frederick Company Inc.
Watson Laboratories, Inc.

**Parties in *County of Delta v. Purdue Pharma, L.P., et al.***

*Plaintiff:*

County of Delta

*Defendants:*

Actavis LLC
Actavis Pharma, Inc. f/k/a Watson Pharma, Inc.
Allergan Finance, LLC f/k/a Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc.
Allergan plc f/k/a Actavis plc
AmerisourceBergen Corporation
Cardinal Health, Inc.
Endo Health Solutions Inc.
Endo Pharmaceuticals Inc.
Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc.
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Purdue Pharma Inc.
The Purdue Frederick Company Inc.
Watson Laboratories, Inc.

## **Parties in *County of Falls v. Purdue Pharma, L.P., et al.***

*Plaintiff:*

County of Falls

*Defendants:*

Actavis LLC
Actavis Pharma, Inc. f/k/a Watson Pharma, Inc.
Allergan Finance, LLC f/k/a Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc.
Allergan plc f/k/a Actavis plc
AmerisourceBergen Corporation
Cardinal Health, Inc.
Endo Health Solutions Inc.
Endo Pharmaceuticals Inc.
Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Janssen Pharmaceuticals, Inc.
Johnson & Johnson
McKesson Corporation
Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Purdue Pharma L.P.
Purdue Pharma Inc.
The Purdue Frederick Company Inc.
Watson Laboratories, Inc.

## Parties in *County of Hopkins v. Purdue Pharma, L.P., et al.*

*Plaintiff:*

County of Hopkins

*Defendants:*

Allergan Finance, LLC f/k/a Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc.
Allergan plc f/k/a Actavis plc
AmerisourceBergen Corporation
AmerisourceBergen Drug Corporation, a wholly-owned subsidiary of AmerisourceBergen Corporation
Cardinal Health, Inc.
Cardinal Health 110, LLC
Howard Gregg Diamond, M.D.
Endo Health Solutions Inc.
Endo Pharmaceuticals Inc.
Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Janssen Pharmaceuticals, Inc.
Johnson & Johnson
Knoll Pharmaceutical Company
McKesson Corporation
Don Martin O'Neal, M.D.
Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Purdue Pharma L.P.
Purdue Pharma Inc.
The Purdue Frederick Company Inc.

**Parties in *County of Van Zandt v. Purdue Pharma, L.P., et al.***

*Plaintiff:*

County of Van Zandt

*Defendants:*

AbbVie Inc.
Actavis LLC
Actavis Pharma, Inc. f/k/a Watson Pharma, Inc.
Allergan Finance, LLC f/k/a Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc.
Allergan plc f/k/a Actavis plc
AmerisourceBergen Corporation
Cardinal Health, Inc.
Endo Health Solutions Inc.
Endo Pharmaceuticals Inc.
Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Janssen Pharmaceuticals, Inc.
Johnson & Johnson
Knoll Pharmaceutical Company, a wholly-owned subsidiary of AbbVie Inc.
McKesson Corporation
Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Purdue Pharma L.P.
Purdue Pharma Inc.
The Purdue Frederick Company
Watson Laboratories, Inc.

**Parties in *County of Travis v. Purdue Pharma, L.P., et al.***

*Plaintiff:*

County of Travis

*Defendants:*

AbbVie Inc.
Actavis LLC
Actavis Pharma, Inc. f/k/a Watson Pharma, Inc.
Allergan Finance, LLC f/k/a Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc.
Allergan plc f/k/a Actavis plc
AmerisourceBergen Drug Corporation
Cardinal Health, Inc.
Endo Health Solutions Inc.
Endo Pharmaceuticals Inc.
Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Janssen Pharmaceuticals, Inc.
Johnson & Johnson
Knoll Pharmaceutical Company, a wholly-owned subsidiary of AbbVie Inc.
McKesson Corporation
Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Purdue Pharma L.P.
Purdue Pharma Inc.
The Purdue Frederick Company Inc.
Watson Laboratories, Inc.

**Parties in *Maverick County, Texas v. Purdue Pharma, L.P., et al.***

*Plaintiff:*

Maverick County, Texas

*Defendants:*

Actavis LLC
Actavis Pharma, Inc. f/k/a Watson Pharma, Inc.
Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc.
Allergan PLC f/k/a Actavis PLC
AmerisourceBergen Corporation
Cardinal Health, Inc.
Cephalon, Inc.
Endo Health Solutions Inc.
Endo Pharmaceuticals Inc.
Perry Fine
Scott Fishman
Insys Therapeutics, Inc.
Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Janssen Pharmaceuticals, Inc.
Johnson & Johnson
McKesson Corporation
Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Russell Portenoy
Purdue Pharma L.P.
Purdue Pharma Inc.
Teva Pharmaceuticals USA, Inc.
The Purdue Frederick Company, Inc.
Walsh Distribution, LLC
Watson Laboratories, Inc.
Lynn Webster

**Parties in *County of Hidalgo v. Purdue Pharma, LP, et al.***

*Plaintiff:*

County of Hidalgo

*Defendants:*

Abbott Laboratories
Actavis LLC
Actavis Pharma, Inc. f/k/a Watson Pharma, Inc.
Allergan PLC, f/k/a Actavis PLC, f/k/a Allergan Finance LLC, f/k/a Actavis, Inc., f/k/a Watson Laboratories, Inc.
AmerisourceBergen Corporation
Cardinal Health, Inc.
Endo Health Solutions Inc.
Insys Therapeutics, Inc.
Insys Manufacturing LLC, a wholly-owned Subsidiary of Insys Therapeutics, Inc.
Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc.
Knoll Pharmaceutical Company, a wholly-owned subsidiary of Abbott Laboratories
Mallinckrodt PLC
McKesson Corporation
Purdue Pharma L.P.
Purdue Pharma Inc.
Purdue Frederick Company
TEV Pharmaceuticals USA, Inc.
Teva Pharmaceutical Industries, Ltd.

## APPENDIX B – continued

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# APPENDIX C

# DALLAS PETITION

FILED
DALLAS COUNTY
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DISTRICT CLERK

DC-18-00290

Angie Avina

**CAUSE NO. _____**

| | | |
|---|---|---|
| **COUNTY OF DALLAS,** | § | **IN THE DISTRICT COURT** |
| | § | |
| *Plaintiff,* | § | |
| | § | |
| vs. | § | **_____ JUDICIAL DISTRICT** |
| | § | |
| **PURDUE PHARMA L.P.**; | § | |
| **PURDUE PHARMA INC.**; | § | |
| **THE PURDUE FREDERICK COMPANY**; | § | |
| **JOHNSON & JOHNSON**; | § | **DALLAS COUNTY, TEXAS** |
| **JANSSEN PHARMACEUTICALS, INC.**; | § | |
| **ORTHO-MCNEIL-JANSSEN PHARMACEUTICALS, INC.** n/k/a JANSSEN PHARMACEUTICALS, INC.; | § | |
| **JANSSEN PHARMACEUTICA, INC.** n/k/a JANSSEN PHARMACEUTICALS, INC.; | § | |
| **ENDO HEALTH SOLUTIONS INC.**; | § | |
| **ENDO PHARMACEUTICALS, INC.**; | § | |
| **ABBVIE INC.**; | § | |
| **KNOLL PHARMACEUTICAL COMPANY,** a wholly-owned subsidiary of ABBVIE INC.; | § | |
| **ALLERGAN PLC** f/k/a ACTAVIS PLC; | § | |
| **ALLERGAN FINANCE LLC** f/k/a ACTAVIS, INC. f/k/a WATSON PHARMACEUTICALS, INC.; | § | |
| **WATSON LABORATORIES, INC.**; | § | |
| **ACTAVIS LLC**; | § | |
| **ACTAVIS PHARMA, INC.** f/k/a WATSON PHARMA, INC.; | § | |
| **MCKESSON CORPORATION**; | § | |
| **CARDINAL HEALTH, INC.**; | § | |
| **AMERISOURCEBERGEN CORPORATION**; | § | |
| **DR. RICHARD ANDREWS**; | § | |
| **DR. THEODORE OKECHUKU**; | § | |
| **DR. NICOLAS PADRON**; and | § | |
| **DOES 1 – 100, INCLUSIVE**, | § | |
| | § | |
| *Defendants.* | § | |

**PLAINTIFF'S ORIGINAL PETITION AND JURY DEMAND WITH DISCOVERY**

**TO THE HONORABLE JUDGE OF SAID COURT:**

Plaintiff, the County of Dallas, Texas, by and through the undersigned attorneys, (hereinafter "Dallas County" or "County") against Defendants Purdue Pharma L.P., Purdue Pharma Inc., The Purdue Frederick Company, Johnson & Johnson, Janssen Pharmaceuticals, Inc., Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc., Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc., Endo Health Solutions Inc., Endo Pharmaceuticals, Inc., Abbvie Inc., Knoll Pharmaceutical Company, a wholly-owned subsidiary of Abbvie Inc., Allergan PLC f/k/a Actavis PLC, Allergan Finance, LLC f/k/a Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc., Watson Laboratories, Inc., Actavis LLC, Actavis Pharma, Inc. f/k/a Watson Pharma, Inc., McKesson Corporation, Cardinal Health, Inc., AmerisourceBergen Corporation, Dr. Richard Andrews, Dr. Theodore Okechuku, and Dr. Nicolas Padron, and Does 1 – 100, alleges as follows:

## I. INTRODUCTION

1. The United States is in the midst of an opioid epidemic caused by Defendants' fraudulent marketing and sales of prescription opioids ("opioids") that has resulted in addiction, criminal activity, and loss of life. The opioid crisis has been described as "the AIDS epidemic of our generation, but even worse."[1] On October 26, 2017, President Donald Trump "declared a nationwide public health emergency to combat the opioid crisis."[2]

2. In 2016 alone, health care providers wrote more than 289 million prescriptions for

[1] David Wright, "Christie on Opioids: "This is the AIDS Epidemic of Our Generation, but even Worse," (Oct. 27, 2017), http://www.cnn.com/2017/10/27/politics/chris-christie-opioid-commission-aids-cnntv/index.html.
[2] Dan Merica, "What Trump's Opioid Announcement Means – and Doesn't Mean," (Oct. 26, 2017), http://www.cnn.com/2017/10/26/politics/national-health-emergency-national-disaster/index.html.

opioids, enough for *every adult in the United States* to have more than one bottle of pills.[3] Americans "consume 85% of all the opioids in the world" and are "the most medicated country in the world . . . ."[4]

3. Unfortunately, using opioids too often leads to addiction and overdose from opioids. In 2014, almost 2 million Americans were addicted to opioids.[5] That same year, more people died from drug overdoses than in any other year, and most overdose deaths involved an opioid. The Texas Legislature has found "that deaths resulting from the use of opioids and other controlled substances constitute a public health crisis."[6] In 2015, Texas "had the second highest total healthcare costs from opioid abuse in the nation ($1.96 billion) . . . ."[7]

4. In fact, accidental drug overdose deaths, of which reportedly at least two-thirds are opioid overdoses, are the leading cause of death for Americans under the age of 50. Accidental drug overdose deaths, predominantly from opioids, exceed the number of deaths caused by cars or guns.

5. The economic burden caused by opioid abuse in the United States is at least $78.5 billion,[8] including lost productivity and increased social services, health insurance costs, increased criminal justice presence and strain on judicial resources, and substance abuse treatment and rehabilitation.

6. This epidemic did not occur by chance. Defendants manufacture, market, distribute,

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[3] *Prevalence of Opioid Misuse*, BupPractice (Sept. 7, 2017), https://www.buppractice.com/node/15576.
[4] David Wright, "Christie on Opioids: "This is the AIDS Epidemic of Our Generation, but even Worse," (Oct. 27, 2017), http://www.cnn.com/2017/10/27/politics/chris-christie-opioid-commission-aids-cnntv/index.html.
[5] Substance Abuse and Mental Health Services Administration, National Survey on Drug Use and Health, 2014.
[6] Opinion of the Attorney General of Texas, KP-0168 (Oct. 4, 2017), *citing* Act of May 26, 2017, 85th Leg., R.S., ch. 534, §3, 2017 Tex. Sess. Law Serv. 1467, 1468.
[7] Kerry Craig, "Opioid Addiction Results in one Woman's Daily Struggle," Oct. 7, 2017, https://www.ssnewstelegram.com/news/opioid-addiction-results-in-one-woman-s-daily-struggle/article_bded4eoa-ab80-11e7-a252-d3f304e26628.html.
[8] *See CDC Foundation's New Business Pulse Focuses on Opioid Overdose Epidemic*, Centers for Disease Control and Prevention (Mar. 15, 2017), https://www.cdc.gov/media/releases/2017/a0315-business-pulse-opioids.html.

and sell prescription opioids, including, but not limited to, brand-name drugs like OxyContin, Vicodin, Opana, Percocet, Percodan, Duragesic, Ultram, Ultracet, and generics like oxycodone, oxymorphone, hydromorphone, hydrocodone, fentanyl, and tramadol, which are powerful narcotics.

7. Historically, opioids were considered too addictive and debilitating for treating non-cancer chronic pain,[9] such as back pain, migraines, and arthritis, and were used only to treat short-term acute pain or for palliative or end-of-life care.

8. By the late 1990s or early 2000s, however, each Manufacturing Defendant began a marketing scheme to persuade doctors and patients that opioids can and should be used ubiquitously and perpetually to treat moderate, non-cancer chronic pain. Each Manufacturing Defendant spent large sums of money to promote the benefits of opioids for non-cancer moderate pain while trivializing, or even denying, their risks. The Manufacturing Defendants' promotional messages deviated substantially from any approved labeling of the drugs and caused prescribing physicians and consuming patients to underappreciate the health risks, and to overestimate the benefits, of opioids.

9. Contrary to the language of their drugs' labels, Defendants falsely and misleadingly, in their marketing: (1) downplayed the serious risk of addiction; (2) promoted and exaggerated the concept of "pseudoaddiction" thereby advocating that the signs of addiction should be treated with more opioids; (3) exaggerated the effectiveness of screening tools in preventing addiction; (4) claimed that opioid dependence and withdrawal are easily managed; (5) denied the risks of higher opioid dosages; and (6) exaggerated the effectiveness of "abuse-deterrent" opioid formulations to prevent abuse and addiction.

[9] "Chronic pain" means non-cancer pain lasting three months or longer.

10. Defendants disseminated these falsehoods through ads and/or their sales representatives and physicians who supported Defendants' message. Sales representatives, working at Defendants' behest, promoted highly addictive opioids through souvenirs and toys including, but not limited to, opioid brand-bearing stuffed plush toys, dolls, coffee cups, fanny packs, water bottles, notepads, pens, refrigerator magnets, clocks, letter openers, rulers, daytime planners, bags, puzzles, posters, hand-held calculators, clipboards, highlighters, flashlights, key chains, clothing, reflex mallets, and mock-ups of the United States Constitution.

11. Defendants also used third parties they controlled by: (a) funding, assisting, encouraging, and directing doctors, known as "key opinion leaders" ("KOLs") and (b) funding, assisting, directing, and encouraging seemingly neutral and credible professional societies and patient advocacy groups (referred to hereinafter as "Front Groups").

12. Defendants worked with KOLs and Front Groups to taint the sources that doctors and patients relied on for ostensibly "neutral" guidance, such as treatment guidelines, Continuing Medical Education ("CME") programs, medical conferences and seminars, and scientific articles. After their individual and concerted efforts, Defendants convinced doctors that instead of being addictive and unsafe for long-term use in most circumstances, opioids were *required* in the compassionate treatment of chronic pain.

13. The Distributor Defendants were not standing by idly while Marketing Defendants were peddling their opioids to physicians and consumers. Cardinal, AmerisourceBergen, and McKesson ("Distributor Defendants") are three of the largest opioid distributors in the United States. Distributor Defendants purchased opioids from Manufacturing Defendants herein and sold them to pharmacies servicing consumers in Dallas County.

14. Despite the alarming and suspicious rise in the ordering of opioids by retailers in

Dallas County, Distributor Defendants did nothing. The Manufacturing Defendants and Distributor Defendants worked hand and glove to glut the U.S. and Dallas County with more opioids than would be consumed for therapeutic purposes. Each Defendant disregarded its legal duty to report suspicious opioid prescriptions, and each Defendant financially benefitted from the other Defendants (both Manufacturing and Distributor Defendants), disregarding their individual duties to report.

15. Essentially each Defendant ignored science and consumer health for profits. Defendants' efforts were so successful that opioids are now the most prescribed class of drugs generating $11 billion in revenue for drug companies in 2014 alone. Even after Purdue reached a $600 million federal settlement in 2007, the settlement failed to impact what is a "$13-billion-a-year opioid industry."[10]

16. As a direct and foreseeable consequence of Defendants' misrepresentations regarding the safety and efficacy of using opioids for chronic pain, Dallas County has spent and continues to spend large sums combatting the public health crisis created by Defendants' negligent and fraudulent marketing campaign.

17. For example, thousands of prescriptions were written for opioids in Dallas County in 2012[11] and in 2012 there were multiple deaths reported from drug overdoses.[12] A substantial number of those overdose deaths were a result, in whole or in part, of opioid ingestion. In each year from 2013-2017, there were multiple deaths in Dallas County caused in whole or in part from ingestion of prescription opioids. Defendants' marketing misconduct, as well as Defendants'

[10] Rebecca L. Haffajee, J.D., Ph.D., M.P.H., and Michelle M. Mello, J.D., Ph.D., *Drug Companies' Liability for the Opioid Epidemic*, N. Engl. J. Med. at 2305, (Dec. 14, 2017).
[11] https://www.cdc.gov/drugoverdose/maps/rxrate-maps.html; https://www.cdc.gov/nchs/data-visualization/drug-poisoning-mortality.
[12] https://www.cdc.gov/nchs/data-visualization/drug-poisoning-mortality.

efforts to sell more prescription opioids that can be consumed therapeutically, were natural and foreseeable causes of overdose deaths and injuries in Dallas County.

18. As a direct and foreseeable consequence of Defendants' conduct described regarding prescription opioids, Dallas County has committed and continues to commit resources to provide and pay for health care, law enforcement, social services, public assistance, pharmaceutical care and other services necessary for its residents.

## II. RULE 47 STATEMENT OF MONETARY RELIEF SOUGHT

19. Per Rule 47 of the Texas Rules of Civil Procedure, the County states that although the full measure of its damages is still being calculated, its damages caused by Defendants' acts and omissions exceed $1,000,000 but are believed to be less than $100,000,000. Accordingly, at this time in the litigation, Dallas County states that it is seeking monetary relief for an amount greater than $1,000,000 and less than $100,000,000, the rightful and just amount to be determined by the jury.

## III. VENUE AND JURISDICTION

20. Venue is proper in Dallas County because all or a substantial part of the events or omissions giving rise to this claim occurred in Dallas County. TEX. CIV. PRAC. & REM. CODE §15.002(a)(2). This Court has subject-matter jurisdiction over this matter because Plaintiff's damages are in excess of the minimal jurisdictional limits of this Court. TEX. GOVT. CODE §24.007(b).

21. This Court has general jurisdiction over Dr. Andrews, Dr. Okechuku, and Dr. Padron as they are Texas residents. This Court also has specific jurisdiction over all Defendants as their activities were directed toward Texas, and injuries complained of herein resulted from their activities. *Guardian Royal Exchange Assur., Ltd. v. English China Clays, P.L.C.*, 815 S.W.2d 223, 227 (Tex. 1991). Each Defendant has a substantial connection with Texas and the requisite minimum contacts with Texas necessary to constitutionally permit the Court to exercise

jurisdiction. *See id.* at 226.

## IV. PARTIES

### A. Plaintiff

22. This action is brought for and on behalf of Dallas County, which provides a wide range of services on behalf of its residents, including services for families and children, public health, public assistance, law enforcement, and emergency care.

### B. Defendants

23. PURDUE PHARMA L.P. is a limited partnership organized under the laws of Delaware, and may be served through its registered agent for service of process, The Prentice-Hall Corporation System, Inc., 251 Little Falls Drive, Wilmington, DE 19808. PURDUE PHARMA L.P. is, through its ownership structure, a Texas resident. PURDUE PHARMA INC. is a New York corporation with its principal place of business in Stamford, Connecticut, and may be served through its registered agent for service of process, Corporation Service Company, 80 State Street, Albany, NY 12207. THE PURDUE FREDERICK COMPANY is a Delaware corporation with its principal place of business in Stamford, Connecticut, and may be served through its registered agent for service of process, The Prentice-Hall Corporation System, Inc., 251 Little Falls Drive, Wilmington, DE 19808 (collectively, "Purdue").

24. Purdue manufactures, promotes, sells, and distributes opioids in the U.S. and Dallas County. Purdue's opioid drug, OxyContin, is among the most addictive and abused prescription drugs in the history of America. Purdue promotes opioids throughout the United States and in Dallas County.

25. JANSSEN PHARMACEUTICALS, INC. is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey, and may be served through its registered

agent for service of process, CT Corporation System, 1999 Bryan Street, Suite 900, Dallas, TX 75201. JANSSEN PHARMACEUTICALS, INC. is a wholly owned subsidiary of JOHNSON & JOHNSON (J&J), a New Jersey corporation with its principal place of business in New Brunswick, New Jersey, and may be served through its registered agent for service of process, Attention: Legal Department, One Johnson & Johnson Plaza, New Brunswick, NJ 08933. ORTHO-MCNEIL-JANSSEN PHARMACEUTICALS, INC., now known as JANSSEN PHARMACEUTICALS, INC., is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey. JANSSEN PHARMACEUTICA INC., now known as JANSSEN PHARMACEUTICALS, INC., is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey. J&J is the only company that owns more than 10% of Janssen Pharmaceuticals' stock, and corresponds with the FDA regarding Janssen's products. Upon information and belief, J&J controls the sale and development of Janssen Pharmaceuticals' drugs and Janssen's profits inure to J&J's benefit. (Janssen Pharmaceuticals, Inc., Ortho-McNeil-Janssen Pharmaceuticals, Inc., Janssen Pharmaceutica, Inc., and J&J are referred to as "Janssen").

26. Janssen manufactures, promotes, sells, and distributes opioids in the U.S. and in Dallas County.

27. ENDO HEALTH SOLUTIONS INC. is a Delaware corporation with its principal place of business in Malvern, Pennsylvania, and may be served through its registered agent for service of process, The Corporation Trust Company, Corporation Trust Center, 1209 Orange St., Wilmington, DE 19801. ENDO PHARMACEUTICALS, INC. is a wholly-owned subsidiary of Endo Health Solutions Inc. and is a Delaware corporation with its principal place of business in Malvern, Pennsylvania, and may be served through its registered agent for service of process, CT Corporation System, 1999 Bryan Street, Suite 900, Dallas, TX 75201. (Endo Health Solutions Inc.

and Endo Pharmaceuticals, Inc. are referred to as "Endo").

28. Endo develops, markets, and sells opioid drugs in the U.S. and in Dallas County. Endo also manufactures and sells generic opioids in the U.S. and Dallas County, by itself and through its subsidiary, Qualitest Pharmaceuticals, Inc.

29. ABBVIE INC. ("Abbvie") is a Delaware corporation with its principal place of business in North Chicago, Illinois, and may be served through its registered agent for service of process, CT Corporation System, 208 S. LaSalle Street, Suite 814, Chicago, IL 60604. KNOLL PHARMACEUTICAL COMPANY ("Knoll") has been a wholly-owned subsidiary of Abbvie from January 1, 2013. KNOLL PHARMACEUTICAL COMPANY is a New Jersey corporation with its principal place of business in Parsippany, New Jersey, and may be served through its registered agent for service of process, CT Corporation System, 208 S. LaSalle Street, Suite 814, Chicago, IL 60604.

30. Knoll irresponsibly marketed narcotics, such as Vicodin, through whimsical toys and souvenirs and did so to boost the sales of opioids. Taking advantage of the fact that Vicodin was not regulated as a Schedule II controlled substance for many years, and the fact physicians and consumers did not fully appreciate the highly addictive nature of Vicodin, Knoll advertised Vicodin with tag lines such as "The Highest Potency Pain Relief You Can Still Phone In." This tag line came as part and parcel of souvenirs like a "Vicodin" fanny pack and water bottle, both bearing the name of Vicodin, the opioid Knoll was promoting. This irresponsible marketing of a narcotic drug caused doctors and patients to believe Vicodin was safer than it really was, to the detriment of people in Dallas County.

31. Abbvie began manufacturing, developing, promoting, marketing, and selling the opioid drug, Vicodin, in the U.S. and in Dallas County beginning January 1, 2013. On information

and belief, it continues to do so at the time of filing this pleading.

32. ALLERGAN PLC is a public limited company incorporated in Ireland with its principal place of business in Dublin, Ireland. ACTAVIS PLC acquired Allergan plc in March 2015, and the combined company changed its name to Allergan plc in January 2013. Before that, WATSON PHARMACEUTICALS, INC. acquired ACTAVIS, INC. in October 2012, and the combined company changed its name to ALLERGAN FINANCE, LLC as of October 2013. ALLERGAN FINANCE, LLC is a Nevada Corporation with its principal place of business in Parsippany, New Jersey, and may be served through its registered agent for service of process, The Corporation Trust Company of Nevada, 701 S. Carson St., Suite 200, Carson City, NV 89701. WATSON LABORATORIES, INC. is a Nevada corporation with its principal place of business in Corona, California, and is a wholly-owned subsidiary of Allergan plc (f/k/a Actavis, Inc., f/k/a Watson Pharmaceuticals, Inc.), and may be served through its registered agent for service of process, Corporate Creations Network, Inc., 8275 South Eastern Ave., #200, Las Vegas, NV 89123. ACTAVIS PHARMA, INC. (f/k/a Actavis, Inc.) is a Delaware corporation with its principal place of business in New Jersey and was formerly known as WATSON PHARMA, INC, and may be served through its registered agent for service of process, CT Corporation System, 1999 Bryan Street, Suite 900, Dallas, TX 75201. ACTAVIS LLC is a Delaware limited liability company with its principal place of business in Parsippany, New Jersey, and may be served through its registered agent for service of process, Corporate Creations Network, Inc., 3411 Silverside Rd., Tatnall Building, Suite 104, Wilmington, DE 19810. Each of these Defendants is owned by Allergan plc, which uses them to market and sell its drugs in the United States.

33. Upon information and belief, Allergan plc exercises control over these marketing and sales efforts and profits from the sale of Allergan/Actavis products ultimately inure to its

benefit. (Allergan plc, Actavis plc, Actavis, Inc., Allergan Finance, LLC, Actavis LLC, Actavis Pharma, Inc., Watson Pharmaceuticals, Inc., Watson Pharma, Inc., and Watson Laboratories, Inc. are referred to as "Actavis"). Actavis acquired the rights to Kadian from King Pharmaceuticals, Inc. on December 30, 2008 and began marketing Kadian in 2009.

34. Actavis manufactures, promotes, sells, and distributes opioids in the U.S. and in Dallas County.

35. MCKESSON CORPORATION ("McKesson") is a Delaware corporation with its principal place of business in San Francisco, California, and may be served through its registered agent for service of process, CSC - Lawyers Incorporating Service, 211 E. 7th Street, Suite 620, Austin, TX 78701. Upon information and belief, McKesson is a pharmaceutical distributor licensed to do business in Texas. McKesson distributes pharmaceuticals to retail pharmacies and institutional providers in all 50 states, including Texas and Dallas County.

36. CARDINAL HEALTH, INC. ("Cardinal") is an Ohio Corporation with its principal place of business in Dublin, Ohio, and may be served through its registered agent for service of process, CT Corporation System, 4400 Easton Commons, Suite 125, Columbus, OH 43219. Cardinal does substantial business in Texas and, upon information and belief, Cardinal is a pharmaceutical distributor licensed to do business in Texas. Cardinal distributes pharmaceuticals to retail pharmacies and institutional providers to customers in all 50 states, including Texas and Dallas County.

37. AMERISOURCEBERGEN DRUG CORPORATION ("Amerisource") is a Delaware Corporation with its principal place of business in Chesterbrook, Pennsylvania, and may be served through its registered agent for service of process, The Corporation Trust Company, Corporation Trust Center, 1209 Orange St., Wilmington, DE 19801. Amerisource does substantial

business in Texas and, upon information and belief, Amerisource is a pharmaceutical distributor licensed to do business in Texas. Amerisource distributes pharmaceuticals to retail pharmacies and institutional providers to customers in all 50 states, including Texas and Dallas County.

38. DR. RICHARD ANDREWS is an individual residing in Dallas, Dallas County, Texas, and may be served with citation at 3905 Highgrove Drive, Dallas, TX 75220, or wherever he may be found. Dr. Andrews was involved in a "pill mill" operation and charged with conspiracy to distribute controlled substances, including oxycodone, to patients in Dallas County and numerous other counties.[13] Dr. Andrews agreed to the revocation of his medical license on March 3, 2017 after pleading guilty to two felony charges.[14] Dallas County is not, however, seeking damages under claims of medical malpractice or medical professional negligence.

39. DR. THEODORE OKECHUKU is an individual who resided in Dallas, Dallas County, Texas until his sentencing date in October 2015; DR. THEODORE OKECHUKU may be served with citation at FCI Texarkana, Federal Correctional Institution, Register Number: 59813-060, 4001 Leopard Drive, Texarkana, TX 75501, or wherever he may be found. Dr. Okechuku was involved in a "pill mill" operation and charged with, among other things, conspiracy to distribute controlled substances, including hydrocodone, to patients in Dallas County and other counties.[15] Dr. Okechuku lost his medical license as of December 17, 2015.[16] Dallas County is not, however, seeking damages under claims of medical malpractice or medical professional negligence.

40. DR. NICOLAS PADRON is an individual who resided in Garland, Dallas County,

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[13] Department of Justice, "Doctor Who Owned McAllen Medical Clinic in Dallas Pleads Guilty in Pill Mill Case," (January 13, 2017), https://www.justice/usao-ndtx/pr/doctor-who-owned-mcallen-medical-clinic-dallas-pleads-guilty-pill-mill-case.
[14] Texas Medical Board, http://reg.tmb/state/tx/us.com, *last viewed* November 13, 2017.
[15] "Trial for Dallas Doctor Accused of Running Pill Mill," (October 6, 2015), http://www.zenlawfirm.com/Law-Blog/2015/October/Trial-for-Dallas-Doctor-Accused-of-Running-Pill-.aspx.
[16] Texas Medical Board, http://reg.tmb/state/tx/us.com, *last viewed* November 13, 2017.

Texas until his sentencing date in March 2014; DR. NICOLAS PADRON may be served with citation at USP Beaumont, U.S. Penitentiary, Register Number: 44575-177, 6200 Knauth Road, Beaumont, TX 77705, or wherever he may be found. Dr. Padron was involved in a "pill mill" operation and charged with conspiracy to distribute controlled substances, including hydrocodone, to patients in Dallas County and other counties.[17] Dr. Padron agreed to the revocation of his medical license on October 1, 2012 in lieu of further disciplinary proceedings after pleading guilty to one charge of conspiracy to commit healthcare fraud.[18] Dallas County is not, however, seeking damages under claims of medical malpractice or medical professional negligence.

41. The County lacks information sufficient to specifically identify the true names or capacities, whether individual, corporate or otherwise, of Defendants sued herein under the fictitious names DOES 1 through 100 inclusive. The County will amend this Petition to show their true names and capacities if and when they are ascertained. Dallas County is informed and believes, and on such information and belief alleges, that each of the Defendants named as a DOE has engaged in conduct that contributed to cause events and occurrences alleged in this Petition and, as such, shares liability for at least some part of the relief sought herein.

## V. FACTUAL ALLEGATIONS

42. Before the 1990s, generally accepted standards of medical practice dictated that opioids should be used only for short-term acute pain – pain relating to recovery from surgery or for cancer or palliative (end-of-life) care. Using opioids for chronic pain was discouraged or even prohibited because there was a lack of evidence that opioids improved patients' ability to overcome pain and function. Instead the evidence demonstrated that patients developed tolerance

[17] "Garland Doctor, other 'Dealers' Sentenced in Dallas 'Pill Mill' Case," (October 29, 2014), http://starlocalmedia.com/rowlettlakeshoretimes/garland-doctor-other-d...llas-pill-mill-case/article_d53be5fc-5fbc-11e4-9186-af37156f06a3.html.
[18] Texas Medical Board, http://reg.tmb/state/tx/us.com, *last viewed* November 13, 2017.

to opioids over time, which increased the risk of addiction and other side effects.

43. Defendants dramatically changed doctors' views regarding opioids through a well-funded deceptive marketing scheme. Each Defendant used direct marketing and unbranded advertising disseminated by seemingly independent third parties to spread false and deceptive statements about the risks and benefits of long-term opioid use.

## A. Defendants Used Multiple Avenues To Disseminate their False and Deceptive Statements about Opioids.

44. Defendants spread their false and deceptive statements by (1) marketing their branded opioids directly to doctors treating patients residing in Dallas County and the Dallas County patients themselves and (2) deploying so-called unbiased and independent third parties to Dallas County.

### 1. Defendants Spread and Continue to Spread Their False and Deceptive Statements Through Direct Marketing of Their Branded Opioids.

45. Defendants' direct marketing of opioids generally proceeded on two tracks. First, each Defendant conducted advertising campaigns touting the purported benefits of their branded drugs. For example, Defendants spent more than $14 million on medical journal advertising of opioids in 2011, nearly triple what they spent in 2001, including $8.3 million by Purdue, $4.9 million by Janssen, and $1.1 million by Endo.

46. A number of Defendants' branded ads deceptively portrayed the benefits of opioids for chronic pain. For example, Endo distributed and made available on its website, www.opana.com, a pamphlet promoting Opana ER with photographs depicting patients with physically demanding jobs like a construction worker and chef, implying that the drug would provide long-term pain-relief and functional improvement. Purdue also ran a series of ads, called "pain vignettes," for OxyContin in 2012 in medical journals. These ads featured chronic pain

patients and recommended OxyContin for each. One ad described a "54-year-old writer with osteoarthritis of the hands" and implied that OxyContin would help the writer work more effectively. Pursuant to a settlement agreement, Endo and Purdue agreed in late 2015 and 2016 to halt these misleading representations in New York, but they may continue to disseminate them in Texas.

47. Second, each Defendant promoted the use of opioids for chronic pain through "detailers" – sales representatives who visited individual doctors and medical staff in their offices – and small-group speaker programs. Defendants devoted massive resources to direct sales contacts with doctors. In 2014 alone, Defendants spent $168 million on detailing branded opioids to doctors, including $108 million by Purdue, $34 million by Janssen, $10 million by Endo, and $2 million by Actavis. This amount is twice as much as Defendants spent on detailing in 2000.

48. Defendants also identified doctors to serve, for payment, on their speakers' bureaus and to attend programs with speakers and meals paid for by Defendants. These speaker programs provided: (1) an incentive for doctors to prescribe a particular opioid (so they might be selected to promote the drug); (2) recognition and compensation for the doctors selected as speakers; and (3) an opportunity to promote the drug through the speaker to his or her peers. These speakers gave the false impression that they were providing unbiased and medically accurate presentations when they were, in fact, presenting a script prepared by Defendants. On information and belief, these presentations conveyed misleading information, omitted material information, and failed to correct Defendants' prior misrepresentations about the risks and benefits of opioids.

49. Defendants employed the same marketing plans, strategies, and messages in and around Dallas County, Texas as they did nationwide. Across the pharmaceutical industry, "core message" development is funded and overseen on a national basis by corporate headquarters. This

comprehensive approach ensures that Defendants' messages are accurately and consistently delivered across marketing channels and in each sales territory. Defendants consider this high level of coordination and uniformity crucial to successfully marketing their drugs.

**2. Defendants Used a Diverse Group of Seemingly Independent Third Parties to Spread False and Deceptive Statements about the Risks and Benefits of Opioids.**

50. Defendants also deceptively marketed opioids in and around Dallas County through unbranded advertising – *i.e.*, advertising that promotes opioid use generally but does not name a specific opioid. This advertising was ostensibly created and disseminated by independent third parties. But by funding, directing, reviewing, editing, and distributing this unbranded advertising, Defendants controlled the deceptive messages disseminated by these third parties and acted in concert with them to falsely and misleadingly promote opioids for treating chronic pain.

51. Unbranded advertising also avoided regulatory scrutiny because Defendants did not have to submit it to the FDA, and therefore it was not reviewed by the FDA.

52. Defendants' deceptive unbranded marketing often contradicted their branded materials reviewed by the FDA. For example, Endo's unbranded advertising contradicted its concurrent, branded advertising for Opana ER:

| **Pain: Opioid Therapy (Unbranded)** | **Opana ER Advertisement (Branded)** |
|---|---|
| "People who take opioids **as prescribed usually do not become addicted**." | "All patients treated with opioids require careful monitoring for signs of abuse and addiction, since **use of opioid analgesic products carries the risk of addiction even under appropriate medical use**." |

**a. Key Opinion Leaders (KOLs)**

53. Defendants spoke through a small circle of doctors who, upon information and belief, were selected, funded, and elevated by Defendants because their public positions supported

using opioids to treat chronic pain. These doctors became known as "key opinion leaders" or "KOLs."

54. Defendants paid KOLs to serve as consultants or on their advisory boards and to give talks or present CMEs, and their support helped these KOLs become respected industry experts. As they rose to prominence, these KOLs touted the benefits of opioids to treat chronic pain, repaying Defendants by advancing their marketing goals. KOLs' professional reputations became dependent on continuing to promote a pro-opioid message, even in activities that were not directly funded by Defendants.

55. KOLs have written, consulted on, edited, and lent their names to books and articles, and given speeches and CMEs supportive of chronic opioid therapy. Defendants created opportunities for KOLs to participate in research studies Defendants suggested or chose and then cited and promoted favorable studies or articles by their KOLs. By contrast, Defendants did not support, acknowledge, or disseminate publications of doctors unsupportive or critical of chronic opioid therapy.

56. Defendants' KOLs also served on committees that developed treatment guidelines that strongly encourage using opioids to treat chronic pain, and on the boards of pro-opioid advocacy groups and professional societies that develop, select, and present CMEs. Defendants were able to direct and exert control over each of these activities through their KOLs.

57. Pro-opioid doctors are one of the most important avenues that Defendants use to spread their false and deceptive statements about the risks and benefits of long-term opioid use. Defendants know that doctors rely heavily and less critically on their peers for guidance, and KOLs provide the false appearance of unbiased and reliable support for chronic opioid therapy.

58. Defendants utilized many KOLs, including many of the same ones. Two of the most

prominent are described below.

**1. Russell Portenoy**

59. Dr. Russell Portenoy, former Chairman of the Department of Pain Medicine and Palliative Care at Beth Israel Medical Center in New York, is one example of a KOL who Defendants identified and promoted to further their marketing campaign. Dr. Portenoy received research support, consulting fees, and honoraria from Endo, Janssen, and Purdue (among others), and was a paid consultant to Purdue.

60. Dr. Portenoy was instrumental in opening the door for the regular use of opioids to treat chronic pain. He served on the American Pain Society ("APS")/American Academy of Pain Medicine ("AAPM") Guidelines Committees, which endorsed the use of opioids to treat chronic pain, first in 1997 and again in 2009. He was also a member of the board of the American Pain Foundation ("APF"), an advocacy organization almost entirely funded by Defendants.

61. Dr. Portenoy also made frequent media appearances promoting opioids. He appeared on *Good Morning America* in 2010 to discuss using opioids long-term to treat chronic pain. On this widely-watched program, broadcast in Texas and across the country, Dr. Portenoy claimed: "Addiction, when treating pain, is distinctly uncommon. If a person does not have a history, a personal history, of substance abuse, and does not have a history in the family of substance abuse, and does not have a very major psychiatric disorder, most doctors can feel very assured that that person is not going to become addicted."[19]

62. Dr. Portenoy later admitted that he "gave innumerable lectures in the late 1980s and '90s about addiction that weren't true."[20] These lectures falsely claimed that less than 1% of

[19] Good Morning America television broadcast, ABC News (Aug. 30, 2010).

[20] Thomas Catan & Evan Perez, *A Pain-Drug Champion Has Second Thoughts*, WALL ST. J., Dec. 17, 2012.

patients would become addicted to opioids. According to Dr. Portenoy, because the primary goal was to "destigmatize" opioids, he and other doctors promoting them overstated their benefits and glossed over their risks. Dr. Portenoy also conceded that "[d]ata about the effectiveness of opioids does not exist."[21] Portenoy candidly stated: "Did I teach about pain management, specifically about opioid therapy, in a way that reflects misinformation? Well…I guess I did."[22]

**2. Lynn Webster**

63. Another KOL, Dr. Lynn Webster, was the co-founder and Chief Medical Director of Lifetree Clinical Research, an otherwise unknown pain clinic in Salt Lake City, Utah. Dr. Webster was President in 2013 and is a current board member of AAPM, a Front Group that ardently supports chronic opioid therapy. He is a Senior Editor of *Pain Medicine*, the same journal that published Endo special advertising supplements touting Opana ER. Dr. Webster authored numerous CMEs sponsored by Endo and Purdue while he was receiving significant funding from Defendants.

64. In 2011, Dr. Webster presented a program via webinar sponsored by Purdue titled, *Managing Patient's Opioid Use: Balancing the Need and the Risk*. Dr. Webster recommended using risk screening tools, such as urine testing and patient agreements as a way to prevent "overuse of prescriptions" and "overdose deaths," which was available to and was intended to reach doctors treating Dallas County residents.

65. Dr. Webster also was a leading proponent of the concept of "pseudoaddiction," the notion that addictive behaviors should be seen not as warnings, but as indications of undertreated pain. In Dr. Webster's description, the only way to differentiate the two was to *increase* a patient's dose of opioids. As he and his co-author wrote in a book entitled *Avoiding Opioid Abuse While*

[21] *Id.*
[22] *Id.*

*Managing Pain* (2007), a book that is still available online, when faced with signs of aberrant behavior, increasing the dose "in most cases . . . should be the clinician's first response." Endo distributed this book to doctors. Years later, Dr. Webster reversed himself, acknowledging that "[pseudoaddiction] obviously became too much of an excuse to give patients more medication."[23]

**b. Front Groups**

66. Defendants entered into arrangements with seemingly unbiased and independent patient and professional organizations to promote opioids for treating chronic pain. Under Defendants' direction and control, these "Front Groups" generated treatment guidelines, unbranded materials, and programs that favored chronic opioid therapy. They also assisted Defendants by responding to negative articles, by advocating against regulatory changes that would limit prescribing opioids in accordance with the scientific evidence, and by conducting outreach to vulnerable patient populations targeted by Defendants.

67. These Front Groups depended on Defendants for funding and, in some cases, for survival. Defendants also exercised control over programs and materials created by these groups by collaborating on, editing, and approving their content, and by funding their dissemination. In doing so, Defendants made sure these Groups would generate only the messages Defendants wanted to distribute. Even so, the Front Groups held themselves out as independent and as serving the needs of their members – whether patients suffering from pain or doctors treating those patients.

68. Defendants Endo, Janssen, and Purdue utilized many Front Groups, including many of the same ones. Several of the most prominent are described below, but there are many others, including the American Pain Society ("APS"), American Geriatrics Society ("AGS"), the Federation of State Medical Boards ("FSMB"), American Chronic Pain Association ("ACPA"),

[23] John Fauber & Ellen Gabler, *Networking Fuels Painkiller Boom*, MILWAUKEE WISC. J. SENTINEL (Feb. 19, 2012).

American Society of Pain Education ("ASPE"), National Pain Foundation ("NPF") and Pain & Policy Studies Group ("PPSG").

**1. American Pain Foundation ("APF")**

69. The most prominent of Defendants' Front Groups was APF, which received more than $10 million in funding from opioid manufacturers from 2007 until it closed its doors in May 2012. Endo alone provided more than half that funding; Purdue was next at $1.7 million.

70. In 2009 and 2010, more than 80% of APF's operating budget came from pharmaceutical industry sources. Including industry grants for specific projects, APF received about $2.3 million from industry sources out of total income of about $2.85 million in 2009; its budget for 2010 projected receipts of roughly $2.9 million from drug companies, out of total income of about $3.5 million. By 2011, APF was entirely dependent on incoming grants from defendants Purdue, Endo, and others to avoid using its line of credit. As one of its board members, Russell Portenoy, explained the lack of funding diversity was one of the biggest problems at APF.

71. APF issued education guides for patients, reporters, and policymakers that touted the benefits of opioids for chronic pain and trivialized their risks, particularly the risk of addiction. APF also engaged in a significant multimedia campaign – through radio, television, and the internet – to educate patients about their "right" to pain treatment, namely opioids. All of the programs and materials were available nationally and were intended to reach patients and consumers in Dallas County.

**2. American Academy of Pain Medicine ("AAPM")**

72. The American Academy of Pain Medicine, with the assistance, prompting, involvement, and funding of Defendants, issued treatment guidelines and sponsored and hosted medical education programs essential to Defendants' deceptive marketing of chronic opioid

therapy.

73. AAPM received over $2.2 million in funding since 2009 from opioid manufacturers. AAPM maintained a corporate relations council, whose members paid $25,000 per year (on top of other funding) to participate. The benefits included allowing members to present educational programs at off-site dinner symposia in connection with AAPM's marquee event – its annual meeting held in Palm Springs, California, or other resort locations. AAPM describes the annual event as an "exclusive venue" for offering education programs to doctors. Membership in the corporate relations council also allows drug company executives and marketing staff to meet with AAPM executive committee members in small settings. Defendants Endo, Purdue, and Actavis were members of the council and presented deceptive programs to doctors who attended this annual event.

74. AAPM is viewed internally by Endo as "industry friendly," with Endo advisors and speakers among its active members. Endo attended AAPM conferences, funded its CMEs, and distributed its publications. The conferences sponsored by AAPM heavily emphasized sessions on opioids – 37 out of roughly 40 at one conference alone. AAPM's presidents have included top industry-supported KOLs Perry Fine, Russell Portenoy, and Lynn Webster. Dr. Webster was even elected president of AAPM while under a DEA investigation. Another past AAPM president, Dr. Scott Fishman, stated that he would place the organization "at the forefront" of teaching that "the risks of addiction are . . . small and can be managed."[24]

75. AAPM's staff understood they and their industry funders were engaged in a common task. Defendants were able to influence AAPM through both their significant and regular

[24] Interview by Paula Moyer with Scott M. Fishman, M.D., Professor of Anesthesiology and Pain Medicine, Chief of the Division of Pain Medicine, Univ. of Cal., Davis (2005), http://www.medscape.org/viewarticle/500829.

funding and the leadership of pro-opioid KOLs within the organization.

76. In 1997, AAPM and the American Pain Society jointly issued a consensus statement, *The Use of Opioids for the Treatment of Chronic Pain*, which endorsed opioids to treat chronic pain and claimed there was a low risk that patients would become addicted to opioids. The co-author of the statement, Dr. Haddox, was at the time a paid speaker for Purdue. Dr. Portenoy was the sole consultant. The consensus statement remained on AAPM's website until 2011, and was taken down from AAPM's website only after a doctor complained, though it still lingers on the internet elsewhere.

77. AAPM and APS issued their own guidelines in 2009 ("AAPM/APS Guidelines") and continued to recommend using opioids to treat chronic pain. Fourteen of the 21 panel members who drafted the AAPM/APS Guidelines, including KOLs Dr. Portenoy and Dr. Perry Fine of the University of Utah, received support from Janssen, Endo, and Purdue.

78. The 2009 Guidelines promote opioids as "safe and effective" for treating chronic pain, despite acknowledging limited evidence, and conclude that the risk of addiction is manageable for patients regardless of past abuse histories. One panel member, Dr. Joel Saper, Clinical Professor of Neurology at Michigan State University and founder of the Michigan Headache & Neurological Institute, resigned from the panel because he was concerned the 2009 Guidelines were influenced by contributions that drug companies, including Defendants, made to the sponsoring organizations and committee members. These AAPM/APS Guidelines have been a particularly effective channel of deception and have influenced not only treating physicians, but also the body of scientific evidence on opioids. The Guidelines have been cited 732 times in academic literature, were disseminated in and around Dallas County during the relevant time period, are still available online, and were reprinted in the *Journal of Pain*.

## B. Defendants' Marketing Scheme Misrepresented the Risks and Benefits of Opioids.

79. To convince doctors treating residents in Dallas County and Dallas County patients that opioids can and should be used to treat chronic pain, Defendants had to convince them that long-term opioid use is both safe and effective. Knowing they could do so only by deceiving those doctors and patients about the risks and benefits of long-term opioid use, Defendants made claims that were not supported by, or were contrary to, the scientific evidence. Even though pronouncements by and guidance from the FDA and the CDC based on that evidence confirm that their claims were false and deceptive, Defendants have not corrected them, or instructed their KOLs or Front Groups to correct them, and continue to spread them today.

## C. Defendants Falsely Trivialized or Failed to Disclose the Known Risks of Long-Term Opioid Use.

80. To convince doctors and patients that opioids are safe, Defendants deceptively trivialized and failed to disclose the risks of long-term opioid use, particularly the risk of addiction, through a series of misrepresentations that have been conclusively debunked by the FDA and CDC. These misrepresentations – which are described below – reinforced each other and created the dangerously misleading impression that: (1) starting patients on opioids was low-risk because most patients would not become addicted, and because those who were at greatest risk of addiction could be readily identified and managed; (2) patients who displayed signs of addiction probably were not addicted and, in any event, could easily be weaned from the drugs; (3) the use of higher opioid doses, which many patients need to sustain pain relief as they develop tolerance to the drugs, do not pose special risks; and (4) abuse-deterrent opioids both prevent overdose and are inherently less addictive. Defendants have not only failed to correct these misrepresentations, they continue to make them today.

81. ***First***, Defendants falsely claimed the risk of addiction is low and unlikely to

develop when opioids are prescribed, as opposed to obtained illicitly; and failed to disclose the greater risk of addiction with prolonged use of opioids. For example:

a. Actavis's predecessor caused a patient education brochure to be distributed in 2007 claiming opioid addiction is possible, but "less likely if you have never had an addiction problem." Upon information and belief, based on Actavis's acquisition of its predecessor's marketing materials along with the rights to Kadian, Actavis continued to use this brochure in 2009 and beyond;

b. Purdue sponsored APF's *Treatment Options: A Guide for People Living with Pain* (2007), which instructed that addiction is rare and limited to extreme cases of unauthorized dose escalations, obtaining duplicative opioid prescriptions from multiple sources, or theft. This publication is still available online;

c. Endo sponsored a website, Painknowledge.com, which claimed in 2009 that "[p]eople who take opioids as prescribed usually do not become addicted." Another Endo website, PainAction.com, stated "Did you know? Most chronic pain patients do not become addicted to the opioid medications that are prescribed for them;"

d. Endo distributed a pamphlet with the Endo logo entitled *Living with Someone with Chronic Pain*, which stated that: "Most health care providers who treat people with pain agree that most people do not develop an addiction problem." A similar statement appeared on the Endo website, www.opana.com;

e. Janssen reviewed, edited, approved, and distributed a patient education guide entitled *Finding Relief: Pain Management for Older Adults* (2009), which described as "myth" the claim that opioids are addictive, and asserted as fact that "[m]any studies show that opioids are rarely addictive when used properly for the management of chronic pain;"

f. Janssen currently runs a website, Prescriberesponsibly.com (last updated July 2, 2015), which claims that concerns about opioid addiction are "overestimated;"

g. Purdue sponsored APF's *A Policymaker's Guide to Understanding Pain & Its Management* – which claims that less than 1% of children prescribed opioids will become addicted and that pain is undertreated due to "misconceptions about opioid addiction[]." This publication is still available online; and

h. Detailers for Purdue, Endo, and Janssen in and around Dallas County

> minimized or omitted any discussion with doctors of the risk of addiction; misrepresented the potential for opioid abuse with purportedly abuse-deterrent formulations; and routinely did not correct the misrepresentations noted above.

82. These claims contradict longstanding scientific evidence, as the FDA and CDC have conclusively declared. As noted in the 2016 CDC Guideline endorsed by the FDA, there is "extensive evidence" of the "possible harms of opioids (including opioid use disorder [an alternative term for opioid addiction])."[25] The guideline points out that "[o]pioid pain medication use presents serious risks, including…opioid use disorder" and that "continuing opioid therapy for 3 months substantially increases risk for opioid use disorder."[26]

83. The FDA further exposed the falsity of Defendants' claims about the low risk of addiction when it announced changes to the labels for ER/LA opioids in 2013 and for IR opioids in 2016. In its announcements, the FDA discussed the risks related to opioid use and that IR opioids are associated with "persistent abuse, addiction, overdose mortality, and risk of NOWS [neonatal opioid withdrawal syndrome]."[27]

84. According to the FDA, because of the risks associated with long-term opioid use, including "the serious risk of addiction, abuse, misuse, overdose, and death,"[28] opioids should be "reserved for pain severe enough to require opioid treatment and for which alternative treatment options (e.g., non-opioid analgesics or opioid combination products, as appropriate) are inadequate or not tolerated."[29]

85. The warnings on Defendants' own FDA-approved drug labels caution that opioids

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[25] *CDC Guideline for Prescribing Opioids for Chronic Pain – United States 2016*, Centers for Disease Control and Prevention (Mar. 18, 2016).
[26] *Id.*
[27] *FDA Announcement of Enhanced Warnings for Immediate-Release Opioid Pain Medications Related to Risks of Misuse, Abuse, Addiction, Overdose and Death*, Federal Drug Administration (Mar. 22, 2016).
[28] *Id.*
[29] *Id.*

"exposes users to risks of addiction, abuse and misuse, which can lead to overdose and death"[30] and that addiction "can occur in patients appropriately prescribed"[31] opioids.

86. ***Second***, Defendants falsely instructed doctors and patients that signs of addiction are actually signs of undertreated pain and should be treated by prescribing more opioids. Defendants called this phenomenon "pseudoaddiction" – a term coined by Dr. David Haddox, who went to work for Purdue, and popularized by Dr. Russell Portenoy, a KOL for Endo, Janssen, and Purdue – and claimed that pseudoaddiction is substantiated by scientific evidence. For example:

a. Purdue sponsored *Responsible Opioid Prescribing* (2007), which taught that behaviors such as "requesting drugs by name," "demanding or manipulative behavior," seeing more than one doctor to obtain opioids, and hoarding, are all signs of pseudoaddiction, rather than true addiction. Responsible Opioid Prescribing remains for sale online. The 2012 edition continues to teach that pseudoaddiction is real;

b. Janssen sponsored, funded, and edited the *Let's Talk Pain* website, which in 2009 stated: "pseudoaddiction . . . refers to patient behaviors that may occur when pain is under-treated . . . . Pseudoaddiction is different from true addiction because such behaviors can be resolved with effective pain management;"

c. Endo sponsored a National Initiative on Pain Control (NIPC) CME program in 2009 titled *Chronic Opioid Therapy: Understanding Risk While Maximizing Analgesia*, which promoted pseudoaddiction by teaching that a patient's aberrant behavior was the result of untreated pain. Endo substantially controlled NIPC by funding NIPC projects; developing, specifying, and reviewing content; and distributing NIPC materials;

d. Purdue published a pamphlet in 2011 entitled *Providing Relief, Preventing Abuse*, which described pseudoaddiction as a concept that "emerged in the literature" to describe the inaccurate interpretation of [drug-seeking behaviors] in patients who have pain that has not been effectively treated;" and

e. Purdue sponsored a CME program entitled *Path of the Patient, Managing Chronic Pain in Younger Adults at Risk for Abuse*. In a role play, a

[30] *See, e.g.*, OxyContin label and insert at *OxyContin.com.*
[31] *Id.*

> chronic pain patient with a history of drug abuse tells his doctor that he is taking twice as many hydrocodone pills as directed. The narrator notes that because of pseudoaddiction, the doctor should not assume the patient is addicted even if he persistently asks for a specific drug, seems desperate, hoards medicine, or "overindulges in unapproved escalating doses." The doctor treats this patient by prescribing a high-dose, long-acting opioid.

87. The 2016 CDC Guideline rejects the concept of pseudoaddiction. The Guideline nowhere recommends that opioid dosages be increased if a patient is not experiencing pain relief. To the contrary, the Guideline explains that "[p]atients who do not experience clinically meaningful pain relief early in treatment…are unlikely to experience pain relief with longer-term use,"[32] and that physicians should "reassess[] pain and function within 1 month"[33] in order to decide whether to "minimize risks of long-term opioid use by discontinuing opioids"[34] because the patient is "not receiving a clear benefit."[35]

88. ***Third,*** Defendants falsely instructed doctors and patients that addiction risk screening tools, patient contracts, urine drug screens, and similar strategies allow them to reliably identify and safely prescribe opioids to patients predisposed to addiction. These misrepresentations were especially insidious because Defendants aimed them at general practitioners and family doctors who lack the time and expertise to closely manage higher-risk patients. Defendants' misrepresentations made these doctors feel more comfortable prescribing opioids to their patients, and patients more comfortable starting opioid therapy for chronic pain. For example:

a. Endo paid for a 2007 supplement in the *Journal of Family Practice* written by a doctor who became a member of Endo's speakers' bureau in 2010. The supplement, entitled *Pain Management Dilemmas in Primary Care: Use of Opioids*, emphasized the effectiveness of screening tools, claiming that patients at high risk of addiction could safely receive chronic opioid therapy using a "maximally structured approach" involving toxicology screens and

[32] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[33] *Id.*
[34] *Id.*
[35] *Id.*

pill counts;

b. Purdue sponsored a 2011 webinar, *Managing Patient's Opioid Use: Balancing the Need and Risk*, which claimed that screening tools, urine tests, and patient agreements prevent "overuse of prescriptions" and "overdose deaths;" and

c. As recently as 2015, Purdue has represented in scientific conferences that "bad apple" patients – and not opioids – are the source of the addiction crisis and that once those "bad apples" are identified, doctors can safely prescribe opioids without causing addiction.

89. Once again, the 2016 CDC Guideline confirms these representations are false. The Guideline notes that there are no studies assessing the effectiveness of risk mitigation strategies – such as screening tools, patient contracts, urine drug testing, or pill counts – widely believed by doctors to detect and deter outcomes related to addiction and overdose.[36] As a result, the Guideline recognizes that doctors should not overestimate the risk screening tools for classifying patients as high or low risk for opioid addiction because they are insufficient to rule out the risks of long-term opioid therapy.[37]

90. ***Fourth***, to underplay the risk and impact of addiction and make doctors feel more comfortable starting patients on opioids, Defendants falsely claimed that opioid dependence can easily be addressed by tapering and that opioid withdrawal is not a problem thereby failing to disclose the increased difficulty of stopping opioids after long-term use.

91. For example, a CME sponsored by Endo, entitled *Persistent Pain in the Older Adult*, claimed that withdrawal symptoms can be avoided by tapering a patient's opioid dose by 10%-20% for 10 days. And Purdue sponsored APF's *A Policymaker's Guide to Understanding Pain & Its Management*, which claimed that "[s]ymptoms of physical dependence can often be ameliorated by gradually decreasing the dose of medication during discontinuation."

[36] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[37] *See id.*

92. Defendants deceptively minimized the significant symptoms of opioid withdrawal, which, as explained in the 2016 CDC Guideline, include drug cravings, anxiety, insomnia, abdominal pain, vomiting, diarrhea, sweating, tremor, tachycardia (rapid heartbeat), spontaneous abortion and premature labor in pregnant women, and the unmasking of anxiety, depression, and addiction – and grossly understated the difficulty of tapering, particularly after long-term opioid use.

93. Yet the 2016 CDC Guideline recognizes that the duration of opioid use and the dosage of opioids prescribed should be limited to "minimize the need to taper opioids to prevent distressing or unpleasant withdrawal symptoms,"[38] because "physical dependence on opioids is an expected physiologic response in patients exposed to opioids for more than a few days."[39] The Guideline further states that "tapering opioids can be especially challenging after years on high dosages because of physical and psychological dependence"[40] and highlights the difficulties, including the need to carefully identify "a taper slow enough to minimize symptoms and signs of opioid withdrawal"[41] and pausing and restarting tapers depending on the patient's response.

94. The CDC also acknowledges the lack of any "high-quality studies comparing the effectiveness of different tapering protocols for use when opioid dosage is reduced or opioids are discontinued."[42]

95. ***Fifth***, Defendants falsely claimed that doctors and patients could increase opioid dosages indefinitely without added risk and failed to disclose the greater risks to patients at higher dosages. The ability to escalate dosages was critical to Defendants' efforts to market opioids for

[38] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[39] *Id.*
[40] *Id.*
[41] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[42] *Id.*

long-term use to treat chronic pain because, absent this misrepresentation, doctors would have abandoned treatment when patients built up tolerance and lower dosages did not provide pain relief. For example:

a. Actavis's predecessor created a patient brochure for Kadian in 2007 that stated, "Over time, your body may become tolerant of your current dose. You may require a dose adjustment to get the right amount of pain relief. This is not addiction." Upon information and belief, based on Actavis's acquisition of its predecessor's marketing materials along with the rights to Kadian, Actavis continued to use these materials in 2009 and beyond;

b. Purdue sponsored *APF's Treatment Options: A Guide for People Living with Pain* (2007), which claims that some patients "need" a larger dose of an opioid, regardless of the dose currently prescribed. The guide stated that opioids have "no ceiling dose" and are therefore the most appropriate treatment for severe pain. This guide is still available for sale online;

c. Endo sponsored a website, painknowledge.com, which claimed in 2009 that opioid dosages may be increased until "you are on the right dose of medication for your pain;"

d. Endo distributed a pamphlet edited by a KOL entitled *Understanding Your Pain: Taking Oral Opioid Analgesics*, which was available during the time period of this Complaint on Endo's website. In Q&A format, it asked "If I take the opioid now, will it work later when I really need it?" The response is, "The dose can be increased. . . . You won't 'run out' of pain relief;"

e. Janssen sponsored a patient education guide entitled *Finding Relief: Pain Management for Older Adults* (2009), which was distributed by its sales force. This guide listed dosage limitations as "disadvantages" of other pain medicines but omitted any discussion of risks of increased opioid dosages;

f. Purdue's In the Face of Pain website promotes the notion that if a patient's doctor does not prescribe what, in the patient's view, is a sufficient dosage of opioids, he or she should find another doctor who will;

g. Purdue sponsored APF's *A Policymaker's Guide to Understanding Pain & Its Management*, which taught that dosage escalations are "sometimes necessary," even unlimited ones, but did not disclose the risks from high opioid dosages. This publication is still available online;

h. Purdue sponsored a CME entitled *Overview of Management Options* that is still available for CME credit. The CME was edited by a KOL and taught that NSAIDs and other drugs, but not opioids, are unsafe at high dosages;

and

i. Purdue presented a 2015 paper at the College on the Problems of Drug Dependence, the "the oldest and largest organization in the US dedicated to advancing a scientific approach to substance use and addictive disorders," challenging the correlation between opioid dosage and overdose.

96. These claims conflict with the scientific evidence, as confirmed by the FDA and CDC. As the CDC explains in its 2016 Guideline, the "[b]enefits of high-dose opioids for chronic pain are not established"[43] while the "risks for serious harms related to opioid therapy increase at higher opioid dosage."[44]

97. More specifically, the CDC explains that "there is now an established body of scientific evidence showing that overdose risk is increased at higher opioid dosages."[45] Similarly, there is an "increased risk for opioid use disorder, respiratory depression, and death at higher dosages."[46] That is why the CDC advises doctors to avoid increasing dosages above 90 morphine milligram equivalents per day.

98. The 2016 CDC Guideline reinforces earlier findings announced by the FDA. In 2013, the FDA acknowledged that available data suggested that increasing the opioid dosage likewise increased certain adverse events. For example, the FDA noted that studies suggest a positive association between high-dose opioid use and overdoses.

99. ***Finally***, Defendants' deceptive marketing of the so-called abuse-deterrent properties of some of their opioids has created false impressions that these opioids can curb addiction and abuse.

100. More specifically, Defendants have made misleading claims about the ability of

[43] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[44] *Id.*
[45] *Id.*
[46] *Id.*

their so-called abuse-deterrent opioid formulations to deter addiction and overdose. For example, Endo's advertisements for the 2012 reformulation of Opana ER claimed that it was designed to be crush resistant in a way that suggested it was more difficult to misuse the product. This claim was false.

101. The FDA warned in a 2013 letter that there was no evidence Endo's design would provide a reduction in oral, intranasal or intravenous use.[47] Moreover, Endo's own studies, which it failed to disclose, showed that Opana ER could still be ground and chewed.

102. In a 2016 settlement with the State of New York, Endo agreed not to make statements in New York that Opana ER was designed to be or is crush resistant. The State found those statements false and deceptive because there was no difference in the ability to extract the narcotic from Opana ER.

103. Similarly, the 2016 CDC Guideline states that no studies support the notion that "abuse-deterrent technologies [are] a risk mitigation strategy for deterring or preventing abuse,"[48] noting that the technologies – even when they work – "do not prevent opioid abuse through oral intake, the most common route of opioid abuse, and can still be abused by non-oral routes."[49]

104. These numerous, long-standing misrepresentations of the risks of long-term opioid use spread by Defendants successfully convinced doctors and patients to discount those risks.

**D. Defendants Grossly Overstated the Benefits of Chronic Opioid Therapy.**

105. To convince doctors and patients that opioids should be used to treat chronic pain, Defendants had to persuade them that there was a significant benefit to long-term opioid use. But as the 2016 CDC Guideline makes clear, there is "insufficient evidence to determine the long-

[47] *See FDA Statement: Original Opana ER Relisting Determination* (May 10, 2013).
[48] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[49] *Id.*

term benefits of opioid therapy for chronic pain."[50]

106. In fact, the CDC found no evidence showing "a long-term benefit of opioids in pain and function versus no opioids for chronic pain with outcomes examined at least 1 year later (with most placebo-controlled randomized trials ≤ 6 weeks in duration)"[51] and that other treatments were more or equally beneficial and less harmful than long-term opioid use. The FDA, too, has recognized the lack of evidence to support long-term opioid use.

107. In 2013, the FDA stated that it was unaware of any studies demonstrating the safety and efficacy of opioids for long-term use.[52] Despite this lack of studies, Defendants falsely and misleadingly touted the benefits of long-term opioid use and suggested that these benefits were supported by scientific evidence. Not only have Defendants failed to correct these false and deceptive claims, they continue to make them today. For example:

a. Actavis distributed an advertisement that claimed that the use of Kadian to treat chronic pain would allow patients to return to work, relieve "stress on your body and your mental health," and help patients enjoy their lives;

b. Endo distributed advertisements that claimed that the use of Opana ER for chronic pain would allow patients to perform demanding tasks like construction work or work as a chef and portrayed seemingly healthy, unimpaired subjects;

c. Janssen sponsored and edited a patient education guide entitled *Finding Relief: Pain Management for Older Adults* (2009) – which states as "a fact" that "opioids may make it easier for people to live normally." The guide lists expected functional improvements from opioid use, including sleeping through the night, returning to work, recreation, sex, walking, and climbing stairs;

d. Purdue ran a series of advertisements for OxyContin in 2012 in medical journals entitled "pain vignettes," which were case studies featuring patients with pain conditions persisting over several months and recommending OxyContin for them. The ads implied that OxyContin

[50] *Id.*
[51] *Id.*
[52] Letter from Janet Woodcock, M.D., Dir., Ctr. For Drug Eval. & Res., to Andrew Kolodny, M.D., Pres. Physicians for Responsible Opioid Prescribing, Re Docket No. FDA-2012-P-0818 (Sept. 10, 2013).

improves patients' function;

e. *Responsible Opioid Prescribing* (2007), sponsored and distributed by Endo and Purdue, taught that relief of pain by opioids, by itself, improved patients' function. The book remains for sale online;

f. Purdue sponsored APF's *Treatment Options: A Guide for People Living with Pain* (2007), which counseled patients that opioids "give [pain patients] a quality of life we deserve." The guide was available online until APF shut its doors in 2012;

g. Endo's NIPC website *painknowledge.com* claimed in 2009 that with opioids, "your level of function should improve; you may find you are now able to participate in activities of daily living, such as work and hobbies, that you were not able to enjoy when your pain was worse." Elsewhere, the website touted improved quality of life (as well as "improved function") as benefits of opioid therapy. The grant request that Endo approved for this project specifically indicated NIPC's intent to make misleading claims about function, and Endo closely tracked visits to the site;

h. Endo was the sole sponsor, through NIPC, of a series of CMEs titled *Persistent Pain in the Older Patient*, which claimed that chronic opioid therapy has been "shown to reduce pain and improve depressive symptoms and cognitive functioning." The CME was disseminated via webcast;

i. Janssen sponsored, funded, and edited a website, *Let's Talk Pain*, in 2009, which featured an interview edited by Janssen claiming that opioids allowed a patient to "continue to function." This video is still available today on YouTube;

j. Purdue sponsored the development and distribution of APF's *A Policymaker's Guide to Understanding Pain & Its Management*, which claimed that "multiple clinical studies" have shown that opioids are effective in improving daily function, psychological health, and health-related quality of life for chronic pain patients." The Policymaker's Guide was originally published in 2011 and is still available online today; and

k. Purdue's, Endo's, and Janssen's sales representatives have conveyed and continue to convey the message that opioids will improve patient function.

108. These claims find no support in the scientific literature. Most recently, the 2016 CDC Guideline, approved by the FDA, concluded, "There is no good evidence that opioids

improve pain or function with long-term use"[53] and "complete relief of pain is unlikely."[54] (Emphasis added.) The CDC reinforced this conclusion throughout its 2016 Guideline:

a. "No evidence shows a long-term benefit of opioids in pain and function versus no opioids for chronic pain with outcomes examined at least 1 year later . . ."[55]

b. "Although opioids can reduce pain during short-term use, the clinical evidence review found insufficient evidence to determine whether pain relief is sustained and whether function or quality of life improves with long-term opioid therapy;"[56] and

c. "[E]vidence is limited or insufficient for improved pain or function with long-term use of opioids for several chronic pain conditions for which opioids are commonly prescribed, such as low back pain, headache, and fibromyalgia."[57]

109. The CDC also noted that the risks of addiction and death "can cause distress and inability to fulfill major role obligations."[58] As a matter of common sense (and medical evidence), drugs that can kill patients or commit them to a life of addiction or recovery do not improve their function and quality of life.

110. The 2016 CDC Guideline was not the first time a federal agency repudiated Defendants' claim that opioids improved function and quality of life. In 2010, the FDA warned Actavis that "[w]e are not aware of substantial evidence or substantial clinical experience demonstrating that the magnitude of the effect of the drug [Kadian] has in alleviating pain, taken together with any drug-related side effects patients may experience…results in any overall positive impact on a patient's work, physical and mental functioning, daily activities, or enjoyment of

[53] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[54] *Id.*
[55] *Id.*
[56] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[57] *Id.*
[58] *Id.*

life."[59]

111. Defendants also falsely emphasized or exaggerated the risks of competing products like NSAIDs so that doctors and patients would look to opioids first for treating chronic pain. Once again, Defendants' misrepresentations contravene pronouncements by and guidance from the FDA and CDC based on the scientific evidence.

112. Consequently, the FDA changed the labels for ER/LA opioids in 2013 and IR opioids in 2016 to state that opioids should be used only as a last resort where alternative treatments like non-opioid drugs are inadequate. And the 2016 CDC Guideline states that NSAIDs, not opioids, should be the first-line treatment for chronic pain, particularly arthritis and lower back pain.

113. In addition, Purdue misleadingly promoted OxyContin as unique among opioids in providing 12 continuous hours of pain relief with one dose. In fact, OxyContin does not last for 12 hours – a fact that Purdue has known at all times relevant to this action.

114. According to Purdue's own research, OxyContin wears off in under six hours in one quarter of patients and in under 10 hours in more than half. The reason is that OxyContin tablets release approximately 40% of their active medicine immediately, after which release tapers. Although the patient experiences a powerful initial response, there is little or no pain relief at the end of the dosing period because less medicine is released.

115. This phenomenon is known as "end of dose" failure, and the FDA found in 2008 that a substantial number of chronic pain patients taking OxyContin experience it.

116. This "end of dose" failure not only renders Purdue's promise of 12 hours of relief

[59] Warning Letter from Thomas Abrams, Dir., FDA Div. of Mktg., Adver., & Commc'ns, to Doug Boothe, CEO, Actavis Elizabeth LLC (Feb. 18, 2010), http://www.fda.gov/Drugs/GuidanceComplianceRegulatoryInformation/EnforcementActivitiesbyFDA/WarningLettersandNoticeofViolationLetterstoPharmaceuticalCcompanies/ucm259240.htm.

false and deceptive, it also makes OxyContin more dangerous because the declining pain relief patients experience toward the end of each dosing period drives them to take more OxyContin before the next dosing period begins, quickly increasing the amount of drug they are taking and spurring growing dependence.

117. Purdue's competitors were aware of this problem. For example, Endo ran advertisements for Opana ER referring to "real" 12-hour dosing. Nevertheless, Purdue falsely promoted OxyContin as if it were effective for a full 12 hours. Indeed, Purdue's sales representatives continue to tell doctors in and around Dallas County that OxyContin lasts a full 12 hours.

**E. Defendants also engaged in Other Unlawful, Unfair, and Fraudulent Misconduct.**

118. Defendants herein participated in illicit and unlawful prescribing of its drugs. For example, Purdue did not report illegal prescribing of OxyContin until years after law enforcement shut down a Los Angeles clinic that prescribed more than 1.1 million OxyContin tablets. In doing so, Purdue protected its own profits at the expense of public health and safety.

119. The State of New York found that Endo failed to require sales representatives to report signs of addiction, diversion, and inappropriate prescribing; paid bonuses to sales representatives for detailing prescribers who were subsequently arrested or convicted for illegal prescribing; and failed to prevent sales representatives from visiting prescribers whose suspicious conduct had caused them to be placed on a no-call list.

**F. Defendants Targeted Susceptible Prescribers and Vulnerable Patient Populations.**

120. As a part of their deceptive marketing scheme, Defendants identified and targeted susceptible prescribers and vulnerable patient populations in the U.S. and in and around Dallas County. For example, Defendants focused their deceptive marketing on primary care doctors, who

were more likely to treat chronic pain patients and prescribe opioids, but were less likely to be educated about treating pain and the risks and benefits of opioids.

121. Defendants also targeted vulnerable patient populations like the elderly and veterans, who tend to suffer from chronic pain. Defendants targeted these vulnerable patients even though the risks of long-term opioid use were significantly greater for them.

122. For example, the 2016 CDC Guideline observes that existing evidence shows that elderly patients taking opioids suffer from elevated fall and fracture risks, greater risk of hospitalization, and increased vulnerability to adverse drug effects and interactions. The Guideline therefore concludes that there are "special risks of long-term opioid use for elderly patients" and recommends that doctors use "additional caution and increased monitoring" to minimize the risks of opioid use in elderly patients.

123. The same is true for veterans, who are more likely to use anti-anxiety drugs (benzodiazepines) for post-traumatic stress disorder, which interact dangerously with opioids.

**G. Although Defendants knew that their Marketing of Opioids was False and Deceptive, they Fraudulently Concealed their Misconduct.**

124. Defendants, both individually and collectively, made, promoted, and profited from their misrepresentations about the risks and benefits of opioids for chronic pain even though they knew their misrepresentations were false and deceptive. The history of opioids, as well as research and clinical experience over the last 20 years, established that opioids were highly addictive and responsible for a long list of very serious adverse outcomes.

125. Not only did the FDA and other regulators warn Defendants, but Defendants had access to scientific studies, detailed prescription data, and reports of adverse events, including reports of addiction, hospitalization, and deaths – all of which made clear the harms from long-term opioid use, including the suffering from addiction, overdoses, and death in alarming numbers in

patients using opioids.

126. More recently, the FDA and CDC have issued pronouncements based on the medical evidence that conclusively expose the known falsity of Defendants' misrepresentations, and Endo and Purdue have recently entered agreements prohibiting them from making some of the same misrepresentations described herein in New York.

127. Specifically, three current and former executives from Purdue plead guilty in 2007 to criminal charges that they misled regulators, doctors, and patients about OxyContin's risk of addiction.[60] In pleading guilty to misbranding charges, Purdue admitted it had fraudulently marketed OxyContin as a drug less prone to addiction and as having fewer side effects than other opioids.[61] In reality, unlike other opioids, OxyContin contained pure oxycodone without any other ingredients, which made it a powerful narcotic despite its time-release design that Purdue touted as ameliorating its addictive potential.[62]

128. Moreover, Defendants took steps to avoid detection of and to fraudulently conceal their deceptive marketing and unlawful, unfair, and fraudulent conduct. For example, Defendants disguised their own role in the deceptive marketing of chronic opioid therapy by funding and working through third parties like Front Groups and KOLs.

129. Finally, Defendants manipulated their promotional materials and the scientific literature to make it appear that these items were accurate, truthful, and supported by objective evidence when they were not.

130. Thus, Defendants successfully concealed from the medical community and patients facts sufficient to arouse suspicion of the claims Dallas County now asserts. Dallas County did not

[60] *See* Barry Meier, "In Guilty Plea, OxyContin Maker to Pay $600 Million," (May 10, 2007), http://www.nytimes.com/2007/05/10/business/11drug-web.html.
[61] *See id.*
[62] *See id.*

know of the existence or scope of Defendants' industry-wide fraud and could not have acquired such knowledge earlier through the exercise of reasonable diligence.

## H. By Increasing Opioid Prescriptions and Use, Defendants' Deceptive Marketing Scheme has fueled the Opioid Epidemic and Devastated Dallas County Communities.

131. Defendants' misrepresentations deceived doctors and patients about the risks and benefits of long-term opioid use. Studies reveal that many doctors and patients are unaware of or do not understand the risks or benefits of opioids. Indeed, patients often report that they were not warned they might become addicted to opioids prescribed to them. As reported in January 2016, a 2015 survey of more than 1,000 opioid patients found that 4 out of 10 were not told opioids were potentially addictive.[63]

132. Defendants' deceptive marketing scheme caused and continues to cause doctors in and around Dallas County to prescribe opioids for chronic pain conditions such as back pain, headaches, arthritis, and fibromyalgia. Absent Defendants' deceptive marketing scheme, these doctors would not have been able to over prescribe opioids or become embroiled in pill mills that negatively impacted residents of Dallas County.

133. For example, Defendants' deceptive marketing scheme allowed three doctors located in Dallas County, Texas to promote, overprescribe, and financially benefit from prescribing opioids. Indeed, the doctors herein "knowingly or intentionally manufactured, distributed, dispensed, or possessed with the intent to manufacture, distribute, or dispense a controlled substance, including opioids such as OxyContin, Hydrocodone, and Vicodin in violation of the Texas Controlled Substances Act in 21 C.F.R. §1301 et seq.[64]

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[63] Hazelden Betty Ford Foundation, *Missed Questions, Missed Opportunities* (Jan. 27, 2016), *available at* http://www.hazeldenbettyford.org/about-us/news-and-media/pressrelease/doctors-missing-questions-that-could-prevent-opioid-addiction.

[64] *See, e.g.,* Grand Jury Indictment in *United States v. Sina Athari, et al.,* U.S.D.C-Northern Dist., Dallas Div., No. 3:14-CR-044-D (December 1, 2015).

134. Dr. Andrews, Dr. Okechuku, and Dr. Padron were all involved in a similar conspiracy to distribute opioids. The conspirators employed persons to recruit individuals who were homeless or of limited means.[65] These individuals would be paid a fee to pose as patients at certain medical clinics and to fill these same prescriptions at certain pharmacies.[66] The involved practitioners, such as the doctors herein, were enlisted to write prescriptions for opioids despite there being no legitimate medical purpose.[67] The clinics and the pharmacies accepted cash only, which was funneled through the various physicians, employees, and/or recruiters.[68] The end goal was to sell the opioids on the open market in Dallas County and elsewhere.

135. Dr. Richard Andrews was a co-owner and supervising physician of McAllen Medical Clinic in Dallas, Texas.[69] Dr. Andrews was indicted on December 1, 2015 for, among other things, conspiracy to distribute a controlled substance.[70] On July 26, 2016, Dr. Andrews entered into a plea agreement in which he pleaded guilty.[71] On March 3, 2017, Dr. Andrews and the Texas Medical Board agreed that his license would be revoked in lieu of further disciplinary actions.[72]

136. Dr. Theodore Okechuku operated a pain clinic in Lake Highlands located in Dallas, Texas.[73] Dr. Okechuku was indicted on December 3, 2013 for conspiracy to unlawfully distribute a controlled substance.[74] Dr. Okechuku violated the terms of his pre-trial release because he

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[65] *See, e.g.,* Indictment at p. 7.
[66] *Id.*
[67] *Id.* at 9.
[68] *Id.* at 5.
[69] *Id.* at 5.
[70] *See id.* at 30.
[71] Plea Agreement in *U.S. v. Richard Andrews*, U.S.D.C.-Northern District, Dallas Div., No. 3:15-CR-044-D (July 25, 2016).
[72] Texas Medical Board, http://reg.tmb/state/tx/us.com, *last viewed* November 13, 2017
[73] "Dallas Doctor Sentenced for Operating 'Pill Mill'", March 31, 2016, http://dfw.cbslocal.com/2016/03/31/dallas-doctor-sentenced-for-operating-pill-mill/.
[74] Texas Medical Board, http://reg.tmb/state/tx/us.com, *last viewed* November 13, 2017; *see also* "Dallas Doctor Sentenced for Operating 'Pill Mill'", March 31, 2016, http://dfw.cbslocal.com/2016/03/31/dallas-doctor-sentenced-for-operating-pill-mill/.

continued to prescribe hydrocodone and other controlled substances.[75] Ultimately, Dr. Okechuku was found guilty on 3 counts, one related to the distribution of opioids, and sentenced to 25 years.[76]

137. Dr. Nicolas A. Padron operated a "cash only" clinic in Dallas.[77] He, too, was indicted for conspiracy to unlawfully distribute controlled substances and ultimately sentenced to 87 months in federal prison.[78] On May 2, 2014, Dr. Padron agreed to the revocation of his medical license in lieu of further disciplinary action.[79]

138. If the manufacturing and distributing Defendants were not over-supplying opioids, then physicians like Dr. Andrews, Dr. Okechuku, and Dr. Padron could not devise schemes to prescribe opioids without a legitimate purpose as a means to flood the open market with opioids, such as OxyContin, Hydrocodone, and Vicodin.

139. While Defendants may claim the federal government authorized the amount of annual prescription opioids sold, they know in truth that several Defendants have successfully used their organized money and influence to render the federal government's enforcement agency, the Drug Enforcement Administration, virtually powerless to interrupt the over-supply of prescription opioid drugs.

140. Defendants' deceptive marketing scheme also caused and continues to cause patients to purchase and use opioids for their chronic pain believing they are safe and effective. Absent Defendants' deceptive marketing scheme, fewer patients would be using opioids long-term to treat chronic pain, and those patients using opioids would be using less of them.

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[75] Texas Medical Board, http://reg.tmb/state/tx/us.com, *last viewed* November 13, 2017.
[76] *U.S. v. Theodore E. Okechuku*, U.S.D.C.-Northern District, Dallas Div., No. 3:13-CR-00481-P(1) (March 30, 2016).
[77] "*Garland Doctor, other 'Dealers' Sentenced in Dallas 'Pill Mill' Case,"* (Oct. 29, 2014), http://starlocalmedia.com/rowlettlakeshoretimes/garland-doctor-other-d...llas-pill-mill-case/article_d53be5fc-5fbc-11e4-9186-af37156f06a3.html.
[78] *Id.*
[79] Texas Medical Board, http://reg.tmb/state/tx/us.com, *last viewed* November 13, 2017.

141. Defendants' deceptive marketing has caused and continues to cause the prescribing and use of opioids to explode. Indeed, this dramatic increase in opioid prescriptions and use corresponds with the dramatic increase in Defendants' spending on their deceptive marketing scheme. Defendants' spending on opioid marketing totaled approximately $91 million in 2000. By 2011, that spending had tripled to $288 million.

142. The escalating number of opioid prescriptions written by doctors who were deceived by Defendants' deceptive marketing scheme is the cause of a correspondingly dramatic increase in opioid addiction, overdose, and death throughout the U.S. and Dallas County.

143. Scientific evidence demonstrates a strong correlation between opioid prescriptions and becoming addicted to opioids. In a 2016 report, the CDC explained that prescribing opioids has quadrupled since 1999, which has resulted in a parallel increase in opioid overdoses.[80] Indeed, there has been a two-third increase in overdose deaths from using opioids since 2000.[81] For these reasons, the CDC concluded that efforts to rein in the prescribing of opioids for chronic pain are critical "to reverse the cycle of opioid pain medication misuse that contributes to the opioid overdose epidemic."[82]

144. Due to the increase in opioid overdoses, first responders such as police officers, have been and will continue to be in the position to assist people experiencing opioid-related overdoses.[83] In 2016, "over 1,200 law enforcement departments nationwide carried naloxone in

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[80] CDC. National Vital Statistics System, Mortality. CDC WONDER. Atlanta, GA: US Department of Health and Human Services, CDC; 2016. https://wonder.cdc.gov/; Rudd RA, Seth P, David F, Scholl L. Increases in Drug and Opioid-Involved Overdose Deaths — United States, 2010–2015. MMWR Morb Mortal Wkly Rep. ePub: 16 December 2016.

[81] *National Vital Statistics System*, Mortality file and appearing *Center for Disease Control and Prevention* Morbidity and Mortality Weekly Report, January 1, 2006 / 64(50); 1378-82, Increases in Drug and Opioid Deaths – United States, 2000-2014.

[82] *CDC Guideline for Prescribing Opioids for Chronic Pain, supra; see also* Rudd RA, Seth P, David F, Scholl L. Increases in Drug and Opioid-Involved Overdose Deaths — United States, 2010–2015. MMWR Morb Mortal Wkly Rep. ePub: 16 December 2016.

[83] Opinion of the Attorney General of Texas, KP-0168 (Oct. 4, 2017).

an effort to prevent opioid-related deaths."[84]

145. Defendants' deceptive marketing scheme has also detrimentally impacted children in Dallas County. Overprescribing opioids for chronic pain has made the drugs more accessible to school-aged children, who come into contact with opioids after they have been prescribed to friends or relatives in the same household.

146. Defendants' conduct has adversely affected Dallas County's child protection agencies in the number of children in foster care driven by parental drug addiction. Children with parents addicted to drugs tend to stay in foster care longer, and they often enter the system having experienced significant trauma, which makes these cases more expensive for counties like Dallas County.

147. Opioid addiction is one of the primary reasons that Dallas County residents seek treatment for substance dependence. A significant number of admissions for drug addiction were associated with a primary diagnosis of opiate addiction or dependence.

148. Defendants' creation, through false and deceptive advertising and other unlawful and unfair conduct, of a virtually limitless opioid market has significantly harmed Dallas County communities. Defendants' success in extending the market for opioids to new patients and chronic pain conditions has created an abundance of drugs available for non-medical and criminal use and fueled a new wave of addiction and injury. It has been estimated that 60% of the opioids to which people are addicted come, directly or indirectly, through doctors' prescriptions.[85]

149. Law enforcement agencies have increasingly associated prescription drug addiction

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[84] *Id. citing* http://www.nchrc.org/law-enforcement/us-law-enforcement-who-carry-naloxone/.

[85] Nathaniel P. Katz, *Prescription Opioid Abuse: Challenges and Opportunities for Payers*, Am. J. Managed Care (Apr. 19 2013), at 5 ("The most common source of abused [opioids] is, directly or indirectly, by prescription."), http://www.ajmc.com/publications/issue/2013/2013-1-vol19-n4/Prescription-Opioid-Abuse-Challenges-and-Opportunities-for-Payers.

with violent and property crimes. Despite strict federal regulation of prescription drugs, local law enforcement agencies are faced with increasing diversion from legitimate sources for illicit purposes, including doctor shopping, forged prescriptions, falsified pharmacy records, and employees who steal from their place of employment. The opioid epidemic has prompted a growing trend of crimes against pharmacies including robbery and burglary. This ongoing diversion of prescription narcotics creates a lucrative marketplace.

150. The rise in opioid addiction caused by Defendants' deceptive marketing scheme has also resulted in an explosion in heroin use. For example, heroin use has more than doubled in the past decade among adults aged 18 to 25 years.[86] Moreover, heroin-related overdoses in the United States has more than quadrupled since 2010.[87]

151. The costs and consequences of opioid addiction are staggering. For example, in 2007, the cost of healthcare due to opioid addiction and dependence was estimated at 25 billion, the cost of criminal justice was estimated at 5.1 billion, and the cost of lost workplace productivity was estimated at 25.6 billion.

152. Consequently, prescription opioid addiction and overdose have an enormous impact on the health and safety of individuals, as well as communities at large, because the consequences of this epidemic reach far beyond the addicted individual.

153. Some of the repercussions for residents of Dallas County include job loss, loss of custody of children, physical and mental health problems, homelessness and incarceration, which results in instability in communities often already in economic crisis and contributes to increased demand on community services such as hospitals, courts, child services, treatment centers, and law

[86] Centers for Disease Control and Prevention. Vital Signs: Today's Heroin Epidemic – More People at Risk, Multiple Drugs Abused. (https://www.cdc.gov/vitalsigns/heroin/index.html). MMWR 2015.
[87] https://www.cdc.gov/vitalsigns/heroin/index.html

enforcement.

154. Defendants knew and should have known about these harms that their deceptive marketing has caused and continues to cause and will cause in the future. Defendants closely monitored their sales and the habits of prescribing doctors. Their sales representatives, who visited doctors and attended CMEs, knew which doctors were receiving their messages and how they were responding.

155. Defendants also had access to and carefully watched government and other data that tracked the explosive rise in opioid use, addiction, injury, and death. Defendants not only knew, but intended that their misrepresentations would persuade doctors to prescribe and encourage patients to use their opioids for chronic pain.

156. Defendants' actions are neither permitted nor excused by the fact that their drug labels may have allowed, or did not exclude, the use of opioids for chronic pain. FDA approval of opioids for certain uses did not give Defendants license to misrepresent the risks and benefits of opioids. Indeed, Defendants' misrepresentations were directly contrary to pronouncements by, and guidance from, the FDA based on the medical evidence and their own labels.

157. Nor is Defendants' causal role broken by the involvement of doctors. Defendants' marketing efforts were ubiquitous and highly persuasive. Their deceptive messages tainted virtually every source doctors could rely on for information and prevented them from making informed treatment decisions. Defendants also hijacked what doctors wanted to believe – namely, that opioids represented a means of relieving their patients' suffering and of practicing medicine more compassionately.

158. Defendants' actions and omissions were each a cause-in-fact of Dallas County's past and future damages. Defendants' wrongful conduct caused injuries to Dallas County in the

past, continues to cause injuries to Dallas County, and will continue to cause injuries to Dallas County in the future. Future damages include, but are not limited to, additional resources for counseling and medication assisted treatment of addicts, medical treatment for overdoses, life skills training for adolescents, increased law enforcement, and additional resources to treat the psychological effects of opioids and the underlying conditions that make people susceptible to opioid addiction.

## I. Defendants' Fraudulent Marketing Has Led To Record Profits.

159. While using opioids has taken an enormous toll on Dallas County and its residents, Defendants have realized blockbuster profits. In 2014 alone, opioids generated $11 billion in revenue for drug companies like Defendants. Indeed, financial information indicates that each Defendant experienced a material increase in sales, revenue, and profits from the false and deceptive advertising and other unlawful and unfair conduct described above.

## VI. FIRST CAUSE OF ACTION: PUBLIC NUISANCE AGAINST ALL DEFENDANTS

160. Dallas County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

161. Defendants knowingly encouraged doctors in and around Dallas County to prescribe, and residents to use, highly addictive opioids for chronic pain even though Defendants knew using opioids had a high risk of addiction and reduced quality of life.

162. By doing so, Defendants purposefully interfered with Dallas County's public health, public safety, public peace, public comfort, and public convenience.

163. Defendants, individually and in concert with each other, have contributed to and/or assisted in creating and maintaining a condition that is harmful to the health and safety of Dallas County residents and/or unreasonably interferes with the peace and comfortable enjoyment of life

in violation of Texas law.

164. The public nuisance created by Defendants' actions is substantial and unreasonable – it has caused and continues to cause significant harm to the community – and the harm inflicted outweighs any offsetting benefit.

165. The staggering rates of opioid use resulting from Defendants' marketing efforts have caused, and continues to cause, harm to the community including, but not limited to:

a. Upwards of 30% of all adults use opioids. These high rates of use have led to unnecessary opioid addiction, overdose, injuries, and deaths;

b. Children have been exposed to opioids prescribed to family members or others resulting in injury, addiction, and death. Easy access to prescription opioids has made opioids a recreational drug of choice among Dallas County teenagers; opioid use among teenagers is only outpaced by marijuana use. Even infants have been born addicted to opioids due to prenatal exposure causing severe withdrawal symptoms and lasting developmental impacts;

c. Residents of Dallas County, who have never taken opioids, have endured both the emotional and financial costs of caring for loved ones addicted to or injured by opioids and the loss of companionship, wages, or other support from family members who have used, become addicted to, overdosed on, or been killed by opioids;

d. More broadly, opioid use and addiction have driven Dallas County residents' health care costs higher;

e. Employers have lost the value of productive and healthy employees who have suffered from adverse consequences from opioid use;

f. Defendants' success in extending the market for opioids to new patients and chronic conditions has created an abundance of drugs available for criminal use and fueled a new wave of addiction and injury. Defendants' scheme created both ends of a new secondary market for opioids – providing both the supply of narcotics to sell and the demand of addicts to buy them;

g. This demand has created additional illicit markets in other opiates, particularly heroin. The low cost of heroin has led some of those who initially become addicted to prescription opioids to migrate to cheaper heroin, fueling a new heroin epidemic in the process;

h. Diverting opioids into secondary, criminal markets and increasing the number of individuals who are addicted to opioids has increased the demands on emergency services and law enforcement in Dallas County;

i. All of Defendants' actions have caused significant harm to the community – in lives lost; addictions endured; the creation of an illicit drug market and all its concomitant crime and costs; unrealized economic productivity; and broken families and homes;

j. These harms have taxed the human, medical, public health, law enforcement, and financial resources of Dallas County; and

k. Defendants' interference with the comfortable enjoyment of life of a substantial number of people is entirely unreasonable because there is limited social utility to opioid use and any potential value is outweighed by the gravity of harm inflicted by Defendants' actions.

166. Defendants knew, or should have known, that promoting opioid use would create a public nuisance in the following ways:

a. Defendants have engaged in massive production, promotion, and distribution of opioids for use by the citizens of Dallas County;

b. Defendants' actions created and expanded the market for opioids, promoting its wide use for pain management;

c. Defendants misrepresented the benefits of opioids for chronic pain and fraudulently concealed, misrepresented, and omitted the serious adverse effects of opioids, including the addictive nature of the drugs; and

d. Defendants knew or should have known that their promotion would lead to addiction and other adverse consequences that the larger community would suffer as a result.

167. Defendants' actions were, at the least, a substantial factor in doctors and patients not accurately assessing and weighing the risks and benefits of opioids for chronic pain thereby causing opioids to become widely available and used in Dallas County.

168. Without Defendants' actions, opioid use would not have become so widespread and the enormous public health hazard of opioid addiction would not have existed and could have been averted.

169. The health and safety of the citizens of Dallas County, including those who use, have used, or will use opioids, as well as those affected by opioid users, is a matter of great public interest and legitimate concern to Dallas County's citizens and residents.

170. The public nuisance created, perpetuated, and maintained by Defendants can be abated and further reoccurrence of such harm and inconvenience can be prevented.

171. Defendants' conduct has affected and continues to affect a considerable number of people within Dallas County and is likely to continue to cause significant harm to patients who take opioids, their families, and the community at large.

172. Each Defendant created or assisted in creating the opioid epidemic, and each Defendant is jointly and severally liable for its abatement. Furthermore, each Defendant should be enjoined from continuing to create, perpetuate, or maintain said public nuisance in Dallas County.

## VII. SECOND CAUSE OF ACTION: COMMON LAW FRAUD AGAINST ALL DEFENDANTS

173. Dallas County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

174. At all relevant and material times, Defendants expressly and/or impliedly warranted that opioids were safe, of merchantable quality, and fit for use.

175. Defendants' superior knowledge and expertise, its relationship of trust and confidence with doctors and the public, its specific knowledge regarding the risks and dangers of opioids, and its intentional dissemination of promotional and marketing information about opioids for the purpose of maximizing sales, each gave rise to the affirmative duty to meaningfully disclose and provide all material information about the risks and harms associated with opioids.

176. At all times herein mentioned, Defendants, individually and acting through their

employees and agents, and in concert with each other, fraudulently represented to physicians who Defendants knew would justifiably rely on Defendants' representations that opioids were safe and effective for treating chronic pain.

177. Defendants' false representations were fraudulently made, with the intent or purpose that healthcare providers and patients would justifiably rely upon them, leading to the prescription, administration, filling, purchasing, and consumption of opioids in Dallas County.

178. Defendants' deliberate misrepresentations and/or concealment, suppression, and omission of material facts as alleged herein include, but are not limited to:

a. Making false and misleading claims regarding the known risks of the addictive nature of opioids and suppressing, failing to disclose, and mischaracterizing the addictive nature of opioids and in concomitant costs, such as overdoses, deaths, and heroin addiction;

b. Making false and misleading written and oral statements that opioids are more effective than traditional pain killers for chronic pain, or effective at all and/or omitting material information showing that opioids are no more effective than other non-addictive drugs for chronic pain;

c. Issuing false and misleading warnings and/or failing to issue adequate warnings concerning the risks and dangers of using opioids;

d. Making false and misleading claims downplaying the risk of addiction when using opioids and/or setting forth guidelines that would purportedly identify addictive behavior; and

e. Making false and misleading misrepresentations concerning the safety, efficacy and benefits of opioids without full and adequate disclosure of the underlying facts which rendered such statements false and misleading.

179. Defendants willfully, wantonly, and recklessly disregarded their duty to provide truthful representations regarding the safety and risk of opioids.

180. Defendants made these misrepresentations with the intent that the healthcare community and patients located wherever these opioid drugs were sold or consumed would rely upon them.

181. Defendants' misrepresentations were made with the intent of defrauding and

deceiving the medical community and consumers to induce and encourage the sale of opioids.

182. Defendants' fraudulent representations evidence their callous, reckless, willful, and depraved indifference to the health, safety, and welfare of consumers living in Dallas County.

183. Defendants omitted, misrepresented, suppressed and concealed material facts concerning the dangers and risk of injuries associated with the use of opioids, as well as the fact that the product was unreasonably dangerous.

184. Defendants' purpose was willfully blind to, ignored, downplayed, avoided, and/or otherwise understated the serious nature of the risks associated with the use of opioids.

185. The treating medical community and consumers in Dallas County did not know that Defendants' representations were false and/or misleading and justifiably relied on them.

186. Defendants had sole access to material facts concerning the dangers and unreasonable risks of opioids, which they intentionally concealed.

187. As a direct and proximate result of Defendants' fraudulent misrepresentations and intentional concealment of facts, upon which the medical community and consumers in Dallas County reasonably relied, Dallas County suffered actual and punitive damages.

## VIII. THIRD CAUSE OF ACTION: NEGLIGENCE AGAINST MANUFACTURING AND DISTRIBUTING DEFENDANTS

188. Dallas County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

189. Manufacturing Defendants have a duty to exercise reasonable care in marketing its opioids to physicians treating residents of Dallas County and Dallas County residents. Manufacturing Defendants have breached their duty by knowingly and fraudulently misrepresenting the benefits of, and downplaying the risks of, opioids for chronic pain.

190. Manufacturing Defendants have used deceitful marketing ploys, KOLs, Front Groups, and other schemes to increase profits at the cost of public health causing an opioid epidemic. Manufacturing Defendants have acted willfully, wantonly, and maliciously.

191. Likewise, Distributor Defendants have a duty to exercise ordinary care in distributing opioids. Distributor Defendants have breached their duty by failing to prevent or reduce the distribution of opioids, or to report the increase in the distribution and/or sale of opioids.

192. Distributor Defendants have intentionally failed to prevent or reduce the distribution of opioids, or to report any increases in the sale of opioids, so that they could increase profits and receive rebates or kick-backs from Manufacturing Defendants. Distributor Defendants have acted willfully, wantonly, and maliciously.

193. As a proximate result, Manufacturing and Distributor Defendants and its agents have caused Dallas County to incur excessive costs to treat the opioid epidemic in its county, including but not limited to increased costs of social services, health systems, law enforcement, judicial system, and treatment facilities.

194. Dallas County and its residents are therefore entitled to actual and punitive damages.

## IX. FOURTH CAUSE OF ACTION: GROSS NEGLIGENCE AGAINST MANUFACTURING AND DISTRIBUTING DEFENDANTS

195. Dallas County incorporates the allegations within all prior paragraphs within this Complaint as if they were fully set forth herein.

196. Defendants' marketing scheme to optimize profits by misrepresenting and falsely touting opioids as the panacea to chronic pain was done intentionally.

197. Defendants' hiring of KOLs, Front Groups, and others to spread its fraudulent message that opioids were useful and beneficial for chronic pain was grossly negligent and done with conscious indifference or reckless disregard for the safety of others.

198. Each Defendant's actions and omissions as described herein, singularly or in combination with each other, was malicious resulting in damages and injuries to Dallas County and

its residents.

199. At every stage, Defendants knew or should have known that their conduct would create an unreasonable risk of physical harm to others, including Dallas County and its residents, and should be held liable in punitive and exemplary damages to Dallas County.

## X. FIFTH CAUSE OF ACTION: TEXAS CONTROLLED SUBSTANCES ACT ("TCSA") AGAINST DISTRIBUTOR DEFENDANTS, DR. ANDREWS, DR. OKECHUKU, AND DR. PADRON

200. Dallas County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

201. Distributor Defendants have knowingly distributed, delivered, administered, or dispensed a controlled substance in violation of the Texas Controlled Substances Act §481.128(a)(1) by deceiving practitioners into prescribing, dispensing, delivering, or administering a controlled substance, or causing a controlled substance to be administered when there is no valid medical purpose. Tex. Health & Safety Code §481.071.

202. As alleged herein, each Distributor Defendant, at all times relevant to this Complaint, violated the Texas Controlled Substance Act by making deceptive representations about using opioids to treat chronic pain. Each Distributor Defendant also omitted or concealed material facts and failed to correct prior misrepresentations and omissions about the risks and benefits of opioids. Each Distributor Defendant's omissions rendered even their seemingly truthful statements about opioids deceptive.

203. Distributor Defendants' deceptive representations and concealments were reasonably calculated to deceive practitioners treating Dallas County residents into prescribing opioids without any valid medical purpose, and Distributor Defendants continue to do so to this day.

204. Dr. Andrews, Dr. Okechuku, and Dr. Padron prescribed opioids without a valid medical purpose in violation of Texas Health & Safety Code Section 481.071(a).

205. As a direct and proximate cause of Distributor Defendants' and the physicians' deceptive conduct, Dallas County should be awarded civil penalties pursuant to the Texas Controlled Substances Act.

## XI. SIXTH CAUSE OF ACTION: UNJUST ENRICHMENT AGAINST ALL DEFENDANTS

206. Dallas County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

207. As an expected and intended result of their conscious wrongdoing as set forth in this Complaint, Defendants have profited and benefited from opioid purchases made by Dallas County and its residents.

208. When Dallas County and its residents purchased opioids, they expected that Defendants had provided necessary and accurate information regarding those risks. Instead, Defendants had misrepresented the material facts regarding the risks and benefits of opioids.

209. Defendants have been unjustly enriched at the expense of Dallas County, and Dallas County is therefore entitled to damages to be determined by the jury.

## PRAYER FOR RELIEF

WHEREFORE, Plaintiff respectfully prays:

a. That the acts alleged herein be adjudged and decreed to be unlawful and that the Court enter a judgment declaring them to be so;

b. That Defendants be enjoined from, directly or indirectly through KOLs, Front Groups or other third parties, continuing to misrepresent the risks and benefits of the use of opioids for chronic pain, and from continuing to violate Texas law;

c. That Plaintiff recover all measures of damages, including punitive and

exemplary damages, allowable under the law, and that judgment be entered against Defendants in favor of Plaintiff;

d. That Plaintiff recover restitution on behalf of Dallas County consumers who paid for opioids for chronic pain;

e. That Plaintiff recover the costs and expenses of suit, pre- and post-judgment interest, and reasonable attorneys' fees as provided by law; and

f. That Defendants be ordered to abate the public nuisance that they created in in violation of Texas common law.

Date: January 8, 2018

Respectfully Submitted,

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# APPENDIX D

# DELTA PETITION

Delta County - District Clerk

Filed: 12/19/2017 4:17 PM
Accepted: 12/20/2017 4:51 PM
Jane Jones
Clerk, County/District Court
Delta County, Texas
Grace Crabb

**CAUSE NO.** 11125

| | | |
|---|---|---|
| **COUNTY OF DELTA,** | § | **IN THE DISTRICT COURT** |
| | § | |
| *Plaintiffs,* | § | |
| | § | |
| vs. | § | 62ND **JUDICIAL DISTRICT** |
| | § | |
| **PURDUE PHARMA, L.P.;** | § | |
| **PURDUE PHARMA, INC.;** | § | |
| **THE PURDUE FREDERICK COMPANY;** | § | |
| **JOHNSON & JOHNSON;** | § | **DELTA COUNTY, TEXAS** |
| **JANSSEN PHARMACEUTICALS, INC.;** | § | |
| **ORTHO-MCNEIL-JANSSEN PHARMACEUTICALS, INC.** n/k/a JANSSEN PHARMACEUTICALS, INC.; | § | |
| **JANSSEN PHARMACEUTICA, INC.** n/k/a JANSSEN PHARMACEUTICALS, INC.; | § | |
| **ENDO HEALTH SOLUTIONS INC.;** | § | |
| **ENDO PHARMACEUTICALS, INC.;** | § | |
| **ABBVIE INC.;** | § | |
| **KNOLL PHARMACEUTICAL COMPANY,** a wholly-owned subsidiary of ABBVIE INC.; | § | |
| **ALLERGAN PLC** f/k/a ACTAVIS PLC; | § | |
| **ALLERGAN FINANCE LLC** f/k/a ACTAVIS, INC. f/k/a WATSON PHARMACEUTICALS, INC.; | § | |
| **WATSON LABORATORIES, INC.;** | § | |
| **ACTAVIS LLC;** | § | |
| **ACTAVIS PHARMA, INC.** f/k/a WATSON PHARMA, INC.; | § | |
| **MCKESSON CORPORATION;** | § | |
| **CARDINAL HEALTH, INC.;** | § | |
| **AMERISOURCEBERGEN CORPORATION;** | § | |
| **DON MARTIN O'NEAL, M.D.;** and | § | |
| **DOES 1 – 100, INCLUSIVE,** | § | |
| | § | |
| *Defendants.* | § | |
| | § | |

## PLAINTIFF'S ORIGINAL PETITION AND JURY DEMAND

**TO THE HONORABLE JUDGE OF SAID COURT:**

Plaintiff, the County of Delta, Texas, by and through the undersigned attorneys, (hereinafter "Delta County" or "County"), on behalf of the County Attorney for Delta County, against Defendants Purdue Pharma, L.P., Purdue Pharma, Inc., The Purdue Frederick Company, Johnson & Johnson, Janssen Pharmaceuticals, Inc., Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc., Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc., Endo Health Solutions Inc., Endo Pharmaceuticals, Inc., Abbvie Inc., Knoll Pharmaceutical Company, a wholly-owned subsidiary of Abbvie Inc., Allergan PLC f/k/a Actavis PLC, Allergan Finance, LLC f/k/a Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc., Watson Laboratories, Inc., Actavis LLC, Actavis Pharma, Inc. f/k/a Watson Pharma, Inc., McKesson Corporation, Cardinal Health, Inc., AmerisourceBergen Corporation, Don Martin O'Neal, M.D., and Does 1 – 100, alleges as follows:

## I. INTRODUCTION

1. The United States is in the midst of an opioid epidemic caused by Defendants' fraudulent marketing and sales of prescription opioids ("opioids") that has resulted in addiction, criminal activity, and loss of life. The opioid crisis has been described as "the AIDS epidemic of our generation, but even worse."[1] On October 26, 2017, President Donald Trump "declared a nationwide public health emergency to combat the opioid crisis."[2]

2. In 2016 alone, health care providers wrote more than 289 million prescriptions for

[1] David Wright, "Christie on Opioids: "This is the AIDS Epidemic of Our Generation, but even Worse," (Oct. 27, 2017), http://www.cnn.com/2017/10/27/politics/chris-christie-opioid-commission-aids-cnntv/index.html.
[2] Dan Merica, "What Trump's Opioid Announcement Means – and Doesn't Mean," (Oct. 26, 2017), http://www.cnn.com/2017/10/26/politics/national-health-emergency-national-disaster/index.html.

opioids, enough for *every adult in the United States* to have more than one bottle of pills.[3] Americans "consume 85% of all the opioids in the world" and are "the most medicated country in the world . . . ."[4]

3. Unfortunately, using opioids too often leads to addiction and overdose from opioids. In 2014, almost 2 million Americans abused or were addicted to opioids.[5] That same year, more people died from drug overdoses than in any other year, and most overdose deaths involved an opioid. The Texas Legislature has found "that deaths resulting from the use of opioids and other controlled substances constitute a public health crisis."[6] In 2015, Texas "had the second highest total healthcare costs from opioid abuse in the nation ($1.96 billion) . . . ."[7]

4. In fact, accidental drug overdose deaths, of which reportedly at least two-thirds are opioid overdoses, are the leading cause of death for Americans under the age of 50. Accidental drug overdose deaths, predominantly from opioids, exceed the number of deaths caused by cars or guns.

5. The economic burden caused by opioid abuse in the United States is at least $78.5 billion,[8] including lost productivity and increased social services, health insurance costs, increased criminal justice presence and strain on judicial resources, and substance abuse treatment and rehabilitation.

6. This epidemic did not occur by chance. Defendants manufacture, market, distribute,

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[3] *Prevalence of Opioid Misuse*, BupPractice (Sept. 7, 2017), https://www.buppractice.com/node/15576.
[4] David Wright, "Christie on Opioids: "This is the AIDS Epidemic of Our Generation, but even Worse," (Oct. 27, 2017), http://www.cnn.com/2017/10/27/politics/chris-christie-opioid-commission-aids-cnntv/index.html.
[5] Substance Abuse and Mental Health Services Administration, National Survey on Drug Use and Health, 2014.
[6] Opinion of the Attorney General of Texas, KP-0168 (Oct. 4, 2017), *citing* Act of May 26, 2017, 85th Leg., R.S., ch. 534, §3, 2017 Tex. Sess. Law Serv. 1467, 1468.
[7] Kerry Craig, "Opioid Addiction Results in one Woman's Daily Struggle," Oct. 7, 2017, https://www.ssnewstelegram.com/news/opioid-addiction-results-in-one-woman-s-daily-struggle/article_bded4eoa-ab80-11e7-a252-d3f304e26628.html.
[8] *CDC Foundation's New Business Pulse Focuses on Opioid Overdose Epidemic*, Centers for Disease Control and Prevention (Mar. 15, 2017), https://www.cdc.gov/media/releases/2017/a0315-business-pulse-opioids.html.

and sell prescription opioids, including, but not limited to, brand-name drugs like OxyContin, Vicodin, Opana, Percocet, Percodan, Duragesic, Ultram, Ultracet, and generics like oxymorphone, oxycodone, hydromorphone, and hydrocodone, which are powerful narcotics.

7. Historically, opioids were considered too addictive and debilitating for treating non-cancer chronic pain,[9] such as back pain, migraines, and arthritis, and were used only to treat short-term acute pain or for palliative or end-of-life care.

8. By the late 1990s or early 2000s, however, each Manufacturing Defendant began a marketing scheme to persuade doctors and patients that opioids can and should be used ubiquitously and perpetually to treat moderate, non-cancer chronic pain. Each Manufacturing Defendant spent large sums of money to promote the benefits of opioids for non-cancer moderate pain while trivializing, or even denying, their risks. The Manufacturing Defendants' promotional messages deviated substantially from any approved labeling of the drugs and caused prescribing physicians and consuming patients to underappreciate the health risks, and to overestimate the benefits, of opioids.

9. Contrary to the language of their drugs' labels, Defendants falsely and misleadingly: (1) downplayed the serious risk of addiction; (2) promoted the concept of "pseudoaddiction" thereby advocating that the signs of addiction should be treated with more opioids; (3) exaggerated the effectiveness of screening tools in preventing addiction; (4) claimed that opioid dependence and withdrawal are easily managed; (5) denied the risks of higher opioid dosages; and (6) exaggerated the effectiveness of "abuse-deterrent" opioid formulations to prevent abuse and addiction.

10. Defendants disseminated these falsehoods through their sales representatives and

[9] "Chronic pain" means non-cancer pain lasting three months or longer.

physicians who supported Defendants' message. Sales representatives, working at Defendants' behest, promoted highly addictive opioids through souvenirs and toys including, but not limited to, opioid brand-bearing stuffed plush toys, dolls, coffee cups, fanny packs, water bottles, notepads, pens, refrigerator magnets, clocks, letter openers, rulers, daytime planners, bags, puzzles, posters, clipboards, highlighters, flashlights, key chains, clothing, reflex mallets, and mock-ups of the United States Constitution.

11. Defendants also used third parties they controlled by: (a) funding, assisting, encouraging, and directing doctors, known as "key opinion leaders" ("KOLs") and (b) funding, assisting, directing, and encouraging seemingly neutral and credible professional societies and patient advocacy groups (referred to hereinafter as "Front Groups").

12. Defendants worked with KOLs and Front Groups to taint the sources that doctors and patients relied on for ostensibly "neutral" guidance, such as treatment guidelines, Continuing Medical Education ("CME") programs, medical conferences and seminars, and scientific articles. After their individual and concerted efforts, Defendants convinced doctors that instead of being addictive and unsafe for long-term use in most circumstances, opioids were *required* in the compassionate treatment of chronic pain.

13. The Distributor Defendants were not standing by idly while Marketing Defendants were peddling their opioids to physicians and consumers. Cardinal, AmerisourceBergen, and McKesson ("Distributor Defendants") are three of the largest opioid distributors in the United States. Distributor Defendants purchased opioids from Manufacturing Defendants herein and sold them to pharmacies servicing consumers in Delta County.

14. Despite the alarming and suspicious rise in the ordering of opioids by retailers in Delta County, Distributor Defendants did nothing.

15. Essentially each Defendant ignored science and consumer health for profits. Defendants' efforts were so successful that opioids are now the most prescribed class of drugs generating $11 billion in revenue for drug companies in 2014 alone. Even after Purdue reached a $600 million federal settlement in 2007, the settlement failed to impact what is a "$13-billion-a-year opioid industry."[10]

16. As a direct and foreseeable consequence of Defendants' misrepresentations regarding the safety and efficacy of using opioids for chronic pain, Delta County has spent and continues to spend large sums combatting the public health crisis created by Defendants' negligent and fraudulent marketing campaign.

17. For example, thousands of prescriptions were written for opioids in Delta County in 2012[11] and in 2012 there were multiple deaths reported from drug overdoses.[12] A substantial number of those overdose deaths were a result, in whole or in part, of opioid ingestion. In each year from 2013-2017, there were multiple deaths in Dallas County caused in whole or in part from ingestion of prescription opioids. Defendants' marketing misconduct, as well as Defendants' efforts to sell more prescription opioids that can be consumed therapeutically, were natural and foreseeable causes of overdose deaths and injuries in Dallas County.

18. As a direct and foreseeable consequence of Defendants' conduct described regarding prescription opioids, Delta County has committed and continues to commit resources to provide and pay for health care, law enforcement, social services, public assistance, pharmaceutical care and other services necessary for its residents.

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[10] Rebecca L. Haffajee, J.D., Ph.D., M.P.H., and Michelle M. Mello, J.D., Ph.D., *Drug Companies' Liability for the Opioid Epidemic*, N. Engl. J. Med. at 2305, (Dec. 14, 2017).

[11] https://www.cdc.gov/drugoverdose/maps/rxrate-maps.html; https://www.cdc.gov/nchs/data-visualization/drug-poisoning-mortality.

[12] https://www.cdc.gov/nchs/data-visualization/drug-poisoning-mortality.

## II. RULE 47 STATEMENT OF MONETARY RELIEF SOUGHT

19. Per Rule 47 of the Texas Rules of Civil Procedure, the County states that although the full measure of its damages is still being calculated, its damages caused by Defendants' acts and omissions exceed $1,000,000 but are believed to be less than $50,000,000. Accordingly, at this time in the litigation, Delta County states that it is seeking monetary relief for an amount greater than $1,000,000 and less than $50,000,000, the rightful and just amount to be determined by the jury.

## III. VENUE AND JURISDICTION

20. Venue is proper in Delta County because all or a substantial part of the events or omissions giving rise to this claim occurred in Delta County. TEX. CIV. PRAC. & REM. CODE §15.002(a)(2). This Court has subject-matter jurisdiction over this matter because Plaintiff's damages are in excess of the minimal jurisdictional limits of this Court. TEX. GOVT. CODE §24.007(b).

21. This Court has general jurisdiction over Dr. O'Neal as he is a Texas resident. This Court also has specific jurisdiction over all Defendants as their activities were directed toward Texas, and injuries complained of herein resulted from their activities. *Guardian Royal Exchange Assur., Ltd. v. English China Clays, P.L.C.*, 815 S.W.2d 223, 227 (Tex. 1991). Each Defendant has a substantial connection with Texas and the requisite minimum contacts with Texas necessary to constitutionally permit the Court to exercise jurisdiction. *See id.* at 226.

## IV. PARTIES

### A. Plaintiff

22. This action is brought for and on behalf of Delta County, which provides a wide range of services on behalf of its residents, including services for families and children, public health, public assistance, law enforcement, and emergency care.

**B. Defendants**

23. PURDUE PHARMA L.P. is a limited partnership organized under the laws of Delaware, and may be served through its registered agent for service of process, The Prentice-Hall Corporation System, Inc., 251 Little Falls Drive, Wilmington, DE 19808. PURDUE PHARMA INC. is a New York corporation with its principal place of business in Stamford, Connecticut, and may be served through its registered agent for service of process, Corporation Service Company, 80 State Street, Albany, NY 12207. THE PURDUE FREDERICK COMPANY is a Delaware corporation with its principal place of business in Stamford, Connecticut, and may be served through its registered agent for service of process, The Prentice-Hall Corporation System, Inc., 251 Little Falls Drive, Wilmington, DE 19808 (collectively, "Purdue").

24. Purdue manufactures, promotes, sells, and distributes opioids in the U.S. and Delta County. Purdue's opioid drug, OxyContin, is among the most addictive and abused prescription drugs in the history of America. Purdue promotes opioids throughout the United States and in Delta County.

25. JANSSEN PHARMACEUTICALS, INC. is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey, and may be served through its registered agent for service of process, CT Corporation System, 1999 Bryan Street, Suite 900, Delta, TX 75201. JANSSEN PHARMACEUTICALS, INC. is a wholly owned subsidiary of JOHNSON & JOHNSON (J&J), a New Jersey corporation with its principal place of business in New Brunswick, New Jersey, and may be served through its registered agent for service of process, Attention: Legal Department, One Johnson & Johnson Plaza, New Brunswick, NJ 08933. ORTHO-MCNEIL-JANSSEN PHARMACEUTICALS, INC., now known as JANSSEN PHARMACEUTICALS, INC., is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey.

JANSSEN PHARMACEUTICA INC., now known as JANSSEN PHARMACEUTICALS, INC., is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey. J&J is the only company that owns more than 10% of Janssen Pharmaceuticals' stock, and corresponds with the FDA regarding Janssen's products. Upon information and belief, J&J controls the sale and development of Janssen Pharmaceuticals' drugs and Janssen's profits inure to J&J's benefit. (Janssen Pharmaceuticals, Inc., Ortho-McNeil-Janssen Pharmaceuticals, Inc., Janssen Pharmaceutica, Inc., and J&J are referred to as "Janssen").

26. Janssen manufactures, promotes, sells, and distributes opioids in the U.S. and in Delta County.

27. ENDO HEALTH SOLUTIONS INC. is a Delaware corporation with its principal place of business in Malvern, Pennsylvania, and may be served through its registered agent for service of process, The Corporation Trust Company, Corporation Trust Center, 1209 Orange St., Wilmington, DE 19801. ENDO PHARMACEUTICALS, INC. is a wholly-owned subsidiary of Endo Health Solutions Inc. and is a Delaware corporation with its principal place of business in Malvern, Pennsylvania, and may be served through its registered agent for service of process, CT Corporation System, 1999 Bryan Street, Suite 900, Delta, TX 75201. (Endo Health Solutions Inc. and Endo Pharmaceuticals, Inc. are referred to as "Endo").

28. Endo develops, markets, and sells opioid drugs in the U.S. and in Delta County. Endo also manufactures and sells generic opioids in the U.S. and Delta County, by itself and through its subsidiary, Qualitest Pharmaceuticals, Inc.

29. ABBVIE INC. ("Abbvie") is a Delaware corporation with its principal place of business in North Chicago, Illinois, and may be served through its registered agent for service of process, CT Corporation System, 208 S. LaSalle Street, Suite 814, Chicago, IL 60604. KNOLL

PHARMACEUTICAL COMPANY ("Knoll") has been a wholly-owned subsidiary of Abbvie from January 1, 2013. KNOLL PHARMACEUTICAL COMPANY is a New Jersey corporation with its principal place of business in Parsippany, New Jersey, and may be served through its registered agent for service of process, CT Corporation System, 208 S. LaSalle Street, Suite 814, Chicago, IL 60604.

30. Knoll irresponsibly marketed narcotics, such as Vicodin, through whimsical toys and souvenirs and did so to boost the sales of opioids. Taking advantage of the fact that Vicodin was not regulated as a Schedule II controlled substance for many years, and the fact physicians and consumers did not fully appreciate the highly addictive nature of Vicodin, Knoll advertised Vicodin with tag lines such as "The Highest Potency Pain Relief You Can Still Phone In." This tag line came as part and parcel of souvenirs like a "Vicodin" fanny pack and water bottle, both bearing the name of Vicodin, the opioid Knoll was promoting. This irresponsible marketing of a narcotic drug caused doctors and patients to believe Vicodin was safer than it really was, to the detriment of people in Delta County.

31. Abbvie began manufacturing, developing, promoting, marketing, and selling the opioid drug, Vicodin, in the U.S. and in Delta County beginning January 1, 2013. On information and belief, it continues to do so at the time of filing this pleading.

32. ALLERGAN PLC is a public limited company incorporated in Ireland with its principal place of business in Dublin, Ireland. ACTAVIS PLC acquired Allergan plc in March 2015, and the combined company changed its name to Allergan plc in January 2013. Before that, WATSON PHARMACEUTICALS, INC. acquired ACTAVIS, INC. in October 2012, and the combined company changed its name to ALLERGAN FINANCE, LLC as of October 2013. ALLERGAN FINANCE, LLC is a Nevada Corporation with its principal place of business in

Parsippany, New Jersey, and may be served through its registered agent for service of process, The Corporation Trust Company of Nevada, 701 S. Carson St., Suite 200, Carson City, NV 89701. WATSON LABORATORIES, INC. is a Nevada corporation with its principal place of business in Corona, California, and is a wholly-owned subsidiary of Allergan plc (f/k/a Actavis, Inc., f/k/a Watson Pharmaceuticals, Inc.), and may be served through its registered agent for service of process, Corporate Creations Network, Inc., 8275 South Eastern Ave., #200, Las Vegas, NV 89123. ACTAVIS PHARMA, INC. (f/k/a Actavis, Inc.) is a Delaware corporation with its principal place of business in New Jersey and was formerly known as WATSON PHARMA, INC, and may be served through its registered agent for service of process, CT Corporation System, 1999 Bryan Street, Suite 900, Delta, TX 75201. ACTAVIS LLC is a Delaware limited liability company with its principal place of business in Parsippany, New Jersey, and may be served through its registered agent for service of process, Corporate Creations Network, Inc., 3411 Silverside Rd., Tatnall Building, Suite 104, Wilmington, DE 19810. Each of these Defendants is owned by Allergan plc, which uses them to market and sell its drugs in the United States.

33. Upon information and belief, Allergan plc exercises control over these marketing and sales efforts and profits from the sale of Allergan/Actavis products ultimately inure to its benefit. (Allergan plc, Actavis plc, Actavis, Inc., Allergan Finance, LLC, Actavis LLC, Actavis Pharma, Inc., Watson Pharmaceuticals, Inc., Watson Pharma, Inc., and Watson Laboratories, Inc. are referred to as "Actavis"). Actavis acquired the rights to Kadian from King Pharmaceuticals, Inc. on December 30, 2008 and began marketing Kadian in 2009.

34. Actavis manufactures, promotes, sells, and distributes opioids in the U.S. and in Delta County.

35. MCKESSON CORPORATION ("McKesson") is a Delaware corporation with its

principal place of business in San Francisco, California, and may be served through its registered agent for service of process, CSC - Lawyers Incorporating Service, 211 E. 7th Street, Suite 620, Austin, TX 78701. Upon information and belief, McKesson is a pharmaceutical distributor licensed to do business in Texas. McKesson distributes pharmaceuticals to retail pharmacies and institutional providers in all 50 states, including Texas and Delta.County.

36. CARDINAL HEALTH, INC. ("Cardinal") is an Ohio Corporation with its principal place of business in Dublin, Ohio, and may be served through its registered agent for service of process, CT Corporation System, 4400 Easton Commons, Suite 125, Columbus, OH 43219. Cardinal does substantial business in Texas and, upon information and belief, Cardinal is a pharmaceutical distributor licensed to do business in Texas. Cardinal distributes pharmaceuticals to retail pharmacies and institutional providers to customers in all 50 states, including Texas and Delta County.

37. AMERISOURCEBERGEN DRUG CORPORATION ("Amerisource") is a Delaware Corporation with its principal place of business in Chesterbrook, Pennsylvania, and may be served through its registered agent for service of process, The Corporation Trust Company, Corporation Trust Center, 1209 Orange St., Wilmington, DE 19801. Amerisource does substantial business in Texas and, upon information and belief, Amerisource is a pharmaceutical distributor licensed to do business in Texas. Amerisource distributes pharmaceuticals to retail pharmacies and institutional providers to customers in all 50 states, including Texas and Delta County.

38. DON MARTIN O'NEAL, M.D. is an individual residing in Smith County, Texas, and may be served with citation at 3309 Cripple Creek, Tyler, TX 75707, or wherever he may be found. Dr. O'Neal overprescribed methadone, oxycodone, and hydrocodone[13] to patients in Delta

[13] *"Sulphur Springs Doctor's License Suspended"* (May 24, 2010), http://www/kltv.com/story/12533527/sulphur-springs-doctors-license-suspended.

County and continued to do so after relinquishing his DEA license by using a colleague's signature stamp.[14] Dr. O'Neal's license has been revoked by the Texas Medical Board.[15] Delta County is not, however, seeking damages under claims of medical malpractice or medical professional negligence.

39. The County lacks information sufficient to specifically identify the true names or capacities, whether individual, corporate or otherwise, of Defendants sued herein under the fictitious names DOES 1 through 100 inclusive. The County will amend this Petition to show their true names and capacities if and when they are ascertained. Delta County is informed and believes, and on such information and belief alleges, that each of the Defendants named as a DOE has engaged in conduct that contributed to cause events and occurrences alleged in this Petition and, as such, shares liability for at least some part of the relief sought herein.

## V. FACTUAL ALLEGATIONS

40. Before the 1990s, generally accepted standards of medical practice dictated that opioids should be used only for short-term acute pain – pain relating to recovery from surgery or for cancer or palliative (end-of-life) care. Using opioids for chronic pain was discouraged or even prohibited because there was a lack of evidence that opioids improved patients' ability to overcome pain and function. Instead the evidence demonstrated that patients developed tolerance to opioids over time, which increased the risk of addiction and other side effects.

41. Defendants dramatically changed doctors' views regarding opioids through a well-funded deceptive marketing scheme. Each Defendant used direct marketing and unbranded advertising disseminated by seemingly independent third parties to spread false and deceptive

[14] *Id.*

[15] "Multiple ETX Doctors Disciplined by Medical Board," *last viewed* October 3, 2017, http://easttexasmatters.com/news/local-news-mutiple-etx-doctors-disciplined-by-medical-board/301673397.

statements about the risks and benefits of long-term opioid use.

## A. Defendants Used Multiple Avenues To Disseminate their False and Deceptive Statements about Opioids.

42. Defendants spread their false and deceptive statements by (1) marketing their branded opioids directly to doctors treating patients residing in Delta County and the Delta County patients themselves and (2) deploying so-called unbiased and independent third parties to Delta County.

### 1. Defendants Spread and Continue to Spread Their False and Deceptive Statements Through Direct Marketing of Their Branded Opioids.

43. Defendants' direct marketing of opioids generally proceeded on two tracks. First, each Defendant conducted advertising campaigns touting the purported benefits of their branded drugs. For example, Defendants spent more than $14 million on medical journal advertising of opioids in 2011, nearly triple what they spent in 2001, including $8.3 million by Purdue, $4.9 million by Janssen, and $1.1 million by Endo.

44. A number of Defendants' branded ads deceptively portrayed the benefits of opioids for chronic pain. For example, Endo distributed and made available on its website, www.opana.com, a pamphlet promoting Opana ER with photographs depicting patients with physically demanding jobs like a construction worker and chef, implying that the drug would provide long-term pain-relief and functional improvement. Purdue also ran a series of ads, called "pain vignettes," for OxyContin in 2012 in medical journals. These ads featured chronic pain patients and recommended OxyContin for each. One ad described a "54-year-old writer with osteoarthritis of the hands" and implied that OxyContin would help the writer work more effectively. Pursuant to a settlement agreement, Endo and Purdue agreed in late 2015 and 2016 to halt these misleading representations in New York, but they may continue to disseminate them in

Texas.

45. Second, each Defendant promoted the use of opioids for chronic pain through "detailers" – sales representatives who visited individual doctors and medical staff in their offices – and small-group speaker programs. Defendants devoted massive resources to direct sales contacts with doctors. In 2014 alone, Defendants spent $168 million on detailing branded opioids to doctors, including $108 million by Purdue, $34 million by Janssen, $10 million by Endo, and $2 million by Actavis. This amount is twice as much as Defendants spent on detailing in 2000.

46. Defendants also identified doctors to serve, for payment, on their speakers' bureaus and to attend programs with speakers and meals paid for by Defendants. These speaker programs provided: (1) an incentive for doctors to prescribe a particular opioid (so they might be selected to promote the drug); (2) recognition and compensation for the doctors selected as speakers; and (3) an opportunity to promote the drug through the speaker to his or her peers. These speakers gave the false impression that they were providing unbiased and medically accurate presentations when they were, in fact, presenting a script prepared by Defendants. On information and belief, these presentations conveyed misleading information, omitted material information, and failed to correct Defendants' prior misrepresentations about the risks and benefits of opioids.

47. Defendants employed the same marketing plans, strategies, and messages in and around Delta County, Texas as they did nationwide. Across the pharmaceutical industry, "core message" development is funded and overseen on a national basis by corporate headquarters. This comprehensive approach ensures that Defendants' messages are accurately and consistently delivered across marketing channels and in each sales territory. Defendants consider this high level of coordination and uniformity crucial to successfully marketing their drugs.

### 2. Defendants Used a Diverse Group of Seemingly Independent Third Parties to Spread False and Deceptive Statements about the Risks and Benefits of Opioids.

48. Defendants also deceptively marketed opioids in and around Delta County through unbranded advertising – *i.e.*, advertising that promotes opioid use generally but does not name a specific opioid. This advertising was ostensibly created and disseminated by independent third parties. But by funding, directing, reviewing, editing, and distributing this unbranded advertising, Defendants controlled the deceptive messages disseminated by these third parties and acted in concert with them to falsely and misleadingly promote opioids for treating chronic pain.

49. Unbranded advertising also avoided regulatory scrutiny because Defendants did not have to submit it to the FDA, and therefore it was not reviewed by the FDA.

50. Defendants' deceptive unbranded marketing often contradicted their branded materials reviewed by the FDA. For example, Endo's unbranded advertising contradicted its concurrent, branded advertising for Opana ER:

| **Pain: Opioid Therapy (Unbranded)** | **Opana ER Advertisement (Branded)** |
|---|---|
| "People who take opioids **as prescribed usually do not become addicted.**" | "All patients treated with opioids require careful monitoring for signs of abuse and addiction, since **use of opioid analgesic products carries the risk of addiction even under appropriate medical use.**" |

#### a. Key Opinion Leaders (KOLs)

51. Defendants spoke through a small circle of doctors who, upon information and belief, were selected, funded, and elevated by Defendants because their public positions supported using opioids to treat chronic pain. These doctors became known as "key opinion leaders" or "KOLs."

52. Defendants paid KOLs to serve as consultants or on their advisory boards and to

give talks or present CMEs, and their support helped these KOLs become respected industry experts. As they rose to prominence, these KOLs touted the benefits of opioids to treat chronic pain, repaying Defendants by advancing their marketing goals. KOLs' professional reputations became dependent on continuing to promote a pro-opioid message, even in activities that were not directly funded by Defendants.

53. KOLs have written, consulted on, edited, and lent their names to books and articles, and given speeches and CMEs supportive of chronic opioid therapy. Defendants created opportunities for KOLs to participate in research studies Defendants suggested or chose and then cited and promoted favorable studies or articles by their KOLs. By contrast, Defendants did not support, acknowledge, or disseminate publications of doctors unsupportive or critical of chronic opioid therapy.

54. Defendants' KOLs also served on committees that developed treatment guidelines that strongly encourage using opioids to treat chronic pain, and on the boards of pro-opioid advocacy groups and professional societies that develop, select, and present CMEs. Defendants were able to direct and exert control over each of these activities through their KOLs.

55. Pro-opioid doctors are one of the most important avenues that Defendants use to spread their false and deceptive statements about the risks and benefits of long-term opioid use. Defendants know that doctors rely heavily and less critically on their peers for guidance, and KOLs provide the false appearance of unbiased and reliable support for chronic opioid therapy.

56. Defendants utilized many KOLs, including many of the same ones. Two of the most prominent are described below.

**1. Russell Portenoy**

57. Dr. Russell Portenoy, former Chairman of the Department of Pain Medicine and

Palliative Care at Beth Israel Medical Center in New York, is one example of a KOL who Defendants identified and promoted to further their marketing campaign. Dr. Portenoy received research support, consulting fees, and honoraria from Endo, Janssen, and Purdue (among others), and was a paid consultant to Purdue.

58. Dr. Portenoy was instrumental in opening the door for the regular use of opioids to treat chronic pain. He served on the American Pain Society ("APS")/American Academy of Pain Medicine ("AAPM") Guidelines Committees, which endorsed the use of opioids to treat chronic pain, first in 1997 and again in 2009. He was also a member of the board of the American Pain Foundation ("APF"), an advocacy organization almost entirely funded by Defendants.

59. Dr. Portenoy also made frequent media appearances promoting opioids. He appeared on *Good Morning America* in 2010 to discuss using opioids long-term to treat chronic pain. On this widely-watched program, broadcast in Texas and across the country, Dr. Portenoy claimed: "Addiction, when treating pain, is distinctly uncommon. If a person does not have a history, a personal history, of substance abuse, and does not have a history in the family of substance abuse, and does not have a very major psychiatric disorder, most doctors can feel very assured that that person is not going to become addicted."[16]

60. Dr. Portenoy later admitted that he "gave innumerable lectures in the late 1980s and '90s about addiction that weren't true."[17] These lectures falsely claimed that less than 1% of patients would become addicted to opioids. According to Dr. Portenoy, because the primary goal was to "destigmatize" opioids, he and other doctors promoting them overstated their benefits and glossed over their risks. Dr. Portenoy also conceded that "[d]ata about the effectiveness of opioids

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[16] Good Morning America television broadcast, ABC News (Aug. 30, 2010).

[17] Thomas Catan & Evan Perez, *A Pain-Drug Champion Has Second Thoughts*, WALL ST. J., Dec. 17, 2012.

does not exist."[18] Portenoy candidly stated: "Did I teach about pain management, specifically about opioid therapy, in a way that reflects misinformation? Well...I guess I did."[19]

**2. Lynn Webster**

61. Another KOL, Dr. Lynn Webster, was the co-founder and Chief Medical Director of Lifetree Clinical Research, an otherwise unknown pain clinic in Salt Lake City, Utah. Dr. Webster was President in 2013 and is a current board member of AAPM, a Front Group that ardently supports chronic opioid therapy. He is a Senior Editor of *Pain Medicine*, the same journal that published Endo special advertising supplements touting Opana ER. Dr. Webster authored numerous CMEs sponsored by Endo and Purdue while he was receiving significant funding from Defendants.

62. In 2011, Dr. Webster presented a program via webinar sponsored by Purdue titled, *Managing Patient's Opioid Use: Balancing the Need and the Risk.* Dr. Webster recommended using risk screening tools, such as urine testing and patient agreements as a way to prevent "overuse of prescriptions" and "overdose deaths," which was available to and was intended to reach doctors treating Delta County residents.

63. Dr. Webster also was a leading proponent of the concept of "pseudoaddiction," the notion that addictive behaviors should be seen not as warnings, but as indications of undertreated pain. In Dr. Webster's description, the only way to differentiate the two was to *increase* a patient's dose of opioids. As he and his co-author wrote in a book entitled *Avoiding Opioid Abuse While Managing Pain* (2007), a book that is still available online, when faced with signs of aberrant behavior, increasing the dose "in most cases . . . should be the clinician's first response." Endo distributed this book to doctors. Years later, Dr. Webster reversed himself, acknowledging that

[18] *Id.*
[19] *Id.*

"[pseudoaddiction] obviously became too much of an excuse to give patients more medication."[20]

**b. Front Groups**

64. Defendants entered into arrangements with seemingly unbiased and independent patient and professional organizations to promote opioids for treating chronic pain. Under Defendants' direction and control, these "Front Groups" generated treatment guidelines, unbranded materials, and programs that favored chronic opioid therapy. They also assisted Defendants by responding to negative articles, by advocating against regulatory changes that would limit prescribing opioids in accordance with the scientific evidence, and by conducting outreach to vulnerable patient populations targeted by Defendants.

65. These Front Groups depended on Defendants for funding and, in some cases, for survival. Defendants also exercised control over programs and materials created by these groups by collaborating on, editing, and approving their content, and by funding their dissemination. In doing so, Defendants made sure these Groups would generate only the messages Defendants wanted to distribute. Even so, the Front Groups held themselves out as independent and as serving the needs of their members – whether patients suffering from pain or doctors treating those patients.

66. Defendants Endo, Janssen, and Purdue utilized many Front Groups, including many of the same ones. Several of the most prominent are described below, but there are many others, including the American Pain Society ("APS"), American Geriatrics Society ("AGS"), the Federation of State Medical Boards ("FSMB"), American Chronic Pain Association ("ACPA"), American Society of Pain Education ("ASPE"), National Pain Foundation ("NPF") and Pain & Policy Studies Group ("PPSG").

[20] John Fauber & Ellen Gabler, *Networking Fuels Painkiller Boom*, MILWAUKEE WISC. J. SENTINEL (Feb. 19, 2012).

**1. American Pain Foundation ("APF")**

67. The most prominent of Defendants' Front Groups was APF, which received more than $10 million in funding from opioid manufacturers from 2007 until it closed its doors in May 2012. Endo alone provided more than half that funding; Purdue was next at $1.7 million.

68. In 2009 and 2010, more than 80% of APF's operating budget came from pharmaceutical industry sources. Including industry grants for specific projects, APF received about $2.3 million from industry sources out of total income of about $2.85 million in 2009; its budget for 2010 projected receipts of roughly $2.9 million from drug companies, out of total income of about $3.5 million. By 2011, APF was entirely dependent on incoming grants from defendants Purdue, Endo, and others to avoid using its line of credit. As one of its board members, Russell Portenoy, explained the lack of funding diversity was one of the biggest problems at APF.

69. APF issued education guides for patients, reporters, and policymakers that touted the benefits of opioids for chronic pain and trivialized their risks, particularly the risk of addiction. APF also engaged in a significant multimedia campaign – through radio, television, and the internet – to educate patients about their "right" to pain treatment, namely opioids. All of the programs and materials were available nationally and were intended to reach patients and consumers in Delta County.

**2. American Academy of Pain Medicine ("AAPM")**

70. The American Academy of Pain Medicine, with the assistance, prompting, involvement, and funding of Defendants, issued treatment guidelines and sponsored and hosted medical education programs essential to Defendants' deceptive marketing of chronic opioid therapy.

71. AAPM received over $2.2 million in funding since 2009 from opioid

manufacturers. AAPM maintained a corporate relations council, whose members paid $25,000 per year (on top of other funding) to participate. The benefits included allowing members to present educational programs at off-site dinner symposia in connection with AAPM's marquee event – its annual meeting held in Palm Springs, California, or other resort locations. AAPM describes the annual event as an "exclusive venue" for offering education programs to doctors. Membership in the corporate relations council also allows drug company executives and marketing staff to meet with AAPM executive committee members in small settings. Defendants Endo, Purdue, and Actavis were members of the council and presented deceptive programs to doctors who attended this annual event.

72. AAPM is viewed internally by Endo as "industry friendly," with Endo advisors and speakers among its active members. Endo attended AAPM conferences, funded its CMEs, and distributed its publications. The conferences sponsored by AAPM heavily emphasized sessions on opioids – 37 out of roughly 40 at one conference alone. AAPM's presidents have included top industry-supported KOLs Perry Fine, Russell Portenoy, and Lynn Webster. Dr. Webster was even elected president of AAPM while under a DEA investigation. Another past AAPM president, Dr. Scott Fishman, stated that he would place the organization "at the forefront" of teaching that "the risks of addiction are . . . small and can be managed."[21]

73. AAPM's staff understood they and their industry funders were engaged in a common task. Defendants were able to influence AAPM through both their significant and regular funding and the leadership of pro-opioid KOLs within the organization.

74. In 1997, AAPM and the American Pain Society jointly issued a consensus

[21] Interview by Paula Moyer with Scott M. Fishman, M.D., Professor of Anesthesiology and Pain Medicine, Chief of the Division of Pain Medicine, Univ. of Cal., Davis (2005), http://www.medscape.org/viewarticle/500829.

statement, *The Use of Opioids for the Treatment of Chronic Pain*, which endorsed opioids to treat chronic pain and claimed there was a low risk that patients would become addicted to opioids. The co-author of the statement, Dr. Haddox, was at the time a paid speaker for Purdue. Dr. Portenoy was the sole consultant. The consensus statement remained on AAPM's website until 2011, and was taken down from AAPM's website only after a doctor complained, though it still lingers on the internet elsewhere.

75. AAPM and APS issued their own guidelines in 2009 ("AAPM/APS Guidelines") and continued to recommend using opioids to treat chronic pain. Fourteen of the 21 panel members who drafted the AAPM/APS Guidelines, including KOLs Dr. Portenoy and Dr. Perry Fine of the University of Utah, received support from Janssen, Endo, and Purdue.

76. The 2009 Guidelines promote opioids as "safe and effective" for treating chronic pain, despite acknowledging limited evidence, and conclude that the risk of addiction is manageable for patients regardless of past abuse histories. One panel member, Dr. Joel Saper, Clinical Professor of Neurology at Michigan State University and founder of the Michigan Headache & Neurological Institute, resigned from the panel because he was concerned the 2009 Guidelines were influenced by contributions that drug companies, including Defendants, made to the sponsoring organizations and committee members. These AAPM/APS Guidelines have been a particularly effective channel of deception and have influenced not only treating physicians, but also the body of scientific evidence on opioids. The Guidelines have been cited 732 times in academic literature, were disseminated in and around Delta County during the relevant time period, are still available online, and were reprinted in the *Journal of Pain.*

**B. Defendants' Marketing Scheme Misrepresented the Risks and Benefits of Opioids.**

77. To convince doctors treating residents in Delta County and Delta County patients

that opioids can and should be used to treat chronic pain, Defendants had to convince them that long-term opioid use is both safe and effective. Knowing they could do so only by deceiving those doctors and patients about the risks and benefits of long-term opioid use, Defendants made claims that were not supported by, or were contrary to, the scientific evidence. Even though pronouncements by and guidance from the FDA and the CDC based on that evidence confirm that their claims were false and deceptive, Defendants have not corrected them, or instructed their KOLs or Front Groups to correct them, and continue to spread them today.

### C. Defendants Falsely Trivialized or Failed to Disclose the Known Risks of Long-Term Opioid Use.

78. To convince doctors and patients that opioids are safe, Defendants deceptively trivialized and failed to disclose the risks of long-term opioid use, particularly the risk of addiction, through a series of misrepresentations that have been conclusively debunked by the FDA and CDC. These misrepresentations – which are described below – reinforced each other and created the dangerously misleading impression that: (1) starting patients on opioids was low-risk because most patients would not become addicted, and because those who were at greatest risk of addiction could be readily identified and managed; (2) patients who displayed signs of addiction probably were not addicted and, in any event, could easily be weaned from the drugs; (3) the use of higher opioid doses, which many patients need to sustain pain relief as they develop tolerance to the drugs, do not pose special risks; and (4) abuse-deterrent opioids both prevent abuse and overdose and are inherently less addictive. Defendants have not only failed to correct these misrepresentations, they continue to make them today.

79. ***First***, Defendants falsely claimed the risk of addiction is low and unlikely to develop when opioids are prescribed, as opposed to obtained illicitly; and failed to disclose the greater risk of addiction with prolonged use of opioids. For example:

a. Actavis's predecessor caused a patient education brochure to be distributed in 2007 claiming opioid addiction is possible, but "less likely if you have never had an addiction problem." Upon information and belief, based on Actavis's acquisition of its predecessor's marketing materials along with the rights to Kadian, Actavis continued to use this brochure in 2009 and beyond;

b. Purdue sponsored APF's *Treatment Options: A Guide for People Living with Pain* (2007), which instructed that addiction is rare and limited to extreme cases of unauthorized dose escalations, obtaining duplicative opioid prescriptions from multiple sources, or theft. This publication is still available online;

c. Endo sponsored a website, Painknowledge.com, which claimed in 2009 that "[p]eople who take opioids as prescribed usually do not become addicted." Another Endo website, PainAction.com, stated "Did you know? Most chronic pain patients do not become addicted to the opioid medications that are prescribed for them;"

d. Endo distributed a pamphlet with the Endo logo entitled *Living with Someone with Chronic Pain*, which stated that: "Most health care providers who treat people with pain agree that most people do not develop an addiction problem." A similar statement appeared on the Endo website, www.opana.com;

e. Janssen reviewed, edited, approved, and distributed a patient education guide entitled *Finding Relief: Pain Management for Older Adults* (2009), which described as "myth" the claim that opioids are addictive, and asserted as fact that "[m]any studies show that opioids are rarely addictive when used properly for the management of chronic pain;"

f. Janssen currently runs a website, Prescriberesponsibly.com (last updated July 2, 2015), which claims that concerns about opioid addiction are "overestimated;"

g. Purdue sponsored APF's *A Policymaker's Guide to Understanding Pain & Its Management* – which claims that less than 1% of children prescribed opioids will become addicted and that pain is undertreated due to "misconceptions about opioid addiction[]." This publication is still available online; and

h. Detailers for Purdue, Endo, and Janssen in and around Delta County minimized or omitted any discussion with doctors of the risk of addiction; misrepresented the potential for opioid abuse with purportedly abuse-deterrent formulations; and routinely did not correct the misrepresentations noted above.

80. These claims contradict longstanding scientific evidence, as the FDA and CDC have conclusively declared. As noted in the 2016 CDC Guideline endorsed by the FDA, there is "extensive evidence" of the "possible harms of opioids (including opioid use disorder [an alternative term for opioid addiction])."[22] The guideline points out that "[o]pioid pain medication use presents serious risks, including...opioid use disorder" and that "continuing opioid therapy for 3 months substantially increases risk for opioid use disorder."[23]

81. The FDA further exposed the falsity of Defendants' claims about the low risk of addiction when it announced changes to the labels for ER/LA opioids in 2013 and for IR opioids in 2016. In its announcements, the FDA discussed the risks related to opioid use and that IR opioids are associated with "persistent abuse, addiction, overdose mortality, and risk of NOWS [neonatal opioid withdrawal syndrome]."[24]

82. According to the FDA, because of the risks associated with long-term opioid use, including "the serious risk of addiction, abuse, misuse, overdose, and death,"[25] opioids should be "reserved for pain severe enough to require opioid treatment and for which alternative treatment options (e.g., non-opioid analgesics or opioid combination products, as appropriate) are inadequate or not tolerated."[26]

83. The warnings on Defendants' own FDA-approved drug labels caution that opioids "exposes users to risks of addiction, abuse and misuse, which can lead to overdose and death"[27]

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[22] *CDC Guideline for Prescribing Opioids for Chronic Pain – United States 2016*, Centers for Disease Control and Prevention (Mar. 18, 2016).
[23] *Id.*
[24] *FDA Announcement of Enhanced Warnings for Immediate-Release Opioid Pain Medications Related to Risks of Misuse, Abuse, Addiction, Overdose and Death*, Federal Drug Administration (Mar. 22, 2016).
[25] *Id.*
[26] *Id.*
[27] *See, e.g.*, OxyContin label and insert at *OxyContin.com.*

and that addiction "can occur in patients appropriately prescribed"[28] opioids.

84. ***Second***, Defendants falsely instructed doctors and patients that signs of addiction are actually signs of undertreated pain and should be treated by prescribing more opioids. Defendants called this phenomenon "pseudoaddiction" – a term coined by Dr. David Haddox, who went to work for Purdue, and popularized by Dr. Russell Portenoy, a KOL for Endo, Janssen, and Purdue – and claimed that pseudoaddiction is substantiated by scientific evidence. For example:

a. Purdue sponsored *Responsible Opioid Prescribing* (2007), which taught that behaviors such as "requesting drugs by name," "demanding or manipulative behavior," seeing more than one doctor to obtain opioids, and hoarding, are all signs of pseudoaddiction, rather than true addiction. Responsible Opioid Prescribing remains for sale online. The 2012 edition continues to teach that pseudoaddiction is real;

b. Janssen sponsored, funded, and edited the *Let's Talk Pain* website, which in 2009 stated: "pseudoaddiction . . . refers to patient behaviors that may occur when pain is under-treated . . . . Pseudoaddiction is different from true addiction because such behaviors can be resolved with effective pain management;"

c. Endo sponsored a National Initiative on Pain Control (NIPC) CME program in 2009 titled *Chronic Opioid Therapy: Understanding Risk While Maximizing Analgesia*, which promoted pseudoaddiction by teaching that a patient's aberrant behavior was the result of untreated pain. Endo substantially controlled NIPC by funding NIPC projects; developing, specifying, and reviewing content; and distributing NIPC materials;

d. Purdue published a pamphlet in 2011 entitled *Providing Relief, Preventing Abuse*, which described pseudoaddiction as a concept that "emerged in the literature" to describe the inaccurate interpretation of [drug-seeking behaviors] in patients who have pain that has not been effectively treated;" and

e. Purdue sponsored a CME program entitled *Path of the Patient, Managing Chronic Pain in Younger Adults at Risk for Abuse*. In a role play, a chronic pain patient with a history of drug abuse tells his doctor that he is taking twice as many hydrocodone pills as directed. The narrator notes that because of pseudoaddiction, the doctor should not assume the patient

[28] *Id.*

is addicted even if he persistently asks for a specific drug, seems desperate, hoards medicine, or "overindulges in unapproved escalating doses." The doctor treats this patient by prescribing a high-dose, long-acting opioid.

85. The 2016 CDC Guideline rejects the concept of pseudoaddiction. The Guideline nowhere recommends that opioid dosages be increased if a patient is not experiencing pain relief. To the contrary, the Guideline explains that "[p]atients who do not experience clinically meaningful pain relief early in treatment...are unlikely to experience pain relief with longer-term use,"[29] and that physicians should "reassess[] pain and function within 1 month"[30] in order to decide whether to "minimize risks of long-term opioid use by discontinuing opioids"[31] because the patient is "not receiving a clear benefit."[32]

86. ***Third,*** Defendants falsely instructed doctors and patients that addiction risk screening tools, patient contracts, urine drug screens, and similar strategies allow them to reliably identify and safely prescribe opioids to patients predisposed to addiction. These misrepresentations were especially insidious because Defendants aimed them at general practitioners and family doctors who lack the time and expertise to closely manage higher-risk patients. Defendants' misrepresentations made these doctors feel more comfortable prescribing opioids to their patients, and patients more comfortable starting opioid therapy for chronic pain. For example:

a. Endo paid for a 2007 supplement in the *Journal of Family Practice* written by a doctor who became a member of Endo's speakers' bureau in 2010. The supplement, entitled *Pain Management Dilemmas in Primary Care: Use of Opioids*, emphasized the effectiveness of screening tools, claiming that patients at high risk of addiction could safely receive chronic opioid therapy using a "maximally structured approach" involving toxicology screens and pill counts;

b. Purdue sponsored a 2011 webinar, *Managing Patient's Opioid Use:*

[29] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[30] *Id.*
[31] *Id.*
[32] *Id.*

*Balancing the Need and Risk*, which claimed that screening tools, urine tests, and patient agreements prevent "overuse of prescriptions" and "overdose deaths;" and

c. As recently as 2015, Purdue has represented in scientific conferences that "bad apple" patients – and not opioids – are the source of the addiction crisis and that once those "bad apples" are identified, doctors can safely prescribe opioids without causing addiction.

87. Once again, the 2016 CDC Guideline confirms these representations are false. The Guideline notes that there are no studies assessing the effectiveness of risk mitigation strategies – such as screening tools, patient contracts, urine drug testing, or pill counts, widely believed by doctors to detect and deter abuse, "for improving outcomes related to overdose, addiction, abuse, or misuse."[33] As a result, the Guideline recognizes that available risk screening tools "show insufficient accuracy for classification of patients as at low or high risk for [opioid] abuse or misuse"[34] and counsels that doctors "should not overestimate the ability of these tools to rule out risks from long-term opioid therapy."[35]

88. ***Fourth***, to underplay the risk and impact of addiction and make doctors feel more comfortable starting patients on opioids, Defendants falsely claimed that opioid dependence can easily be addressed by tapering and that opioid withdrawal is not a problem thereby failing to disclose the increased difficulty of stopping opioids after long-term use.

89. For example, a CME sponsored by Endo, entitled *Persistent Pain in the Older Adult*, claimed that withdrawal symptoms can be avoided by tapering a patient's opioid dose by 10%-20% for 10 days. And Purdue sponsored APF's *A Policymaker's Guide to Understanding Pain & Its Management*, which claimed that "[s]ymptoms of physical dependence can often be ameliorated by gradually decreasing the dose of medication during discontinuation."

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[33] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[34] *Id.*
[35] *Id.*

90. Defendants deceptively minimized the significant symptoms of opioid withdrawal, which, as explained in the 2016 CDC Guideline, include drug cravings, anxiety, insomnia, abdominal pain, vomiting, diarrhea, sweating, tremor, tachycardia (rapid heartbeat), spontaneous abortion and premature labor in pregnant women, and the unmasking of anxiety, depression, and addiction – and grossly understated the difficulty of tapering, particularly after long-term opioid use.

91. Yet the 2016 CDC Guideline recognizes that the duration of opioid use and the dosage of opioids prescribed should be limited to "minimize the need to taper opioids to prevent distressing or unpleasant withdrawal symptoms,"[36] because "physical dependence on opioids is an expected physiologic response in patients exposed to opioids for more than a few days."[37] The Guideline further states that "tapering opioids can be especially challenging after years on high dosages because of physical and psychological dependence"[38] and highlights the difficulties, including the need to carefully identify "a taper slow enough to minimize symptoms and signs of opioid withdrawal"[39] and pausing and restarting tapers depending on the patient's response.

92. The CDC also acknowledges the lack of any "high-quality studies comparing the effectiveness of different tapering protocols for use when opioid dosage is reduced or opioids are discontinued."[40]

93. ***Fifth***, Defendants falsely claimed that doctors and patients could increase opioid dosages indefinitely without added risk and failed to disclose the greater risks to patients at higher dosages. The ability to escalate dosages was critical to Defendants' efforts to market opioids for

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[36] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[37] *Id.*
[38] *Id.*
[39] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[40] *Id.*

long-term use to treat chronic pain because, absent this misrepresentation, doctors would have abandoned treatment when patients built up tolerance and lower dosages did not provide pain relief. For example:

a. Actavis's predecessor created a patient brochure for Kadian in 2007 that stated, "Over time, your body may become tolerant of your current dose. You may require a dose adjustment to get the right amount of pain relief. This is not addiction." Upon information and belief, based on Actavis's acquisition of its predecessor's marketing materials along with the rights to Kadian, Actavis continued to use these materials in 2009 and beyond;

b. Purdue sponsored *APF's Treatment Options: A Guide for People Living with Pain* (2007), which claims that some patients "need" a larger dose of an opioid, regardless of the dose currently prescribed. The guide stated that opioids have "no ceiling dose" and are therefore the most appropriate treatment for severe pain. This guide is still available for sale online;

c. Endo sponsored a website, painknowledge.com, which claimed in 2009 that opioid dosages may be increased until "you are on the right dose of medication for your pain;"

d. Endo distributed a pamphlet edited by a KOL entitled *Understanding Your Pain: Taking Oral Opioid Analgesics*, which was available during the time period of this Complaint on Endo's website. In Q&A format, it asked "If I take the opioid now, will it work later when I really need it?" The response is, "The dose can be increased. . . . You won't 'run out' of pain relief;"

e. Janssen sponsored a patient education guide entitled *Finding Relief: Pain Management for Older Adults* (2009), which was distributed by its sales force. This guide listed dosage limitations as "disadvantages" of other pain medicines but omitted any discussion of risks of increased opioid dosages;

f. Purdue's In the Face of Pain website promotes the notion that if a patient's doctor does not prescribe what, in the patient's view, is a sufficient dosage of opioids, he or she should find another doctor who will;

g. Purdue sponsored APF's *A Policymaker's Guide to Understanding Pain & Its Management*, which taught that dosage escalations are "sometimes necessary," even unlimited ones, but did not disclose the risks from high opioid dosages. This publication is still available online;

h. Purdue sponsored a CME entitled *Overview of Management Options* that is still available for CME credit. The CME was edited by a KOL and taught that NSAIDs and other drugs, but not opioids, are unsafe at high dosages;

and

i. Purdue presented a 2015 paper at the College on the Problems of Drug Dependence, the "the oldest and largest organization in the US dedicated to advancing a scientific approach to substance use and addictive disorders," challenging the correlation between opioid dosage and overdose.

94. These claims conflict with the scientific evidence, as confirmed by the FDA and CDC. As the CDC explains in its 2016 Guideline, the "[b]enefits of high-dose opioids for chronic pain are not established"[41] while the "risks for serious harms related to opioid therapy increase at higher opioid dosage."[42]

95. More specifically, the CDC explains that "there is now an established body of scientific evidence showing that overdose risk is increased at higher opioid dosages."[43] Similarly, there is an "increased risk for opioid use disorder, respiratory depression, and death at higher dosages."[44] That is why the CDC advises doctors to avoid increasing dosages above 90 morphine milligram equivalents per day.

96. The 2016 CDC Guideline reinforces earlier findings announced by the FDA. In 2013, the FDA acknowledged that available data suggested that increasing the opioid dosage likewise increased certain adverse events. For example, the FDA noted that studies suggest a positive association between high-dose opioid use and overdoses.

97. ***Finally***, Defendants' deceptive marketing of the so-called abuse-deterrent properties of some of their opioids has created false impressions that these opioids can curb addiction and abuse.

98. More specifically, Defendants have made misleading claims about the ability of

[41] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[42] *Id.*
[43] *Id.*
[44] *Id.*

their so-called abuse-deterrent opioid formulations to deter abuse. For example, Endo's advertisements for the 2012 reformulation of Opana ER claimed that it was designed to be crush resistant in a way that suggested it was more difficult to abuse. This claim was false.

99. The FDA warned in a 2013 letter that there was no evidence Endo's design would provide a reduction in oral, intranasal or intravenous abuse.[45] Moreover, Endo's own studies, which it failed to disclose, showed that Opana ER could still be ground and chewed.

100. In a 2016 settlement with the State of New York, Endo agreed not to make statements in New York that Opana ER was designed to be or is crush resistant. The State found those statements false and deceptive because there was no difference in the ability to extract the narcotic from Opana ER.

101. Similarly, the 2016 CDC Guideline states that no studies support the notion that "abuse-deterrent technologies [are] a risk mitigation strategy for deterring or preventing abuse,"[46] noting that the technologies – even when they work – "do not prevent opioid abuse through oral intake, the most common route of opioid abuse, and can still be abused by non-oral routes."[47]

102. These numerous, long-standing misrepresentations of the risks of long-term opioid use spread by Defendants successfully convinced doctors and patients to discount those risks.

**D. Defendants Grossly Overstated the Benefits of Chronic Opioid Therapy.**

103. To convince doctors and patients that opioids should be used to treat chronic pain, Defendants had to persuade them that there was a significant benefit to long-term opioid use. But as the 2016 CDC Guideline makes clear, there is "insufficient evidence to determine the long-term benefits of opioid therapy for chronic pain."[48]

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[45] *See FDA Statement: Original Opana ER Relisting Determination* (May 10, 2013).
[46] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[47] *Id.*
[48] *Id.*

104. In fact, the CDC found no evidence showing "a long-term benefit of opioids in pain and function versus no opioids for chronic pain with outcomes examined at least 1 year later (with most placebo-controlled randomized trials ≤ 6 weeks in duration)"[49] and that other treatments were more or equally beneficial and less harmful than long-term opioid use. The FDA, too, has recognized the lack of evidence to support long-term opioid use.

105. In 2013, the FDA stated that it was unaware of any studies demonstrating the safety and efficacy of opioids for long-term use.[50] Despite this lack of studies, Defendants falsely and misleadingly touted the benefits of long-term opioid use and suggested that these benefits were supported by scientific evidence. Not only have Defendants failed to correct these false and deceptive claims, they continue to make them today. For example:

a. Actavis distributed an advertisement that claimed that the use of Kadian to treat chronic pain would allow patients to return to work, relieve "stress on your body and your mental health," and help patients enjoy their lives;

b. Endo distributed advertisements that claimed that the use of Opana ER for chronic pain would allow patients to perform demanding tasks like construction work or work as a chef and portrayed seemingly healthy, unimpaired subjects;

c. Janssen sponsored and edited a patient education guide entitled *Finding Relief: Pain Management for Older Adults* (2009) – which states as "a fact" that "opioids may make it easier for people to live normally." The guide lists expected functional improvements from opioid use, including sleeping through the night, returning to work, recreation, sex, walking, and climbing stairs;

d. Purdue ran a series of advertisements for OxyContin in 2012 in medical journals entitled "pain vignettes," which were case studies featuring patients with pain conditions persisting over several months and recommending OxyContin for them. The ads implied that OxyContin improves patients' function;

e. *Responsible Opioid Prescribing* (2007), sponsored and distributed by Endo

[49] *Id.*

[50] Letter from Janet Woodcock, M.D., Dir., Ctr. For Drug Eval. & Res., to Andrew Kolodny, M.D., Pres. Physicians for Responsible Opioid Prescribing, Re Docket No. FDA-2012-P-0818 (Sept. 10, 2013).

and Purdue, taught that relief of pain by opioids, by itself, improved patients' function. The book remains for sale online;

f. Purdue sponsored APF's *Treatment Options: A Guide for People Living with Pain* (2007), which counseled patients that opioids "give [pain patients] a quality of life we deserve." The guide was available online until APF shut its doors in 2012;

g. Endo's NIPC website *painknowledge.com* claimed in 2009 that with opioids, "your level of function should improve; you may find you are now able to participate in activities of daily living, such as work and hobbies, that you were not able to enjoy when your pain was worse." Elsewhere, the website touted improved quality of life (as well as "improved function") as benefits of opioid therapy. The grant request that Endo approved for this project specifically indicated NIPC's intent to make misleading claims about function, and Endo closely tracked visits to the site;

h. Endo was the sole sponsor, through NIPC, of a series of CMEs titled *Persistent Pain in the Older Patient*, which claimed that chronic opioid therapy has been "shown to reduce pain and improve depressive symptoms and cognitive functioning." The CME was disseminated via webcast;

i. Janssen sponsored, funded, and edited a website, *Let's Talk Pain*, in 2009, which featured an interview edited by Janssen claiming that opioids allowed a patient to "continue to function." This video is still available today on YouTube;

j. Purdue sponsored the development and distribution of APF's *A Policymaker's Guide to Understanding Pain & Its Management*, which claimed that "multiple clinical studies" have shown that opioids are effective in improving daily function, psychological health, and health-related quality of life for chronic pain patients." The Policymaker's Guide was originally published in 2011 and is still available online today; and

k. Purdue's, Endo's, and Janssen's sales representatives have conveyed and continue to convey the message that opioids will improve patient function.

106. These claims find no support in the scientific literature. Most recently, the 2016 CDC Guideline, approved by the FDA, concluded, "There is no good evidence that opioids improve pain or function with long-term use"[51] and "complete relief of pain is unlikely."[52]

[51] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[52] *Id.*

(Emphasis added.) The CDC reinforced this conclusion throughout its 2016 Guideline:

a. "No evidence shows a long-term benefit of opioids in pain and function versus no opioids for chronic pain with outcomes examined at least 1 year later . . ."[53]

b. "Although opioids can reduce pain during short-term use, the clinical evidence review found insufficient evidence to determine whether pain relief is sustained and whether function or quality of life improves with long-term opioid therapy;"[54] and

c. "[E]vidence is limited or insufficient for improved pain or function with long-term use of opioids for several chronic pain conditions for which opioids are commonly prescribed, such as low back pain, headache, and fibromyalgia."[55]

107. The CDC also noted that the risks of addiction and death "can cause distress and inability to fulfill major role obligations."[56] As a matter of common sense (and medical evidence), drugs that can kill patients or commit them to a life of addiction or recovery do not improve their function and quality of life.

108. The 2016 CDC Guideline was not the first time a federal agency repudiated Defendants' claim that opioids improved function and quality of life. In 2010, the FDA warned Actavis that "[w]e are not aware of substantial evidence or substantial clinical experience demonstrating that the magnitude of the effect of the drug [Kadian] has in alleviating pain, taken together with any drug-related side effects patients may experience...results in any overall positive impact on a patient's work, physical and mental functioning, daily activities, or enjoyment of life."[57]

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[53] *Id.*
[54] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[55] *Id.*
[56] *Id.*
[57] Warning Letter from Thomas Abrams, Dir., FDA Div. of Mktg., Adver., & Commc'ns, to Doug Boothe, CEO, Actavis Elizabeth LLC (Feb. 18, 2010), http://www.fda.gov/Drugs/GuidanceComplianceRegulatoryInformation/EnforcementActivitiesbyFDA/WarningLettersandNoticeofViolationLetterstoPharmaceuticalCcompanies/ucm259240.htm.

109. Defendants also falsely emphasized or exaggerated the risks of competing products like NSAIDs so that doctors and patients would look to opioids first for treating chronic pain. Once again, Defendants' misrepresentations contravene pronouncements by and guidance from the FDA and CDC based on the scientific evidence.

110. Consequently, the FDA changed the labels for ER/LA opioids in 2013 and IR opioids in 2016 to state that opioids should be used only as a last resort where alternative treatments like non-opioid drugs are inadequate. And the 2016 CDC Guideline states that NSAIDs, not opioids, should be the first-line treatment for chronic pain, particularly arthritis and lower back pain.

111. In addition, Purdue misleadingly promoted OxyContin as unique among opioids in providing 12 continuous hours of pain relief with one dose. In fact, OxyContin does not last for 12 hours – a fact that Purdue has known at all times relevant to this action.

112. According to Purdue's own research, OxyContin wears off in under six hours in one quarter of patients and in under 10 hours in more than half. The reason is that OxyContin tablets release approximately 40% of their active medicine immediately, after which release tapers. Although the patient experiences a powerful initial response, there is little or no pain relief at the end of the dosing period because less medicine is released.

113. This phenomenon is known as "end of dose" failure, and the FDA found in 2008 that a substantial number of chronic pain patients taking OxyContin experience it.

114. This "end of dose" failure not only renders Purdue's promise of 12 hours of relief false and deceptive, it also makes OxyContin more dangerous because the declining pain relief patients experience toward the end of each dosing period drives them to take more OxyContin before the next dosing period begins, quickly increasing the amount of drug they are taking and

spurring growing dependence.

115. Purdue's competitors were aware of this problem. For example, Endo ran advertisements for Opana ER referring to "real" 12-hour dosing. Nevertheless, Purdue falsely promoted OxyContin as if it were effective for a full 12 hours. Indeed, Purdue's sales representatives continue to tell doctors in and around Delta County that OxyContin lasts a full 12 hours.

**E. Defendants also engaged in Other Unlawful, Unfair, and Fraudulent Misconduct.**

116. Defendants herein participated in illicit and unlawful prescribing of its drugs. For example, Purdue did not report illegal prescribing of OxyContin until years after law enforcement shut down a Los Angeles clinic that prescribed more than 1.1 million OxyContin tablets. In doing so, Purdue protected its own profits at the expense of public health and safety.

117. The State of New York also found that Endo failed to require sales representatives to report signs of abuse, diversion, and inappropriate prescribing; paid bonuses to sales representatives for detailing prescribers who were subsequently arrested or convicted for illegal prescribing; and failed to prevent sales representatives from visiting prescribers whose suspicious conduct had caused them to be placed on a no-call list.

**F. Defendants Targeted Susceptible Prescribers and Vulnerable Patient Populations.**

118. As a part of their deceptive marketing scheme, Defendants identified and targeted susceptible prescribers and vulnerable patient populations in the U.S. and in and around Delta County. For example, Defendants focused their deceptive marketing on primary care doctors, who were more likely to treat chronic pain patients and prescribe opioids, but were less likely to be educated about treating pain and the risks and benefits of opioids.

119. Defendants also targeted vulnerable patient populations like the elderly and

veterans, who tend to suffer from chronic pain. Defendants targeted these vulnerable patients even though the risks of long-term opioid use were significantly greater for them.

120. For example, the 2016 CDC Guideline observes that existing evidence shows that elderly patients taking opioids suffer from elevated fall and fracture risks, greater risk of hospitalization, and increased vulnerability to adverse drug effects and interactions. The Guideline therefore concludes that there are "special risks of long-term opioid use for elderly patients" and recommends that doctors use "additional caution and increased monitoring" to minimize the risks of opioid use in elderly patients.

121. The same is true for veterans, who are more likely to use anti-anxiety drugs (benzodiazepines) for post-traumatic stress disorder, which interact dangerously with opioids.

**G. Although Defendants knew that their Marketing of Opioids was False and Deceptive, they Fraudulently Concealed their Misconduct.**

122. Defendants, both individually and collectively, made, promoted, and profited from their misrepresentations about the risks and benefits of opioids for chronic pain even though they knew their misrepresentations were false and deceptive. The history of opioids, as well as research and clinical experience over the last 20 years, established that opioids were highly addictive and responsible for a long list of very serious adverse outcomes.

123. Not only did the FDA and other regulators warn Defendants, but Defendants had access to scientific studies, detailed prescription data, and reports of adverse events, including reports of addiction, hospitalization, and deaths – all of which made clear the harms from long-term opioid use, including the suffering from addiction, overdoses, and death in alarming numbers in patients using opioids.

124. More recently, the FDA and CDC have issued pronouncements based on the medical evidence that conclusively expose the known falsity of Defendants' misrepresentations,

and Endo and Purdue have recently entered agreements prohibiting them from making some of the same misrepresentations described herein in New York.

125. Specifically, three current and former executives from Purdue plead guilty in 2007 to criminal charges that they misled regulators, doctors, and patients about OxyContin's risk of addiction.[58] In pleading guilty to misbranding charges, Purdue admitted it had fraudulently marketed OxyContin as a drug less prone to addiction and as having fewer side effects than other opioids.[59] In reality, unlike other opioids, OxyContin contained pure oxycodone without any other ingredients, which made it a powerful narcotic despite its time-release design that Purdue touted as ameliorating its addictive potential.[60]

126. Moreover, Defendants took steps to avoid detection of and to fraudulently conceal their deceptive marketing and unlawful, unfair, and fraudulent conduct. For example, Defendants disguised their own role in the deceptive marketing of chronic opioid therapy by funding and working through third parties like Front Groups and KOLs.

127. Finally, Defendants manipulated their promotional materials and the scientific literature to make it appear that these items were accurate, truthful, and supported by objective evidence when they were not.

128. Thus, Defendants successfully concealed from the medical community and patients facts sufficient to arouse suspicion of the claims Delta County now asserts. Delta County did not know of the existence or scope of Defendants' industry-wide fraud and could not have acquired such knowledge earlier through the exercise of reasonable diligence.

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[58] *See* Barry Meier, "In Guilty Plea, OxyContin Maker to Pay $600 Million," (May 10, 2007), http://www.nytimes.com/2007/05/10/business/11drug-web.html.
[59] *See id.*
[60] *See id.*

## H. By Increasing Opioid Prescriptions and Use, Defendants' Deceptive Marketing Scheme has fueled the Opioid Epidemic and Devastated Delta County Communities.

129. Defendants' misrepresentations deceived doctors and patients about the risks and benefits of long-term opioid use. Studies reveal that many doctors and patients are unaware of or do not understand the risks or benefits of opioids. Indeed, patients often report that they were not warned they might become addicted to opioids prescribed to them. As reported in January 2016, a 2015 survey of more than 1,000 opioid patients found that 4 out of 10 were not told opioids were potentially addictive.[61]

130. Defendants' deceptive marketing scheme caused and continues to cause doctors in and around Delta County to prescribe opioids for chronic pain conditions such as back pain, headaches, arthritis, and fibromyalgia. Absent Defendants' deceptive marketing scheme, these doctors would not have been able to over prescribe opioids or become embroiled in pill mills that negatively impacted residents of Delta County.

131. For example, Defendants' deceptive marketing scheme allowed Dr. Don Martin O'Neal to promote, over-prescribe, and financially benefit from prescribing opioids for chronic pain to elderly patients. Indeed, Dr. O'Neal herein "knowingly or intentionally manufactured, distributed, dispensed, or possessed with the intent to manufacture, distribute, or dispense a controlled substance, including opioids such as OxyContin, Hydrocodone, and Vicodin in violation of the Texas Controlled Substances Act in 21 C.F.R. §1301 et seq.[62]

132. Dr. O'Neal was employed by the Sulphur Spring Medical-Surgical Clinic in Sulphur

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[61] Hazelden Betty Ford Foundation, *Missed Questions, Missed Opportunities* (Jan. 27, 2016), *available at* http://www.hazeldenbettyford.org/about-us/news-and-media/pressrelease/doctors-missing-questions-that-could-prevent-opioid-addiction.

[62] *See, e.g.,* Grand Jury Indictment in *United States v. Sina Athari, et al.*, U.S.D.C-Northern Dist., Dallas Div., No. 3:14-CR-044-D (December 1, 2015).

Springs, which services residents of Delta County.[63] This clinic specialized in nursing home patient treatment.[64]

133. Dr. O'Neal prescribed 40 to 50 of his patients "significant quantities" of methadone, oxycodone, hydrocodone, and benzodiazepines as reported by a pharmacist in November 2008.[65] Essentially 80% of Dr. O'Neal's patients were receiving high dosages of opioids.[66]

134. To avoid further investigation and potential criminal charges, Dr. O'Neal relinquished his DEA certificate in August 2009, which had enabled him to prescribe controlled substances.[67]

135. But the lack of a DEA certificate did not stop Dr. O'Neal from prescribing his patients controlled substances. In January 2010, one of Dr. O'Neal's colleagues discovered that Dr. O'Neal had been using his signature stamp to prescribe controlled substances, such as opioids, to 38 of Dr. O'Neal's 78 patients.[68]

136. In April 2010, Dr. O'Neal was charged with "33 felony counts alleging fraud for ordering narcotic prescriptions without a valid DEA registration by fraudulently using the license or registration stamp of another physician. On April 26, 2010, Dr. O'Neal was arrested again and charged with an additional 22 counts alleging fraud for ordering and administering controlled substances without a valid DEA registration."[69] These narcotics and controlled substances included opioids.

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[63] Kerry Craig, "Change of Venue Sought in O'Neal Cases," Sulphur Springs News-Telegram (Mar. 12, 2011).
[64] *"Texas Physician Arrested yet Again on Illegal Money Scheme"* (July 13, 2013), https://medicalmiscreants.com/2013/07/13/texas-physician-arrested-yet-again-on-illegal-money-scheme/.
[65] *"Sulphur Springs Doctor's License Suspended"* (May 24, 2010), http://www/kltv.com/story/12533527/sulphur-springs-doctors-license-suspended.
[66] *"Texas Physician Arrested yet Again on Illegal Money Scheme"* (July 13, 2013), https://medicalmiscreants.com/2013/07/13/texas-physician-arrested-yet-again-on-illegal-money-scheme/.
[67] *"Sulphur Springs Doctor's License Suspended"* (May 24, 2010), http://www/kltv.com/story/12533527/sulphur-springs-doctors-license-suspended.
[68] *Id.*
[69] *Id.*

137. The Texas Medicine Board suspended Dr. O'Neal's license on May 24, 2010 because he constituted "a continuing threat to the public health and safety."[70] The Panel further concluded that Dr. O'Neal is a "continuing and real danger to the health of [his] patients or to the public from the acts or omissions of [Dr. O'Neal] caused through [his] lack of competence, impaired status, or failure to care adequately for [his] patients."[71]

138. Dr. O'Neal was not running a medical practice, but a "pill mill" in which his net worth ballooned to an amount in excess of $12 million in addition to investments and real estate holdings.[72]

139. On December 3, 2015, Dr. O'Neal's medical license was revoked by the Texas Medical Board after he plead guilty to the first degree felony of theft of property.[73]

140. Dr. O'Neal caused damage to the health and welfare of numerous Delta County residents. His conduct subverted public health and violated the Texas Controlled Substances Act.

141. Dr. O'Neal's disregard of his patient's health in favor of his pursuit of profits has added to the opioid epidemic that has affected and continues to affect Delta County.

142. If the manufacturing and distributing Defendants were not over-supplying opioids, then physicians like Dr. O'Neal could not devise schemes to prescribe opioids without a legitimate purpose.

143. While Defendants may claim the federal government authorized the amount of annual prescription opioids sold, they know in truth that several Defendants have successfully used

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[70] Order of Temporary Suspension, *In the Matter of the License of Don Martin O'Neal, M.D.*, License No. E-2769, May 24, 2010, at p. 3; *see also "Sulphur Springs Doctor's License Suspended"* (May 24, 2010), http://www/kltv.com/story/12533527/sulphur-springs-doctors-license-suspended.
[71] Order of Temporary Suspension, at p. 3.
[72] Kerry Craig, *"D.A. to Ask for Higher Bonds and Passport in O'Neal Case,"* Sulphur Springs News-Telegram.
[73] *"Multiple ETX Doctors Disciplined by Medical Board," viewed on* October 3, 2017, http://easttexasmatters.com/news/local-news-mutiple-etx-doctors-disciplined-by-medical-board/301673397.

their organized money and influence to render the federal government's enforcement agency, the Drug Enforcement Administration, virtually powerless to interrupt the over-supply of prescription opioid drugs.

144. Defendants' deceptive marketing scheme also caused and continues to cause patients to purchase and use opioids for their chronic pain believing they are safe and effective. Absent Defendants' deceptive marketing scheme, fewer patients would be using opioids long-term to treat chronic pain, and those patients using opioids would be using less of them.

145. Defendants' deceptive marketing has caused and continues to cause the prescribing and use of opioids to explode. Indeed, this dramatic increase in opioid prescriptions and use corresponds with the dramatic increase in Defendants' spending on their deceptive marketing scheme. Defendants' spending on opioid marketing totaled approximately $91 million in 2000. By 2011, that spending had tripled to $288 million.

146. The escalating number of opioid prescriptions written by doctors who were deceived by Defendants' deceptive marketing scheme is the cause of a correspondingly dramatic increase in opioid addiction, overdose, and death throughout the U.S. and Delta County.

147. Scientific evidence demonstrates a strong correlation between opioid prescriptions and opioid abuse. In a 2016 report, the CDC explained that prescribing opioids has quadrupled since 1999, which has resulted in a parallel increase in opioid overdoses.[74] Indeed, there has been a two-third increase in overdose deaths from using opioids since 2000.[75] For these reasons, the CDC concluded that efforts to rein in the prescribing of opioids for chronic pain are critical "to reverse

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[74] CDC. National Vital Statistics System, Mortality. CDC WONDER. Atlanta, GA: US Department of Health and Human Services, CDC; 2016. https://wonder.cdc.gov/; Rudd RA, Seth P, David F, Scholl L. Increases in Drug and Opioid-Involved Overdose Deaths — United States, 2010–2015. MMWR Morb Mortal Wkly Rep. ePub: 16 December 2016.

[75] *National Vital Statistics System*, Mortality file and appearing *Center for Disease Control and Prevention* Morbidity and Mortality Weekly Report, January 1, 2006 / 64(50); 1378-82, Increases in Drug and Opioid Deaths – United States, 2000-2014.

the cycle of opioid pain medication misuse that contributes to the opioid overdose epidemic."[76]

148. Due to the increase in opioid overdoses, first responders such as police officers, have been and will continue to be in the position to assist people experiencing opioid-related overdoses.[77] In 2016, "over 1,200 law enforcement departments nationwide carried naloxone in an effort to prevent opioid-related deaths."[78]

149. Defendants' deceptive marketing scheme has also detrimentally impacted children in Delta County. Overprescribing opioids for chronic pain has made the drugs more accessible to school-aged children, who come into contact with opioids after they have been prescribed to friends or relatives in the same household.

150. Defendants' conduct has adversely affected Delta County's child protection agencies in the number of children in foster care driven by parental drug addiction. Children with parents addicted to drugs tend to stay in foster care longer, and they often enter the system having experienced significant trauma, which makes these cases more expensive for counties like Delta County.

151. Opioid addiction is one of the primary reasons that Delta County residents seek substance abuse treatment. A significant number of admissions for drug abuse were associated with a primary diagnosis of opiate abuse or dependence.

152. Defendants' creation, through false and deceptive advertising and other unlawful and unfair conduct, of a virtually limitless opioid market has significantly harmed Delta County communities. Defendants' success in extending the market for opioids to new patients and chronic

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[76] *CDC Guideline for Prescribing Opioids for Chronic Pain, supra; see also* Rudd RA, Seth P, David F, Scholl L. Increases in Drug and Opioid-Involved Overdose Deaths — United States, 2010–2015. MMWR Morb Mortal Wkly Rep. ePub: 16 December 2016.

[77] Opinion of the Attorney General of Texas, KP-0168 (Oct. 4, 2017).

[78] *Id. citing* http://www.nchrc.org/law-enforcement/us-law-enforcement-who-carry-naloxone/.

pain conditions has created an abundance of drugs available for non-medical and criminal use and fueled a new wave of addiction and injury. It has been estimated that 60% of the opioids that are abused come, directly or indirectly, through doctors' prescriptions.[79]

153. Law enforcement agencies have increasingly associated prescription drug abuse with violent and property crimes. Despite strict federal regulation of prescription drugs, local law enforcement agencies are faced with increasing diversion from legitimate sources for illicit purposes, including doctor shopping, forged prescriptions, falsified pharmacy records, and employees who steal from their place of employment. The opioid epidemic has prompted a growing trend of crimes against pharmacies including robbery and burglary. This ongoing diversion of prescription narcotics creates a lucrative marketplace.

154. The rise in opioid addiction caused by Defendants' deceptive marketing scheme has also resulted in an explosion in heroin use. For example, heroin use has more than doubled in the past decade among adults aged 18 to 25 years.[80] Moreover, heroin-related overdoses in the United States has more than quadrupled since 2010.[81]

155. The costs and consequences of opioid addiction are staggering. For example, in 2007, the cost of healthcare due to opioid abuse, dependence, and misuse was estimated at 25 billion, the cost of criminal justice was estimated at 5.1 billion, and the cost of lost workplace productivity was estimated at 25.6 billion.

156. Consequently, prescription opioid misuse, abuse, and overdose have an enormous

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[79] Nathaniel P. Katz, *Prescription Opioid Abuse: Challenges and Opportunities for Payers*, Am. J. Managed Care (Apr. 19 2013), at 5 ("The most common source of abused [opioids] is, directly or indirectly, by prescription."), http://www.ajmc.com/publications/issue/2013/2013-1-vol19-n4/Prescription-Opioid-Abuse-Challenges-and-Opportunities-for-Payers.

[80] Centers for Disease Control and Prevention. Vital Signs: Today's Heroin Epidemic – More People at Risk, Multiple Drugs Abused. (https://www.cdc.gov/vitalsigns/heroin/index.html). MMWR 2015.

[81] https://www.cdc.gov/vitalsigns/heroin/index.html

impact on the health and safety of individuals, as well as communities at large, because the consequences of this epidemic reach far beyond the addicted individual.

157. Some of the repercussions for residents of Delta County include job loss, loss of custody of children, physical and mental health problems, homelessness and incarceration, which results in instability in communities often already in economic crisis and contributes to increased demand on community services such as hospitals, courts, child services, treatment centers, and law enforcement.

158. Defendants knew and should have known about these harms that their deceptive marketing has caused and continues to cause and will cause in the future. Defendants closely monitored their sales and the habits of prescribing doctors. Their sales representatives, who visited doctors and attended CMEs, knew which doctors were receiving their messages and how they were responding.

159. Defendants also had access to and carefully watched government and other data that tracked the explosive rise in opioid use, addiction, injury, and death. Defendants not only knew, but intended that their misrepresentations would persuade doctors to prescribe and encourage patients to use their opioids for chronic pain.

160. Defendants' actions are neither permitted nor excused by the fact that their drug labels (with the exception of the Actiq/Fentora labels) may have allowed, or did not exclude, the use of opioids for chronic pain. FDA approval of opioids for certain uses did not give Defendants license to misrepresent the risks and benefits of opioids. Indeed, Defendants' misrepresentations were directly contrary to pronouncements by, and guidance from, the FDA based on the medical evidence and their own labels.

161. Nor is Defendants' causal role broken by the involvement of doctors. Defendants'

marketing efforts were ubiquitous and highly persuasive. Their deceptive messages tainted virtually every source doctors could rely on for information and prevented them from making informed treatment decisions. Defendants also hijacked what doctors wanted to believe – namely, that opioids represented a means of relieving their patients' suffering and of practicing medicine more compassionately.

162. Defendants' actions and omissions were each a cause-in-fact of Delta County's past and future damages. Defendants' wrongful conduct caused injuries to Delta County in the past, continues to cause injuries to Delta County, and will continue to cause injuries to Delta County in the future. Future damages include, but are not limited to, additional resources for counseling and medication assisted treatment of addicts, medical treatment for overdoses, life skills training for adolescents, increased law enforcement, and additional resources to treat the psychological effects of opioids and the underlying conditions that make people susceptible to opioid addiction.

## I. Defendants' Fraudulent Marketing Has Led To Record Profits.

163. While using opioids has taken an enormous toll on Delta County and its residents, Defendants have realized blockbuster profits. In 2014 alone, opioids generated $11 billion in revenue for drug companies like Defendants. Indeed, financial information indicates that each Defendant experienced a material increase in sales, revenue, and profits from the false and deceptive advertising and other unlawful and unfair conduct described above.

## VI. FIRST CAUSE OF ACTION: PUBLIC NUISANCE AGAINST ALL DEFENDANTS

164. Delta County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

165. Defendants knowingly encouraged doctors in and around Delta County to prescribe, and residents to use, highly addictive opioids for chronic pain even though Defendants

knew using opioids had a high risk of addiction and reduced quality of life.

166. By doing so, Defendants purposefully interfered with Delta County's public health, public safety, public peace, public comfort, and public convenience.

167. Defendants, individually and in concert with each other, have contributed to and/or assisted in creating and maintaining a condition that is harmful to the health and safety of Delta County residents and/or unreasonably interferes with the peace and comfortable enjoyment of life in violation of Texas law.

168. The public nuisance created by Defendants' actions is substantial and unreasonable – it has caused and continues to cause significant harm to the community – and the harm inflicted outweighs any offsetting benefit.

169. The staggering rates of opioid use resulting from Defendants' marketing efforts have caused, and continues to cause, harm to the community including, but not limited to:

a. Upwards of 30% of all adults use opioids. These high rates of use have led to unnecessary opioid abuse, addiction, overdose, injuries, and deaths;

b. Children have been exposed to opioids prescribed to family members or others resulting in injury, addiction, and death. Easy access to prescription opioids has made opioids a recreational drug of choice among Delta County teenagers; opioid use among teenagers is only outpaced by marijuana use. Even infants have been born addicted to opioids due to prenatal exposure causing severe withdrawal symptoms and lasting developmental impacts;

c. Residents of Delta County, who have never taken opioids, have endured both the emotional and financial costs of caring for loved ones addicted to or injured by opioids and the loss of companionship, wages, or other support from family members who have used, abused, become addicted to, overdosed on, or been killed by opioids;

d. More broadly, opioid use and misuse have driven Delta County residents' health care costs higher;

e. Employers have lost the value of productive and healthy employees who have suffered from adverse consequences from opioid use;

f. Defendants' success in extending the market for opioids to new patients and chronic conditions has created an abundance of drugs available for criminal use and fueled a new wave of addiction, abuse, and injury. Defendants' scheme created both ends of a new secondary market for opioids – providing both the supply of narcotics to sell and the demand of addicts to buy them;

g. This demand has created additional illicit markets in other opiates, particularly heroin. The low cost of heroin has led some of those who initially become addicted to prescription opioids to migrate to cheaper heroin, fueling a new heroin epidemic in the process;

h. Diverting opioids into secondary, criminal markets and increasing the number of individuals who abuse or are addicted to opioids has increased the demands on emergency services and law enforcement in Delta County;

i. All of Defendants' actions have caused significant harm to the community – in lives lost; addictions endured; the creation of an illicit drug market and all its concomitant crime and costs; unrealized economic productivity; and broken families and homes;

j. These harms have taxed the human, medical, public health, law enforcement, and financial resources of Delta County; and

k. Defendants' interference with the comfortable enjoyment of life of a substantial number of people is entirely unreasonable because there is limited social utility to opioid use and any potential value is outweighed by the gravity of harm inflicted by Defendants' actions.

170. Defendants knew, or should have known, that promoting opioid use would create a public nuisance in the following ways:

a. Defendants have engaged in massive production, promotion, and distribution of opioids for use by the citizens of Delta County;

b. Defendants' actions created and expanded the market for opioids, promoting its wide use for pain management;

c. Defendants misrepresented the benefits of opioids for chronic pain and fraudulently concealed, misrepresented, and omitted the serious adverse effects of opioids, including the addictive nature of the drugs; and

d. Defendants knew or should have known that their promotion would lead to addiction and other adverse consequences that the larger community would suffer as a result.

171. Defendants' actions were, at the least, a substantial factor in doctors and patients not accurately assessing and weighing the risks and benefits of opioids for chronic pain thereby causing opioids to become widely available and used in Delta County.

172. Without Defendants' actions, opioid use would not have become so widespread and the enormous public health hazard of opioid overuse, abuse, and addiction not existing would have been averted.

173. The health and safety of the citizens of Delta County, including those who use, have used, or will use opioids, as well as those affected by opioid users, is a matter of great public interest and legitimate concern to Delta County's citizens and residents.

174. The public nuisance created, perpetuated, and maintained by Defendants can be abated and further reoccurrence of such harm and inconvenience can be prevented.

175. Defendants' conduct has affected and continues to affect a considerable number of people within Delta County and is likely to continue to cause significant harm to patients who take opioids, their families, and the community at large.

176. Each Defendant created or assisted in creating the opioid epidemic, and each Defendant is jointly and severally liable for its abatement. Furthermore, each Defendant should be enjoined from continuing to create, perpetuate, or maintain said public nuisance in Delta County.

## VII. SECOND CAUSE OF ACTION: COMMON LAW FRAUD AGAINST ALL DEFENDANTS

177. Delta County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

178. At all relevant and material times, Defendants expressly and/or impliedly warranted that opioids were safe, of merchantable quality, and fit for use.

179. Defendants' superior knowledge and expertise, its relationship of trust and confidence with doctors and the public, its specific knowledge regarding the risks and dangers of opioids, and its intentional dissemination of promotional and marketing information about opioids for the purpose of maximizing sales, each gave rise to the affirmative duty to meaningfully disclose and provide all material information about the risks and harms associated with opioids.

180. At all times herein mentioned, Defendants, individually and acting through their employees and agents, and in concert with each other, fraudulently represented to physicians who Defendants knew would justifiably rely on Defendants' representations that opioids were safe and effective for treating chronic pain.

181. Defendants' false representations were fraudulently made, with the intent or purpose that healthcare providers and patients would justifiably rely upon them, leading to the prescription, administration, filling, purchasing, and consumption of opioids in Delta County.

182. Defendants' deliberate misrepresentations and/or concealment, suppression, and omission of material facts as alleged herein include, but are not limited to:

a. Making false and misleading claims regarding the known risks of the addictive nature of opioids and suppressing, failing to disclose, and mischaracterizing the addictive nature of opioids and in concomitant costs, such as overdoses, deaths, and heroin addiction;

b. Making false and misleading written and oral statements that opioids are more effective than traditional pain killers for chronic pain, or effective at all and/or omitting material information showing that opioids are no more effective than other non-addictive drugs for chronic pain;

c. Issuing false and misleading warnings and/or failing to issue adequate warnings concerning the risks and dangers of using opioids;

d. Making false and misleading claims downplaying the risk of addiction when using opioids and/or setting forth guidelines that would purportedly identify addictive behavior; and

e. Making false and misleading misrepresentations concerning the safety, efficacy and benefits of opioids without full and adequate disclosure of the underlying facts which rendered such statements false and misleading.

183. Defendants willfully, wantonly, and recklessly disregarded their duty to provide truthful representations regarding the safety and risk of opioids.

184. Defendants made these misrepresentations with the intent that the healthcare community and patients located wherever these opioid drugs were sold or consumed would rely upon them.

185. Defendants' misrepresentations were made with the intent of defrauding and deceiving the medical community and consumers to induce and encourage the sale of opioids.

186. Defendants' fraudulent representations evidence their callous, reckless, willful, and depraved indifference to the health, safety, and welfare of consumers living in Delta County.

187. Defendants omitted, misrepresented, suppressed and concealed material facts concerning the dangers and risk of injuries associated with the use of opioids, as well as the fact that the product was unreasonably dangerous.

188. Defendants' purpose was willfully blind to, ignored, downplayed, avoided, and/or otherwise understated the serious nature of the risks associated with the use of opioids.

189. The treating medical community and consumers in Delta County did not know that Defendants' representations were false and/or misleading and justifiably relied on them.

190. Defendants had sole access to material facts concerning the dangers and unreasonable risks of opioids, which they intentionally concealed.

191. As a direct and proximate result of Defendants' fraudulent misrepresentations and intentional concealment of facts, upon which the medical community and consumers in Delta County reasonably relied, Delta County suffered actual and punitive damages.

## VIII. THIRD CAUSE OF ACTION: NEGLIGENCE AGAINST MANUFACTURING AND DISTRIBUTING DEFENDANTS

192. Delta County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

193. Manufacturing Defendants have a duty to exercise reasonable care in marketing its opioids to physicians treating residents of Delta County and Delta County residents. Manufacturing Defendants have breached their duty by knowingly and fraudulently misrepresenting the benefits of, and downplaying the risks of, opioids for chronic pain.

194. Manufacturing Defendants have used deceitful marketing ploys, KOLs, Front Groups, and other schemes to increase profits at the cost of public health causing an opioid epidemic. Manufacturing Defendants have acted willfully, wantonly, and maliciously.

195. Likewise, Distributor Defendants have a duty to exercise ordinary care in distributing opioids. Distributor Defendants have breached their duty by failing to prevent or reduce the distribution of opioids, or to report the increase in the distribution and/or sale of opioids.

196. Distributor Defendants have intentionally failed to prevent or reduce the distribution of opioids, or to report any increases in the sale of opioids, so that they could increase profits and receive rebates or kick-backs from Manufacturing Defendants. Distributor Defendants have acted willfully, wantonly, and maliciously.

197. As a proximate result, Manufacturing and Distributor Defendants and its agents have caused Delta County to incur excessive costs to treat the opioid epidemic in its county, including but not limited to increased costs of social services, health systems, law enforcement, judicial system, and treatment facilities.

198. Delta County and its residents are therefore entitled to actual and punitive damages.

## IX. FOURTH CAUSE OF ACTION: GROSS NEGLIGENCE AGAINST MANUFACTURING AND DISTRIBUTING DEFENDANTS

199. Delta County incorporates the allegations within all prior paragraphs within this Complaint as if they were fully set forth herein.

200. Defendants' marketing scheme to optimize profits by misrepresenting and falsely

touting opioids as the panacea to chronic pain was done intentionally.

201. Defendants' hiring of KOLs, Front Groups, and others to spread its fraudulent message that opioids were useful and beneficial for chronic pain was grossly negligent and done with conscious indifference or reckless disregard for the safety of others.

202. Each Defendant's actions and omissions as described herein, singularly or in combination with each other, was malicious resulting in damages and injuries to Delta County and its residents.

203. At every stage, Defendants knew or should have known that their conduct would create an unreasonable risk of physical harm to others, including Delta County and its residents, and should be held liable in punitive and exemplary damages to Delta County.

## X. FIFTH CAUSE OF ACTION: TEXAS CONTROLLED SUBSTANCES ACT ("TCSA") AGAINST DISTRIBUTOR DEFENDANTS AND DR. O'NEAL

204. Delta County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

205. Distributor Defendants have knowingly distributed, delivered, administered, or dispensed a controlled substance in violation of the Texas Controlled Substances Act §481.128(a)(1) by deceiving practitioners into prescribing, dispensing, delivering, or administering a controlled substance, or causing a controlled substance to be administered when there is no valid medical purpose. Tex. Health & Safety Code §481.071.

206. As alleged herein, each Distributor Defendant, at all times relevant to this Complaint, violated the Texas Controlled Substance Act by making deceptive representations about using opioids to treat chronic pain. Each Distributor Defendant also omitted or concealed material facts and failed to correct prior misrepresentations and omissions about the risks and benefits of

opioids. Each Distributor Defendant's omissions rendered even their seemingly truthful statements about opioids deceptive.

207. Distributor Defendants' deceptive representations and concealments were reasonably calculated to deceive practitioners treating Delta County residents into prescribing opioids without any valid medical purpose, and Distributor Defendants continue to do so to this day.

208. Dr. O'Neal prescribed opioids, a controlled substance, without the federal registration required under Texas Health & Safety Code Section 481.061. Due to his lack of a DEA registration, Dr. O'Neal was not allowed to "manufacture, distribute, prescribe, possess, analyze or dispense" a controlled substance in Texas, including Delta County. Tex. Health & Safety Code §481.061(a).

209. Even when Dr. O'Neal possessed the requisite registration, Dr. O'Neal prescribed opioids without a valid medical purpose in violation of Texas Health & Safety Code Section 481.071(a).

210. As a direct and proximate cause of Distributor Defendants' and Dr. O'Neal's deceptive conduct, Delta County should be awarded civil penalties pursuant to the Texas Controlled Substances Act.

## XI. SIXTH CAUSE OF ACTION: UNJUST ENRICHMENT AGAINST ALL DEFENDANTS

211. Delta County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

212. As an expected and intended result of their conscious wrongdoing as set forth in this Complaint, Defendants have profited and benefited from opioid purchases made by Delta County and its residents.

213. When Delta County and its residents purchased opioids, they expected that Defendants had provided necessary and accurate information regarding those risks. Instead, Defendants had misrepresented the material facts regarding the risks and benefits of opioids.

214. Defendants have been unjustly enriched at the expense of Delta County, and Delta County is therefore entitled to damages to be determined by the jury.

## PRAYER FOR RELIEF

WHEREFORE, Plaintiff respectfully prays:

a. That the acts alleged herein be adjudged and decreed to be unlawful and that the Court enter a judgment declaring them to be so;

b. That Defendants be enjoined from, directly or indirectly through KOLs, Front Groups or other third parties, continuing to misrepresent the risks and benefits of the use of opioids for chronic pain, and from continuing to violate Texas law;

c. That Plaintiff recover all measures of damages, including punitive and exemplary damages, allowable under the law, and that judgment be entered against Defendants in favor of Plaintiff;

d. That Plaintiff recover restitution on behalf of Delta County consumers who paid for opioids for chronic pain;

e. That Plaintiff recover the costs and expenses of suit, pre- and post-judgment interest, and reasonable attorneys' fees as provided by law; and

f. That Defendants be ordered to abate the public nuisance that they created in in violation of Texas common law.

Date: December 19, 2017

Respectfully Submitted,

**County Attorney, Delta County**

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# APPENDIX E

## FALLS PETITION

Filed 1/4/2018 1:41 PM
Christy Wideman
District Clerk
Falls County, Texas

Tonya Martinez

**CAUSE NO. _____**

| | | |
|---|---|---|
| **COUNTY OF FALLS,** | § | **IN THE DISTRICT COURT** |
| | § | |
| *Plaintiff,* | § | |
| | § | |
| vs. | § | **_____ JUDICIAL DISTRICT** |
| | § | |
| **PURDUE PHARMA L.P.**; | § | |
| **PURDUE PHARMA INC.**; | § | |
| **THE PURDUE FREDERICK COMPANY**; | § | |
| **JOHNSON & JOHNSON**; | § | **FALLS COUNTY, TEXAS** |
| **JANSSEN PHARMACEUTICALS, INC.**; | § | |
| **ORTHO-MCNEIL-JANSSEN PHARMACEUTICALS, INC.** n/k/a JANSSEN PHARMACEUTICALS, INC.; | § | |
| **JANSSEN PHARMACEUTICA, INC.** n/k/a JANSSEN PHARMACEUTICALS, INC.; | § | |
| **ENDO HEALTH SOLUTIONS INC.**; | § | |
| **ENDO PHARMACEUTICALS, INC.**; | § | |
| **ABBVIE INC.**; | § | |
| **KNOLL PHARMACEUTICAL COMPANY,** a wholly-owned subsidiary of ABBVIE INC.; | § | |
| **ALLERGAN PLC** f/k/a ACTAVIS PLC; | § | |
| **ALLERGAN FINANCE LLC** f/k/a ACTAVIS, INC. f/k/a WATSON PHARMACEUTICALS, INC.; | § | |
| **WATSON LABORATORIES, INC.**; | § | |
| **ACTAVIS LLC**; | § | |
| **ACTAVIS PHARMA, INC.** f/k/a WATSON PHARMA, INC.; | § | |
| **MCKESSON CORPORATION**; | § | |
| **CARDINAL HEALTH, INC.**; | § | |
| **AMERISOURCEBERGEN CORPORATION**; and | § | |
| **DOES 1 – 100, INCLUSIVE,** | § | |
| | § | |
| *Defendants.* | § | |

**PLAINTIFF'S ORIGINAL PETITION AND JURY DEMAND**

**TO THE HONORABLE JUDGE OF SAID COURT:**

Plaintiff, the County of Falls, Texas, by and through the undersigned attorneys, (hereinafter

"Falls County" or "County"), on behalf of the County Attorney for Falls County, against Defendants Purdue Pharma L.P., Purdue Pharma Inc., The Purdue Frederick Company, Johnson & Johnson, Janssen Pharmaceuticals, Inc., Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc., Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc., Endo Health Solutions Inc., Endo Pharmaceuticals, Inc., Abbvie Inc., Knoll Pharmaceutical Company, a wholly-owned subsidiary of Abbvie Inc., Allergan PLC f/k/a Actavis PLC, Allergan Finance, LLC f/k/a Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc., Watson Laboratories, Inc., Actavis LLC, Actavis Pharma, Inc. f/k/a Watson Pharma, Inc., McKesson Corporation, Cardinal Health, Inc., AmerisourceBergen Corporation, and Does 1 – 100, alleges as follows:

## I. INTRODUCTION

1. The United States is in the midst of an opioid epidemic caused by Defendants' fraudulent marketing and sales of prescription opioids ("opioids") that has resulted in addiction, criminal activity, and loss of life. The opioid crisis has been described as "the AIDS epidemic of our generation, but even worse."[1] On October 26, 2017, President Donald Trump "declared a nationwide public health emergency to combat the opioid crisis."[2]

2. In 2016 alone, health care providers wrote more than 289 million prescriptions for opioids, enough for *every adult in the United States* to have more than one bottle of pills.[3] Americans "consume 85% of all the opioids in the world" and are "the most medicated country in

[1] David Wright, "Christie on Opioids: "This is the AIDS Epidemic of Our Generation, but even Worse," (Oct. 27, 2017), http://www.cnn.com/2017/10/27/politics/chris-christie-opioid-commission-aids-cnntv/index.html.
[2] Dan Merica, "What Trump's Opioid Announcement Means – and Doesn't Mean," (Oct. 26, 2017), http://www.cnn.com/2017/10/26/politics/national-health-emergency-national-disaster/index.html.
[3] *Prevalence of Opioid Misuse*, BupPractice (Sept. 7, 2017), https://www.buppractice.com/node/15576.

the world . . . ."[4]

3. Unfortunately, using opioids too often leads to addiction and overdose from opioids. In 2014, almost 2 million Americans were addicted to opioids.[5] That same year, more people died from drug overdoses than in any other year, and most overdose deaths involved an opioid. The Texas Legislature has found "that deaths resulting from the use of opioids and other controlled substances constitute a public health crisis."[6] In 2015, Texas "had the second highest total healthcare costs from opioid abuse in the nation ($1.96 billion) . . . ."[7]

4. In fact, accidental drug overdose deaths, of which reportedly at least two-thirds are opioid overdoses, are the leading cause of death for Americans under the age of 50. Accidental drug overdose deaths, predominantly from opioids, exceed the number of deaths caused by cars or guns.

5. The economic burden caused by opioid abuse in the United States is at least $78.5 billion,[8] including lost productivity and increased social services, health insurance costs, increased criminal justice presence and strain on judicial resources, and substance abuse treatment and rehabilitation.

6. This epidemic did not occur by chance. Defendants manufacture, market, distribute, and sell prescription opioids, including, but not limited to, brand-name drugs like OxyContin, Vicodin, Opana, Percocet, Percodan, Duragesic, Ultram, Ultracet, and generics like oxycodone,

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[4] David Wright, "Christie on Opioids: "This is the AIDS Epidemic of Our Generation, but even Worse," (Oct. 27, 2017), http://www.cnn.com/2017/10/27/politics/chris-christie-opioid-commission-aids-cnntv/index.html.
[5] Substance Abuse and Mental Health Services Administration, National Survey on Drug Use and Health, 2014.
[6] Opinion of the Attorney General of Texas, KP-0168 (Oct. 4, 2017), *citing* Act of May 26, 2017, 85th Leg., R.S., ch. 534, §3, 2017 Tex. Sess. Law Serv. 1467, 1468.
[7] Kerry Craig, "Opioid Addiction Results in one Woman's Daily Struggle," Oct. 7, 2017, https://www.ssnewstelegram.com/news/opioid-addiction-results-in-one-woman-s-daily-struggle/article_bded4eoa-ab80-11e7-a252-d3f304e26628.html.
[8] *See CDC Foundation's New Business Pulse Focuses on Opioid Overdose Epidemic*, Centers for Disease Control and Prevention (Mar. 15, 2017), https://www.cdc.gov/media/releases/2017/a0315-business-pulse-opioids.html.

oxymorphone, hydromorphone, hydrocodone, fentanyl, and tramadol, which are powerful narcotics.

7. Historically, opioids were considered too addictive and debilitating for treating non-cancer chronic pain,[9] such as back pain, migraines, and arthritis, and were used only to treat short-term acute pain or for palliative or end-of-life care.

8. By the late 1990s or early 2000s, however, each Manufacturing Defendant began a marketing scheme to persuade doctors and patients that opioids can and should be used ubiquitously and perpetually to treat moderate, non-cancer chronic pain. Each Manufacturing Defendant spent large sums of money to promote the benefits of opioids for non-cancer moderate pain while trivializing, or even denying, their risks. The Manufacturing Defendants' promotional messages deviated substantially from any approved labeling of the drugs and caused prescribing physicians and consuming patients to underappreciate the health risks, and to overestimate the benefits, of opioids.

9. Contrary to the language of their drugs' labels, Defendants falsely and misleadingly, in their marketing: (1) downplayed the serious risk of addiction; (2) promoted and exaggerated the concept of "pseudoaddiction" thereby advocating that the signs of addiction should be treated with more opioids; (3) exaggerated the effectiveness of screening tools in preventing addiction; (4) claimed that opioid dependence and withdrawal are easily managed; (5) denied the risks of higher opioid dosages; and (6) exaggerated the effectiveness of "abuse-deterrent" opioid formulations to prevent abuse and addiction.

10. Defendants disseminated these falsehoods through ads and/or their sales representatives and physicians who supported Defendants' message. Sales representatives,

[9] "Chronic pain" means non-cancer pain lasting three months or longer.

working at Defendants' behest, promoted highly addictive opioids through souvenirs and toys including, but not limited to, opioid brand-bearing stuffed plush toys, dolls, coffee cups, fanny packs, water bottles, notepads, pens, refrigerator magnets, clocks, letter openers, rulers, daytime planners, bags, puzzles, posters, hand-held calculators, clipboards, highlighters, flashlights, key chains, clothing, reflex mallets, and mock-ups of the United States Constitution.

11. Defendants also used third parties they controlled by: (a) funding, assisting, encouraging, and directing doctors, known as "key opinion leaders" ("KOLs") and (b) funding, assisting, directing, and encouraging seemingly neutral and credible professional societies and patient advocacy groups (referred to hereinafter as "Front Groups").

12. Defendants worked with KOLs and Front Groups to taint the sources that doctors and patients relied on for ostensibly "neutral" guidance, such as treatment guidelines, Continuing Medical Education ("CME") programs, medical conferences and seminars, and scientific articles. After their individual and concerted efforts, Defendants convinced doctors that instead of being addictive and unsafe for long-term use in most circumstances, opioids were *required* in the compassionate treatment of chronic pain.

13. The Distributor Defendants were not standing by idly while Marketing Defendants were peddling their opioids to physicians and consumers. Cardinal, AmerisourceBergen, and McKesson ("Distributor Defendants") are three of the largest opioid distributors in the United States. Distributor Defendants purchased opioids from Manufacturing Defendants herein and sold them to pharmacies servicing consumers in Falls County.

14. Despite the alarming and suspicious rise in the ordering of opioids by retailers in Falls County, Distributor Defendants did nothing. The Manufacturing Defendants and Distributor Defendants worked hand and glove to glut the U.S. and Falls County with more opioids than would

be consumed for therapeutic purposes. Each Defendant disregarded its legal duty to report suspicious opioid prescriptions, and each Defendant financially benefitted from the other Defendants (both Manufacturing and Distributor Defendants), disregarding their individual duties to report.

15. Essentially each Defendant ignored science and consumer health for profits. Defendants' efforts were so successful that opioids are now the most prescribed class of drugs generating $11 billion in revenue for drug companies in 2014 alone. Even after Purdue reached a $600 million federal settlement in 2007, the settlement failed to impact what is a "$13-billion-a-year opioid industry."[10]

16. As a direct and foreseeable consequence of Defendants' misrepresentations regarding the safety and efficacy of using opioids for chronic pain, Falls County has spent and continues to spend large sums combatting the public health crisis created by Defendants' negligent and fraudulent marketing campaign.

17. For example, thousands of prescriptions were written for opioids in Falls County in 2013[11] and in 2014 there were multiple deaths reported from drug overdoses.[12] A substantial number of those overdose deaths were a result, in whole or in part, of opioid ingestion. Upon information and belief, in each year from 2013-2017, there were multiple deaths in Falls County caused in whole or in part from ingestion of prescription opioids. Defendants' marketing misconduct, as well as Defendants' efforts to sell more prescription opioids that can be consumed therapeutically, were natural and foreseeable causes of overdose deaths and injuries in Falls

[10] Rebecca L. Haffajee, J.D., Ph.D., M.P.H., and Michelle M. Mello, J.D., Ph.D., *Drug Companies' Liability for the Opioid Epidemic*, N. Engl. J. Med. at 2305, (Dec. 14, 2017).
[11] https://www.cdc.gov/drugoverdose/maps/rxrate-maps.html; https://www.cdc.gov/nchs/data-visualization/drug-poisoning-mortality.
[12] https://www.cdc.gov/nchs/data-visualization/drug-poisoning-mortality.

County.

18. As a direct and foreseeable consequence of Defendants' conduct described regarding prescription opioids, Falls County has committed and continues to commit resources to provide and pay for health care, law enforcement, social services, public assistance, pharmaceutical care and other services necessary for its residents.

## II. RULE 47 STATEMENT OF MONETARY RELIEF SOUGHT

19. Per Rule 47 of the Texas Rules of Civil Procedure, the County states that although the full measure of its damages is still being calculated, its damages caused by Defendants' acts and omissions exceed $1,000,000 but are believed to be less than $50,000,000. Accordingly, at this time in the litigation, Falls County states that it is seeking monetary relief for an amount greater than $1,000,000 and less than $50,000,000, the rightful and just amount to be determined by the jury.

## III. VENUE AND JURISDICTION

20. Venue is proper in Falls County because all or a substantial part of the events or omissions giving rise to this claim occurred in Falls County. TEX. CIV. PRAC. & REM. CODE §15.002(a)(2). This Court has subject-matter jurisdiction over this matter because Plaintiff's damages are in excess of the minimal jurisdictional limits of this Court. TEX. GOVT. CODE §24.007(b).

21. This Court has specific jurisdiction over all Defendants as their activities were directed toward Texas, and injuries complained of herein resulted from their activities. *Guardian Royal Exchange Assur., Ltd. v. English China Clays, P.L.C.*, 815 S.W.2d 223, 227 (Tex. 1991). Each Defendant has a substantial connection with Texas and the requisite minimum contacts with Texas necessary to constitutionally permit the Court to exercise jurisdiction. *See id.* at 226.

## IV. PARTIES

### A. Plaintiff

22. This action is brought for and on behalf of Falls County, which provides a wide range of services on behalf of its residents, including services for families and children, public health, public assistance, law enforcement, and emergency care.

### B. Defendants

23. PURDUE PHARMA L.P. is a limited partnership organized under the laws of Delaware, and may be served through its registered agent for service of process, The Prentice-Hall Corporation System, Inc., 251 Little Falls Drive, Wilmington, DE 19808. PURDUE PHARMA L.P. is, through its ownership structure, a Texas resident. PURDUE PHARMA INC. is a New York corporation with its principal place of business in Stamford, Connecticut, and may be served through its registered agent for service of process, Corporation Service Company, 80 State Street, Albany, NY 12207. THE PURDUE FREDERICK COMPANY is a Delaware corporation with its principal place of business in Stamford, Connecticut, and may be served through its registered agent for service of process, The Prentice-Hall Corporation System, Inc., 251 Little Falls Drive, Wilmington, DE 19808 (collectively, "Purdue").

24. Purdue manufactures, promotes, sells, and distributes opioids in the U.S. and Falls County. Purdue's opioid drug, OxyContin, is among the most addictive and abused prescription drugs in the history of America. Purdue promotes opioids throughout the United States and in Falls County.

25. JANSSEN PHARMACEUTICALS, INC. is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey, and may be served through its registered agent for service of process, CT Corporation System, 1999 Bryan Street, Suite 900, Dallas, TX

75201. JANSSEN PHARMACEUTICALS, INC. is a wholly owned subsidiary of JOHNSON & JOHNSON (J&J), a New Jersey corporation with its principal place of business in New Brunswick, New Jersey, and may be served through its registered agent for service of process, Attention: Legal Department, One Johnson & Johnson Plaza, New Brunswick, NJ 08933. ORTHO-MCNEIL-JANSSEN PHARMACEUTICALS, INC., now known as JANSSEN PHARMACEUTICALS, INC., is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey. JANSSEN PHARMACEUTICA INC., now known as JANSSEN PHARMACEUTICALS, INC., is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey. J&J is the only company that owns more than 10% of Janssen Pharmaceuticals' stock, and corresponds with the FDA regarding Janssen's products. Upon information and belief, J&J controls the sale and development of Janssen Pharmaceuticals' drugs and Janssen's profits inure to J&J's benefit. (Janssen Pharmaceuticals, Inc., Ortho-McNeil-Janssen Pharmaceuticals, Inc., Janssen Pharmaceutica, Inc., and J&J are referred to as "Janssen").

26. Janssen manufactures, promotes, sells, and distributes opioids in the U.S. and in Falls County.

27. ENDO HEALTH SOLUTIONS INC. is a Delaware corporation with its principal place of business in Malvern, Pennsylvania, and may be served through its registered agent for service of process, The Corporation Trust Company, Corporation Trust Center, 1209 Orange St., Wilmington, DE 19801. ENDO PHARMACEUTICALS, INC. is a wholly- owned subsidiary of Endo Health Solutions Inc. and is a Delaware corporation with its principal place of business in Malvern, Pennsylvania, and may be served through its registered agent for service of process, CT Corporation System, 1999 Bryan Street, Suite 900, Dallas, TX 75201. (Endo Health Solutions Inc. and Endo Pharmaceuticals, Inc. are referred to as "Endo").

28. Endo develops, markets, and sells opioid drugs in the U.S. and in Falls County. Endo also manufactures and sells generic opioids in the U.S. and Falls County, by itself and through its subsidiary, Qualitest Pharmaceuticals, Inc.

29. ABBVIE INC. ("Abbvie") is a Delaware corporation with its principal place of business in North Chicago, Illinois, and may be served through its registered agent for service of process, CT Corporation System, 208 S. LaSalle Street, Suite 814, Chicago, IL 60604. KNOLL PHARMACEUTICAL COMPANY ("Knoll") has been a wholly-owned subsidiary of Abbvie from January 1, 2013. KNOLL PHARMACEUTICAL COMPANY is a New Jersey corporation with its principal place of business in Parsippany, New Jersey, and may be served through its registered agent for service of process, CT Corporation System, 208 S. LaSalle Street, Suite 814, Chicago, IL 60604.

30. Knoll irresponsibly marketed narcotics, such as Vicodin, through whimsical toys and souvenirs and did so to boost the sales of opioids. Taking advantage of the fact that Vicodin was not regulated as a Schedule II controlled substance for many years, and the fact physicians and consumers did not fully appreciate the highly addictive nature of Vicodin, Knoll advertised Vicodin with tag lines such as "The Highest Potency Pain Relief You Can Still Phone In." This tag line came as part and parcel of souvenirs like a "Vicodin" fanny pack and water bottle, both bearing the name of Vicodin, the opioid Knoll was promoting. This irresponsible marketing of a narcotic drug caused doctors and patients to believe Vicodin was safer than it really was, to the detriment of people in Falls County.

31. Abbvie began manufacturing, developing, promoting, marketing, and selling the opioid drug, Vicodin, in the U.S. and in Falls County beginning January 1, 2013. On information and belief, it continues to do so at the time of filing this pleading.

32. ALLERGAN PLC is a public limited company incorporated in Ireland with its principal place of business in Dublin, Ireland. ACTAVIS PLC acquired Allergan plc in March 2015, and the combined company changed its name to Allergan plc in January 2013. Before that, WATSON PHARMACEUTICALS, INC. acquired ACTAVIS, INC. in October 2012, and the combined company changed its name to ALLERGAN FINANCE, LLC as of October 2013. ALLERGAN FINANCE, LLC is a Nevada Corporation with its principal place of business in Parsippany, New Jersey, and may be served through its registered agent for service of process, The Corporation Trust Company of Nevada, 701 S. Carson St., Suite 200, Carson City, NV 89701. WATSON LABORATORIES, INC. is a Nevada corporation with its principal place of business in Corona, California, and is a wholly-owned subsidiary of Allergan plc (f/k/a Actavis, Inc., f/k/a Watson Pharmaceuticals, Inc.), and may be served through its registered agent for service of process, Corporate Creations Network, Inc., 8275 South Eastern Ave., #200, Las Vegas, NV 89123. ACTAVIS PHARMA, INC. (f/k/a Actavis, Inc.) is a Delaware corporation with its principal place of business in New Jersey and was formerly known as WATSON PHARMA, INC, and may be served through its registered agent for service of process, CT Corporation System, 1999 Bryan Street, Suite 900, Dallas, TX 75201. ACTAVIS LLC is a Delaware limited liability company with its principal place of business in Parsippany, New Jersey, and may be served through its registered agent for service of process, Corporate Creations Network, Inc., 3411 Silverside Rd., Tatnall Building, Suite 104, Wilmington, DE 19810. Each of these Defendants is owned by Allergan plc, which uses them to market and sell its drugs in the United States.

33. Upon information and belief, Allergan plc exercises control over these marketing and sales efforts and profits from the sale of Allergan/Actavis products ultimately inure to its benefit. (Allergan plc, Actavis plc, Actavis, Inc., Allergan Finance, LLC, Actavis LLC, Actavis

Pharma, Inc., Watson Pharmaceuticals, Inc., Watson Pharma, Inc., and Watson Laboratories, Inc. are referred to as "Actavis"). Actavis acquired the rights to Kadian from King Pharmaceuticals, Inc. on December 30, 2008 and began marketing Kadian in 2009.

34. Actavis manufactures, promotes, sells, and distributes opioids in the U.S. and in Falls County.

35. MCKESSON CORPORATION ("McKesson") is a Delaware corporation with its principal place of business in San Francisco, California, and may be served through its registered agent for service of process, CSC - Lawyers Incorporating Service, 211 E. 7th Street, Suite 620, Austin, TX 78701. Upon information and belief, McKesson is a pharmaceutical distributor licensed to do business in Texas. McKesson distributes pharmaceuticals to retail pharmacies and institutional providers in all 50 states, including Texas and Falls County.

36. CARDINAL HEALTH, INC. ("Cardinal") is an Ohio Corporation with its principal place of business in Dublin, Ohio, and may be served through its registered agent for service of process, CT Corporation System, 4400 Easton Commons, Suite 125, Columbus, OH 43219. Cardinal does substantial business in Texas and, upon information and belief, Cardinal is a pharmaceutical distributor licensed to do business in Texas. Cardinal distributes pharmaceuticals to retail pharmacies and institutional providers to customers in all 50 states, including Texas and Falls County.

37. AMERISOURCEBERGEN DRUG CORPORATION ("Amerisource") is a Delaware Corporation with its principal place of business in Chesterbrook, Pennsylvania, and may be served through its registered agent for service of process, The Corporation Trust Company, Corporation Trust Center, 1209 Orange St., Wilmington, DE 19801. Amerisource does substantial business in Texas and, upon information and belief, Amerisource is a pharmaceutical distributor

licensed to do business in Texas. Amerisource distributes pharmaceuticals to retail pharmacies and institutional providers to customers in all 50 states, including Texas and Falls County.

38. The County lacks information sufficient to specifically identify the true names or capacities, whether individual, corporate or otherwise, of Defendants sued herein under the fictitious names DOES 1 through 100 inclusive. The County will amend this Petition to show their true names and capacities if and when they are ascertained. Falls County is informed and believes, and on such information and belief alleges, that each of the Defendants named as a DOE has engaged in conduct that contributed to cause events and occurrences alleged in this Petition and, as such, shares liability for at least some part of the relief sought herein.

## V. FACTUAL ALLEGATIONS

39. Before the 1990s, generally accepted standards of medical practice dictated that opioids should be used only for short-term acute pain – pain relating to recovery from surgery or for cancer or palliative (end-of-life) care. Using opioids for chronic pain was discouraged or even prohibited because there was a lack of evidence that opioids improved patients' ability to overcome pain and function. Instead the evidence demonstrated that patients developed tolerance to opioids over time, which increased the risk of addiction and other side effects.

40. Defendants dramatically changed doctors' views regarding opioids through a well-funded deceptive marketing scheme. Each Defendant used direct marketing and unbranded advertising disseminated by seemingly independent third parties to spread false and deceptive statements about the risks and benefits of long-term opioid use.

### A. Defendants Used Multiple Avenues To Disseminate their False and Deceptive Statements about Opioids.

41. Defendants spread their false and deceptive statements by (1) marketing their branded opioids directly to doctors treating patients residing in Falls County and the Falls

County patients themselves and (2) deploying so-called unbiased and independent third parties to Falls County.

**1. Defendants Spread and Continue to Spread Their False and Deceptive Statements Through Direct Marketing of Their Branded Opioids.**

42. Defendants' direct marketing of opioids generally proceeded on two tracks. First, each Defendant conducted advertising campaigns touting the purported benefits of their branded drugs. For example, Defendants spent more than $14 million on medical journal advertising of opioids in 2011, nearly triple what they spent in 2001, including $8.3 million by Purdue, $4.9 million by Janssen, and $1.1 million by Endo.

43. A number of Defendants' branded ads deceptively portrayed the benefits of opioids for chronic pain. For example, Endo distributed and made available on its website, www.opana.com, a pamphlet promoting Opana ER with photographs depicting patients with physically demanding jobs like a construction worker and chef, implying that the drug would provide long-term pain-relief and functional improvement. Purdue also ran a series of ads, called "pain vignettes," for OxyContin in 2012 in medical journals. These ads featured chronic pain patients and recommended OxyContin for each. One ad described a "54-year-old writer with osteoarthritis of the hands" and implied that OxyContin would help the writer work more effectively. Pursuant to a settlement agreement, Endo and Purdue agreed in late 2015 and 2016 to halt these misleading representations in New York, but they may continue to disseminate them in Texas.

44. Second, each Defendant promoted the use of opioids for chronic pain through "detailers" – sales representatives who visited individual doctors and medical staff in their offices – and small-group speaker programs. Defendants devoted massive resources to direct sales contacts with doctors. In 2014 alone, Defendants spent $168 million on detailing branded opioids

to doctors, including $108 million by Purdue, $34 million by Janssen, $10 million by Endo, and $2 million by Actavis. This amount is twice as much as Defendants spent on detailing in 2000.

45. Defendants also identified doctors to serve, for payment, on their speakers' bureaus and to attend programs with speakers and meals paid for by Defendants. These speaker programs provided: (1) an incentive for doctors to prescribe a particular opioid (so they might be selected to promote the drug); (2) recognition and compensation for the doctors selected as speakers; and (3) an opportunity to promote the drug through the speaker to his or her peers. These speakers gave the false impression that they were providing unbiased and medically accurate presentations when they were, in fact, presenting a script prepared by Defendants. On information and belief, these presentations conveyed misleading information, omitted material information, and failed to correct Defendants' prior misrepresentations about the risks and benefits of opioids.

46. Defendants employed the same marketing plans, strategies, and messages in and around Falls County, Texas as they did nationwide. Across the pharmaceutical industry, "core message" development is funded and overseen on a national basis by corporate headquarters. This comprehensive approach ensures that Defendants' messages are accurately and consistently delivered across marketing channels and in each sales territory. Defendants consider this high level of coordination and uniformity crucial to successfully marketing their drugs.

**2. Defendants Used a Diverse Group of Seemingly Independent Third Parties to Spread False and Deceptive Statements about the Risks and Benefits of Opioids.**

47. Defendants also deceptively marketed opioids in and around Falls County through unbranded advertising – *i.e.*, advertising that promotes opioid use generally but does not name a specific opioid. This advertising was ostensibly created and disseminated by independent third parties. But by funding, directing, reviewing, editing, and distributing this unbranded advertising,

Defendants controlled the deceptive messages disseminated by these third parties and acted in concert with them to falsely and misleadingly promote opioids for treating chronic pain.

48. Unbranded advertising also avoided regulatory scrutiny because Defendants did not have to submit it to the FDA, and therefore it was not reviewed by the FDA.

49. Defendants' deceptive unbranded marketing often contradicted their branded materials reviewed by the FDA. For example, Endo's unbranded advertising contradicted its concurrent, branded advertising for Opana ER:

| **Pain: Opioid Therapy (Unbranded)** | **Opana ER Advertisement (Branded)** |
|---|---|
| "People who take opioids **as prescribed usually do not become addicted.**" | "All patients treated with opioids require careful monitoring for signs of abuse and addiction, since **use of opioid analgesic products carries the risk of addiction even under appropriate medical use.**" |

**a. Key Opinion Leaders (KOLs)**

50. Defendants spoke through a small circle of doctors who, upon information and belief, were selected, funded, and elevated by Defendants because their public positions supported using opioids to treat chronic pain. These doctors became known as "key opinion leaders" or "KOLs."

51. Defendants paid KOLs to serve as consultants or on their advisory boards and to give talks or present CMEs, and their support helped these KOLs become respected industry experts. As they rose to prominence, these KOLs touted the benefits of opioids to treat chronic pain, repaying Defendants by advancing their marketing goals. KOLs' professional reputations became dependent on continuing to promote a pro-opioid message, even in activities that were not directly funded by Defendants.

52. KOLs have written, consulted on, edited, and lent their names to books and articles,

and given speeches and CMEs supportive of chronic opioid therapy. Defendants created opportunities for KOLs to participate in research studies Defendants suggested or chose and then cited and promoted favorable studies or articles by their KOLs. By contrast, Defendants did not support, acknowledge, or disseminate publications of doctors unsupportive or critical of chronic opioid therapy.

53. Defendants' KOLs also served on committees that developed treatment guidelines that strongly encourage using opioids to treat chronic pain, and on the boards of pro-opioid advocacy groups and professional societies that develop, select, and present CMEs. Defendants were able to direct and exert control over each of these activities through their KOLs.

54. Pro-opioid doctors are one of the most important avenues that Defendants use to spread their false and deceptive statements about the risks and benefits of long-term opioid use. Defendants know that doctors rely heavily and less critically on their peers for guidance, and KOLs provide the false appearance of unbiased and reliable support for chronic opioid therapy.

55. Defendants utilized many KOLs, including many of the same ones. Two of the most prominent are described below.

**1. Russell Portenoy**

56. Dr. Russell Portenoy, former Chairman of the Department of Pain Medicine and Palliative Care at Beth Israel Medical Center in New York, is one example of a KOL who Defendants identified and promoted to further their marketing campaign. Dr. Portenoy received research support, consulting fees, and honoraria from Endo, Janssen, and Purdue (among others), and was a paid consultant to Purdue.

57. Dr. Portenoy was instrumental in opening the door for the regular use of opioids to treat chronic pain. He served on the American Pain Society ("APS")/American Academy of Pain

Medicine ("AAPM") Guidelines Committees, which endorsed the use of opioids to treat chronic pain, first in 1997 and again in 2009. He was also a member of the board of the American Pain Foundation ("APF"), an advocacy organization almost entirely funded by Defendants.

58. Dr. Portenoy also made frequent media appearances promoting opioids. He appeared on *Good Morning America* in 2010 to discuss using opioids long-term to treat chronic pain. On this widely-watched program, broadcast in Texas and across the country, Dr. Portenoy claimed: "Addiction, when treating pain, is distinctly uncommon. If a person does not have a history, a personal history, of substance abuse, and does not have a history in the family of substance abuse, and does not have a very major psychiatric disorder, most doctors can feel very assured that that person is not going to become addicted."[13]

59. Dr. Portenoy later admitted that he "gave innumerable lectures in the late 1980s and '90s about addiction that weren't true."[14] These lectures falsely claimed that less than 1% of patients would become addicted to opioids. According to Dr. Portenoy, because the primary goal was to "destigmatize" opioids, he and other doctors promoting them overstated their benefits and glossed over their risks. Dr. Portenoy also conceded that "[d]ata about the effectiveness of opioids does not exist."[15] Portenoy candidly stated: "Did I teach about pain management, specifically about opioid therapy, in a way that reflects misinformation? Well...I guess I did."[16]

**2. Lynn Webster**

60. Another KOL, Dr. Lynn Webster, was the co-founder and Chief Medical Director of Lifetree Clinical Research, an otherwise unknown pain clinic in Salt Lake City, Utah. Dr.

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[13] Good Morning America television broadcast, ABC News (Aug. 30, 2010).

[14] Thomas Catan & Evan Perez, *A Pain-Drug Champion Has Second Thoughts*, WALL ST. J., Dec. 17, 2012.

[15] *Id.*

[16] *Id.*

Webster was President in 2013 and is a current board member of AAPM, a Front Group that ardently supports chronic opioid therapy. He is a Senior Editor of *Pain Medicine*, the same journal that published Endo special advertising supplements touting Opana ER. Dr. Webster authored numerous CMEs sponsored by Endo and Purdue while he was receiving significant funding from Defendants.

61. In 2011, Dr. Webster presented a program via webinar sponsored by Purdue titled, *Managing Patient's Opioid Use: Balancing the Need and the Risk.* Dr. Webster recommended using risk screening tools, such as urine testing and patient agreements as a way to prevent "overuse of prescriptions" and "overdose deaths," which was available to and was intended to reach doctors treating Falls County residents.

62. Dr. Webster also was a leading proponent of the concept of "pseudoaddiction," the notion that addictive behaviors should be seen not as warnings, but as indications of undertreated pain. In Dr. Webster's description, the only way to differentiate the two was to *increase* a patient's dose of opioids. As he and his co-author wrote in a book entitled *Avoiding Opioid Abuse While Managing Pain* (2007), a book that is still available online, when faced with signs of aberrant behavior, increasing the dose "in most cases . . . should be the clinician's first response." Endo distributed this book to doctors. Years later, Dr. Webster reversed himself, acknowledging that "[pseudoaddiction] obviously became too much of an excuse to give patients more medication."[17]

**b. Front Groups**

63. Defendants entered into arrangements with seemingly unbiased and independent patient and professional organizations to promote opioids for treating chronic pain. Under Defendants' direction and control, these "Front Groups" generated treatment guidelines, unbranded

[17] John Fauber & Ellen Gabler, *Networking Fuels Painkiller Boom*, MILWAUKEE WISC. J. SENTINEL (Feb. 19, 2012).

materials, and programs that favored chronic opioid therapy. They also assisted Defendants by responding to negative articles, by advocating against regulatory changes that would limit prescribing opioids in accordance with the scientific evidence, and by conducting outreach to vulnerable patient populations targeted by Defendants.

64. These Front Groups depended on Defendants for funding and, in some cases, for survival. Defendants also exercised control over programs and materials created by these groups by collaborating on, editing, and approving their content, and by funding their dissemination. In doing so, Defendants made sure these Groups would generate only the messages Defendants wanted to distribute. Even so, the Front Groups held themselves out as independent and as serving the needs of their members – whether patients suffering from pain or doctors treating those patients.

65. Defendants Endo, Janssen, and Purdue utilized many Front Groups, including many of the same ones. Several of the most prominent are described below, but there are many others, including the American Pain Society ("APS"), American Geriatrics Society ("AGS"), the Federation of State Medical Boards ("FSMB"), American Chronic Pain Association ("ACPA"), American Society of Pain Education ("ASPE"), National Pain Foundation ("NPF") and Pain & Policy Studies Group ("PPSG").

**1. American Pain Foundation ("APF")**

66. The most prominent of Defendants' Front Groups was APF, which received more than $10 million in funding from opioid manufacturers from 2007 until it closed its doors in May 2012. Endo alone provided more than half that funding; Purdue was next at $1.7 million.

67. In 2009 and 2010, more than 80% of APF's operating budget came from pharmaceutical industry sources. Including industry grants for specific projects, APF received about $2.3 million from industry sources out of total income of about $2.85 million in 2009; its budget for 2010 projected receipts of roughly $2.9 million from drug companies, out of total

income of about $3.5 million. By 2011, APF was entirely dependent on incoming grants from defendants Purdue, Endo, and others to avoid using its line of credit. As one of its board members, Russell Portenoy, explained the lack of funding diversity was one of the biggest problems at APF.

68. APF issued education guides for patients, reporters, and policymakers that touted the benefits of opioids for chronic pain and trivialized their risks, particularly the risk of addiction. APF also engaged in a significant multimedia campaign – through radio, television, and the internet – to educate patients about their "right" to pain treatment, namely opioids. All of the programs and materials were available nationally and were intended to reach patients and consumers in Falls County.

**2. American Academy of Pain Medicine ("AAPM")**

69. The American Academy of Pain Medicine, with the assistance, prompting, involvement, and funding of Defendants, issued treatment guidelines and sponsored and hosted medical education programs essential to Defendants' deceptive marketing of chronic opioid therapy.

70. AAPM received over $2.2 million in funding since 2009 from opioid manufacturers. AAPM maintained a corporate relations council, whose members paid $25,000 per year (on top of other funding) to participate. The benefits included allowing members to present educational programs at off-site dinner symposia in connection with AAPM's marquee event – its annual meeting held in Palm Springs, California, or other resort locations. AAPM describes the annual event as an "exclusive venue" for offering education programs to doctors. Membership in the corporate relations council also allows drug company executives and marketing staff to meet with AAPM executive committee members in small settings. Defendants Endo, Purdue, and Actavis were members of the council and presented deceptive programs to doctors who attended

this annual event.

71. AAPM is viewed internally by Endo as "industry friendly," with Endo advisors and speakers among its active members. Endo attended AAPM conferences, funded its CMEs, and distributed its publications. The conferences sponsored by AAPM heavily emphasized sessions on opioids – 37 out of roughly 40 at one conference alone. AAPM's presidents have included top industry-supported KOLs Perry Fine, Russell Portenoy, and Lynn Webster. Dr. Webster was even elected president of AAPM while under a DEA investigation. Another past AAPM president, Dr. Scott Fishman, stated that he would place the organization "at the forefront" of teaching that "the risks of addiction are . . . small and can be managed."[18]

72. AAPM's staff understood they and their industry funders were engaged in a common task. Defendants were able to influence AAPM through both their significant and regular funding and the leadership of pro-opioid KOLs within the organization.

73. In 1997, AAPM and the American Pain Society jointly issued a consensus statement, *The Use of Opioids for the Treatment of Chronic Pain*, which endorsed opioids to treat chronic pain and claimed there was a low risk that patients would become addicted to opioids. The co-author of the statement, Dr. Haddox, was at the time a paid speaker for Purdue. Dr. Portenoy was the sole consultant. The consensus statement remained on AAPM's website until 2011, and was taken down from AAPM's website only after a doctor complained, though it still lingers on the internet elsewhere.

74. AAPM and APS issued their own guidelines in 2009 ("AAPM/APS Guidelines") and continued to recommend using opioids to treat chronic pain. Fourteen of the 21 panel members

[18] Interview by Paula Moyer with Scott M. Fishman, M.D., Professor of Anesthesiology and Pain Medicine, Chief of the Division of Pain Medicine, Univ. of Cal., Davis (2005), http://www.medscape.org/viewarticle/500829.

who drafted the AAPM/APS Guidelines, including KOLs Dr. Portenoy and Dr. Perry Fine of the University of Utah, received support from Janssen, Endo, and Purdue.

75. The 2009 Guidelines promote opioids as "safe and effective" for treating chronic pain, despite acknowledging limited evidence, and conclude that the risk of addiction is manageable for patients regardless of past abuse histories. One panel member, Dr. Joel Saper, Clinical Professor of Neurology at Michigan State University and founder of the Michigan Headache & Neurological Institute, resigned from the panel because he was concerned the 2009 Guidelines were influenced by contributions that drug companies, including Defendants, made to the sponsoring organizations and committee members. These AAPM/APS Guidelines have been a particularly effective channel of deception and have influenced not only treating physicians, but also the body of scientific evidence on opioids. The Guidelines have been cited 732 times in academic literature, were disseminated in and around Falls County during the relevant time period, are still available online, and were reprinted in the *Journal of Pain.*

**B. Defendants' Marketing Scheme Misrepresented the Risks and Benefits of Opioids.**

76. To convince doctors treating residents in Falls County and Falls County patients that opioids can and should be used to treat chronic pain, Defendants had to convince them that long-term opioid use is both safe and effective. Knowing they could do so only by deceiving those doctors and patients about the risks and benefits of long-term opioid use, Defendants made claims that were not supported by, or were contrary to, the scientific evidence. Even though pronouncements by and guidance from the FDA and the CDC based on that evidence confirm that their claims were false and deceptive, Defendants have not corrected them, or instructed their KOLs or Front Groups to correct them, and continue to spread them today.

## C. Defendants Falsely Trivialized or Failed to Disclose the Known Risks of Long-Term Opioid Use.

77. To convince doctors and patients that opioids are safe, Defendants deceptively trivialized and failed to disclose the risks of long-term opioid use, particularly the risk of addiction, through a series of misrepresentations that have been conclusively debunked by the FDA and CDC. These misrepresentations – which are described below – reinforced each other and created the dangerously misleading impression that: (1) starting patients on opioids was low-risk because most patients would not become addicted, and because those who were at greatest risk of addiction could be readily identified and managed; (2) patients who displayed signs of addiction probably were not addicted and, in any event, could easily be weaned from the drugs; (3) the use of higher opioid doses, which many patients need to sustain pain relief as they develop tolerance to the drugs, do not pose special risks; and (4) abuse-deterrent opioids both prevent overdose and are inherently less addictive. Defendants have not only failed to correct these misrepresentations, they continue to make them today.

78. ***First***, Defendants falsely claimed the risk of addiction is low and unlikely to develop when opioids are prescribed, as opposed to obtained illicitly; and failed to disclose the greater risk of addiction with prolonged use of opioids. For example:

a. Actavis's predecessor caused a patient education brochure to be distributed in 2007 claiming opioid addiction is possible, but "less likely if you have never had an addiction problem." Upon information and belief, based on Actavis's acquisition of its predecessor's marketing materials along with the rights to Kadian, Actavis continued to use this brochure in 2009 and beyond;

b. Purdue sponsored APF's *Treatment Options: A Guide for People Living with Pain* (2007), which instructed that addiction is rare and limited to extreme cases of unauthorized dose escalations, obtaining duplicative opioid prescriptions from multiple sources, or theft. This publication is still available online;

c. Endo sponsored a website, Painknowledge.com, which claimed in 2009 that "[p]eople who take opioids as prescribed usually do not become addicted." Another Endo website, PainAction.com, stated "Did you know? Most chronic pain patients do not become addicted to the opioid medications that are prescribed for them;"

d. Endo distributed a pamphlet with the Endo logo entitled *Living with Someone with Chronic Pain*, which stated that: "Most health care providers who treat people with pain agree that most people do not develop an addiction problem." A similar statement appeared on the Endo website, www.opana.com;

e. Janssen reviewed, edited, approved, and distributed a patient education guide entitled *Finding Relief: Pain Management for Older Adults* (2009), which described as "myth" the claim that opioids are addictive, and asserted as fact that "[m]any studies show that opioids are rarely addictive when used properly for the management of chronic pain;"

f. Janssen currently runs a website, Prescriberesponsibly.com (last updated July 2, 2015), which claims that concerns about opioid addiction are "overestimated;"

g. Purdue sponsored APF's *A Policymaker's Guide to Understanding Pain & Its Management* – which claims that less than 1% of children prescribed opioids will become addicted and that pain is undertreated due to "misconceptions about opioid addiction[]." This publication is still available online; and

h. Detailers for Purdue, Endo, and Janssen in and around Falls County minimized or omitted any discussion with doctors of the risk of addiction; misrepresented the potential for opioid abuse with purportedly abuse-deterrent formulations; and routinely did not correct the misrepresentations noted above.

79. These claims contradict longstanding scientific evidence, as the FDA and CDC have conclusively declared. As noted in the 2016 CDC Guideline endorsed by the FDA, there is "extensive evidence" of the "possible harms of opioids (including opioid use disorder [an alternative term for opioid addiction])."[19] The guideline points out that "[o]pioid pain medication use presents serious risks, including...opioid use disorder" and that "continuing opioid therapy for

[19] *CDC Guideline for Prescribing Opioids for Chronic Pain – United States 2016*, Centers for Disease Control and Prevention (Mar. 18, 2016).

3 months substantially increases risk for opioid use disorder."[20]

80. The FDA further exposed the falsity of Defendants' claims about the low risk of addiction when it announced changes to the labels for ER/LA opioids in 2013 and for IR opioids in 2016. In its announcements, the FDA discussed the risks related to opioid use and that IR opioids are associated with "persistent abuse, addiction, overdose mortality, and risk of NOWS [neonatal opioid withdrawal syndrome]."[21]

81. According to the FDA, because of the risks associated with long-term opioid use, including "the serious risk of addiction, abuse, misuse, overdose, and death,"[22] opioids should be "reserved for pain severe enough to require opioid treatment and for which alternative treatment options (e.g., non-opioid analgesics or opioid combination products, as appropriate) are inadequate or not tolerated."[23]

82. The warnings on Defendants' own FDA-approved drug labels caution that opioids "exposes users to risks of addiction, abuse and misuse, which can lead to overdose and death"[24] and that addiction "can occur in patients appropriately prescribed"[25] opioids.

83. ***Second***, Defendants falsely instructed doctors and patients that signs of addiction are actually signs of undertreated pain and should be treated by prescribing more opioids. Defendants called this phenomenon "pseudoaddiction" – a term coined by Dr. David Haddox, who went to work for Purdue, and popularized by Dr. Russell Portenoy, a KOL for Endo, Janssen, and Purdue – and claimed that pseudoaddiction is substantiated by scientific evidence. For example:

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[20] *Id.*
[21] *FDA Announcement of Enhanced Warnings for Immediate-Release Opioid Pain Medications Related to Risks of Misuse, Abuse, Addiction, Overdose and Death*, Federal Drug Administration (Mar. 22, 2016).
[22] *Id.*
[23] *Id.*
[24] *See, e.g.*, OxyContin label and insert at *OxyContin.com.*
[25] *Id.*

a. Purdue sponsored *Responsible Opioid Prescribing* (2007), which taught that behaviors such as "requesting drugs by name," "demanding or manipulative behavior," seeing more than one doctor to obtain opioids, and hoarding, are all signs of pseudoaddiction, rather than true addiction. Responsible Opioid Prescribing remains for sale online. The 2012 edition continues to teach that pseudoaddiction is real;

b. Janssen sponsored, funded, and edited the *Let's Talk Pain* website, which in 2009 stated: "pseudoaddiction . . . refers to patient behaviors that may occur when pain is under-treated . . . . Pseudoaddiction is different from true addiction because such behaviors can be resolved with effective pain management;"

c. Endo sponsored a National Initiative on Pain Control (NIPC) CME program in 2009 titled *Chronic Opioid Therapy: Understanding Risk While Maximizing Analgesia*, which promoted pseudoaddiction by teaching that a patient's aberrant behavior was the result of untreated pain. Endo substantially controlled NIPC by funding NIPC projects; developing, specifying, and reviewing content; and distributing NIPC materials;

d. Purdue published a pamphlet in 2011 entitled *Providing Relief, Preventing Abuse*, which described pseudoaddiction as a concept that "emerged in the literature" to describe the inaccurate interpretation of [drug-seeking behaviors] in patients who have pain that has not been effectively treated;" and

e. Purdue sponsored a CME program entitled *Path of the Patient, Managing Chronic Pain in Younger Adults at Risk for Abuse*. In a role play, a chronic pain patient with a history of drug abuse tells his doctor that he is taking twice as many hydrocodone pills as directed. The narrator notes that because of pseudoaddiction, the doctor should not assume the patient is addicted even if he persistently asks for a specific drug, seems desperate, hoards medicine, or "overindulges in unapproved escalating doses." The doctor treats this patient by prescribing a high-dose, long-acting opioid.

84. The 2016 CDC Guideline rejects the concept of pseudoaddiction. The Guideline nowhere recommends that opioid dosages be increased if a patient is not experiencing pain relief. To the contrary, the Guideline explains that "[p]atients who do not experience clinically meaningful pain relief early in treatment...are unlikely to experience pain relief with longer-term

use,"[26] and that physicians should "reassess[] pain and function within 1 month"[27] in order to decide whether to "minimize risks of long-term opioid use by discontinuing opioids"[28] because the patient is "not receiving a clear benefit."[29]

85. ***Third,*** Defendants falsely instructed doctors and patients that addiction risk screening tools, patient contracts, urine drug screens, and similar strategies allow them to reliably identify and safely prescribe opioids to patients predisposed to addiction. These misrepresentations were especially insidious because Defendants aimed them at general practitioners and family doctors who lack the time and expertise to closely manage higher-risk patients. Defendants' misrepresentations made these doctors feel more comfortable prescribing opioids to their patients, and patients more comfortable starting opioid therapy for chronic pain. For example:

a. Endo paid for a 2007 supplement in the *Journal of Family Practice* written by a doctor who became a member of Endo's speakers' bureau in 2010. The supplement, entitled *Pain Management Dilemmas in Primary Care: Use of Opioids*, emphasized the effectiveness of screening tools, claiming that patients at high risk of addiction could safely receive chronic opioid therapy using a "maximally structured approach" involving toxicology screens and pill counts;

b. Purdue sponsored a 2011 webinar, *Managing Patient's Opioid Use: Balancing the Need and Risk*, which claimed that screening tools, urine tests, and patient agreements prevent "overuse of prescriptions" and "overdose deaths;" and

c. As recently as 2015, Purdue has represented in scientific conferences that "bad apple" patients – and not opioids – are the source of the addiction crisis and that once those "bad apples" are identified, doctors can safely prescribe opioids without causing addiction.

86. Once again, the 2016 CDC Guideline confirms these representations are false. The Guideline notes that there are no studies assessing the effectiveness of risk mitigation strategies –

[26] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[27] *Id.*
[28] *Id.*
[29] *Id.*

such as screening tools, patient contracts, urine drug testing, or pill counts – widely believed by doctors to detect and deter outcomes related to addiction and overdose.[30] As a result, the Guideline recognizes that doctors should not overestimate the risk screening tools for classifying patients as high or low risk for opioid addiction because they are insufficient to rule out the risks of long-term opioid therapy.[31]

87. ***Fourth***, to underplay the risk and impact of addiction and make doctors feel more comfortable starting patients on opioids, Defendants falsely claimed that opioid dependence can easily be addressed by tapering and that opioid withdrawal is not a problem thereby failing to disclose the increased difficulty of stopping opioids after long-term use.

88. For example, a CME sponsored by Endo, entitled *Persistent Pain in the Older Adult*, claimed that withdrawal symptoms can be avoided by tapering a patient's opioid dose by 10%-20% for 10 days. And Purdue sponsored APF's *A Policymaker's Guide to Understanding Pain & Its Management*, which claimed that "[s]ymptoms of physical dependence can often be ameliorated by gradually decreasing the dose of medication during discontinuation."

89. Defendants deceptively minimized the significant symptoms of opioid withdrawal, which, as explained in the 2016 CDC Guideline, include drug cravings, anxiety, insomnia, abdominal pain, vomiting, diarrhea, sweating, tremor, tachycardia (rapid heartbeat), spontaneous abortion and premature labor in pregnant women, and the unmasking of anxiety, depression, and addiction – and grossly understated the difficulty of tapering, particularly after long-term opioid use.

90. Yet the 2016 CDC Guideline recognizes that the duration of opioid use and the dosage of opioids prescribed should be limited to "minimize the need to taper opioids to prevent

[30] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[31] *See id.*

distressing or unpleasant withdrawal symptoms,"[32] because "physical dependence on opioids is an expected physiologic response in patients exposed to opioids for more than a few days."[33] The Guideline further states that "tapering opioids can be especially challenging after years on high dosages because of physical and psychological dependence"[34] and highlights the difficulties, including the need to carefully identify "a taper slow enough to minimize symptoms and signs of opioid withdrawal"[35] and pausing and restarting tapers depending on the patient's response.

91. The CDC also acknowledges the lack of any "high-quality studies comparing the effectiveness of different tapering protocols for use when opioid dosage is reduced or opioids are discontinued."[36]

92. ***Fifth***, Defendants falsely claimed that doctors and patients could increase opioid dosages indefinitely without added risk and failed to disclose the greater risks to patients at higher dosages. The ability to escalate dosages was critical to Defendants' efforts to market opioids for long-term use to treat chronic pain because, absent this misrepresentation, doctors would have abandoned treatment when patients built up tolerance and lower dosages did not provide pain relief. For example:

a. Actavis's predecessor created a patient brochure for Kadian in 2007 that stated, "Over time, your body may become tolerant of your current dose. You may require a dose adjustment to get the right amount of pain relief. This is not addiction." Upon information and belief, based on Actavis's acquisition of its predecessor's marketing materials along with the rights to Kadian, Actavis continued to use these materials in 2009 and beyond;

b. Purdue sponsored *APF's Treatment Options: A Guide for People Living with Pain* (2007), which claims that some patients "need" a larger dose of an opioid, regardless of the dose currently prescribed. The guide stated that opioids have "no ceiling dose" and are therefore the most appropriate

[32] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[33] *Id.*
[34] *Id.*
[35] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[36] *Id.*

treatment for severe pain. This guide is still available for sale online;

c. Endo sponsored a website, painknowledge.com, which claimed in 2009 that opioid dosages may be increased until "you are on the right dose of medication for your pain;"

d. Endo distributed a pamphlet edited by a KOL entitled *Understanding Your Pain: Taking Oral Opioid Analgesics*, which was available during the time period of this Complaint on Endo's website. In Q&A format, it asked "If I take the opioid now, will it work later when I really need it?" The response is, "The dose can be increased. . . . You won't 'run out' of pain relief;"

e. Janssen sponsored a patient education guide entitled *Finding Relief: Pain Management for Older Adults* (2009), which was distributed by its sales force. This guide listed dosage limitations as "disadvantages" of other pain medicines but omitted any discussion of risks of increased opioid dosages;

f. Purdue's In the Face of Pain website promotes the notion that if a patient's doctor does not prescribe what, in the patient's view, is a sufficient dosage of opioids, he or she should find another doctor who will;

g. Purdue sponsored APF's *A Policymaker's Guide to Understanding Pain & Its Management*, which taught that dosage escalations are "sometimes necessary," even unlimited ones, but did not disclose the risks from high opioid dosages. This publication is still available online;

h. Purdue sponsored a CME entitled *Overview of Management Options* that is still available for CME credit. The CME was edited by a KOL and taught that NSAIDs and other drugs, but not opioids, are unsafe at high dosages; and

i. Purdue presented a 2015 paper at the College on the Problems of Drug Dependence, the "the oldest and largest organization in the US dedicated to advancing a scientific approach to substance use and addictive disorders," challenging the correlation between opioid dosage and overdose.

93. These claims conflict with the scientific evidence, as confirmed by the FDA and CDC. As the CDC explains in its 2016 Guideline, the "[b]enefits of high-dose opioids for chronic pain are not established"[37] while the "risks for serious harms related to opioid therapy increase at

[37] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*

higher opioid dosage."[38]

94. More specifically, the CDC explains that "there is now an established body of scientific evidence showing that overdose risk is increased at higher opioid dosages."[39] Similarly, there is an "increased risk for opioid use disorder, respiratory depression, and death at higher dosages."[40] That is why the CDC advises doctors to avoid increasing dosages above 90 morphine milligram equivalents per day.

95. The 2016 CDC Guideline reinforces earlier findings announced by the FDA. In 2013, the FDA acknowledged that available data suggested that increasing the opioid dosage likewise increased certain adverse events. For example, the FDA noted that studies suggest a positive association between high-dose opioid use and overdoses.

96. ***Finally***, Defendants' deceptive marketing of the so-called abuse-deterrent properties of some of their opioids has created false impressions that these opioids can curb addiction and abuse.

97. More specifically, Defendants have made misleading claims about the ability of their so-called abuse-deterrent opioid formulations to deter addiction and overdose. For example, Endo's advertisements for the 2012 reformulation of Opana ER claimed that it was designed to be crush resistant in a way that suggested it was more difficult to misuse the product. This claim was false.

98. The FDA warned in a 2013 letter that there was no evidence Endo's design would provide a reduction in oral, intranasal or intravenous use.[41] Moreover, Endo's own studies, which it failed to disclose, showed that Opana ER could still be ground and chewed.

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[38] *Id.*
[39] *Id.*
[40] *Id.*
[41] *See FDA Statement: Original Opana ER Relisting Determination* (May 10, 2013).

99. In a 2016 settlement with the State of New York, Endo agreed not to make statements in New York that Opana ER was designed to be or is crush resistant. The State found those statements false and deceptive because there was no difference in the ability to extract the narcotic from Opana ER.

100. Similarly, the 2016 CDC Guideline states that no studies support the notion that "abuse-deterrent technologies [are] a risk mitigation strategy for deterring or preventing abuse,"[42] noting that the technologies – even when they work – "do not prevent opioid abuse through oral intake, the most common route of opioid abuse, and can still be abused by non-oral routes."[43]

101. These numerous, long-standing misrepresentations of the risks of long-term opioid use spread by Defendants successfully convinced doctors and patients to discount those risks.

**D. Defendants Grossly Overstated the Benefits of Chronic Opioid Therapy.**

102. To convince doctors and patients that opioids should be used to treat chronic pain, Defendants had to persuade them that there was a significant benefit to long-term opioid use. But as the 2016 CDC Guideline makes clear, there is "insufficient evidence to determine the long-term benefits of opioid therapy for chronic pain."[44]

103. In fact, the CDC found no evidence showing "a long-term benefit of opioids in pain and function versus no opioids for chronic pain with outcomes examined at least 1 year later (with most placebo-controlled randomized trials $\leq$ 6 weeks in duration)"[45] and that other treatments were more or equally beneficial and less harmful than long-term opioid use. The FDA, too, has recognized the lack of evidence to support long-term opioid use.

104. In 2013, the FDA stated that it was unaware of any studies demonstrating the

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[42] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[43] *Id.*
[44] *Id.*
[45] *Id.*

safety and efficacy of opioids for long-term use.[46] Despite this lack of studies, Defendants falsely and misleadingly touted the benefits of long-term opioid use and suggested that these benefits were supported by scientific evidence. Not only have Defendants failed to correct these false and deceptive claims, they continue to make them today. For example:

a. Actavis distributed an advertisement that claimed that the use of Kadian to treat chronic pain would allow patients to return to work, relieve "stress on your body and your mental health," and help patients enjoy their lives;

b. Endo distributed advertisements that claimed that the use of Opana ER for chronic pain would allow patients to perform demanding tasks like construction work or work as a chef and portrayed seemingly healthy, unimpaired subjects;

c. Janssen sponsored and edited a patient education guide entitled *Finding Relief: Pain Management for Older Adults* (2009) – which states as "a fact" that "opioids may make it easier for people to live normally." The guide lists expected functional improvements from opioid use, including sleeping through the night, returning to work, recreation, sex, walking, and climbing stairs;

d. Purdue ran a series of advertisements for OxyContin in 2012 in medical journals entitled "pain vignettes," which were case studies featuring patients with pain conditions persisting over several months and recommending OxyContin for them. The ads implied that OxyContin improves patients' function;

e. *Responsible Opioid Prescribing* (2007), sponsored and distributed by Endo and Purdue, taught that relief of pain by opioids, by itself, improved patients' function. The book remains for sale online;

f. Purdue sponsored APF's *Treatment Options: A Guide for People Living with Pain* (2007), which counseled patients that opioids "give [pain patients] a quality of life we deserve." The guide was available online until APF shut its doors in 2012;

g. Endo's NIPC website *painknowledge.com* claimed in 2009 that with opioids, "your level of function should improve; you may find you are now able to participate in activities of daily living, such as work and hobbies, that you were not able to enjoy when your pain was worse." Elsewhere, the website touted improved quality of life (as well as "improved function") as

[46] Letter from Janet Woodcock, M.D., Dir., Ctr. For Drug Eval. & Res., to Andrew Kolodny, M.D., Pres. Physicians for Responsible Opioid Prescribing, Re Docket No. FDA-2012-P-0818 (Sept. 10, 2013).

benefits of opioid therapy. The grant request that Endo approved for this project specifically indicated NIPC's intent to make misleading claims about function, and Endo closely tracked visits to the site;

h. Endo was the sole sponsor, through NIPC, of a series of CMEs titled *Persistent Pain in the Older Patient*, which claimed that chronic opioid therapy has been "shown to reduce pain and improve depressive symptoms and cognitive functioning." The CME was disseminated via webcast;

i. Janssen sponsored, funded, and edited a website, *Let's Talk Pain*, in 2009, which featured an interview edited by Janssen claiming that opioids allowed a patient to "continue to function." This video is still available today on YouTube;

j. Purdue sponsored the development and distribution of APF's *A Policymaker's Guide to Understanding Pain & Its Management*, which claimed that "multiple clinical studies" have shown that opioids are effective in improving daily function, psychological health, and health-related quality of life for chronic pain patients." The Policymaker's Guide was originally published in 2011 and is still available online today; and

k. Purdue's, Endo's, and Janssen's sales representatives have conveyed and continue to convey the message that opioids will improve patient function.

105. These claims find no support in the scientific literature. Most recently, the 2016 CDC Guideline, approved by the FDA, concluded, "There is no good evidence that opioids improve pain or function with long-term use"[47] and "complete relief of pain is unlikely."[48] (Emphasis added.) The CDC reinforced this conclusion throughout its 2016 Guideline:

a. "No evidence shows a long-term benefit of opioids in pain and function versus no opioids for chronic pain with outcomes examined at least 1 year later . . ."[49]

b. "Although opioids can reduce pain during short-term use, the clinical evidence review found insufficient evidence to determine whether pain relief is sustained and whether function or quality of life improves with long-term opioid therapy;"[50] and

c. "[E]vidence is limited or insufficient for improved pain or function with

[47] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[48] *Id.*
[49] *Id.*
[50] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*

> long-term use of opioids for several chronic pain conditions for which opioids are commonly prescribed, such as low back pain, headache, and fibromyalgia."[51]

106. The CDC also noted that the risks of addiction and death "can cause distress and inability to fulfill major role obligations."[52] As a matter of common sense (and medical evidence), drugs that can kill patients or commit them to a life of addiction or recovery do not improve their function and quality of life.

107. The 2016 CDC Guideline was not the first time a federal agency repudiated Defendants' claim that opioids improved function and quality of life. In 2010, the FDA warned Actavis that "[w]e are not aware of substantial evidence or substantial clinical experience demonstrating that the magnitude of the effect of the drug [Kadian] has in alleviating pain, taken together with any drug-related side effects patients may experience...results in any overall positive impact on a patient's work, physical and mental functioning, daily activities, or enjoyment of life."[53]

108. Defendants also falsely emphasized or exaggerated the risks of competing products like NSAIDs so that doctors and patients would look to opioids first for treating chronic pain. Once again, Defendants' misrepresentations contravene pronouncements by and guidance from the FDA and CDC based on the scientific evidence.

109. Consequently, the FDA changed the labels for ER/LA opioids in 2013 and IR opioids in 2016 to state that opioids should be used only as a last resort where alternative treatments like non-opioid drugs are inadequate. And the 2016 CDC Guideline states that NSAIDs,

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[51] *Id.*

[52] *Id.*

[53] Warning Letter from Thomas Abrams, Dir., FDA Div. of Mktg., Adver., & Commc'ns, to Doug Boothe, CEO, Actavis Elizabeth LLC (Feb. 18, 2010), http://www.fda.gov/Drugs/GuidanceComplianceRegulatoryInformation/EnforcementActivitiesbyFDA/WarningLettersandNoticeofViolationLetterstoPharmaceuticalCompanies/ucm259240.htm.

not opioids, should be the first-line treatment for chronic pain, particularly arthritis and lower back pain.

110. In addition, Purdue misleadingly promoted OxyContin as unique among opioids in providing 12 continuous hours of pain relief with one dose. In fact, OxyContin does not last for 12 hours – a fact that Purdue has known at all times relevant to this action.

111. According to Purdue's own research, OxyContin wears off in under six hours in one quarter of patients and in under 10 hours in more than half. The reason is that OxyContin tablets release approximately 40% of their active medicine immediately, after which release tapers. Although the patient experiences a powerful initial response, there is little or no pain relief at the end of the dosing period because less medicine is released.

112. This phenomenon is known as "end of dose" failure, and the FDA found in 2008 that a substantial number of chronic pain patients taking OxyContin experience it.

113. This "end of dose" failure not only renders Purdue's promise of 12 hours of relief false and deceptive, it also makes OxyContin more dangerous because the declining pain relief patients experience toward the end of each dosing period drives them to take more OxyContin before the next dosing period begins, quickly increasing the amount of drug they are taking and spurring growing dependence.

114. Purdue's competitors were aware of this problem. For example, Endo ran advertisements for Opana ER referring to "real" 12-hour dosing. Nevertheless, Purdue falsely promoted OxyContin as if it were effective for a full 12 hours. Indeed, Purdue's sales representatives continue to tell doctors in and around Falls County that OxyContin lasts a full 12 hours.

**E. Defendants also engaged in Other Unlawful, Unfair, and Fraudulent Misconduct.**

115. Defendants herein participated in illicit and unlawful prescribing of its drugs. For example, Purdue did not report illegal prescribing of OxyContin until years after law enforcement shut down a Los Angeles clinic that prescribed more than 1.1 million OxyContin tablets. In doing so, Purdue protected its own profits at the expense of public health and safety.

116. The State of New York found that Endo failed to require sales representatives to report signs of addiction, diversion, and inappropriate prescribing; paid bonuses to sales representatives for detailing prescribers who were subsequently arrested or convicted for illegal prescribing; and failed to prevent sales representatives from visiting prescribers whose suspicious conduct had caused them to be placed on a no-call list.

**F. Defendants Targeted Susceptible Prescribers and Vulnerable Patient Populations.**

117. As a part of their deceptive marketing scheme, Defendants identified and targeted susceptible prescribers and vulnerable patient populations in the U.S. and in and around Falls County. For example, Defendants focused their deceptive marketing on primary care doctors, who were more likely to treat chronic pain patients and prescribe opioids, but were less likely to be educated about treating pain and the risks and benefits of opioids.

118. Defendants also targeted vulnerable patient populations like the elderly and veterans, who tend to suffer from chronic pain. Defendants targeted these vulnerable patients even though the risks of long-term opioid use were significantly greater for them.

119. For example, the 2016 CDC Guideline observes that existing evidence shows that elderly patients taking opioids suffer from elevated fall and fracture risks, greater risk of hospitalization, and increased vulnerability to adverse drug effects and interactions. The Guideline therefore concludes that there are "special risks of long-term opioid use for elderly patients" and

recommends that doctors use "additional caution and increased monitoring" to minimize the risks of opioid use in elderly patients.

120. The same is true for veterans, who are more likely to use anti-anxiety drugs (benzodiazepines) for post-traumatic stress disorder, which interact dangerously with opioids.

**G. Although Defendants knew that their Marketing of Opioids was False and Deceptive, they Fraudulently Concealed their Misconduct.**

121. Defendants, both individually and collectively, made, promoted, and profited from their misrepresentations about the risks and benefits of opioids for chronic pain even though they knew their misrepresentations were false and deceptive. The history of opioids, as well as research and clinical experience over the last 20 years, established that opioids were highly addictive and responsible for a long list of very serious adverse outcomes.

122. Not only did the FDA and other regulators warn Defendants, but Defendants had access to scientific studies, detailed prescription data, and reports of adverse events, including reports of addiction, hospitalization, and deaths – all of which made clear the harms from long-term opioid use, including the suffering from addiction, overdoses, and death in alarming numbers in patients using opioids.

123. More recently, the FDA and CDC have issued pronouncements based on the medical evidence that conclusively expose the known falsity of Defendants' misrepresentations, and Endo and Purdue have recently entered agreements prohibiting them from making some of the same misrepresentations described herein in New York.

124. Specifically, three current and former executives from Purdue plead guilty in 2007 to criminal charges that they misled regulators, doctors, and patients about OxyContin's risk of

addiction.[54] In pleading guilty to misbranding charges, Purdue admitted it had fraudulently marketed OxyContin as a drug less prone to addiction and as having fewer side effects than other opioids.[55] In reality, unlike other opioids, OxyContin contained pure oxycodone without any other ingredients, which made it a powerful narcotic despite its time-release design that Purdue touted as ameliorating its addictive potential.[56]

125. Moreover, Defendants took steps to avoid detection of and to fraudulently conceal their deceptive marketing and unlawful, unfair, and fraudulent conduct. For example, Defendants disguised their own role in the deceptive marketing of chronic opioid therapy by funding and working through third parties like Front Groups and KOLs.

126. Finally, Defendants manipulated their promotional materials and the scientific literature to make it appear that these items were accurate, truthful, and supported by objective evidence when they were not.

127. Thus, Defendants successfully concealed from the medical community and patients facts sufficient to arouse suspicion of the claims Falls County now asserts. Falls County did not know of the existence or scope of Defendants' industry-wide fraud and could not have acquired such knowledge earlier through the exercise of reasonable diligence.

## H. By Increasing Opioid Prescriptions and Use, Defendants' Deceptive Marketing Scheme has fueled the Opioid Epidemic and Devastated Falls County Communities.

128. Defendants' misrepresentations deceived doctors and patients about the risks and benefits of long-term opioid use. Studies reveal that many doctors and patients are unaware of or do not understand the risks or benefits of opioids. Indeed, patients often report that they were not

[54] *See* Barry Meier, "In Guilty Plea, OxyContin Maker to Pay $600 Million," (May 10, 2007), http://www.nytimes.com/2007/05/10/business/11drug-web.html.
[55] *See id.*
[56] *See id.*

warned they might become addicted to opioids prescribed to them. As reported in January 2016, a 2015 survey of more than 1,000 opioid patients found that 4 out of 10 were not told opioids were potentially addictive.[57]

129. Defendants' deceptive marketing scheme caused and continues to cause doctors in and around Falls County to prescribe opioids for chronic pain conditions such as back pain, headaches, arthritis, and fibromyalgia. Absent Defendants' deceptive marketing scheme, these doctors would not have been able to over prescribe opioids or become embroiled in pill mills that negatively impacted residents of Falls County.

130. While Defendants may claim the federal government authorized the amount of annual prescription opioids sold, they know in truth that several Defendants have successfully used their organized money and influence to render the federal government's enforcement agency, the Drug Enforcement Administration, virtually powerless to interrupt the over-supply of prescription opioid drugs.

131. Defendants' deceptive marketing scheme also caused and continues to cause patients to purchase and use opioids for their chronic pain believing they are safe and effective. Absent Defendants' deceptive marketing scheme, fewer patients would be using opioids long-term to treat chronic pain, and those patients using opioids would be using less of them.

132. Defendants' deceptive marketing has caused and continues to cause the prescribing and use of opioids to explode. Indeed, this dramatic increase in opioid prescriptions and use corresponds with the dramatic increase in Defendants' spending on their deceptive marketing scheme. Defendants' spending on opioid marketing totaled approximately $91 million in 2000.

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[57] Hazelden Betty Ford Foundation, *Missed Questions, Missed Opportunities* (Jan. 27, 2016), *available at* http://www.hazeldenbettyford.org/about-us/news-and-media/pressrelease/doctors-missing-questions-that-could-prevent-opioid-addiction.

By 2011, that spending had tripled to $288 million.

133. The escalating number of opioid prescriptions written by doctors who were deceived by Defendants' deceptive marketing scheme is the cause of a correspondingly dramatic increase in opioid addiction, overdose, and death throughout the U.S. and Falls County.

134. Scientific evidence demonstrates a strong correlation between opioid prescriptions and becoming addicted to opioids. In a 2016 report, the CDC explained that prescribing opioids has quadrupled since 1999, which has resulted in a parallel increase in opioid overdoses.[58] Indeed, there has been a two-third increase in overdose deaths from using opioids since 2000.[59] For these reasons, the CDC concluded that efforts to rein in the prescribing of opioids for chronic pain are critical "to reverse the cycle of opioid pain medication misuse that contributes to the opioid overdose epidemic."[60]

135. Due to the increase in opioid overdoses, first responders such as police officers, have been and will continue to be in the position to assist people experiencing opioid-related overdoses.[61] In 2016, "over 1,200 law enforcement departments nationwide carried naloxone in an effort to prevent opioid-related deaths."[62]

136. Defendants' deceptive marketing scheme has also detrimentally impacted children in Falls County. Overprescribing opioids for chronic pain has made the drugs more accessible to

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[58] CDC. National Vital Statistics System, Mortality. CDC WONDER. Atlanta, GA: US Department of Health and Human Services, CDC; 2016. https://wonder.cdc.gov/; Rudd RA, Seth P, David F, Scholl L. Increases in Drug and Opioid-Involved Overdose Deaths — United States, 2010–2015. MMWR Morb Mortal Wkly Rep. ePub: 16 December 2016.

[59] *National Vital Statistics System*, Mortality file and appearing *Center for Disease Control and Prevention* Morbidity and Mortality Weekly Report, January 1, 2006 / 64(50); 1378-82, Increases in Drug and Opioid Deaths – United States, 2000-2014.

[60] *CDC Guideline for Prescribing Opioids for Chronic Pain, supra; see also* Rudd RA, Seth P, David F, Scholl L. Increases in Drug and Opioid-Involved Overdose Deaths — United States, 2010–2015. MMWR Morb Mortal Wkly Rep. ePub: 16 December 2016.

[61] Opinion of the Attorney General of Texas, KP-0168 (Oct. 4, 2017).

[62] *Id. citing* http://www.nchrc.org/law-enforcement/us-law-enforcement-who-carry-naloxone/.

school-aged children, who come into contact with opioids after they have been prescribed to friends or relatives in the same household.

137. Defendants' conduct has adversely affected Falls County's child protection agencies in the number of children in foster care driven by parental drug addiction. Children with parents addicted to drugs tend to stay in foster care longer, and they often enter the system having experienced significant trauma, which makes these cases more expensive for counties like Falls County.

138. Opioid addiction is one of the primary reasons that Falls County residents seek treatment for substance dependence. A significant number of admissions for drug addiction were associated with a primary diagnosis of opiate addiction or dependence.

139. Defendants' creation, through false and deceptive advertising and other unlawful and unfair conduct, of a virtually limitless opioid market has significantly harmed Falls County communities. Defendants' success in extending the market for opioids to new patients and chronic pain conditions has created an abundance of drugs available for non-medical and criminal use and fueled a new wave of addiction and injury. It has been estimated that 60% of the opioids to which people are addicted come, directly or indirectly, through doctors' prescriptions.[63]

140. Law enforcement agencies have increasingly associated prescription drug addiction with violent and property crimes. Despite strict federal regulation of prescription drugs, local law enforcement agencies are faced with increasing diversion from legitimate sources for illicit purposes, including doctor shopping, forged prescriptions, falsified pharmacy records, and employees who steal from their place of employment. The opioid epidemic has prompted a

[63] Nathaniel P. Katz, *Prescription Opioid Abuse: Challenges and Opportunities for Payers*, Am. J. Managed Care (Apr. 19 2013), at 5 ("The most common source of abused [opioids] is, directly or indirectly, by prescription."), http://www.ajmc.com/publications/issue/2013/2013-1-vol19-n4/Prescription-Opioid-Abuse-Challenges-and-Opportunities-for-Payers.

growing trend of crimes against pharmacies including robbery and burglary. This ongoing diversion of prescription narcotics creates a lucrative marketplace.

141. The rise in opioid addiction caused by Defendants' deceptive marketing scheme has also resulted in an explosion in heroin use. For example, heroin use has more than doubled in the past decade among adults aged 18 to 25 years.[64] Moreover, heroin-related overdoses in the United States has more than quadrupled since 2010.[65]

142. The costs and consequences of opioid addiction are staggering. For example, in 2007, the cost of healthcare due to opioid addiction and dependence was estimated at 25 billion, the cost of criminal justice was estimated at 5.1 billion, and the cost of lost workplace productivity was estimated at 25.6 billion.

143. Consequently, prescription opioid addiction and overdose have an enormous impact on the health and safety of individuals, as well as communities at large, because the consequences of this epidemic reach far beyond the addicted individual.

144. Some of the repercussions for residents of Falls County include job loss, loss of custody of children, physical and mental health problems, homelessness and incarceration, which results in instability in communities often already in economic crisis and contributes to increased demand on community services such as hospitals, courts, child services, treatment centers, and law enforcement.

145. Defendants knew and should have known about these harms that their deceptive marketing has caused and continues to cause and will cause in the future. Defendants closely monitored their sales and the habits of prescribing doctors. Their sales representatives, who visited

[64] Centers for Disease Control and Prevention. Vital Signs: Today's Heroin Epidemic – More People at Risk, Multiple Drugs Abused. (https://www.cdc.gov/vitalsigns/heroin/index.html). MMWR 2015.
[65] https://www.cdc.gov/vitalsigns/heroin/index.html

doctors and attended CMEs, knew which doctors were receiving their messages and how they were responding.

146. Defendants also had access to and carefully watched government and other data that tracked the explosive rise in opioid use, addiction, injury, and death. Defendants not only knew, but intended that their misrepresentations would persuade doctors to prescribe and encourage patients to use their opioids for chronic pain.

147. Defendants' actions are neither permitted nor excused by the fact that their drug labels may have allowed, or did not exclude, the use of opioids for chronic pain. FDA approval of opioids for certain uses did not give Defendants license to misrepresent the risks and benefits of opioids. Indeed, Defendants' misrepresentations were directly contrary to pronouncements by, and guidance from, the FDA based on the medical evidence and their own labels.

148. Nor is Defendants' causal role broken by the involvement of doctors. Defendants' marketing efforts were ubiquitous and highly persuasive. Their deceptive messages tainted virtually every source doctors could rely on for information and prevented them from making informed treatment decisions. Defendants also hijacked what doctors wanted to believe – namely, that opioids represented a means of relieving their patients' suffering and of practicing medicine more compassionately.

149. Defendants' actions and omissions were each a cause-in-fact of Falls County's past and future damages. Defendants' wrongful conduct caused injuries to Falls County in the past, continues to cause injuries to Falls County, and will continue to cause injuries to Falls County in the future. Future damages include, but are not limited to, additional resources for counseling and medication assisted treatment of addicts, medical treatment for overdoses, life skills training for adolescents, increased law enforcement, and additional resources to treat the psychological effects

of opioids and the underlying conditions that make people susceptible to opioid addiction.

## I. Defendants' Fraudulent Marketing Has Led To Record Profits.

150. While using opioids has taken an enormous toll on Falls County and its residents, Defendants have realized blockbuster profits. In 2014 alone, opioids generated $11 billion in revenue for drug companies like Defendants. Indeed, financial information indicates that each Defendant experienced a material increase in sales, revenue, and profits from the false and deceptive advertising and other unlawful and unfair conduct described above.

## VI. FIRST CAUSE OF ACTION: PUBLIC NUISANCE AGAINST ALL DEFENDANTS

151. Falls County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

152. Defendants knowingly encouraged doctors in and around Falls County to prescribe, and residents to use, highly addictive opioids for chronic pain even though Defendants knew using opioids had a high risk of addiction and reduced quality of life.

153. By doing so, Defendants purposefully interfered with Falls County's public health, public safety, public peace, public comfort, and public convenience.

154. Defendants, individually and in concert with each other, have contributed to and/or assisted in creating and maintaining a condition that is harmful to the health and safety of Falls County residents and/or unreasonably interferes with the peace and comfortable enjoyment of life in violation of Texas law.

155. The public nuisance created by Defendants' actions is substantial and unreasonable – it has caused and continues to cause significant harm to the community – and the harm inflicted outweighs any offsetting benefit.

156. The staggering rates of opioid use resulting from Defendants' marketing efforts

have caused, and continues to cause, harm to the community including, but not limited to:

a. Upwards of 30% of all adults use opioids. These high rates of use have led to unnecessary opioid addiction, overdose, injuries, and deaths;

b. Children have been exposed to opioids prescribed to family members or others resulting in injury, addiction, and death. Easy access to prescription opioids has made opioids a recreational drug of choice among Falls County teenagers; opioid use among teenagers is only outpaced by marijuana use. Even infants have been born addicted to opioids due to prenatal exposure causing severe withdrawal symptoms and lasting developmental impacts;

c. Residents of Falls County, who have never taken opioids, have endured both the emotional and financial costs of caring for loved ones addicted to or injured by opioids and the loss of companionship, wages, or other support from family members who have used, become addicted to, overdosed on, or been killed by opioids;

d. More broadly, opioid use and addiction have driven Falls County residents' health care costs higher;

e. Employers have lost the value of productive and healthy employees who have suffered from adverse consequences from opioid use;

f. Defendants' success in extending the market for opioids to new patients and chronic conditions has created an abundance of drugs available for criminal use and fueled a new wave of addiction and injury. Defendants' scheme created both ends of a new secondary market for opioids – providing both the supply of narcotics to sell and the demand of addicts to buy them;

g. This demand has created additional illicit markets in other opiates, particularly heroin. The low cost of heroin has led some of those who initially become addicted to prescription opioids to migrate to cheaper heroin, fueling a new heroin epidemic in the process;

h. Diverting opioids into secondary, criminal markets and increasing the number of individuals who are addicted to opioids has increased the demands on emergency services and law enforcement in Falls County;

i. All of Defendants' actions have caused significant harm to the community – in lives lost; addictions endured; the creation of an illicit drug market and all its concomitant crime and costs; unrealized economic productivity; and broken families and homes;

j. These harms have taxed the human, medical, public health, law enforcement, and financial resources of Falls County; and

k. Defendants' interference with the comfortable enjoyment of life of a substantial number of people is entirely unreasonable because there is limited social utility to opioid use and any potential value is outweighed by the gravity of harm inflicted by Defendants' actions.

157. Defendants knew, or should have known, that promoting opioid use would create a public nuisance in the following ways:

a. Defendants have engaged in massive production, promotion, and distribution of opioids for use by the citizens of Falls County;

b. Defendants' actions created and expanded the market for opioids, promoting its wide use for pain management;

c. Defendants misrepresented the benefits of opioids for chronic pain and fraudulently concealed, misrepresented, and omitted the serious adverse effects of opioids, including the addictive nature of the drugs; and

d. Defendants knew or should have known that their promotion would lead to addiction and other adverse consequences that the larger community would suffer as a result.

158. Defendants' actions were, at the least, a substantial factor in doctors and patients not accurately assessing and weighing the risks and benefits of opioids for chronic pain thereby causing opioids to become widely available and used in Falls County.

159. Without Defendants' actions, opioid use would not have become so widespread and the enormous public health hazard of opioid addiction would not have existed and could have been averted.

160. The health and safety of the citizens of Falls County, including those who use, have used, or will use opioids, as well as those affected by opioid users, is a matter of great public interest and legitimate concern to Falls County's citizens and residents.

161. The public nuisance created, perpetuated, and maintained by Defendants can be abated and further reoccurrence of such harm and inconvenience can be prevented.

162. Defendants' conduct has affected and continues to affect a considerable number of people within Falls County and is likely to continue to cause significant harm to patients who take opioids, their families, and the community at large.

163. Each Defendant created or assisted in creating the opioid epidemic, and each Defendant is jointly and severally liable for its abatement. Furthermore, each Defendant should be enjoined from continuing to create, perpetuate, or maintain said public nuisance in Falls County.

## VII. SECOND CAUSE OF ACTION: COMMON LAW FRAUD AGAINST ALL DEFENDANTS

164. Falls County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

165. At all relevant and material times, Defendants expressly and/or impliedly warranted that opioids were safe, of merchantable quality, and fit for use.

166. Defendants' superior knowledge and expertise, its relationship of trust and confidence with doctors and the public, its specific knowledge regarding the risks and dangers of opioids, and its intentional dissemination of promotional and marketing information about opioids for the purpose of maximizing sales, each gave rise to the affirmative duty to meaningfully disclose and provide all material information about the risks and harms associated with opioids.

167. At all times herein mentioned, Defendants, individually and acting through their employees and agents, and in concert with each other, fraudulently represented to physicians who Defendants knew would justifiably rely on Defendants' representations that opioids were safe and effective for treating chronic pain.

168. Defendants' false representations were fraudulently made, with the intent or purpose that healthcare providers and patients would justifiably rely upon them, leading to the prescription, administration, filling, purchasing, and consumption of opioids in Falls County.

169. Defendants' deliberate misrepresentations and/or concealment, suppression, and omission of material facts as alleged herein include, but are not limited to:

a. Making false and misleading claims regarding the known risks of the addictive nature of opioids and suppressing, failing to disclose, and mischaracterizing the addictive nature of opioids and in concomitant costs, such as overdoses, deaths, and heroin addiction;

b. Making false and misleading written and oral statements that opioids are more effective than traditional pain killers for chronic pain, or effective at all and/or omitting material information showing that opioids are no more effective than other non-addictive drugs for chronic pain;

c. Issuing false and misleading warnings and/or failing to issue adequate warnings concerning the risks and dangers of using opioids;

d. Making false and misleading claims downplaying the risk of addiction when using opioids and/or setting forth guidelines that would purportedly identify addictive behavior; and

e. Making false and misleading misrepresentations concerning the safety, efficacy and benefits of opioids without full and adequate disclosure of the underlying facts which rendered such statements false and misleading.

170. Defendants willfully, wantonly, and recklessly disregarded their duty to provide truthful representations regarding the safety and risk of opioids.

171. Defendants made these misrepresentations with the intent that the healthcare community and patients located wherever these opioid drugs were sold or consumed would rely upon them.

172. Defendants' misrepresentations were made with the intent of defrauding and deceiving the medical community and consumers to induce and encourage the sale of opioids.

173. Defendants' fraudulent representations evidence their callous, reckless, willful, and depraved indifference to the health, safety, and welfare of consumers living in Falls County.

174. Defendants omitted, misrepresented, suppressed and concealed material facts concerning the dangers and risk of injuries associated with the use of opioids, as well as the fact that the product was unreasonably dangerous.

175. Defendants' purpose was willfully blind to, ignored, downplayed, avoided, and/or

otherwise understated the serious nature of the risks associated with the use of opioids.

176. The treating medical community and consumers in Falls County did not know that Defendants' representations were false and/or misleading and justifiably relied on them.

177. Defendants had sole access to material facts concerning the dangers and unreasonable risks of opioids, which they intentionally concealed.

178. As a direct and proximate result of Defendants' fraudulent misrepresentations and intentional concealment of facts, upon which the medical community and consumers in Falls County reasonably relied, Falls County suffered actual and punitive damages.

## VIII. THIRD CAUSE OF ACTION: NEGLIGENCE AGAINST MANUFACTURING AND DISTRIBUTING DEFENDANTS

179. Falls County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

180. Manufacturing Defendants have a duty to exercise reasonable care in marketing its opioids to physicians treating residents of Falls County and Falls County residents. Manufacturing Defendants have breached their duty by knowingly and fraudulently misrepresenting the benefits of, and downplaying the risks of, opioids for chronic pain.

181. Manufacturing Defendants have used deceitful marketing ploys, KOLs, Front Groups, and other schemes to increase profits at the cost of public health causing an opioid epidemic. Manufacturing Defendants have acted willfully, wantonly, and maliciously.

182. Likewise, Distributor Defendants have a duty to exercise ordinary care in distributing opioids. Distributor Defendants have breached their duty by failing to prevent or reduce the distribution of opioids, or to report the increase in the distribution and/or sale of opioids.

183. Distributor Defendants have intentionally failed to prevent or reduce the distribution of opioids, or to report any increases in the sale of opioids, so that they could increase profits and receive rebates or kick-backs from Manufacturing Defendants. Distributor Defendants have acted

willfully, wantonly, and maliciously.

184. As a proximate result, Manufacturing and Distributor Defendants and its agents have caused Falls County to incur excessive costs to treat the opioid epidemic in its county, including but not limited to increased costs of social services, health systems, law enforcement, judicial system, and treatment facilities.

185. Falls County and its residents are therefore entitled to actual and punitive damages.

## IX. FOURTH CAUSE OF ACTION: GROSS NEGLIGENCE AGAINST MANUFACTURING AND DISTRIBUTING DEFENDANTS

186. Falls County incorporates the allegations within all prior paragraphs within this Complaint as if they were fully set forth herein.

187. Defendants' marketing scheme to optimize profits by misrepresenting and falsely touting opioids as the panacea to chronic pain was done intentionally.

188. Defendants' hiring of KOLs, Front Groups, and others to spread its fraudulent message that opioids were useful and beneficial for chronic pain was grossly negligent and done with conscious indifference or reckless disregard for the safety of others.

189. Each Defendant's actions and omissions as described herein, singularly or in combination with each other, was malicious resulting in damages and injuries to Falls County and its residents.

190. At every stage, Defendants knew or should have known that their conduct would create an unreasonable risk of physical harm to others, including Falls County and its residents, and should be held liable in punitive and exemplary damages to Falls County.

## X. FIFTH CAUSE OF ACTION: TEXAS CONTROLLED SUBSTANCES ACT ("TCSA") AGAINST DISTRIBUTOR DEFENDANTS

191. Falls County re-alleges and incorporates by reference each of the allegations

contained in the preceding paragraphs of this Complaint as though fully alleged herein.

192. Distributor Defendants have knowingly distributed, delivered, administered, or dispensed a controlled substance in violation of the Texas Controlled Substances Act §481.128(a)(1) by deceiving practitioners into prescribing, dispensing, delivering, or administering a controlled substance, or causing a controlled substance to be administered when there is no valid medical purpose. Tex. Health & Safety Code §481.071.

193. As alleged herein, each Distributor Defendant, at all times relevant to this Complaint, violated the Texas Controlled Substance Act by making deceptive representations about using opioids to treat chronic pain. Each Distributor Defendant also omitted or concealed material facts and failed to correct prior misrepresentations and omissions about the risks and benefits of opioids. Each Distributor Defendant's omissions rendered even their seemingly truthful statements about opioids deceptive.

194. Distributor Defendants' deceptive representations and concealments were reasonably calculated to deceive practitioners treating Falls County residents into prescribing opioids without any valid medical purpose, and Distributor Defendants continue to do so to this day.

195. As a direct and proximate cause of Distributor Defendants' conduct, Falls County should be awarded civil penalties pursuant to the Texas Controlled Substances Act.

## XI. SIXTH CAUSE OF ACTION: UNJUST ENRICHMENT AGAINST ALL DEFENDANTS

196. Falls County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

197. As an expected and intended result of their conscious wrongdoing as set forth in this Complaint, Defendants have profited and benefited from opioid purchases made by Falls

County and its residents.

198. When Falls County and its residents purchased opioids, they expected that Defendants had provided necessary and accurate information regarding those risks. Instead, Defendants had misrepresented the material facts regarding the risks and benefits of opioids.

199. Defendants have been unjustly enriched at the expense of Falls County, and Falls County is therefore entitled to damages to be determined by the jury.

## PRAYER FOR RELIEF

WHEREFORE, Plaintiff respectfully prays:

a. That the acts alleged herein be adjudged and decreed to be unlawful and that the Court enter a judgment declaring them to be so;

b. That Defendants be enjoined from, directly or indirectly through KOLs, Front Groups or other third parties, continuing to misrepresent the risks and benefits of the use of opioids for chronic pain, and from continuing to violate Texas law;

c. That Plaintiff recover all measures of damages, including punitive and exemplary damages, allowable under the law, and that judgment be entered against Defendants in favor of Plaintiff;

d. That Plaintiff recover restitution on behalf of Falls County consumers who paid for opioids for chronic pain;

e. That Plaintiff recover the costs and expenses of suit, pre- and post-judgment interest, and reasonable attorneys' fees as provided by law; and

f. That Defendants be ordered to abate the public nuisance that they created in in violation of Texas common law.

Date: January 4, 2018

Respectfully Submitted,

Falls County Attorney's Office

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# APPENDIX F

## HOPKINS THIRD AMENDED PETITION

**CAUSE NO. CV43486**

| | | |
|---|---|---|
| **COUNTY OF HOPKINS,** | § | **IN THE DISTRICT COURT** |
| | § | |
| *Plaintiff,* | § | |
| | § | |
| vs. | § | **62nd JUDICIAL DISTRICT** |
| | § | |
| **PURDUE PHARMA, L.P.**; | § | |
| **PURDUE PHARMA, INC.**; | § | |
| **THE PURDUE FREDERICK COMPANY**; | § | |
| **JOHNSON & JOHNSON**; | § | **HOPKINS COUNTY, TEXAS** |
| **JANSSEN PHARMACEUTICALS, INC.**; | § | |
| **ORTHO-MCNEIL-JANSSEN** | § | |
| **PHARMACEUTICALS, INC.** n/k/a | § | |
| JANSSEN PHARMACEUTICALS, INC.; | § | |
| **JANSSEN PHARMACEUTICA, INC.** n/k/a | § | |
| JANSSEN PHARMACEUTICALS, INC.; | § | |
| **ENDO HEALTH SOLUTIONS INC.**; | § | |
| **ENDO PHARMACEUTICALS, INC.**; | § | |
| **ALLERGAN PLC** f/k/a ACTAVIS PLC; | § | |
| **ALLERGAN FINANCE LLC** f/k/a | § | |
| ACTAVIS, INC. f/k/a WATSON | § | |
| PHARMACEUTICALS, INC.; | § | |
| **MCKESSON CORPORATION**; | § | |
| **CARDINAL HEALTH, INC.**; | § | |
| **AMERISOURCEBERGEN** | § | |
| **CORPORATION**; | § | |
| **HOWARD GREGG DIAMOND, M.D.**; | § | |
| **CARDINAL HEALTH 110, LLC**, a wholly- | § | |
| owned subsidiary of CARDINAL HEALTH, | § | |
| INC.; | § | |
| **AMERISOURCEBERGEN DRUG** | § | |
| **CORPORATION**, a wholly-owned subsidiary | § | |
| of AMERISOURCEBERGEN | § | |
| CORPORATION; and | § | |
| **DOES 1 – 100, INCLUSIVE**, | § | |
| | § | |
| *Defendants.* | § | |

**PLAINTIFF'S THIRD AMENDED PETITION AND JURY DEMAND**

**TO THE HONORABLE JUDGE OF SAID COURT:**

Plaintiff, the County of Hopkins, Texas, by and through the undersigned attorneys, (hereinafter "Hopkins County" or "County"), on behalf of the County Attorney for Hopkins County, against Defendants Purdue Pharma L.P., Purdue Pharma Inc., The Purdue Frederick Company, Johnson & Johnson, Janssen Pharmaceuticals, Inc., Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc., Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc., Endo Health Solutions Inc., Endo Pharmaceuticals, Inc., Allergan PLC f/k/a Actavis PLC, Allergan Finance, LLC f/k/a Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc., McKesson Corporation, Cardinal Health, Inc., AmerisourceBergen Corporation, Howard Gregg Diamond, M.D., Cardinal Health 110, LLC, a wholly-owned subsidiary of Cardinal Health, Inc., AmerisourceBergen Drug Corporation, a wholly-owned subsidiary of AmerisourceBergen Corporation, and Does 1 – 100, alleges as follows:

## I. INTRODUCTION

1. The United States is in the midst of an opioid epidemic caused by Defendants' fraudulent marketing, sales, and distribution of prescription opioids ("opioids") that has resulted in addiction, criminal activity, and loss of life.[1] Americans "consume 85% of all the opioids in the world" and are "the most medicated country in the world . . . ."[2] The opioid crisis has been described as "the AIDS epidemic of our generation, but even worse."[3] On October 26, 2017, President Donald Trump "declared a nationwide public health emergency to combat the opioid

[1] L. Manchikanti, *Opioid Epidemic in the United States*, Pain Physician, Jul. 2012, at 1, www.painphysicianjournal.org, attached hereto as Exhibit A.
[2] David Wright, *Christie on Opioids: "This is the AIDS Epidemic of Our Generation, but even Worse,"* CNN, Oct. 27, 2017, *available at* http://www.cnn.com/2017/10/27/politics/chris-christie-opioid-commission-aids-cnntv/index.html; Manchikanti, Ex. A, at 16 ("Gram for gram, people in the United States consume more narcotic medication than any other nation worldwide.").
[3] Wright, *supra*.

crisis."[4]

2. In 1997, each person in the United States, on average, consumed 96 mg morphine equivalents. In 2010 that number increased to 710 mg per person.[5] This amount has been estimated as the equivalent to 7.1 kg of opioids per 10,000 people – or enough to supply each American with 5 mg of hydrocodone every 6 hours for 45 days.[6]

3. It's no surprise that in 2016 alone, health care providers wrote more than 289 million prescriptions for opioids, enough for *every adult in the United States* to have more than one bottle of pills.[7] The prescription rate in Hopkins County is substantially higher than the national average.[8]

4. Unfortunately, using opioids too often leads to addiction and overdose from opioids. It was estimated as early as 2001 that up to 40% of chronic pain patients were addicted to opioid pain medication.[9] Almost 2 million Americans were addicted to opioids in 2014.[10] To put the opioid crisis in perspective, the statistics demonstrate:

- Roughly 21 to 29 percent of patients prescribed opioids for chronic pain misuse them;
- Between 8 and 12 percent develop an opioid use disorder; and
- About 80 percent of people who use heroin first misused prescription opioids.[11]

5. In 2014, more people died from drug overdoses than in any other year, and most overdose deaths involved an opioid. Currently more than 90 Americans die every day after

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[4] Dan Merica, *What Trump's Opioid Announcement Means – and Doesn't Mean*, CNN, Oct. 26, 2017, *available at* http://www.cnn.com/2017/10/26/politics/national-health-emergency-national-disaster/index.html.
[5] Manchikanti, Ex. A, at 14.
[6] *Id.*
[7] *Prevalence of Opioid Misuse*, BupPractice, Sept. 7, 2017, *available at* https://www.buppractice.com/node/15576.
[8] Center for Disease Control, *available at* https://www.cdc.gov/drugoverdose/maps/rxrate-maps.html.
[9] *Prescription Drugs: Abuse and Addiction*, National Institute of Drug Abuse (NIH Publication), Jul. 2001, at 13.
[10] *National Survey on Drug Use and Health*, Substance Abuse and Mental Health Services Administration, 2014.
[11] *Opioid Overdose Crisis*, National Institute on Drug Abuse, Jan. 2018, *available at* https://www.drugabuse.gov/drugs-abuse/opioids/opioid-overdose-crisis.

overdosing on opioids.[12] The Texas Legislature has found "that deaths resulting from the use of opioids and other controlled substances constitute a public health crisis."[13]

6. In fact, accidental drug overdose deaths, of which reportedly at least two-thirds are opioid overdoses, are the leading cause of death for Americans under the age of 50. Accidental drug overdose deaths, predominantly from opioids, exceed the number of deaths caused by cars or guns. A report from the CDC found that from July 2016 to September 2017, emergency visits due to suspected opioid overdoses continued to climb approximately 30% across the nation.[14] The increase was seen in adults of all age groups and in men and women in all geographic areas.[15]

7. Over the next decade, the average number of deaths due to opioids is expected to be 500,000.[16] The economic burden caused by opioid abuse in the United States is at least $78.5 billion,[17] including lost productivity and increased social services, health insurance costs, increased criminal justice presence and strain on judicial resources, and substance abuse treatment and rehabilitation.[18] In 2015, Texas "had the second highest total healthcare costs from opioid abuse in the nation ($1.96 billion) . . . ."[19]

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[12] *Id*.
[13] Tex. Att'y Gen. Op. No. KP-0168 (2017), *citing* Act of May 26, 2017, 85th Leg., R.S., ch. 534, § 3, 2017 Tex. Sess. Law Serv. 1467, 1468.
[14] Jacqueline Howard, *ER Visits for Opioid Overdose up 30%, CDC Study Finds*, CNN, Mar. 6, 2018.
[15] *Id.*
[16] Max Blau, *STAT forecast: Opioids Could Kill Nearly 500,000 American in the next Decade*, STAT, June 27, 2017, *available at* https://www.statnews.com/2017/06/27/opioid-deaths-forecast/; *see also* Wes Rapaport, *Advocates for Painkiller Advocates Wants Society to Meet Them Halfway*, Big Country, Feb. 18, 2018 (stating the number of opioid overdose deaths is going to go up for at least several more years and explaining how Operation Naloxone has administered more than $1 million of the powerful antidote).
[17] *CDC Foundation's New Business Pulse Focuses on Opioid Overdose Epidemic*, Centers for Disease Control and Prevention, Mar. 15, 2017, *available at* https://www.cdc.gov/media/releases/2017/a0315-business-pulse-opioids.html.
[18] *Opioid Overdose Crisis*, *supra.*
[19] Kerry Craig, *Opioid Addiction Results in one Woman's Daily Struggle*, Sulphur Springs News-Telegram, Oct. 7, 2017, *available at* https://www.ssnewstelegram.com/news/opioid-addiction-results-in-one-woman-s-daily-struggle/article_bded4eoa-ab80-11e7-a252-d3f304e26628.html.

8. This epidemic did not occur by chance. Defendants manufacture, market, distribute, and sell prescription opioids, including, but not limited to, brand-name drugs like OxyContin, Vicodin, Opana, Percocet, Percodan, Duragesic, Ultram, Ultracet, and generics like oxycodone, oxymorphone, hydromorphone, hydrocodone, fentanyl, and tramadol, which are powerful narcotics.

9. Historically, opioids were considered too addictive and debilitating for treating non-cancer chronic pain,[20] such as back pain, migraines, and arthritis, and were used only to treat short-term acute pain or for palliative or end-of-life care.

10. By the late 1990s or early 2000s, however, each Manufacturing Defendant began a marketing scheme to persuade doctors and patients that opioids were not addictive and should be used ubiquitously and perpetually to treat moderate, non-cancer chronic pain.[21] Defendants' efforts to "increase opioid use" and their campaign emphasizing "the alleged undertreatment of pain continue to be significant factors of the [opioid] escalation."[22] Defendants reassured the medical community that opioids were not addictive, and doctors prescribed them at a higher rate.[23] Consequently, the National Institute of Drug Abuse attributes the opioid crises to Defendants' successful marketing campaign.[24] Each Manufacturing Defendant spent, and continues to spend large sums of money to promote the benefits of opioids for non-cancer moderate pain while trivializing or even denying their risks.

[20] "Chronic pain" means non-cancer pain lasting three months or longer.
[21] *See e.g., Opioid Overdose Crisis*, National Institute on Drug Abuse, Jan. 2018, *available at* https://www.drugabuse.gov/drugs-abuse/opioids/opioid-overdose-crisis (explaining the greater rate of prescribing opioids due to misinformation to physicians, which led to a diversion and misuse of opioids before anyone knew opioids were highly addictive).
[22] Manchikanti, Ex. A, at 1.
[23] CDC/NCHS, *National Vital Statistics System, Mortality*, CDC Wonder, Atlanta, Ga: US Department of Health and Human Services, 2017, *available at https://wonder.cdc.gov.*
[24] *See id.*

11. The Manufacturing Defendants' promotional messages deviated substantially from any approved labeling of the drugs and caused prescribing physicians and consuming patients to underappreciate the health risks, and to overestimate the benefits of opioids.

12. Contrary to the language of their drugs' labels, Defendants falsely and misleadingly in their marketing: (1) downplayed the serious risk of addiction; (2) promoted and exaggerated the concept of "pseudoaddiction" thereby advocating that the signs of addiction should be treated with more opioids; (3) exaggerated the effectiveness of screening tools in preventing addiction; (4) claimed that opioid dependence and withdrawal are easily managed; (5) denied the risks of higher opioid dosages; and (6) exaggerated the effectiveness of "abuse-deterrent" opioid formulations to prevent abuse and addiction.

13. Manufacturing Defendants disseminated these falsehoods through ads and/or their sales representatives and hand-picked physicians who supported Defendants' message. Sales representatives, working at Manufacturing Defendants' behest, promoted highly addictive opioids through souvenirs and toys including, but not limited to, opioid brand-bearing stuffed plush toys, dolls, coffee cups, fanny packs, water bottles, notepads, pens, refrigerator magnets, clocks, letter openers, rulers, daytime planners, bags, puzzles, posters, hand-held calculators, clipboards, highlighters, flashlights, key chains, clothing, reflex mallets, and mock-ups of the United States Constitution.

14. Defendants also used third parties they controlled by: (a) funding, assisting, encouraging, and directing doctors, known as "key opinion leaders" ("KOLs") and (b) funding, assisting, directing, and encouraging seemingly neutral and credible professional societies and patient advocacy groups (referred to hereinafter as "Front Groups").

15. Manufacturing Defendants worked with KOLs and Front Groups to taint the sources that doctors and patients relied on for ostensibly "neutral" guidance, such as treatment guidelines, Continuing Medical Education ("CME") programs, medical conferences and seminars, and scientific articles. Through their individual and concerted efforts, Defendants convinced doctors that, instead of being addictive and unsafe for long-term use in most circumstances, opioids were *required* for the compassionate treatment of chronic pain, which Defendants termed an epidemic in America.

16. Manufacturing Defendants' aggressive marketing of opioids for chronic pain is "based on unsound science and blatant misinformation, and accompanied by the dangerous assumptions that opioids are highly effective and safe, and devoid of adverse events when prescribed by physicians."[25] Nevertheless, Defendants' marketing was effective and by 2011, there were 136.7 million prescriptions for hydrocodone alone, with all opioids exceeding 238 million.[26] Data demonstrates that "[o]ver 90% of patients received opioids for chronic pain management."[27]

17. Essentially each Defendant ignored science and consumer health for profits. Defendants' efforts were so successful that opioids are now the most prescribed class of drugs generating $11 billion in revenue for drug companies in 2014 alone. Sales for Purdue's OxyContin grew from $48 million in 1996 to $1.1 billion in 2000 after it successfully and aggressively marketed and promoted its opioid.[28] In fact, OxyContin was a "leading drug of abuse" by 2004 through its availability.[29] Even after Purdue reached a $600 million federal settlement in 2007, the

[25] Manchikanti, Ex. A, at 1-4.
[26] *Id.*
[27] *Id.* at 19.
[28] Art Van Zee, M.D., *The Promotion and Marketing of OxyContin: Commercial Triumph, Public Health Tragedy*, 99 Am. J. Public Health 221, Feb. 2009, at 1, attached hereto as Exhibit B.
[29] Zee, *supra*.

settlement failed to impact what is a "$13-billion-a-year opioid industry."[30]

18. As a direct and foreseeable consequence of Manufacturing Defendants' misrepresentations and misleading marketing campaign to Hopkins County physicians and residents regarding the safety and efficacy of using opioids for chronic non-cancer pain, Hopkins County has spent and continues to spend large sums of money combatting the public health crisis.

19. The Distributor Defendants were not standing by idly while Marketing Defendants were peddling their opioids to physicians and consumers. Cardinal, AmerisourceBergen, and McKesson ("Distributor Defendants") are three of the largest opioid distributors in the United States. Distributor Defendants purchased opioids from Manufacturing Defendants herein and sold them to pharmacies throughout Hopkins County. Distributor Defendants function as "trusted partners" with Manufacturing Defendants in maximizing market share and success of pharmaceutical products. AmerisourceBergen states on its website that it is "[a] trusted partner [to manufacturers] in the commercialization process" and works with manufacturers "to optimize each stage—and each decision—along the product lifecycle."[31] McKesson claims that it works in partnership with manufacturers "to develop, market, and deliver pharmaceuticals to patients."[32] In fact, McKesson claims that its "health care informatics expertise" allows it to provide manufacturers with "painstaking market research" to help manufacturers develop and refine their "product launch and market penetration strategy."[33] Cardinal Heath likewise advertises itself as a

[30] Rebecca L. Haffajee, J.D., Ph.D., M.P.H. and Michelle M. Mello, J.D., Ph.D., *Drug Companies' Liability for the Opioid Epidemic*, N. Engl. J. Med., Dec. 14, 2017, at 2305.
[31] AmerisourceBergen, "Brand and Specialty Manufacturer Solutions," https://www.amerisourcebergen.com/abcnew/solutions-manufacturers/brand-and-specialty.
[32] McKesson, "Pharmaceutical Manufacturers: Better Partnerships, Better Heath," http://www.mckesson.com/pharmaceutical-manufacturers/.
[33] McKesson, "Health Care Informatics for Manufacturers," http://www.mckesson.com/manufacturers/health-care-informatics/.

"consultative partner" to pharmaceutical manufacturers.[34]

20. Distributor Defendants promote themselves as partners and resources for pharmaceutical manufacturers in "pharmacovigilance," representing that they can "detect, assess, and monitor[ ] therapies throughout the patient journey."[35] Distributor Defendants like Cardinal Health provide Risk Evaluation and Mitigation Strategy (REMS) programs that "design[ ] and maintain[ ] registries of prescribers, pharmacies, distributors, and patients" and "assess[ whether] patients are receiving and understanding medication guides."[36] Cardinal Health even advertises that it can help pharmaceutical manufacturers "ensure the highest level of patient touch by providing high-quality clinical services during therapy."[37] These Risk Evaluation and Mitigation Strategies require, among other things, that the Distributor Defendants provide through the chain of distribution a plain-English medication guide delivered to each patient that describes the serious risks of taking the prescription drug. This duty of the Distributor Defendants is not mitigated by the prescribing doctor.

21. Distributor Defendants have acknowledged and claim to have undertaken a duty to prevent prescription drug diversion and abuse based on their unique role in the opioid supply chain.[38] Distributor Defendants laud on their website their ability to detect and prevent prescription

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[34] Cardinal Health, *Pharmaceutical Manufacturer*, http://www.cardinalhealth.com/en/services/manufacturer/pharma-manufacturer.html.

[35] Cardinal Health, *Pharmacovigilance and Medical Information*, http://www.cardinalhealth.com/en/services/manufacturer/pharma-manufacturer/cardinal-health-specialty-solutions/patient-solutions/access-and-patient-support/access-and-adherence/pharmacovigilance.html.

[36] Cardinal Health, *Risk Evaluation and Mitigation Strategy: Ensure the Specific Safety Needs of Your Therapy Are Met,* http://www.cardinalhealth.com/en/services/manufacturer/pharma-manufacturer/cardinal-health-specialty-solutions/patient-solutions/access-and-patient-support/access-and-adherence/rems.html.

[37] Cardinal Health, *Patient Centric Clinical Solutions*, http://www.cardinalhealth.com/en/services/manufacturer/pharma-manufacturer/cardinal-health-specialty-solutions/patient-solutions/access-and-patient-support/access-and-adherence/clinical-services.html.

[38] Charles Ornstein, *Drug Distributors Penalized for Turning Blind Eye in Opioid Epidemic*, APR, Health News, Jan. 27, 2017; Letter from Pete Slone, Senior Vice President, Public Affairs, of McKesson, to The Honorable Chris Christie dated October 31, 2017; AmerisourceBergen Foundation, *AmerisourceBergen Foundation Launches Municipal Support Program to Help Combat Opioid Abuse*, Dec. 14, 2017 press release; Cardinal Health, *Cardinal Health's*

drug diversion to improper purposes. AmerisourceBergen claims it uses "complex algorithms [that] identify and stop orders that are deemed to be suspicious."[39] And Cardinal Health claims it uses a "state-of-the-art, constantly adaptive system to combat opioid diversion."[40]

22. Despite the alarming and suspicious rise in the number of opioids ordered by retailers in Hopkins County and Distributor Defendants self-claimed duty to stop suspicious opioid orders, Distributor Defendants simply continued to flood of opioids into the County. In continuing to oversupply opioids in Hopkins County, Distributor Defendants put their partnership with pharmaceutical manufacturers—to increase market penetration—above their obligations to secure the opioid supply claim. Manufacturing Defendants and Distributor Defendants worked hand and glove to glut Hopkins County with more opioids than could possibly be consumed for therapeutic purposes, resulting in an opioid prescription rate in Hopkins County that remains well above the already insupportable national rate. Each Defendant disregarded its legal duty to ensure that not only that opioids were safe and effective, but that they were being prescribed for a valid medical purpose.

23. As a direct and foreseeable consequence of Distributor Defendants' failure to act as the gatekeeper and distributing opioids even though suspicion for diversionary purposes existed, Hopkins County has spent and continues to spend large sums of money combatting the public health crisis.

24. The money Hopkins County has spent comes directly from its taxpayers. These taxpayers include Hopkins County physicians, who passed on Defendants' misleading safety and efficacy information and prescribed more opioids to taxpaying residents in Hopkins County.

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*Commitment to Opioid Anti-Diversion, Education and Misuse Prevention,* www.cardinalhealth.com; Cardinal Health, *No Demographic Group, supra.*.

[39] *Id.*

[40] Cardinal Health, *Opioid Action Program: Reclaiming our Communities,* www.cardinalhealth.com.

These taxpayers necessarily included Hopkins County residents who either suffered the addictive effects of consuming opioids or overdosed using Defendants' opioids that had been over-prescribed and over-supplied to Hopkins County as intended be Defendants herein. Thus, this group of Hopkins County residents has suffered not only injury to property, but also bodily injury, as a result of Defendants' misconduct in the false promotion and/or over-supply of prescription opioids.

25. Hopkins County has spent and continues to spend large sums of money combatting the opioid crisis created by Defendants' negligent and fraudulent marketing campaign. Across the country, including Texas, increased opioid prescribing has caused and continues to cause an increase in overdoses and death. Defendants tracked the CDC data and knew that the more they promoted opioid prescribing and distributed more opioids that non-therapeutic outcomes, such as overdose, addiction, and criminality (e.g. pill mills like those of Dr. Diamond herein) would occur. By 2010, enough opioids had been sold to medicate every American adult with a typical dose of 5 mg of hydrocodone every 4 hours for 1 month.[41] The increased use of opioids has contributed to the increased rate of overdose deaths and nonmedical use with the varying rates of sales in each states impacting the outcomes in each state.[42] "Given that 3% of physicians accounted for 62% of the [opioids] prescribed in one study, the proliferation of high-volume prescribers can have a large impact on state use of [opioids] and overdose death rates."[43] Not surprisingly, "[l]arge increases in overdoses involving the types of drugs sold by illegitimate pain clinics (i.e., 'pill mills') have been reported in Florida and Texas."[44] In Hopkins County, the prescription rate per 100 people in

[41] Center for Disease Control, *Vital Signs: Overdoses of Prescription Opioid Pain Relievers – United States, 1999-2008*, Morbidity and Mortality Weekly Report (MMWR), Nov. 4, 2011.
[42] *Id.*
[43] *Id.*
[44] *Id.*

2014 was 112.6, which means that every man, woman, and child in Hopkins County could have had at least one bottle of opioids.[45] By 2016, even though the average prescription rate was a little under 67 per 100, the prescription rate in Hopkins County per 100 people remained at 92.7.[46] Essentially, Hopkins County's prescription rate has been twice that of the national average. In 2015 there were 12.1 to 14 deaths per 100,000 people reported from drug overdoses.[47] A substantial number of those overdose deaths were a result, in whole or in part, of opioid ingestion. Defendants' marketing misconduct, as well as Defendants' efforts to sell more prescription opioids than can be consumed therapeutically, were natural and foreseeable causes of overdose deaths and injuries in Hopkins County.

26. As a direct and foreseeable consequence of Defendants' conduct described regarding prescription opioids, Hopkins County has committed and continues to commit resources to provide and pay additional health care, law enforcement, social services, public assistance, pharmaceutical care and other services necessary for its residents.

## II. RULE 47 STATEMENT OF MONETARY RELIEF SOUGHT

27. Per Rule 47 of the Texas Rules of Civil Procedure, the County states that although the full measure of its damages is still being calculated, its damages caused by Defendants' acts and omissions exceed $1,000,000 but are believed to be less than $50,000,000. Accordingly, at this time in the litigation, Hopkins County states that it is seeking monetary relief for an amount greater than $1,000,000 and less than $50,000,000, the rightful and just amount to be determined by the jury.

## III. STANDING

28. Hopkins County has standing to bring this lawsuit because it has suffered an injury-

[45] Center for Disease Control, *available at* https://www.cdc.gov/drugoverdose/maps/rxrate-maps.html.
[46] *Id.*
[47] Center for Disease Control, *available at* https://www.cdc.gov/nchs/data-visualization/drug-poisoning-mortality.

in-fact caused by Defendants' misconduct, and that harm can be redressed through this action. Having decided that it was necessary to pursue these claims to protect the County's interests, the County hired outside counsel to handle the litigation.[48] The contract governing the County's representation in this litigation was approved by the Texas Comptroller of Public Accounts pursuant to Tex. Gov't Code § 403.0305.[49]

29. Defendants' misconduct has placed an unreasonable burden on Hopkins County's resources and ability to provide the public services and employee benefits it is obligated to and/or has authority to provide to its residents and employees. Hopkins County has the statutory duty and/or authority to provide public safety and health services, including, but not limited to, the following:

- Supporting paupers;[50]
- Providing county jails;[51]
- Providing health care in county jails;[52]
- Providing fire protection;[53]
- Enforcing drug laws;[54]
- Contracting with drug centers;[55]
- Commissioning drug education and counseling programs;[56] and
- Paying county and precinct officers and employee compensation, office and

[48] *See* Resolution for approval of bringing suit on behalf of Hopkins County, Texas, vs. various drug manufacturers, developers, suppliers and others of a class of pharmaceutical class of drugs commonly referred to as opioids and approval of Professional Services Agreement for Special Counsel attached hereto as Exhibit G and Executed and Approved Retention Agreement attached hereto as Exhibit H.
[49] *Id.*
[50] Tex. Local Gov't Code § 81.027.
[51] *Id.* at § 351.001.
[52] *Id.* at § 351.045.
[53] *Id.* at § 352.001.
[54] *Id.* at § 370.003.
[55] Tex. Health & Safety Code at § 464.032.
[56] *Id.* at § 465.001

travel expenses, and any other allowances.[57]

30. Defendants' misconduct—including Manufacturing Defendants' calculated marketing campaign of misinformation to physicians and patients, and Distributor Defendants' disbursement and distribution of prescription opioids even though suspicion for diversionary purposes existed—caused the damages to the County. They misled physicians into overprescribing opioids, which directly created the need for dramatically increased public services. The County relied on these misrepresentations in paying for its employees' healthcare costs causing the County to incur increased healthcare costs for its own employees.

31. The harm caused by Defendants' misconduct can be redressed by the Court in this action. Defendants should be enjoined from continuing to manufacture, distribute, and sell opioids in Hopkins County without a medical purpose and without educating physicians and patients about the actual risks and benefits of its drugs. Furthermore, Defendants should compensate Hopkins County for the funds it has expended and continues to expend for medical insurance claims for opioids that were not medically necessary, as well as increased costs of social services, health systems, law enforcement, the judicial system, and treatment facilities.

## IV. VENUE AND JURISDICTION

32. Venue is proper in Hopkins County because all or a substantial part of the events or omissions giving rise to this claim occurred in Hopkins County. TEX. CIV. PRAC. & REM. CODE §15.002(a)(2). This Court has subject-matter jurisdiction over this matter because Plaintiff's damages are in excess of the minimal jurisdictional limits of this Court. TEX. GOVT. CODE §24.007(b).

33. This Court has general jurisdiction over Dr. Diamond as he is a Texas resident. This Court also has specific jurisdiction over all Defendants as their activities were directed toward

[57] Tex. Local Gov't Code § 152.011.

Texas, and injuries complained of herein resulted from their activities. *Guardian Royal Exchange Assur., Ltd. v. English China Clays, P.L.C.*, 815 S.W.2d 223, 227 (Tex. 1991). Each Defendant has a substantial connection with Texas and the requisite minimum contacts with Texas necessary to constitutionally permit the Court to exercise jurisdiction. *See id.* at 226.

## V. PARTIES

### A. Plaintiff

34. This action is brought for and on behalf of Hopkins County, which provides a wide range of services on behalf of its residents, including services for families and children, public health, public assistance, law enforcement, and emergency care.

### B. Defendants

35. PURDUE PHARMA L.P. is a limited partnership organized under the laws of Delaware, and has appeared and/or answered herein. PURDUE PHARMA L.P. is, through its ownership structure, a Texas resident. PURDUE PHARMA INC. is a New York corporation with its principal place of business in Stamford, Connecticut, and has appeared and/or answered herein. THE PURDUE FREDERICK COMPANY is a Delaware corporation with its principal place of business in Stamford, Connecticut, and has appeared and/or answered herein (Purdue Pharma L.P., Purdue Pharma Inc., and The Purdue Frederick Company are hereinafter referred to as "Purdue").

36. Purdue manufactures, promotes, sells, and distributes opioids in the U.S. and Hopkins County. Purdue's opioid drug, OxyContin, is one of the most addictive and abused prescription drugs in American history. Purdue promotes opioids throughout the United States and in Hopkins County.

37. JANSSEN PHARMACEUTICALS, INC. is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey, and has appeared and/or answered herein.

JANSSEN PHARMACEUTICALS, INC. is a wholly owned subsidiary of JOHNSON & JOHNSON. JOHNSON & JOHNSON ("J&J") is a New Jersey corporation with its principal place of business in New Brunswick, New Jersey, and has appeared and/or answered herein. ORTHO-MCNEIL-JANSSEN PHARMACEUTICALS, INC., now known as JANSSEN PHARMACEUTICALS, INC., is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey, and has appeared and/or answered herein. JANSSEN PHARMACEUTICA INC., now known as JANSSEN PHARMACEUTICALS, INC., is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey, and has appeared and/or answered herein. J&J is the only company that owns more than 10% of Janssen Pharmaceuticals' stock, and corresponds with the FDA regarding Janssen's products. Upon information and belief, J&J controls the sale and development of Janssen Pharmaceuticals' drugs and Janssen's profits inure to J&J's benefit (Janssen Pharmaceuticals, Inc., Ortho-McNeil-Janssen Pharmaceuticals, Inc., Janssen Pharmaceutica, Inc., and J&J are hereinafter referred to as "Janssen").

38. Janssen manufactures, promotes, sells, and distributes opioids in the U.S. and in Hopkins County.

39. ENDO HEALTH SOLUTIONS INC. is a Delaware corporation with its principal place of business in Malvern, Pennsylvania, and has appeared and/or answered herein. ENDO PHARMACEUTICALS, INC., a wholly-owned subsidiary of ENDO HEALTH SOLUTIONS INC., is a Delaware corporation with its principal place of business in Malvern, Pennsylvania, and has appeared and/or answered herein (Endo Health Solutions Inc. and Endo Pharmaceuticals, Inc. are hereinafter referred to as "Endo").

40. Endo develops, markets, and sells opioid drugs in the U.S. and in Hopkins County. Endo also manufactures and sells generic opioids in the U.S. and Hopkins County, by itself and through its subsidiary, Qualitest Pharmaceuticals, Inc.

41. ALLERGAN PLC is a public limited company incorporated in Ireland with its principal place of business in Dublin, Ireland. ACTAVIS PLC acquired Allergan plc in March 2015, and the combined company changed its name to Allergan plc in January 2013. Before that, WATSON PHARMACEUTICALS, INC. acquired ACTAVIS, INC. in October 2012, and the combined company changed its name to ALLERGAN FINANCE, LLC as of October 2013. ALLERGAN FINANCE, LLC is a Nevada Corporation with its principal place of business in Parsippany, New Jersey, and has appeared and/or answered herein. Each of these Defendants is owned by Allergan plc, which uses them to market and sell its drugs in the United States.

42. Upon information and belief, Allergan plc exercises control over these marketing and sales efforts and profits from the sale of Allergan/Actavis products ultimately inure to its benefit. (Allergan plc f/k/a Actavis plc and Allergan Finance, LLC f/k/a Watson Pharmaceuticals, Inc. are hereinafter referred to as "Actavis"). Actavis acquired the rights to Kadian from King Pharmaceuticals, Inc. on December 30, 2008 and began marketing Kadian in 2009.

43. Actavis manufactures, promotes, sells, and distributes opioids in the U.S. and in Hopkins County.

44. MCKESSON CORPORATION ("McKesson") is a Delaware corporation with its principal place of business in San Francisco, California, and has appeared and/or answered herein. Upon information and belief, McKesson is a pharmaceutical distributor licensed to do business in Texas. McKesson distributes pharmaceuticals to retail pharmacies and institutional providers in all 50 states, including Texas and Hopkins County.

45. CARDINAL HEALTH, INC. is an Ohio Corporation with its principal place of business in Dublin, Ohio, and has appeared and/or answered herein. CARDINAL HEALTH 110, LLC, a wholly-owned subsidiary of CARDINAL HEALTH, INC., is a Delaware Corporation with its principal place of business in Dublin, Ohio, and has appeared and/or answered herein (Cardinal Health, Inc. and Cardinal Health 110, LLC are hereinafter referred to as "Cardinal"). Cardinal does substantial business in Texas and, upon information and belief, Cardinal is a pharmaceutical distributor licensed to do business in Texas. Cardinal distributes pharmaceuticals to retail pharmacies and institutional providers to customers in all 50 states, including Texas and Hopkins County.

46. AMERISOURCEBERGEN CORPORATION is a Delaware Corporation with its principal place of business in Chesterbrook, Pennsylvania, and has appeared and/or answered herein. AMERISOURCEBERGEN DRUG CORPORATION is a Delaware Corporation with its principal place of business in Conshohocken, Pennsylvania, and has appeared and/or answered herein (AmerisourceBergen Corporation and AmerisourceBergen Drug Corporation are hereinafter referred to as "Amerisource"). Amerisource does substantial business in Texas and, upon information and belief, Amerisource is a pharmaceutical distributor licensed to do business in Texas. Amerisource distributes pharmaceuticals to retail pharmacies and institutional providers to customers in all 50 states, including Texas and Hopkins County.

47. HOWARD GREGG DIAMOND, M.D. is an individual residing in Sherman, Grayson County, Texas, and was served with a copy of Plaintiff's Original Complaint and Citation on October 31, 2017. Dr. Diamond, who ran Diamondback Pain and Wellness Center in Sherman,

Grayson County, Texas overprescribed fentanyl, hydrocodone, and morphine[58] to many patients, including those who lived in Hopkins County. Dr. Diamond's overprescribing resulted in seven deaths from 2012 to 2016, one of which was a resident of Hopkins County.[59] Dr. Diamond's license has been suspended by the Texas Medical Board.[60] Hopkins County is not, however, seeking damages under claims of medical malpractice or medical professional negligence.

48. The County lacks information sufficient to specifically identify the true names or capacities, whether individual, corporate or otherwise, of Defendants sued herein under the fictitious names DOES 1 through 100 inclusive. The County will amend this Petition to show their true names and capacities if and when they are ascertained. Hopkins County is informed and believes, and on such information and belief alleges, that each of the Defendants named as a DOE has engaged in conduct that contributed to cause events and occurrences alleged in this Petition and, as such, shares liability for at least some part of the relief sought herein.

## VI. FACTUAL ALLEGATIONS

49. Before the 1990s, generally accepted standards of medical practice dictated that opioids should be used only for short-term acute pain – pain relating to recovery from surgery or for cancer or palliative (end-of-life) care. Using opioids for chronic pain was discouraged or even prohibited because there was a lack of evidence that opioids improved patients' ability to overcome pain and function. Instead the evidence demonstrated that patients developed tolerance to opioids over time, which increased the risk of addiction and other side effects.

[58] Emma Ruby, *Texas Suspends Doctor's License after 7 Drug Deaths, including McKinney Overdose,* Dallas Morning News, Jul. 21, 2012, *available at* http://www.dallasnews.com/news/crime/2017/07/12/texas-doctor-accused-prescribing-unnecessary-drugs-led-7-deaths.
[59] *Id.*
[60] Texas Medical Board, *available at* http://reg.tmb/state/tx/us/OnLineVerif/Phy_ReportVerif_new.asp.\.

50. After the 1990s, Defendants dramatically changed doctors' views regarding opioids through a well-funded deceptive marketing scheme. Defendants were so successful that, according to the National Safety Council, 74% of ***all*** doctors prescribe opioids for chronic back pain and 55% prescribe opioids for dental pain, "neither of which is appropriate in most cases."[61] And 99% of doctors are prescribing them for longer than the three-day recommended period as recommended by the CDC.[62] Twenty three percent prescribe at least a month's worth of opioids and evidence shows that just 30 days of usage can cause brain damage.[63]

51. Each Defendant used direct marketing and unbranded advertising (*i.e.*, advertising that promotes opioid use generally but does not name a specific opioid) disseminated by seemingly independent third parties to spread false and deceptive statements about the risks and benefits of long-term opioid use. Defendants advocated the widespread use of opioids for chronic pain even though it contravened the "cardinal principles of medical intervention – that there be compelling evidence of the benefit of a therapy prior to its large-scale use."[64]

## A. Defendants Used Multiple Avenues To Disseminate their False and Deceptive Statements about Opioids.

52. Defendants spread their false and deceptive statements by (1) marketing their branded opioids directly to doctors treating patients residing in Hopkins County and the Hopkins County patients themselves and (2) deploying so-called unbiased and independent third parties to Hopkins County.

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[61] National Safety Council, *NSC Poll: 99% of Doctors Prescribe Highly-Addictive Opioids Longer than CDC Recommends*, 2017 (The NSC was founded in 1913 and chartered by Congress and is a non-profit organization whose mission is to save lives by preventing injuries and deaths at work, in homes, and in the communities through leadership, research, education, and advocacy).
[62] *Id.*
[63] *Id.*
[64] Manchikanti, Ex. A, at 2.

### 1. Defendants Spread and Continue to Spread Their False and Deceptive Statements Through Direct Marketing of Their Branded Opioids.

53. Defendants' direct marketing of opioids generally proceeded on two tracks. First, each Manufacturing Defendant conducted advertising campaigns touting the purported benefits of their branded drugs. For example, Purdue spent $200 million promoting and marketing OxyContin in various forms.[65] Defendants spent more than $14 million on medical journal advertising of opioids in 2011, nearly triple what they spent in 2001, including $8.3 million by Purdue, $4.9 million by Janssen, and $1.1 million by Endo.

54. A number of Defendants' branded ads deceptively portrayed the benefits of opioids for chronic pain. For example, Endo distributed and made available on its website, www.opana.com, a pamphlet promoting Opana ER with photographs depicting patients with physically demanding jobs like a construction worker and chef, implying that the drug would provide long-term pain-relief and functional improvement.

55. Purdue also ran a series of ads, called "pain vignettes," for OxyContin in 2012 in medical journals. These ads featured chronic pain patients and recommended OxyContin for each. One ad described a "54-year-old writer with osteoarthritis of the hands" and implied that OxyContin would help the writer work more effectively. Second, each Defendant promoted the use of opioids for chronic pain through "detailers" – sales representatives who visited individual doctors and medical staff in their offices – and small-group speaker programs.

56. Defendants devoted massive resources to direct sales contacts with doctors. In 2014 alone, Defendants spent $154 million on detailing branded opioids to doctors, including $108 million by Purdue, $34 million by Janssen, and $10 million by Endo.

[65] Zee, Ex. B, at 2.

57. Defendants sent their sales representatives to prescribers based on their specialties and prescribing habits obtained from sales data through IMS Health. Defendants used this data to monitor, and thereby target, specific physicians through the initial and renewal prescribing rates. To ensure that their sales representatives were properly incentivized, Defendants motivated them through bonuses. In 2001, Purdue paid $20 million in "sales incentive bonuses" to its sales representatives.[66]

58. Defendants also utilized "influence mapping" to use decile rankings or similar breakdowns to identify high-volume prescribers. The underlying strategy was that detailers would have the biggest sales impact on high-volume prescribers. For example, Endo identified prescribers representing 30% of its nationwide sales volume and planned sales visits three times per month to these physicians. These detailers visited physicians across the nation, including physicians in Hopkins County. Defendants also had access to data from IMS Health, which provides Defendants specific details about which medications physicians prescribe and how frequently they do so. This data was collected from more than 50% of the pharmacies in the United States, which would inform Defendants which doctors to target to convince them to prescribe more opioids or to start prescribing opioids instead of the medications they had been prescribing.

59. Another manner in which Defendants expanded their sales was to target prescribers in individual zip codes and local boundaries. Defendants would send a detailer based on ease of in-person access and the likelihood of convincing the physician to prescribe a higher number of opioids and at higher doses.

60. As part and parcel of their detailing of opioids to physicians, Purdue trained its sales representatives to inform physicians that the risk of addiction was "less than one percent" even

[66] Zee, Ex. B, at 2.

though studies demonstrated that there was a high incidence of drug abuse associated with prescription opioid use for chronic pain.[67]

61. As Defendants' marketing efforts grew, they targeted nurse practitioners and physician assistants who, a 2012 Endo business plan noted, were "share acquisition" opportunities because they were more responsive than physicians to details and wrote most of their prescriptions without a physician consult.

62. Studies demonstrate that visits from sales representatives influence the prescribing practices of residents and physicians by curtailing the prescription of generic drugs and rapidly expanding the prescription of new drugs, such as opioids for chronic pain.[68]

63. Defendants also paid doctors to serve on speakers' bureaus, to attend programs, and for meals.[69] In 2017, Dr. Hadland identified some of these payments from pharmaceutical companies to physicians prescribing opioids.[70] It was the first time "industry payments to physicians related to opioid marketing" could be collated because of the "Open Payments program database" authorized under the "Physician Payments Sunshine Act."[71] Dr. Hadland explained that it was the first large-scale examination of these payments.[72]

64. One statistic Dr. Hadland gleaned from the data is that nearly 1 in 5 family physicians in 2013, out of 108,971 active family physicians, received an opioid-related payment.[73] After culling through the Open Payments program database, Dr. Hadland concluded that "[f]inancial transfers" from pharmaceutical companies to physicians prescribing opioids "were

[67] Zee, Ex. B, at 3.
[68] *Id.* at 6.
[69] *See* Scott E. Hadland, M.D., M.P.H, M.S., *Industry Payments to Physicians for Opioid Products, 2013-2015*, 107 Am. J of Pub. Health 9, Sept. 2017, attached hereto as Exhibit C.
[70] *See id.* at 1493.
[71] *Id.*
[72] *Id.* at 1495.
[73] Hadland, Ex. C, at 1494.

substantial and widespread and may be increasing in number and value."[74]

65. Some of the financial transfers most likely involved speaker programs, which provided: (1) an incentive for doctors to prescribe a particular opioid (so they might be selected to promote the drug); (2) recognition and compensation for the doctors selected as speakers; and (3) an opportunity to promote the drug through the speaker to his or her peers. These speakers gave the false impression that they were providing unbiased and medically accurate presentations when they were, in fact, presenting a script prepared by Defendants. On information and belief, these presentations conveyed misleading information, omitted material information, and failed to correct Defendants' prior misrepresentations about the risks and benefits of opioids.

66. Defendants employed the same marketing plans, strategies, and messages in and around Hopkins County, Texas as they did nationwide. Across the pharmaceutical industry, "core message" development is funded and overseen on a national basis by corporate headquarters. This comprehensive approach ensures that Defendants' messages are accurately and consistently delivered across marketing channels and in each sales territory. Defendants consider this high level of coordination and uniformity crucial to successfully marketing their drugs.

**2. Defendants Used a Diverse Group of Seemingly Independent Third Parties to Spread False and Deceptive Statements about the Risks and Benefits of Opioids.**

67. Defendants also deceptively marketed opioids in and around Hopkins County through unbranded advertising. This advertising was ostensibly created and disseminated by independent third parties. But by funding, directing, reviewing, editing, and distributing this unbranded advertising, Defendants controlled the deceptive messages disseminated by these third parties and acted in concert with them to falsely and misleadingly promote opioids for treating

[74] *Id.* at 1495.

chronic pain. Unbranded advertising also avoided regulatory scrutiny because Defendants did not have to submit it to the FDA, and therefore it was not reviewed by the FDA. But it is illegal for a drug company to distribute materials that exclude contrary evidence or information about the drug's safety or efficacy that "clearly cannot be supported by the results of the study."[75] Moreover, a drug company cannot compare or suggest that its "drug is safer or more effective than another drug…when it has not been demonstrated to be safer or more effective in such particular by substantial evidence of substantial clinical experience."[76] It is therefore Defendants' responsibility to ensure that not only is its label accurate and complete, but that any and all materials they distribute is accurate and complete.[77]

68. Defendants' deceptive unbranded marketing often contradicted their branded materials. For example, Endo's unbranded advertising contradicted its concurrent, branded advertising for Opana ER:

| **Pain: Opioid Therapy (Unbranded)** | **Opana ER Advertisement (Branded)** |
|---|---|
| "People who take opioids **as prescribed usually do not become addicted**." | "All patients treated with opioids require careful monitoring for signs of abuse and addiction, since **use of opioid analgesic products carries the risk of addiction even under appropriate medical use**." |

69. Drug companies that make, market, and distribute opioids are generally subject to rules requiring truthful marketing of prescription drugs. A drug company's branded marketing, which identifies and promotes a specific drug, must: (a) be consistent with its label and supported

[75] 21 C.F.R. § 99.101(a)(4).
[76] 21 C.F.R. § 202.1 (e)(6)(ii).
[77] *See* 21 C.F.R. § 201.56 (providing general requirements for prescription drug labeling); 21 C.F.R. § 314.70(c)(6)(iii)(A-C) (providing for changes to labels that strengthen precautions, warnings, or adverse reactions, as well as statements about drug abuse, dependence, or overdosage); *see also Wyeth v. Levine*, 555 U.S. 555 (2009) (holding that a drug company bears responsibility for the content of its drug label at all times).

by substantial scientific evidence; (b) not include false or misleading statements or material omissions; and (c) fairly balance the drug's benefits and risks.[78]

70. This framework ensures that drug companies, which are best suited to understand the properties and effect of their drugs, bear the responsibility of providing accurate information so that prescribers and users can assess the risks and benefits of the drugs.

71. Defendants did not follow this framework in assisting, creating, and/or distributing third-party publications that included warnings and instructions either mandated by the FDA-required drug labels or that described the risks and benefits known to Defendants. The publications either failed to disclose the risk of addiction and misuse or affirmatively denied the risk of addiction. The publications also "appeared" to be independent third-party materials that had the effect of carrying more weight and credibility to convince physicians that opioids were safe for chronic pain.

**a. Defendants Utilized Treatment Guidelines to Promote their Deception.**

72. Defendants used treatment guidelines to normalize the use of opioids for chronic pain. Doctors, especially general practitioners and family doctors, rely upon treatment guidelines when faced with patients complaining of chronic pain. Scientific literature references treatment guidelines in making its conclusions and third-party payers use treatment guidelines to determine coverage. Even Endo's internal documents indicate that sales representatives discussed treatment guidelines with doctors during individual sales visits.

**1. The FSMB Wrote or Sponsored Misleading and Deceptive Guidelines.**

73. Headquartered in Euless, Texas, the Federation of State Medical Boards ("FSMB") is a trade organization representing the various state medical boards in the United States. The state

[78] 21 U.S.C. § 352(a); 21 C.F.R. §§ 1.21(a); 202.1(e)(3); 202.1(e)(6).

boards that comprise the FSMB membership have the power to license doctors, investigate complaints, and discipline doctors. The FSMB finances opioid and pain-specific programs through grants from Defendants.

74. In 1998, the FSMB developed *Model Guidelines for the Use of Controlled Substances for the Treatment of Pain* ("FSMB Guidelines"), which was produced in collaboration with pharmaceutical companies. The FSMB guidelines instructed that opioids were "essential" for the treatment of chronic pain, even as a first prescription option.

75. A book adapted from the 2007 FSMB guidelines, *Responsible Opioid Prescribing: A Physician's Guide* ("*Opioid Prescribing*"), released March 1, 2009 makes these same claims. *Opioid Prescribing* was supported by a consortium of pharmaceutical companies and Front Groups with an interest in ensuring that "effective" pain management included the use of opioids.

76. The author of *Opioid Prescribing*, Scott Fishman, M.D., chaired the board of the American Pain Foundation and served as president of the American Academy of Pain Medicine. *Opioid Prescribing* was sponsored by the Alliance of State Pain Initiatives, Federation of State Medical Boards, and the University of Wisconsin School of Medicine and Public Health.[79]

77. Dr. Fishman was a paid consultant to Cephalon and Eli Lilly. Dr. Fishman was also a paid consultant, on the Speakers' Bureau, and part of the research support for Endo, Merck, Janssen, Pfizer and Purdue.[80]

78. *Opioid Prescribing* was designed for continued medical education ("CME") in which a physician had to read the book, complete questions, and fulfill administrative steps to receive 7.5 hours of credit. The first page of *Opioid Prescribing* specifically states that opioids

[79] Scott M. Fishman, M.D., *Responsible Opioid Prescribing, A Physician's Guide*, FSMB Foundation, Waterford Life Sciences, 2009.
[80] *Id.*

are the "drugs of choice" and "essential in the treatment of persons with chronic non-cancer pain" and that the CME will inform physicians about the laws and regulations governing the prescribing of opioids for pain control.[81] It also specifically teaches physicians how to protect their practices from unwarranted federal scrutiny.[82]

79. *Opioid Prescribing* marketed "[o]pioid analgesics" as the "drugs of choice for the management of moderate to severe pain . . . [which] may be ***essential*** in the treatment of persons with chronic non-cancer pain."[83] The goal was to "change patient care, medical knowledge, practice-based learning, interpersonal and communication skills, and professionalism . . . ."[84] The argument was that opioids were "underutilized" despite their "effectiveness."[85] The truth, known to Dr. Fishman and Defendants herein, was that using opioids "for other than legitimate medical purposes pose[d] a threat to the individual and society," posed high risks for overdose and addiction, and remained unproven as safe and effective for the long-term treatment of non-cancer pain.[86]

80. It was even conveyed to doctors that undertreating pain would be officially disciplined whereas doctors prescribing opioids for chronic pain would not be disciplined. *Opioid Prescribing* described a case in which a physician was sued for "elder abuse" and the jury awarded $1.5 million to the plaintiff as an example of a physician that had been "successfully sued for not treating pain aggressively."[87] *Opioid Prescribing* cautioned that "these legal precedents sound a warning that there are risks associated with under-treating."[88] In actuality, it was a threat that

[81] *Id.*
[82] *Id.*
[83] *Id.* at i.
[84] *Id.*
[85] *Id.*
[86] *Id.* at 6, 9.
[87] *Id.* at 28.
[88] Fishman, *supra.*

doctors would be punished if they *failed* to prescribe opioids to patients who complained about pain. That teaching has held true given that according to the National Safety Council, 67% of doctors prescribe opioids, in part, based on a patient's expectations.[89] Moreover, approximately 74% of doctors incorrectly believe morphine and oxycodone are the most effective ways to treat pain even though research shows that over-the-counter medications such as ibuprofen and acetaminophen are the most effective pain relief for acute pain.[90]

81. Defendants also allayed any concerns doctors may have about patients exhibiting addictive behavior by highlighting the now debunked myth of "pseudoaddiction." Dr. Fishman described pseudoaddiction as a sign that patients were receiving an inadequate dose to obtain pain relief, not as a sign that the patient was exhibiting drug-seeking or addictive behavior.[91]

82. *Prescribing Opioids* taught physicians that the following signs were evidence of "pseudoaddiction" and *not* drug seeking behavior or signs of addiction so long as prescribing additional opioids resolves the pain:

- Requesting analgesics by name;
- Demanding or manipulative behavior,
- Clock watching;
- Taking opioid drugs for an extended period;
- Obtaining opioid drugs from more than one physician; and
- Hoarding opioids.[92]

83. Indeed, the types of behaviors that Dr. Fishman posed as "MORE indicative of addiction" included:

[89] National Safety Council, *supra*.
[90] *Id.*
[91] Fishman, *supra.* at 62.
[92] *Id.*

- Stealing money to obtain drugs;
- Performing sex for drugs;
- Stealing drugs from others;
- Prostituting others for money to obtain drugs;
- Prescription forgery; and
- Selling prescription drugs.[93]

84. Certainly by the time a patient is performing sex for drugs, the patient has long been addicted and exhibited addictive behavior that was ignored by physicians at the explicit direction of Defendants. This conclusion is supported by the American Psychiatric Association.

85. In the DSM-IV, addiction is "manifested" by three (or more) of the following in a 12-month period, including:

a) Tolerance described as:

A need for markedly increased amounts of the substance to achieve intoxication or the desired effect

*or*

Markedly diminished effect with continued use of the same amount of the substance;

b) Withdrawal manifested by:

The characteristic withdrawal syndrome for the substance

*or*

The same (or closely related) substance is taken to relieve or avoid withdrawal symptoms;

c) The substance is taken in larger amounts or over a longer period than intended; and

d) Spending a great deal of time to obtain the substance, such as visiting

[93] *Id.* at 63.

multiple doctors or driving long distances.[94]

86. According to Defendants, as seen in *Prescribing Opioids* and other publications, signs of addiction as defined by the American Psychiatric Association are not signs of addiction, but of pseudoaddiction that justifies taking *more* opioids for a longer period of time.

87. The reason not to discontinue the use of opioids – indeed, the foundation upon which Defendants built its opioid empire – was "the undertreatment of pain."[95] *Opioid Prescribing* claimed the undertreatment of pain has "been recognized as a public health crisis for decades. The cost of human suffering is immeasurable. Turning away patients in pain simply is not an option."[96] However, according to Dr. Donald Treater, medical advisor at The National Safety Council: "Opioids do not kill pain; they kill people."[97]

88. *Prescribing Opioids* acknowledged that by 2005, more than 10 million Americans were abusing prescription drugs, which is more than the combined number of people abusing cocaine, heroin, hallucinogens, and inhalants combined.[98] It also acknowledged that prescription opioids are associated with more overdose deaths than cocaine and heroin combined.[99] Yet the book then cautioned that the "undertreatment" of non-cancer pain was a public health crisis of equal importance that justified more opioid prescribing.

89. Under the guise of addressing "legitimate cause of undertreated pain" that "patients and advocates have been pushing to address,"[100] Manufacturing Defendants tailored opioid

[94] American Psychiatric Association, *Diagnostic and Statistical Manual of Mental Disorders*, Fourth Ed., Washington, D.C., American Psychiatric Assoc., 2000.
[95] Fishman, *supra,* at 105.
[96] *Id*.; *see also id.* at 80 (stating that efforts have been made to reduce the undertreatment or non-treatment of pain in children, the elderly, and in other vulnerable patient populations).
[97] National Safety Council, *supra*.
[98] *Responsible Opioid Prescribing*, *supra,* at 6.
[99] *Id.*; *Prescribing Opioids* even recognized that "[b]ehind these figures lie millions of individual stories of personal tragedy: untimely death, fractures families, shattered dreams and wasted lives." *Id.* at 7.
[100] *Id.* at 8.

marketing campaigns to affect children and the elderly. The Defendants made significant efforts to promote more opioid prescribing for "untreated or undertreated pain in children, older patients, and in all other vulnerable patient populations."[101]

90. Defendants also taught physicians that "[p]ain is what the patient says it is" and that a physician "cannot measure or even confirm the pain that a patient is experiencing."[102] As such, "pain remains an untestable hypothesis."[103] Furthermore, "[p]atients should not be denied opioid medications except in light of clear evidence that such medications are harmful to the patient."[104] All in all, opioids would cure the "pain epidemic" facing Americans. And yet, chronic pain continues to be a problem facing Americans, as well as an opioid epidemic of addiction and death.

91. A total of 200,000 copies of *Opioid Prescribing,* which Dr. Fisherman wrote for the FSMB, has been delivered to U.S. prescribers through 20 state medical boards in all 50 states, including Texas. The FSMB earned approximately $250,000 from the sale. The FSMB website describes the book as the "leading continuing medication education (CME) activity for prescribers of opioid medications."

92. The guidelines for *Opioid Prescribing* were posted online for use and reliance by physicians throughout America, including but not limited to, those servicing patients in Hopkins County. State medical boards even encouraged physicians to buy the book and participate in the CME. The North Carolina Medical Board stated on its website that *Prescribing Opioids* "has been ***widely used and supported*** in the medical and regulatory communities as the leading continuing medical education (CME) activity for prescribers of opioid medications."[105] The website then

[101] Fishman, *supra*, at 8.
[102] *See id.* at 14.
[103] *Id.* at 13.
[104] *Id.* at 9.
[105] North Carolina Medical Board, *FSMB Foundation Publishes Second Edition of Prescribing Book*, Forum Newsletter, July 31, 2012; *see also* University of Wisconsin School of Medicine and Public Health, Federation of

informs physicians that a CME accompanies the book and directs them to the book and how to claim the CME. The FSMB also hosted free CMEs in Texas, including Houston, Dallas, and Austin, related to extended-release and long-acting opioids.[106] The CME taught physicians the "safe and responsible prescribing of opioid medications and [was] aimed at improving prescriber training and counseling for patients while providing more thorough information on extended-release or long-acting (ER/LA) opioid products on the market."[107]

93. The impact of *Opioid Prescribing* was even studied through a survey sent to 12,666 licensed Georgia physicians six weeks after receiving the book.[108] The lead author was a member of FSMB.[109] A total of 508 physicians completed the online survey and of those, 82.1% rated the book either "very good" or "good" for improving care for their patients in pain.[110] Almost one-third (32.2%) claimed that they intended to make changes to their practice after reading the book.[111] Of note, 42.8% of solo practitioners and 41.6% of primary care providers were more likely to make changes to their practice than doctors in other areas.[112] Of the respondents, 57.7% said that the book was better than others with regard to prescribing opioids and on pain management.[113]

94. *Opioid Prescribing* was therefore an effective tool that impacted specific doctors and their prescribing practices, as concluded by the study. Specifically, the study provided "insight into which physician population would be the most receptive to the type of information presented

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State Medical Boards, *Responsible Opioid Prescribing – Book Helps Physicians Reduce Risk of Opioid Diversion and Abuse*, April 1, 2009 (describing the book and CME activity).

[106] Texas Medical Board, *Extended-Release and Long-Acting Opioid Analgesics Risk Evaluation and Mitigation Strategy*, www.tmb.state.tx.us.

[107] *Id.*

[108] A. Young, *Physician Survey Examining the Impact of an Educational Tool for Responsible Opioid Prescribing*, J. Opioid Management, Mar-Apr. 2012.

[109] *Id.*

[110] *Id.*

[111] *Id.*

[112] *Id.*

[113] *Id.*

in Dr. Fishman's book" and that population was to "first target[] solo and primary care physicians."[114] Defendants found out that their educational efforts "significantly altered prescription practices."[115]

**2. The Joint Commission also Spread Deceptive Information.**

95. The Joint Commission on Accreditation of Healthcare Organizations ("JCAHO") is a United States-based non-profit, tax-exempt organization that "accredits and certifies nearly 21,000 health care organizations and programs in the United States."[116] A majority of state governments recognize accreditation from the Joint Commission as a condition of licensure and for receiving Medicaid and Medicare reimbursements.[117] Hopkins County Hospital is part of the Trinity Mother Frances Health System based in Tyler, Texas, which subscribed to the JCAHO.[118]

96. According to the JCAHO, it "continuously improve[s] health care for the public" and inspires health care organizations "to excel in providing safe and effective care of the highest quality and value."[119] The JCAHO is not independent, but has been influenced by Manufacturing Defendants and those Defendants used the JCAHO as a marketing shill to spread the misleading message that opioids are non-addictive and safe as a first-line analgesic to treat any complaint of pain.

97. In 2000, the JCAHO published *Pain Assessment and Management: An Organizational Approach* ("*Pain Assessment"*), which was paid for by Purdue and reviewed by June L. Dahl, Ph.D., who had worked for Endo and Purdue.[120]

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[114] *Id.*
[115] *Id.*
[116] www.jointcommission.org.
[117] Anthony Anonimo, *Poppy Seed. Revealing the Roots of the Opioid Epidemic,* Trinity Mother Frances Health System, 2017, at 65.
[118] www.jointcommission.org.
[119] *Id.*
[120] Joint Commission on Accreditation of Healthcare Organizations, *Pain Assessment and Management*, 2000.

98. The JCAHO mission statement on the inside cover page of the book explains that it aspires "to continuously improve the safety and quality of care provided to the public through the provision of health care accreditation and related services that support the performance improvement in health care organizations."[121] One of its big achievements, however, is its endorsements of new pain management standards that underscored Defendants' fraudulent message.

99. JCAHO, with the help of the American Pain Society ("APS"), a Front Group, loosened pain management standards thereby allowing doctors to prescribe opioids for any complaint of pain. To that end, "[t]he Joint Commission recognize[d] pain as a major, yet largely avoidable, problem . . . .[and] has expanded the scope of its pain management standards, which have been endorsed by the American Pain Society (APS), ***to cover all pain scenarios in accredited health care organizations rather than limiting the scope to end-of-life care***."[122] (Emphasis added.) On January 1, 2001, Texas incorporated JCAHO pain management standards for hospital and healthcare group accreditation.[123] The Texas Medical Association advertises that *Pain Assessment* "provides practical help in integrating pain assessment and management into organizational systems . . . ."[124]

100. *Pain Assessment* established the cornerstone of Defendants' message that "all pain scenarios" should be included in pain management practices.[125] It explained that "[p]ain is the most common reason individuals seek medical attention. According to the American Pain Society (APS), 50 million Americans are partially or totally disabled by pain."[126] "The conclusion? Pain

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[121] *Id.*
[122] *Id.*
[123] Texas Medical Association, *JCAHO Pain Management Services*, *available at* https://www.texmed.org/Template.aspx?id=2389&terms=The%20war%20on%20pain.
[124] *Pain Assessment*, *supra*.
[125] *Pain Assessment*, *supra,* at p. 1.
[126] *Id.*

is undertreated – despite the availability of effective pharmacologic and nonpharmacologic therapies. Why?"[127]

101. The answer is on the first page of *Pain Assessment*. There is a chronic pain epidemic. Chronic pain is undertreated. Chronic pain can be managed and even cured with opioids, which are safe and effective, according to *Pain Assessment*. And the JCAHO encouraged organizations to establish standards for recording and responding to patient pain reports and monitoring staff performance and compliance with those standards, so that a physician who did not agree with the JCAHO standards faced the spectre of poor performance evaluations.[128]

102. According to *Pain Assessment*, the reasons healthcare professionals had not used opioids previously included: (1) inadequate knowledge of opioids pharmacology and pain therapy, (2) poor pain assessment practices, (3) unfounded concerns about regulatory oversight, and (4) fear of opioids' side effects of opioids such as tolerance and addiction.[129]

103. *Pain Assessment* asserted that few practitioners received adequate training in pain management in medical school or during their residency resulting in the failure to prescribe opioids or nonsteroidal anti-inflammatory drugs (NSAIDS) on a regular basis leaving patients without pain relief.[130] "[Many] health care professionals lack the knowledge and skills to manage pain effectively, and they fear the effects of treatment."[131] Too few health care systems make pain management a priority.[132] Some clinicians had "inaccurate and exaggerated concerns about addiction, tolerance, respiratory depression, and other opioid side effects, which lead them to be extremely cautious about the use of drugs."[133] Instead of expanding upon and explaining the risks

[127] *Id.*
[128] *Id.* at 41-42.
[129] *Id.*
[130] *Id.*
[131] *Id.* at 3.
[132] *Id.* at 1.
[133] *Pain Assessment*, *supra*, at 4.

of opioids, *Pain Assessment* states: "***This attitude prevails despite the fact there is no evidence that addiction is a signification issue when persons are given opioids for pain control.***"[134] (Emphasis added). That claim of insignificant addiction risk was false when made and remains false today. Yet it worked as intended to mislead treating doctors, medical staff, and patients into believing opioids could and should be utilized more often. Indeed, 74% of doctors "incorrectly believe morphine and oxycodone" are the "most effective ways to treat pain" even though research shows that over-the-counter pain relievers are the most effective for acute pain.[135] Even worse, 20% of doctors prescribing opioids prescribed at least a month's worth, even though the evidence shows that "30-day use causes brain changes."[136]

104. Patients also contributed to the pain epidemic by their reluctance to report their pain and to take medications,[137] according to *Pain Assessment.* Doctors were instructed to engage patients in conversations about their pain before prescribing opioids by: (1) asking for pain relief when the pain begins; (2) helping the doctor or nurse assess the pain; and (3) telling the doctor or nurse if the pain is not relieved.[138] Doctors were taught that "[t]he single most reliable indicator of the existence and intensity of pain is the individual's self-report."[139] Indeed, the individual's self-report was to be the ***primary*** source of information for the doctor and deemed more reliable than the observations of others.[140]

105. The bombardment of information, instruction, books, pamphlets, seminars, ads, and marketing regarding this "pain epidemic" was so successful that pain has been included as the "fifth vital sign" to be recorded along with the individual's temperature, pulse, respiration, and

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[134] *Id.*
[135] National Safety Council, *supra.*
[136] *Id.*
[137] *Pain Assessment*, *supra*, at 4.
[138] *Id.* at 8.
[139] *Id.* at 13.
[140] *Id.*

blood pressure.[141] This strategy was first pitched by the APS to ensure that pain management gained acceptance in the medical community, which it did.[142]

106. Beginning in 1999, the Veteran's Health Administration began routinely assessing pain as the fifth vital sign in every individual.[143] And according to *Pain Assessment*, the research showed that "when pain assessment information is included in clinical charts, those individuals' analgesics [meaning opioids] are more likely to be increased."[144] In other words, including pain as a fifth element results in not only the prescribing of more opioids, it results in the prescribing of higher doses of opioids.

107. *Pain Assessment* also framed the role of key opinion leaders ("KOL") as trustworthy people "to evaluate new clinical information, assess new practices, and then determine their value within the context of the local setting."[145] Doctors were expected to accept KOLs opinions even though KOLs are ***not*** "necessarily innovators or authority figures."[146] KOLs convinced practitioners that their current chronic pain treatment was "outdated, inappropriate, unsupported by research evidence, or no longer accepted by colleagues."[147]

108. Expert leaders, on the other hand, influenced and implemented protocols with individuals or small groups.[148] These "academic strategies" included "conducting interviews to determine baseline knowledge, stimulating active participation during educational sessions, using concise graphic educational materials, and highlighting or replicating essential messages."[149] Academic detailing was modeled after pharmaceutical detailing practices in which representatives

[141] *Pain Assessment, supra*, at 20.
[142] *See id.* 20-21.
[143] *Id.* at 21.
[144] *Id.*
[145] *Id.* at p. 24.
[146] *Id.*
[147] *See id.* at 25.
[148] *Id.*
[149] *Pain Assessment*, *supra*, at 25.

visited physicians to talk about specific medicines, just as Defendants' representatives met with physicians to about opioids.[150] Simply put, *Pain Assessment* was a part of a marketing campaign to plow ground for Manufacturing Defendants to sell more opioids, and the book set forth sophisticated, multi-layered marketing strategies that were most effective in executing the campaign.

109. If a doctor was not available to prescribe opioids, a nurse would suffice. A nurse specializing in oncology, surgery, critical care, or a nurse anesthetist, as well as a clinical pharmacist, can "serv[e] as role models, provid[e] pain management education and consultation, and act[s] as agents of change."[151] These educational efforts "significantly altered prescription practices."[152]

110. To succeed in prescribing opioids for chronic pain, Defendants had to create a market for chronic pain. To do so, Defendants literally encouraged patients not to tolerate pain and to fear pain *more* than opioid addiction.[153] Physicians and their staff were encouraged to educate their patients about "effective pain management," which included the use of opioids.[154] *Pain Assessment* explained research that showed Americans would rather bear pain because they were afraid of "addiction, dependence on drugs, and tolerance to medications," which affected not only the patient's willingness to report pain, but to use adequate amount of opioids to control the pain.[155] A patient's reluctance to take opioids out of fear they would not function normally meant that the problem was "underreported" and the pain went "untreated."[156]

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[150] *See id.*
[151] *Id.*
[152] *Id.*
[153] *Id.* at 33.
[154] *Id.*
[155] *Id.*
[156] *Pain Assessment*, *supra*, at 33.

111. Consequently, the answer was to inform and educate the patient that unrelieved pain is harmful and that he or she should communicate pain.[157] *Pain Assessment* instructed the use of pain assessment instruments, including pain intensity scales, to describe the nature of the pain and stressed that the "most reliable indicator of pain" was the individual's self-report.[158] Once the patient reported the pain, the physicians and staff were taught to tell the patient about opioids, explain that opioids were safe and effective, describe the name, dosage, and duration of the opioid therapy, and explain the risk of pain versus the importance of pain management.[159]

112. To ensure that patients self-reported pain prior to hospital visits, *Pain Assessment* encouraged health care systems to provide individuals and families with pain management information ***prior*** to being admitted.[160] And health care systems were told to leave individuals and family members with audio and videotapes to watch and listen to about the "importance" of "pain relief" so that they truly understood the message – that is, if you have "pain," tell us and we will provide opioids.

113. The JCAHO was not independent and did not improve the safety or quality of healthcare. Instead it was hijacked by Defendants to standardize pain management criteria that required the use of opioids for chronic pain. The JCAHO was merely a pawn in the Manufacturing Defendants' larger game.

114. Like other books and pamphlets used by Defendants to spread their "message," *Pain Assessment* was distributed throughout the nation and in Texas. As of today, anyone can buy a used copy of *Pain Assessment* on Amazon.com for $26.48 plus $5.99 in shipping costs from a seller in Texas.

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[157] *Id.* at 35.
[158] *Id.*
[159] *Id.*
[160] *Id.* at 36.

**b. Key Opinion Leaders (KOLs) were another Means of Disseminating False Information.**

115. Defendants also sponsored KOLs, a small circle of doctors who, upon information and belief, were selected, funded, and elevated by Defendants because they publicly supported dispensing opioids more widely and indiscriminately.

116. Defendants paid KOLs to serve as consultants or to appear on their advisory boards and to give talks or present CMEs, and Defendants' support helped these KOLs become respected industry experts. As they rose to prominence, these KOLs promoted the benefits of opioids to treat chronic non-cancer pain, repaying Defendants by advancing their marketing goals.

117. KOLs wrote articles and books, gave speeches, and taught CMEs to promote the utilization of opioids to treat moderate non-cancer pain. Defendants created opportunities for KOLs to participate in "studies" and write papers for the purpose of advancing the Manufacturing Defendants marketing theme: opioids should be dispensed regularly and perpetually to treat a broad array of pain complaints.

118. Defendants' KOLs also served on committees that developed treatment guidelines that strongly encourage using opioids to treat chronic pain, and on the boards of pro-opioid advocacy groups and professional societies that develop, select, and present CMEs. Defendants were able to direct and exert control over each of these activities through their KOLs.

119. Pro-opioid doctors are one of the most important avenues that Defendants use to spread their false and deceptive statements about the risks and benefits of long-term opioid use. Defendants know that doctors rely heavily and less critically on their peers for guidance, and KOLs provide the false appearance of unbiased and reliable support for using opioids for chronic pain.

120. Different Defendants utilized many of the same KOLs. Two of the most prominent are described below.

### 1. Russell Portenoy

121. Dr. Russell Portenoy, former Chairman of the Department of Pain Medicine and Palliative Care at Beth Israel Medical Center in New York, is one example of a KOL who Defendants identified and promoted to further their marketing campaign. Dr. Portenoy received research support, consulting fees, and honoraria from Endo, Janssen, and Purdue (among others), and was a paid consultant to Purdue.

122. Dr. Portenoy was instrumental in opening the door for the regular use of opioids to treat chronic pain. He served on the American Pain Society ("APS")/American Academy of Pain Medicine ("AAPM") Guidelines Committees, which endorsed the use of opioids to treat chronic pain, first in 1997 and again in 2009. He was also a member of the board of the American Pain Foundation ("APF"), an advocacy organization almost entirely funded by Defendants.

123. Dr. Portenoy also made frequent media appearances promoting opioids. He appeared on *Good Morning America* in 2010 to discuss using opioids long-term to treat chronic pain. On this widely-watched program, broadcast in Texas and across the country, Dr. Portenoy claimed: "Addiction, when treating pain, is distinctly uncommon. If a person does not have a history, a personal history, of substance abuse, and does not have a history in the family of substance abuse, and does not have a very major psychiatric disorder, most doctors can feel very assured that that person is not going to become addicted."[161]

124. Perhaps realizing that "[m]ore than 16,000 people die from opioid overdoses every year," Dr. Portenoy is now having "second thoughts" about the "wider prescription" of drugs like Vicodin, OxyContin, and Percocet.[162] Dr. Portenoy later admitted in a 2010 videotaped interview

[161] Good Morning America television broadcast, ABC News, Aug. 30, 2010.
[162] Thomas Catan & Evan Perez, *A Pain-Drug Champion Has Second Thoughts*, WALL ST. J., Dec. 17, 2012, attached hereto as Exhibit D.

that he "gave innumerable lectures in the late 1980s and '90s about addiction that weren't true."[163] According to Dr. Portenoy, because the primary goal was to "destigmatize" opioids, he and other doctors promoting them overstated their benefits and glossed over their risks.

125. Dr. Portenoy put doctors' fear that opioids were dangerous and addictive, and meant only for cancer patients, to rest by arguing that they could be taken safely for months, even years, by patients with chronic pain.[164] Dr. Portenoy, as well as other doctors making the speaker rounds, asserted that "[l]ess than 1% of opioid users became addicted, the drugs were easy to discontinue and overdoses were extremely rare in pain patients."[165]

126. Dr. Portenoy also conceded that "[d]ata about the effectiveness of opioids does not exist."[166] Dr. Portenoy candidly stated: "Did I teach about pain management, specifically about opioid therapy, in a way that reflects misinformation? Well…I guess I did."[167]

127. Before his moment of clarity, Dr. Portenoy co-authored a guide to publicize the benefits of opioids for chronic pain, which was paid for by an unrestricted education grant from Endo, titled *A Clinical Guide to Opioid Analgesia* ("*Opioid Analgesia*").[168] *Opioid Analgesia* reiterated that opioids are "absolutely necessary" for pain relief.[169]

128. Although *Opioid Analgesia* claimed "to help clinicians make practical sense of the varied and often conflicting pharmacologic, clinical and regulatory issues to promote the most healthful outcomes possible for patients in pain,"[170] the reality was that it expressed regret that federal and state governments had passed controlled substances acts to stem addiction, which had

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[163] *Id*.
[164] Catan, *supra*.
[165] *Id.*
[166] *Id.*
[167] *Id.*
[168] Perry G. Fine, M.D. and Russell K. Portenoy, M.D., *A Clinical Guide to Opioid Analgesia*, McGraw-Hill, 2004.
[169] *Id.* at 2.
[170] *Id.* at 3.

curtailed the prescription of opioids.[171] This regulation, explained *Opioid Analgesia*, "contributed to the underuse of opioid medications."[172]

129. As with all other books, guidelines, and CMEs promoted by Front Groups and KOLs, *Opioid Analgesia* establishes the absolute need for opioids in light of the chronic pain epidemic. "Because pain is inherently subjective, patient self-report is the 'gold standard' for assessment."[173] If there's no discernible reason for the pain, then it should be characterized as "idiopathic."[174] Regardless of how the pain is characterized, the solution, per *Opioid Analgesia,* is opioids.

130. "While opioid analgesics are controlled substances, they are also essential medication and are absolutely necessary for relief of pain."[175] "Opioid analgesics should be accessible to all patients who need them for relief of pain."[176] Brushing away any concerns about addiction, *Opioid Analgesia* posits that "[a] patient who has reached middle age without developing compulsive use behaviors to potentially abusable drugs, including alcohol and nicotine, appears to be at a very low risk" of addiction, especially if "there is no family history of addiction."[177]

131. Underplaying the risks of addiction, *Opioid Analgesia* falsely claimed that "[o]verall, the literature provides evidence that the outcomes of the drug abuse and addiction are rare among patients who receive opioids for a short period (i.e., for acute pain) and among those with no history of abuse who receive long-term therapy for medical indications."[178] Even while

[171] *Id.*
[172] *Id.* at 6.
[173] Fine, *supra,* at 34.
[174] *Id.* at 35.
[175] *Id.* at Table 1.
[176] *Id.*
[177] *Id.* at 21.
[178] *Id.*

admitting there is "very little information about the risks of misuse, abuse, or addiction among different opioid-treated populations" and even admitting the "[w]hen misused, opioids pose a threat to society,"[179] Defendants' intentionally marketed opioids as effective and safe for treatment of chronic pain and summed up the risk of addiction for short-term therapy as "rare."[180]

132. Of course when addiction is as narrowly defined as it is in the books, CMEs, and guidelines that Defendants publishes, the risk of addiction would be termed as "rare." The behaviors cited in *Opioid Analgesia* as "probably more suggestive" of addiction included:

- Selling prescription drugs;
- Forging prescriptions;
- Stealing or "borrowing" drugs from others;
- Injecting or inhaling (snorting, smoking) oral formulations; and
- Obtaining the prescription drugs from nonmedical sources.[181]

133. Whereas the following behaviors are "probably less suggestive" of addiction:

- Aggressive complaining about the need for more drug;
- Drug hoarding during periods of reduced symptoms;
- Requesting specific drugs; and
- Using the drug, without approval, to treat another symptom.[182]

134. Instead of these behaviors being symptoms of possible addiction, Dr. Portenoy terms these behaviors as a "phenomenon" termed "pseudoaddiction."[183] Pseudoaddiction allows physicians to discount these behaviors because "they are driven by desperation surrounding

[179] *Id.* at 31, 2.
[180] Fine, *supra*, at 34.
[181] *Id.* at 85.
[182] *Id.*
[183] *Id. at 35.*

unrelieved pain" and are "eliminated by measures that relieve the pain, such as an increase in medication."[184] Instead of treating the "less suggestive" symptoms for what they are – signs of addiction.

135. *Opioid Analgesic* was a success for Defendants in that it has been and continues to be used extensively in CMEs, pamphlets, and reading lists for physicians looking for information regarding opioids. For example, *Opioid Analgesia* was cited ***just this year*** in a presentation at the University of North Texas College of Pharmacy on April 28, 2017, entitled *Adverse Drug Events Associated with Opiate-Based Pain Management*. It has also been listed as a reference for a CME entitled *The Management of Opioid-Induced Constipation* published by the University of North Texas Health Science Center, which was valid for CME from May 2009 to May 2010. Finally, the book was included in the suggested reading list for a seminar entitled *When Opioids Are Indicated for Chronic Pain* presented on March 26, 2011, in Houston, Texas.

**2. Lynn Webster**

136. Another KOL, Dr. Lynn Webster, was the co-founder and Chief Medical Director of Lifetree Clinical Research, an otherwise-unknown pain clinic in Salt Lake City, Utah. Dr. Webster was President in 2013 and is a current board member of AAPM, a Front Group that ardently supports using opioids for chronic pain. He is a Senior Editor of *Pain Medicine*, the same journal that published Endo special advertising supplements recommending Opana ER. Dr. Webster authored numerous CMEs sponsored by Endo and Purdue while he was receiving significant funding from Defendants.

137. In 2011, Dr. Webster presented a program via webinar sponsored by Purdue titled *Managing Patient's Opioid Use: Balancing the Need and the Risk*. Dr. Webster recommended

[184] *Id.*

using risk screening tools, such as urine testing and patient agreements as a way to prevent "overuse of prescriptions" and "overdose deaths," which was available to and was intended to reach doctors treating Hopkins County residents.

138. Dr. Webster also was a leading proponent of the concept of "pseudoaddiction," the notion that addictive behaviors should be seen not as warnings, but as indications of undertreated pain. In Dr. Webster's description, the only way to differentiate the two was to *increase* a patient's dose of opioids. As he and his co-author wrote in a book entitled *Avoiding Opioid Abuse While Managing Pain* (2007), a book that is still available online, when faced with signs of aberrant behavior, increasing the dose "in most cases . . . should be the clinician's first response." Endo distributed this book to doctors.

139. Years later, Dr. Webster reversed himself, acknowledging that "[pseudoaddiction] obviously became too much of an excuse to give patients more medication."[185] Dr. Webster also admits that "[i]t's obviously crazy to think that only 1% of the population is at risk for opioid addiction."[186]

**c. Front Groups Affirmed Defendants' Falsities.**

140. Defendants entered into arrangements with seemingly unbiased and independent patient and professional organizations to promote opioids for treating chronic pain. Under Defendants' direction and control, these "Front Groups" generated treatment guidelines, unbranded materials, and programs that favored using opioids for chronic non-cancer pain. They also assisted Defendants by responding to negative articles, by advocating against regulatory changes that would limit prescribing opioids in accordance with the scientific evidence, and by conducting outreach to vulnerable patient populations targeted by Defendants.

[185] John Fauber & Ellen Gabler, *Networking Fuels Painkiller Boom*, MILWAUKEE WISC. J. SENTINEL, Feb. 19, 2012.
[186] Thomas Catan & Evan Perez, *A Pain-Drug Champion Has Second Thoughts*, WALL ST. J., Dec.17, 2012.

141. These Front Groups depended on Defendants for funding and, in some cases, for survival. Defendants also exercised control over programs and materials created by these groups by collaborating on, editing, and approving their content, and by funding their dissemination. In doing so, Defendants made sure these Front Groups would generate only the messages Defendants wanted to distribute. Even so, the Front Groups held themselves out as independent and as serving the needs of their members – whether patients suffering from pain or doctors treating those patients.

142. Defendants Endo, Janssen, and Purdue utilized many Front Groups, including many of the same ones. Several of the most prominent are described below, but there are many others, including the American Pain Society ("APS"), American Geriatrics Society ("AGS"), the Federation of State Medical Boards ("FSMB"), American Chronic Pain Association ("ACPA"), American Society of Pain Education ("ASPE"), National Pain Foundation ("NPF") and Pain & Policy Studies Group ("PPSG").

**1. American Pain Foundation ("APF")**

143. APF was founded in 1997 and professed to be an independent non-profit 501(c)3 organization "serving people with pain through information, advocacy and support."[187] It had a membership of "close to 100,000 and growing" in 2010 and claimed to be the "largest advocacy group for people with pain."[188] The APF lauded its participation in "close to 100 policy activities," which included testifying at legislative hearings to securing state and local proclamations for Pain Awareness Month.[189]

144. APF, however, as the most prominent of Manufacturing Defendants' Front Groups, received more than $10 million in funding from opioid manufacturers from 2007 until it closed its

[187] American Pain Foundation, *Treatment Options: A Guide for People Living with Pain*, www.painfoundation.org; *see also 2010 Annual Report*, American Pain Foundation.
[188] *2010 Annual Report*, *supra*.
[189] *Id.*

doors in May 2012. Endo alone provided more than half that funding; Purdue was next at $1.7 million. Despite the influx of funds from pharmaceutical companies, APF claimed to be an independent patient advocacy group.

145. In 2009 and 2010, more than 80% of APF's operating budget came from pharmaceutical industry sources. Including industry grants for specific projects, APF received about $2.3 million from industry sources out of total income of about $2.85 million in 2009. In 2010, Endo paid APF more than $1 million and Purdue paid APF between $1 million and 4.9 million.[190] By 2011, APF was entirely dependent on incoming grants from Purdue, Endo, and others to avoid using its line of credit. One of its board members, Russell Portenoy, explained the lack of funding diversity was one of the biggest problems at APF.

146. APF issued education guides for patients, reporters, and policymakers that recommended opioids for chronic pain while trivializing their risks, particularly the risk of addiction. Its *Pain Community News*, an "esteemed" quarterly newsletter, had a print circulation of more than 68,000 plus additional online readers.[191] Its monthly electronic newsletter, *Pain Monitor*, was a monthly newsletter that provided links to pain-related news and research.[192] The APF also provided "patient representatives" for Defendants' promotional activities, including Purdue's *Partners Against Pain*[193] and Janssen's *Let's Talk Pain.*[194]

147. In one of its publications, *Treatment Options: A Guide for People Living with Pain*, ("*Treatment Options*"), APF recognized contributions from Cephalon and Purdue.[195] *Treatment*

[190] *Id*.
[191] *Id.* at 2.
[192] *2010 Annual Report, supra*, at 2.
[193] In its "Partner against Pain" website, Purdue claimed that the risk of addiction from the use of OxyContin in treating "chronic non-cancer pain" was "extremely small"; *see also* Zee, Ex. B, at 3.
[194] *Let's Talk Pain* was a "coalition effort that focus[ed] on supporting positive patient-provided communications" regarding pain.
[195] *Treatment Options, supra,* at ii.

*Options* was reviewed by Scott Fishman, M.D., Vice Chairman of the APF Board of Directors, and Russell Portenoy, M.D., a Member of the APF Board of Directors and also a KOL.[196] *Treatment Options* set the stage for prescribing opioids by explaining their underuse despite their benefits.[197] It dismissed the risk of addiction with the rhetoric that physical dependence was nothing more than symptoms or signs of withdrawal that occurred when opioids were stopped suddenly or the dose lowered too quickly.[198]

148. *Responsible Opioid Prescribing* and *The War on Pain* both had a tremendous impact on doctors' prescribing habits. In 2000, Scott Fishman, M.D., who served on APF's board, co-authored *The War on Pain* ("*Pain War*") as general authoritative information about pain medicine."[199]

149. *Pain War* seeks new specialties in which opioids can be prescribed for chronic pain. Rheumatologists treating arthritis have been overlooked because they were more prone to prescribe NSAIDS instead of opioids, such as morphine.[200] But such "outdated ideas about addiction and concerns about social stigmas" need to evolve because opioids offer "substantial relief" with "less severe long-term side effects than chronic anti-inflammatories."[201]

150. *Pain War* advocates for physical dependence to opioids, and equates withdrawal symptoms from opioid drugs to that of cessation of coffee drinking. A "pain patient who is dependent on opioids finds life restored," the book advises, and then explains that removing a patient from opioids causes physical, not psychological, consequences, like quitting *coffee*.[202] Addiction to opioids is treated as a "phobia" or "notion" that "using opioids" are "always

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[196] *Id.* at iv.
[197] *Id.* at 11.
[198] *Id.* at 14 (referring to symptoms such as sweating, rapid heart rate, nausea, diarrhea, goosebumps, and anxiety).
[199] Scott Fishman, M.D., with Lisa Berger, *The War on Pain,* First Quill, 1st ed., 2000.
[200] *Id.* at 154.
[201] Fishman, *War on Pain*, *supra,* at *155.*
[202] *Id.* at 187.

addictive."[203]

151. *Pain War* censures the failure to prescribe opioids and even suggests that such failure is a criticism of the patient. For example:

> Doses tend to be too low, the right narcotic preparation tends to be avoided, and the prescribing period is often too short. Medicine's reluctance to use appropriate doses of opioid drugs gives patients the wrong message – their pain isn't that important, they are not trustworthy, they may be addicts, they are bad people if they take drugs even if they are prescribed.[204]

152. *Pain War* was distributed across the nation, and sold in Texas, as evidence by a seller from Texas offering the used book for $9.56 plus $5.99 in shipping costs on Amazon.com.

153. As late as 2008, the APF was still relaying the same message. In *A Reporter's Guide: Covering Pain and Its Management* ("*Reporter's Guide*"), the APF extolled that "[t]he person with pain is the authority on the existence and severity of his/her pain. The self-report is [the] most reliable indicator."[205] The *Reporter's Guide* referred to pain as a health crisis and concluded that it affected more Americans than "diabetes, heart disease and cancer combined."[206]

154. Yet APF, Defendants' Front Group also admitted that:

- 71% of people abusing prescription pain relievers received them from a friend or family member without a prescription;
- Approximately 2.2 million Americans abused pain medication for the first time in 2006; and
- Between 1992 and 2002, reported abuse by teenagers increased by 542%.[207]

155. Even though Defendants knew about the risks involved in prescribing opioids or ingesting opioids, they continued to disseminate a story about a "pain epidemic" that could be

[203] *Id.* at 185.
[204] *Id.*
[205] American Pain Foundation, *A Reporter's Guide: Covering Pain and Its Management*, Oct. 2008, at 1, attached hereto as Exhibit E.
[206] *Id.* at 29.
[207] *Reporter's Guide*, Ex. E, at 29.

treated only through the use of opioids. Even a 542% increase in abuse by teenagers in the United States in the span of ten years did not make Defendants change their marketing strategy or otherwise modify their educational or promotional materials concerning the risks associated with the use of opioids.

156. In addition to these publications, APF also engaged in a significant multimedia campaign – through radio, television, and the internet – to educate patients about their "right" to pain treatment, namely opioids. APF's local and national media efforts resulted in 1,600 media stories on pain in 2010, which was an increase of 1,255% from 2009.[208] APF surmised that it reached more than 600 million people with information and education related to pain.[209] All of the programs and materials were available nationally and were intended to reach patients and consumers in Hopkins County.

157. APF's website was visited by nearly 275,000 people in 2010 and a National Pain Foundation was expected to be complete in 2011.[210] In May 2012, the U.S. Senate Finance Committee began investigating the financial ties between Front Groups and trade organizations, such as APF and the FSMB, and the opioid manufacturers. This investigation not only caused damage to APF's credibility but caused Defendants to cease its funding.

158. The Senate Finance Committee intended to investigate whether pharmaceutical companies were responsible for the opioid epidemic by "promoting misleading information about the drugs' safety and effectiveness."[211] The Senate Finance Committee was concerned that a "network of national organizations and researchers with financial connections to the makers of narcotic painkillers…helped create a body of dubious information 'favoring opioids' that can be

[208] *Id.* at 15.
[209] *Id.*
[210] *2010 Annual Budget*, *supra*, at 6
[211] *See* Letter to Dr. Humayun J. Chaudhy dated May 8, 2012 from Charles E. Grassley and Max Baucus, at p. 2.

found in prescribing guidelines, patient literature, position statements, books and doctor education courses."[212]

159. The Senate Finance Committee was especially concerned that "[a]mong the FSMB's educational initiatives has been the development and distribution of a guidebook intended to help physicians recognize the risk of opioids and follow responsible and safe prescribing standards."[213] (Emphasis in original.) Hence, Dr. Fishman and his book *Opioid Prescribing: A Physician's Guide*, the first edition of which was released in 2007 and later accredited by the University of Wisconsin School of Medicine and Public Health, was at the center of the investigation.[214]

160. The Senate Finance Committee asked for any grants or financial transfers used to produce the book, the revenue generated from the sale of the book, each state that distributed the book, and the names of any people or organization involved in writing or editing the book.[215]

161. Within days, APF's board voted to dissolve the organization and it ceased to exist. The FSMB responded to the Senate Finance Committee's inquiry, however, and agreed that "the abuse and misuse of opioids is a serious national problem."[216] Dr. Chaudhy, speaking on behalf of the FSMB, acknowledged that "prescription drug abuse and related deaths has grown at an alarming pace in the United States."[217] Dr. Chaudhy described Dr. Fishman, the author of *Opioid Prescribing*, as "one of the nation's leading experts in pain medicine."[218]

162. *Opioid Prescribing* was released from 2007 through January 2012, was distributed in each of the 50 states, including Texas, and supported in the medical community as an educational

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[212] *Id. quoting* Milwaukee Journal Sentinel/MedPage Today, *Follow the Money: Pain, Policy, and Profit*, Feb. 19, 2012, *available at* at http://medpagetoday.com/Neurology/PainManagement/31256.
[213] Chaudhy Letter, *supra*, at 5.
[214] *Id.*
[215] *Id.* at 3.
[216] Letter to Max Baucus and Charles Grassley dated June 8, 2012 from Humayun J. Chaudhy, DO, FACP, at 1.
[217] Chaudhy Letter, *supra*, at 1.
[218] *Id.* at 5.

resource for doctors.[219] The book is still being sold today. For example, a used copy of the book is being sold on Amazon.com by Delta River Books, located in Texas, for $51.49 plus $3.99 in shipping. Dr. Fishman also toured and gave keynote speeches about *Opioid Prescribing*. For example, Dr. Fishman presented the keynote at the Federation of State Medical Board Meeting in Fort Worth, Texas on April 28, 2012, which lasted three days.[220] The book was also used extensively by state regulators to make safe and responsible decisions about prescribing opioids.[221]

163. As described herein, Dr. Fishman and his book was partly funded by Endo and Purdue, among others as evidenced in the response. In 2004, Purdue paid $87,895 in the form of a grant to the FSMB to update the FSMB *Model Guidelines for the Use of Controlled Substances in the Treatment of Pain*, along with other objectives related to opioids.[222] In 2005, Purdue paid $244,000 to FSMB and in 2006, Purdue paid $207,000 to FSMB for the continuation of the same project.[223] In 2008, Endo and Purdue each paid $100,000 in the form of a grant for the distribution of *Responsible Opioid Prescribing*.[224] Thus, from 2000-2012, Purdue paid $734,505.06 and Endo paid $411,620.00 to the FSMB and FSMB Foundation.

164. Dr. Chaudhy's response merely underscored Defendants' role, through KOLs and Front Groups, in controlling the message these groups conveyed about opioids.

**2. American Academy of Pain Medicine ("AAPM")**

165. The American Academy of Pain Medicine, with Defendants' assistance, prompting, involvement, and funding, issued treatment guidelines and sponsored and hosted medical education programs essential to Defendants' deceptive marketing of chronic opioid therapy.

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[219] *Id.*
[220] U.C. Davis, *Fishman Gives Keynote at Federation of State Medical Boards Meeting*, May 1, 2012, *available at* https://ucdmc.ucdavis.edu/publish/news/newsroom/6523.
[221] Chaudhy, *supra*, at 5, 17.
[222] *Id.* at 11.
[223] *Id.* at 11-12.
[224] *Id.* at 12.

166. AAPM received over $2.2 million in funding since 2009 from opioid manufacturers. AAPM maintained a corporate relations council, whose members paid $25,000 per year (on top of other funding) to participate. The benefits included allowing members to present educational programs at off-site dinner symposia in connection with AAPM's marquee event – its annual meeting held in Palm Springs, California, or other resort locations. AAPM describes the annual event as an "exclusive venue" for offering education programs to doctors. Membership in the corporate relations council also allows drug company executives and marketing staff to meet with AAPM executive committee members in small settings. Defendants Endo, Purdue, and Actavis were members of the council and presented deceptive programs to doctors who attended this annual event.

167. AAPM is viewed internally by Endo as "industry friendly," with Endo advisors and speakers among its active members. Endo attended AAPM conferences, funded its CMEs, and distributed its publications. The conferences sponsored by AAPM heavily emphasized sessions on opioids – 37 out of roughly 40 at one conference alone. AAPM's presidents have included top industry-supported KOLs Perry Fine, Russell Portenoy, and Lynn Webster. Dr. Webster was even elected president of AAPM while under a DEA investigation. Another past AAPM president, Dr. Scott Fishman, stated that he would place the organization "at the forefront" of teaching that "the risks of addiction are . . . small and can be managed."[225]

168. Defendants influenced AAPM through both their significant and regular funding and the leadership of pro-opioid KOLs within the organization. AAPM's staff understood they and their industry funders were engaged in a common task – propagate a "pain epidemic" and solve it

[225] Interview by Paula Moyer with Scott M. Fishman, M.D., Professor of Anesthesiology and Pain Medicine, Chief of the Division of Pain Medicine, Univ. of Cal., Davis (2005), *available at* http://www.medscape.org/viewarticle/500829.

by teaching that opioids were safe and effective for treating chronic pain.

169. In 1997, AAPM and the American Pain Society jointly issued a consensus statement, *The Use of Opioids for the Treatment of Chronic Pain*, which endorsed opioids to treat chronic pain and claimed there was a low risk that patients would become addicted to opioids. The co-author of the statement, Dr. Haddox, was a paid speaker for Purdue at the time. Dr. Portenoy, Defendants' KOL, was the sole consultant. The consensus statement remained on AAPM's website until 2011.

170. AAPM and APS issued their own guidelines in 2009 ("AAPM/APS Guidelines") and continued to recommend using opioids to treat chronic pain. Fourteen of the 21 panel members who drafted the AAPM/APS Guidelines, including KOLs Dr. Portenoy and Dr. Perry Fine of the University of Utah, received support from Janssen, Endo, and Purdue.

171. The 2009 Guidelines promote opioids as "safe and effective" for treating chronic pain, despite acknowledging limited evidence, and conclude that the risk of addiction is manageable for patients regardless of past abuse histories. One panel member, Dr. Joel Saper, Clinical Professor of Neurology at Michigan State University and founder of the Michigan Headache & Neurological Institute, resigned from the panel because he was concerned the 2009 Guidelines were influenced by contributions that drug companies, including Defendants, made to the sponsoring organizations and committee members. These AAPM/APS Guidelines have been a particularly effective channel of deception and have influenced not only treating physicians, but also the body of scientific evidence on opioids. The Guidelines have been cited 732 times in academic literature, were disseminated in and around Hopkins County during the relevant time period, are still available online, and were reprinted in the *Journal of Pain*.

## B. Defendants' Marketing Scheme Misrepresented the Risks and Benefits of Opioids.

172. To convince doctors and patients in Hopkins County that opioids can and should be used to treat chronic pain, Defendants had to convince them that long-term opioid use is non-addictive, safe, and effective. Knowing they could do so only by deceiving those doctors and patients about the risks and benefits of long-term opioid use, Manufacturing Defendants made claims that were not supported by, and were contrary to, the scientific evidence. Defendants have not corrected their misrepresentations.

## C. Defendants Falsely Trivialized or Failed to Disclose the Known Risks of Long-Term Opioid Use.

173. Defendants deceptively trivialized and failed to disclose the risks of long-term opioid use, particularly the risks of addiction and overdose, through a series of misrepresentations that have since been conclusively debunked by numerous published studies and the magnitude of human misery caused by Defendants' deceptions. These misrepresentations – which are described below – reinforced each other and created the dangerously misleading impression that opioids are the best treatment option for any recurrent moderate pain because: (1) only a miniscule number of patients, if any, would become addicted; (2) all patients with a substantial risk of becoming addicted to opioids could be readily identified; (3) patients who displayed signs of addiction probably were not addicted and, in any event, could easily be weaned from the drugs; (4) the use of higher opioid doses do not escalate risk of addiction or overdose; and (5) "abuse-deterrent" opioids are reliably safe and effective for perpetual use. Defendants still espouse these misrepresentations today.

174. ***First***, Defendants falsely claimed the risk of addiction is low and unlikely to develop when opioids are prescribed, as opposed to those obtained illicitly; and failed to disclose

the greater risk of addiction with prolonged use of opioids.[226] For example:

a) Actavis's predecessor caused a patient education brochure to be distributed in 2007 claiming opioid addiction is possible, but "less likely if you have never had an addiction problem." Upon information and belief, based on Actavis's acquisition of its predecessor's marketing materials along with the rights to Kadian, Actavis continued to use this brochure in 2009 and beyond;

b) Purdue sponsored APF's *Treatment Options: A Guide for People Living with Pain* (2007), which instructed that addiction is rare and limited to extreme cases of unauthorized dose escalations, obtaining duplicative opioid prescriptions from multiple sources, or theft. This publication is still available online;

c) Endo sponsored a website, Painknowledge.com, which claimed in 2009 that "[p]eople who take opioids as prescribed usually do not become addicted." Another Endo website, PainAction.com, stated "Did you know? Most chronic pain patients do not become addicted to the opioid medications that are prescribed for them.";

d) Endo distributed a pamphlet with the Endo logo entitled *Living with Someone with Chronic Pain*, which stated that: "Most health care providers who treat people with pain agree that most people do not develop an addiction problem." A similar statement appeared on the Endo website, www.opana.com;

e) Janssen reviewed, edited, approved, and distributed a patient education guide entitled *Finding Relief: Pain Management for Older Adults* (2009), which described as "myth" the claim that opioids are addictive, and asserted as fact that "[m]any studies show that opioids are rarely addictive when used properly for the management of chronic pain.";

f) Janssen currently runs a website, Prescriberesponsibly.com (last updated July 2, 2015), which claims that concerns about opioid addiction are "overestimated";

g) Purdue sponsored APF's *A Policymaker's Guide to Understanding Pain & Its Management* – which claims that less than 1% of children prescribed opioids will become addicted and that pain is undertreated due to "misconceptions about opioid addiction[]." This publication is still available online; and

[226] *See, e.g.*, Manchikanti, Ex. A, at 22 (blaming adverse consequences on abuses and overuses instead of appropriately blaming opioids used as directed).

h) Detailers for Purdue, Endo, and Janssen in and around Hopkins County minimized or omitted any discussion with doctors of the risk of addiction; misrepresented the potential for opioid abuse with purportedly abuse-deterrent formulations; and routinely did not correct the misrepresentations noted above.

175. These claims contradict empirical evidence. As noted by the CDC, there is "extensive evidence" of the "possible harms of opioids (including opioid use disorder [an alternative term for opioid addiction])."[227] The CDC has explained that "[o]pioid pain medication use presents serious risks, including…opioid use disorder" and that "continuing opioid therapy for 3 months substantially increases risk for opioid use disorder."[228] In fact, as many as "1 in 4 patients receiving long-term opioid therapy in primary care settings struggle with opioid use disorder."[229] Among the 12 recommendations by the new CDC guidelines to improve patient care and safety is that non-opioid therapy is preferred for chronic pain unless there is active cancer or it is for palliative and end-of-life care.[230]

176. Defendants' long-standing claims that opioid addiction and overdose are anomalies largely attributable to patient abuse of the drug, are demonstrably false. Indeed, the majority of cases "involving injury and death occur in people using opioids ***exactly*** as prescribed . . ."[231]

177. In 2010, a study addressed the rates of opioid overdose with patients receiving average prescribed daily opioids versus patients receiving medically prescribed chronic opioid therapy.[232] The patients included those receiving three-plus opioid prescriptions within 90-days for chronic non-cancer pain between 1997 and 2005.[233] Patients who received 50-99 mg had a 3.7-

[227] Centers for Disease Control and Prevention, *CDC Guideline for Prescribing Opioids for Chronic Pain – United States 2016*, Mar. 18, 2016.
[228] *Id.*
[229] *Id.*
[230] *Id.*
[231] Manchikanti, Ex. A, at 22.
[232] Kate M. Dunn, Ph.D., Kathleen W. Saunders, J.D., *Overdose and Prescribed Opioids: Association among Chronic Non-Cancer Pain Patients*, Ann. Intern. Med., Dec. 10, 2010, at 2.
[233] *Id.*

fold increase in overdose risk (95% C.I. 1.5, 9.5) and a 0.7 annual overdose rate.[234]

178. The authors determined that even though opioids provide some pain relief for chronic pain, balancing the long-term risks with the benefits was still "poorly understood."[235] Those patients who had not received opioids lately had a lower risk of overdose, however, than patients consistently receiving opioids at a low dosage.[236]

179. The authors pointed to previous studies that indicated a rise in opioid-related overdoses with an increase in prescribing opioids for non-cancer pain, but the belief that such phenomenon was caused by patients obtaining opioids from non-medical sources.[237] This study, however, proves for the first time that the risk of overdose is directly linked to the prescription and use of medically prescribed opioids.[238]

180. The authors of a Washington study in which the authors obtained Washington Medicaid data from the Washington Heath Care Authority reached a similar conclusion.[239] The opioid prescription claim history was examined for each "opioid poisoning" for the months that enrollees received Medicaid FFS prescription benefits.[240] The authors concluded that a large percentage of opioid poisonings happened at lower prescribed doses and in individuals who were not considered chronic users.[241]

181. The authors noted that previous opioid guidelines focused on opioid doses above 80-120 mg/d MED even though previous studies showed risk of opioid deaths and poisonings at much lower doses and that most non-methadone opioid poisonings had been prescribed below

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[234] *Id.*
[235] *Id.*
[236] Dunn, *supra,* at 6.
[237] *Id.* at 7.
[238] *Id.*
[239] Deborah Fulton-Kehoe, Ph.D., *Opioid Poisonings in Washington State Medicaid: Trends, Dosing, and Guidelines*, 53 Medical Care 8, Aug. 2015, at 680.
[240] *Id.*
[241] *Id.*

these guidelines levels.[242] The authors concluded that only a small percentage of patients are prescribed opioids at a dosage greater than 120 mg/d MED, but that a large percentage of the opioids poisonings have been occurring in patients taking lower doses and in patients not considered chronic users.[243] Overdoses were therefore occurring in patients prescribed opioids for chronic non-cancer pain at increased rates and the overdose risk increased with an average prescription dose.[244] The guidelines and other educational material regarding opioids need to be changed to reflect the opioid poisoning among this population.[245]

182. In fact, "[t]he majority of deaths (60%) occur in patients when they are given prescriptions based on prescribing guidelines by medical boards with 20% of deaths in low dose opioid therapy . . . ."[246] The way to cure the "crisis of opioid use in the United States" is to change "inappropriate prescribing patterns, which are largely based on a lack of knowledge, perceived safety, and inaccurate belief of undertreatment of pain."[247]

183. Another study found that approximately 60% of overdoses occur in medical users of opioids prescribed by a single physician to manage chronic pain.[248] Non-medical users comprise only a statistical minority of opioid overdoses.[249]

184. Scientific evidence underscores the conclusion that low-dose opioid therapy for chronic pain, opioids taken as prescribed, opioids obtained from a single doctor, and opioids prescribed pursuant to prescribing guidelines cause many overdoses. Manufacturing Defendants, however, disseminated contrary messaging throughout their marketing campaigns to sell more

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[242] *Id.* at 683.
[243] *Id.* at 684.
[244] *Id.*
[245] *Id.*
[246] Manchikanti, Ex. A, at 1.
[247] *Id.*
[248] Barbara Zedler, M.D., *Risk Factors for Serious Prescription Opioid-Related Toxicity or Overdose Among Veterans Health Administration Patients*, Pain Medicine, 2014, at 1912, attached hereto as Exhibit F.
[249] *Id.*

opioids.

185. ***Second***, Defendants falsely instructed doctors and patients that signs of addiction are actually signs of undertreated pain and should be treated by prescribing more opioids. Defendants called this phenomenon "pseudoaddiction" – a term coined by Dr. David Haddox, who went to work for Purdue, and popularized by Dr. Russell Portenoy, a KOL for Endo, Janssen, and Purdue – and claimed that pseudoaddiction is substantiated by scientific evidence. For example:

a) Purdue sponsored *Responsible Opioid Prescribing* (2007), which taught that behaviors such as "requesting drugs by name," "demanding or manipulative behavior," seeing more than one doctor to obtain opioids, and hoarding, are all signs of pseudoaddiction, rather than true addiction. *Responsible Opioid Prescribing* remains for sale online. The 2012 edition continues to teach that pseudoaddiction is real;

b) Janssen sponsored, funded, and edited the *Let's Talk Pain* website, which in 2009 stated: "pseudoaddiction . . . refers to patient behaviors that may occur when pain is under-treated . . . . Pseudoaddiction is different from true addiction because such behaviors can be resolved with effective pain management.";

c) Endo sponsored a National Initiative on Pain Control (NIPC) CME program in 2009 titled *Chronic Opioid Therapy: Understanding Risk While Maximizing Analgesia*, which promoted pseudoaddiction by teaching that a patient's aberrant behavior was the result of untreated pain. Endo substantially controlled NIPC by funding NIPC projects; developing, specifying, and reviewing content; and distributing NIPC materials;

d) Purdue published a pamphlet in 2011 entitled *Providing Relief, Preventing Abuse*, which described pseudoaddiction as a concept that "emerged in the literature" to describe the inaccurate interpretation of [drug-seeking behaviors] in patients who have pain that has not been effectively treated."; and

e) Purdue sponsored a CME program entitled *Path of the Patient, Managing Chronic Pain in Younger Adults at Risk for Abuse*. In a role play, a chronic pain patient with a history of drug abuse tells his doctor that he is taking twice as many hydrocodone pills as directed. The narrator notes that because of pseudoaddiction, the doctor should not assume the patient is addicted even if he persistently asks for a specific drug, seems desperate, hoards medicine, or "overindulges in unapproved escalating

doses." The doctor treats this patient by prescribing a high-dose, long-acting opioid.

186. The 2016 CDC Guideline rejects the concept of pseudoaddiction. The CDC Guideline nowhere recommends that opioid dosages be increased if a patient is not experiencing pain relief. To the contrary, the Guideline explains that "[p]atients who do not experience clinically meaningful pain relief early in treatment…are unlikely to experience pain relief with longer-term use,"[250] and that physicians should "reassess[] pain and function within 1 month"[251] in order to decide whether to "minimize risks of long-term opioid use by discontinuing opioids"[252] because the patient is "not receiving a clear benefit."[253]

187. ***Third,*** Defendants falsely instructed doctors and patients that addiction risk screening tools, patient contracts, urine drug screens, and similar strategies allowed them to reliably identify and safely prescribe opioids to patients predisposed to addiction. These misrepresentations were especially insidious because Defendants aimed them at general practitioners and family doctors who lack the time and expertise to closely manage higher-risk patients. Defendants' misrepresentations made these doctors feel more comfortable prescribing opioids to their patients, and patients more comfortable starting opioid therapy for chronic pain. For example:

a) Endo paid for a 2007 supplement in the *Journal of Family Practice* written by a doctor who became a member of Endo's speakers' bureau in 2010. The supplement, entitled *Pain Management Dilemmas in Primary Care: Use of Opioids*, emphasized the effectiveness of screening tools, claiming that patients at high risk of addiction could safely receive chronic opioid therapy using a "maximally structured approach" involving toxicology screens and pill counts;

[250] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[251] *Id.*
[252] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[253] *Id.*

b) Purdue sponsored a 2011 webinar, *Managing Patient's Opioid Use: Balancing the Need and Risk*, which claimed that screening tools, urine tests, and patient agreements prevent "overuse of prescriptions" and "overdose deaths;" and

c) As recently as 2015, Purdue has represented in scientific conferences that "bad apple" patients – and not opioids – are the source of the addiction crisis and that once those "bad apples" are identified, doctors can safely prescribe opioids without causing addiction.

188. Once again, the 2016 CDC Guideline confirms these representations are false. The Guideline notes that there are no studies assessing the effectiveness of risk mitigation strategies – such as screening tools, patient contracts, urine drug testing, or pill counts – widely believed by doctors to detect and deter outcomes related to addiction and overdose.[254] As a result, the Guideline recognizes that doctors should not overestimate the risk screening tools for classifying patients as high or low risk for opioid addiction because they are insufficient to rule out the risks of long-term opioid therapy.[255]

189. ***Fourth***, to underplay the risk and impact of addiction and make doctors feel more comfortable starting patients on opioids, Defendants falsely claimed that opioid dependence can easily be addressed by tapering and that opioid withdrawal is not a problem thereby failing to disclose the increased difficulty of stopping opioids after long-term use.

190. For example, a CME sponsored by Endo, entitled *Persistent Pain in the Older Adult*, claimed that withdrawal symptoms can be avoided by tapering a patient's opioid dose by 10%-20% for 10 days. And Purdue sponsored APF's *A Policymaker's Guide to Understanding Pain & Its Management*, which claimed that "[s]ymptoms of physical dependence can often be ameliorated by gradually decreasing the dose of medication during discontinuation."

[254] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[255] *Id.*

191. Defendants deceptively minimized the significant symptoms of opioid withdrawal, which, as explained in the 2016 CDC Guideline, include drug cravings, anxiety, insomnia, abdominal pain, vomiting, diarrhea, sweating, tremor, tachycardia (rapid heartbeat), spontaneous abortion and premature labor in pregnant women, and the unmasking of anxiety, depression, and addiction – and grossly understated the difficulty of tapering, particularly after long-term opioid use.

192. Yet the 2016 CDC Guideline recognizes that the duration of opioid use and the dosage of opioids prescribed should be limited to "minimize the need to taper opioids to prevent distressing or unpleasant withdrawal symptoms,"[256] because "***physical dependence on opioids is an expected physiologic response in patients exposed to opioids for more than a few days.***"[257] (Emphasis Added.) The Guideline further states that "tapering opioids can be especially challenging after years on high dosages because of physical and psychological dependence"[258] and highlights the difficulties, including the need to carefully identify "a taper slow enough to minimize symptoms and signs of opioid withdrawal"[259] and pausing and restarting tapers depending on the patient's response.

193. The CDC also acknowledges the lack of any "high-quality studies comparing the effectiveness of different tapering protocols for use when opioid dosage is reduced or opioids are discontinued."[260] Contrary to the *Treatment Options* distributed by the APF, withdrawal from opioids involves much more than mere "physical" dependence occurring only when opioids are stopped suddenly or the dose lowered too quickly.

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[256] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[257] *Id.*
[258] *Id.*
[259] *Id.*
[260] *Id.*

194. ***Fifth***, Defendants falsely claimed that doctors and patients could increase opioid dosages indefinitely without added risk and failed to disclose the greater risks to patients at higher dosages. The ability to escalate dosages was critical to Defendants' efforts to market opioids for long-term use to treat chronic pain because, absent this misrepresentation, doctors would have abandoned treatment when patients built up tolerance and lower dosages did not provide pain relief. For example:

a) Actavis's predecessor created a patient brochure for Kadian in 2007 that stated, "Over time, your body may become tolerant of your current dose. You may require a dose adjustment to get the right amount of pain relief. This is not addiction." Upon information and belief, based on Actavis's acquisition of its predecessor's marketing materials along with the rights to Kadian, Actavis continued to use these materials in 2009 and beyond;

b) Purdue sponsored *APF's Treatment Options: A Guide for People Living with Pain* (2007), which claims that some patients "need" a larger dose of an opioid, regardless of the dose currently prescribed. The guide stated that opioids have "no ceiling dose" and are therefore the most appropriate treatment for severe pain. This guide is still available for sale online;

c) Endo sponsored a website, painknowledge.com, which claimed in 2009 that opioid dosages may be increased until "you are on the right dose of medication for your pain.";

d) Endo distributed a pamphlet edited by a KOL entitled *Understanding Your Pain: Taking Oral Opioid Analgesics*, which was available during the time period of this Complaint on Endo's website. In Q&A format, it asked "If I take the opioid now, will it work later when I really need it?" The response is, "The dose can be increased. . . . You won't 'run out' of pain relief.";

e) Janssen sponsored a patient education guide entitled *Finding Relief: Pain Management for Older Adults* (2009), which was distributed by its sales force. This guide listed dosage limitations as "disadvantages" of other pain medicines but omitted any discussion of risks of increased opioid dosages;

f) Purdue's In the Face of Pain website promotes the notion that if a patient's doctor does not prescribe what, in the patient's view, is a sufficient dosage of opioids, he or she should find another doctor who will;

g) Purdue sponsored APF's *A Policymaker's Guide to Understanding Pain & Its Management*, which taught that dosage escalations are "sometimes

necessary," even unlimited ones, but did not disclose the risks from high opioid dosages. This publication is still available online;

h) Purdue sponsored a CME entitled *Overview of Management Options* that is still available for CME credit. The CME was edited by a KOL and taught that NSAIDs and other drugs, but not opioids, are unsafe at high dosages; and

i) Purdue presented a 2015 paper at the College on the Problems of Drug Dependence, the "the oldest and largest organization in the US dedicated to advancing a scientific approach to substance use and addictive disorders," challenging the correlation between opioid dosage and overdose.

195. These claims conflict with the scientific evidence, as confirmed by the FDA and CDC. As the CDC explains in its 2016 Guideline, the "[b]enefits of high-dose opioids for chronic pain are not established"[261] while the "risks for serious harms related to opioid therapy increase at higher opioid dosage."[262]

196. More specifically, the CDC explains, "there is now an established body of scientific evidence showing that overdose risk is increased at higher opioid dosages."[263] Similarly, there is an "increased risk for opioid use disorder, respiratory depression, and death at higher dosages."[264] That is why the CDC advises doctors to avoid increasing dosages above 90 morphine milligram equivalents per day.

197. ***Finally***, Defendants' deceptive marketing of the so-called abuse-deterrent properties of some of their opioids has created false impressions that these opioids reliably curb addiction and abuse.

198. More specifically, Defendants have made misleading claims about the ability of their so-called abuse-deterrent opioid formulations to deter use. For example, Endo's

[261] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[262] *Id.*
[263] *Id.*
[264] *Id.*

advertisements for the 2012 reformulation of Opana ER claimed that it was designed to be crush resistant in a way that suggested it was more difficult to misuse the product. This claim was false.

199. The FDA warned in a 2013 letter that there was no evidence Endo's design would provide a reduction in oral, intranasal or intravenous use.[265] Moreover, Endo's own studies, which it failed to disclose, showed that Opana ER could still be ground and chewed.

200. In a 2016 settlement with the State of New York, Endo agreed not to make statements in New York that Opana ER was designed to be or is crush-resistant. The State found those statements false and deceptive because there was no difference in the ability to extract the narcotic from Opana ER.

201. Similarly, the 2016 CDC Guideline states that no studies support the notion that "abuse-deterrent technologies [are] a risk mitigation strategy for deterring or preventing abuse,"[266] noting that the technologies – even when they work – "do not prevent opioid abuse through oral intake, the most common route of opioid abuse, and can still be abused by non-oral routes."[267]

202. These numerous, long-standing misrepresentations of the risks of long-term opioid use spread by Defendants successfully convinced doctors and patients to underestimate those risks.

**D. Defendants Grossly Overstated the Benefits of Chronic Opioid Therapy.**

203. To convince doctors and patients that opioids should be used to treat chronic pain, Defendants had to persuade them that there was a significant benefit to long-term opioid use. But as the 2016 CDC Guideline makes clear, there is "insufficient evidence to determine the long-term benefits of opioid therapy for chronic pain."[268]

[265] *See FDA Statement: Original Opana ER Relisting Determination*, May 10, 2013.
[266] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[267] *Id.*
[268] *Id.*

204. In fact, the CDC found no evidence showing "a long-term benefit of opioids in pain and function versus no opioids for chronic pain with outcomes examined at least 1 year later (with most placebo-controlled randomized trials ≤ 6 weeks in duration)"[269] and that other treatments were more or equally beneficial and less harmful than long-term opioid use.

205. Nonetheless, Manufacturing Defendants were legion in their misrepresentations that opioid drugs were appropriate for use as a long-term lifestyle. For example:

a) Actavis distributed an advertisement that claimed that the use of Kadian to treat chronic pain would allow patients to return to work, relieve "stress on your body and your mental health," and help patients enjoy their lives;

b) Endo distributed advertisements that claimed that the use of Opana ER for chronic pain would allow patients to perform demanding tasks like construction work or work as a chef and portrayed seemingly healthy, unimpaired subjects;

c) Janssen sponsored and edited a patient education guide entitled *Finding Relief: Pain Management for Older Adults* (2009) – which states as "a fact" that "opioids may make it easier for people to live normally." The guide lists expected functional improvements from opioid use, including sleeping through the night, returning to work, recreation, sex, walking, and climbing stairs;

d) Purdue ran a series of advertisements for OxyContin in 2012 in medical journals entitled "pain vignettes," which were case studies featuring patients with pain conditions persisting over several months and recommending OxyContin for them. The ads implied that OxyContin improves patients' function;

e) *Responsible Opioid Prescribing* (2007), sponsored and distributed by Endo and Purdue, taught that relief of pain by opioids, by itself, improved patients' function. The book remains for sale online;

f) Purdue sponsored APF's *Treatment Options: A Guide for People Living with Pain* (2007), which counseled patients that opioids "give [pain patients] a quality of life we deserve." The guide was available online until APF shut its doors in 2012;

g) Endo's NIPC website *painknowledge.com* claimed in 2009 that with opioids, "your level of function should improve; you may find you are now

[269] *Id.*

able to participate in activities of daily living, such as work and hobbies, that you were not able to enjoy when your pain was worse." Elsewhere, the website promoted improved quality of life (as well as "improved function") as benefits of opioid therapy. The grant request that Endo approved for this project specifically indicated NIPC's intent to make misleading claims about function, and Endo closely tracked visits to the site;

h) Endo was the sole sponsor, through NIPC, of a series of CMEs titled *Persistent Pain in the Older Patient*, which claimed that chronic opioid therapy has been "shown to reduce pain and improve depressive symptoms and cognitive functioning." The CME was disseminated via webcast;

i) Janssen sponsored, funded, and edited a website, *Let's Talk Pain*, in 2009, which featured an interview edited by Janssen claiming that opioids allowed a patient to "continue to function." This video is still available today on YouTube;

j) Purdue sponsored the development and distribution of APF's *A Policymaker's Guide to Understanding Pain & Its Management*, which claimed that "multiple clinical studies" have shown that opioids are effective in improving daily function, psychological health, and health-related quality of life for chronic pain patients." The Policymaker's Guide was originally published in 2011 and is still available online today; and

k) Purdue's, Endo's, and Janssen's sales representatives have conveyed and continue to convey the message that opioids will improve patient function.

206. These claims are unsupported by the scientific literature. The 2016 CDC Guideline explained, "There is no good evidence that opioids improve pain or function with long-term use"[270] and "complete relief of pain is unlikely."[271] The CDC reinforced this conclusion throughout its 2016 Guideline:

a) "No evidence shows a long-term benefit of opioids in pain and function versus no opioids for chronic pain with outcomes examined at least 1 year later . . . .";[272]

b) "Although opioids can reduce pain during short-term use, the clinical evidence review found insufficient evidence to determine whether pain relief is sustained and whether function or quality of life improves with

[270] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[271] *Id.* (emphasis added).
[272] *Id.*

long-term opioid therapy.";[273] and

c) "[E]vidence is limited or insufficient for improved pain or function with long-term use of opioids for several chronic pain conditions for which opioids are commonly prescribed, such as low back pain, headache, and fibromyalgia."[274]

207. The CDC also noted that the risks of addiction and death "can cause distress and inability to fulfill major role obligations."[275]

208. Defendants also falsely emphasized or exaggerated the risks of competing products like NSAIDs so that doctors and patients would look to opioids first for treating chronic pain. Once again, Defendants' misrepresentations contradicted non-industry sponsored scientific evidence. In addition, Purdue misleadingly promoted OxyContin as unique among opioids in providing 12 continuous hours of pain relief with one dose. In fact, OxyContin does not last for 12 hours – a fact that Purdue has known at all times relevant to this action.

209. According to Purdue's own research, OxyContin wears off in under six hours in one quarter of patients and in under 10 hours in more than half. The reason is that OxyContin tablets release approximately 40% of their active medicine immediately, after which release tapers. Although the patient experiences a powerful initial response, there is little or no pain relief at the end of the dosing period because less medicine is released.

210. This phenomenon is known as "end of dose" failure, and the FDA found in 2008 that a substantial number of chronic pain patients taking OxyContin experience it.

211. This "end of dose" failure not only renders Purdue's promise of 12 hours of relief false and deceptive, it also makes OxyContin more dangerous because the declining pain relief patients experience toward the end of each dosing period drives them to take more OxyContin

[273] *Id.*
[274] *Id.*
[275] *Id.*

before the next dosing period begins, quickly increasing the amount of drug they are taking and spurring growing dependence.

212. Purdue's competitors were aware of this problem. For example, Endo ran advertisements for Opana ER referring to "real" 12-hour dosing. Nevertheless, Purdue falsely promoted OxyContin as if it were effective for a full 12 hours. Indeed, Purdue's sales representatives continue to tell doctors in and around Hopkins County that OxyContin lasts a full 12 hours.

**E. Defendants also engaged in Other Unlawful, Unfair, and Fraudulent Misconduct.**

213. Other Defendants herein participated in illicit and unlawful prescribing of its drugs. For example, Purdue did not report illegal prescribing of OxyContin until years after law enforcement shut down a Los Angeles clinic that prescribed more than 1.1 million OxyContin tablets. In doing so, Purdue protected its own profits at the expense of public health and safety.

214. The State of New York also found that Endo failed to require sales representatives to report signs of addiction, diversion, and inappropriate prescribing; paid bonuses to sales representatives for detailing prescribers who were subsequently arrested or convicted for illegal prescribing; and failed to prevent sales representatives from visiting prescribers whose suspicious conduct had caused them to be placed on a no-call list.

**F. Defendants Targeted Susceptible Prescribers and Vulnerable Patient Populations.**

215. As part of their deceptive marketing scheme, Defendants identified and targeted susceptible prescribers and vulnerable patient populations in the U.S. and in and around Hopkins County. For example, Defendants focused their deceptive marketing on primary care doctors, who were more likely to treat chronic pain patients and prescribe opioids, but were less likely to be educated about treating pain and the risks and benefits of opioids.

216. Defendants also targeted vulnerable patient populations like the elderly and veterans, who tend to suffer from chronic pain. Defendants targeted these vulnerable patients even though the risks of long-term opioid use were significantly greater for them.

217. For example, the 2016 CDC Guideline observes that existing evidence shows that elderly patients taking opioids suffer from elevated fall and fracture risks, greater risk of hospitalization, and increased vulnerability to adverse drug effects and interactions. The Guideline therefore concludes that there are "special risks of long-term opioid use for elderly patients" and recommends that doctors use "additional caution and increased monitoring" to minimize the risks of opioid use in elderly patients.

218. The same is true for veterans, who are more likely to use anti-anxiety drugs (benzodiazepines) for post-traumatic stress disorder, which interact dangerously with opioids.

219. Manufacturing Defendants achieved their goal in targeting these vulnerable populations when the Arthritis Foundation published its *Guide to Pain Management* in 2003 ("*Pain Management Guide*").[276] The *Pain Management Guide* was published by a neutral third-party that not only believed the message Defendants had been selling for years, but it continued to relay that message to patients experiencing chronic pain – elderly patients with arthritis.[277]

220. The *Pain Management Guide* was intended for a population of "70 million Americans who have arthritis or other related diseases."[278] It parroted falsities, such as the low risk of developing an addiction to opioids and cited Defendants' false statistic: "The addiction rate from narcotics is approximately one percent."[279]

[276] Susan Bernstein, *The Arthritis Foundation's Guide to Pain Management*, Arthritis Foundation, 2003.
[277] *Id.*
[278] *Id.*
[279] *Id.* at 70-71.

221. The Arthritis Foundation even accepted and repeated Defendants' distinction between dependence and addiction. A person with dependence suggests he or she would experience withdrawal symptoms upon stopping opioids while addiction "is a self-destructive, habitual use" of opioids.[280] The *Pain Management Guide* brushes aside concerns about addiction and recommends higher doses of opioids for patients who develop a dependence on opioids[281] – the exact message that Defendants had been spouting for years.

222. The fact that neutral third parties were relying on and buying Defendants' false propositions only verifies Defendants' successful fraud on the medical and non-medical community at large.

## G. Although Defendants knew that their Marketing of Opioids was False and Deceptive, they Fraudulently Concealed their Misconduct.

223. Defendants, both individually and collectively, made, promoted, and profited from their misrepresentations about the risks and benefits of opioids for chronic pain even though they knew their misrepresentations were false and deceptive. Manufacturing Defendants and Distributor Defendants alike knew that the marketing scheme being promoted by the Manufacturer Defendants was misleading, inaccurate, and simply false. The history of opioids, as well as research and clinical experience over the last 20 years, established that opioids were highly addictive and responsible for a long list of very serious adverse outcomes.

224. In The Journal of the American Medical Association November 2002 edition, which Defendants meant to reach physicians throughout the nation, Purdue advertised OxyContin as a safe drug with minimal safety risks.[282] The ad depicts a man and boy fishing with a title in large white letters exclaiming that "THERE CAN BE LIFE WITH RELIEF" with "LIFE WITH

[280] *Id.* at 70.
[281] *Id.*
[282] The Journal of American Medical Association, Nov. 13, 2002.

RELIEF" as the largest words in the advertisement.[283] Purdue then informs physicians that "[t]he most serious risk associated with opioids, including OxyContin, is respiratory depression."[284]

225. Purdue fraudulently represented that respiratory depression was not only the most serious risk for its own drug OxyContin, but for opioids in general, even though it knew that opioids carried a risk of addiction and death.

226. The ad continues with benign side effects that may occur with the use of OxyContin, such as "constipation, nausea, sedation, dizziness, vomiting, pruritus, headache, dry mouth, sweating, and weakness."[285] These side effects are certainly a far cry from addiction or death. Of course this ad also claims that OxyContin is a "continuous around-the-clock analgesic," which is equally false.[286]

227. Because of the bold misrepresentations and omissions in its ads occurring in the October 2, 2002 JAMA issue, and one occurring in the November 13, 2002 issue, the FDA wrote a warning letter to Michael Friedman, the Executive Vice President and Chief Operating Officer of Purdue.[287] Mr. Abrams explained that "[y]our journal advertisements omit and minimize the serious safety risks associated with OxyContin, and promote it for uses beyond which have been proven safe and effective."[288] Mr. Abrams reprimanded Purdue for failing to present "any information" in the advertisement about the "potentially fatal risks" or the potential for abuse associated with OxyContin.[289]

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[283] *Id.* at 1, 3.
[284] *Id.*
[285] *Id.*
[286] JAMA, *supra*, at 1, 3.
[287] Warning Letter from Thomas Abrams, Dir., FDA Div. of Mktg., Adver., & Commc'ns, to Michael Friedman, Exec. Vice Pres. and COO, Purdue Pharma L.P.
[288] *Id.* at 1.
[289] *Id.*

228. Mr. Abrams was concerned that these advertisements suggested such a "broad use of [OxyContin] to treat pain without disclosing the potential for abuse with the drug and the serious, potentially fatal risks associated with its use…"[290] Purdue's actions were "especially egregious and alarming" given "its potential impact on the public health."[291] Mr. Abrams pointed out to Purdue the reality that "[i]t is particularly disturbing that your November Ad would tout 'Life with Relief,' yet fail to warn that patients can die from taking OxyContin."[292]

229. Not surprisingly, three current and former executives from Purdue plead guilty in 2007 to criminal charges that they misled regulators, doctors, and patients about OxyContin's risk of addiction.[293] In pleading guilty to misbranding charges, Purdue admitted it had fraudulently marketed OxyContin as a drug less prone to addiction and as having fewer side effects than other opioids.[294] In reality, unlike most other opioids, OxyContin contained pure oxycodone without any other ingredients, which made it a higher-dose narcotic despite its time-release design that Purdue hawked as ameliorating its addictive potential.[295]

230. Manufacturing Defendants avoided detection of their fraudulent conduct by disguising their role in the deceptive marketing through funding and using third parties, such as Front Groups and KOLs. Doctors and patients trusted these third parties and did not realize that it was the pharmaceutical companies that were actually feeding them false and misleading information.

231. Defendants also manipulated their promotional materials and the scientific literature to make it appear that the information promoted was accurate, truthful, and supported by

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[290] *Id.* at 2.
[291] *Id.*
[292] *Id.* at 4.
[293] *See* Barry Meier, *In Guilty Plea, OxyContin Maker to Pay $600 Million*, May 10, 2007, *available at* http://www.nytimes.com/2007/05/10/business/11drug-web.html; *see also* Zee, Ex. B, at 3-4.
[294] *See* Meier, *supra.*
[295] *See id.*

objective evidence when it was not.

232. Thus, Defendants successfully concealed from the medical community and patients facts sufficient to arouse suspicion of the claims Hopkins County now asserts. Hopkins County did not know of the existence or scope of Defendants' industry-wide fraud and could not have acquired such knowledge earlier through the exercise of reasonable diligence.

## H. By Increasing Opioid Prescriptions and Use, Defendants' Deceptive Marketing Scheme has fueled the Opioid Epidemic and Damaged Hopkins County Communities.

233. Defendants' misrepresentations deceived doctors and patients about the risks and benefits of long-term opioid use. Studies reveal that many doctors and patients are unaware of or do not understand the risks or benefits of opioids. Indeed, patients often report that they were not warned they might become addicted to opioids prescribed to them. As reported in January 2016, a 2015 survey of more than 1,000 opioid patients found that 4 out of 10 were not told opioids were potentially addictive.[296]

234. Defendants' deceptive marketing scheme caused, and continues to cause, doctors in and around Hopkins County to prescribe opioids for chronic pain conditions such as back pain, headaches, arthritis, and fibromyalgia. Absent Defendants' fraud, these doctors would not have prescribed as many opioids that negatively impacted residents of Hopkins County.

235. Defendants' deceptive marketing scheme allowed Dr. Howard Gregg Diamond to promote, over-prescribe, and financially benefit from prescribing opioids for to patients complaining of chronic pain.

236. Dr. Diamond used the glut of opioids flooding the market to assist him in over-prescribing controlled substances. These opioids included fentanyl, morphine, oxycodone,

[296] Hazelden Betty Ford Foundation, *Missed Questions, Missed Opportunities*, Jan. 27, 2016, *available at* http://www.hazeldenbettyford.org/about-us/news-and-media/pressrelease/doctors-missing-questions-that-could-prevent-opioid-addiction.

oxymorphone, methadone, hydrocodone, alprazolam, and zolpidem, which he routinely prescribed without "a legitimate medical purpose."[297]

237. Dr. Diamond was the principal physician at the Diamondback Pain and Wellness Center in Sherman, Texas, which served residents of Hopkins County. Dr. Diamond was arrested on July 11, 2017 on charges including "conspiracy to distribute controlled substances, possession with intent to distribute controlled substances, health care fraud, and money laundering."[298]

238. The controlled substances Dr. Diamond prescribed included opioids and, unfortunately, his prescriptions have been linked to at least seven opioid overdose deaths, one of which is in Hopkins County.[299]

239. Dr. Diamond's license was suspended on July 20, 2017 as his practice of medicine posed "a continuing threat to public welfare" and was effective immediately.[300] Dr. Diamond remains in jail while he is awaiting trial.[301] Dr. Diamond sought to be released pending trial, but the judge denied his request citing testimony that "even after federal agents executed search warrants at [Dr. Diamond's] Diamondback Pain and Wellness Centers in Sherman and Paris on May 19, he prescribed almost 278,000 dosage units."[302] Even after his arrest, according to a DEA agent's testimony, Dr. Diamond "filled at least seven new prescriptions."[303]

240. Dr. Diamond has entered a not guilty plea to all charges. If convicted, he faces up to

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[297] No. 4:17 CR 118, "*United States of America v. Howard Gregg Diamond,"* Indictment filed in the U.S. Eastern District of Texas, Jul. 6, 2017, at 5.
[298] Emma Ruby, *Texas Suspends Doctor's License after 7 Drug Deaths, including McKinney Overdose,* Dallas Morning News, Jul. 21, 2012, *available at* http://www.dallasnews.com/news/crime/2017/07/12/texas-doctor-accused-prescribing-unnecessary-drugs-led-7-deaths.
[299] Walt Zwirko, *Judge: No Freedom for Texoma Doctor Linked to Overdose Deaths*, KTEN, Jul. 24, 2017, *available at* http://www.kten.com/story/35959439/judge-no-freedom-for-texoma-doctor-linked-to-seven-overdose-deaths.
[300] Texas Medical Board, http://reg. tmb/state/tx/us/OnLineVerif/Phy_ReportVerif_new.asp.
[301] Zwirko, *supra*.
[302] *Id.*
[303] *Id.*

life in prison.[304] Prosecutors are also seeking to have Dr. Diamond's medical license revoked.[305]

241. If the Manufacturing and Distributing Defendants were not over-supplying opioids, then physicians like Dr. Diamond could not devise schemes to prescribe opioids without a legitimate purpose as a means to flood the open market with opioids, such as OxyContin, Hydrocodone, and Vicodin.

242. Defendants' deceptive marketing scheme also caused, and continues to cause, patients to purchase and use opioids for their chronic pain believing they are safe and effective. Absent Defendants' deceptive marketing scheme, fewer patients would be using opioids long-term to treat chronic pain, and those patients using opioids would be using less of them.

243. Defendants' deceptive marketing has caused and continues to cause the prescription and use of opioids to explode. Indeed, this dramatic increase in opioid prescriptions and use corresponds with the dramatic increase in Defendants' spending on their deceptive marketing scheme. Defendants' spending on opioid marketing totaled approximately $91 million in 2000. By 2011, that spending had tripled to $288 million.

244. The escalating number of opioid prescriptions written by doctors who were deceived by Defendants' deceptive marketing scheme is the cause of a correspondingly dramatic increase in opioid addiction, overdose, and death throughout the U.S. and Hopkins County. The increase in opioid prescriptions equals an increase in "disability, medical costs, subsequent surgery, and continued or late opioid use."[306]

245. Scientific evidence demonstrates a strong correlation between opioid prescriptions and addiction to opioids. In a 2016 report, the CDC explained that prescribing opioids has

[304] *Id.*
[305] Ruby, *supra.*
[306] Manchikanti, Ex. A, at 23.

quadrupled since 1999, which has resulted in a parallel increase in opioid overdoses.[307] Indeed, there has been a two-third increase in overdose deaths from using opioids since 2000.[308] For these reasons, the CDC concluded that efforts to rein in the prescribing of opioids for chronic pain are critical "to reverse the cycle of opioid pain medication misuse that contributes to the opioid overdose epidemic."[309]

246. Due to the increase in opioid overdoses, first responders, such as police officers, have been and will continue to be in the position to assist people experiencing opioid-related overdoses.[310] In 2016, "over 1,200 law enforcement departments nationwide carried naloxone in an effort to prevent opioid-related deaths."[311]

247. Defendants' deceptive marketing scheme has also detrimentally impacted children in Hopkins County. Overprescribing opioids for chronic pain has made the drugs more accessible to school-aged children, who come into contact with opioids after they have been prescribed to friends or relatives in the same household.

248. Defendants' conduct has adversely affected Hopkins County's child protection agencies in the number of children in foster care driven by parental drug addiction. Children with parents addicted to drugs tend to stay in foster care longer, and they often enter the system having experienced significant trauma, which makes these cases more expensive for counties like Hopkins County.

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[307] CDC/NCHS, *National Vital Statistics System, Mortality*, CDC Wonder, Atlanta, GA: US Department of Health and Human Services, 2016, *available at* https://wonder.cdc.gov/; Rudd RA, Seth P, David F, Scholl L, *Increases in Drug and Opioid-Involved Overdose Deaths — United States, 2010–2015*, Morb Mortal Wkly Rep., Dec. 16, 2016.
[308] CDC, *National Vital Statistics System*, *Mortality*, Morb Mortal Wkly Rep., Jan. 1, 2006, at 1378-82, Increases in Drug and Opioid Deaths – United States, 2000-2014.
[309] *CDC Guideline for Prescribing Opioids for Chronic Pain, supra; see also* Rudd, *supra*.
[310] Tex. Att'y Gen. Op. No. KP-0168 (2017).
[311] *Id. citing* http://www.nchrc.org/law-enforcement/us-law-enforcement-who-carry-naloxone/.

249. Opioid addiction is one of the primary reasons that Hopkins County residents seek treatment for substance dependence. A significant number of admissions for drug addiction were associated with a primary diagnosis of opiate addiction or dependence.

250. Defendants' creation, through false and deceptive advertising and other unlawful and unfair conduct, of a virtually limitless opioid market has significantly harmed Hopkins County communities. Defendants' success in extending the market for opioids to new patients and chronic pain conditions has created an abundance of drugs available for non-medical and criminal use and fueled a new wave of addiction and injury. It has been estimated that 60% of the opioids to which people are addicted come, directly or indirectly, through doctors' prescriptions.[312]

251. In Hopkins County, a woman called Amber became addicted to opioids after gall bladder surgery and later became a user of methamphetamine and crystal methamphetamines, after she could no longer afford doctors or prescription opioids.[313] The opioids never cured her pain, but were addictive and destructive.[314] Similarly, a young man named Kevin Roberts became addicted to hydrocodone, and later turned to illegal drugs, after having two knee surgeries in his early teens.[315] He was given the pills to "feel better" and never told that they were addictive.[316]

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[312] Nathaniel P. Katz, *Prescription Opioid Abuse: Challenges and Opportunities for Payers*, Am. J. Managed Care, Apr. 19 2013, at 5 ("The most common source of abused [opioids] is, directly or indirectly, by prescription."), *available at* http://www.ajmc.com/publications/issue/2013/2013-1-vol19-n4/Prescription-Opioid-Abuse-Challenges-and-Opportunities-for-Payers.

[313] Kerry Craig, *PANDEMIC - Opioid Addiction Results in one Woman's Daily Struggle*, Sulphur Springs News-Telegram, Oct. 7, 2017, *available at* https://www.ssnewstelegram.com/news/opioid-addiction-results-in-one-woman-s-daily-struggle/article_bded4eoa-ab80-11e7-a252-d3f304e26628.html. Amber used opioids after gall bladder surgery and was told to drink a lot of water without being told the risks of addition. *Id.* Amber's doctor continued prescribing opioids due to Amber's physical issues. *Id.* Amber became so addicted, she wrote hot check to see the doctor and to get her opioids. *Id.* Luckily Amber has been "clean" for almost five years. *Id.*

[314] Craig, *Pandemic*, *supra*.

[315] Kerry Craig, *ADDICTION – One Man's Personal Struggle with Prescriptions and More*, Sulphur Springs News-Telegram, Dec. 2, 2017 (insisting that his real name be used to help others avoid the things he experienced due to addiction).

[316] *Id.* (stating that he was told to take the pills until they were gone but not told about the effects of taking them).

252. Law enforcement agencies have increasingly associated prescription drug addiction with violent and property crimes. Despite strict federal regulation of prescription drugs, local law enforcement agencies are faced with increasing diversion from legitimate sources for illicit purposes, including doctor shopping, forged prescriptions, falsified pharmacy records, and employees who steal from their place of employment. The opioid epidemic has prompted a growing trend of crimes against pharmacies including robbery and burglary. This ongoing diversion of prescription narcotics creates a lucrative marketplace.

253. In Hopkins County, the illegal activity even struck a former police officer and sheriff's deputy who entered guilty pleas in 2014 to charges of theft of property, among other things, after stealing hydrocodone pills from evidence.[317] A second officer, a criminal investigator for Hopkins County, was also arrested and charged with tampering with evidence related to the same missing hydrocodone pills.[318]

254. The rise in opioid addiction caused by Defendants' deceptive marketing scheme has also resulted in an explosion in heroin use. For example, heroin use has more than doubled in the past decade among adults aged 18 to 25 years.[319] Moreover, heroin-related overdoses in the United States has more than quadrupled since 2010.[320]

255. The costs and consequences of opioid addiction are staggering. For example, in 2007, the cost of healthcare due to opioid addiction and dependence was estimated at 25 billion, the cost of criminal justice was estimated at 5.1 billion, and the cost of lost workplace productivity was estimated at 25.6 billion.

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[317] Kerry Craig, *Former Lawman Pleads to Two Felony Charges*, Sulphur Springs News-Telegram, Sept. 29, 2014.
[318] Kerry Craig, *Deputy Arrested on Felony Charges*, Sulphur Springs News-Telegram, Oct. 23, 2014.
[319] Centers for Disease Control and Prevention, *Vital Signs: Today's Heroin Epidemic – More People at Risk, Multiple Drugs Abused*, MMWR 2015, *available at* https://www.cdc.gov/vitalsigns/heroin/index.html.
[320] *Id.*

256. Texas had the second highest healthcare costs in 2015 from opioid abuse in the nation totaling $1.96 billion.[321] One in five Texas high school students has taken prescription drugs without a valid prescription.[322] And four of the top 25 cities for abuse in the United States – two of them located in East Texas – is in Texas.[323]

257. Prescription opioid addiction and overdose have an enormous impact on the health and safety of individuals, as well as communities at large, because the consequences of this epidemic reach far beyond the addicted individual.

258. Hopkins County has also expended funds for false claims submitted on the County's health plans that were paid as medically necessary when they were not and prescriptions for opioids through worker's compensation benefits.

259. The repercussions for residents of Hopkins County therefore include job loss, loss of custody of children, physical and mental health problems, homelessness and incarceration, which results in instability in communities often already in economic crisis and contributes to increased demand on community services such as hospitals, courts, child services, treatment centers, and law enforcement. Manufacturing Defendants knew, and should have known, about the harms that their deceptive marketing has caused, and continues to cause, and will cause in the future. Manufacturing Defendants closely monitored their sales and the habits of prescribing doctors. Their sales representatives, who visited doctors and attended CMEs, knew which doctors were receiving their messages and how they were responding.

260. Manufacturing Defendants also had access to and carefully watched government and other data that tracked the explosive rise in opioid use, addiction, injury, and death.

[321] Craig, *Pandemic*, *supra*.
[322] *Id.*
[323] *Id.*

Manufacturing Defendants not only knew, but intended that their misrepresentations would persuade doctors to prescribe and encourage patients to use their opioids for chronic pain.

261. Manufacturing Defendants' actions are neither permitted nor excused by the fact that their drug labels may have allowed, or did not exclude, the use of opioids for chronic pain. FDA approval of opioids for certain uses did not give Defendants license to misrepresent the risks and benefits of opioids. Indeed, Manufacturing Defendants' misrepresentations were directly contrary to pronouncements by, and guidance from, the FDA based on the medical evidence and their own labels.

262. Nor is Manufacturing Defendants' causal role broken by the involvement of doctors. Defendants' marketing efforts were ubiquitous and highly persuasive. Their deceptive messages tainted virtually every source doctors could rely on for information and prevented them from making informed decisions. And both doctors and patients in Hopkins County relied on information Manufacturing Defendants distributed whether it was through ads, magazines, trade journals, websites, CMEs, KOLs, and/or front groups. Manufacturing Defendants also hijacked what doctors wanted to believe – namely, that opioids represented a means of relieving their patients' suffering and of practicing medicine more compassionately.

263. Likewise, Distributor Defendants knew when there was suspicious opioid prescription activity as they do today. They were in a unique position to forestall the epidemic in Hopkins County before it began. Instead, Distributor Defendants allowed a flood of opioids to be poured into Hopkins County. Hopkins County relied on Distributor Defendants to prevent oversupply with distribute prescription drugs, including opioids, only if a valid medical purpose existed. At the very least, Hopkins County depended on Distributor Defendants to act as watchful and effective gatekeepers in the opioid pipeline as they represent in their public statements.

Distributor Defendants did neither to Hopkins County's detriment, proximately causing damage to Hopkins County.

264. The funds that Hopkins County has used and will continue to use for all the costs associated with Defendants' false, misleading, and fraudulent marketing are taxpayer funds. Defendants specifically targeted physicians in Hopkins County with fraudulent claims concerning the benefits of opioids for chronic pain while omitting the lack of efficacy.

265. Defendants also fraudulently omitted the fact that opioids were addictive even though they knew, or should have known, that physicians in Hopkins County would either use the misinformation Defendants relayed to them to prescribe opioids to Hopkins County residents or give this information to Hopkins County residents, resulting in the over-prescribing and/or overuse of opioids in Hopkins County.

266. Defendants' actions and omissions were each a cause-in-fact of Hopkins County's past and future damages. Defendants' wrongful conduct caused injuries to Hopkins County in the past, continues to cause injuries to Hopkins County, and will continue to cause injuries to Hopkins County in the future. Future damages include, but are not limited to, additional resources for counseling and medication assisted treatment of addicts, medical treatment for overdoses, life skills training for adolescents, increased law enforcement, and additional resources to treat the psychological effects of opioids and the underlying conditions that make people susceptible to opioid addiction, all of which will be obtained through taxpayer resources.

## I. Defendants' Fraudulent Marketing Has Led To Record Profits.

267. While using opioids has taken an enormous toll on Hopkins County and its residents, Defendants have realized blockbuster profits. In 2014 alone, opioids generated $11 billion in revenue for drug companies like Defendants. Indeed, financial information indicates that

each Defendant experienced a material increase in sales, revenue, and profits from the false and deceptive advertising and other unlawful and unfair conduct described above.

## J. Distributor Defendants Knew that Opioids were being Fraudulently Prescribed and Failed to Act.

268. Distributor Defendants are not innocent sellers of opioid drugs. Distributor Defendants knew that the marketing scheme promoted by Manufacturing Defendants was misleading, false, and deceptive. They knew that opioids were being industry promoted for the treatment of virtually any complaint of recurrent pain, and advertised as less addictive, less prone to abuse, less threatening for overdose, and more effective for perpetual use than was true. Nevertheless, they have deliberately shirked their duties to monitor suspiciously high prescription patterns, and have continued to feed the over-prescribing of opioid drugs. Distributor Defendants have long been aware of an opioid overuse epidemic in America, in Texas, and in Hopkins County, but chose in each instance to profit by stoking those epidemics with more opioids. Distributor Defendants knew that opioids were too often being prescribed without legitimate therapeutic purpose, but continued to inundate the market with opioids. Distributor Defendants were and continue to be an integral part of the Hopkins County opioid epidemic.

269. As early as 2008, Distributor Defendants knew there was an opioid crisis and they were failing in their "critical role" in the supply chain to change or decrease the number of opioids being distributed into the market. McKesson paid a $13.25 million fine to settle claims regarding suspicious orders of opioids from internet pharmacies in 2008.[324] Cardinal paid a $34 million fine for failing regarding suspicious distribution of hydrocodone in 2008.[325] Cardinal also had to close down its Lakeland, Florida warehouse because it turned a blind eye to the abundance of opioids

[324] Eric Eyre, *"Suspicious" Drug Order Rules Never Enforced by State*, Charleston Gazette Mail, Dec. 18, 2016, *available at* www.wvgazettemail.com.
[325] *Id.*

funneling through only four pharmacies.[326] As such, McKesson and Cardinal knew by 2008, that their opioids were too often being oversupplied and distributed and, upon information and belief, that suspicion for diversionary purposes existed. This knowledge should have caused McKesson and Cardinal to better perform their duties to monitor non-therapeutic opioid prescription orders and to refrain from filling these orders as they occurred in Hopkins County. But instead McKesson and Cardinal doubled-down on profiting from an opioid epidemic, and did so in the Hopkins County opioid epidemic.

270. No later than 2011, all Distributor Defendants knew there was a public health crisis throughout America created by opioid use. In 2011, the CDC announced that very thing. And in 2017, the Sulphur Spring News-Telegram announced a "PANDEMIC" of opioid addiction in America that included Hopkins County.[327]

271. Texas law specifically requires that dispensers like Distributor Defendants monitor opioid prescription orders and to refuse to fill prescription orders for opioids that are without valid medical purpose. In spite of the existence of an opioid epidemic in Hopkins County, and the fact that Distributor Defendants knew or should have known of that epidemic, Distributor Defendants continued to fill each opioid prescription order in Hopkins County, including those that were without valid medical purpose--thus stoking the epidemic.[328]

272. In 2017, Distributor Defendant McKesson publicly acknowledged its essential duty to monitor and to curb to therapeutic levels its distribution of opioid drugs. The chairman and chief executive officer of McKesson, John H. Hammergren, has said: "Pharmaceutical distributors play

[326] *See* Eyre, *supra*.
[327] Kerry Craig, *PANDEMIC - Opioid Addiction Results in one Woman's Daily Struggle*, Sulphur Springs News-Telegram, Oct. 7, 2017, *available at* https://www.ssnewstelegram.com/news/opioid-addiction-results-in-one-woman-s-daily-struggle/article_bded4eoa-ab80-11e7-a252-d3f304e26628.html.
[328] CDC, *Prescription Painkiller Overdoses at Epidemic Levels*, Nov. 1, 2011, www.cdc.gov.

an important role in identifying and combatting prescription drug diversion and abuse… McKesson, as the nation's largest distributors, takes our role seriously."[329]

273. In an October 31, 2017 letter to Chris Christie, Chair of the President's Commission on Fighting the Drug and Opioid Crisis, Mr. Pete Slone, the Senior Vice President, Public Affairs, of McKesson admitted that the opioid crisis was the "public health crisis of our times" and was affecting communities at alarming rates. [330] Mr. Slone stated that both manufacturers and distributors should address the complicated opioid heath crisis.[331] Certainly in public McKesson admits its responsibility and duty to the public with regard to distributing and disbursing opioids. It is only when McKesson is named in a lawsuit that it is suddenly blameless and has no responsibilities in stemming the tide of the opioid epidemic it helps fuel.

274. Amerisource agrees it has the same essential duty to monitor and to refrain from supplying suspicious opioid prescribing. On December 14, 2017, a press release announced that Amerisource, as a global healthcare solutions leader, "plays a critical role in the pharmaceutical supply chain, working as a link between manufacturers and healthcare providers to help patients have access to the medications they ***need,*** when they ***need*** them."[332] (Emphasis added).

275. Indeed, Amerisource's website is dedicated to its role as the "core strength" in U.S. drug distribution citing its "[t]remendous cash generation" and its "[d]iverse base of high quality provider customers."[333] Recognizing that the opioid epidemic could strike as many as 650,000 Americans over the next decade, Amersource's CEO, Steve Collis, has assured the public that

[329] Charles Ornstein, *Drug Distributors Penalized for Turning Blind Eye in Opioid Epidemic*, APR, Health News, Jan. 27, 2017; Letter from Pete Slone, Senior Vice President, Public Affairs, of McKesson, to The Honorable Chris Christie dated October 31, 2017.
[330] Pete Slone Letter, *supra*.
[331] *Id.*
[332] AmerisourceBergen Foundation, *AmerisourceBergen Foundation Launches Municipal Support Program to Help Combat Opioid Abuse*, Dec. 14, 2017 press release.
[333] Amerisource Bergen, *Distributor's Duty*, www.amerisourcebergen.com.

distributors like Amerisource are responsible for safely delivering medication to pharmacies given its "unique perspective into how [the] supply chain works."[334]

276. Mr. Collis agrees with people who are "rightfully demanding action on this tragic issue" and agrees to "push forward practical solutions that can yield results in the near-term on opioids."[335] Remarking that it is "difficult to avoid the epidemic of opioid abuse," Mr. Collis adds that the opioid crisis is "demands attention, action, and accountability."[336] Mr. Collis explains that large pharmaceutical distribution companies, of which Amerisource is one, should be held accountable because "nearly every prescription in the United States moves through distributors who purchase drugs from pharmaceutical manufacturers and sell them to pharmacies . . . ."[337] Mr. Collis explains that distributors like Amerisource "must create a supply chain that is safe and secure."[338] Mr. Collis even admits that as more opioid-based pain treatments were prescribed, more opioids were distributed.[339] The same views are expressed on Amerisource's website.[340]

277. On its website, Amerisource demonstrates that Distributor Defendants' ability to create a safe and secure supply chain is possible through "complex algorithms to identify and stop orders that are deemed to be suspicious."[341] Mr. Collis admits that Amerisource has "reported and stopped ***tens of thousands*** of suspicious orders since 2007, not to mention countless other orders that pharmacies never had the opportunity to place because [Amerisource] declined to service them altogether."[342] But not in Hopkins County.

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[334] Steven H. Collis, *Sound Policy and More Transparency can Help Companies Fight the Opioid Crisis*, Politics, Dec. 15, 2017.
[335] *Id.*
[336] Steve Collis, *The Surprising Morality of Opioid Distribution*, Sept. 18, 2017, *available at* https://www.amerisourcebergen.com/abcnew/fighting-the-opioid-epidemic.
[337] *Id.*
[338] *Id.*
[339] *Id.*
[340] https://www.amerisourcebergen.com/abcnew/fighting-the-opioid-epidemic.
[341] *Id.*
[342] *Id.* (emphasis added).

278. Cardinal acknowledges on its website that the "opioid abuse crises dates back decades" and is due to "changes in prescribing patterns" for pain.[343] Like McKesson, Cardinal admits that the opioid epidemic is a "serious and complex" public health issue.[344]

279. Cardinal has been aware of this public health crisis for many years because it tracks and reports CDC opioid prescription and overdose death data. For example, Cardinal cites the fact that in 2015, 4.4 billion opioid prescriptions were filled, which equals about 12 prescriptions per person in the United States.[345] Out of these prescriptions, 11% were prescribed following surgery, injury, or for health conditions such as cancer.[346] Moreover, Cardinal recites that 1 in 4 patients using opioids long-term in a primary care situation struggle with opioid addiction.[347] Cardinal is also aware that in 2015, according to the CDC, there were more than 33,000 deaths that involved prescribed and non-prescribed opioids, and that opioid overdoses have quadrupled since 1999.[348] After tracking the opioid prescription data, Cardinal cannot now claim ignorance of the fact that Hopkins County's opioid prescription rate was approximately twice that of the Texas opioid prescription rate.

280. To deny such knowledge would be to deny Cardinal's role. Cardinal is part of a "multi-faceted and highly regulated healthcare system" in which "everyone in that chain, including [Cardinal] must do their part."[349] Cardinal believes it has a responsibility to provide a safe and secure channel to deliver medications, such as opioids.[350] This belief is underscored by Cardinal's Opioid Acton Program: Reclaiming Our Communities in which Cardinal states on its website that

[343] Cardinal Health, *No Demographic Group is Immune to this Crisis,* www.cardinalheatlh.com.
[344] *Id.*
[345] *Id.*
[346] *Id.*
[347] *Id.*
[348] *Id.*
[349] Cardinal Health, *Cardinal Health's Commitment to Opioid Anti-Diversion, Education and Misuse Prevention,* www.cardinalhealth.com.
[350] Cardinal Health, *No Demographic Group, supra.*

it operates a "state-of-the-art, constantly adaptive system to combat opioid diversion."[351] Cardinal claims it knows its pharmacy customers and uses a "multi-factor process to evaluate pharmacies" even before the pharmacies become Cardinal's customers.[352] To identify diversion and flag suspicious activity, Cardinal "engage[s] directly with pharmacists to understand their business, their purchasing patterns, the ration of controlled to non-controlled substances ordered and the demographics of their customers."[353] As a result, not only could Cardinal identify the suspicious ordering patterns in Hopkins County, it had the power to stop the influx of opioids into Hopkins County.

281. Cardinal electronically monitors every order prior to fulfillment, especially controlled substances.[354] As a distributor, Cardinal has access to, and checks, the requisite information to see "whether the order deviates from historic ordering patterns in [Cardinal's] strict anti-diversion standards."[355] Suspicious orders are flagged for further "scrutiny and evaluation, including potentially canceling the order."[356]

282. Cardinal states on its website that it "refuse[s] to supply controlled substances to any pharmacy customer where [Cardinal] believe[s] there is an unreasonable risk of medication diversion."[357] This refusal is part of Cardinal's commitment to help solve the "complex national public health crisis."[358] Yet Cardinal continued to supply opioids to pharmacies in Hopkins County

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[351] Cardinal Health, *Opioid Action Program: Reclaiming our Communities,* www.cardinalhealth.com.
[352] *Id.* (making announced and unannounced site visits to pharmacies, which also includes hiring independent investigators specializing in pharmacy diversion and surveillance).
[353] *Id.*; Cardinal also uses an investigative team to perform on-the-ground investigations to determine whether heightened scrutiny is necessary. *Id.* For high-volume orders or orders that flag additional scrutiny, Cardinal has a senior committee of anti-diversion experts who recommend safeguards, which can include ending a business relationship with a customer or denying a business relationship with a new customer. *Id.*
[354] *Id.*
[355] *Id.*
[356] *Id.*
[357] Cardinal Health, *Cardinal Health's Commitment, supra.*
[358] *Id.*

despite the data it reviewed.

283. Distributor Defendants knew their duty. Distributor Defendants had the means to carry out their duty and claim to have successfully done so at times in the past. Distributor Defendants acknowledge that their duty is ongoing. With regards to opioids, however, Distributor Defendants continuously evade their gatekeeping duties, including but not limited to, in Hopkins County.

284. According to *The Charleston Gazette-Mail*, Distributor Defendants shipped nearly 9 million hydrocodone pills over two years to one pharmacy in the town of Kermit, West Virginia.[359] Kermit, West Virginia has a population of 392. Drug wholesalers distributed 780 million pills of oxycodone and hydrocodone in the state over six years. According to the *Gazette*, "[t]he unfettered shipments amount to 433 pain pills for every man, woman and child in West Virginia."[360]

285. Refusing to take action or turning a blind eye to suspicious spikes in opioid prescriptions has for the Distributor Defendants simply become a pesky cost of doing business. McKesson and Cardinal did not take sufficient heed of the multi-million dollar fines each had paid in 2008. McKesson agreed to pay a $150 million fine on January 17, 2017. Cardinal Health reached a $44 million settlement in December 2016; Cardinal also agreed in January 2017 to pay $20 million to the state of West Virginia; and Amerisource agreed to pay $16 million to the state of West Virginia.[361]

[359] Charles Ornstein, *Drug Distributors Penalized for Turning Blind Eye in Opioid Epidemic*, APR, Health News, Jan. 27, 2017.
[360] *Id.*
[361] Ornstein, *supra.*

286. According to a 60 Minutes/Washington Post joint investigation, "[t]he pharmaceutical industry is doing everything it can to keep [the opioid] epidemic going."[362] McKesson certainly did not recognize any sort of "due diligence" while it was providing "millions and millions and millions of pills to countless pharmacies through the United States."[363] Since the 1990s, McKesson has made billions from distributing addictive opioids.[364] McKesson has admitted that when it came to the opioid crisis and pills flooding American communities, there was plenty of blame to go around, including drug makers, other distributors, doctors, and pharmacies.[365]

287. If McKesson had used their authority in the supply chain, the opioid epidemic would not be "nowhere near" where it is today.[366] In McKesson's role as a distributor, not a day went by that something suspicious was not happening, but McKesson never reported any suspicious activity, according to the 60 Minutes investigation.[367] McKesson "fueled the explosive prescription drug abuse problem in this country."[368]

288. McKesson repeatedly filled suspicious orders of the most commonly abused and prescribed opioid drugs oxycodone and hydrocodone.[369] McKesson, as did other Distributor Defendants, failed in their duty to act and also violated the Texas Controlled Substances Act.

289. Distributor Defendants have knowingly distributed, delivered, or dispensed a controlled substance in violation of the Texas Controlled Substances Act §481.128(a)(1). Distributor Defendants dispensed or delivered a controlled substance without any valid medical

[362] 60 Minutes, *Whistleblowers: DEA Attorneys Went Easy on McKesson, the Country's Largest Drug Distributor,* CBS News, Dec. 17, 2017.
[363] *Id.*
[364] *Id.*
[365] *Id.*
[366] *Id.*
[367] 60 Minutes, *supra.*
[368] Too Big to Prosecute, *supra*.
[369] *Id.*

purpose. Tex. Health & Safety Code §481.071. Upon information and belief, Distributor Defendants knew of or had notice of a suspicious rise in the prescribing of opioids in Hopkins County, but chose to open opioid floodgates rather than regulate them. Pill-mill doctors need Distributors to be complicit in the over-supply of opioids. Distributor Defendants were so, causing damages to Hopkins County.

290. Distributor Defendants were in a unique position to see the results of Manufacturing Defendants' fraudulent marketing scheme. Distributor Defendants knew or should have known that there was a sharp increase in the prescription and distribution of opioids. Distributor Defendants undertook the responsibility to prevent opioids from being dispensed or disbursed for diversionary purposes —with a multi-faceted system to monitor and prevent they developed long ago, *according to their own published public proclamations* – and breached that duty Like Manufacturing Defendants, Distributor Defendants chose profits over duty, in breach of duty.

## VII. FIRST CAUSE OF ACTION: NEGLIGENT AND/OR INTENTIONAL CREATION OF A PUBLIC NUISANCE AGAINST MANUFACTURING AND DISTRIBUTOR DEFENDANTS

291. Hopkins County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Third Amended Complaint as though fully alleged herein.

292. Manufacturing Defendants knowingly encouraged doctors in and around Hopkins County to prescribe, and residents to use, highly addictive opioids for chronic pain even though Manufacturing Defendants knew using opioids had a high risk of addiction and reduced quality of life. Distributor Defendants knew or should have known that many of those prescription orders were not for a valid medical purpose. Nevertheless, Distributor Defendants continued to disburse and distribute opioids even though upon information and belief, the evidence would suggest

suspicion for diversionary purposes.

293. By doing so, Defendants purposefully interfered with Hopkins County's public health, public safety, public peace, public comfort, and public convenience.

294. Defendants, individually and in concert with each other, have contributed to and/or assisted in creating and maintaining a condition that is harmful to the health and safety of Hopkins County residents, and/or unreasonably interferes with the peace and comfortable enjoyment of life in violation of Texas law.

295. The public nuisance created by Defendants' actions is substantial and unreasonable – it has caused and continues to cause significant harm to the community – and the harm inflicted outweighs any offsetting benefit.

296. The staggering rates of opioid use resulting from Manufacturing Defendants' marketing efforts, combined with the high number of opioids distributed by Distributor Defendants, have caused, and continues to cause, harm to the community including, but not limited to:

a) Upwards of 30% of all adults use opioids. These high rates of use have led to unnecessary opioid addiction, overdose, injuries, and deaths;

b) Children have been exposed to opioids prescribed to family members or others resulting in injury, addiction, and death. Easy access to prescription opioids has made opioids a recreational drug of choice among Hopkins County teenagers; opioid use among teenagers is only outpaced by marijuana use. Even infants have been born addicted to opioids due to prenatal exposure causing severe withdrawal symptoms and lasting developmental impacts;

c) Residents of Hopkins County, who have never taken opioids, have endured both the emotional and financial costs of caring for loved ones addicted to or injured by opioids and the loss of companionship, wages, or other support from family members who have used, become addicted to, overdosed on, or been killed by opioids;

d) More broadly, opioid use and addiction have driven Hopkins County residents' health care costs higher[370];

e) Employers have lost the value of productive and healthy employees who have suffered from adverse consequences from opioid use;

f) Defendants' success in extending the market for opioids to new patients and chronic conditions has created an abundance of drugs available for criminal use and fueled a new wave of addiction and injury. Defendants' scheme created both ends of a new secondary market for opioids – providing both the supply of narcotics to sell and the demand of addicts to buy them;

g) This demand has created additional illicit markets in other opiates, particularly heroin. The low cost of heroin has led some of those who initially become addicted to prescription opioids to migrate to cheaper heroin, fueling a new heroin epidemic in the process;

h) Diverting opioids into secondary, criminal markets and increasing the number of individuals who are addicted to opioids has increased the demands on emergency services and law enforcement in Hopkins County;

i) All of Defendants' actions have caused significant harm to the community – in lives lost; addictions endured; the creation of an illicit drug market and all its concomitant crime and costs; unrealized economic productivity; and broken families and homes;

j) These harms have taxed the human, medical, public health, law enforcement, and financial resources of Hopkins County; and

k) Defendants' interference with the comfortable enjoyment of life of a substantial number of people is entirely unreasonable because there is limited social utility to opioid use and any potential value is outweighed by the gravity of harm inflicted by Defendants' actions.

297. Manufacturing Defendants knew, or should have known, that promoting opioid use would create a public nuisance in the following ways:

a) Defendants have engaged in massive production, promotion, and distribution of opioids for use by the citizens of Hopkins County;

b) Defendants' actions created and expanded the market for opioids, promoting its wide use for pain management;

[370] *See, e.g.,* Manchikanti, Ex. A, at 14 (stating that the escalating use of opioids in high doses over long periods of time, lifetime use of long-acting drugs, or the combination has serious consequences for the costs of health care and economic stability).

c) Defendants misrepresented the benefits of opioids for chronic pain and fraudulently concealed, misrepresented, and omitted the serious adverse effects of opioids, including the addictive nature of the drugs; and

d) Defendants knew, or should have known, that their promotion would lead to addiction and other adverse consequences that the larger community would suffer as a result.

298. Manufacturing Defendants' actions were, at the least, a substantial factor in doctors and patients not accurately assessing and weighing the risks and benefits of opioids for chronic pain thereby causing opioids to become widely available and used in Hopkins County.

299. Without Defendants' actions, opioid use would not have become so widespread and the enormous public health hazard of opioid addiction would not have existed and could have been averted.

300. The health and safety of the citizens of Hopkins County, including those who use, have used, or will use opioids, as well as those affected by opioid users, is a matter of great public interest and legitimate concern to Hopkins County's citizens and residents. It was foreseeable to all Defendants that the burden of the opioid crisis would fall to counties like Hopkins County in the form of social and economic costs. Specifically it was foreseeable that Hopkins County would sustain damages as an employer obligated to provide healthcare coverage to its employees and as a local government obligated to provide public services to its citizens.

301. The public nuisance created, perpetuated, and maintained by Defendants can be abated and further reoccurrence of such harm and inconvenience can be prevented.

302. Defendants' conduct has affected and continues to affect a considerable number of people within Hopkins County and is likely to continue to cause significant harm to patients who take opioids, their families, and the community at large.

303. Each Defendant created or assisted in creating the opioid epidemic, and each Defendant is jointly and severally liable for its abatement. Furthermore, each Defendant should be enjoined from continuing to create, perpetuate, or maintain said public nuisance in Hopkins County. Furthermore, Defendants should compensate Hopkins County for the funds it has expended and continues to expend for medical insurance claims for opioids that were not medically valid, as well as increased costs of social services, health systems, law enforcement, judicial system, and treatment facilities.

## VIII. SECOND CAUSE OF ACTION: COMMON LAW FRAUD AGAINST ALL DEFENDANTS

304. Hopkins County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Third Amended Complaint as though fully alleged herein.

305. At all relevant and material times, Defendants expressly and/or impliedly warranted that opioids were safe, of merchantable quality, and fit for use.

306. Manufacturing Defendants' superior knowledge and expertise, its relationship of trust and confidence with doctors and the public, its specific knowledge regarding the risks and dangers of opioids, and its intentional dissemination of promotional and marketing information about opioids for the purpose of maximizing sales, each gave rise to the affirmative duty to meaningfully disclose and provide all material information about the risks and harms associated with opioids.

307. At all relevant and material times, Manufacturing Defendants, individually and acting through their employees and agents, and in concert with each other, fraudulently represented to physicians, who Defendants knew would justifiably rely on Manufacturing Defendants' representations, that opioids were safe and effective for treating chronic pain.

308. Manufacturing Defendants' false representations were fraudulently made, with the intent or purpose that healthcare providers and patients would justifiably rely upon them, leading to the prescription, administration, filling, purchasing, and consumption of opioids in Hopkins County.

309. Distributor Defendants knowingly and deliberately took advantage of the Manufacturing Defendants' false and fraudulent representations to disburse and distribute an immense amount of opioids in Hopkins County.

310. Distributor Defendants made representation that they were taking action to prevent the opioid oversupply and abuse while recognizing they were in a unique position to do so. Hopkins County relied on Distributor Defendants to act as a gatekeeper in the supply chain as they represented to the public.

311. Defendants' deliberate misrepresentations and/or concealment, suppression, and omission of material facts as alleged herein include, but are not limited to:

a) Making false and misleading claims regarding the known risks of the addictive nature of opioids and suppressing, failing to disclose, and mischaracterizing the addictive nature of opioids and in concomitant costs, such as overdoses, deaths, and heroin addiction;

b) Making false and misleading written and oral statements that opioids are more effective than traditional pain killers for chronic pain, or effective at all and/or omitting material information showing that opioids are no more effective than other non-addictive drugs for chronic pain;

c) Issuing false and misleading warnings and/or failing to issue adequate warnings concerning the risks and dangers of using opioids;

d) Making false and misleading claims downplaying the risk of addiction when using opioids and/or setting forth guidelines that would purportedly identify addictive behavior;

e) Making false and misleading misrepresentations concerning the safety, efficacy and benefits of opioids without full and adequate disclosure of the underlying facts which rendered such statements false and misleading; and

f) Disbursing and distributing opioids when suspicion existed that opioids were being diverted.

312. Defendants willfully, wantonly, and recklessly disregarded their duty to provide truthful representations regarding the safety and risk of opioids and distributed and disbursed opioids, including the fact that upon information and belief, there was suspicion for diversionary purposes

313. Defendants made these misrepresentations with the intent that the healthcare community and patients would rely to their detriment.

314. Defendants' misrepresentations were made with the intent of defrauding and deceiving the medical community and consumers to induce and encourage the sale of opioids.

315. Defendants' fraudulent representations evidence their callous, reckless, willful, and depraved indifference to the health, safety, and welfare of consumers living in Hopkins County.

316. Defendants omitted, misrepresented, suppressed and concealed material facts concerning the dangers and risk of injuries associated with the use of opioids, as well as the fact that the product was unreasonably dangerous.

317. Defendants' purpose was willfully blind to, ignored, downplayed, avoided, and/or otherwise understated the serious nature of the risks associated with the use of opioids.

318. Defendants' failure to stem, rather than fuel spikes of opioid sales was intended to encourage the sale of opioids, even if the circumstances provided suspicion for diversionary purposes.

319. The treating medical community and consumers in Hopkins County did not know that Defendants' representations were false and/or misleading and justifiably relied on them.

320. Defendants had sole access to material facts concerning the dangers and unreasonable risks of opioids, which they intentionally concealed.

321. As a direct and proximate result of Defendants' fraudulent misrepresentations and intentional concealment of facts, upon which the medical community and consumers in Hopkins County reasonably relied, Hopkins County suffered actual and punitive damages.

## IX. THIRD CAUSE OF ACTION: NEGLIGENCE AGAINST MANUFACTURING AND DISTRIBUTING DEFENDANTS

322. Hopkins County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Third Amended Complaint as though fully alleged herein.

323. Manufacturing Defendants have a duty to exercise reasonable care in marketing its opioids to physicians treating residents of Hopkins County and Hopkins County residents. Manufacturing Defendants have breached their duty by knowingly and fraudulently misrepresenting the benefits of, and downplaying the risks of, opioids for chronic pain.

324. Manufacturing Defendants have used deceitful marketing ploys, KOLs, Front Groups, and other schemes to increase profits at the cost of public health causing an opioid epidemic. Manufacturing Defendants have acted willfully, wantonly, and maliciously.

325. Likewise, Distributor Defendants have a duty to exercise ordinary care in distributing opioids. Distributor Defendants have breached their duty by failing to prevent or reduce the distribution of opioids even if there existed suspicion for diversionary purposes. Distributor Defendants have intentionally failed to prevent or reduce the distribution of opioids so that they could increase profits. Distributor Defendants have acted willfully, wantonly, and maliciously.

326. As a proximate result, Manufacturing and Distributor Defendants and its agents have caused Hopkins County to incur excessive costs to treat the opioid epidemic in its county including, but not limited to, increased costs of social services, health systems, law enforcement, judicial system, and treatment facilities. It was foreseeable to all Defendants that the burden of the

opioid crisis would fall to counties like Hopkins County in the form of social and economic costs. Specifically it was foreseeable that Hopkins County would sustain damages as an employer obligated to provide healthcare coverage to its employees and as a local government obligated to provide public services to its citizens.

327. Hopkins County and its residents are therefore entitled to actual and punitive damages.

## X. FOURTH CAUSE OF ACTION: GROSS NEGLIGENCE AGAINST MANUFACTURING AND DISTRIBUTING DEFENDANTS

328. Hopkins County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Third Amended Complaint as though fully alleged herein.

329. Defendants' marketing scheme to optimize profits by misrepresenting and falsely promoting opioids as the panacea to chronic pain was done intentionally.

330. Defendants' hiring of KOLs, Front Groups, and others to spread its fraudulent message that opioids were useful and beneficial for chronic pain was grossly negligent and done with conscious indifference or reckless disregard for the safety of others.

331. Distributor Defendants' distribution of opioids despite the obvious signs that there was no valid medical purpose for a large number of prescription for opioids was grossly negligent and done with conscious indifference or reckless disregard for the safety of others.

332. Each Defendant's actions and omissions as described herein, singularly or in combination with each other, were malicious resulting in damages and injuries to Hopkins County and its residents.

333. At every stage, Defendants knew, or should have known, that their conduct would create an unreasonable risk of physical harm to others, including Hopkins County and its residents,

and should be held liable in punitive and exemplary damages to Hopkins County.

## XI. FIFTH CAUSE OF ACTION: TEXAS CONTROLLED SUBSTANCES ACT ("TCSA") AGAINST DISTRIBUTOR DEFENDANTS AND DR. DIAMOND

334. Hopkins County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Third Amended Complaint as though fully alleged herein.

335. Distributor Defendants have knowingly distributed, delivered, administered, or dispensed a controlled substance in violation of the Texas Controlled Substances Act §481.128(a)(1) by dispensing or delivering a controlled substance, or causing a controlled substance to be administered, when there is no valid medical purpose. Tex. Health & Safety Code §481.071.

336. As alleged herein, each Distributor Defendant, at all times relevant to this Complaint, had a duty to monitor the flow of opioids by acting as a gatekeeper between physicians and pharmacies and patients. Distributor Defendants wholly failed in its duty and knew, or should have known, that they were distributing or disbursing opioids without a valid medial purpose.

337. Moreover, Distributor Defendants' failure to rein in the endless supply of opioids was reasonably calculated to deceive practitioners treating Hopkins County residents into prescribing opioids without any valid medical purpose, and Distributor Defendants continue to do so to this day.

338. Dr. Diamond prescribed opioids, a controlled substance, without the federal registration required under Texas Health & Safety Code Section § 481.061. Due to his lack of a DEA registration, Dr. Diamond was not allowed to "manufacture, distribute, prescribe, possess, analyze or dispense" a controlled substance in Texas, including Hopkins County. Tex. Health &

Safety Code § 481.061(a).

339. Even when Dr. Diamond possessed the requisite registration, Dr. Diamond prescribed opioids without a valid medical purpose in violation of Texas Health & Safety Code Section 481.071(a).

340. As a direct and proximate cause of Distributor Defendants' and Dr. Diamond's deceptive conduct, Hopkins County should be awarded civil penalties pursuant to the Texas Controlled Substances Act.

## XII. SIXTH CAUSE OF ACTION: TEXAS CONTROLLED SUBSTANCES ACT ("TCSA") AGAINST DISTRIBUTOR DEFENDANTS

341. Hopkins County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Third Amended Complaint as though fully alleged herein.

342. Distributor Defendants have knowingly distributed opioids even though it had reason to suspect that the opioids were being diverted, or suspicion for diversionary purposes.

343. As alleged herein, each Distributor Defendant, at all times relevant to this Complaint, has stated in public through its agents and on its websites that they act as gatekeepers between physicians and pharmacies and patients. In fact, Distributor Defendants have called their role in the chain of supply critical to stopping suspicious opioid prescribing and assisting in stopping the opioid epidemic.

344. For many years, Distributor Defendants have had the ability to track prescription orders and they have undertaken the duty to exercise reasonable care to track and halt any and all suspicious opioid prescriptions. Distributor Defendants claim they have stopped tens of thousands of prescriptions suspected of diversion while admitting that the opioid epidemic is a serious public

health crisis.

345. But Distributor Defendants have breached its duty by failing to track and halt the overwhelming supply of opioids into Hopkins County despite its layers of oversight and commitment to provide a safe and secure channel to deliver medications, including opioids. Distributor Defendants have intentionally failed to prevent or reduce the distribution of opioids so that they could increase profits and have done so willfully, wantonly, and maliciously.

346. As a proximate result, Distributor Defendants and its agents have caused Hopkins County to incur excessive costs to treat the opioid epidemic in its county including, but not limited to, increased costs of social services, health systems, law enforcement, judicial system, and treatment facilities. It was foreseeable to all Defendants that the burden of the opioid crisis would fall to counties like Hopkins County in the form of social and economic costs. Specifically it was foreseeable that Hopkins County would sustain damages as an employer obligated to provide healthcare coverage to its employees and as a local government obligated to provide public services to its citizens.

347. Hopkins County and its residents are therefore entitled to actual and punitive damages.

## XIII. SEVENTH CAUSE OF ACTION: UNJUST ENRICHMENT AGAINST ALL DEFENDANTS

348. Hopkins County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Third Amended Complaint as though fully alleged herein.

349. As an expected and intended result of their conscious wrongdoing as set forth in this Complaint, Defendants have profited and benefited from opioid purchases made by Hopkins County and its residents.

350. When Hopkins County and its residents purchased opioids, they expected that Defendants had provided necessary and accurate information regarding those risks. Instead, Defendants had misrepresented the material facts regarding the risks and benefits of opioids and distributed or disbursed opioids even though, upon information and belief, there was suspicion for diversionary purposes.

351. Defendants took undue advantage and received a benefit because the County bore the cost of the externalities of Defendants' wrongful conduct. Moreover, the County had no choice and was effectively required to cover these costs to Defendants' benefit.

352. Defendants have been unjustly enriched at the expense of Hopkins County, and Hopkins County is therefore entitled to damages to be determined by the jury.

## PRAYER FOR RELIEF

WHEREFORE, Plaintiff respectfully prays:

a. That the acts alleged herein be adjudged and decreed to be unlawful and that the Court enter a judgment declaring them to be so;

b. That Defendants be enjoined from, directly or indirectly through KOLs, Front Groups or other third parties, continuing to misrepresent the risks and benefits of the use of opioids for chronic pain, and from continuing to violate Texas law;

c. That Plaintiff recover all measures of damages, including punitive and exemplary damages, allowable under the law, and that judgment be entered against Defendants in favor of Plaintiff;

d. That Plaintiff recover restitution on behalf of Hopkins County consumers who paid for opioids for chronic pain;

e. That Plaintiff recover the costs and expenses of suit, pre- and post-judgment interest, and reasonable attorneys' fees as provided by law; and

f. That Defendants be ordered to abate the public nuisance that they created in in violation of Texas common law.

Date: April 23, 2018

Respectfully Submitted,

**County Attorney, Hopkins County**

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# EXHIBIT A

Pain Physician 2012; 15:ES9-ES38 • ISSN 2150-1149

Health Policy

# Opioid Epidemic in the United States

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Over the past two decades, as the prevalence of chronic pain and health care costs have exploded, an opioid epidemic with adverse consequences has escalated. Efforts to increase opioid use and a campaign touting the alleged undertreatment of pain continue to be significant factors in the escalation. Many arguments in favor of opioids are based solely on traditions, expert opinion, practical experience and uncontrolled anecdotal observations. Over the past 20 years, the liberalization of laws governing the prescribing of opioids for the treatment of chronic non-cancer pain by the state medical boards has led to dramatic increases in opioid use. This has evolved into the present stage, with the introduction of new pain management standards by the Joint Commission on the Accreditation of Healthcare Organizations (JCAHO) in 2000, an increased awareness of the right to pain relief, the support of various organizations supporting the use of opioids in large doses, and finally, aggressive marketing by the pharmaceutical industry. These positions are based on unsound science and blatant misinformation, and accompanied by the dangerous assumptions that opioids are highly effective and safe, and devoid of adverse events when prescribed by physicians.

Results of the 2010 National Survey on Drug Use and Health (NSDUH) showed that an estimated 22.6 million, or 8.9% of Americans, aged 12 or older, were current or past month illicit drug users, The survey showed that just behind the 7 million people who had used marijuana, 5.1 million had used pain relievers. It has also been shown that only one in 6 or 17.3% of users of non-therapeutic opioids indicated that they received the drugs through a prescription from one doctor.

The escalating use of therapeutic opioids shows hydrocodone topping all prescriptions with 136.7 million prescriptions in 2011, with all narcotic analgesics exceeding 238 million prescriptions. It has also been illustrated that opioid analgesics are now responsible for more deaths than the number of deaths from both suicide and motor vehicle crashes, or deaths from cocaine and heroin combined. A significant relationship exists between sales of opioid pain relievers and deaths. The majority of deaths (60%) occur in patients when they are given prescriptions based on prescribing guidelines by medical boards, with 20% of deaths in low dose opioid therapy of 100 mg of morphine equivalent dose or less per day and 40% in those receiving morphine of over 100 mg per day. In comparison, 40% of deaths occur in individuals abusing the drugs obtained through multiple prescriptions, doctor shopping, and drug diversion.

The purpose of this comprehensive review is to describe various aspects of crisis of opioid use in the United States. The obstacles that must be surmounted are primarily inappropriate prescribing patterns, which are largely based on a lack of knowledge, perceived safety, and inaccurate belief of undertreatment of pain.

**Key words:** Opioid abuse, opioid misuse, nonmedical use of psychotherapeutic drugs, nonmedical use of opioids, National Survey on Drug Use and Health, opioid guidelines.

**Pain Physician 2012; 15:ES9-ES38**

The Institute of Medicine (IOM) recently published a report on relieving pain in America (1,2). The report identified multiple facts, including that there are more than 116 million Americans with pain persisting from weeks to years, with financial costs ranging from $560 billion to $635 billion per year. The report alluded to the serious problem of the diversion and abuse of opioid drugs, questioning their long-term usefulness. The IOM committee reported that when opioids are used as prescribed; they can be safe and effective for acute postoperative pain, procedural pain, and patients nearing the end of life who desire more pain relief. While the IOM committee does promote pain treatment, including opioids, they do acknowledge a serious crisis in the diversion and abuse of opioids and a lack of evidence for the long-term usefulness of opioids in treating chronic pain. Along with increases in the prevalence of chronic pain, health care costs, and adverse consequences due to opioid use, the opioid crisis is escalating (1-49). Despite mounting evidence, efforts to increase opioid use based on the alleged undertreatment of pain continue (50-63). In fact, Stein (64) summarized the evidence succinctly, noting that "many arguments in favor of opioids are solely based on traditions, expert opinion, practical experience, and uncontrolled anecdotal observations."

Starting in the late 1990's, state medical boards curtailed restrictions on laws governing the prescribing of opioids for the treatment of chronic non-cancer pain, resulting in a dramatic increase in the number of prescriptions (65). This development gathered momentum with the introduction of new pain management standards for in-patient and out-patient medical care implemented by the Joint Commission on the Accreditation of Health Care Organizations (JCAHO) in 2000 (66) and an increased awareness of the right to pain relief, both of which provided justification for physicians. (67-70). Other factors fueling an increase in prescriptions included aggressive marketing by the pharmaceutical industry, the promotion of opioids by numerous physicians and a call for for the increased use of opioids in the treatment of chronic non-cancer pain by myriad organizations. These positions, alongside continued assertions that pain is undertreated, were largely based on untenable science and misinformation, and contended that opioids are highly effective and safe without adverse effects when prescribed by physicians (31,60,66,71-90). Moreover, a recent examination of model guidelines for curtailing controlled substance abuse revealed that the guidelines appeared instead to condone an increase in prescribing (50,91-93). This is illustrated by the language in the model guidelines, which state (65), "no disciplinary action will be taken against a practitioner based solely on the quantity and/or frequency of opioids prescribed." Thus, the use of opioids in general, including long-acting and potent forms of opioids, have dramatically increased due to a shift in regulations largely driven by published, albeit extremely weak, evidence suggesting that opioids are not only highly effective, but also safe in selected persons with chronic non-cancer pain, even though this selection criteria are extremely weak and these guidelines have only facilitated overuse of opioids (31,71,94-98). Nearly 2 decades later, the scientific evidence for the effectiveness of opioids for chronic non-cancer pain remains unclear (35,71,96,99-119). In addition to ongoing concerns with regard to the lack of effectiveness of opioids in chronic non-cancer pain (31-38,96,99-119), there is growing evidence of multiple physiologic and non-physiologic adverse effects, such as opioid hyperalgesia (32,95,96,107,112-124), misuse and abuse (31-39,71,95,96,102,103,110-115,125-140), the inability of providers to identify and monitor misuse and overuse (31,32,36,95,96,126,127,130,138-151), and a steady increase in opioid-related fatalities (32,34,37,129,130,152-163). In fact, in 2008 drug poisoning in the United States has been reported to contribute to one death every 15 minutes (160). Furthermore, opioids have been shown to contribute to one death every 36 minutes in the United States in 2008. Correlating with these fatalities, sales and substance abuse treatment admissions have increased substantially (125-127,159,160,164-168).

With the above background highlighting a steady increase in fatalities with opioid use and very little evidence of effectiveness, it remains to be seen who will ultimately bear the responsibility for the premature adoption of opioids as a treatment standard (116). It has been speculated that in the coming years, there will likely be an extensive "postmortem" on the massive opioid treatment movement and the escalating social crisis that has accompanied it (116). It is universally accepted that this massive treatment movement has led to huge collateral damage in terms of diversion, misuse, and abuse of opioids. The widespread use of opioids for chronic non-cancer pain is in direct violation of the established cardinal principles of medical intervention – that there be compelling evidence of the benefit of a therapy prior to its large-scale use (116).

A cautious approach has been advocated in recent years by many (17,33,35,49,110-115,117-119,169). This

manuscript is undertaken to evaluate the escalating opioid crisis which although heavily regulated, continues to be uncontrolled.

## 1.0 Non-Medical Use of Psychotherapeutic Drugs

### 1.1 Current Non-Medical Use

Results of the 2010 National Survey on Drug Use and Health (NSDUH) (170), an annual survey sponsored by the Substance Abuse and Mental Health Services Administration (SAMHSA), showed that an estimated 22.6 million, or 8.9% of Americans, age 12 or older, were current (past month) illicit drug users. Illicit drugs include marijuana, cocaine, heroin, hallucinogens, inhalants, or prescription-type psychotherapeutics (defined in this survey as prescription-type pain relievers, tranquilizers, stimulants, and sedatives) used non-medically. Marijuana was the most commonly used illicit drug with 17.4 million current (past month) users, or 6.9% of the US population. Cocaine was used by 1.5 million, whereas hallucinogens were used in the past month by 1.2 million persons (Fig. 1 and Table 1). Next to marijuana, 7.0 million (27%) persons age 12 or older had used prescription-type psychotherapeutic drugs non-medically in the past month (current use). Of these, 5.1 million had used pain relievers. The category of psychotherapeutics used in the tables and figures includes the nonmedical use of any prescription-type pain relievers, tranquilizers, stimulants, or sedatives. However, over-the-counter substances are not included in these studies. The categories of nonmedical use of psychotherapeutics and pain relievers were well ahead of the illicit use of cocaine, hallucinogens, inhalants, methamphetamine, heroin, and lysergic acid diethylamide (LSD).

Overall, there has been an increase in the current use of all illicit drugs and marijuana, without any change for psychotherapeutics and hallucinogens and a decrease for cocaine from 2002 to 2010, as shown in Fig. 2.

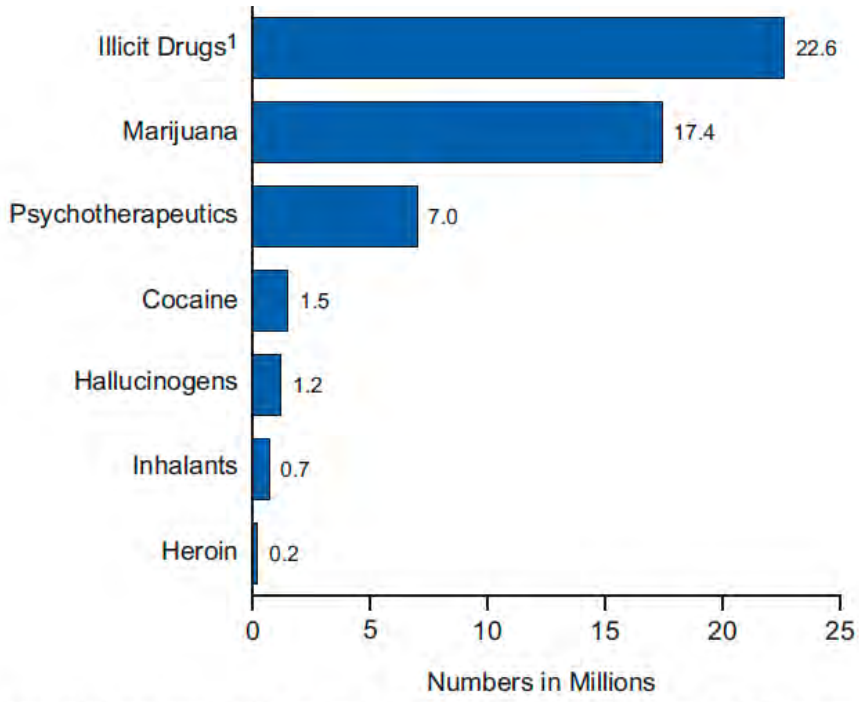



Fig. 1. *Past month illicit drug use among persons aged 12 or older: 2010.*

Source: Substance Abuse and Mental Health Services Administration. *Results from the 2010 National Survey on Drug Use and Health: Summary of National Findings*. http://www.samhsa.gov/data/NSDUH/2k10NSDUH/2k10Results.pdf (170) Access date 2/22/2012.

Table 1. *Types of illicit drug use in the past month among persons aged 12 or older: Numbers in thousands, from 1998 to 2010.*

| Drugs | 1998 | 1999 | 2000 | 2001 | 2002 | 2003 | 2004 | 2005 | 2006 | 2007 | 2008 | 2009 | 2010 | 12-Year % change from 1998 to 2010 |
|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|
| Nonmedical Use of Psychotherapeutics[2,3] | 2,477 (1.1%) | 3,952 (1.8%) | 3,849 (1.7%) | 4,811 (2.1%) | 6,287 (2.7%) | 6,451 (2.7%) | 6,110 (2.5% ) | 6,491 (2.7%) | 7,095[b] (2.9%[b]) | 6,895[a] (2.8%[a]) | 6,224 (2.5%) | 6,953 (2.8%) | 6,967 (2.7%) | 181% |
| Pain Relievers | -- | 2,621 (1.2%) | 2,782 (1.2%) | 3,497 (1.6%) | 4,377 (1.9%) | 4,693 (2.0%) | 4,404 (1.8%) | 4,658 (1.9%) | 5,220 (2.1%) | 5,174 (2.1%) | 4,747 (1.9%) | 5,257 (2.1%) | 5,100 (2.0%) | NA |
| OxyContin® | -- | -- | -- | -- | -- | -- | 325 (0.1%) | 334 (0.1%) | 276 (0.1%[a]) | 369 (0.1%) | 435 (0.2%) | 510 (0.2%) | 564 (0.2%) | NA |
| Tranquilizers | 655 (0.3%) | 1,097 (0.5%) | 1,000 (0.4%) | 1,358 (0.6%) | 1,804 (0.8%) | 1,830 (0.8%) | 1,616 (0.7%) | 1,817 (0.7%) | 1,766 (0.7%) | 1,835 (0.7%) | 1,800 (0.7%) | 2,010 (0.8%) | 2,160 (0.9%) | 230% |
| Stimulants | 633 (0.3%) | 950 (0.4%) | 788 (0.4%) | 1,018 (0.5%) | 1,303[b] (0.6%[b]) | 1,310[b] (0.6%[b]) | 1,312[b] (0.5%[b]) | 1,188[b] (0.5%[b]) | 1,385[b] (0.6%[b]) | 1,053 (0.4%) | 904 (0.4%) | 1,290 (0.5%) | 1,077 (0.4%) | 70% |
| Sedatives[3] | 210 (0.1%) | 229 (0.1%) | 175 (0.1%) | 306 (0.1%) | 436[b] (0.2%[b]) | 294 (0.1%) | 265 (0.1%) | 272 (0.1%) | 385 (0.2%[a]) | 346 (0.1%) | 234 (0.1%) | 370 (0.1%) | 374 (0.1%) | 78% |
| Marijuana and Hashish | 11,016 (5.0%) | 10,458 (4.7%) | 10,714 (4.8) | 12,122 (5.4%) | 14,584 (6.2%) | 14,638 (6.2%) | 14,576 (6.1%) | 14,626 (6.0%) | 14,813 (6.0%) | 14,448 (5.8%) | 15,203 (6.1%) | 16,718 (6.6%) | 17,373 (6.9%) | 58% |
| Cocaine | 1,750 (0.8%) | 1,552 (0.7%) | 1,213 (0.5%) | 1,667 (0.7%) | 2,020 (0.9%) | 2,281 (1.0%) | 2,021 (0.8%) | 2,397 (1.0%) | 2,421 (1.0%) | 2,075 | 1,855 | 1,637 (0.7%) | 1,466 (0.6%) | -16% |
| TOTAL ILLICIT DRUGS[1] | 13,615 (6.2%) | 13,829 (6.3%) | 14,027 (6.3%) | 15,910 (7.1%) | 19,522 (8.3%) | 19,470 (8.2%) | 19,071 (7.9%) | 19,720 (8.1%) | 20,357 (8.3%) | 19,857 (8.0%) | 20,077 (8.0%) | 21,813 (8.7%) | 22,622 (8.9%) | 66% |

-- Not available.

Note: 2002 to 2008 data is based on 2008 National Survey on Drug Use and Health Survey Report.

a Difference between estimate and 2008 estimate is statistically significant at the 0.05 level.b Difference between estimate and 2008 estimate is statistically significant at the 0.01 level.

1 Illicit Drugs include marijuana/hashish, cocaine (including crack), heroin, hallucinogens, inhalants, or prescription-type psychotherapeutics used nonmedically. Illicit Drugs Other

Than Marijuana include cocaine (including crack), heroin, hallucinogens, inhalants, or prescription-type psychotherapeutics used nonmedically. The estimates for Nonmedical Use of Psychotherapeutics, Stimulants, and Methamphetamine incorporated in these summary estimates do not include data from the methamphetamine items added in 2005 and 2006.

2 Nonmedical use of prescription-type psychotherapeutics includes the nonmedical use of pain relievers, tranquilizers, stimulants, or sedatives and does not include over-the counter drugs.

3 Estimates of Nonmedical Use of Psychotherapeutics, Stimulants, and Methamphetamine in the designated rows include data from methamphetamine items added in 2005 and 2006 and are not comparable with estimates presented in NSDUH reports prior to the 2007 National Findings report. For the 2002 through 2005 survey years, a Bernoulli stochastic imputation procedure was used to generate adjusted estimates comparable with estimates for survey years 2006 and later.

Source: SAMHSA, Office of Applied Studies, National Survey on Drug Use and Health, 1998 - 2010.

www.samhsa.gov/data/NSDUH/2k10NSDUH/2k10Results.pdf (170) Access date 2/22/2012

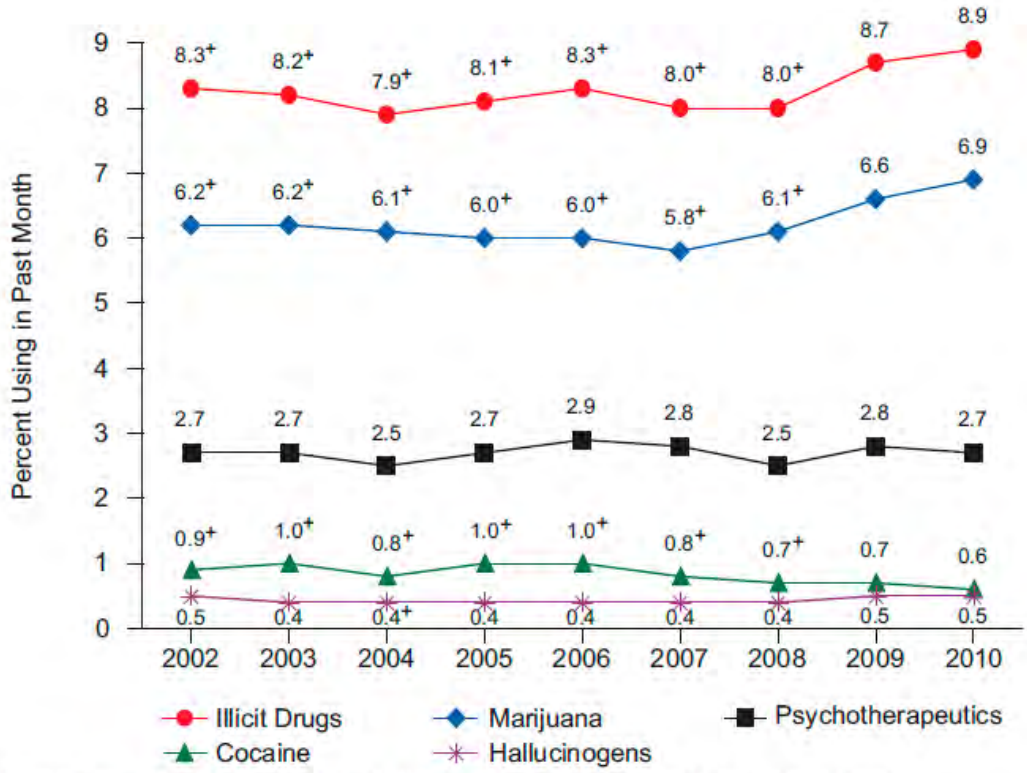



Fig. 2. *Past month use of selected illicit drugs among persons aged 12 or older: 2002-2010.*
Source: Substance Abuse and Mental Health Services Administration. Results from the 2010 National Survey on Drug Use and Health: Summary of National Findings. http://www.samhsa.gov/data/NSDUH/2k10NSDUH/2k10Results.pdf (170) Access date 2/22/2012

### 1.2 Past Year Initiates

In 2010, there were 2.4 million persons age 12 or older who used psychotherapeutics non-medically for the first time within the past year. Numbers of new users for specific psychotherapeutics in 2010 were 2.0 million for pain relievers, 1.2 million for tranquilizers, 624,000 for stimulants, and 252,000 for sedatives (Table 2 and Fig. 3). The specific drug categories with the largest number of recent initiatives among persons age 12 or older were nonmedical use of pain relievers (2,004 million) and marijuana (2,426 million), followed by nonmedical use of tranquilizers (1,238 million), ecstasy (0.937 million), inhalants (0.793 million), cocaine (0.637 million), and stimulants (0.624 million) (Fig. 3). More strikingly, in 2010, the number of new nonmedical users of OxyContin (oxycodone) age 12 or older was 598,000 with an average age at first use of 22.8 years among those age 12 to 49 (170).

### 1.3 Past Year Use

The analysis of long-term statistics based on yearly use of illicit drugs is disturbing. The past year use of illicit drugs in 2010 was 38.806 million, or 15.3% of the population (Table 3). Nonmedical use of psychotherapeutics for the past year in the 2010 survey was 16.031 million or 6.3% population age 12 or older, compared to 2.6% of the population in 1998. Of importance is the fact that nonmedical use of psychotherapeutics was just behind marijuana and hashish with use by 11.5% of the population age 12 or older in 2010, increased from 8.6% in 1998. Overall, nonmedical use of psychotherapeutics increased 178% from 1998 to 2010, compared to marijuana 56% and cocaine at 17%.

### 1.4 Lifetime Use

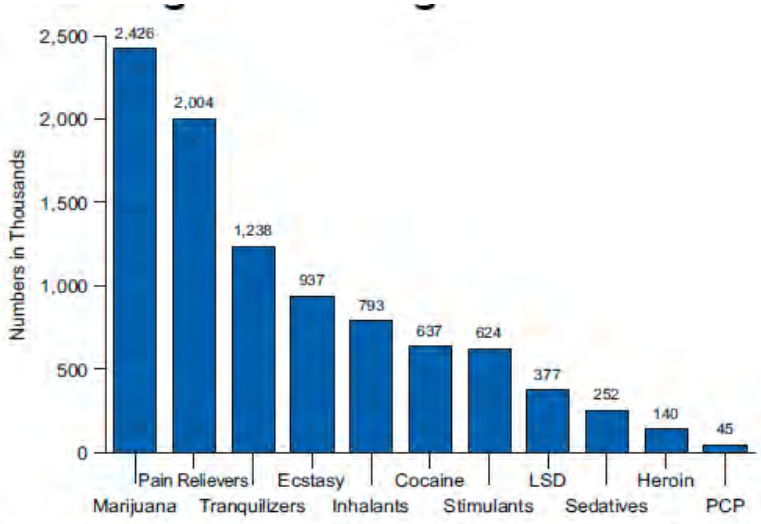
Lifetime use of illicit drugs (lifetime use indicates use of a specific drug at least once in the respondent's

Table 2. *Past year initiates for illicit drugs from 1998 to 2010 (numbers in thousands) for 12 years.*

| Drugs | 1998 | 1999 | 2000 | 2001 | 2002 | 2003 | 2004 | 2005 | 2006 | 2007 | 2008 | 2009 | 2010 | 12-Year % change from 1998 to 2010 |
|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|
| Pain Relievers[2] | 1,548 | 1,810 | 2,268 | 2,400 | 2,320 | 2,456 | 2,422 | 2,193 | 2,150 | 2,147 | 2,176 | 2,179 | 2,004 | 29% |
| Tranquilizers | 860 | 916 | 1,298 | 1,212 | 1,184 | 1,071 | 1,180 | 1,286 | 1,112 | 1,232 | 1,127 | 1,226 | 1,238 | 44% |
| Stimulants[2] | 648 | 706 | 808 | 853 | 783 | 715 | 793[a] | 647 | 845[b] | 642 | 599 | 702 | 624 | -4% |
| Sedatives | 147 | 164 | 191 | 225 | 209 | 194 | 240 | 247 | 267 | 198 | 181 | 186 | 252 | 71% |
| Marijuana | 2,498 | 2,640 | 2,746 | 2,793 | 2,196 | 1,973 | 2,142 | 2,114 | 2,063 | 2,090 | 2,208 | 2,361 | 2,426 | -3% |
| Cocaine | 868 | 917 | 1,002 | 1,140 | 1,032[b] | 986[b] | 998[b] | 872[a] | 977[b] | 906[b] | 722 | 617 | 637 | -27% |
| Heroin | 140 | 121 | 114 | 154 | 117 | 92 | 118 | 108 | 91 | 106 | 114 | 180 | 140 | 0% |

Note: 2002 to 2008 data is based on 2008 National Survey on Drug Use and Health Survey Report.
-- Not available.
a Difference between estimate and 2008 estimate is statistically significant at the 0.05 level.
b Difference between estimate and 2008 estimate is statistically significant at the 0.01 level.
1 Illicit Drugs include marijuana/hashish, cocaine (including crack), heroin, hallucinogens, inhalants, or prescription-type psychotherapeutics used nonmedically. Illicit Drugs Other Than Marijuana include cocaine (including crack), heroin, hallucinogens, inhalants, or prescription-type psychotherapeutics used nonmedically. The estimates for Nonmedical Use of Psychotherapeutics, Stimulants, and Methamphetamine incorporated in these summary estimates do not include data from the methamphetamine items added in 2005 and 2006. See Section B.4.8 in Appendix B of the Results from the 2008 National Survey on Drug Use and Health: National Findings.
2 Nonmedical use of prescription-type psychotherapeutics includes the nonmedical use of pain relievers, tranquilizers, stimulants, or sedatives and does not include over-the counter
drugs.
3 Estimates of Nonmedical Use of Psychotherapeutics, Stimulants, and Methamphetamine in the designated rows include data from methamphetamine items added in 2005 and 2006 and are not comparable with estimates presented in NSDUH reports prior to the 2007 National Findings report. For the 2002 through 2005 survey years, a Bernoulli stochastic imputation procedure was used to generate adjusted estimates comparable with estimates for survey years 2006 and later.
Source: Substance Abuse and Mental Health Services Administration. Results from the 2010 National Survey on Drug Use and Health: Summary of National Findings. http://www.samhsa.gov/data/NSDUH/2k10NSDUH/2k10Results.pdf (170) Access date 2/22/2012

lifetime), including psychotherapeutics, among persons age 12 or older has been increasing over the years (Table 4). In 2010, the lifetime use of illicit drugs among persons age 12 or older was slightly more than 2009 with 119,508 or 47.1% of the population. Similarly, nonmedical use of psychotherapeutics remained the same from 2009 with 20.4% in 2010, or almost 51.6 million using prescription psychotherapeutic drugs for nonmedical purposes. Among the subgroups, only OxyContin increased significantly from 1.9 million in 2005 to 6.1 million in 2010, or 0.8% of the population in 2005 to 2.4% in 2010 (171). Lifetime use of illicit drugs in per-




Note: The specific drug refers to the one that was used for the first time, regardless of whether it was the first drug used or not.

Fig. 3. *Past year initiates for specific illicit drugs among persons aged 12 or older: 2010*.
Source: Substance Abuse and Mental Health Services Administration. Results from the 2010 National Survey on Drug Use and Health: Summary of National Findings. http://www.samhsa.gov/data/NSDUH/2k10NSDUH/2k10Results.pdf (170) Access date 2/22/2012

sons age 12 or older was topped by marijuana (41.9% of the population) followed by nonmedical use of psychotherapeutics (20.4% of the population).

### 1.5 Abuse Based on Age

In 2010, young adults age 18 to 25 demonstrated rates of current use of illicit drugs to be higher (21.5%) than for youths age 12 to 17 (10.1%) and adults age 26 or older (6.6%), with 6.9% using marijuana, 2.7% using psychotherapeutics non-medically, 0.6% using cocaine, and 0.5% using hallucinogens among young adults 18-25 (Fig. 4). Past month nonmedical use of prescription-type drugs among young adults increased from 20.2% in 2002 to 21.5% in 2010. This was primarily due to an increase in the rate of pain reliever use which was 4.1% in 2002 and 4.9% in 2006 (170). As illustrated in Figure 5, overall illicit drug use increased from 8.3% to 8.9% in 2010 in the age group from 18 to 25.

Rates of past month illicit drug use varied with age. Through the adolescent years from 12 to 17, the rates of current illicit drug use in 2010 increased from 4.0% at ages 12 or 13, to 9.3% at ages 14 or 15, to 16.6% at ages 16 or 17 (170). The highest rate of 23.1% was noted among persons age 18 to 20, with the next highest rate among 21 to 25 year olds 20.5% (Fig. 6) (144). In 2010, adults age 26 or older were less likely to be current drug users than youths age 12 to 17 or young adults age 18 to 25 (6.6 versus 10.1 and 21.5%, respectively). However, there were more drug users age 26 or older (12.8 million) than users in the 12-to-17-year age group (2.5 million) and 18-to-25-year age group (7.3 million) combined.

### 1.6 Abuse Based on Gender

In 2010, the survey results were similar to prior years with males being more likely than females to be current illicit drug users (11.2% versus 6.8%). Males were more likely than females to be past month users of marijuana (9.1% versus 4.7%). Rates of past month nonmedical use of psychotherapeutic drugs among males and females was 3% and 2.5%, pain relievers was 2.3% and 1.7%, cocaine was 0.8% and 0.4% and hallucinogens was 0.6% and 0.3% (170).

### 1.7 Abuse During Pregnancy

Among pregnant woman age 15 to 44 years, a significantly lower proportion of women used illicit drugs in the past month (4.4%) compared to 10.9% of their

Table 3. *Types of illicit drug use in the past year among persons aged 12 or older: numbers in thousands from 1998 to 2010 (12 years).*

| Drugs | 1998 | 1999 | 2000 | 2001 | 2002 | 2003 | 2004 | 2005 | 2006 | 2007 | 2008 | 2009 | 2010 | 12-year % change from 1998 to 2010 |
|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|
| Nonmedical Use of Psychotherapeutics[2, 3] | 5,759 (2.6%) | 9,220 (4.2%) | 8,761 (3.9%) | 11,102 (4.9%) | 14,795 (6.3%) | 15,163 (6.4%) | 14,849 (6.2%) | 15,346 (6.3%) | 16,482 [b] (6.7% [b]) | 16,280[a] (6.6%[a]) | 15,166 (6.1%) | 16,006 (6.4%) | 16,031 (6.3%) | 178% |
| Pain Relievers | -- | 6,582 (3.0%) | 6,466 (2.9%) | 8,353 (3.7%) | 10,992[a] (4.7%) | 11,671 (4.9%) | 11,256 (4.7%) | 11,815 (4.9%) | 12,649 (5.1% [a]) | 12,466 (5.0%) | 11,885 (4.8%) | 12,405 (4.9%) | 12,213 (4.8%) | 85% From 1999 |
| OxyContin® | -- | -- | -- | -- | -- | -- | 1,213[a] (0.5%) | 1,226 (0.5%) | 1,323 (0.5%) | 1,422 (0.6%) | 1,459 (0.6%) | 1,677 (0.7%) | 1,869 (0.7%) | 54% From 2004 |
| Tranquilizers | 1,940 (0.9%) | 2,728 (1.2%) | 2,731 (1.2%) | 3,673 (1.6%) | 4,849 (2.1%) | 5,051 (2.1%) | 5,068 (2.1%) | 5,249 (2.2%) | 5,058 (2.1%) | 5,282 (2.1%) | 5,103 (2.0%) | 5,460 (2.2%) | 5,581 (2.2%) | 188% |
| Stimulants3 | 1,489 (0.7%) | 2,291 (1.0%) | 2,112 (0.9%) | 2,486 (1.1%) | 3,380[b] (1.4%[b]) | 3,031[a] (1.3%[b]) | 3,254[b] (1.4%[b]) | 3,088[a] (1.3%[b]) | 3,791[b] (1.5%[b]) | 2,998 (1.2%) | 2,639 (1.1%) | 3,060 (1.2%) | 2,887 (1.1%) | 94% |
| Sedatives | 522 (0.2%) | 631 (0.3%) | 611 (0.3%) | 806 (0.4%) | 981[b] (0.4%[b]) | 831 a (0.3%[a]) | 737 (0.3%) | 750 (0.3%) | 926 b (0.4%[b]) | 864 [a] (0.3%[a]) | 621 (0.2%) | 811 (0.3%) | 907 (0.4%) | 56% |
| Marijuana and Hashish | 18,710 (8.6%) | 19,102 (8.6%) | 18,589 (8.3%) | 21,086 (9.3%c) | 25,755 (11.0%[a]) | 25,231 (10.6%) | 25,451 (10.6%) | 25,375 (10.4%) | 25,378 (10.3%) | 25,085 (10.1%) | 25,768 (10.3%) | 28,521 (11.3%) | 29,206 (11.5%) | 56% |
| Cocaine | 3,811 (1.7%) | 3,742 (1.7%) | 3,328 (1.5%) | 4,186 (1.9%c) | 5,902[a] (2.5%[b]) | 5,908[a] (2.5%[b]) | 5,658 (2.4% a) | 5,523 (2.3%) | 6,069[b] (2.5%[b]) | 5,738 (2.3%) | 5,255 (2.1%) | 4,797 (1.9%) | 4,449 (1.8%) | 17% |
| TOTAL ILLICIT DRUGS1 | 23,115 (10.6%) | 25,402 (11.5%) | 24,535 (11.0%) | 28,409 (12.6%[c]) | 35,132 (14.9%[a]) | 34,993 (14.7%) | 34,807 (14.5%) | 35,041 (14.4%) | 35,775 (14.5%) | 35,692 (14.4%) | 35,525 (14.2%) | 37,957 (15.1%) | 38,806 (15.3%) | 68% |

-- Not available.

Note: 2002 to 2010 data is based on 2010 National Survey on Drug Use and Health Survey Report. a Difference between estimate and 2010 estimate is statistically significant at the 0.05 level. b Difference between estimate and 2010 estimate is statistically significant at the 0.01 level.

1 Illicit Drugs include marijuana/hashish, cocaine (including crack), heroin, hallucinogens, inhalants, or prescription-type psychotherapeutics used non-medically. Illicit drugs other than marijuana include cocaine (including crack), heroin, hallucinogens, inhalants, or prescription-type psychotherapeutics used non-medically. The estimates for nonmedical use of psychotherapeutics, stimulants, and methamphetamine incorporated in these summary estimates do not include data from the methamphetamine items added in 2005 and 2006.

2 Nonmedical use of prescription-type psychotherapeutics includes the nonmedical use of pain relievers, tranquilizers, stimulants, or sedatives and does not include over-the counter drugs.

3 Estimates of nonmedical use of psychotherapeutics, stimulants, and methamphetamine in the designated rows include data from methamphetamine items added in 2005 and 2006 and are not comparable with estimates presented in NSDUH reports prior to the 2007 National Findings report. For the 2002 through 2005 survey years, a Bernoulli stochastic imputation procedure was used to generate adjusted estimates comparable with estimates for survey years 2006 and later.

Source: Substance Abuse and Mental Health Services Administration. Results from the 2010 National Survey on Drug Use and Health: Summary of National Findings. http://www.samhsa.gov/data/NSDUH/2k10NSDUH/2k10Results.pdf (170). Access date 2/22/2012

Table 4. *Types of illicit drugs of lifetime use among persons aged 12 or older: numbers in thousands, 1998 – 2010.*

| Drug | 1998 | 1999 | 2000 | 2001 | 2002 | 2003 | 2004 | 2005 | 2006 | 2007 | 2008 | 2009 | 2010 | 12-Year % change from 1998 to 2010 |
|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|
| Nonmedical Use of Psychotherapeutics[2] | 20,193 (9.2%) | 34,076 (15.4%) | 32,443 (14.5%) | 36,028 (16.0%) | 47,958 [b] (20.4%) | 49,001[b] (20.6%) | 49,157 (20.4%) | 49,571 a (20.4%) | 50,965 (20.7%) | 50,415 (20.3%) | 51,970 (20.8%) | 51,771 (20.6%) | 51,641 (20.4%) | 156% |
| Pain Relievers | -- | 19,888 (9.0%) | 19,210 (8.6%) | 22,133 (9.8%) | 29,611 [b] (12.6% [b]) | 31,207 [b] (13.1% [a]) | 31,768 [b] (13.2% [a]) | 32,692 [b] (13.4%) | 33,472 (13.6%) | 33,060 [a] (13.3%) | 34,861 (14.0%) | 35,046 (13.9%) | 34,776 (13.7%) | 75% From 1999 |
| OxyContin® | -- | -- | -- | -- | 1,924 [b] (0.8% [b]) | 2,832 [b] (1.2% [b]) | 3,072 [b] (1.3% [b]) | 3,481 [b] (1.4% [b]) | 4,098 [b] (1.7% [b]) | 4,354 (1.8%) | 4,842 (1.9%) | 5,829 (2.3%) | 6,121 (2.4%) | 218% From 2002 |
| Tranquilizers | 7,726 (3.5%) | 13,860 (6.3%) | 13,007 (5.8%) | 13,945 (6.2%) | 19,267 [b] (8.2%) | 20,220 (8.5%) | 19,852 [a] (8.3%) | 21,041 (8.7%) | 21,303 (8.7%) | 20,208 (8.2%) | 21,476 (8.6%) | 21,755 8.6%) | 22,103 (8.7%) | 186% |
| Stimulants | 9,614 (4.4%) | 15,922 (7.2%) | 14,661 (6.6%) | 16,007 (7.1%) | 23,496 [b] (10.0% [b]) | 23,004 [a] (9.7% [b]) | 22,297 (9.3% b) | 20,983 (8.6%) | 22,468 (9.1% [a]) | 21,654 (8.7%) | 21,206 (8.5%) | 21,930 (8.7%) | 21,660 (8.5%) | 125% |
| Sedatives | 4,640 (2.1%) | 7,747 (3.5%) | 7,142 (3.2%) | 7,477 (3.3%) | 9,960 [a] (4.2% [b]) | 9,510 (4.0% [a]) | 9,891 (4.1% a) | 8,982 (3.7%) | 8,822 (3.6%) | 8,396 (3.4%) | 8,882 (3.6%) | 8,605 (3.4%a) | 7,631 (3.2%) | 64% |
| Marijuana and Hashish | 72,070 (33.0%) | 76,428 (34.6%) | 76,321 (34.2%) | 83,272 (36.9%[c]) | 94,946 [b] (40.4%) | 96,611 [b] (40.6%) | 96,772 b (40.2%) | 97,545 [b] (40.1%) | 97,825 [b] (39.8% [a]) | 100,518 (40.6%) | 102,404 (41.0%) | 104,446 (41.5%) | 106,232 (41.9%) | 47% |
| Cocaine | 23,089 (10.6%) | 25,406 (11.5%) | 24,896 (11.2%) | 27,788 (12.3%) | 33,910 [b] (14.4%) | 34,891 [a] (14.7%) | 34,153 b (14.2%) | 33,673 [b] (13.8%) | 35,298 (14.3%) | 35,882 (14.5%) | 36,773 (14.7%) | 36,599 (14.5%) | 37,210 (14.7%) | 61% |
| TOTAL ILLICIT DRUGS1 | 78,123 (35.8%) | 87,734 (39.7%) | 86,931 (38.9%) | 94,140 (41.7%[c]) | 108,255 b (46.0%) | 110,205 [b] (46.4%) | 110,057 b (45.8% a) | 112,085 [b] (46.1%) | 111,774 [b] (45.4% [b]) | 114,275[a] (46.1%) | 117,325 (47.0%) | 118,705 (47.1%) | 119,508 (47.1%) | 53% |

-- Not available.

Note: 2002 to 2010 data is based on 2010 National Survey on Drug Use and Health Survey Report.

a Difference between estimate and 2010 estimate is statistically significant at the 0.05 level.

b Difference between estimate and 2010 estimate is statistically significant at the 0.01 level.

1 Illicit Drugs include marijuana/hashish, cocaine (including crack), heroin, hallucinogens, inhalants, or prescription-type psychotherapeutics used non-medically. Illicit drugs other than marijuana include cocaine (including crack), heroin, hallucinogens, inhalants, or prescription-type psychotherapeutics used non-medically. The estimates for nonmedical use of psychotherapeutics, stimulants, and methamphetamine incorporated in these summary estimates do not include data from the methamphetamine items added in 2005 and 2006.

2 Nonmedical use of prescription-type psychotherapeutics includes the nonmedical use of pain relievers, tranquilizers, stimulants, or sedatives and does not include over-the counter drugs.

3 Estimates of nonmedical use of psychotherapeutics, stimulants, and methamphetamine in the designated rows include data from methamphetamine items added in 2005 and 2006 and are not comparable with estimates presented in NSDUH reports prior to the 2007 National Findings report. For the 2002 through 2005 survey years, a Bernoulli stochastic imputation procedure was used to generate adjusted estimates comparable with estimates for survey years 2006 and later.

Source: Substance Abuse and Mental Health Services Administration. Results from the 2010 National Survey on Drug Use and Health: Summary of National Findings. http://www.samhsa.gov/data/NSDUH/2k10NSDUH/2k10Results.pdf (170) Access date 2/22/2012

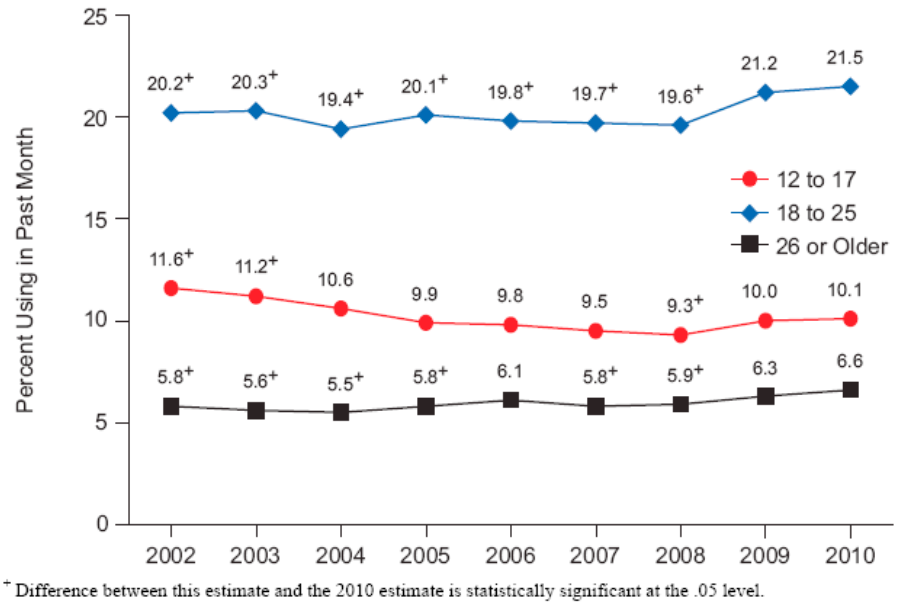



Fig. 4. ***Comparative analysis of past month use of illicit drugs among various age groups.***
Source: Substance Abuse and Mental Health Services Administration. Results from the 2010 National Survey on Drug Use and Health: Summary of National Findings. http://www.samhsa.gov/data/NSDUH/2k10NSDUH/2k10Results.pdf (170) Access date 2/22/2012

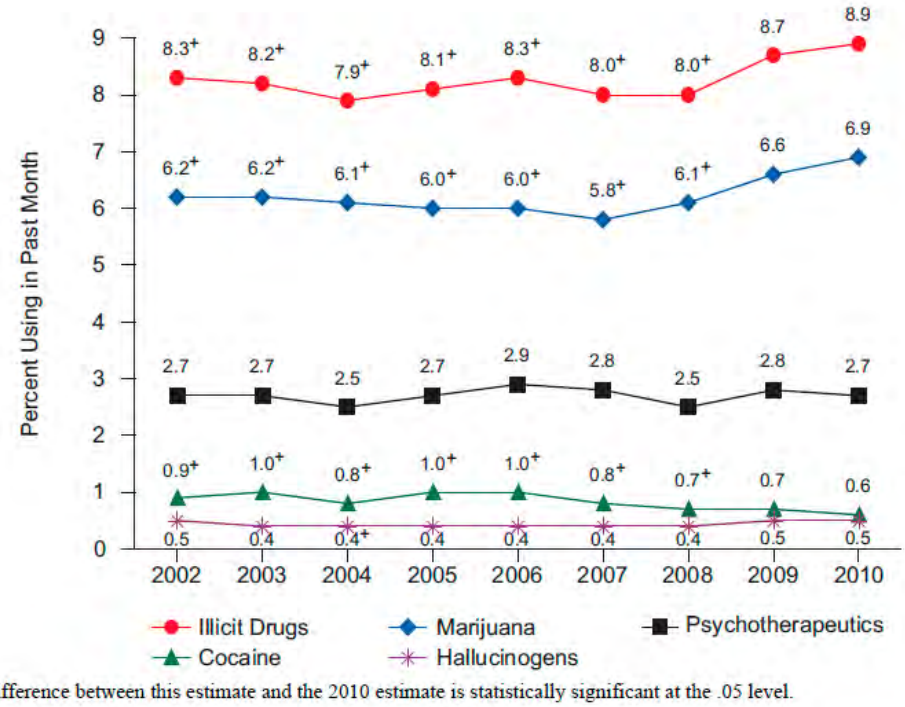



Fig. 5. ***Past month use of selected illicit drugs among young adults aged 18 to 25: 2002-2010.***
Source: Substance Abuse and Mental Health Services Administration. Results from the 2010 National Survey on Drug Use and Health: Summary of National Findings. http://www.samhsa.gov/data/NSDUH/2k10NSDUH/2k10Results.pdf (170) Access date 2/22/2012

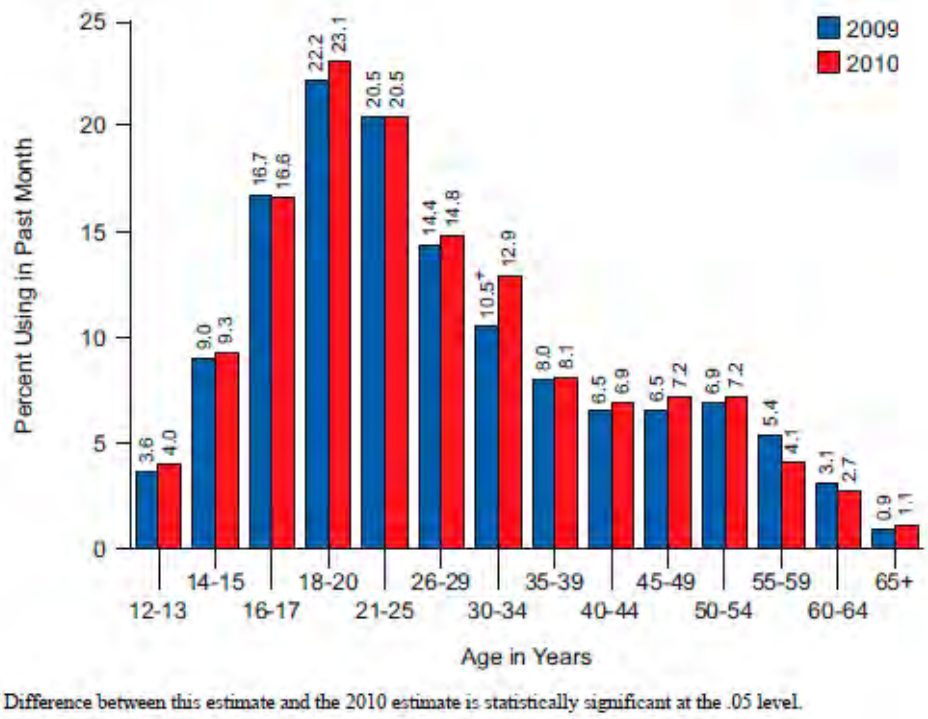



Fig. 6. *Past month illicit drug use among persons aged 12 or older, by age: 2009 and 2010.*
Source: Substance Abuse and Mental Health Services Administration. Results from the 2010 National Survey on Drug Use and Health: Summary of National Findings. http://www.samhsa.gov/data/NSDUH/2k10NSDUH/2k10Results.pdf (170) Access date 2/22/2012

nonpregnant counterparts. These figures are based on data averaged for 2009 and 2010 (170).

### 1.8 Abuse Based on Employment

Employment also seemed to have a significant influence in 2010. Among adults age 18 or older, the rate of illicit drug use was higher for unemployed persons (17.5%) than for those who were employed full time (8.4%) or part time (11.2%) (170).

### 1.9 Regional Variations

There were also differences based on geographic area among persons age 12 or older in 2010. The rate of current illicit drug use in 2010 was 11.0% in the West, 9.4% in the Northeast, 8.2% in the Midwest, and 7.8% in the South (170). Further, the rate of current illicit drug use in metropolitan areas was higher than the rate in non-metropolitan areas with 9.4% in large metropolitan counties, 8.8% in small metropolitan counties, and 7.5% in non-metropolitan counties as a group (170).

### 1.10 Drug Abuse Among Criminals

In 2010, an estimated 1.5 million adults age 18 or older who were on parole or supervised release from jail during the past year had higher rates of dependence on or abuse of a substance (27%) than their counterparts who were not on parole or supervised release during the past year (8.7%). In 2010, probation status was associated with substance dependence or abuse. The rate of substance dependence or abuse was 29.9% among adults who were on probation during the past year, which was significantly higher than the rate among adults who were not on probation during the past year was 8.3% (170).

### 1.11 Driving Under the Influence

Driving under the influence of illicit drugs is a criminal act and dangerous to the public. In 2010, 10.6 million persons, or 4.2% of the population age 12 or older, reported driving under the influence of illicit drugs during the past year. This rate was highest among young adults age 18 to 25 with 12.7% (170).

### 1.12 Frequency of Abuse

Among past year marijuana users age 12 or older in 2010, the following patterns were revealed (170):

- 15.7% used marijuana on 300 or more days within the past 12 months, translating to 4.6 million using marijuana on a daily or almost daily basis over a 12-month period.
- 39.9%, or 6.9 million, used the drug on 20 or more days in the past month (current use).

## 2.0 Mental Health Problems and Nonmedical Use Of Drugs

The NSDUH survey of 2010 evaluated the prevalence and treatment of serious mental illness (SMI), serious psychological distress (SPD), and major depressive episode (MDE) and the association of these problems with substance use and substance dependency or abuse. SPD is an overall indicator of the past 30 days of psychological distress, whereas MDE is defined as a period of at least 2 weeks when a person experienced a depressed mood or loss of interest or pleasure in daily activities and had symptoms that met the criteria for a major depressive disorder (171). Further, SPD indicates a respondent recently experienced heightened distress symptomatology that may be affecting health and behavior during the past 30 days. However, this distress may be part of a chronic psychological disturbance (even SMI) or may represent a temporary disturbance that could subside after a brief period of adjustment.

### 2.1 Serious Medical Illness and Drug Abuse

The prevalence of SMI in 2010 was shown in 11.4 million adults, representing 5.0% of all adults, with the highest rates being in adults age 18 to 25 (7.7%) and lowest for adults age 50 or older (3.2%) as shown in Figure 7 (171). The prevalence of SPD among women age 18 or older was higher (6.5%) than among men (3.4%) in that age group (171).

### 2.2 Major Depressive Episodes and Drug Abuse

The prevalence of a MDE in 2010 was 6.8% of persons age 18 or older, or 15.5 million adults, with at least one MDE in the past year. The number of adults who had past year MDE was 6.8%. Even then, the past year

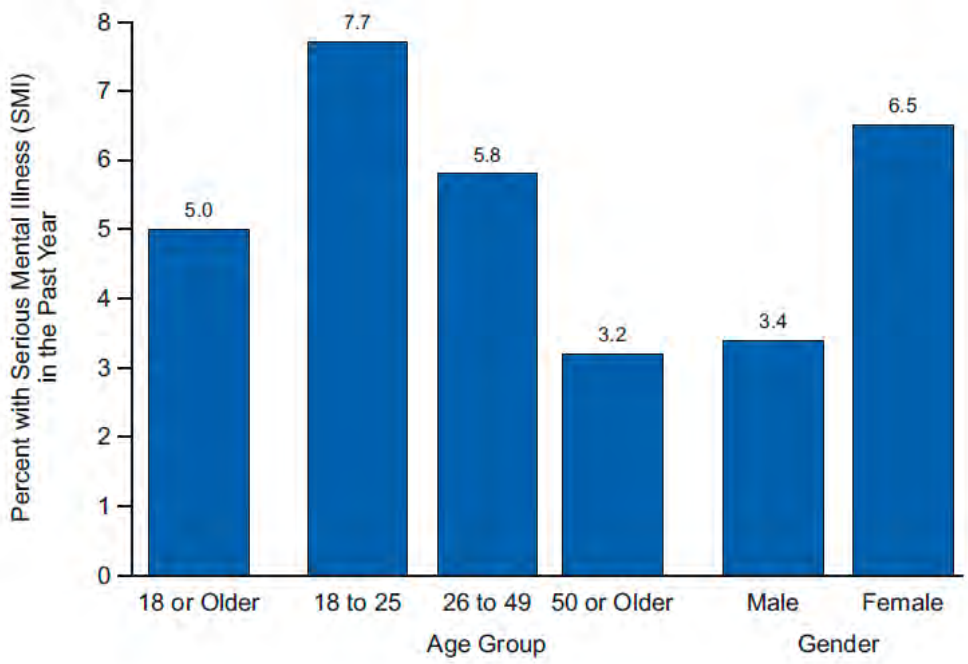



Fig. 7. *Serious mental illness, psychological distress, and nontherapeutic drug use, among persons age 18 and older, by age, 2010.* Source: Substance Abuse and Mental Health Services Administration. Results from the 2010 National Survey on Drug Use and Health: Mental Health Findings. www.samhsa.gov/data/NSDUH/2k10MH_Findings/2k10MHResults.pdf (171) Access date 2/23/2012

prevalence of MDE in 2010 was lower for those age 50 or older (5.6%) compared with rates among persons age 18 to 25 (8.2%) and those age 26 to 49 (7.5%). However, the past year prevalence of MDE was higher among adult females than among adult males, 8.4% versus 5.1%. In addition, among women, past year MDE rates were higher with 11.3% for 18 to 25 year olds, 9.2 for 26 to 49 year olds compared with those of 50 or older with only 6.7%. Further, the prevalence of MDE also varied by race and ethnicity with the highest rate among persons reporting 2 or more races (10.8%), while rates for single race groups were 7.3% among whites, 5.6% among Hispanics, 7.7% among American Indians or Alaska Natives, 5.8% among blacks, and 3.8% among Asians.

In addition, in 2010 the past prevalence of MDE with severe impairment for adults age 18 or older was higher among unemployed persons (9.3%) than among persons employed full time (5.4%).

In 2010, an adult age 18 or older with a combination of a MDE and substance use and dependence or abuse in the past year was more likely than those with MDE to have used an illicit drug in the past year (22.0% versus 7.9%) (171). A similar pattern was observed for specific types of past year illicit drug use, such as marijuana and the nonmedical use of prescription-type psychotherapeutics. Figure 8 illustrates substance abuse in adults by MDE.

The prevalence of a MDE in youths age 12 to 17 in 2010 showed that 1.9 million (8.9%) reported at least one MDE during the past year. Among youths age 12 to 17, the past year prevalence of MDE ranged from 3.3% among 12-year-olds to 10.9% among those age 16, and 10.3% among those age 17 (171).

Among youths with MDE age 12 to 17, 37.2% had used illicit drugs in 2010, in contrast to 37.4% in 2008. This was higher than the 17.8% of youths in the past year that did not have a MDE but had used illicit drugs. This pattern, however, was similar to specific types of illicit drug use including marijuana and the nonmedical use of prescription-type psychotherapeutics (171).

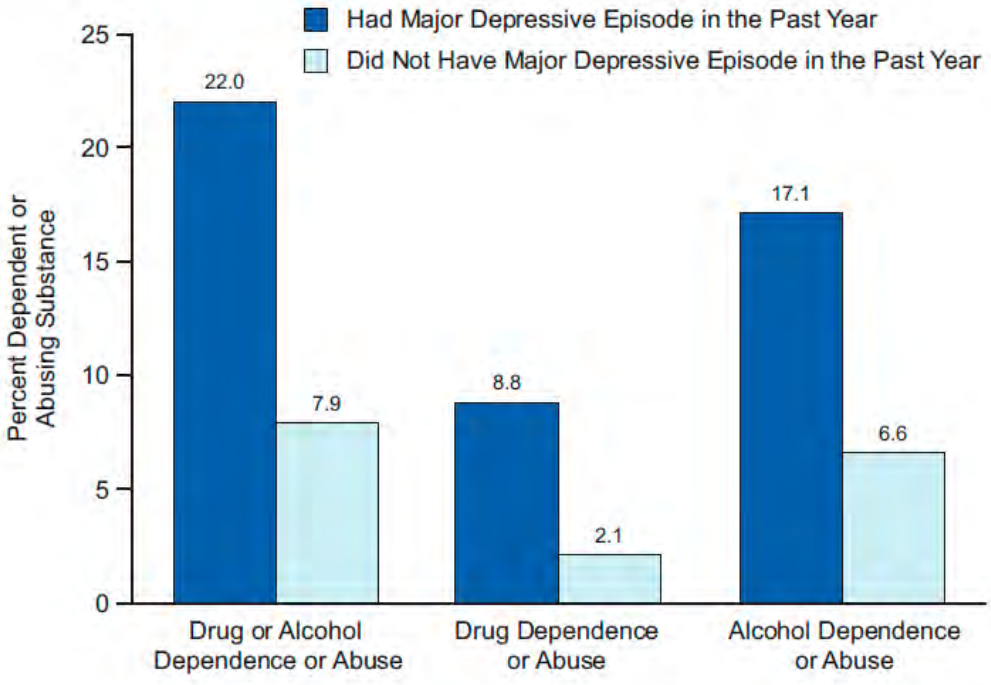



Fig. 8. *Substance dependence or abuse among adults age 18 or older, by major depressive episode in the past years, 2010.* Source: Substance Abuse and Mental Health Services Administration. Results from the 2010 National Survey on Drug Use and Health: Mental Health Findings. www.samhsa.gov/data/NSDUH/2k10MH_Findings/2k10MHResults.pdf (171) Access date 2/23/2012

## 3.0 Where Do Non-Therapeutic Drugs Come From?

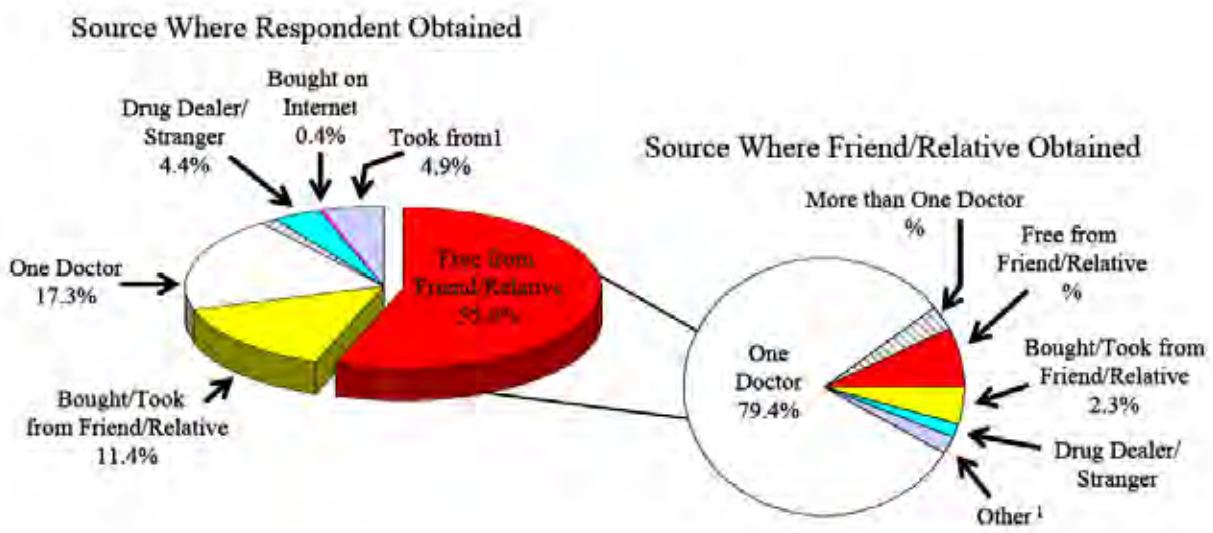
Among persons aged 12 or older in 2009-2010 who used pain relievers nonmedically in the past 12 months, 55% obtained pain relievers from a friend or relative for free (170). Among the remaining 45%, 11.4% bought them from a friend or relative (which was significantly higher than the 8.9% from 2007-2008), and 4.8% essentially stole them from a friend or relative (Fig. 9). However, only one in 6 or 17.3% indicated that they received the drugs through a prescription from one doctor, while only 4.4% received pain relievers from a drug dealer or other stranger, and 0.4% bought them on the Internet, with no significant changes from 2007 to 2008.

Even more striking is the fact that in 2009-2010, 41.5% of past year methamphetamine users reported that they obtained the methamphetamine they used most recently for free from a friend or relative, with an additional 30.7% buying it from a friend or relative (170).

## 4.0 Escalating Use Of Therapeutic Opioids

The escalating use of therapeutic opioids, specifically in high doses over long periods of time or even lifetime use of long-acting drugs, and the combination of long and short-acting drugs continue to have serious consequences for costs of health care and economic stability.

The data overwhelmingly suggest that the increased supply of opioids, high medical users, doctor shoppers, and patients with multiple comorbid factors contribute to the majority of fatalities. The quadrupled sales of opioid analgesics between 1999 and 2010 are a perfect example of the therapeutic opioid explosion. The data on sales and distribution of opioids show an increase from 96 mg morphine equivalents per person in the United States in 1997 to 710 mg per person in 2010 (34,153). This has been estimated to be the equivalent of 7.1 kg of opioid medication per 10,000 persons or enough to supply every adult American with 5 mg of hydrocodone every 6 hours for 45 days. Sales of hy-




**Note: Totals may not sum to 100% because of rounding or because suppressed estimates are not shown.**

One took them from a friend or relative without asking

Fig. 9. *Source where pain relievers were obtained for most recent nonmedical use among past year users age 12 or older: 2009-2010.* Source: Substance Abuse and Mental Health Services Administration. Results from the 2010 National Survey on Drug Use and Health: Mental Health Findings. www.samhsa.gov/data/NSDUH/2k10MH_Findings/2k10MHResults.pdf (171) Access date 2/23/2012

drocodone have increased by 280% from 1997 to 2007, whereas methadone usage has increased 1,293% and oxycodone usage by 866%, as illustrated in Table 5 (32). The estimated number of prescriptions filled for opioids exceeded 256 million in the United States in 2009, with 234 million prescriptions for immediate-release (IR) opioids and 22.9 million for extended-release (ER) opioids with significant increases from 21.3 million for

Table 5. *Retail sales of opioid medications (grams of medication) from 1997 to 2007.*

| Drug | 1997 | 1998 | 1999 | 2000 | 2001 | 2002 | 2003 | 2004 | 2005 | 2006 | 2007 | % of Change from 1997 |
|---|---|---|---|---|---|---|---|---|---|---|---|---|
| Methadone | 518,737 | 692,675 (34%) | 964,982 (39%) | 1,428,840* (48%) | 1,892,691 (32%) | 2,649,559 (40%) | 3,683,881 (39%) | 4,730,157 (28%) | 5,362,815 (13%) | 6,621,687 (23%) | 7,228,219 (9%) | 1293% |
| Oxycodone | 4,449,562 | 6,579,719 (48%) | 9,717,600 (48%) | 15,305,913 (58%) | 19,927,286 (30%) | 22,376,892 (12%) | 26,655,152 (19%) | 29,177,530 (9%) | 30,628,973 (5%) | 37,034,220 (21%) | 42,977,043 (16%) | 866% |
| Fentanyl Base | 74,086 | 90,618 (22%) | 107,141 (18%) | 146,612* (37%) | 186,083 (27%) | 242,027 (30%) | 317,200 (31%) | 370,739 (17%) | 387,928 (5%) | 428,668 (11%) | 463,340 (8%) | 525% |
| Hydromorphone | 241,078 | 260,009 (8%) | 292,506 (12%) | 346,574* (18%) | 400,642 (16%) | 473,362 (18%) | 579,372 (22%) | 655,395 (13%) | 781,287 (19%) | 901,663 (15% | 1,011,028 (12%) | 319% |
| Hydrocodone | 8,669,311 | 10,389,503 (20%) | 12,101,621 (16%) | 14,118,637 (17%) | 15,594,692 (10%) | 18,822,619 (21%) | 22,342,174 (19%) | 24,081,900 (8%) | 25,803,543 (7%) | 29,856,368 (16%) | 32,969,527 (10%) | 280% |
| Morphine | 5,922,872 | 6,408,322 (8%) | 6,804,935 (6%) | 7,807,511 (15%) | 8,810,700 (13%) | 10,264,264 (16%) | 12,303,956 (20%) | 14,319,243 (16%) | 15,054,846 (5%) | 17,507,148 (16%) | 19,051,426 (9%) | 222% |
| Codeine | 25,071,410 | 26,018,054 (4%) | 23,917,088 (-8%) | 23,474,865* (-2%) | 23,032,641 (-2%) | 22,633,733 (-2%) | 21,865,409 (-3%) | 20,264,555 (-7%) | 18,960,038 (-6%) | 18,762,919 (-1%) | 18,840,329 (0.4%) | -25% |
| Meperidine (Pethidine) | 5,765,954 | 5,834,294 (1%) | 5,539,592 (-5%) | 5,494,898* (-1%) | 5,450,204 (-1%) | 5,412,389 (-1%) | 5,239,932 (-3%) | 4,856,644 (-7%) | 4,272,520 (-12%) | 4,160,033 (-3%) | 3,936,179 (-5%) | -32% |
| Total | 50,713,010 | 56,273,194 (11%) | 59,445,465 (6%) | 35,962,089.84 (15%) | 75,294,939 (11%) | 82,874,845 (10%) | 92,987,076 (12%) | 98,456,163 (6%) | 101,251,950 (6%) | 115,272,706 (14%) | 126,477,091 (10%) | 149% |

Number in parenthesis is percentage of change from previous year.
* For year 2000 data is not available, the average of 1999 and 2001 was taken.
Source: www.deadiversion.usdoj.gov/arcos/retail_drug_summary/index.html Access date: 8/25/2010
Source for 2007 data - www.justice.gov/ndic/pubs33/33775/dlinks.htm
Adapted from: Manchikanti L, et al. Therapeutic use, abuse, and nonmedical use of opioids: A ten-year perspective. Pain Physician 2010; 13:401-435 (32).

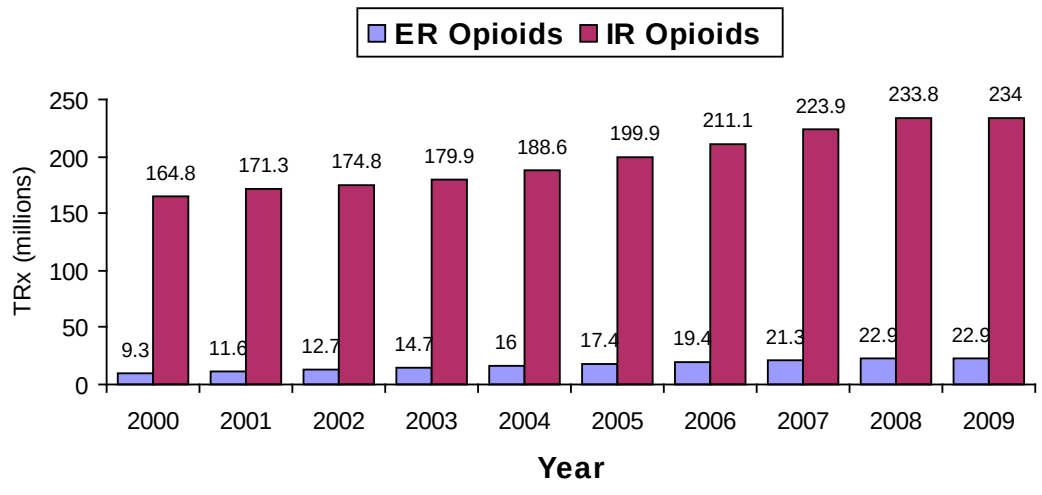



Fig. 10. *Total number of prescriptions dispensed for ER/LA and IR opioids from U. S. outpatient retail pharmacies, year 2000 – 2009 (173).*
Source: SDI, Vector One ®: National (174).

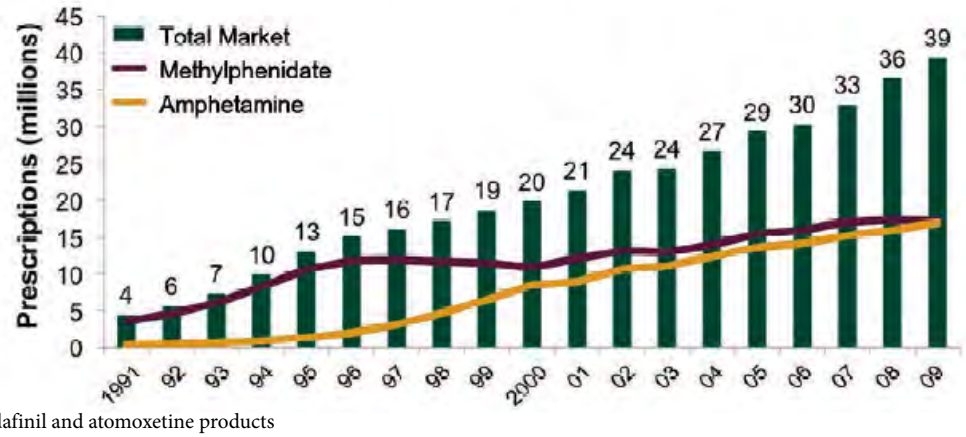
ER opioids and from 223.9 million for IR opioids from 2007 as illustrated in Figure 10 (172-174). The data are even more compelling when compared from 2002 to 2009 with an increase from 9.3 million for ER opioids to 22.9 million, a 146% increase, and from 164.8 million to 234 million for IR opioids, a 42% increase with an annual increase of 21% for ER opioids and 6% for IR opioids. Most prescriptions were for hydrocodone and oxycodone-containing products (84.9%) and issued for short treatment courses, 19.1% for less than 2 weeks, 65.4% for 2-3 weeks. Of these, however, approximately 12% of the prescriptions were issued to those aged 10 to 29 years. This may signal a potential problem for this population, as this is also the population most likely to abuse drugs and develop addictions (172). In addition, the data also illustrates an 8-fold increase in stimulant prescriptions from 1991 to 2009 as illustrated in Fig. 11.

Table 6 illustrates hydrocodone with acetaminophen being the number one prescription from 2006 through 2011 (175). However, narcotic analgesics constitute number 4 in the proportion of patients treated in selected therapies with hypertension, topping at 42.4 million and narcotic analgesics at 15.6 million, constituting number 10 in spending in leading therapy areas with oncologicals constituting 23.2 billion and narcotic analgesics constituting 8.3 billion in 2011 as illustrated in Tables 7 and 8 and Fig. 12 (175).

The United Nations Office on Drugs and Crime, in an evaluation of the world supply of opioid, shows 90% of the global consumption of morphine, fentanyl, and oxycodone registered in 2009 occurring in Australia, Canada, New Zealand, the United States and several European countries (60,85).

Another World Health Organization (WHO) report (87) showed that based on the statistics from the International Narcotics Control Board (INCB) in 2003, 6 developed countries accounted for 79% of global morphine consumption, whereas developing countries which represent 80% of the world population accounted for only about 6% of global morphine consumption. In addition, the most recent data showed that in 2007, 6 developed countries reported the highest level of morphine consumption and 132 of the 160 signatory countries that require reporting of consumption were below the global mean as illustrated in Fig. 13. This simply illustrates that millions of patients with moderate to severe pain caused by different diseases and conditions may not be getting treatment to alleviate their suffering in some countries, while more of them are receiving it in other countries such as the United States, which uses 99% of the world's supply of hydrocodone and 83% of the world's oxycodone (176-178).

Gram for gram, people in the United States consume more narcotic medication than any other nation worldwide. The International Narcotic Control Board, a division of the United States, estimates global pharmaceutical companies produce more than 75 tons a year of oxycodone, compared with 11.5 tons in 1999,




*excludes modafinil and atomoxetine products

*Fig. 11. Projected number of prescriptions for stimulants* dispensed by U.S. retail pharmacies, 1991-2009.*
Source: SDI, Vector One ®: National (174).

Table 6. *Top medicines by prescriptions.*

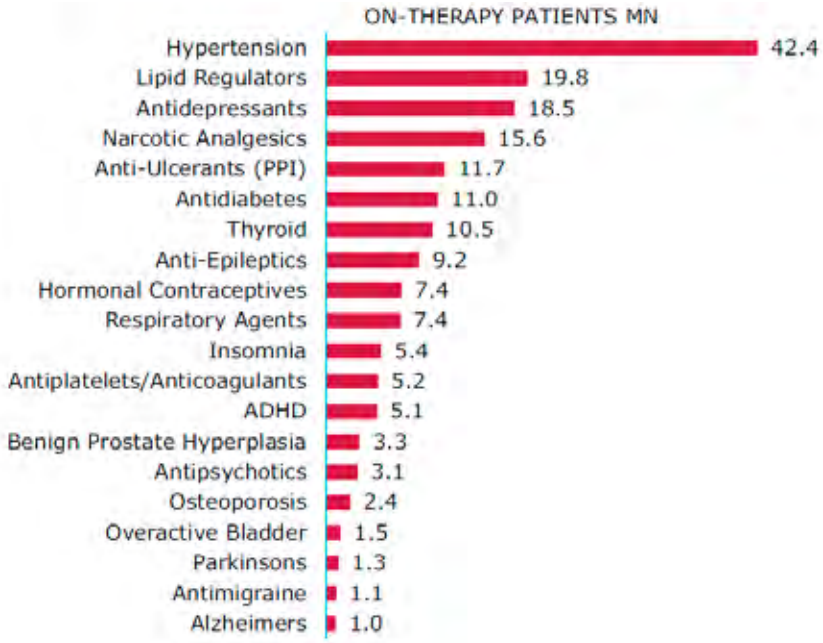
| DISPENSED PRESCRIPTIONS MN | | 2007 | 2008 | 2009 | 2010 | 2011 |
|---|---|---|---|---|---|---|
| Total US Market | | 3,825 | 3,866 | 3,949 | 3,993 | 4,024 |
| 1 | Hydrocodone/acetaminophen | 120.9 | 125.5 | 129.4 | 132.1 | 136.7 |
| 2 | Levothyroxine sodium | 97.4 | 98.9 | 100.2 | 103.2 | 104.7 |
| 3 | Simvastatin | 49.0 | 68.0 | 84.1 | 94.4 | 96.8 |
| 4 | Lisinopril | 71.5 | 77.2 | 83.0 | 87.6 | 88.8 |
| 5 | Amlodipine besylate | 40.8 | 46.0 | 52.1 | 57.8 | 62.5 |
| 6 | Omeprazole (RX) | 27.7 | 35.8 | 45.6 | 53.5 | 59.4 |
| 7 | Metformin HCL | 49.2 | 51.6 | 53.8 | 57.0 | 59.1 |
| 8 | Azithromycin | 47.1 | 51.9 | 54.7 | 53.6 | 56.2 |
| 9 | Amoxicillin | 54.0 | 51.3 | 52.9 | 52.4 | 53.8 |
| 10 | Alprazolam | 41.4 | 43.3 | 45.3 | 47.7 | 49.1 |
| 11 | Hydrochlorothiazide | 48.5 | 48.5 | 47.9 | 47.8 | 48.1 |
| 12 | Zolpidem tartrate | 34.5 | 39.1 | 42.7 | 43.7 | 44.6 |
| 13 | Atorvastatin | 65.8 | 58.5 | 51.7 | 45.3 | 43.3 |
| 14 | Furosemide | 44.7 | 44.4 | 43.8 | 43.6 | 42.3 |
| 15 | Oxycodone/acetaminophen | 31.3 | 33.6 | 36.7 | 37.9 | 38.8 |
| 16 | Fluticasone | 23.9 | 26.2 | 30.1 | 34.8 | 38.4 |
| 17 | Citalopram HBR | 18.1 | 22.6 | 27.3 | 32.2 | 37.8 |
| 18 | Metoprolol tartrate | 43.5 | 38.4 | 41.1 | 38.9 | 37.8 |
| 19 | Sertraline HCL | 33.4 | 33.7 | 34.8 | 36.2 | 37.6 |
| 20 | Metoprolol succinate | 33.0 | 41.5 | 26.9 | 33.0 | 34.5 |
| 21 | Warfarin sodium | 34.4 | 34.9 | 35.7 | 35.6 | 33.9 |
| 22 | Tramadol HCL | 20.6 | 23.3 | 25.5 | 28.0 | 33.9 |
| 23 | Potassium | 36.7 | 35.8 | 35.2 | 34.7 | 33.7 |
| 24 | Prednisone | 25.9 | 27.1 | 27.8 | 28.7 | 33.7 |
| 25 | Atenolol | 45.0 | 42.0 | 39.5 | 36.4 | 33.4 |

Source: IMS Health, National Prescription Audit, Dec. 2011 (175).
Notes: Report reflects prescription-bound products including insulins and excluding other products such as OTC. Table shows leading active-ingredients or ingredient fixed-combinations, and includes those produced by both branded and generic manufacturers. Includes all prescriptions dispensed through retail pharmacies - including independent and chain drug stores, food store pharmacies and mail order as well as long-term care facilities. Prescription counts are not adjusted for length of therapy. 90-day and 30-day prescriptions are both counted as one prescription.

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of which more than 80% of is consumed in the United States. The International Narcotics Board also reports that U.S. demand for hydrocodone, the most commonly prescribed opioid, is about 27.4 million grams annually compared to 3,237 grams for Britain, France, Germany, and Italy combined (61,177,178).

Caudill-Slosberg et al (165) in one of the earliest evaluations demonstrated that opioid use doubled from 8% in 1980 to 16% in 2000. The data also illustrates that from 1999 to 2002, 4.2% of U.S. adults reported the use of opioid analgesics for pain within the past month (179). In a report of opioid use in one of the states in the United States (Utah) (180), the data showed that 20.8% of adults had been prescribed an opioid in the last year and that 29.1% of these prescriptions were for long-term pain. Sullivan et al (181) also showed over a 6 year period that the proportion of enrollees receiving opioids with a diagnosis of chronic




Source: IMS Health, LifeLink, Dec 2011

Fig. 12. *Treated patients in selected therapy.*

Table 7. *Spending based on the therapeutic class.*

| SPENDING $BN | | 2007 | 2008 | 2009 | 2010 | 2011 |
|---|---|---|---|---|---|---|
| Total US Market | | 280.5 | 285.7 | 300.7 | 308.6 | 319.9 |
| 1 | Oncologics | 18.1 | 19.7 | 21.5 | 22.3 | 23.2 |
| 2 | Respiratory Agents | 15.1 | 16.0 | 18.1 | 19.3 | 21.0 |
| 3 | Lipid Regulators | 19.4 | 18.1 | 18.6 | 18.8 | 20.1 |
| 4 | Antidiabetics | 12.2 | 13.6 | 15.8 | 17.7 | 19.6 |
| 5 | Antipsychotics | 12.8 | 14.3 | 14.7 | 16.2 | 18.2 |
| 6 | Autoimmune Diseases | 7.6 | 8.6 | 9.7 | 10.6 | 12.0 |
| 7 | Antidepressant | 11.7 | 11.7 | 11.5 | 11.6 | 11.0 |
| 8 | HIV Antivirals | 6.2 | 7.1 | 8.2 | 9.3 | 10.3 |
| 9 | Anti-Ulcerants | 14.6 | 14.2 | 14.1 | 11.9 | 10.1 |
| 10 | Narcotic Analgesics | 6.7 | 7.3 | 8.0 | 8.4 | 8.3 |
| 11 | ADHD | 4.0 | 4.7 | 5.8 | 6.7 | 7.9 |

| SPENDING $BN | | 2007 | 2008 | 2009 | 2010 | 2011 |
|---|---|---|---|---|---|---|
| 12 | Platelet Aggregation Inhibitors | 5.0 | 5.7 | 6.5 | 7.1 | 7.8 |
| 13 | Angiotensin II Inhibitors | 6.5 | 7.6 | 8.6 | 8.7 | 7.6 |
| 14 | Multiple Sclerosis | 3.4 | 4.1 | 5.0 | 5.8 | 7.1 |
| 15 | Vaccines (Pure, Comb, Other) | 5.9 | 5.0 | 4.7 | 5.7 | 6.3 |
| 16 | Anti-Epileptics | 10.0 | 11.1 | 6.9 | 5.6 | 5.9 |
| 17 | Erythropoietins | 4.1 | 4.5 | 4.7 | 4.8 | 5.2 |
| 18 | Immunostimulating Agents | 8.4 | 6.9 | 6.3 | 6.1 | 5.1 |
| 19 | Hormonal Contraceptives | 4.1 | 4.1 | 4.1 | 4.2 | 4.5 |
| 20 | Antivirals, excl. Anti-HIV | 3.6 | 3.9 | 4.8 | 3.2 | 3.7 |

Source: IMS Health, National Prescription Audit, Dec. 2011 (175).

Notes:
Therapy areas are based on proprietary IMS Health definitions. Report reflects prescription-bound products including insulins and excluding other products such as OTC. Includes all prescriptions dispensed through retail pharmacies - including independent and chain drug stores, food store pharmacies and mail order as well as long-term care facilities. Prescription counts are not adjusted for length of therapy. 90-day and 30-day prescriptions are both counted as one prescription.

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Table 8. *Top therapeutic classes by prescriptions.*

| DISPENSED PRESCRIPTIONS MN | | 2007 | 2008 | 2009 | 2010 | 2011 |
|---|---|---|---|---|---|---|
| Total US Market | | 3,825 | 3,866 | 3,949 | 3,993 | 4,024 |
| 1 | Antidepressants | 237 | 241 | 247 | 254 | 264 |
| 2 | Lipid Regulators | 233 | 242 | 254 | 260 | 260 |
| 3 | Narcotic Analgesics | 231 | 239 | 241 | 244 | 238 |
| 4 | Antidiabetics | 165 | 166 | 169 | 172 | 173 |
| 5 | Ace Inhibitors (Plain & Combo) | 159 | 163 | 166 | 168 | 164 |
| 6 | Beta Blockers (Plain & Combo) | 162 | 164 | 163 | 162 | 161 |
| 7 | Respiratory Agents | 147 | 147 | 152 | 153 | 153 |
| 8 | Anti-Ulcerants | 134 | 139 | 146 | 147 | 150 |
| 9 | Diuretics | 137 | 135 | 132 | 131 | 128 |
| 10 | Anti-Epileptics | 102 | 110 | 116 | 122 | 128 |
| 11 | Tranquillizers | 98 | 101 | 104 | 108 | 111 |
| 12 | Thyroid Preparations | 103 | 104 | 105 | 107 | 110 |
| 13 | Calcium Antagonists (Plain & Combo) | 87 | 90 | 93 | 96 | 98 |
| 14 | Antirheumatic Non-Steroid | 90 | 91 | 92 | 93 | 97 |
| 15 | Hormonal Contraceptives | 94 | 94 | 93 | 91 | 90 |
| 16 | Angiotensin II Inhibitors | 83 | 86 | 85 | 84 | 86 |
| 17 | Broad Spectrum Penicillins | 77 | 74 | 77 | 76 | 77 |
| 18 | Macrolides & Similar Type Antibiotics | 63 | 66 | 69 | 67 | 69 |
| 19 | Hypnotics & Sedatives | 58 | 60 | 63 | 63 | 63 |
| 20 | Vitamins & Minerals | 60 | 59 | 58 | 58 | 60 |

Source: IMS Health, National Prescription Audit, Dec. 2011 (175).
Appendix notes:
Therapy areas are based on proprietary IMS Health definitions. Report reflects prescription-bound products including insulins and excluding other products such as OTC. Includes all prescriptions dispensed through retail pharmacies - including independent and chain drug stores, food store pharmacies and mail order as well as long-term care facilities. Prescription counts are not adjusted for length of therapy. 90-day and 30-day prescriptions are both counted as one prescription.

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non-cancer pain and opioid prescriptions increase. Opioids are also used commonly in combination with sedative hypnotics. Vogt et al (182) in an evaluation of analgesic usage for low back pain and its impact on health care costs and service use showed that in 2001, a total of $1.4 million was spent on opioids, which constituted 68% of prescriptions for analgesics.

The data from reports and pain management settings is disconcerting. Over 90% of patients received opioids for chronic pain management (32,169,172,183-188). Even more alarming, however, is the fact that the majority of the prescriptions are from outside pain management settings. Volkow et al (172) showed that only a small proportion of prescriptions were from pain clinics or specialists from anesthesiology in 2009. Moreover, Deyo et al (31) illustrated that approximately 20% of patients in primary care settings were long-time opioid users with 61% receiving a course of opioids. In young veterans, Wu et al (189) showed that prevalence of chronic opioid use increased from 3% in 2003 to 4.5% in 2007. Patients on average were exposed to 2 different opioids and had 3 different opioid prescribers. Not surprisingly, 80% of the opioid prescriptions during the study were prescribed by pri-

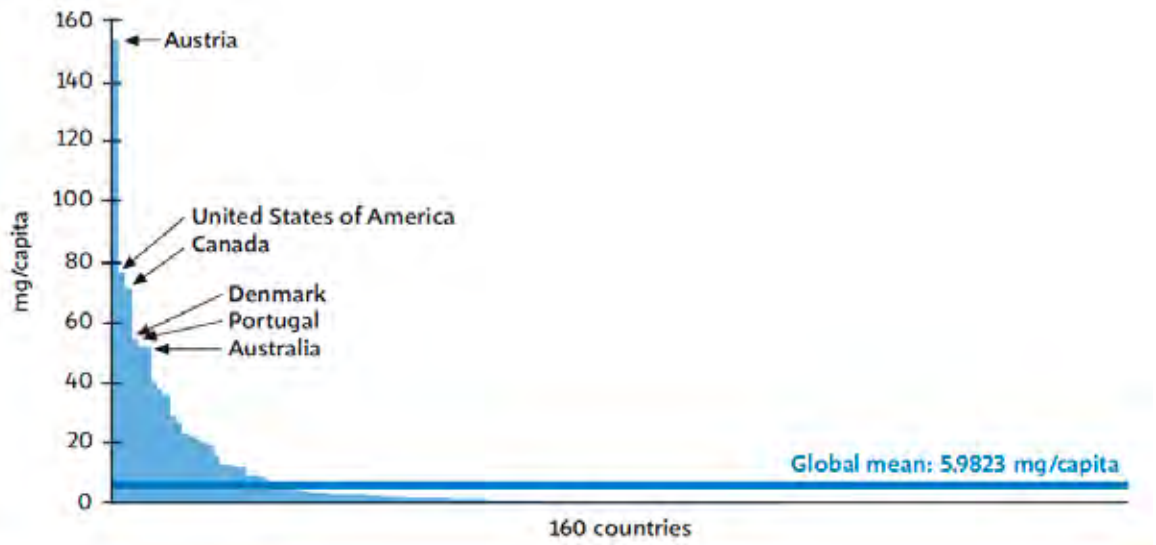



Fig. 13. *Global morphine consumption in 2007 (mg/capita).*
Source: International Narcotics Control Board, United Nations data. Graphic created by the Pain and Policy Study Group, University of Wisconsin/WHO Collaborating Center, 2009.

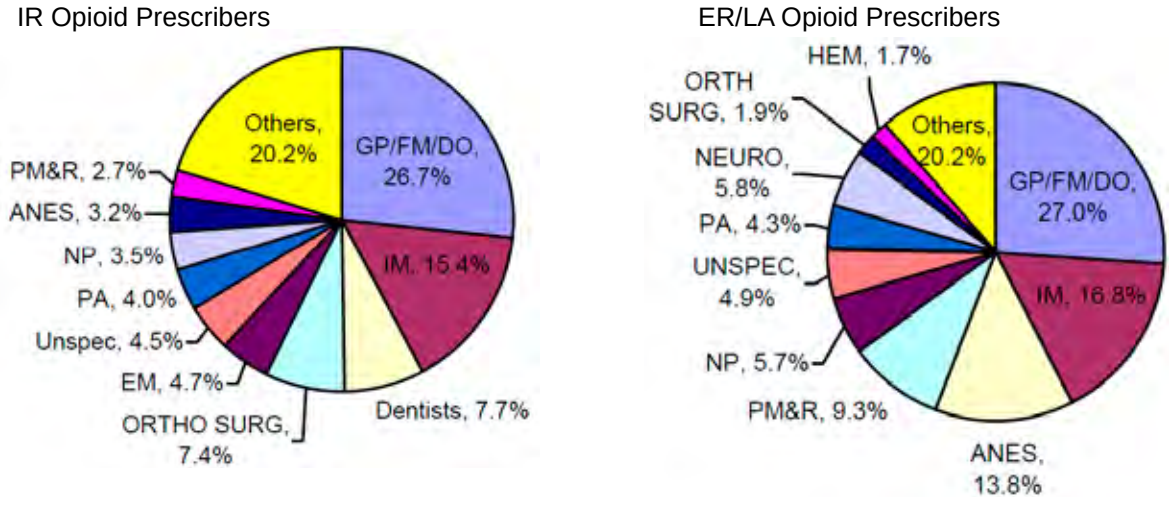



Fig. 14. *Total number of prescriptions dispensed in the U.S. by top 10 prescribing specialties for IR and ER/ LA opioids, year 2009 (173).* Source: SDI, Vector One ®: National (174).

mary care providers, and less than 1% was from pain specialists.

In fact, the data illustrates that in 2009 (Fig. 14). among the top 10 specialties of those prescribing immediate release opioids were general practitioners/family medicine 26.7%, internal medicine 15.4%, anesthesiologists constituting 3.2%, and physical medicine and rehabilitation specialists constituting 2.7% (173,174). In contrast, for ER or long-acting opioids in 2009, anesthesiologists constituted 13.8% and physical medicine and rehabilitation constituted 9.3%, with general practitioners, family medical doctors, osteopaths, and internal medicine specialists still dominating the field with 27% and 16.8%, in essence exceeding their prescriptions of immediate release opioids (173,174).

## 5.0 Relationship of Escalating Opioid Use and Adverse Consequences

While numerous adverse effects have been reported, ever increasing opioid related fatalities, including drug poisoning deaths, are crucial. In the United States, in 2008, one or more prescription drugs were involved in 20,044 of the 27,153 deaths with a specified drug. Opioid pain relievers were involved in 14,800 drug overdose deaths, compared to 11,500 of 27,500 fatal unintended drug overdose deaths in 2007 – an increase of 3,300 in just one year (160). Alarmingly, in 2007 there were more opioid analgesic overdose deaths than overdoses involving heroin and cocaine combined (Fig. 15). In addition, during the same time frame, drug-related suicides also increased, with opioid analgesics being involved in roughly 3,000 of the 8,400 overdose deaths in the United States in 2007 that were suicide or of undetermined intent (190). Complicating these grave statistics, for every unintentional overdose death related to an opioid analgesic, 9 are admitted for substance abuse treatment, 35 visit emergency departments, 161 report drug abuse or dependence, and 461 report non-medical uses of opioid analgesics (34). Not surprisingly, in 2007, non-suicidal drug poisoning deaths exceeded both motor vehicle traffic and suicide deaths in 20 states, with data from Ohio illustrating that the number of deaths from unintentional drug poisoning surpassed the numbers of deaths from both suicide and motor vehicle crashes combined (190-192). Thus, it has been concluded that opioid analgesics contributed to fatalities based on opioid abuse and increasing doses, doctor shopping, and other aspects of drug abuse as illustrated in Fig. 16 (160). The data from emergency department visits sadly illustrate that opioids, sedatives, and non-prescription sleep aides are often taken more than prescribed or solely for the feeling they cause, and that this trend is steadily increasing (170).

The Centers for Disease Control and Prevention (CDC) (34) also reported the percentage of prescription drug overdoses by risk group in the United States. This report showed that approximately 80% of prescribed low-doses (less than 100 mg of morphine equivalent dose per day – considered as high dose by many) were by a single practitioner, accounting for an estimated 20% of all prescription overdoses (Fig. 17). In contrast, among the remaining 20% of patients, 10% of prescribed high doses (greater than 100 mg morphine equivalent dose per day) (193-195) per day of opioids by single prescribers account for an estimated 40% of the prescription opioid overdoses (131,195). The remaining 10% of patients seeing multiple doctors and typically involved in drug diversion contribute to 40% of overdoses (152). Furthermore, among persons who died of opioid overdoses, a significant proportion did not have a prescription in their records for the opioid that killed them; in West Virginia, Utah, and Ohio, 25% to 66% of those who died of pharmaceutical overdose used opioids originally prescribed to someone

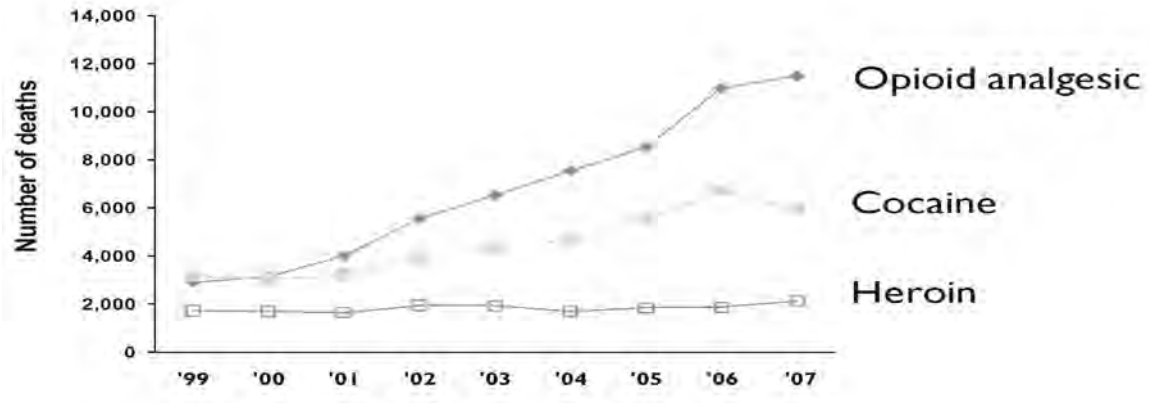
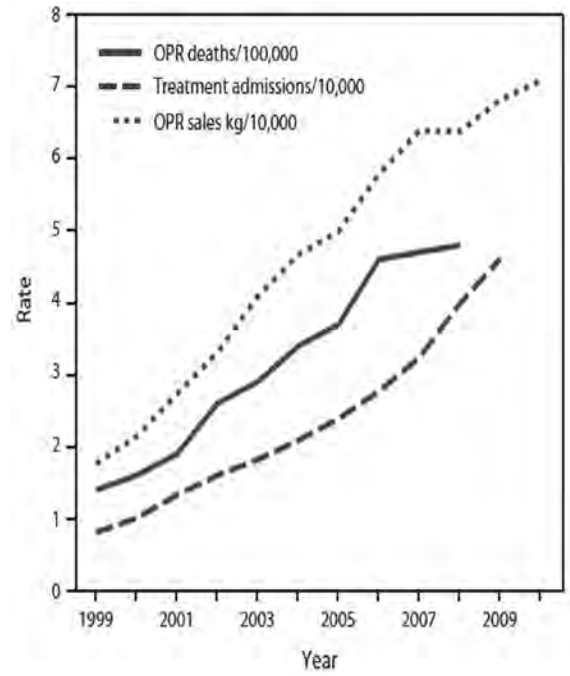



Fig. 15. *Deaths from unintentional drug overdoses in the United States according to major type of drug, 1999-2007.*
Source: Centers for Disease Control and Prevention. Unintentional Drug Poisoning in the United States. July 2010. http://www.cdc.gov/HomeandRecreationalSafety/pdf/poison-issue-brief.pdf (190).




* Age-adjusted rates per 100,000 population for OPR deaths, crude rates per 10,000 population for OPR abuse treatment admissions, and crude rates per 10,000 population for kilograms of OPR sold.

Fig. 16. *Rates of opioid pain reliever overdose death, opioid pain relief treatment admissions, and kilograms of opioid pain relievers sold – United States, 1999-2010.*
Source: Centers for Disease Control and Prevention. Vital signs: Overdoses of prescription opioid pain relievers – United States, 1999-2008. MMWR. Morb. Mortal Wkly. Rep. 60, 1487-1492 (2011) (160).

else (152,192,196).

The responsible opioid prescription community considers that the adverse consequences of appropriately prescribed and used opioids are least considered, as the blame is placed predominantly on abuses and overuses (49,71,116-119). Consequently, it is coupled with a lack of evidence regarding long-term benefits and ample evidence that the increased prescription of opioids is fueling an epidemic of addiction and overdose deaths. This crisis is rooted in a lack of education and misinformation, leading to overprescribing and a tendency to focus on ineffective strategies (49,71,197-199). In fact, the majority of cases involving injury and death occur in people using opioids exactly as prescribed, not just those misusing or abusing them (71). Even more importantly, most studies indicate that patients on long-term opioid therapy are unlikely to stop even if analgesia and function are poor and safety issues arise. Frequently, despite good relief and improvement in function with modalities other than opioids including interventional techniques and surgery, patients continue on opioids (200-215).

Even though there is no evidence to support the previous teaching that long-acting opioids can provide better analgesia, and less risk for abuse than immediate release products (32,71,96,100,103,107,116-119,216), the use of higher doses, with a combination of short-acting and long-acting opioids, continues to escalate. Thus, it is believed that commencing long-acting opioid therapy is often the starting point for high dose opi-

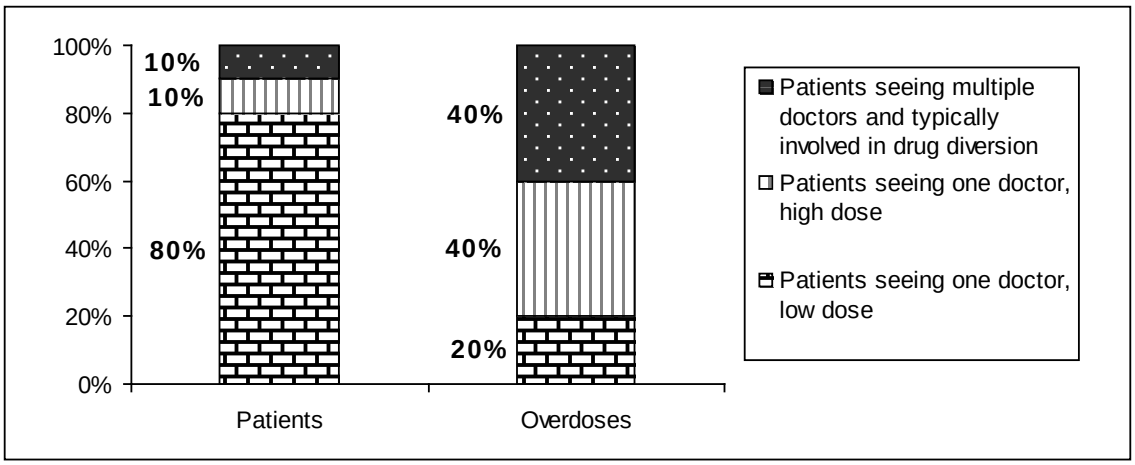



Fig. 17. *Percentage of patients and prescription drug overdoses, by risk group – United States.*
Source: Centers for Disease Control and Prevention. CDC grand rounds: Prescription drug overdoses – a U.S. epidemic. MMWR. Morb. Mortal Wkly. Rep. 61, 10-13 (2012) (34).

oid therapy, a practice that growing evidence suggests is harmful to patients and increases the black market availability of opioids through diversion (71,217-222).

Multiple studies in the literature (23,32,37,46-49,223-236) have reported an association between opioid prescribing and overall health status, with increased disability, medical costs, subsequent surgery, and continued or late opioid use. Overall, the epidemiologic studies are less positive with regards to improvement in function and quality of life with opioids in chronic pain patients (110,116-119,170,232,237). In fact, in an epidemiologic study from Denmark (23) where opioids are prescribed liberally for chronic pain, it was demonstrated that in patients receiving opioids, pain was worse, health care utilization was higher, and activity levels were lower compared to a matched cohort of chronic pain patients not using opioids. This study suggested that when opioids are prescribed liberally, even if some patients benefit, the overall population does not. Another study (33) also reported worse pain, higher health care utilization, and lower activity levels in opioid-treated patients compared to matched cohort of chronic pain patients not using opioids. Sjøgren et al (49) in a population-based cohort study on chronic pain and the role of opioids, showed that the odds of recovery from chronic pain were almost 4 times higher among individuals not using opioids compared with individuals using opioids. In addition, they also showed that use of strong opioids was associated with poor health-related quality of life, and higher risk of death. In addition, opioid abuse in chronic pain has been highly prevalent, along with illicit drug usage in addition to misuse or abuse of therapeutic opioids (32,143-152,183-188).

## Conclusion

What emerges from the available data utilized in this review is the conclusion that over the past 20 years there has been an escalation of the therapeutic use of opioids and other psychotherapeutics as well as their abuse and nonmedical use. As a consequence of the fact that hydrocodone has become the number one prescribed medication in America, it is not difficult to see the significant impact that this has had on the overall patterns of abuse and nonmedical use, particularly since the illicit use of prescribed psychotherapeutics (including opioids, which are currently at the top of that list) now overshadows the use of nonprescription illicit drugs. Drug dealers are no longer the primary source of illicit drugs. Our greatest enemy is now inappropriate prescribing patterns, based on a lack of knowledge, perceived safety, and undertreatment of pain.

## Acknowledgments

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## References

1. Institute of Medicine (IOM). *Relieving Pain in America: A Blueprint for Transforming Prevention, Care, Education, and Research*. The National Academies Press, Washington, DC, June 29, 2011.
2. Pizzo PA, Clark NM. Alleviating suffering 101 – Pain relief in the United States. *N Engl J Med* 2012; 367:197-198.
3. Harkness EF, Macfarlane GJ, Silman AJ, McBeth J. Is musculoskeletal pain more common now than 40 years ago?: Two population-based cross-sectional studies. *Rheumatology (Oxford.* 2005; 44:890-895.
4. Freburger JK, Holmes GM, Agans RP, Jackman AM, Darter JD, Wallace AS, Castel LD, Kalsbeek WD, Carey TS. The rising prevalence of chronic low back pain. *Arch Intern Med* 2009; 169:251-258.
5. Manchikanti L, Singh V, Datta S, Cohen SP, Hirsch JA. Comprehensive review of epidemiology, scope, and impact of spinal pain. *Pain Physician* 2009; 12:E35-E70.
6. Hoy DG, Bain C, Williams G, March L, Brooks P, Blyth F, Woolf A, Vos T, Buchbinder R. A systematic review of the global prevalence of low back pain. *Arthritis Rheum.* 2012 Jan 9. [Epub ahead of print].
7. Hoy D, Brooks P, Blyth F, Buchbinder R. The epidemiology of low back pain. *Best Pract Res Clin Rheumatol* 2010; 24:769-781.
8. Hoy DG, Protani M, De R, Buchbinder R. The epidemiology of neck pain. *Best Pract Res Clin Rheumatol* 2010; 24:783-792.
9. Reid KJ, Harker J, Bala MM, Truyers C, Kellen E, Bekkering GE, Kleijnen J. Epidemiology of chronic non-cancer pain in Europe: Narrative review of prevalence, pain treatments and pain impact. *Curr Med Res Opin* 2011; 27:449-462.
10. Bekkering GE, Bala MM, Reid K, Kellen E, Harker J, Riemsma R, Huygen FJ, Kleijnen J. Epidemiology of chronic pain and its treatment in The Netherlands. *Neth J Med* 2011; 69:141-153.
11. Langley PC. The prevalence, correlates and treatment of pain in the European Union. *Curr Med Res Opin* 2011; 27:463-480.
12. Tosato M, Lukas A, van der Roest HG, Danese P, Antocicco M, Finne-Soveri H,

Nikolaus T, Landi F, Bernabei R, Onder G. Association of pain with behavioral and psychiatric symptoms among nursing home residents with cognitive impairment: Results from the SHELTER study. *Pain* 2012; 153:305-310.

13. Clark JD. Chronic pain prevalence and analgesic prescribing in a general medical population. *J Pain Symptom Manage* 2002; 23:131-137.
14. Eriksen J. Epidemiology of chronic nonmalignant pain in Denmark. *Pain* 2003; 106:221-228.
15. Gureje O, Von Korff M, Simon GE, Gater R. Persistent pain and well-being: a World Health Organization study in primary care. *JAMA* 1998; 280:147-151.
16. Moulin DE, Clark AJ, Speechley M, Morley-Forster PK. Chronic pain in Canada – prevalence, treatment, impact and the role of opioid analgesia. *Pain Res Manag* 2002; 7:179-184.
17. Sjøgren P, Ekholm O, Peuckmann V, Grønbæk M. Epidemiology of chronic pain in Denmark: an update. *Eur J Pain* 2009; 13:287-292.
18. Elliott AM, Smith BH, Penny KI, Smith WC, Chambers WA. The epidemiology of chronic pain in the community. *Lancet* 1999; 354:1248-1252.
19. Eriksen J, Ekholm O, Sjøgren P, Rasmussen NK. Development of and recovery from long-term pain. A 6-year follow-up study of a cross-section of the adult Danish population. *Pain* 2004; 108:154-162.
20. Lawrence RC, Helmick CG, Arnett FC. Estimates of the prevalence of arthritis and selected musculoskeletal disorders in the United States. *Arthritis Rheum* 1998; 41:778-799.
21. Verhaak PF, Kerssens JJ, Dekker J, Sorbi MJ, Bensing JM. Prevalence of chronic benign pain disorder among adults: a review of the literature. *Pain* 1998; 77:231-239.
22. Torrance N, Smith BH, Bennett MI, Lee AJ. The epidemiology of chronic pain of predominantly neuropathic origin. Results from a general population survey. *J Pain* 2006; 7:281-289.
23. Breivik H, Collett B, Ventafridda V, Cohen R, Gallacher D. Survey of chronic pain in Europe: Prevalence, impact on daily life, and treatment. *Eur J Pain* 2006; 10:287-333.
24. Blyth FM, Rochat S, Cumming RG, Creasey H, Handelsman DJ, Le Couteur DG, Naganathan V, Sambrook PN, Seibel MJ, Waite LM. Pain, frailty and comorbidity on older men: the CHAMP study. *Pain* 2008; 140:224-230.
25. Cassidy JD, Carroll LJ, Côté P. The Saskatchewan Health and Back Pain Survey. The prevalence of low back pain and related disability in Saskatchewan adults. *Spine (Phila Pa 1976)* 1998; 23:1860-1867.
26. Côté P, Cassidy JD, Carroll L. The Saskatchewan Health and Back Pain Survey. The prevalence of neck pain and related disability in Saskatchewan adults. *Spine (Phila Pa 1976)* 1998; 23:1689-1698.
27. Leboeuf-Yde C, Nielsen J, Kyvik KO, Fejer R, Hartvigsen J. Pain in the lumbar, thoracic or cervical regions: Do age or gender matter? A population-based study of 34,902 Danish twins 20-71 years of age. *BMC Musculoskeletal Disorders* 2009; 39.
28. Carroll LJ, Cassidy JD, Peloso PM, Giles-Smith L, Cheng CS, Greenhalgh SW, Haldeman S, van der Velde G, Hurwitz EL, Côté P, Nordin M, Hogg-Johnson S, Holm LW, Guzman J, Carragee EJ. Bone and Joint Decade 2000–2010 Task Force on Neck Pain and Its Associated Disorders. The burden and determinants of neck pain in the general population: Results of the Bone and Joint Decade 2000–2010 Task Force on Neck Pain and its associated disorders. *Spine (Phila Pa 1976)* 2008; 33:S39-S51.
29. Côté P, Kristman V, Vidmar M, Van Eerd D, Hogg-Johnson S, Beaton D, Smith PM. The prevalence and incidence of work absenteeism involving neck pain: a cohort of Ontario lost-time claimants. *Spine (Phila Pa 1976)* 2008; 33:S192-S198.
30. Côté P, Cassidy JD, Carroll L. The factors associated with neck pain and its related disability in the Saskatchewan population. *Spine (Phila Pa 1976)* 2000; 25:1109-1117.
31. Deyo RA, Smith DH, Johnson ES, Donovan M, Tillotson CJ, Yang X, Petrik AF, Dobscha SK. Opioids for back pain patients: Primary care prescribing patterns and use of services. *J Am Board Fam Med* 2011; 24:717-727.
32. Manchikanti L, Fellows B, Ailinani H, Pampati V. Therapeutic use, abuse, and nonmedical use of opioids: A ten-year perspective. *Pain Physician* 2010; 13:401-435.
33. Eriksen J, Sjøgren P, Bruera E, Ekholm O, Rasmussen NK. Critical issues on opioids in chronic non-cancer pain: An epidemiological study. *Pain* 2006; 125:172-179.
34. Centers for Disease Control and Prevention. CDC grand rounds: Prescription drug overdoses – a U.S. epidemic. *MMWR Morb Mortal Wkly Rep* 2012; 61:10-13.
35. Manchikanti L, Benyamin R, Datta S, Vallejo R, Smith HS. Opioids in chronic noncancer pain. *Expert Rev Neurother* 2010; 10:775-789.
36. Ballantyne J, LaForge K. Opioid dependence and addiction during opioid treatment of chronic pain. *Pain* 2007; 129:235-255.
37. US Government Accountability Office: Report to Congressional Requesters. Prescription Pain Reliever Abuse, December 2011. www.gao.gov/assets/590/587301.pdf
38. Toblin RL, Mack KA, Perveen G, Paulozzi LJ. A population-based survey of chronic pain and its treatment with prescription drugs. *Pain* 2011; 152:1249-1255.
39. Okie S. A flood of opioids, a rising tide of deaths. *N Engl J Med* 2010; 363:1981-1985.
40. Martin BI, Turner JA, Mirza SK, Lee MJ, Comstock BA, Deyo RA. Trends in health care expenditures, utilization, and health status among US adults with spine problems, 1997-2006. *Spine (Phila Pa 1976)* 2009 34:2077-2084.
41. Webster LR. Ending unnecessary opioid-related deaths: A national priority. *Pain Med* 2011; 12:S13-S15.
42. Collen M. Profit-Driven Drug Testing. *J Pain Palliat Care Pharmacother* 2012; 26:13-17.
43. Braden JB, Russo J, Fan MY, Edlund MJ, Martin BC, DeVries A, Sullivan MD. Emergency department visits among recipients of chronic opioid therapy. *Arch Intern Med* 2010; 170:1425-1432.
44. Volkow ND, McLellan TA. Curtailing diversion and abuse of opioid analgesics without jeopardizing pain treatment. *JAMA* 2011; 305:1346-1347.
45. Paulozzi LJ, Kilbourne EM, Shah NG, Nolte KB, Desai HA, Landen MG, Harvey W, Loring LD. A history of being prescribed controlled substances and risk of drug overdose death. *Pain Med* 2012; 13:87-95.
46. Cicero TJ, Wong G, Tian Y, Lynskey M, Todorov A, Isenberg K. Co-morbidity and utilization of medical services by pain patients receiving opioid medications: Data from an insurance claims database. *Pain* 2009; 144:20-27.

47. Lavin R, Park J. Depressive symptoms in community-dwelling older adults receiving opioid therapy for chronic pain. *J Opioid Manag* 2011; 7:309-319.
48. Kidner CL, Mayer TG, Gatchel RJ. Higher opioid doses predict poorer functional outcome in patients with chronic disabling occupational musculoskeletal disorders. *J Bone Joint Surg Am* 2009; 91:919-927.
49. Sjøgren P, Grønbæk M, Peuckmann V, Ekholm O. A population-based cohort study on chronic pain: the role of opioids. *Clin J Pain* 2010; 26:763-769.
50. Fauber J. Painkiller boom fueled by networking. *Journal Sentinel*, Feb. 18, 2012.
51. Murnion BP, Gnjidic D, Hilmer SN. Prescription and administration of opioids to hospital in-patients, and barriers to effective use. *Pain Med* 2010; 11:58-66.
52. Apfelbaum JL, Chen C, Mehta SS, Gan TJ. Postoperative pain experience: Results from a national survey suggest postoperative pain continues to be undermanaged. *Anesth Analg* 2003; 97:534-540.
53. Dix P, Sandhar B, Murdoch J, MacIntyre PA. Pain on medical wards in a district general hospital. *Br J Anaesth* 2004; 92:235-237.
54. Brown D, McCormack B. Determining factors that have an impact upon effective evidence-based pain management with older people, following colorectal surgery: An ethnographic study. *J Clin Nurs* 2006; 15:1287-1298.
55. Ljungberg C, Lindblad AK, Tully MP. Hospital doctors' views of factors influencing their prescribing. *J Eval Clin Pract* 2007; 13:765-771.
56. Auret K, Schug SA. Underutilisation of opioids in elderly patients with chronic pain: Approaches to correcting the problem. *Drugs Aging* 2005; 22:641-654.
57. McCarberg BH, Barkin RL. Long-acting opioids for chronic pain: Pharmacotherapeutic opportunities to enhance compliance, quality of life, and analgesia. *Am J Ther* 2001; 8:181-186.
58. McErlean M, Triner W. Young A. Impact of outside regulatory investigation on opiate administration in the emergency department. *J Pain* 2006; 7:947-950.
59. Gnjidic D, Murnion BP, Hilmer SN. Age and opioid analgesia in an acute hospital population. *Age Ageing* 2008; 37(6):699-702.
60. Ensuring availability of controlled medications for the relief of pain and preventing diversion and abuse: Striking the right balance to achieve the optimal public health outcome. Discussion paper based on a scientific workshop, January 18-19, 2011, UNODC, Vienna.
61. International Narcotics Control Board, Report of the International Narcotics Control Board on the availability of internationally controlled drugs: Ensuring adequate access for medical and scientific purposes. New York 2011.
62. Nicholson BD. Panel Discussion. *Pain Med* 2012; 13:S21-S22.
63. Nicholson BD. Introduction. *Pain Med* 2012; 13:S1-S3.
64. A fresh view of opioids. *The BackLetter* 2011; 26:4, 37.
65. Federation of State Medical Boards of the US. Model guidelines for the use of controlled substances for the treatment of pain: A policy document of the Federation of State Medical Boards of the United States, Inc. Dallas, TX, 1998.
66. Phillips DM. JCAHO pain management standards are unveiled. Joint Commission on Accreditation of Healthcare Organizations. *JAMA* 2000; 284:428-429.
67. Cohen MZ, Easley MK, Ellis C, Hughes B, Ownby K, Rashad BG, Rude M, Taft E, Westbrooks JB. JCAHO cancer pain management and the JCAHO's pain standards: An institutional challenge. *J Pain Symptom Manage* 2003; 25:519-527.
68. Frasco PE, Sprung J, Trentman TL. The impact of the Joint Commission for Accreditation of Healthcare Organizations pain initiative on perioperative opiate consumption and recovery room length of stay. *Anesth Analg* 2005; 100:162-168.
69. Tormoehlen LM, Mowry JB, Bodle JD, Rusyniak DE. Increased adolescent opioid use and complications reported to a poison control center following the 2000 JCAHO pain initiative. *Clin Toxicol (Phila)* 2011; 49:492-498.
70. Mularski RA, White-Chu F, Overbay D, Miller L, Asch SM, Ganzini L. Measuring pain as the 5th vital sign does not improve quality of pain management. *J Gen Intern Med* 2006; 21:607-612.
71. Letter to Janet Woodcock, MD, Director, Center for Drug Evaluation and Research, U.S Food and Drug Administration, from Physicians for Responsible Opioid Prescribing RE Docket No. FDA-2011-D-0771, Draft Blueprint for Prescriber Education for Long-Acting/Extended Release Opioid Class-Wide Risk Evaluation and Mitigation Strategies. 2 December, 2011.
72. Nickerson JW, Attaran A. The inadequate treatment of pain: collateral damage from the war on drugs. *PLoS Med* 2012; 9:e1001153.
73. Small D, Drucker E. Return to Galileo? The Inquisition of the International Narcotic Control Board. *Harm Reduct J* 2008; 5:16.
74. Gilson AM, Joranson DE, Maurer MA. Improving state pain policies: recent progress and continuing opportunities. *CA Cancer J Clin* 2007; 57:341-353.
75. Gilson AM, Maurer MA, Joranson DE. State medical board members' beliefs about pain, addiction, and diversion and abuse: A changing regulatory environment. *J Pain* 2007; 8:682-691.
76. Taylor AL. Addressing the global tragedy of needless pain: Rethinking the United Nations single convention on narcotic drugs. *J Law Med Ethics* 2007; 35:556-570, 511.
77. Lipman AG. Pain as a human right: The 2004 Global Day Against Pain. *J Pain Palliat Care Pharmacother* 2005; 19:85-100.
78. Dilcher AJ. Damned if they do, damned if they don't: The need for a comprehensive public policy to address the inadequate management of pain. *Ann Health Law* 2004; 13:81-144, table of contents.
79. Ghodse H. Pain, anxiety and insomnia--a global perspective on the relief of suffering: Comparative review. *Br J Psychiatry* 2003; 183:15-21.
80. Gostomzyk JG, Heller WD. Prescribing strong opioids for pain therapy and for substitution therapy by established physicians. *Schmerz* 1996; 10:292-298.
81. Lohman D, Schleifer R, Amon JJ. Access to pain treatment as a human right. *BMC Med* 2010; 8:8.
82. Inadequate pain treatment is a public health crisis. Drug war shouldn't claim new victims. *StarTribune,* April 21, 2011. www.startribune.com/opinion/editorials/120420264.html
83. Canadian groups welcome international report condemning failed "War On Drugs." June 2, 2011. www.aidslaw.ca/publications/interfaces/downloadFile.php?ref=1886
84. [No authors listed]. Pain management failing as fears of prescription drug abuse rise. *J Pain Palliat Care Pharmacother* 2010; 24:182-183.
85. Varrassi G, Müller-Schwefe GH. The international CHANGE PAIN Physician Survey: Does specialism influence the

perception of pain and its treatment? *Curr Med Res Opin* 2012; Mar 28 [Epub ahead of print].

86. War on Drugs. Report of the Global Commission on Drug Policy. June 2011. www.globalcommissionondrugs.org/reports/
87. Milani B, Scholten W. World Health Organization. The World Medicines Situation 2011: Access to Controlled Medicines. http://apps.who.int/medicinedocs/documents/s18062en/s18062en.pdf
88. Dasgupta N, Mandl K, Brownstein J. Breaking the news or fueling the epidemic? Temporal association between news media report volume and opioid-related mortality. *PLoS ONE* 2009; 4:e7758.
89. Are we winning the war on drugs? Is it worth the cost? *MD Health Network*. www.mdhealthnetwork.org/prescription-drugs/war_on_drugs.html
90. Painful drug war victory. *The Washington Times*. August 16, 2007. www.washingtontimes.com/news/2007/aug/16/painful-drug-war-victory/?page=all
91. Fauber J. UW a force in pain drug growth. *JSOnline*. 2 April, 2011.
92. Fauber J. Academics profit by making the case for opioid painkillers. *MedPage Today*. 3 April, 2011.
93. Manchikanti L, Benyamin RM, Falco FJE, Caraway DL, Datta S, Hirsch JA. Guidelines warfare over interventional techniques: Is there a lack of discourse or straw man? *Pain Physician* 2012; 15:E1-E26.
94. Portenoy RK, Foley KM. Chronic use of opioid analgesics in non-malignant pain: Report of 38 cases. *Pain* 1986; 25:171-186.
95. Ballantyne JC. Opioid analgesia: Perspectives on right use and utility. *Pain Physician* 2007; 10:479-491.
96. Chou R, Huffman L. *Use of Chronic Opioid Therapy in Chronic Noncancer Pain: Evidence Review*. American Pain Society; Glenview, IL: 2009.

www.ampainsoc.org/pub/pdf/Opioid_Final_Evidence_Report.pdf

97. Hill CS. Government regulatory influences on opioid prescribing and their impact on the treatment of pain of nonmalignant origin. *J Pain Symptom Manage* 1996; 11:287-298.
98. The American Academy of Pain Medicine, the American Pain Society. The use of opioids for the treatment of chronic pain. A consensus statement from the American Academy of Pain Medicine and the American Pain Society. *Clin J Pain* 1997; 13:6-8.
99. Turk DC, Swanson KS, Gatchel RJ. Predicting opioid misuse by chronic pain patients: a systematic review and literature synthesis. *Clin J Pain* 2008; 24:497-508.
100. Manchikanti L, Ailinani H, Koyyalagunta D, Datta S, Singh V, Eriator I, Sehgal N, Shah RV, Benyamin RM, Vallejo R, Fellows B, Christo PJ. A systematic review of randomized trials of long-term opioid management for chronic non-cancer pain. *Pain Physician* 2011; 14:91-121.
101. Noble M, Treadwell JR, Tregear SJ, Coates VH, Wiffen PJ, Akafomo C, Schoelles KM. Long-term opioid management for chronic noncancer pain. *Cochrane Database Syst Rev* 2010; 1:CD006605.
102. Kalso E, Edwards JE, Moore RA, McQuay HJ. Opioids in chronic non-cancer pain: Systematic review of efficacy and safety. *Pain* 2004; 112:372-380.
103. Furlan AD, Sandoval JA, Mailis-Gagnon A, Tunks E. Opioids for chronic noncancer pain: A meta-analysis of effectiveness and side effects. *Can Med Assoc J* 2006; 174:1589-1594.
104. Eisenberg E, McNicol E, Carr DB. Opioids for neuropathic pain (review). *Cochrane Database Syst Rev* 2006, 3:CD006146.
105. Deshpande A, Furlan A, Mailis-Gagnon A, Atlas S, Turk D. Opioids for chronic low back pain (review). *Cochrane Database Syst Rev* 2007; 3:CD004959.
106. Devulder J, Richarz U, Nataraja SH. Impact of long-term use of opioids on quality of life in patients with chronic, non-malignant pain. *Curr Med Res Opin* 2005; 21:1555-1568.
107. Manchikanti L, Vallejo R, Manchikanti KN, Benyamin RM, Datta S, Christo PJ. Effectiveness of long-term opioid therapy for chronic non-cancer pain. *Pain Physician* 2011; 14:E133-E156.
108. Colson J, Koyyalagunta D, Falco FJE, Manchikanti L. A systematic review of observational studies on the effectiveness of opioid therapy for cancer pain. *Pain Physician* 2011; 14:E85-E102.
109. Vallejo R, Barkin RL, Wang VC. Pharmacology of opioids in the treatment of chronic pain syndromes. *Pain Physician* 2011; 14:E343-E360.
110. Trescot AM, Helm S, Hansen H, Benyamin R, Adlaka R, Patel S, Manchikanti L. Opioids in the management of chronic non-cancer pain: An update of American Society of Interventional Pain Physicians' (ASIPP) guidelines. *Pain Physician* 2008; 11:S5-S62.
111. Stein C, Reinecke H, Sorgatz H. Opioid use in chronic noncancer pain: Guidelines revisited. *Curr Opin Anaesthesiol* 2010; 23:598-601.
112. von Korff M, Kolodny A, Deyo R, Chou R. Long-term opioid therapy reconsidered. *Ann Intern Med* 2011; 155:325-328.
113. Grady D, Berkowitz SA, Katz MH. Opioids for chronic pain. *Arch Intern Med* 2011; 171:1426-1427.
114. Dhalla IA, Persaud N, Juurlink DN. Facing up to the prescription opioid crisis. *BMJ* 2011; 343:d5142.
115. Martell BA, O'Connor PG, Kerns RD, Becker WC, Morales KH, Kosten TR, Fiellin DA. Systematic review: Opioid treatment for chronic back pain: Prevalence, efficacy, and association with addiction. *Ann Intern Med* 2007; 146:116-127.
116. Who bears responsibility for the premature adoption of opioids as a treatment standard? *The Back Letter* 2011; 26:46
117. Chou R, Huffman LH; American Pain Society; American College of Physicians. Medications for acute and chronic low back pain: A review of the evidence for an American Pain Society/American College of Physicians clinical practice guideline. *Ann Intern Med* 2007; 147:505-514.
118. Reinecke H, Sorgatz H; German Society for the Study of Pain (DGSS). S3 guideline LONTS. Long-term administration of opioids for non-tumor pain. *Schmerz* 2009; 23:440-447.
119. Sorgatz H, Maier C. Nothing is more damaging to a new truth than an old error: Conformity of new guidelines on opioid administration for chronic pain with the effect prognosis of the DGSS S3 guidelines LONTS (long-term administration of opioids for non-tumor pain). *Schmerz* 2010; 24:309-312.
120. Angst MS, Clark JD. Opioid-induced hyperalgesia: a qualitative systematic review. *Anesthesiology* 2006; 104:570-587.
121. Lee M, Silverman SM, Hansen H, Patel VB, Manchikanti L. A comprehensive review of opioid-induced hyperalgesia. *Pain Physician* 2011; 14:145-161.
122. Cohen SP, Christo PJ, Wang S, Chen L, Stojanovic MP, Shields CH, Brummett C, Mao J. The effect of opioid dose and treatment duration on the perception of

a painful standardized clinical stimulus. *Reg Anesth Pain Med* 2008; 33:199-206.

123. Raffaelli W, Salmosky-Dekel BG. Biological consequences of long-term intrathecal administration of opioids. *Minerva Anestesiol* 2005; 71:475-478.
124. Deer TR, Smith HS, Burton AW, Pope JE, Doleys DM, Levy RM, Staats PS, Wallace MS, Webster LR, Rauck RL, Cousins M. Comprehensive consensus based guidelines on intrathecal drug delivery systems in the treatment of pain caused by cancer pain. *Pain Physician* 2011; 14:E283-E312.
125. Edlund MJ, Martin BC, Devries A, Fan M-Y, Braden JB, Sullivan MD. Trends in use of opioids for chronic noncancer pain among individuals with mental health and substance use disorders: the TROUP Study. *Clin J Pain* 2010; 26:1-8.
126. Sullivan MD, Edlund MJ, Fan MY, Devries A, Brennan Braden J, Martin BC. Risks for possible and probable opioid misuse among recipients of chronic opioid therapy in commercial and medicaid insurance plans: the TROUP Study. *Pain* 2010; 150:332-339.
127. Fleming MF, Balousek SL, Klessig CL, Mundt MP, Brown DD. Substance use disorders in a primary care sample receiving daily opioid therapy. *J Pain* 2007; 8:573-582.
128. Sullivan MD. Limiting the potential harms of high-dose opioid therapy: Comment on "Opioid dose and drug-related mortality in patients with nonmalignant pain." *Arch Intern Med* 2011; 171:691-693.
129. Katz MH. Long-term opioid treatment of nonmalignant pain: a believer loses his faith. *Arch Intern Med* 2010; 170:1422-1424.
130. McLellan AT, Turner BJ. Chronic noncancer pain management and opioid overdose: Time to change prescribing practices. *Ann Intern Med* 2010; 152:123-124.
131. Dunn KM, Saunders KW, Rutter CM, Banta-Green CJ, Merrill JO, Sullivan MD, Weisner CM, Silverberg MJ, Campbell CI, Psaty BM, Von Korff M. Opioid prescriptions for chronic pain and overdose: a cohort study. *Ann Intern Med* 2010; 152:85-92.
132. Manchikanti L, Singh V, Caraway DL, Benyamin RM. Breakthrough pain in chronic non-cancer pain: Fact, fiction, or abuse. *Pain Physician* 2011; 14:E103-E117.
133. Saunders KW, Dunn KM, Merrill JO, Sullivan M, Weisner C, Braden JB, Psaty BM, Von Korff M. Relationship of opioid use and dosage levels to fractures in older chronic pain patients. *J Gen Intern Med* 2010; 25:310-315.
134. Walker JM, Farney RJ, Rhondeau SM, Boyle KM, Valentine K, Cloward TV, Shilling KC. Chronic opioid use is a risk factor for the development of central sleep apnea and ataxic breathing. *J Clin Sleep Med* 2007; 3:455-461.
135. Solomon DH, Rassen JA, Glynn RJ, Lee J, Levin R, Schneeweiss S. The comparative safety of analgesics in older adults with arthritis. *Arch Intern Med* 2010; 170:1968-1976.
136. Martin BC, Fan MY, Edlund MJ, Devries A, Braden JB, Sullivan MD. Long-term chronic opioid therapy discontinuation rates from the TROUP study. *J Gen Intern Med* 2011; 26:1450-1457.
137. Pletcher MJ, Kertesz SG, Kohn MA, Gonzales. R. Trends in opioid prescribing by race/ethnicity for patients seeking care in US emergency departments. *JAMA* 2008; 299:70-78.
138. American Society for Pain Management Nursing (ASPMN); Emergency Nurses Association (ENA); American College of Emergency Physicians (ACEP); American Pain Society (APS). Policy statement. Optimizing the treatment of pain in patients with acute presentations. *Ann Emerg Med* 2010; 56:77-79.
139. American College of Emergency Physicians. Policy statement. Electronic prescription monitoring. Approved by the ACEP Board of Directors, October 2011. *Ann. Emerg. Med.* Pending publication, March 2012.
140. Department of Health and Human Services, Food and Drug Administration. Draft blueprint for prescriber education for long-acting/extended-release opioid class-wide risk evaluation and mitigation strategy. *Fed. Reg.* 76, 68766-68767 (November 7, 2011).
141. Solanki DR, Koyyalagunta D, Shah RV, Silverman SM, Manchikanti L. Monitoring opioid adherence in chronic pain patients: Assessment of risk of substance misuse. *Pain Physician* 2011; 14:E119-E131.
142. Christo PJ, Manchikanti L, Ruan X, Bottros M, Hansen H, Solanki D, Jordan AE, Colson J. Urine drug testing in chronic pain. *Pain Physician* 2011; 14:123-143.
143. Gilbert JW, Wheeler GR, Mick GE, Storey BB, Herder SL, Richardson GB, Watts E, Gyarteng-Dakwa K, Marino BS, Kenney CM, Siddiqi M, Broughton PG. Importance of urine drug testing in the treatment of chronic noncancer pain: Implications of recent medicare policy changes in Kentucky. *Pain Physician* 2010; 13:167-186.
144. Gilbert JW, Wheeler GR, Mick GE, Storey BB, Herder SL, Richardson GB, Watts E, Gyarteng-Dakwa K, Marino BS, Kenney CM, Siddiqi M, Broughton PG. Urine drug testing in the treatment of chronic noncancer pain in a Kentucky private neuroscience practice: The potential effect of Medicare benefit changes in Kentucky. *Pain Physician* 2010; 13:187-194.
145. Manchikanti L, Malla Y, Wargo BW, Fellows B. Comparative evaluation of the accuracy of immunoassay with liquid chromatography tandem mass spectrometry (LC/MS/MS) of urine drug testing (UDT) opioids and illicit drugs in chronic pain patients. *Pain Physician* 2011; 14:175-187.
146. Manchikanti L, Malla Y, Wargo BW, Fellows B. Comparative evaluation of the accuracy of benzodiazepine testing in chronic pain patients utilizing immunoassay with liquid chromatography tandem mass spectrometry (LC/MS/MS) of urine drug testing. *Pain Physician* 2011; 14:259-270.
147. Pesce A, West C, West R, Crews B, Mikel C, Almazan P, Latyshev S, Rosenthal M, Horn P. Reference intervals: a novel approach to detect drug abuse in a pain patient population. *J Opioid Manage* 2010; 6:341-350.
148. Pesce A, Rosenthal M, West R, West C, Mikel C, Almazan P, Latyshev S. An evaluation of the diagnostic accuracy of liquid chromatography-tandem mass spectrometry versus immunoassay drug testing in pain patients. *Pain Physician* 2010; 13:273-281.
149. Manchikanti L, Malla Y, Wargo BW, Cash KA, Pampati V, Damron KS, McManus CD, Brandon DE. Protocol for accuracy of point of care (POC) or in-office urine drug testing (immunoassay) in chronic pain patients: A prospective analysis of immunoassay and liquid chromatography tandem mass spectometry (LC/MS/MS). *Pain Physician* 2010; 13:E1-E22.
150. McCarberg BH. A critical assessment of opioid treatment adherence using urine drug testing in chronic pain management. *Postgrad Med* 2011; 123:124-131.
151. Ling W, Mooney L, Hillhouse M. Prescription opioid abuse, pain and addiction: Clinical issues and implications. *Drug Alcohol Rev* 2011; 30:300-305.

152. Hall AJ, Logan JE, Toblin RL, Kaplan JA, Kraner JC, Bixler D, Crosby AE, Paulozzi LJ. Patterns of abuse among unintentional pharmaceutical overdose fatalities. *JAMA* 2008; 300:2613-2620.
153. United States Department of Justice, Drug Enforcement Administration. Automation of Reports and Consolidated Orders System (ARCOS). Springfield, VA, 2011.
www.deadiversion.usdoj.gov/arcos/index.html.
154. Paulozzi LJ, Logan JE, Hall AJ, McKinstry E, Kaplan JA, Crosby AE. A comparison of drug overdose deaths involving methadone and other opioid analgesics in West Virginia. *Addiction* 2009; 104:1541-1548.
155. Dhalla IA, Mamdani MM, Sivilotti ML, Kopp A, Qureshi O, Juurlink DN. Prescribing of opioid analgesics and related mortality before and after the introduction of long-acting oxycodone. *CMAJ* 2009; 181:891-896.
156. Toblin RL, Paulozzi LJ, Logan JE, Hall AJ, Kaplan JA. Mental illness and psychotropic drug use among prescription drug overdose deaths: A medical examiner chart review. *J Clin Psychiatry* 2010; 71:491-496.
157. Methadone Mortality Working Group Drug Enforcement Administration, Office of Diversion Control, April 2007.
www.deadiversion.usdoj.gov/drugs_concern/methadone/methadone_presentation0407_revised.pdf
158. Xu J, Kochanek KD, Murphy SL, Tejada-Vera B. Deaths: Final data for 2007. National vital statistics reports; Vol. 58 No. 19. National Center for Health Statistics, Hyattsville, MD, 2010.
http://www.cdc.gov/nchs/data/nvsr/nvsr58/nvsr58_01.pdf.
159. Warner M, Chen LH, Makuc DM, Anderson RN, Miniño AM. Drug poisoning deaths in the United States, 1980-2008. NCHS data brief, no. 81. National Center for Health Statistics, Hyattsville, MD, 2011.
160. Centers for Disease Control and Prevention. Vital signs: Overdoses of prescription opioid pain relievers – United States, 1999-2008. *MMWR. Morb Mortal Wkly Rep* 2011; 60:1487-1492.
161. Degenhardt L, Hall W. Extent of illicit drug use and dependence, and their contribution to the global burden of disease. *Lancet* 2012; 379:55-70.
162. National Health Services, The National Treatment Agency for Substance Misuse. Addiction to Medicine. An investigation into the configuration and commissioning of treatment services to support those who develop problems with prescription-only or over-the-counter medicine. May 2011. www.nta.nhs.uk/uploads/addictiontomedicinesmay2011a.pdf
163. Wunsch MJ, Nakamoto K, Behonick G, Massello W. Opioid deaths in rural Virginia: A description of the high prevalence of accidental fatalities involving prescribed medications. *Am J Addict* 2009; 18:5-14.
164. Hudson TJ, Edlund MJ, Steffick DE, Tripathi SP, Sullivan MD. Epidemiology of regular prescribed opioid use: Results from a national, population-based survey. *J Pain Sympt Mgmt* 2008; 36:280-288.
165. Caudill-Slosberg MA, Schwartz LM, Woloshin S. Office visits and analgesic prescriptions for musculoskeletal pain in US: 1980 vs. 2000. *Pain* 2004; 109:514-519.
166. Franklin GM, Mai J, Wickizer T, Turner JA, Fulton-Kehoe D, Grant L. Opioid dosing trends and mortality in Washington State workers' compensation, 1996–2002. *Am J Ind Med* 2005; 48:91-99.
167. Luo X, Pietrobon R, Hey L. Patterns and trends in opioid use among individuals with back pain in the United States. *Spine (Phila Pa 1976)* 2004; 29:884-891.
168. Zerzan JT, Morden NE, Soumerai S, Ross-Degnan D, Roughead E, Zhang F, Simoni-Wastila L, Sullivan SD. Trends and geographic variation of opiate medication use in state Medicaid fee-for-service programs, 1996 to 2002. *Med Care* 2006; 44:1005-1010.
169. Manchikanti L, Pampati S, Damron KS, Cash KA, McManus CD, Fellows B. Identification of doctor shoppers with KASPER: A comparative evaluation over a decade in western Kentucky. *J Ky Med Assoc* 2012; in press.
170. Substance Abuse and Mental Health Services Administration. *Results from the 2010 National Survey on Drug Use and Health: Summary of National Findings*. NSDUH Series H-41, HHS Publication No. (SMA) 11-4658. Substance Abuse and Mental Health Services Administration, Rockville, MD, 2011.
www.samhsa.gov/data/NSDUH/2k10NSDUH/2k10Results.pdf.
171. Substance Abuse and Mental Health Services Administration. *Results from the 2010 National Survey on Drug Use and Health: Mental Health Findings*. NSDUH Series H-42, HHS Publication No. (SMA) 11-4667. Rockville, MD: Substance Abuse and Mental Health Services Administration, 2012. www.samhsa.gov/data/NSDUH/2k10MH_Findings/2k10MHResults.pdf
172. Volkow ND, McLellan TA, Cotto JH. Characteristics of opioid prescriptions in 2009. *JAMA* 2011; 305:1299-1301.
173. Governale L. Outpatient prescription opioid utilization in the U.S., years 2000 – 2009. Drug Utilization Data Analysis Team Leader, Division of Epidemiology, Office of Surveillance and Epidemiology. Presentation for U.S. Food and Drug Administration, July 22, 2010.
www.fda.gov/downloads/AdvisoryCommittees/CommitteesMeetingMaterials/Drugs/AnestheticAndLifeSupportDrugsAdvisoryCommittee/UCM220950.pdf
174. Source: SDI, Vector One ®: National.
175. IMS Institute for Healthcare Informatics. The use of medicines in the United States: Review of 2011. April 2012.
www.imshealth.com/ims/Global/Content/Insights/IMS%20Institute%20for%20Healthcare%20Informatics/IHII_Medicines_in_U.S_Report_2011.pdf
176. Ricks D. UN: US consumes 80% of world's oxycodone. *Newsday*, January 21, 2012.
177. Report of the International Narcotics Control Board for 2004. New York, United Nations, 2005.
www.incb.org/pdf/e/ar/2004/incb_report_2004_full.pdf
178. Report of the International Narcotics Control Board 2008. New York, United Nations, 2009.
www.incb.org/pdf/annual-report/2008/en/AR_08_English.pdf
179. National Center for Health Statistics. Health, United States, 2008 with chartbook. National Center for Health Statistics, Hyattsville, MD, 2009.
180. Centers for Disease Control and Prevention. Adult use of prescription opioid pain medications – Utah, 2008. *MMWR Morb Mortal Wkly Rep* 2010; 59:153-157.
181. Sullivan MD, Edlund MJ, Fan MY, Devries A, Brennan Braden J, Martin BC. Trends in use of opioids for non-cancer pain conditions 2000-2005 in commercial and Medicaid insurance plans: the TROUP study. *Pain* 2008; 138:440-449.
182. Vogt MT, Kwoh CK, Cope DK, Osial TA,

Culyba M, Starz TW. Analgesic usage for low back pain: Impact on health care costs and service use. *Spine (Phila Pa 1976)* 2005; 30:1075-1081.

183. Manchikanti L, Damron KS, McManus CD, Barnhill RC. Patterns of illicit drug use and opioid abuse in patients with chronic pain at initial evaluation: a prospective, observational study. *Pain Physician* 2004; 7:431-437.
184. Manchikanti L, Pampati V, Damron KS, Beyer CD, Barnhill RC, Fellows B. Prevalence of prescription drug abuse and dependency in patients with chronic pain in western Kentucky. *J KY Med Assoc* 2003; 101:511-517.
185. Manchikanti L, Damron KS, Pampati V, McManus CD. Prevalence of illicit drug use among individuals with chronic pain in the Commonwealth of Kentucky: an evaluation of patterns and trends. *J KY Med Assoc* 2005; 103:55-62.
186. Vaglienti RM, Huber SJ, Noel KR, Johnstone RE. Misuse of prescribed controlled substances defined by urinalysis. *WV Med J* 2003; 99:67-70.
187. Michna E, Jamison RN, Pham LD, Ross EL, Janfaza D, Nedeljkovic SS, Narang S, Palombi D, Wasan AD. Urine toxicology screening among chronic pain patients on opioid therapy: Frequency and predictability of abnormal findings. *Clin J Pain* 2007; 23:173-179.
188. Manchikanti L, Manchukonda R, Pampati V, Damron KS, Brandon DE, Cash KA, McManus CD. Does random urine drug testing reduce illicit drug use in chronic pain patients receiving opioids? *Pain Physician* 2006; 9:123-129.
189. Wu PC, Lang C, Hasson NK, Linder SH, Clark DJ. Opioid use in young veterans. *J Opioid Manag* 2010; 6:133-139.
190. Unintentional Drug Poisoning in the United States. Centers for Disease Control and Prevention. July 2010. http://www.cdc.gov/HomeandRecreationalSafety/pdf/poison-issue-brief.pdf
191. Paulozzi LJ, Weisler RH, Patkar AA. A national epidemic of unintentional prescription opioid overdose deaths: how physicians can help control it. *J Clin Psychiatry* 2011; 72:589-592.
192. Ohio Department of Health. Epidemic of prescription drug overdose in Ohio. 2010.
www.healthyohioprogram.org/diseaseprevention/dpoison/drugdata.aspx.
193. Edlund MJ, Martin BC, Fan MY, Braden JB, Devries A, Sullivan MD. An analysis of heavy utilizers of opioids for chronic noncancer pain in the TROUP study. *J Pain Symptom Manage* 2010; 40:279-289.
194. Katz N, Panas L, Kim M, Audet AD, Bilansky A, Eadie J, Kreiner P, Paillard FC, Thomas C, Carrow G. Usefulness of prescription monitoring programs for surveillance--analysis of Schedule II opioid prescription data in Massachusetts, 1996-2006. *Pharmacoepidemiol Drug Saf* 2010; 19:115-123.
195. Bohnert AS, Valenstein M, Bair MJ, Ganoczy D, McCarthy JF, Ilgen MA, Blow FC. Association between opioid prescribing patterns and opioid overdose-related deaths. *JAMA* 2011; 305:1315-1321.
196. Lanier WA. Prescription drug overdose deaths – Utah, 2008-2009. *In Proceedings at the 59th Annual Epidemic Intelligence Service Conference*. Atlanta, GA, April 23, 2010.
197. Manchikanti L, Singh V, Boswell MV. Interventional pain management at crossroads: The perfect storm brewing for a new decade of challenges. *Pain Physician* 2010; 13:E111-E140.
198. Benyamin RM, Datta S, Falco FJE. A perfect storm in interventional pain management: Regulated, but unbalanced. *Pain Physician* 2010; 13:109-116.
199. Office of National Drug Control Policy. 2011 National Drug Control Strategy. http://www.whitehouse.gov/ondcp/2011-national-drug-control-strategy
200. Manchikanti L, Singh V, Cash KA, Pampati V, Damron KS, Boswell MV. A randomized, controlled, double-blind trial of fluoroscopic caudal epidural injections in the treatment of lumbar disc herniation and radiculitis. *Spine (Phila Pa 1976)* 2011; 36:1897-1905.
201. Manchikanti L, Singh V, Cash KA, Pampati V, Datta S. Management of pain of post lumbar surgery syndrome: One-year results of a randomized, double double-blind, active controlled trial of fluoroscopic caudal epidural injections. *Pain Physician* 2010; 13:509-521.
202. Manchikanti L, Cash RA, McManus CD, Pampati V, Fellows B. Fluoroscopic caudal epidural injections with or without steroids in managing pain of lumbar spinal stenosis: One year results of randomized, double-blind, active-controlled trial. *J Spinal Disord* 2011; April 5 [Epub ahead of print].
203. Manchikanti L, Cash KA, McManus CD, Pampati V, Benyamin RM. Preliminary results of a randomized, double-blind, controlled trial of fluoroscopic lumbar interlaminar epidural injections in managing chronic lumbar discogenic pain without disc herniation or radiculitis. *Pain Physician* 2010; 13:E279-E292.
204. Manchikanti L, Cash KA, McManus CD, Damron KS, Pampati V, Falco FJE. Lumbar interlaminar epidural injections in central spinal stenosis: Preliminary results of a randomized, double-blind, active control trial. *Pain Physician* 2012; 15:51-64.
205. Manchikanti L, Malla Y, Cash KA, McManus CD, Pampati V. Fluoroscopic epidural injections in cervical spinal stenosis: Preliminary results of a randomized, double-blind, active control trial. *Pain Physician* 2012; 15:E59-E70.
206. Manchikanti L, Malla Y, Cash KA, McManus CD, Pampati V. Fluoroscopic cervical interlaminar epidural injections in managing chronic pain of cervical post-surgery syndrome: Preliminary results of a randomized, double-blind active control trial. *Pain Physician* 2012; 15:13-26.
207. Manchikanti L, Cash KA, McManus CD, Pampati V, Benyamin RM. A preliminary report of a randomized double-blind, active controlled trial of fluoroscopic thoracic interlaminar epidural injections in managing chronic thoracic pain. *Pain Physician* 2010; 13:E357-E369.
208. Manchikanti L, Singh V, Falco FJE, Cash KA, Pampati V. Evaluation of lumbar facet joint nerve blocks in managing chronic low back pain: a randomized, double-blind, controlled trial with a 2-year follow-up. *Int J Med Sci* 2010; 7:124-135.
209. Manchikanti L, Singh V, Falco FJE, Cash KA, Fellows B. Comparative outcomes of a 2-year follow-up of cervical medial branch blocks in management of chronic neck pain: a randomized, double-blind controlled trial. *Pain Physician* 2010; 13:437-450.
210. Chou R, Huffman L. *Guideline for the Evaluation and Management of Low Back Pain: Evidence Review*. American Pain Society, Glenview, IL, 2009.
www.ampainsoc.org/pub/pdf/LBPEvidRev.pdf.
211. Manchikanti L, Singh V, Falco FJE, Cash KA, Pampati V, Fellows B. Comparative effectiveness of a one-year follow-up of thoracic medial branch blocks in management of chronic thoracic pain: a randomized, double-blind active controlled trial. *Pain Physician* 2010; 13:535-548.
212. Staal JB, de Bie RA, de Vet HC, Hildebrandt J, Nelemans P. Injection therapy

for subacute and chronic low back pain: an updated Cochrane review. *Spine (Phila Pa 1976)* 2009; 34:49-59.

213. American College of Occupational and Environmental Medicine (ACOEM) Low back Disorders. In: *Occupational Medicine Practice Guidelines: Evaluation and Management of Common Health Problems and Functional Recovery of Workers, Second Edition*. American College of Occupational and Environmental Medicine Press, Elk Grove Village, 2007.
214. Manchikanti L, Datta S, Derby R, Wolfer LR, Benyamin RM, Hirsch JA. A critical review of the American Pain Society clinical practice guidelines for interventional techniques: Part 1. Diagnostic interventions. *Pain Physician* 2010; 13:E141-E174.
215. Manchikanti L, Datta S, Gupta S, Munglani R, Bryce DA, Ward SP, Benyamin RM, Sharma ML, Helm II S, Fellows B, Hirsch JA. A critical review of the American Pain Society clinical practice guidelines for interventional techniques: Part 2. Therapeutic interventions. *Pain Physician* 2010; 13:E215-E264.
216. Manchikanti L, Manchukonda R, Pampati V, Damron KS. Evaluation of abuse of prescription and illicit drugs in chronic pain patients receiving short-acting (hydrocodone) or long-acting (methadone) opioids. *Pain Physician* 2005; 8:257-261.
217. Macintyre PE, Loadsman JA, Scott DA. Opioids, ventilation and acute pain management. *Anaesth Intensive Care* 2011; 39:545-558.
218. Webster LR, Cochella S, Dasgupta N, Fakata KL, Fine PG, Fishman SM, Grey T, Johnson EM, Lee LK, Passik SD, Peppin J, Porucznik CA, Ray A, Schnoll SH, Stieg RL, Wakeland W. An analysis of the root causes for opioid-related overdose deaths in the United States. *Pain Med* 2011; 12:S26-S35.
219. Perrin-Terrin A, Pathak A, Lapeyre-Mestre M. QT interval prolongation: Prevalence, risk factors and pharmacovigilance data among methadone-treated patients in France. *Fundam Clin Pharmacol* 2011; 25:503-510.
220. Anchersen K, Hansteen V, Gossop M, Clausen T, Waal H. Opioid maintenance patients with QTc prolongation: Congenital long QT syndrome mutation may be a contributing risk factor. *Drug Alcohol Depend* 2010; 112:216-219.
221. Modesto-Lowe V, Brooks D, Petry N. Methadone deaths: Risk factors in pain and addicted populations. *J Gen Intern Med* 2010; 25:305-309.
222. Mayet S, Gossop M, Lintzeris N, Markides V, Strang J. Methadone maintenance, QTc and torsade de pointes: Who needs an electrocardiogram and what is the prevalence of QTc prolongation? *Drug Alcohol Rev* 2011; 30:388-396.
223. Manchikanti L, Damron KS, Beyer CD, Pampati V. A comparative evaluation of illicit drug use in patients with or without controlled substance abuse in interventional pain management. *Pain Physician* 2003; 6:281-285.
224. Fillingim RB, Doleys DM, Edwards RR, Lowery D. Clinical characteristics of chronic back pain as a function of gender and oral opioid use. *Spine (Phila Pa 1976)* 2003; 28:143-150.
225. Webster BS, Verma SK, Gatchel RJ. Relationship between early opioid prescribing for acute occupational low back pain and disability duration, medical costs, subsequent surgery, and late opioid use. *Spine (Phila Pa 1976)* 2007; 32:2127-2132.
226. Mahmud MA, Webster BS, Courtney TK, Matz S, Tacci JA, Christiani DC. Clinical management and the duration of disability for work-related low back pain. *J Occup Environ Med* 2000; 42:1178-1187.
227. Franklin GM, Stover BD, Turner JA, Fulton- Kehoe D, Wickizer TM; Disability Risk Identification Study Cohort. Early opioid prescription and subsequent disability among workers with back injuries: The Disability Risk Identification Study Cohort. *Spine (Phila Pa 1976)* 2008; 33:199-204.
228. Rhee Y, Taitel MS, Walker DR, Lau DT. Narcotic drug use among patients with lower back pain in employer health plans: a retrospective analysis of risk factors and health care services. *Clin Ther* 2007; 29:2603-2612.
229. Gross DP, Stephens B, Bhambhani Y, Haykowsky M, Bostick GP, Rashiq S. Opioid prescriptions in canadian workers' compensation claimants: Prescription trends and associations between early prescription and future recovery. *Spine (Phila Pa 1976)* 2009; 34:525-531.
230. Volinn E, Fargo JD, Fine PG. Opioid therapy for nonspecific low back pain and the outcome of chronic work loss. *Pain* 2009; 142:194-201.
231. Cifuentes M, Webster B, Genevay S, Pransky G. The course of opioid prescribing for a new episode of disabling low back pain: Opioid features and dose escalation. *Pain* 2010; 151:22-29.
232. Becker N, Sjogren P, Bech P, Olsen AK, Eriksen J. Treatment outcome of chronic non-malignant pain patients managed in a Danish multidisciplinary pain centre compared to general practice: a randomised controlled trial. *Pain* 2000; 84:203-211.
233. Webster BS, Cifuentes M, Verma S, Pransky G. Geographic variation in opioid prescribing for acute, work-related, low back pain and associated factors: A multilevel analysis. *Am J Ind Med* 2009; 52:162-171.
234. Franklin GM, Rahman EA, Turner JA, Daniell WE, Fulton-Kehoe D. Opioid use for chronic low back pain: a prospective, population-based study among injured workers in Washington state, 2002-2005. *Clin J Pain* 2009; 25:743-751.
235. Stover BD, Turner JA, Franklin G, Gluck JV, Fulton-Kehoe D, Sheppard L, Wickizer TM, Kaufman J, Egan K. Factors associated with early opioid prescription among workers with low back injuries. *J Pain* 2006; 7:718-725.
236. White AG, Birnbaum HG, Mareva MN, Daher M, Vallow S, Schein J, Katz N. Direct costs of opioid abuse in an insured population in the United States. *J Manag Care Pharm* 2005; 11:469-479.
237. Fishbain DA, Rosomoff HL, Rosomoff RS. Drug abuse, dependence, and addiction in chronic pain patients. *Clin J Pain* 1992; 8:77-85.

# EXHIBIT B

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# The Promotion and Marketing of OxyContin: Commercial Triumph, Public Health Tragedy

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## Abstract

I focus on issues surrounding the promotion and marketing of controlled drugs and their regulatory oversight. Compared with noncontrolled drugs, controlled drugs, with their potential for abuse and diversion, pose different public health risks when they are overpromoted and highly prescribed. An in-depth analysis of the promotion and marketing of OxyContin illustrates some of the associated issues.

Modifications of the promotion and marketing of controlled drugs by the pharmaceutical industry and an enhanced capacity of the Food and Drug Administration to regulate and monitor such promotion can have a positive impact on the public health.

**CONTROLLED DRUGS, WITH** their potential for abuse and diversion, can pose public health risks that are different from—and more problematic than—those of uncontrolled drugs when they are overpromoted and highly prescribed. An in-depth analysis of the promotion and marketing of OxyContin (Purdue Pharma, Stamford, CT), a sustained-release oxycodone preparation, illustrates some of the key issues. When Purdue Pharma introduced OxyContin in 1996, it was aggressively marketed and highly promoted. Sales grew from $48 million in 1996 to almost $1.1 billion in 2000.[1] The high availability of OxyContin correlated with increased abuse, diversion, and addiction, and by 2004 OxyContin had become a leading drug of abuse in the United States.[2]

Under current regulations, the Food and Drug Administration (FDA) is limited in its oversight of the marketing and promotion of controlled drugs. However, fundamental changes in the promotion and marketing of controlled drugs by the pharmaceutical industry, and an enhanced capacity of the FDA to regulate and monitor such promotion, can positively affect public health.

OxyContin's commercial success did not depend on the merits of the drug compared with other available opioid preparations. The *Medical Letter on Drugs and Therapeutics* concluded in 2001 that oxycodone offered no advantage over appropriate doses of other potent opioids.[3] Randomized double-blind studies comparing OxyContin given every 12 hours with immediate-release oxycodone given 4 times daily showed comparable efficacy and safety for use with chronic back pain[4] and cancer-related pain.[5,6] Randomized double-blind studies that compared OxyContin with controlled-release morphine for cancer-related pain also found comparable efficacy and safety.[7–9] The FDA's medical review officer, in evaluating the efficacy of OxyContin in Purdue's 1995 new drug application, concluded that OxyContin had not been shown to

have a significant advantage over conventional, immediate-release oxycodone taken 4 times daily other than a reduction in frequency of dosing.[10] In a review of the medical literature, Chou et al. made similar conclusions.[11]

The promotion and marketing of OxyContin occurred during a recent trend in the liberalization of the use of opioids in the treatment of pain, particularly for chronic non–cancer-related pain. Purdue pursued an "aggressive" campaign to promote the use of opioids in general and OxyContin in particular.[1,12–17] In 2001 alone, the company spent $200 million[18] in an array of approaches to market and promote OxyContin.

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# PROMOTION OF OXYCONTIN

From 1996 to 2001, Purdue conducted more than 40 national pain-management and speaker-training conferences at resorts in Florida, Arizona, and California. More than 5000 physicians, pharmacists, and nurses attended these all-expenses-paid symposia, where they were recruited and trained for Purdue's national speaker bureau.[19(p22)] It is well documented that this type of pharmaceutical company symposium influences physicians' prescribing, even though the physicians who attend such symposia believe that such enticements do not alter their prescribing patterns.[20]

One of the cornerstones of Purdue's marketing plan was the use of sophisticated marketing data to influence physicians' prescribing. Drug companies compile prescriber profiles on individual physicians—detailing the prescribing patterns of physicians nationwide—in an effort to influence doctors' prescribing habits. Through these profiles, a drug company can identify the highest and lowest prescribers of particular drugs in a single zip code, county, state, or the entire country.[21] One of the critical foundations of Purdue's marketing plan for OxyContin was to target the physicians who were the highest prescribers for opioids across the country.[1,12–17,22] The resulting database would help identify physicians with large numbers of chronic-pain patients. Unfortunately, this same database would also identify which physicians were simply the most frequent prescribers of opioids and, in some cases, the least discriminate prescribers.

A lucrative bonus system encouraged sales representatives to increase sales of OxyContin in their territories, resulting in a large number of visits to physicians with high rates of opioid prescriptions, as well as a multifaceted information campaign aimed at them. In 2001, in addition to the average sales representative's annual salary of $55 000, annual bonuses averaged $71 500, with a range of $15 000 to nearly $240 000. Purdue paid $40 million in sales incentive bonuses to its sales representatives that year.[19]

From 1996 to 2000, Purdue increased its internal sales force from 318 sales representatives to 671, and its total physician call list from approximately 33 400 to 44 500 to approximately 70 500 to 94 000 physicians.[19] Through the sales representatives, Purdue used a patient starter coupon program for OxyContin that provided patients with a free limited-time prescription for a 7- to 30-day supply. By 2001, when the program was ended, approximately 34 000 coupons had been redeemed nationally.[19]

The distribution to health care professionals of branded promotional items such as OxyContin fishing hats, stuffed plush toys, and music compact discs ("Get in the Swing With OxyContin") was unprecedented for a schedule II opioid, according to the Drug Enforcement Administration.[19]

Purdue promoted among primary care physicians a more liberal use of opioids, particularly sustained-release opioids. Primary care physicians began to use more of the increasingly popular OxyContin; by 2003, nearly half of all physicians prescribing OxyContin were primary care physicians.[19] Some experts were concerned that primary care physicians were not sufficiently trained in pain management or addiction issues.[23] Primary care physicians, particularly in a managed care environment of time constraints, also had the least amount of time for evaluation and follow-up of patients with complicated chronic pain.

Purdue “aggressively” promoted the use of opioids for use in the “non-malignant pain market.”[15(p187)] A much larger market than that for cancer-related pain, the non–cancer-related pain market constituted 86% of the total opioid market in 1999.[17] Purdue's promotion of OxyContin for the treatment of non–cancer-related pain contributed to a nearly tenfold increase in OxyContin prescriptions for this type of pain, from about 670 000 in 1997 to about 6.2 million in 2002, whereas prescriptions for cancer-related pain increased about fourfold during that same period.[19] Although the science and consensus for the use of opioids in the treatment of acute pain or pain associated with cancer are robust, there is still much controversy in medicine about the use of opioids for chronic non–cancer-related pain, where their risks and benefits are much less clear. Prospective, randomized, controlled trials lasting at least 4 weeks that evaluated the use of opioids for chronic, non–cancer-related pain showed statistically significant but small to modest improvement in pain relief, with no consistent improvement in physical functioning.[24–38] A recent review of the use of opioids in chronic back pain concluded that opioids may be efficacious for short-term pain relief, but longer-term efficacy ( > 16 weeks) is unclear.[39]

In the long-term use of opioids for chronic non–cancer-related pain, the proven analgesic efficacy must be weighed against the following potential problems and risks: well-known opioid side effects, including respiratory depression, sedation, constipation, and nausea; inconsistent improvement in functioning; opioid-induced hyperalgesia; adverse hormonal and immune effects of long-term opioid treatment; a high incidence of prescription opioid abuse behaviors; and an ill-defined and unclarified risk of iatrogenic addiction.[40]

# MISREPRESENTING THE RISK OF ADDICTION

A consistent feature in the promotion and marketing of OxyContin was a systematic effort to minimize the risk of addiction in the use of opioids for the treatment of chronic non–cancer-related pain. One of the most critical issues regarding the use of opioids in the treatment of chronic non–cancer-related pain is the potential of iatrogenic addiction. The lifetime prevalence of addictive disorders has been estimated at 3% to 16% of the general population.[41] However, we lack any large, methodically rigorous prospective study addressing the issue of iatrogenic addiction during long-term opioid use for chronic nonmalignant pain.[42]

In much of its promotional campaign—in literature and audiotapes for physicians, brochures and videotapes for patients, and its “Partners Against Pain” Web site—Purdue claimed that the risk of addiction from OxyContin was extremely small.[43–49]

Purdue trained its sales representatives to carry the message that the risk of addiction was “less than one percent.”[50(p99)] The company cited studies by Porter and Jick,[51] who found iatrogenic addiction in only 4 of 11 882 patients using opioids and by Perry and Heidrich,[52] who found no addiction among 10 000 burn patients treated with opioids. Both of these studies, although shedding some light on the risk of addiction for acute pain, do not help establish the risk of iatrogenic addiction when opioids are used daily for a prolonged time in treating chronic pain. There are a number of studies, however, that demonstrate that in the treatment of chronic non–cancer-related pain with opioids, there is a high incidence of prescription drug abuse. Prescription drug abuse in a substantial minority of chronic-pain patients has been demonstrated in studies by Fishbain et al. (3%–18% of patients),[53] Hoffman et al. (23%),[54] Kouyanou et al. (12%),[55] Chabal et al. (34%),[56] Katz et al. (43%),[57] Reid et al. (24%–31%),[58] and Michna et al. (45%).[59] A recent literature review showed that the prevalence of addiction in patients with long-term opioid treatment for chronic non–cancer-related pain varied from 0% to 50%, depending on the criteria used and the subpopulation studied.[60]

Misrepresenting the risk of addiction proved costly for Purdue. On May 10, 2007, Purdue Frederick Company Inc, an affiliate of Purdue Pharma, along with 3 company executives, pled guilty to criminal charges of misbranding OxyContin by claiming that it was less addictive and less subject to abuse and diversion than other opioids, and will pay $634 million in fines.[61]

Although research demonstrated that OxyContin was comparable in efficacy and safety to other available opioids,[11,63] marketing catapulted OxyContin to blockbuster drug status. Sales escalated from $44 million (316 000 prescriptions dispensed) in 1996 to a 2001 and 2002 combined sales of nearly $3 billion (over 14 million prescriptions).[19]

The remarkable commercial success of OxyContin, however, was stained by increasing rates of abuse and addiction. Drug abusers learned how to simply crush the controlled-release tablet and swallow, inhale, or inject the high-potency opioid for an intense morphinelike high.[64] There had been some precedence for the diversion and abuse of controlled-release opioid preparations. Purdue's own MS Contin had been abused in the late 1980s in a fashion similar to how OxyContin was later to be; by 1990, MS Contin had become the most abused prescription opioid in one major metropolitan area.[65] Purdue's own testing in 1995 had demonstrated that 68% of the oxycodone could be extracted from an OxyContin tablet when crushed.[66]

Opioid prescribing has had significant geographical variations. In some areas, such as Maine, West Virginia, eastern Kentucky, southwestern Virginia, and Alabama, from 1998 through 2000, hydrocodone and (non-OxyContin) oxycodone were being prescribed 2.5 to 5.0 times more than the national average. By 2000, these same areas had become high OxyContin-prescribing areas—up to 5 to 6 times higher than the national average in some counties (Table 1).[67] These areas, in which OxyContin was highly available, were the first in the nation to witness increasing OxyContin abuse and diversion, which began surfacing in 1999 and 2000.[23] From 1995 to 2001, the number of patients treated for opioid abuse in Maine increased 460%, and from 1997 to 1999 the state had a 400% increase in the number of chronic hepatitis C cases reported.[68] In eastern Kentucky from 1995 to 2001, there was a 500% increase in the number of patients entering methadone maintenance treatment programs, about 75% of whom were OxyContin dependent (Mac Bell, administrator, Narcotics Treatment Programs, Kentucky Division of Substance Abuse, written communication, March 2002). In West Virginia, the first methadone maintenance treatment program opened in August 2000, largely in response to the increasing number of people with OxyContin dependence. By October 2003, West Virginia had 7 methadone maintenance treatment clinics with 3040 patients in treatment (M. Moore, Office of Behavioral Health Services, Office of Alcoholism and Drug Abuse, West Virginia, written communication, March 16, 2004). In southwestern Virginia, the first methadone maintenance treatment program opened in March 2000, and within 3 years it had 1400 admissions (E. Jennings, Life Center of Galax, Galax, Virginia, written communication, March 12, 2004).

TABLE 1

**Distribution of OxyContin, Oxycodone (Excluding OxyContin), and Hydrocodone per 100 000 Population: Virginia, West Virginia, and Kentucky, 2000**

With increasing diversion and abuse, opioid-related overdoses escalated. In southwest Virginia, the number of deaths related to opioid prescriptions increased 830%, from 23 in 1997 to 215 in 2003 (William Massello III, MD, assistant chief medical examiner, Office of Chief Medical Examiner, Western District, Virginia Department of Health, written communication, January 12, 2007). The high availability of OxyContin in these 5 regions seemed to be a simple correlate of its abuse, diversion, and addiction.

With the growing availability of OxyContin prescriptions, the once-regional problem began to spread nationally. By 2002, OxyContin accounted for 68% of oxycodone sales.[69] Lifetime nonmedical use of OxyContin increased from 1.9 million to 3.1 million people between 2002 and 2004, and in 2004 there were 615 000 new nonmedical users of OxyContin.[70] By 2004, OxyContin had become the most prevalent prescription opioid abused in the United States.[2]

The increasing OxyContin abuse problem was an integral part of the escalating national prescription opioid abuse problem. Liberalization of the use of opioids, particularly for the treatment of chronic non–cancer-related pain, increased the availability of all opioids as well as their abuse. Nationwide, from 1997 to 2002, there was a 226%, 73%, and 402% increase in fentanyl, morphine, and oxycodone prescribing, respectively

(in grams per 100 000 population). During that same period, the Drug Abuse Warning Network reported that hospital emergency department mentions for fentanyl, morphine, and oxycodone increased 641%, 113%, and 346%, respectively.[71] Among new initiates to illicit drug use in 2005, a total of 2.1 million reported prescription opioids as the first drug they had tried, more than for marijuana and almost equal to the number of new cigarette smokers (2.3 million).[72] Most abusers of prescription opioids get their diverted drugs directly from a doctor's prescription or from the prescriptions of friends and family.[73]

In terms of illicit drug abuse, prescription opioids are now ahead of cocaine and heroin and second only to marijuana.[72] Mortality rates from drug overdose have climbed dramatically; by 2002, unintentional overdose deaths from prescription opioids surpassed those from heroin and cocaine nationwide.[74] Nationally, as well as regionally, the high availability of OxyContin and all prescription opioids was correlated with high rates of abuse and diversion.

# THE FOOD AND DRUG ADMINISTRATION

Under the Food, Drug, and Cosmetics Act and implementing regulations, the FDA regulates the advertising and promotion of prescription drugs and is responsible for ensuring that prescription drug advertising and promotion are truthful, balanced, and accurately communicated. There is no distinction in the act between controlled and noncontrolled drugs regarding the oversight of promotional activities. Although regulations require that all promotional materials for prescription drugs be submitted to the FDA for review when the materials are initially disseminated or used, it is generally not required that these materials be approved by the FDA prior to their use. The FDA has a limited number of staff for overseeing the enormous amount of promotional materials. In 2002, for example, 39 FDA staff members were responsible for reviewing roughly 34 000 pieces of promotional materials.[19] This limited staffing significantly diminishes the FDA's ability to ensure that the promotion is truthful, balanced, and accurately communicated.

In 1998, Purdue distributed 15 000 copies of an OxyContin video to physicians without submitting it to the FDA for review, an oversight later acknowledged by Purdue. In 2001, Purdue submitted to the FDA a second version of the video, which the FDA did not review until October 2002—after the General Accounting Office inquired about its content. After its review, the FDA concluded that the video minimized the risks from OxyContin and made unsubstantiated claims regarding its benefits to patients.[19]

When OxyContin entered the market in 1996, the FDA approved its original label, which stated that iatrogenic addiction was “very rare” if opioids were legitimately used in the management of pain. In July 2001, to reflect the available scientific evidence, the label was modified to state that data were not available for establishing the true incidence of addiction in chronic-pain patients. The 2001 labeling also deleted the original statement that the delayed absorption of OxyContin was believed to reduce the abuse liability of the drug.[19] A more thorough review of the available scientific evidence prior to the original labeling might have prevented some of the need for the 2001 label revision.

# CONCLUSIONS

OxyContin appears to be as efficacious and safe as other available opioids and as oxycodone taken 4 times daily.[11,63] Its commercial success, fueled by an unprecedented promotion and marketing campaign, was stained by escalating OxyContin abuse and diversion that spread throughout the country.[2,75] The regions of the country that had the earliest and highest availability of prescribed OxyContin had the greatest initial abuse and diversion.[23,67] Nationally, the increasing availability of OxyContin was associated with higher rates of abuse, and it became the most prevalent abused prescription opioid by 2004.[2]

Compared with noncontrolled drugs, controlled drugs, with their potential for abuse and diversion, pose different public health risks when overpromoted and highly prescribed. Several marketing practices appear to be especially questionable.

The extraordinary amount of money spent in promoting a sustained-release opioid was unprecedented. During OxyContin's first 6 years on the market, Purdue spent approximately 6 to 12 times more on promoting it than the company had spent on promoting MS Contin, or than Janssen Pharmaceutical Products LP had spent on Duragesic, one of OxyContin's competitors.[19] Although OxyContin has not been shown to be superior to other available potent opioid preparations,[11,63] by 2001 it had become the most frequently prescribed brand-name opioid in the United States for treating moderate to severe pain.[19] Carefully crafted limits on the marketing and promotion of controlled drugs would help to realign their actual use with the principles of evidence-based medicine.

Physicians' interactions with pharmaceutical sales representatives have been found to influence the prescribing practices of residents and physicians in terms of decreased prescribing of generic drugs, prescribing cost, nonrational prescribing, and rapid prescribing of new drugs.[76] Carefully crafted limits on the promotion of controlled drugs by the pharmaceutical sales force and enhanced FDA oversight of the training and performance of sales representatives would also reduce over- and misprescribing.

Although there are no available data for evaluating the promotional effect of free starter coupons for controlled drugs, it seems likely that the over- and misprescribing of a controlled drug are encouraged by such promotional programs and the public health would be well served by eliminating them.

The use of prescriber profiling data to influence prescribing and improve sales is imbedded in pharmaceutical detailing. Very little data are publicly available for understanding to what extent this marketing practice boosts sales. One market research report indicated that profiling improved profit margins by as much as 3 percentage points and the initial uptake of new drugs by 30%.[77] The use of prescriber profiling data to target high-opioid prescribers—coupled with very lucrative incentives for sales representatives—would seem to fuel increased prescribing by some physicians—perhaps the most liberal prescribers of opioids and, in some cases, the least discriminate. Regulations eliminating this marketing tool might decrease some potential overprescribing of controlled drugs.

The public health would be better protected if the FDA reviewed all advertising and promotional materials as well as associated educational materials—for their truthfulness, accuracy, balance, and scientific validity—*before* dissemination. Such a change would require a considerable increase in FDA support, staffing, and funding from what is currently available. Public monies spent on the front end of the problem could prevent another such tragedy.

The pharmaceutical industry's role and influence in medical education is problematic. From 1996 through July 2002, Purdue funded more than 20 000 pain-related educational programs through direct sponsorship or financial grants,[19] providing a venue that had enormous influence on physicians' prescribing throughout the country. Particularly with controlled drugs, the potential for blurring marketing and education carries a much higher public health risk than with uncontrolled drugs. At least in the area of controlled drugs, with their high potential for abuse and diversion, public health would best be served by severing the pharmaceutical industry's direct role and influence in medical education.

Marketing and promotion by the pharmaceutical industry have considerably amplified the prescription sales and availability of opioids. A number of factors have contributed to the marked growth of opioid abuse in the United States, but one factor is certainly the much increased availability of prescription opioids.[78] The public interest and public health would be better served by a redefinition of acceptable and allowable marketing practices for opioids and other controlled drugs.

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# References


1. “OxyContin Marketing Plan, 2002.” Purdue Pharma, Stamford, CN, 2002
2. Cicero T, Inciardi J, Munoz A. Trends in abuse of OxyContin and other opioid analgesics in the United States: 2002–2004. J Pain 2005;6:662–672 [PubMed]
3. Oxycodone and OxyContin. Med Lett Drugs Ther 2001;43:80–81 [PubMed]
4. Hale ME, Fleischmann R, Salzman R, et al. Efficacy and safety of controlled-release versus immediate-release oxycodone: randomized, double-blind evaluation in chronic back pain. Clin J Pain 1999;15:179–183 [PubMed]
5. Kaplan R, Parris WC, Citron MI, et al. Comparison of controlled-release and immediate-release oxycodone in cancer pain. J Clin Oncol 1998;16:3230–3237 [PubMed]
6. Staumbaugh JE, Reder RF, Stambaugh MD, et al. Double-blind, randomized comparison of the analgesic and pharmacokinetic profiles of controlled- and immediate-release oral oxycodone in cancer pain patients. J Clin Pharmacol 2001;41:500–506 [PubMed]
7. Heiskanen T, Kalso E. Controlled-release oxycodone and morphine in cancer related pain. Pain 1997;73:37–45 [PubMed]
8. Mucci-LoRusso P, Berman BS, Silberstein PT, et al. Controlled-release oxycodone compared with controlled-release morphine in treatment of cancer pain: a randomized, double-blind, parallel-group study. Eur J Pain 1998;2:239–249 [PubMed]
9. Bruera E, Belzile M, Pituskin E, et al. Randomized, double-blind, cross-over trial comparing safety and efficacy of oral controlled-release oxycodone with controlled-release morphine in patients with cancer pain. J Clin Oncol 1998;16:3222–3229 [PubMed]
10. “New Drug Application for OxyContin.” Purdue Pharma, Stamford, CN December 1995.
11. Chou R, Clark E, Helfand M. Comparative efficacy and safety of long-acting oral opioids for chronic non-cancer pain. J Pain Symptom Manage 2003;26(5):1026–1048 [PubMed]
12. “OxyContin Marketing Plan, 1996.” Purdue Pharma, Stamford, CN.
13. “OxyContin Marketing Plan, 1997.” Purdue Pharma, Stamford, CN.
14. “OxyContin Marketing Plan, 1998.” Purdue Pharma, Stamford, CN.
15. “OxyContin Marketing Plan, 1999.” Purdue Pharma, Stamford, CN.
16. “OxyContin Marketing Plan, 1996.” Purdue Pharma, Stamford, CN.
17. “OxyContin Marketing Plan, 2001.” Purdue Pharma, Stamford, CN.
18. “OxyContin: balancing risks and benefits,” in *Hearing of the Committee on Health, Education, Labor, and Pensions, United States Senate*, February 12, 2002, p 87 (testimony of Paul Goldenheim, Purdue Pharma)
19. Prescription Drugs: OxyContin Abuse and Diversion and Efforts to Address the Problem. Washington, DC: General Accounting Office; December 2003. Publication GAO-04-110
20. Orlowski JP, Wateska L. The effect of pharmaceutical firm enticements on physician prescribing patterns. There's no such thing as a free lunch. Chest 1992;102:270–273 [PubMed]
21. Stolberg SG, Gerth J. High-tech stealth being used to sway doctor prescriptions. New York Times November 16, 2000. Available at: http://query.nytimes.com/gst/fullpage.html?res=9502EEDF153BF935A25752C1A9669C8B63&sec=&spon=&pagewanted=1. Accessed September 11, 2008 [PubMed]
22. Adams C. Painkiller's sales far exceeded levels anticipated by maker. Wall Street Journal May 16, 2002
23. Tough P. The alchemy of OxyContin: from pain relief to drug addiction. New York Times Magazine July 29, 2001:37
24. Moulin DE, Iezzi A, Amireh R, et al. Randomized trial of oral morphine for chronic non-cancer pain. Lancet 1996;346:143–147 [PubMed]
25. Watson CP, Babul N. Efficacy of oxycodone in neuropathic pain: a randomized trial in postherpetic pain. Neurology 1998;50:1837–1841 [PubMed]
26. Caldwell JR, Rapoport RJ, Davis JC, et al. Efficacy and safety of a once-daily morphine formulation in chronic, moderate-to-severe osteoarthritis pain: results from a randomized, placebo-controlled, double-blind trial and an open-label extension trial. J Pain Symptom Manage 2002;23:178–291 [PubMed]
27. Gimbel J, Richards P, Portenoy R. Controlled-release oxycodone for pain in diabetic neuropathy: a randomized controlled trial. Neurology 2003;60:927–934 [PubMed]

28. Peloso P, Bellamy N, Bensen W, et al. Double blind randomized placebo controlled trial of controlled release codeine in the treatment of osteoarthritis of the hip or knee. J Rheumatol 2000;27:764–771 [PubMed]
29. Roth SH, Fleischmann RM, Burch FX, et al. Around-the-clock controlled-release oxycodone therapy for osteoarthritis-related pain. Arch Intern Med 2000;160:853–860 [PubMed]
30. Caldwell JR, Hale ME, Boyd RE, et al. Treatment of osteoarthritis pain with controlled release oxycodone or fixed combination oxycodone plus acetaminophen added to nonsteroidal anti-inflammatory drugs: a double blind, randomized, multicenter, placebo controlled trial. J Rheumatol 1999;26:862–869 [PubMed]
31. Rowbotham MD, Twilling LO, Davies PS, et al. Oral opioid therapy for chronic peripheral and central neuropathic pain. N Engl J Med 2003;348:1223–1232 [PubMed]
32. Kjaersgaard-Andersen P, Nafei A, Skov O, et al. Codeine plus paracetamol versus paracetamol in longer-term treatment of chronic pain due to osteoarthritis of the hip: a randomized, double-blind multi-centre study. Pain 1990;43:309–318 [PubMed]
33. Raja SN, Haythornthwaite JA, Pappagallo M, et al. A placebo-controlled trial comparing the analgesic and cognitive effects of opioids and tricyclic antidepressants in postherpetic neuralgia. Neurology 2002;59:1015–1021 [PubMed]
34. Huse E, Larbig W, Flor H, et al. The effect of opioids on phantom limb pain and cortical reorganization. Pain 2001;90:47–55 [PubMed]
35. Moran C. MS continuous tablets and pain control in severe rheumatoid arthritis. Br J Clin Res 1991;2:1–12
36. Jamison RN, Raymond SA, Slawsby EA, et al. Opioid therapy for chronic noncancer back pain: a randomized prospective study. Spine 1998;23:2591–2600 [PubMed]
37. Arkinstall W, Sandler A, Groghnour B, et al. Efficacy of controlled-release codeine in chronic non-malignant pain: a randomized placebo-controlled trial. Pain 1995;62:168–178 [PubMed]
38. Sheather-Reid RB, Cohen ML. Efficacy of analgesics in chronic pain: a series of N-of-1 studies. J Pain Symptom Manage 1998;15:244–252 [PubMed]
39. Martell BA, O'Connor PG, Kerns RD, et al. Systematic review: opioid treatment for chronic back pain: prevalence, efficacy, and association with addiction. Ann Intern Med 2007;146:116–127 [PubMed]
40. Ballantyne JC. Opioids for chronic nonterminal pain. South Med J 2006;99:1245–1255 [PubMed]
41. Regier DA, Myers JK, Kramer M, et al. The NIMH epidemiological catchment area program. Historical context, major objectives, and study population characteristics. Arch Gen Psychiatry 1984;41:934–941 [PubMed]
42. Katz N. Opioids: after thousands of years, still getting to know you. Clin J Pain 2007;23:303–306 [PubMed]
43. Irick N, Lipman A, Gitlin M. *Overcoming Barriers to Effective Pain Management* [audiotape]. Rochester, NY: Solutions Unlimited; March 2000
44. Carr B, Kulich R, Sukiennik A, et al. *The Impact of Chronic Pain—An Interdisciplinary Perspective*. Continuing Medical Education program. New York, NY: Power-Pak Communications; 2000:925 Program 424-000-99-010-H01
45. Lipman A, Jackson K., II *Use of Opioids in Chronic Noncancer Pain*. Continuing Medical Education program. New York, NY: Power-Pak Communications; April 2000:6
46. *How You Can Be a Partner Against Pain and Gain Control Over Your Own Pain* [patient brochure]. Stamford, CN: Purdue Pharma; 1998
47. “Partners Against Pain” Web site, under “Professional Education” menu and “Opioids and back pain: the last taboo”—2000. Available at: http://www.partnersagainstpain.com/html/proofed/pmc/pe_pmc6.htm. Accessed March 19, 2001.
48. *Pain Management* [CD and slide instructional program for physicians]. Stamford, CN: Purdue Pharma; 2002
49. *Dispelling the Myths About Opioids* [brochure for physicians]. Stamford, CN: Purdue Pharma; 1998
50. Meier B. Pain Killer Emmaus, PA: Rodale Press; 2003:99
51. Porter J, Jick H. Addiction rare in patients treated with narcotics. N Engl J Med 1980;302:123. [PubMed]
52. Perry S, Heidrich G. Management of pain during debridement: a survey of US burn units. Pain 1982;13:267–280 [PubMed]

53. Fishbain DA, Rosomoff HL, Rosomoff RS. Drug abuse, dependence, and addiction in chronic pain patients. Clin J Pain 1992;8:77–85 [PubMed]
54. Hoffmann NG, Olofsson S, Salen B, Wickstrom L. Prevalence of abuse and dependence in chronic pain patients. Int J Addict 1995;30:919–927 [PubMed]
55. Kouyanou K, Pither CE, Wessely S. Medication misuse, abuse, and chronic dependence in chronic pain patients. J Psychosom Res 1997;43:497–504 [PubMed]
56. Chabal C, Erjaved MK, Jacobson L, et al. Prescription opiate abuse in chronic pain patients: clinical criteria, incidence, and predictors. Clin J Pain 13;150–155 [PubMed]
57. Katz NP, Sherburne S, Beach M, et al. Behavioral monitoring and urine toxicology testing in patients receiving long-term opioid therapy. Anesth Analg 2003;97:1097–1102 [PubMed]
58. Reid M, Engles-Horton L, Weber M, et al. Use of opioid medications for chronic non-cancer pain. J Gen Intern Med 2002;17:173–179 [PMC free article] [PubMed]
59. Michna E, Jamison RN, Pham LD, et al. Urine toxicology screening among chronic pain patients on opioid therapy: frequency and predictability of abnormal findings. Clin J Pain 2007;23:173–179 [PubMed]
60. Hojsted J, Sjogren P. Addiction to opioids in chronic pain patients: a literature review. Eur J Pain 2007;11:490–518 [PubMed]
61. United States Attorney's Office Western District of Virginia [news release]. Available at: http://www.dodig.osd.mil/IGInformation/IGInformationReleases/prudue_frederick_1.pdf. Accessed September 11, 2008
62. *United States of America v The Purdue Frederick Company Inc et al.*, (WD Va, May 10, 2007), Case 1:07CR00029.
63. Rischitelli DG, Karbowicz SH. Safety and efficacy of controlled-release oxycodone: a systematic literature review. Pharmacotherapy 2002;22:898–904 [PubMed]
64. Drug Enforcement Administration, Office of Diversion Control Action plan to prevent the diversion and abuse of OxyContin. Available at: http://www.deadiversion.usdoj.gov/drugs_concern/oxycodone/abuse_oxy.htm. Accessed March 12, 2008
65. Crews JC, Denson DD. Recovery of morphine from a controlled-release preparation: a source of opioid abuse. Cancer 1990;66:2642–2644 [PubMed]
66. "New Drug Application to FDA for OxyContin, Pharmacology Review: 'Abuse Liability of Oxycodone.' " Purdue Pharma, Stamford, CN, 1995
67. States of Alabama, Maine, Kentucky, Virginia, and West Virginia Drug Profile by County—OxyContin, Oxycodone (Excluding OxyContin), and Hydrocodone—2000. Washington, DC: Office of Diversion Control, Drug Enforcement Administration; 2002
68. OxyContin Abuse: Maine's Newest Epidemic. Augusta: Maine Office of Substance Abuse; January 2002
69. Paulozzi LJ. Opioid analgesic involvement in drug abuse deaths in American metropolitan areas. Am J Public Health 2006;96:1755–1757 [PMC free article] [PubMed]
70. Substance Abuse and Mental Health Services Administration. National Survey on Drug Use and Health. Available at: http://www.oas.samhsa.gov/NSDUH/2k4nsduh/2k4Results/2k4Results.pdf. Accessed March 12, 2008
71. Gilson AM, Ryan KM, Joranson DE, et al. A reassessment of trends in the medical use and abuse of opioid analgesics and implications for diversion control: 1997–2002. J Pain Symptom Manage 2004;28:176–188 [PubMed]
72. Substance Abuse and Mental Health Services Administration Results from the 2005 National Survey on Drug Use and Health: national findings. Available at: http://www.oas.samhsa.gov/nsduh/2k5nsduh/2k5Results.pdf. Accessed March 12, 2008
73. Substance Abuse and Mental Health Services Administration Results from the 2006 National Survey on Drug Use and Health. Available at: http://www.oas.samhsa.gov/nsduh/2k6nsduh/2k6Results.pdf. Accessed March 12, 2008
74. Paulozzi LJ, Budnitz DS, Yongli X. Increasing deaths from opioid analgesics in the United States. Pharmacoepidemiol Drug Saf 2006;15:618–627 [PubMed]
75. Pulse Check: Trends in Drub Abuse. Washington, DC: Office of National Drug Control Policy, Executive Office of the President; November 2002
76. Wazana A. Physicians and the pharmaceutical industry: is a gift ever just a gift? JAMA 2000;283:373–380 [PubMed]
77. Grande D. Prescribing profiling: time to call it quits. Ann Intern Med 2007;146:751–752 [PubMed]

78. Compton W, Volkow N. Major increases in opioid analgesic abuse in the United States: concerns and strategies. Drug Alcohol Depend 2006;81:103–107 [PubMed]

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- Review Opioids for chronic nonterminal pain.[South Med J. 2006]

- The NIMH Epidemiologic Catchment Area program. Historical context, major objectives, and study population characteristics.[Arch Gen Psychiatry. 1984]
- Review Opioids: after thousands of years, still getting to know you.[Clin J Pain. 2007]

- Addiction rare in patients treated with narcotics.[N Engl J Med. 1980]
- Management of pain during debridement: a survey of U.S. burn units.[Pain. 1982]
- Review Drug abuse, dependence, and addiction in chronic pain patients.[Clin J Pain. 1992]
- Prevalence of abuse and dependency in chronic pain patients.[Int J Addict. 1995]
- Medication misuse, abuse and dependence in chronic pain patients.[J Psychosom Res. 1997]
- Prescription opiate abuse in chronic pain patients: clinical criteria, incidence, and predictors.[Clin J Pain. 1997]

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- Review Safety and efficacy of controlled-release oxycodone: a systematic literature review.[Pharmacotherapy. 2002]

- Recovery of morphine from a controlled-release preparation. A source of opioid abuse.[Cancer. 1990]

- Opioid analgesic involvement in drug abuse deaths in American metropolitan areas.[Am J Public Health. 2006]
- Trends in abuse of Oxycontin and other opioid analgesics in the United States: 2002-2004.[J Pain. 2005]

- A reassessment of trends in the medical use and abuse of opioid analgesics and implications for diversion control: 1997-2002.[J Pain Symptom Manage. 2004]

- Increasing deaths from opioid analgesics in the United States.[Pharmacoepidemiol Drug Saf. 2006]

- Review Comparative efficacy and safety of long-acting oral opioids for chronic non-cancer pain: a systematic review.[J Pain Symptom Manage. 2003]
- Review Safety and efficacy of controlled-release oxycodone: a systematic literature review.[Pharmacotherapy. 2002]

- Trends in abuse of Oxycontin and other opioid analgesics in the United States: 2002-2004.[J Pain. 2005]

- Review Comparative efficacy and safety of long-acting oral opioids for chronic non-cancer pain: a systematic review.[J Pain Symptom Manage. 2003]
- Review Safety and efficacy of controlled-release oxycodone: a systematic literature review.[Pharmacotherapy. 2002]

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Support Center Support Center

# EXHIBIT C

# Industry Payments to Physicians for Opioid Products, 2013–2015

*Scott E. Hadland, MD, MPH, MS, Maxwell S. Krieger, BS, and Brandon D. L. Marshall, PhD*

*Objectives.* To identify payments that involved opioid products from the pharmaceutical industry to physicians.

*Methods.* We used the Open Payments program database from the Centers for Medicare and Medicaid Services to identify payments involving an opioid to physicians between August 2013 and December 2015. We used medians, interquartile ranges, and ranges as a result of heavily skewed distributions to examine payments according to opioid product, abuse-deterrent formulation, nature of payment, state, and physician specialty.

*Results.* During the study, 375 266 nonresearch opioid-related payments were made to 68 177 physicians, totaling $46 158 388. The top 1% of physicians received 82.5% of total payments in dollars. Abuse-deterrent formulations constituted 20.3% of total payments, and buprenorphine marketed for addiction treatment constituted 9.9%. Most payments were for speaking fees or honoraria (63.2% of all dollars), whereas food and beverage payments were the most frequent (93.9% of all payments). Physicians specializing in anesthesiology received the most in total annual payments (median = $50; interquartile range = $16–$151).

*Conclusions.* Approximately 1 in 12 US physicians received a payment involving an opioid during the 29-month study. These findings should prompt an examination of industry influences on opioid prescribing. (*Am J Public Health.* 2017;107:1493–1495. doi: 10.2105/AJPH.2017.303982)

The nonmedical use of opioids and overdose mortality have reached unprecedented levels in the United States.[1] To respond to concerns about overprescribing of opioids, the Centers for Disease Control and Prevention recently released chronic pain management guidelines that call on physicians to consider nonopioid pain medications as an alternative to opioids.[2] Additionally, some physicians and pharmaceutical industry representatives have suggested that abuse-deterrent formulations—newly marketed brand-name opioids with pill properties that render misuse more difficult—offer a safer option for prescribers.[3,4]

Under the recently implemented Physician Payments Sunshine Act, drug companies are now required to report all transfers of value ("payments") to US physicians.[5] Research suggests that pharmaceutical company payments promote increased prescribing for marketed brand-name medications, even when payments are of low monetary value (e.g., industry-sponsored meals).[6] To date, industry payments to physicians involving opioids have not been studied and deserve further examination because they may impede national efforts to reduce overprescribing.

It is currently unclear which opioids are most heavily marketed, to whom, and in exchange for which physician activities. The extent to which abuse-deterrent formulations and nonopioid alternatives are marketed is also poorly understood. For the first time, exhaustive data on payments are now available through the Open Payments program database implemented under the Physician Payments Sunshine Act.[5,7] We used this novel data set to characterize industry payments to physicians related to opioid marketing.

## METHODS

We extracted all payments between August 1, 2013 (when mandated reporting began), and December 31, 2015, that listed a US Food and Drug Administration (FDA)–approved opioid product. We included buprenorphine but examined buprenorphine and buprenorphine/naloxone marketed for addiction treatment separately from the buprenorphine transdermal patch marketed for pain control. We excluded remifentanil (which is marketed exclusively for anesthesia) and 2 fentanyl products (1 marketed exclusively for anesthesia, and 1 marketed exclusively for in-hospital pain).

We also identified payments involving FDA-recognized abuse-deterrent opioid formulations.[8] For comparison with a nonopioid class of pain medications, we quantified payments for all actively marketed nonsteroidal anti-inflammatory drugs (NSAIDs) in the database. We chose NSAIDs for this comparison because unlike other medication classes used for pain that have additional indications (e.g., medications marketed not only for pain but also for depression or neuralgia), NSAIDs are almost exclusively used for pain control.

We limited the current analysis to nonresearch payments to physicians; we excluded research payments, which are made in

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association with established research protocols, do not explicitly target prescribing behaviors, and may be provided to physicians not actively practicing medicine. We summarized payments in terms of total dollars and number of payments made and identified changes from 2014 to 2015 (the 2 years for which all 12 months of data were available). We used medians, interquartile ranges (IQRs), and ranges as a result of heavily skewed distributions to examine payments according to opioid product, abuse-deterrent formulation, nature of payment (i.e., physician activity leading to the payment), state, and physician specialty. We also assessed payments to physicians receiving the top 1% of payments for opioids. We used Stata version 13.1 (StataCorp LP, College Station, TX) for analyses.

## RESULTS

Over the study period, 375 266 nonresearch payments involving a marketed opioid were made to 68 177 physicians, totaling $46 158 388. Total payments increased from $18 958 125 in 2014 to $20 996 858 in 2015, an increase of 10.7%. The number of payments increased from 145 715 in 2014 to 184 237 in 2015, an increase of 26.4%.

The 5 opioid products constituting the greatest proportion of payments were fentanyl ($21 240 794; 46.0% of total dollars), hydrocodone ($7 123 421; 15.4%), buprenorphine transdermal patch ($5 141 808; 11.1%), oxycodone ($4 487 978; 9.7%), and tapentadol ($4 296 130; 9.3%). Overall, payments for FDA-approved abuse-deterrent formulations totaled $9 352 959 (20.3%), and payments for buprenorphine or buprenorphine/naloxone marketed for addiction treatment totaled $4 561 729 (9.9%). By comparison, payments for NSAIDs amounted to $13 758 385 (not included in previous totals).

Speaking fees or honoraria constituted the largest proportion of payments in dollars, whereas payments involving food and beverage were the most common (Table 1). Payments varied widely according to US state (Figure A, available as a supplement to the online version of this article at http://www.ajph.org). The median paid per physician annually was $15 (IQR = $7–$42; maximum = $1 539 471), with physicians receiving a median of 1 payment annually (IQR = 1–2; maximum = 157). Payments were positively skewed, with the top 1% of physicians (n = 681) receiving $2639 or more annually (Table A, available as a supplement to the online version of this article at http://www.ajph.org). These physicians collectively received $38 073 796 (82.5% of total payments) during the study period.

**TABLE 1—Characteristics of Payments Involving Opioid Products to Physicians: Open Payments Program Database, United States, August 1, 2013–December 31, 2015**

| Nature of Payment | Total Payment Amount, $ (%) | Median Payment, $ (IQR) | No. of Payments (%) |
|---|---|---|---|
| Speaking fees or honoraria | 29 190 854 (63.2) | 2 010 (1 000–3 750) | 9 161 (2.4) |
| Food and beverages | 7 872 581 (17.1) | 14 (11–18) | 352 298 (93.9) |
| Consulting fees | 5 886 461 (12.8) | 1 000 (500–2 500) | 2 145 (0.6) |
| Travel and lodging | 2 904 940 (6.3) | 537 (100–1 131) | 4 048 (1.1) |
| Education | 222 869 (0.5) | 14 (5–25) | 7 422 (2.0) |
| Other[a] | 80 683 (0.2) | 100 (14–500) | 192 (0.1) |

Note. IQR = interquartile ranges.
[a]Includes gifts, entertainment, and space rental or facility fees.

Physicians specializing in anesthesiology received the most in total annual payments (median = $50; IQR = $16–$151; n = 4339), followed by physical medicine and rehabilitation (median = $48; IQR = $14–$145; n = 3502) and pain medicine (median = $43; IQR = $12–$125; n = 3090). Physicians specializing in family medicine received the largest total number of payments (n = 20 592).

## DISCUSSION

According to the Association of American Medical Colleges, there were 829 962 active physicians in the United States at the beginning of the study period in 2013[9]; thus, our results suggest that 1 in 12 physicians received an industry payment involving an opioid during the 29-month study period. Although half of all the annual payments were $15 or less, even small payments (including meals) are associated with increased prescribing of marketed products.[6] FDA-approved abuse-deterrent formulations, which have properties expected to render misuse less likely, constituted only one fifth of the total payments, suggesting that such medications may not be as heavily marketed as other opioids are. Additionally, despite Centers for Disease Control and Prevention recommendations to consider use of nonopioid medications for pain, NSAIDs, a prominent family of nonopioid pain medications, were not as heavily marketed as opioids were.[2]

Fentanyl was the most common opioid involved in payments to physicians. National data implicate fentanyl in a rapidly increasing number of overdose deaths, although most are caused by illicitly manufactured fentanyl.[10] Further studies should clarify the extent to which industry payments contribute to prescribing patterns and overdose rates across geographic regions, particularly given the heterogeneity we observed in payments among states. Although payment amounts in dollar terms were greatest to physicians specializing in anesthesiology, physical medicine and rehabilitation, and pain medicine—specialists with expertise in pain management—family medicine physicians received the largest number of payments, indicating extensive marketing of opioid products to primary care physicians. Because there were 108 917 active family physicians in the United States in 2013,[9] our data highlight that nearly 1 in 5 received an opioid-related payment.

A limitation of this study was the absence of further details about industry-physician interactions; some payments may have supported education on appropriate prescribing behaviors.[11] One tenth of the payments involved buprenorphine marketed for addiction treatment, which may have resulted in improved education on addiction care. Risk Evaluation and Mitigation Strategies programs imposed by the FDA require education on extended-release/long-acting

opioids and on transmucosal fentanyl products, and some industry payments to physicians may have been related to this regulation. Another limitation was that some abuse-deterrent formulations were approved partway through the study period; in future years, such medications might be associated with a greater portion of industry payments.

## PUBLIC HEALTH IMPLICATIONS

To our knowledge, this was the first large-scale examination of industry payments involving opioids. Financial transfers were substantial and widespread and may be increasing in number and value. Although opioid prescribing declined nationally during the study period,[12] these results should prompt an examination of industry influences on prescribing amid an ongoing opioid crisis. Further research should examine whether payments are related to opioid misuse and overdose, and policymakers might consider whether caps should be imposed on certain payments. AJPH

### CONTRIBUTORS

S. E. Hadland and B. D. L. Marshall designed the study and wrote the protocol. S. E. Hadland conducted the literature review and wrote the first draft of the article. M. S. Krieger undertook data management and statistical analyses with additional input from S. E. Hadland and B. D. L. Marshall. All authors contributed to and approved the final article.

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### HUMAN PARTICIPANT PROTECTION

The study was considered exempt by the Brown University institutional review board.

### REFERENCES

1. Rudd RA, Aleshire N, Zibbell JE, Gladden RM. Increases in drug and opioid overdose deaths - United States, 2000-2014. *MMWR Morb Mortal Wkly Rep*. 2016; 64(50-51):1378–1382.

2. Dowell D, Haegerich TM, Chou R. CDC guideline for prescribing opioids for chronic pain–United States, 2016. *JAMA*. 2016;315(15):1624–1645.

3. Webster LR, Markman J, Cone EJ, Niebler G. Current and future development of extended-release, abuse-deterrent opioid formulations in the United States. *Postgrad Med*. 2017;129(1):102–110.

4. Alexander L, Mannion RO, Weingarten B, Fanelli RJ, Stiles GL. Development and impact of prescription opioid abuse deterrent formulation technologies. *Drug Alcohol Depend*. 2014;138:1–6.

5. Agrawal S, Brown D. The Physician Payments Sunshine Act — two years of the open payments program. *N Engl J Med*. 2016;374(10):906–909.

6. DeJong C, Aguilar T, Tseng C-W, Lin GA, Boscardin WJ, Dudley RA. Pharmaceutical industry-sponsored meals and physician prescribing patterns for Medicare beneficiaries. *JAMA Intern Med*. 2016;176(8):1114–1122.

7. Centers for Medicare & Medicaid Services. Open Payments. 2016. Available at: https://www.cms.gov/openpayments. Accessed February 12, 2017.

8. US Food and Drug Administration. FDA facts: abuse-deterrent opioid medications. April 2017. Available at: http://www.fda.gov/NewsEvents/Newsroom/FactSheets/ucm514939.htm. Accessed July 3, 2017.

9. Association of American Medical Colleges. *2014 Physician Specialty Data Book*. November 2014. Available at: https://members.aamc.org/eweb/upload/PhysicianSpecialtyDatabook2014.pdf. Accessed February 12, 2017.

10. Gladden RM, Martinez P, Seth P. Fentanyl law enforcement submissions and increases in synthetic opioid–involved overdose deaths — 27 states, 2013–2014. *MMWR Morb Mortal Wkly Rep*. 2016;65(33):837–843.

11. Sismondo S. Key opinion leaders and the corruption of medical knowledge: what the Sunshine Act will and won't cast light on. *J Law Med Ethics*. 2013;41(3):635–643.

12. Goodnough A, Tavernise S. Opioid prescriptions drop for first time in two decades. *New York Times*. May 20, 2016:A1.

# EXHIBIT D

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U.S.

# A Pain-Drug Champion Has Second Thoughts

*By Thomas Catan and Evan Perez*

Updated Dec. 17, 2012 11:36 a.m. ET

It has been his life's work. Now, Russell Portenoy appears to be having second thoughts.

Two decades ago, the prominent New York pain-care specialist drove a movement to help people with chronic pain. He campaigned to rehabilitate a group of painkillers derived from the opium poppy that were long shunned by physicians because of their addictiveness.

Dr. Portenoy's message was wildly successful. Today, drugs containing opioids like Vicodin, OxyContin and Percocet are among the most widely prescribed pharmaceuticals in America.

Opioids are also behind the country's deadliest drug epidemic. More than 16,500 people die of overdoses annually, more than all illegal drugs combined.

Now, Dr. Portenoy and other pain doctors who promoted the drugs say they erred by overstating the drugs' benefits and glossing over risks. "Did I teach about pain management, specifically about opioid therapy, in a way that reflects misinformation? Well, against the standards of 2012, I guess I did," Dr. Portenoy said in an interview with The Wall Street Journal. "We didn't know then what we know now."

Recent research suggests a significantly higher risk of addiction than previously thought, and questions whether opioids are effective against long-term chronic pain.

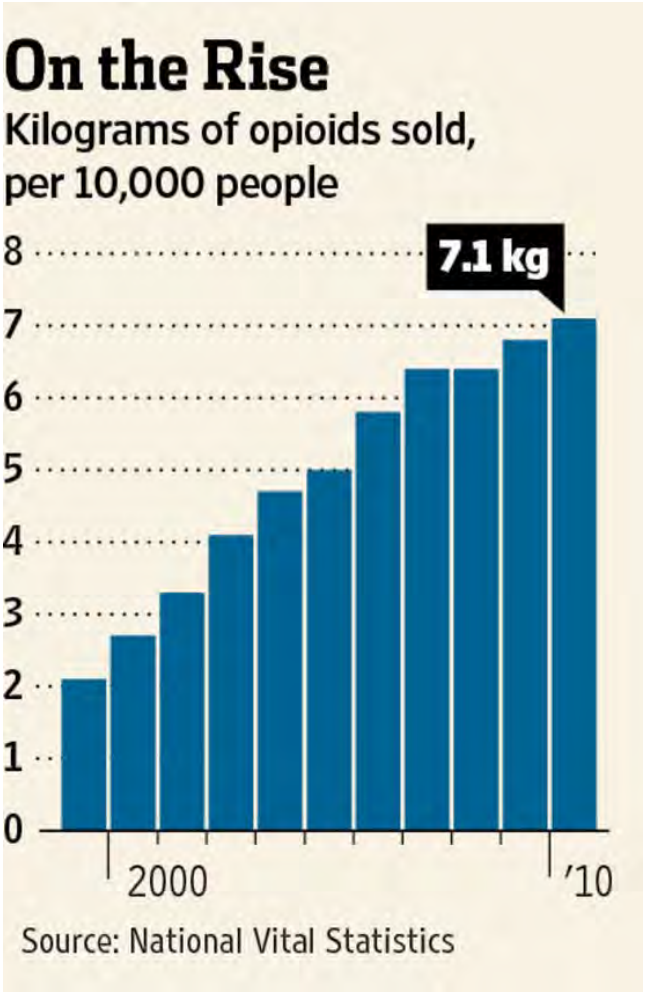
The change of heart among former champions of opioid use has happened quietly, largely beyond the notice of many doctors. New York psychiatrist Joseph Carmody said he was "shocked" after attending a recent lecture outlining the latest findings on opioid risk.

"It goes in the face of everything you've learned," he said. "You saw other doctors come around to it and saying, 'Oh my God, what are we doing?'"

Because doctors feared they were dangerous and addictive, opioids were long reserved mainly for cancer patients. But Dr. Portenoy argued that they could be also safely be taken for months or years by people suffering from chronic pain. Among the assertions he and his followers made in the 1990s: Less than 1% of opioid users became addicted, the drugs were easy to discontinue and overdoses were extremely rare in pain patients.

Many of those experts now say those claims were weren't based on sound scientific evidence. "I gave innumerable lectures in the late 1980s and '90s about addiction that weren't true," Dr. Portenoy said in a 2010 videotaped interview with a fellow doctor. The Journal reviewed the conversation, much of which is previously unpublished.

In it, Dr. Portenoy said it was "quite scary" to think how the growth in opioid prescribing driven by people like him had contributed to soaring rates of addiction and overdose deaths. "Clearly, if I had an inkling of what I know now then, I wouldn't have spoken in the way that I spoke. It was clearly the wrong thing to do," Dr. Portenoy said in the recording.




Speaking to the Journal in September, Dr. Portenoy tempered that statement with cautions about overturning what he sees as the positive change he achieved. He cited his 82-year-old mother, who has taken hydrocodone to control arthritis for 15 years. "If you insist on regulation, then you're consigning my mother and many millions of people like my mother to live in chronic pain," he said.

Virtually no one wants to return to a time when doctors were reluctant to use opioids even for cancer patients. All sides also agree that there is a group of people who do well on opioids long-term, taming their pain while avoiding addiction or excessive sedation, although there is no research on how large this group is or how to identify them before they begin a treatment. There is also widespread agreement that they can be used, with caution, for acute pain, such as after an operation.

But some specialists now question whether the drugs should be prescribed so freely for

months or years to people with chronic pain that isn't related to cancer, as Dr. Portenoy proposed 25 years ago. "People lost sight of the fact that these are dangerous drugs that are highly addictive," said Jane Ballantyne, a pain specialist at the University of Washington. She once agreed with Dr. Portenoy and proponents of broad opioid use but now believes they need to be used more selectively.

Opium-derived painkillers have been around for thousands of years. Early in the 20th century, heroin was sold as a cough suppressant. Heroin addiction in the U.S. skyrocketed. Congress banned the drug in 1924 and doctors became deeply wary about using opioids.

Dr. Portenoy set out to change that. As a young doctor at Memorial Sloan-Kettering hospital in New York, he noticed that opioids were effective in cancer patients with terrible pain.

In 1986, at the age of 31, he co-wrote a seminal paper arguing that opioids could also be used in the much larger group of people without cancer who suffered chronic pain. The paper was based on just 38 cases and included several caveats. Nevertheless, it opened the door to much broader prescribing of the drugs for more common complaints such as nerve or back pain.

Charming and articulate, he became a sought-after public speaker. He argued that opioids are a "gift from nature" that were being forsaken because of "opiophobia" among doctors. "We had to destigmatize these drugs," said Dr. Portenoy.

He rose to chairman of pain medicine and palliative care at Beth Israel Medical Center in New York. His small office is studded with awards and evidence of his offbeat sense of humor. He prominently displays a magazine mock-up that jokingly dubs him "The King of Pain."

At medical conferences, his confident, knowing manner helped smooth the way for his message. Before an audience of government regulators, he once joked that he might tell a patient at low risk of abuse: "Here, [have] six months of drugs. See you later," he said, according to a Food and Drug Administration transcript. Amid laughter, he added, "It's just hyperbole. I don't actually do that."

Steven Passik, a psychologist who once worked closely with Dr. Portenoy and describes him as his mentor, says their message wasn't based on scientific evidence so much as a zeal to improve patients' lives. "It had all the makings of a religious movement at the time," he says. "It had that kind of a spirit to it."

Drug companies took notice. In 1996, Purdue Pharma LP released OxyContin, a form of oxycodone in a patented, time-release form, and rivals followed suit. Today, sales of opioid painkillers total more than $9 billion a year, according to IMS Health, which tracks sales for drug companies.

In 2007, Purdue Pharma and three executives pleaded guilty to "misbranding" of the drug as less addictive and less subject to abuse than other pain medicines and paid $635 million in fines.

Purdue Pharma says it has worked to discourage abuse of its drugs, adding that OxyContin is safe and effective when used properly.

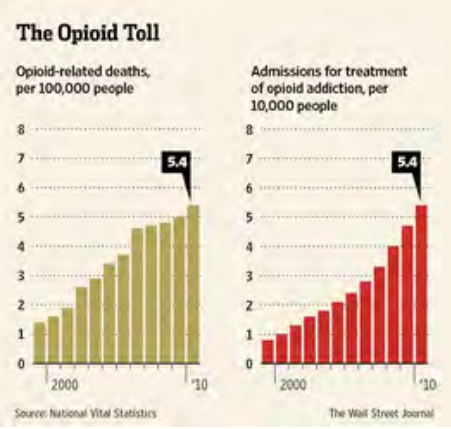
In the late 1990s, groups such as the American Pain Foundation, of which Dr. Portenoy was a director, urged tackling what they called an epidemic of untreated pain. The American Pain Society, of which he was president, campaigned to make pain what it called the "fifth vital sign" that doctors should monitor, alongside blood pressure, temperature, heartbeat and breathing.

Dr. Portenoy helped write a landmark 1996 consensus statement by two professional pain societies that said there was little risk of addiction or overdose among pain patients. In lectures he cited the statistic that less than 1% of opioid users became addicted.

Today, even proponents of opioid use say that figure was wrong. "It's obviously crazy to think that only 1% of the population is at risk for opioid addiction," said Lynn Webster, president-elect of the American Academy of Pain Medicine, one of the publishers of the 1996 statement. "It's just not true."

The figure came from a single-paragraph report in the New England Journal of Medicine in 1980 describing hospitalized patients briefly given opioids. Dr. Portenoy now says he shouldn't have used the information in lectures because it wasn't relevant for patients with chronic noncancer pain.

For such a widely used therapy, there is relatively little scientific evidence that opioid drugs are safe and effective for long-term use. "Data about the effectiveness of opioids does not exist," Dr. Portenoy said in his recent Journal interview. To get a painkiller approved, companies must prove that it is better at reducing pain than a sugar pill during short trials often lasting less than 12 weeks.




"Do they work for five years, 10 years, 20 years?" Dr. Portenoy said in the Journal interview. "We're at the level of anecdote." Even so, he says, the drugs can still benefit carefully selected patients.

Dr. Portenoy's ideas about opioids reached into mainstream medicine and attracted outspoken advocates. In a 1998 talk in Houston, Alan Spanos, a South Carolina pain specialist, said patients with chronic noncancer pain could be trusted to decide themselves how many painkillers to take without risk of overdose. According to a recording, Dr. Spanos said he understood that a patient would simply "go to sleep" before stopping breathing. While asleep, he said, the patient "can't take a dangerous dose. It sounds scary, but as far as I know, nobody anywhere is getting burned by doing it this way."

Dr. Spanos declined to say whether he still agreed with his previous statements. He said opioids can be helpful and safe with proper use.

One of Dr. Portenoy's chief complaints was that doctors were reluctant to prescribe opioids because they feared scrutiny by regulators or law enforcement. In the second half of the 1990s, he and his followers campaigned successfully for policies to change that.

In 1998, the Federation of State Medical Boards released a recommended policy reassuring doctors that they wouldn't face regulatory action for prescribing even large amounts of narcotics, as long as it was in the course of medical treatment. In 2004 the group called on state medical boards to make undertreatment of pain punishable for the first time.

That policy was drawn up with the help of several people with links to opioid makers, including David Haddox, a senior Purdue Pharma executive then and now. The federation said it received nearly $2 million from opioid makers since 1997. The federation says it derives the majority of its funding from administering medical licensing exams, credential verification, and data services.

A federation-published book outlining the opioid policy was funded by opioid makers including Purdue Pharma, Endo Health Solutions Inc. and others, with proceeds totaling $280,000 going to the federation. Endo declined to comment.

Purdue Pharma said, "Dr. Haddox was recruited by the FSMB, so he did not have undue or inappropriate influence" on the federation's output. Purdue declined to make Dr. Haddox available to comment.

The federation said it didn't believe its model policy contributed to increased prescriptions and said drug makers didn't influence its guidelines.

In 2001, the Joint Commission, which accredits U.S. hospitals, issued new standards telling hospitals to regularly ask patients about pain and to make treating it a priority. The now-familiar pain scale was introduced in many hospitals, with patients being asked to rate their pain from one to 10 and circle a smiling or frowning face.

The Joint Commission published a guide sponsored by Purdue Pharma. "Some clinicians have inaccurate and exaggerated concerns" about addiction, tolerance and risk of death, the guide said. "This attitude prevails despite the fact there is no evidence that addiction is a significant issue when persons are given opioids for pain control."

Purdue said the booklet emerged from a process that "represented the consensus of a broad range of interested stakeholders." Drug makers regularly pay for educational materials for physicians as an element of their marketing.

The Joint Commission said its standards didn't encourage physicians and hospitals to increase prescriptions. "I think that's a very distorted and not helpful explanation of what's going on," said Ana McKee, the Joint Commission's chief medical officer.

Over his career, Dr. Portenoy has disclosed relationships with more than a dozen companies, most of which produce opioid painkillers. "My viewpoint is that I can have those relationships, they would benefit my educational mission, they benefit in my research mission, and to some extent, they can benefit my own pocketbook, without producing in me any tendency to engage in undue influence or misinformation," he said.

Dr. Portenoy and Beth Israel declined to provide details of their funding by drug companies. A 2007 fundraising prospectus from Dr. Portenoy's program shows that his program received millions of dollars over the preceding decade in funding from opioid

makers including Endo, Abbott Laboratories , Cephalon, Purdue Pharma and Johnson & Johnson .

Endo, Abbott, Janssen and Purdue declined to comment. Cephalon's current owner, Teva Pharmaceutical Industries Ltd. , didn't immediately have a comment.

In May of this year, the Senate Finance Committee opened an investigation into the financial ties between the pharmaceutical makers and the doctors and groups that advocated broader use of opioids. It asked opioid makers to disclose how much they had paid Dr. Portenoy, his program and several organizations he was involved with.

After spending most of his professional life advocating greater use of the drugs, Dr. Portenoy said there is still little research to show whether patients who embark on long-term opioid therapy will ever be able to stop.

Earlier this year, he said, he asked his mother whether she would stop taking her hydrocodone as part of a scientific study. She said no.

"How difficult is it for her to get off these drugs?" Dr. Portenoy asked. "You have no idea and neither do I, because no one knows."

*—Devlin Barrett contributed to this article.*

**Write to** Thomas Catan at thomas.catan@wsj.com and Evan Perez at evan.perez@wsj.com

**Corrections & Amplifications**

In the United States in 2010, there were 5.4 opioid-related deaths per 100,000 people and 5.4 admissions for treatment of opioid addiction per 10,000 people. An earlier version of a graphic accompanying this article reversed the labels on the two charts.

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# EXHIBIT E

American Pain Foundation
A United Voice of Hope and Power over Pain



# A Reporter's Guide:
## *Covering Pain and Its Management*



*Pain is a complex perception that differs enormously from one person to another, even those with seemingly identical injuries or illnesses.*

# Introduction

*Everyone has experienced pain—whether it's a pounding headache at the end of a long day, a throbbing toothache warning of a cavity or infection, an open wound or sprained ankle from a fall, or a stinging burn from touching a hot pan.*

*There are hundreds of pain syndromes, and pain is often a chief symptom of most chronic conditions, including cancer, diabetes, arthritis, fibromyalgia and a host of neurological disorders. For millions of Americans, pain persists, interfering with everyday activities and enjoyment of life. People living with chronic pain will often avoid certain movements or activities, fearful they will cause more injury or to avoid the anxiety of anticipated pain.*

**Pain is complex and frequently misunderstood by the public. The issue of pain is riddled with myths and misperceptions, which makes the task of informing and educating people about pain and its management that much more challenging.**

## SOME COMMON MISCONCEPTIONS ABOUT PAIN

- **Pain is "all in your head."** Although this is partially true because we need our brains for the perception of pain, that does not mean pain is imaginary when the source of pain is not well understood. Pain is all too real to the person who lives with it day in and out.
- **Pain is just something one has to live with**—an inevitable part of a disease or condition. The fact is most pain can be relieved with proper pain management.
- **Pain is a natural part of growing older**. While pain is more common as we age because conditions that cause pain (e.g., arthritis, degenerative joint diseases, cancer, shingles, osteoporosis) are more frequent in older adults, it should not be something people have to struggle with.
- **The best judge of pain is the physician or nurse.** Studies have shown that there is little correlation between what a physician or nurse might "guess" about someone's actual pain. The person with pain is the authority on the existence and severity of his/her pain. The self-report is most reliable indicator.
- **Seeking medical care for pain is a sign of weakness.** Pain carries a stigma, and many people hesitate talking about their pain and how it affects their daily life; they also don't want to be considered a "bad" patient.
- **Use of strong pain medication leads to addiction.** Many people living with pain and even some healthcare providers falsely believe opioids (strong pain medicines) are universally addictive. Studies have shown that the risk of addiction is small when these medicines are properly prescribed and taken as directed. As with any medication, there are risks, but these risks can be managed.

# *Key Reporting Challenges*

*A limited and informal survey of reporters, editors and producers revealed the following challenges when researching and covering the pain/pain management story:*

- Stigma of pain management, especially among legal and government regulatory bodies
- Hesitancy on the part of patients and providers to discuss opioids for legitimate chronic pain management given misperceptions about opioids and addiction
- Ability to find unbiased, credible information about pain
- Limited number of randomized controlled trials
- Finding pain patients who live with the type of pain and/or use the pain management approach being reported in the news story
- Accurately characterizing the pain experience given that every person experiences pain differently, even if they have a similar injury or illness

## THE UBIQUITOUS NATURE OF PAIN

Consider the following…

- Most Americans (80%) will suffer from back pain at some point in their lives.
- As we age, arthritis hinders the normally smooth sliding motion of our joints and connective tissues, resulting in stiffness and discomfort. Arthritis is the leading cause of disability in people over the age of 55.
- Pain associated with pediatric immunizations is a significant source of anxiety for children receiving the immunizations, and evidence suggests that the way children and parents cope can set the stage for future pain responses.
- Damage to or dysfunction of the central nervous system, due to stroke, multiple sclerosis, epilepsy, brain or spinal cord injuries or Parkinson's disease, also stimulates pain pathways.
- An estimated 30 to 50% of patients undergoing active treatment for cancer and 70% of those with advanced stages of the disease experience significant levels of pain and may be reluctant to discuss their pain with their doctors.

*Sources:* The American Academy of Physical Medicine and Rehabilitation, Arthritis Foundation, Mayday Fund, National Institute of Neurological Disorders and Stroke, National Cancer Institute.

# *Purpose of This Guide*

*The American Pain Foundation (APF) has developed this Guide as a primer on pain and pain management to help meet the informational needs of busy reporters, editors and producers covering the pain story. We know it's a complex topic, and hope you will find this to be a useful resource.*

## INSIDE YOU WILL FIND:


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Be sure to visit the Newsroom section of the American Pain Foundation's web site at www.painfoundation.org to download additional copies and to check for posted updates and new Topic Briefs as they are added. Here you will also find recent news releases, press statements and background information on a wide variety of issues related to pain care.

### ABOUT THE AMERICAN PAIN FOUNDATION

APF's mission is to improve the quality of life for people with pain by:

- Raising public awareness;
- Providing practical information, education and support;
- Advocating to remove barriers and increase access to effective pain management; and,
- Promoting research.

Since its founding in 1997, the American Pain Foundation (APF) has been at the forefront of advocating for people living with a wide variety of pain conditions and their caregivers.

Our grassroots effort, *Power Over Pain Action Network,* is now active in nearly 40 states and is comprised of people living with pain, caregivers, healthcare providers and advocates, who are working hard to call attention to the urgent need for positive changes in pain policy, practice and research investment.

### EXPERTS AVAILABLE FOR INTERVIEW

APF can connect reporters with a wide array of leading pain experts, as well as people living with pain and their caregivers. Whether you are working on a national or local story, we can help coordinate interviews about pain-specific conditions and other important issues related to pain (e.g., depression, coping skills, financial matters, disparities, treatment options).

If you are interested in interviewing someone at APF or need additional resources, please contact Tina Regester, APF's communications manager, at (443) 690-4707 or tregester@painfoundation.org.

# A Primer on Pain and Its Management

## BURDEN OF PAIN IN AMERICA: AN EVOLVING PUBLIC HEALTH CRISIS

*Pain is a serious and costly public health issue. It affects more Americans than diabetes, heart disease and cancer combined, and is a leading cause of disability in the United States. Even though pain is one of the most common reasons patients consult a healthcare provider, it is often inadequately assessed and treated, resulting in needless suffering and poor patient outcomes.*

### PAIN IS WOEFULLY UNDERTREATED FOR A VARIETY OF REASONS, INCLUDING:

- Misconceptions about opioid addiction
- Lack of access to care
- Cultural norms and the stigma associated with admitting pain
- Limited or no professional training in pain management, which leaves healthcare providers ill-equipped to effectively respond to patients' reports of pain
- Concerns among physicians about prescribing pain medications for chronic pain, and fears of scrutiny by regulators or law enforcement
- Inadequate funding for pain research (less than 2% of NIH research budget was dedicated to pain studies)

Untreated or poorly managed pain can compromise every aspect of life, including a person's physical and mental health, social and intimate relations, ability to sleep and perform everyday tasks, work productivity and financial well being.

Chronic pain is not only emotionally and physically debilitating for patients, it also places a tremendous burden on families and caregivers, and contributes to excessive healthcare costs. The economic toll of chronic pain exceeds $100 billion each year in the United States alone. As the 75 million Baby Boomers move toward retirement, the epidemic of untreated or undertreated pain is expected to continue.

*More than one-quarter of Americans (26%) age 20 years and over—or, an estimated 76.5 million people—report that they have had a problem with pain. This number does not account for acute pain.*

Source: National Center for Health Statistics,2006.

## PAIN BASICS

*The International Association for the Study of Pain defines pain as: An unpleasant sensory and emotional experience associated with actual or potential tissue damage or described in terms of such damage.*

### COMMON PAIN CONDITIONS

- Headaches or migraine
- Back pain and sciatica
- Neck and shoulder pain
- Joint pain due to arthritis, bursitis, fibromyalgia or degenerative joint disease
- Muscle pain from overuse or strain, injury or fibromyalgia
- Post-surgical pain

***For definitions of these and other pain conditions, as well as common pain terms, please refer to the Pain A to Z listing at the back of this resource.***

At its best, pain is the body's natural alarm system, alerting us to injury (or further injury if already injured). It prompts us to stop a harmful behavior or seek medical attention. For example, lifting too much weight might result in a piercing pain in a person's back. Within moments of touching a hot surface, the fiery sensation of a burn warns us to quickly pull away. Worsening abdominal pain may be a sign of appendicitis or other serious infection. Pain also triggers inflammation, which directs healing cells to the area of injury. The experience of pain also beckons the injured person to rest, promoting healing.

At its worst, unrelenting pain robs people of their livelihood and well being. When pain persists, it is often a sign that the body's alert system has broken down. In other words, pain signals remain active. Over time, this heightened response may:

- Harm the nerves, blood vessels and organs
- Suppress immune function
- Result in excessive inflammation
- Delay healing

Since the brain remembers pain, pain may be imprinted into the nerve tissue and continue to send pain sensations even in the absence of painful stimuli.

### *Chronic Pain-Brain Connection*

New research is unraveling how chronic activation of the biological pathways transmitting pain is associated with structural and chemical changes in the brain. A recent study suggests that constant pain signals can result in mental rewiring that affects the frontal cortex, the area of the brain mainly associated with emotion and attention. According to researchers, this provides the first objective proof of brain disturbances in patients with chronic pain that is unrelated to the sensation of physical pain.

## ACUTE VS. CHRONIC PAIN

*There are two main types of pain: acute and chronic.*

| | ACUTE PAIN | CHRONIC PAIN |
|---|---|---|
| **Onset** | Usually sudden | Sudden or gradual development |
| **Cause** | Typically linked to an event, such as an injury or disease | Contributing factors are less certain |
| **Duration** | Temporary (up to 3 months) | Persistent (beyond usual healing time or longer than 3 months) |
| **Pain Identification** | Painful areas are generally well identified | Painful areas are less easily differentiated |
| **Pattern** | Self-limiting or readily corrected | Continuous or intermittent; intensity may vary or remain constant |
| **Course** | Pain usually lessens over time | Pain usually increases over time |
| **Response** | Stress response may be present (increased heart and/or breathing rate, increase in blood pressure) | Stress response often absent |
| **Prognosis** | Total relief typically possible | Total relief often impossible |

*Adapted from:* McCance K, Huether SE, eds. Pathophysiology: the biologic basis for disease in adults and children. 5th ed. New York, NY: Elsevier, 2006:447-489.

**Acute Pain** occurs suddenly due to illness, inflammation, injury or surgery. It has a short duration that subsides when the injured tissue heals. The cause of the pain can usually be diagnosed and treated.

**Chronic Pain** is pain that lasts long enough (after normal healing or for at least three months), or is intense enough, to affect a person's normal activities and well-being. Failure to treat acute pain promptly and appropriately at the time of injury, during initial medical and surgical care or at the time of transition to community-based care, contributes to the development of chronic pain syndromes.

With chronic pain, pain signals may remain active in the nervous system for weeks, months or even years. Unlike acute pain, chronic pain has no value or benefit; it is a disease in its own right. It can also be especially challenging to treat.



## PAIN ASSESSMENT

Timely access to quality pain management is the best way to minimize the suffering and disability often associated with undertreated pain and to avoid additional problems down the road. Science is revealing the role of unrelieved acute pain in the development of chronic, persistent pain.

Most hospitals, nursing homes and other healthcare facilities are now required to assess and treat pain. To correctly diagnose pain, healthcare professional will:

- Perform a thorough physical exam
- Complete a pain assessment
- Ask detailed questions about the patient's medical history and lifestyle
- Order blood work, X-rays, electrical tests to detect nerve damage, or other diagnostic and laboratory tests

Pain is a subjective experience, and it is critical for healthcare professionals to have a complete picture of the patient's pain history. He/she may ask about seven characteristics of pain to help LOCATE the pain and make the correct diagnosis.

**L** the exact Location of the pain and whether it travels to other body parts
**O** Other associated symptoms such as nausea, numbness, or weakness
**C** The Character of the pain, whether it's throbbing, sharp, dull or burning
**A** Aggravating or Alleviating factors. What makes the pain better or worse?
**T** the Timing of the pain, how long it lasts, is it constant or intermittent?
**E** the Environment where the pain occurs, for example, while working or at home

The type of pain someone is experiencing is often a clue to its cause; for example, chronic pain that is burning or tingling is often the result of nerve disease (neuropathy).

### EFFECTS OF UNRELIEVED CHRONIC PAIN ON PHYSICAL AND MENTAL HEALTH

If untreated, pain can have serious physiological, psychological and social consequences. It can:

- Limit the ability to work, sleep, exercise or perform everyday tasks (for example, dressing, going to the grocery store)
- Reduce mobility
- Impair strength
- Diminish appetite
- Make it difficult to recover from an injury or fight infection by weakening the immune system
- Aggravate other health problems
- Lead to depression and/or anxiety, which often worsen pain sensations
- Make it difficult to concentrate or reason
- Place added strain on relationships and interfere with intimacy
- Result in a loss of self-esteem and independence

Pain scales are additional tools available to help patients describe the intensity of their pain. These assessment tools help healthcare professionals diagnose or measure a patient's level of pain. These include numeric, verbal or visual scales.

With **numerical scales**, patients use numbers from 0-10 (0 being no pain and 10 being the worst pain ever) to rate the intensity of the pain.

**Verbal scales** contain commonly used words such as "mild," "moderate" and "severe" to help patient's describe the severity of the pain.

**Visual scales** use aids like pictures of facial expressions, colors or gaming objects, such as poker chips, to help explain the severity of pain. One type, the Wong Baker Faces Pain Rating Scale, shows six different facial expressions from happy (no hurt) to agony (hurts the worst) to help show healthcare professionals how much pain a patient feels. Body diagrams may also be used to help pinpoint where the pain occurs.




From Hockenberry MJ, Wilson D, Winkelstein ML: *Wong's Essentials of Pediatric Nursing*, ed. 7, St. Louis, 2005, p. 1259. Used with permission. Copyright, Mosby.

**Multidimensional pain assessment tools**, such as the McGill Pain Questionnaire (MPQ) and the Brief Pain Inventory (BPI), have been developed to quantify different aspects of pain, including location and quality of pain and its effect on mood and function. However, these take longer to administer than the simpler scales and some patients who are cognitively impaired or poorly educated may find them difficult to complete. They are generally used in pain research, but can be adapted for clinical use if appropriate and valuable.

Our processing of pain is complex. A basic explanation is that the pain signals of acute pain are initiated when receptors on the skin, within an organ, tissue or nerve are triggered by injury or disease, known or unknown. A series of events follow: an electrical impulse, or pain message, is generated that is then carried on nerve fibers to the spinal cord. The spinal cord transmits the pain signal to various levels of the brain for interpretation and response. At any time during the transport of pain messaging, these noxious signals can be blocked, enhanced or modified. Signaling associated with chronic pain is much more complicated than acute pain as science is beginning to show.

## TREATING PAIN

Successful pain management aims to:

1) lessen the pain
2) improve functioning and
3) enhance quality of life

Pain treatment needs to be individualized and, in most cases, requires a team of providers, as well as social support from family and friends. Most often, an integrative approach is needed to provide pain relief, which includes a combination of treatment options; this also encourages patients to actively participate in self-care. Treatment options may include:

- Medication (anti-inflammatory medicines, opioids or other classes of drugs)
- Psychosocial interventions (cognitive-behavioral counseling, guided imagery)
- Rehabilitative approaches (exercise, application of heat/cold, myofascial release, occupational therapy, if needed)
- Complementary alternative medicine (massage, acupuncture, hypnosis)
- Injection or infusion therapies
- Implantable devices and surgical procedures

Research shows that pain can affect patients' emotions and behavior and interfere with the ability to concentrate, manage everyday tasks and cope with stress. Likewise, stress and emotional pressures can make pain worse, provoking "flare ups" and contributing to alterations in the immune system response. These relationships are not always easily recognized or readily fixed by medical procedures or medications alone.

New treatments under investigation are aimed at the physical, psychological and environmental components of chronic pain. Research is also examining the role of genetic predisposition and the immune system in mitigating pain signals.

For a detailed description of the different treatment modalities for managing pain, please refer to the America Pain Foundation's *Treatment Options: A Guide for People Living with Pain*.

### MEDICATIONS & PAIN MANAGEMENT

Medications play an important role in the treatment of pain. There are three major classes of medications for pain control:

**Non-opioids:** non-steroidal anti-inflammatory drugs (NSAIDs) and acetaminophen

**Opioids**: morphine, oxycodone, methadone, codeine and fentanyl are examples

**Adjuvant analgesics**: a loose term referring to the many medications originally used to treat conditions other than pain, but now also used to help relieve specific pain problems; examples include some antidepressants and anticonvulsants. Some of these drugs have been shown to work well for specific types of pain.

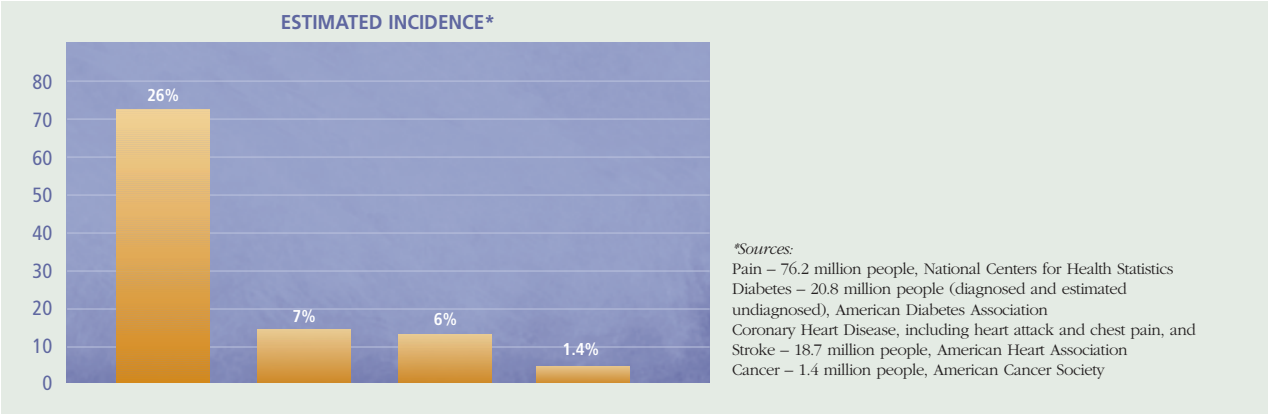
Drugs that have no direct pain-relieving properties may also be prescribed as part of a pain management plan. These include medications to treat insomnia, anxiety, depression and muscle spasms, and can help a great deal in the overall management of pain in some persons.

# Pain Facts & Stats

## PREVALENCE OF PAIN

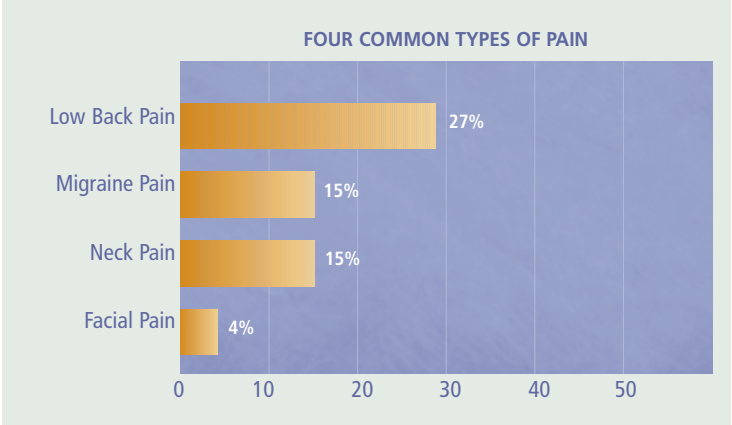
***Pain is a serious and costly public health problem.***

- A hallmark of many chronic conditions, pain affects more Americans than diabetes, heart disease and cancer combined.



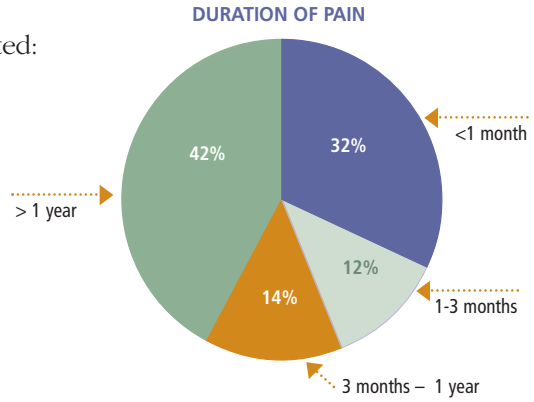

*Sources:*
Pain – 76.2 million people, National Centers for Health Statistics
Diabetes – 20.8 million people (diagnosed and estimated undiagnosed), American Diabetes Association
Coronary Heart Disease, including heart attack and chest pain, and Stroke – 18.7 million people, American Heart Association
Cancer – 1.4 million people, American Cancer Society

- More than one-quarter of Americans (26%) age 20 years and over—or, an estimated 76.5 million Americans—reported that they have had a problem with pain of any sort that persisted for more than 24 hours in duration. This number does not account for acute pain.[1]
- About one-third of people who report pain indicate that their pain is "disabling," defined as both severe and having a high impact on functions of daily life.[2]
- More women (27.1%) than men (24.4%) report that they are in pain.[1]
- Non-Hispanic white adults reported pain more often than adults of other races and ethnicities (27.8% vs. 22.1% Black only or 15.3% Mexican).[1]
- Adults living in families with income less than twice the poverty level reported pain more often than higher income adult.[1]
- When asked about four common types of pain, respondents of a National Institute of Health Statistics survey indicated that low back pain was the most common (27%), followed by severe headache or migraine pain (15%), neck pain (15%) and facial ache or pain (4%).[1]



## DURATION OF PAIN

- Adults 20 years of age and over who report pain said that it lasted:
  - Less than one month – 32%
  - One to three months – 12%
  - Three months to one year – 14%
  - Longer than one year – 42%




## ECONOMIC AND WORKPLACE BURDEN OF PAIN

- The annual cost of chronic pain in the United States, including healthcare expenses, lost income, and lost productivity, is estimated to be $100 billion.[3] However, more recent studies have indicated that costs associated with low back pain alone are an estimated $85.9 billion.[4] The total cost of arthritis—the nation's leading cause of disability—is estimated at $128 billion.[5]
- Undertreated pain drives up the cost of healthcare because it extends lengths of stay in hospitals, increases emergency room visits and results in unplanned clinic visits.
- Pain is the second leading cause of medically related work absenteeism, resulting in more than 50 million lost workdays each year.[6]
- Lost productive time due to headache, arthritis, back pain and other musculoskeletal conditions is estimated to cost $61.2 billion per year.[7]
  - Headache was the most common (5.4%) pain condition resulting in lost productive time. It was followed by back pain (3.2%), arthritis pain (2.0%), and other musculoskeletal pain (2.0%).
  - Most (76%) of the pain-related lost productive time was in the form of reduced performance occurring while the employees were at work, rather than absenteeism.
  - Workers who experienced lost productive time from a pain condition lost an average of 4.6 hours per week.

### MUCH WORK REMAINS

- Currently, less than 2% of the NIH research budget is dedicated to pain.
- More than half of all hospitalized patients experienced pain in the last days of their lives and although therapies are present to alleviate most pain for those dying of cancer, research shows that 50-75% of patients die in moderate to severe pain.[8]

For more statistics and research findings, see our **Topic Briefs** on:

- Special Considerations: Pain in Specific Populations
- Disparities and Pain Management
- Integrative Medicine: Non-Drug Treatment Options for Pain Management
- Chronic Pain and Opioid Treatment

**Be sure to visit the American Pain Foundation at www.painfoundation.org for posted updates and additional Topic Briefs.**

## REFERENCES


1. National Center for Health Statistics. Health, United States, 2006, Special Feature on Pain With Chartbook on Trends in the Health of Americans. Hyattsville, MD. Available at http://www.cdc.gov/nchs/data/hus/hus06.pdf.
2. Portenoy, R, Ugarte C, Fuller I, Haas G. "Population-based Survey of Pain in the United States: Differences Among White, African American, and Hispanic Subjects" *Journal of Pain*, Vol 5, Issue 6, 2004; pp 317-318.
3. National Institutes of Health. NIH Guide: New Directions in Pain Research I. September 4, 1998. Available from http://grants.nih.gov/grants/guide/pa-files/PA-98-102.html.
4. Brook I. Martin, MPH; Richard A. Deyo, MD, MPH; Sohail K. Mirza, MD, MPH; Judith A. Turner, PhD; Bryan A. Comstock, MS; William Hollingworth, PhD; Sean D. Sullivan, PhD. "Expenditures and Health Status Among Adults With Back and Neck Problems." JAMA. 2008;299(6):656-664.
5. Centers for Disease Control and Prevention. "Targeting Arthritis: Improving Quality of Life for More than 46 Million Americans." At a Glance 2008. Retrieved March 6, 2008 from http://www.cdc.gov/nccdphp/publications/aag/arthritis.htm.
6. American Pain Society. "Pain Assessment and Treatment in the Managed Care Environment." January 11, 2000. Available at http://www.ampainsoc.org/advocacy/assess_treat_mce.htm.
7. Stewart WF, Ricci JA, Chee E, Morganstein D, Lipton R. Lost Productive Time and Cost Due to Common Pain Conditions in the US Workforce. *JAMA*. 2003;290:2443-2454.
8. Weiss SC, Emanuel LL, Fairclough DL, Emanuel EJ. Understanding the experience of pain in terminally ill patients. Lancet. 2001; 357:1311-1315.




*"The moral test of a society is how that society treats those who are in the dawn of life...the children; those who are in the twilight of life...the elderly; and those who are in the shadows of life...the sick, the needy and the handicapped."*

*—Hubert Humphrey*

# SPECIAL CONSIDERATIONS: PAIN IN SPECIFIC POPULATIONS

Although pain is a significant problem among all Americans, certain populations are more susceptible to and at greater risk for undertreatment, including children, minorities and those with advanced, life-limiting medical illness. Studies conducted in emergency departments suggest that women receive less attention in response to reports of severe pain than men. As well, active duty military and veterans tend to experience pain differently and present greater challenges to achieving optimal pain relief.

In order to provide the most effective pain care possible and minimize pain-related morbidity, characteristics of vulnerable populations must be taken into consideration when performing pain assessment and implementing treatment plans. Healthcare professionals must also become aware of their own biases and understand that, regardless of demographic or social position, every individual with pain requires evaluation and treatment tailored to his or her specific clinical circumstances.

## Children and Pain

Every child will experience pain at one time or another, whether it's from everyday bumps and bruises, or more chronic conditions such as headaches, gastrointestinal problems or diabetes. In fact, research shows that as many as 40% of children and adolescents complain of pain that occurs at least once weekly, and chronic pain affects at least 15 to 20% of children.[1] And pediatric pain stems from a wide range of chronic conditions:

- Each year, 1.5 million children have surgery, and many receive inadequate pain relief. In 20% of cases, the pain becomes chronic.[2]
- Of children aged 5 to 17 years, 20% suffer headaches.[2]
- More than one-third of children complain of abdominal pain lasting two weeks or longer.[3]
- Juvenille arthritis, which causes joint inflammation and aches, affects nearly 250,000 people under the age of 16 years.[4]
- By 2010, 1 in 1,000 U.S. children will be a survivor of childhood cancer and may have to deal with late and long-term effects of treatment (e.g., chronic fatigue and pain syndromes, nerve damage).[2]
- Recent evidence reveals reduced pain sensitivity is a common feature of children with autism and Asperger's syndrome.[2]
- Musculoskeletal pain can result from "growing pains," a normal occurrence in about 25 to 40% of children.[5]

### COMMON CAUSES OF PAIN IN CHILDREN

- Scrapes and bruises
- Needlestick pain from immunizations (most children receive up to 24 immunizations by their 2nd birthday)
- Sports injuries (e.g., sprains, concussion, fractures)
- Chronic illnesses (e.g., sickle cell disease, Type I diabetes)
- Headaches
- Abdominal pain (e.g., ulcerative colitis)

According to the American Medical Association, children and infants are at increased risk of inadequate pain management, with age-related factors playing a major role. Physical and psychological changes that occur during childhood development can make understanding and managing pain in children significantly more complicated than treating pain in adults.

Many things affect the way a child experiences, communicates and responds to pain, including:

- Their age
- Their beliefs and understanding of what is causing the pain
- Their ability to cope
- Their activity and anxiety levels
- Previous experiences with pain and how they learned to respond
- Support from parents and siblings
  - Preliminary data suggest that a mother's anxiety may be transmitted more strongly to her daughters than her sons, resulting in increased anxiety and pain in girls, but not boys.[6]

If pain is not addressed and treated early on, it can greatly impact a child's quality of life by interfering with mood, sleep, appetite, school attendance, academic performance, and participation in sports and other extracurricular activities. Further, if unrelieved, childhood pain can enhance a child's vulnerability to pain later in life.[7] It is essential that healthcare providers begin to approach pediatric pain so that appropriate strategies can be devised to target and reduce

children's distress and pain-related disability.

Unaddressed pain can also result in significant financial stress for families who not only have to cover healthcare expenses, but who may also have to miss work to care for a sick child.[8]

Inadequate prevention and relief of pediatric pain are still widespread. Many obstacles exist to providing appropriate pain care to children and adolescents:[10]

- Beliefs and attitudes about the experience of pediatric pain.
- General lack of understanding about the best course of action for treating children in pain.
- Belief that pain should be treated less aggressively in children than adults.
- Pediatric pain management research has not been effectively translated into routine clinical practice.
- Pain in children with disabilities or other special health care needs may be more difficult to assess.

## MYTHS AND TRUTHS ABOUT PAIN IN CHILDREN

| | |
|---|---|
| **MYTH:** | Children who are playing or sleeping must not be in pain. |
| **TRUTH:** | Children cope with pain by distracting themselves, often through play. Sleep may also be a coping mechanism, and/or because they are exhausted. |
| **MYTH:** | Young infants do not feel pain because their nervous systems are immature and unable to perceive and experience pain the way adults do. |
| **TRUTH:** | Decades ago it was believed that a newborn couldn't feel pain, and surgery was routinely performed on infants without anesthetic. Today, we know that the central nervous system of a 26-week-old fetus has the capability of experiencing pain. There is strong evidence that children experience increasing anxiety and perception of pain with multiple procedures or painful stimuli.[8] |
| **MYTH:** | Children can easily become addicted to pain medications. |
| **TRUTH:** | Less than 1% of children treated with opioids become addicted.[9] |
| **MYTH:** | Children cannot effectively communicate their pain; it is difficult to know when they have pain. |
| **TRUTH:** | Children don't communicate, respond to, or feel pain the same way adults do, so it's difficult for health professionals and parents to understand what they are experiencing. But, it is very real and not something they easily forget about. There are many tools available to assess pain in children. Adults need to recognize how children of different ages express pain in both behaviors and words. |
| **MYTH:** | Children will tell adults when they are having pain. |
| **TRUTH:** | Children may not have the words to express pain (e.g., hurt, "ouch") or know to point to where it hurts. They may also be afraid of the consequences (e.g., extra visits to the pediatrician, shots, medicine). |

## Potential barriers to the effective treatment of pain in children[10]

- The myth that children, especially infants, do not feel pain the way adults do;
- Lack of routine assessment for the presence of pain in children;
- The idea that treating pediatric pain takes too much time and effort;
- Fears of adverse effects of analgesic medications, including respiratory depression and addiction;
- Differing personal values and beliefs of healthcare professionals about the meaning and value of pain in the development of the child (e.g., the belief that pain builds character).

## WEB RESOURCES

**American Pain Society**
www.ampainsoc.org

**National Children's Pain Center**
www.pediatricpain.org/ncpc.php

**Pediatric Pain Sourcebook**
http://painsourcebook.ca/

**UCLA Pediatric Pain Program**
www.mattel.ucla.edu/pedspain/home.php

**American Academy of Pediatrics**
http://www.aap.org

# Gender and Pain

Although it has long been thought that women and men have similar pain experiences, recent research reveals significant differences in the way male and female brains process pain,[1] as well as in women's expression of pain and their responsiveness to analgesics and pain stimulus.[2,3]

Historically, women have been categorized as being emotional and overly sensitive; often influencing the way physicians assessed and managed their pain.[4] Even though research now shows that chronic pain conditions are generally more prevalent among women, they continue to be treated less aggressively for their pain than men.[5,6] And while women are more likely than men to seek treatment for their pain, they are less likely to receive it.[7]

Women report pain more often than men do and in more body regions, and they also tend to have more severe, recurrent and persistent pain, as well as a reduced pain threshold when compared with men.[3] However, despite their increased pain burden, women reportedly cope with pain better than men, possibly due to the fact that they experience pain more often throughout the course of their lives (e.g., menstruation, pregnancy and child birth, and other health issues specific to women).[3]

Female hormones are also likely to play a role in pain perception. Some pain conditions like migraine tend to vary with a woman's menstrual cycle, and many of the observed gender differences in pain appear to diminish following the reproductive years.[8]

### Hormones May Influence Pain Experience

- Estrogen administration in women and in men can increase the incidence of chronic pain conditions.[9,10]
- Variations in women's estrogen levels, like those that occur during the menstrual cycle or during pregnancy, may regulate the brain's natural ability to suppress pain.[11]
- Some pain conditions such as migraine and fibromyalgia tend to fluctuate with a woman's menstrual cycle.
- Observed gender differences in pain appear to diminish following menopause.

Additionally, cultural conditioning may impact the expression of pain among women and men. As children, girls are more likely to be permitted to express pain and show emotion than boys, and attitudes about the social acceptability of gender and pain often carry into adulthood.[3]

### Potential Sources of Gender Differences in Pain

*Biological factors including:*

- sex hormones
- genetics
- anatomical differences

*Psychosocial influences including:*

- emotion (e.g., anxiety, depression)
- coping strategies
- gender roles
- cultural conditioning
- health behaviors
- use of healthcare services

As advances in brain imaging technology provide further insights into gender variations in the experience of pain, it is becoming evident that different pain experiences among men and women will call for different approaches to pain management.

Ongoing research is essential to achieve:

- A better understanding of the biological and psychosocial factors that influence gender differences in pain
- A greater appreciation of the different health needs of men and women
- More effective and targeted pain treatments for women

### PAIN DISORDERS WITH HIGHER PREVALENCE IN WOMEN

- Migraine
- Irritable bowel syndrome
- Fibromyalgia
- Chronic pelvic pain
- Interstitial cystitis
- Temporomandibular joint disorder (TMJ)
- Breast pain (mastalgia)
- Autoimmune disorders (e.g. Lupus and Chronic Fatigue Syndrome)
- Rheumatoid arthritis
- Osteoarthritis

### WEB RESOURCES

**International Association for the Study of Pain: Real Women, Real Pain**
www.iasp-pain.org

**National Institutes of Health: Gender & Pain**
http://painconsortium.nih.gov/genderandpain/summary.htm

**National Women's Health Resource Center**
www.healthywomen.org/

**Society for Neuroscience: Gender & Pain**
www.sfn.org/index.cfm?pagename=brainBriefings_gender_and_pain

# Older Adults and Pain

As we age, pain becomes a more common problem due to the high prevalence of chronic and progressive pain-producing conditions associated with aging. It is estimated that up to 50% of older persons living in the community have pain that interferes with normal function, and 59 to 80% of nursing home residents experience persistent pain.[1,2] Alarmingly, being older than 70 is the leading risk factor for inadequate pain management.[3]

Diagnosing and treating pain in older adults can be challenging. Those 65 and older often present with multiple medical and nutritional problems, take multiple medications and have many potential sources of pain. Older persons with dementia or communication problems are at even greater risk of undertreatment of pain due to difficulties communicating their pain.[4] Use of certain medications in older patients becomes problematic because of physiological changes.[5]

The most common cause of persistent pain in older adults is musculoskeletal in nature, typically from osteoarthritis or other bone, joint and spine disorders. According to the Arthritis Foundation, arthritis affects up to 80% of older adults, who report being fearful of recurring pain and disability. But the predilection for painful conditions does not mean that older adults need to live with uncontrolled pain. Quite the opposite; older patients can be effectively treated, and in so doing, pain-related morbidity—and even premature mortality—can and should be obviated.

## COMMON PAIN CONDITIONS IN OLDER ADULTS

- Arthritis
- Lower back and neck pain; vertebral compression fractures from osteoporosis
- Abdominal pain (e.g., gallstones, bowel obstruction, peptic ulcer disease, abdominal aortic aneurysm)
- Cancer-related pain (symptom of disease or effect of nerve damage from treatments)
- Neuropathic pain due to diabetes, herpes zoster ("shingles"), kidney disease or other medical problems
- Muscle cramps, restless leg pain, itchy skin and sores due to circulatory problems or vitamin D deficiency
- Fibromyalgia
- Complex Regional Pain Syndrome (CRPS), which develops after an illness or injury and often affects the leg, arm, foot or hand
- Injuries, especially from falls



## WEB RESOURCES

**Handbook of Pain Relief in Older Adults — An Evidence-Based Approach**
By Gloth III, F. Michael
http://www.humanapress.com/Product.pasp?txtCatalog=HumanaBooks&txtProductID=1-58829-217-7

**American Medical Association**
**Assessing and Treating Pain in Older Adults**
http://www.ama-cmeonline.com/pain_mgmt/module05/index.htm

**American Geriatrics Society Foundation**
**The Management of Persistent Pain: Resources for Older Adults and Caregivers**
http://www.healthinaging.org/public_education/pain

# End-of-Life and Pain

Pain control is one of the most challenging aspects of end-of-life care.[1] Terminal illness is often accompanied by severe pain, and a significant number of patients suffer needlessly at the end-of-life. While the goal of end-of-life care should be making patients more comfortable, the health care system has been designed to take a curative approach to disease, rather than focusing on symptom relief.[2] Hospital research reveals that healthcare providers continue to inadequately treat pain, and tend to under-medicate terminal pain.

Patients at end-of-life may have their pain undertreated for variety of reasons, including a lack of knowledgeable and experienced physicians and myths about addiction to pain medication, leading unnecessarily to patient and family suffering.[3]

Despite advances in research on end-of-life pain treatment, physicians remain influenced by social and legal concerns, as well as misconceptions about medications including addiction, overdose, lasting side effects and diminished physical capacity.[5] Patients and their families may also hesitate to begin using pain medications as they often associate such treatment with imminent death, thereby allowing patient suffering to worsen and continue.[4]

However, thorough and ongoing pain assessment, paired with well-designed and aggressive medication plans, as well as counseling for patients and their families can have a significant impact on pain relief and side effects among dying patients.[4,5]

### IN DYING PATIENTS, PAIN MAY BE EXACERBATED BY MANY OTHER SYMPTOMS INCLUDING:

- Dry mouth
- Nausea
- Water retention and swelling
- Lack of appetite
- Shortness of breath
- Mental distress and anxiety caused by fear or denial of impending death

Effective pain management at the end-of-life requires addressing the total pain experience, including physical causes, as well as interpersonal and spiritual pain.[3,4]

Pain associated with terminal illness often requires special treatment that can be best provided by hospice and palliative care programs available in many medical facilities. Hospice focuses on relieving symptoms and supporting patients who are nearing the end of their life, while palliative care is designed to provide comfort and pain relief at any time during a person's illness.[7] The goal of both programs is to alleviate suffering and ultimately assist patients in achieving a pain-free and dignified death.

*"When someone is dying, time is a luxury and wait-and-see is not an option. What matters most in the final days is that patients are free of crippling pain and unbearable suffering so that they can finish their lives in ways that bring comfort, peace, and completion. Concerns about lasting side effects or diminished physical capacity from months of using a drug become secondary to making a patient comfortable. No one has to die in pain."*

*— Dr. Scott Fishman*

*"Suicidal wishes in patients with advanced disease are closely linked to unrelieved pain and to mood alterations such as depression and anxiety, which like pain, frequently respond to clinician treatment if the clinician identifies and addresses them."*[2,6]

### Essential Components of End-of-Life Care[8]

- Continual assessment and management of pain and other physical symptoms
- Assessment and management of psychological and spiritual needs
- Helping patients identify personal goals for pain treatment and end-of-life care
- Assessment of the patient's support system

### WEB RESOURCES

**American Academy of Family Physicians: Challenges in Pain Management at the End of Life**
www.aafp.org/afp/20011001/1227.html

**American Pain Society: Treatment of Pain at the End of Life**
www.ampainsoc.org/advocacy/treatment.htm

**Discovery Health Center: End of Life Q&A with Dr. Scott Fishman**
http://health.discovery.com/centers/pain/endoflife/endoflife.html

**National Hospice and Palliative Care Organization**
www.nhpco.org/i4a/pages/index.cfm?pageid=3254

# Military/Veterans and Pain[1]

Pain is a major issue among military personnel and veterans, who are at heightened risk for injury and combat wounds. Although today's body armor and rapid evacuation to medical care is saving lives, there are more maimed and shattered limbs than ever before, with instances of amputation double previous rates. Hundreds of thousands of returning veterans will seek medical care and claim disability compensation for a wide variety of injuries and health problems they endured during their tours of duty. It is estimated that the U.S. will be paying the cost of related medical care and disability claims for the next 40 years.

Veterans are more likely to experience psychological distress and other medical conditions, including post traumatic stress disorder, depression, amputations, traumatic brain injuries, substance abuse and other injuries, which further complicate effective pain management.

## COMMON PAIN CONDITIONS AMONG MILITARY MEMBERS

**Post traumatic stress disorder (PTSD)** commonly affects soldiers returning from war, and is triggered by exposure to a situation or event that is or could be perceived as highly threatening to a person's life or those around him/her. PTSD may not emerge for years after the initial trauma. Chronic pain symptoms and PTSD frequently co-occur and may intensify an individual's experience of both conditions. Together, they result in fear, avoidance behaviors, anxiety and feelings of isolation.

**Amputations** have long been a tragic, unavoidable consequence of combat injury— "one of the most visible and enduring reminders of the cost of war," according to the Amputee Coalition of America. While there have been major advances in medicine, prosthetics and technologies that allow amputees to lead more independent lives, most of these patients continue to need specialized long-term or lifelong support. Managing wound, post-operative, phantom and stump pain is important to reduce suffering and improve quality of life.

A **traumatic brain injury** (TBI) is a blow or jolt to the head or a penetrating head injury that disrupts the function of the brain and is a major cause of life long disability and death. Managing pain in veterans with TBIs may be complicated by memory lapses affecting medication management, difficulty organizing and following complicated and sometimes even simple pain management regimens, and difficulty learning new coping skills. Rehabilitation should incorporate efforts to relieve associated pain.

Veterans have significantly worse pain than the general public, and while military medical care is among the best in the world, there are still long-term problems and challenges with managing disability and chronic pain.

Military culture may also present a significant barrier to appropriate patient care. The persisting stigma around pain and pain treatment is particularly pronounced in the military, and pain is often perceived as a sign of weakness leading many individuals to choose to suffer in silence. Seeking mental health care for PTSD and depression, which so often accompany pain is important; pain is best managed when depression and PTSD are treated simultaneously.

A recent analysis found that the Veterans Health Administration (VHA) is already overwhelmed by the sheer number of returning veterans and the seriousness of their health care needs. Without increased staffing and funding for veterans medical care, it will not be able to provide quality care in a timely fashion.

# Military/Veterans and Pain[1]

**Barriers to optimal pain management among veterans and military personnel may include fears about:**

- No longer being physically capable of fulfilling their duties
- Being discharged and no longer having a sense of purpose
- Letting down or losing the respect of their peers
- Becoming addicted to pain medications
- Experiencing personality changes or problems with sexual relations due to pain medications
- Losing their benefits/pension if they acknowledge a pain condition

## THE UNITED STATES CONGRESS HAS STATED THE FOLLOWING:

(1) Acute and chronic pain are prevalent conditions among active duty and retired military personnel.

(2) Characteristics of modern warfare, including the use of improvised explosive devices, produce substantial numbers of battlefield casualties with significant damage to both the central and peripheral nervous systems.

(3) The successes of military health care both on and off the battlefield result in high survival rates of severely injured military personnel who will be afflicted with significant pain disorders on either an acute or chronic basis.

(4) Failure to treat acute pain promptly and appropriately at the time of injury, during initial medical and surgical care, and at the time of transition to community-based care, contributes to the development of long-term chronic pain syndromes, in some cases accompanied by long-term mental health and substance abuse disorders.

(5) Pain is a leading cause of short- and long-term disability among military personnel.

(6) The military health care systems have implemented important pain care programs at some facilities and in some areas, but comprehensive pain care is not consistently provided on a uniform basis throughout the systems to all patients in need of such care.

(7) Inconsistent and ineffective pain care leads to pain-related impairments, occupational disability, and medical and mental complications with long-term costs for the military health and disability systems, and for society at large.

(8) Research, diagnosis, treatment, and management of acute and chronic pain in the active duty and retired military populations constitute health care priorities of the United States.

From the Military Pain Care Act of 2008

*The U.S. Veterans Health Administration is instructing physicians and nurses who treat veterans to regard pain as a "fifth vital sign," to be routinely assessed along with blood pressure, pulse, temperature and respiration.*

## WEB RESOURCES

**American Pain Foundation: Military/Veterans and Pain**
www.painfoundation.org/page.asp?file=Veterans/Intro.htm

**Amputee Coalition of America**
www.amputee-coalition.org

**Defense and Veterans Brain Injury Center**
www.dvbic.org

**Disabled American Veterans (DAV)**
www.dav.org

**Military Pain Care Act of 2008**
http://www.govtrack.us/congress/bill.xpd?bill=h110-5465

**U.S. Department of Veterans Affairs**
www.va.gov



## HOT TOPICS

### Children & Pain: HOT TOPICS

- Maternal anxiety influencing daughters' experience of pain
- Some neonatologists still do not treat pain in pre-term low birth weight babies because they "won't remember it"
- Investigations into "chronic daily headaches" in children
- Unraveling pediatric pain conditions and their impact into adulthood (e.g., whether Complex Regional Pain Syndrome in children leads to adult CRPS, whether irritable bowel syndrome in adolescents is this the same as IBS in adults)
- Complementary and alternative medicine: how and what is safe to use in children with chronic pain?
- Factors leading to pain-related disability in children (e.g., missing school, not sleeping, avoiding physical and social activities, not eating)

### Gender & Pain: HOT TOPICS

- Prevalent pain conditions in women (e.g., fibromyalgia, chronic pelvic pain)
- Interface of hormones and the pain experience
- Brain imaging, uncovering routes of pain transmission and tolerance
- Differential effects of medicines across genders
- Impact of chronic pain on sexuality and self-image

## HOT TOPICS

### Older Adults and End-of-Life Care & Pain: HOT TOPICS

- False belief that pain is an inevitable part of aging
- Vitamin deficiencies and musculoskeletal pain
- Limited consumer awareness of the options that exist other than traditional "acute care" approaches (e.g., doctor's office visits, ER visits, hospitalizations)
- Insufficient numbers of adequately trained and skilled healthcare professionals to manage the myriad issues confronting patients/families with advanced medical illness; limited number of providers with specialty in geriatrics
- Variability in delivery of hospice and palliative care services across the country
- Lack of clinical research data on pain care among elders

### Military/Veterans & Pain: HOT TOPICS

- President Bush recently signing the Military and Veterans Pain Care Acts into law
- Emerging Options: Interdisciplinary approaches to pain care
- Acupuncture now being incorporated into treatment plans at Walter Reed Army Medical Center
- Competitive athletics as a form of therapy
- New Veteran centers open for drop-in counseling



# References


### Children and Pain

1. Goodman, JE, McGrath, PJ. (1991). The epidemiology of pain in children and adolescents: A review. *Pain*, 46:247–264.
2. Zeltzer LK, Schlank CB. (2005) Conquering Your Child's Chronic Pain: A Pediatrician's Guide for Reclaiming a Normal Childhood. New York, NY: HarperCollins.
3. *Chronic Abdominal Pain in Childhood: Diagnosis and Management*. Retrieved October 12, 2008 from American Academy of Family Physicians. Web site: http://www.aafp.org/afp/990401ap/1823.html
4. Juvenile Arthritis. Retrieved October 13, 2008 from American Academy of Orthopaedic Surgeons. Web site:http://orthoinfo.aaos.org/topic.cfm?topic=A00075
5. Nemours Foundation. Growing Pains Fact Sheet. Retrieved October 13, 2008 from http://kidshealth.org/parent/general/aches/growing_pains.html.
6. Tsao JCI, Lu Q, Kim SC, Zeltzer LK. (2006). Relationships among anxious sympto-matology, anxiety sensitivity and laboratory pain responsivity in children. *Cognitive Behaviour Therapy*, 35:207-215.
7. Zeltzer LK, Anderson CTM, Schechter NL. (1990) Pediatric Pain: Current status and new directions. *Current Problems in Pediatrics*, 20(8):415-486.
8. Anand KJS. (2006). Fetal pain. *Pain-Clinical Updates,* 14:1-4.
9. Foley KM. (1996) Controlling the pain of cancer. *Sci Am,* 275(3): 164-165.
10. *The Assessment and Management of Acute Pain in Infants, Children, and Adolescents: A Position Statement from the American Academy of Pediatrics Committee on Psychosocial Aspects of Child and Family Health and American Pain Society Task Force on Pain in Infants, Children, and Adolescents.* Retrieved October 12, 2008 from American Pain Society. Web site: http://www.ampainsoc.org/advocacy/pediatric2.htm

### Gender and Pain

1. Paulson PM, Minoshima S, Morrow TJ, Casey KL. (1998) Gender differences in pain perception and patterns of cerebral activation during noxious heat stimulation in humans. *Pain,* 76:2239.
2. Fillingim RB, Maixner W. (1995) Gender differences in the response to noxious stimuli. *Pain Forum*,4:209-21.
3. Berkley KJ. (1997) Sex differences in pain. *Behav Brain Sci*, 20:371-80.
4. Fishbain DA, Goldberg M, Meagher BR, et al. (1986) Male and female chronic pain patients categorized by DSM-III psychiatric diagnostic criteria. *Pain*, 26(2):181-97.
5. LeResche L. (1999) Gender considerations in the epidemiology of chronic pain. Chapter in I. Crombie (ed.), *Epidemiology of Pain.* IASP Press, Seattle.
6. Green CR. "Pain, Disparities, and Practice: Opportunities to Improve Health Policy and Healthcare Quality," Presented September 9, 2008 at the American Academy of Pain Management annual meeting.
7. Hoffmann DE, Tarzian AJ. (2001) The Girl Who Cried Pain: A Bias Against Women in the Treatment of Pain. *J Law Med Ethics*, 29:13-27.
8. Gagliese L, Fillingim RB. (2003) Age and sex interactions in the experience of pain. XX vs. XY: *The International Journal of Sex Differences in the Study of Health, Disease and Aging* 1,124131.
9. Dao TT, LeResche L. (2000) Gender differences in pain. *J Orofac Pain*, 14: 169184.
10. Aloisi AM, Bachiocco V, Costantino A, Stefani R, Ceccarelli I, et al. (2007) Crosssex hormone administration changes pain in transsexual women and men. *Pain.* [Epub ahead of print]
11. Zubieta JK. "Systems Integration and Neuroimaging in the Neurobiology of Pain" Presented February 18, 2003 at the American Association for the Advancement of Science annual meeting.

### Older Adults and Pain

1. Ferrell BA. (1995). Pain evaluation and management in the nursing home. *Ann Intern Med*, 123:681-687.
2. Helme RD, Gibson SJ. Pain in Older People. In: Crombie IK, ed. *Epidemiology of Pain.* Seattle: IASP Press; 1999:103-112.
3. Cleeland, CS, Gonin R, Hatfield AK, Edmonson JH, et al. (1994) Pain and Its Treatment in Outpatients with Metastatic Cancer. *N Engl J Med*, 330(9):592-596.
4. Parmalee PA, Smith B, Katz IR. (1993). Pain complaints and cognitive status among elderly institution residents. *J Am Geriatr Soc*, 41:517-22.
5. Schmucker DL. (2001) Liver Function and Phase I Drug Metabolism in the Elderly: A Paradox. *Drugs Aging,* 18:837-851.

### End-of-Life and Pain

1. *Challenges in Pain Management at the End of Life.* Retrieved October 13, 2008 from American Academy of Family Physicians. Web site: http://www.aafp.org/afp/20011001/1227.html
2. *Treatment of Pain at the End of Life: A Position Statement from the American Pain Society.* Retrieved October 12, 2008 from American Pain Society. Web site: http://www.ampainsoc.org/advocacy/treatment.htm
3. *Pain Management at the End of Life: A Physician's Self-Study Packet.* Retrieved October 12, 2008 from Maine Hospice Council. Web Site: http://www.mainehospicecouncil.org/Pain%20Management%20web%20version.pdf
4. Leleszi JP, Lewandowski JG. (2005) Pain Management in End-of-Life Care. *J Am Osteopath Assoc*, 105(3_suppl), 6S-11.
5. *Pain Mangement: Q&A with Dr. Scott Fishman.* Retrieved October 12, 2008 from Discovery Health. Web site: http://health.discovery.com/centers/pain/endoflife/endoflife.html
6. Foley KM. (1995) A review of ethical and legal aspects of terminating medical care. *Amer J Med*, 84:291-301.
7. *End-of-Life Care.* Retrieved October 12, 2008 from National Hospice and Palliative Care Organization. Web site: http://www.nhpco.org/i4a/pages/index.cfm?pageid=3254
8. *End-of-Life Care Eases Pain and Prepares Patient for Death.* Retrieved October 12, 2008 from Medical College of Wisconsin. Web site: http://healthlink.mcw.edu/article/1001710698.html

### Military/Veterans and Pain

1. *Military/Veterans and Pain* Fact Sheet. Retrieved October 13, 3008 from the American Pain Foundation. Web site: www.painfoundation.com.

# PAIN MANAGEMENT & DISPARITIES

The undertreatment of pain in America is a growing public health crisis, especially among underserved populations, including ethnic minorities, women, the elderly and those who are socioeconomically disadvantaged. Despite an overall improvement in health for most Americans, certain segments of the population continue to experience poor health status.[1] There is compelling evidence that minorities are less likely to have access to routine, coordinated medical care or health insurance than whites. They are also more likely to receive inappropriate or insufficient care, resulting in poorer health outcomes.

As the U.S. population becomes increasingly diverse, there is an urgent need to eliminate health disparities. Patients have a right to appropriate assessment and treatment of their pain without regard to race, ethnicity or other factors.

*"Of all the forms of inequality, injustice in health is the most shocking and the most inhumane."*
*–Martin Luther King, Jr.*

## Health Disparities Defined

According to the National Institutes of Health, health disparities are defined as "differences in the incidence, prevalence, mortality, and burden of diseases and other adverse health conditions that exist among specific population groups in the United States."

Disparities in health care are complex and multifaceted resulting from:

- Patient/personal factors (e.g., low socioeconomic status, communication barriers)
- Healthcare provider factors (e.g., bias, cultural insensitivity)
- Systematic/health system factors (e.g., health insurance status, access to care)



### Snapshot of U.S. Population, An Older and More Diverse Nation

According to projections by the U.S. Census Bureau:

- Minorities now comprise roughly one-third of the U.S. population.
- By 2023, more than half of all children will be from minority groups.
- Minorities are expected to become the majority in 2042.
- In 2050, the nation is projected to be 54% minority.
- The Latino population, already the nation's largest minority group, will triple in size between 2005 and 2050.
- The nation's elderly population will more than double in size from 2005 through 2050 as the baby boom generation enters traditional retirement years.

Source: U.S. Census Bureau, 2008, http://www.census.gov/PressRelease/www/releases/archives/population/012496.html; Pew Hispanic Center.

## Disparities in Pain Care

Pain is widely recognized as an undertreated health problem in the general population.[2] However, a growing body of research reveals even more extensive gaps in pain assessment and treatment among racial and ethnic populations, with minorities receiving less care for pain than non-Hispanic whites.[3,4,5,6,]

Differences in pain care occur across all types of pain (e.g., acute, chronic, cancer-related) and medical settings (e.g., emergency departments and primary care).[3,4,5,6,7] Even when income, insurance status and access to health care are accounted for, minorities are still less likely than whites to receive necessary pain treatments.[3,4,8]

***Minorities are less likely to:***

- Have access to pain management services and treatments
- Have their pain documented by healthcare providers
- Receive pain medications

***And more likely to:***

- Use the emergency department for pain care, but less likely to receive adequate care
- Experience greater severity of pain
- Experience and report physical disability
- Experience poorer health and quality of life related to pain

There are clear variations in the way pain is assessed and managed among all minority populations. Significant gaps exist in the provision of effective quality pain care due to the lack of research and medical training focused on pain care disparities.[3,4,9]

Research also shows gender differences in the experience and treatment of pain. Most chronic pain conditions are more prevalent among women; however, women's pain complaints tend to be poorly assessed and undertreated.[3]

Additionally, gender differences have been identified in patient responsiveness to analgesics and pain stimuli. While estrogen and progesterone play a role in how pain signals are received in men and women, psychology and culture may also account for some of the difference. For example, children may learn how to respond to pain later in life depending on how their pain complaints were treated in their formative years (e.g., receiving comfort and validation versus being encouraged to tough it out or dismiss the pain).[14] For more information, see the *Special Considerations: Pain in Specific Populations* Topic Brief.

In response to the overwhelming discrepancies in pain treatment among minority groups, the Joint Commission issued a statement recognizing the rights of all patients to receive appropriate assessment and management of pain, and the World Health Organization has declared that pain relief is a human right.

### RESEARCH ON DISPARITIES IN PAIN CARE HAVE SHOWN:

- Blacks were less likely than whites to receive pain medication and had a 66% greater risk of receiving no pain medication at all.[5,6,7,9]
- Hispanics were twice as likely as non-Hispanic whites to receive no pain medication in the emergency department (55% of Hispanics received no pain medication vs. 26% of non-Hispanic whites).[7,10]
- Minority patients were less likely to have pain recorded relative to whites, which is critical to providing quality patient care.[11]
- Only 25% of pharmacies in predominantly nonwhite neighborhoods had opioid supplies that were sufficient to treat patients in severe pain, as compared with 72% of pharmacies in white neighborhoods.[12]
- In a study of minority outpatients with recurrent or metastatic cancer, 65% did not receive guideline-recommended analgesic prescriptions compared with 50% of nonminority patients ($P < 0.001$). Hispanic patients in particular reported less pain relief and had less adequate analgesia.[13]

### Patient and provider factors drive pain disparities

Multiple factors contribute to racial and ethnic disparities in pain care, including beliefs about pain, preconceived bias and cultural insensitivity and poor patient-provider communication.

Positive physician-patient interaction and communication is critical in accurate pain assessment.[2] Some research has shown that patients take a more active role in their own pain treatment when their healthcare providers are of similar ethnic backgrounds.[3,4]

*"Pain is a complex, subjective response with several quantifiable features, including intensity, time course, quality, impact, and personal meaning. The reporting of pain is a social transaction between caregiver and patient."*[15]

*Patient sources of racial and ethnic disparities:*[3]

- Low socioeconomic status
- Patients' attitudes or beliefs regarding pain and patient-level decision making and preferences
  - Stoicism and the belief that pain is an inevitable part of disease
- Minority patients more likely to:
  - Refuse recommended pain therapies
  - Poorly adhere to treatment regimens
  - Delay seeking medical care
- Mistrust of physicians or previous negative experiences with health care system
- Limited health literacy
- Language barriers that hinder communication with providers

*Physician sources of racial and ethnic disparities:*[3]

- Perceptions of race and ethnicity
- Racism or bias
- Poor cross cultural communication skills/cultural insensitivity
- Underrepresentation of physicians from racially/ethnically diverse backgrounds/lack of cultural sensitivity

## HOT TOPICS

### Disparities & Pain: HOT TOPICS

- Aging and increasingly diverse U.S. population could lead to greater disease burden if pain remains untreated
- Undertreatment of minorities in emergency departments
- Minority pain complaints receive less attention than others
- Impact of pain on productivity and quality of life among minority patients
- Pain relief as a human right

**Minorities lack access to effective pain care**

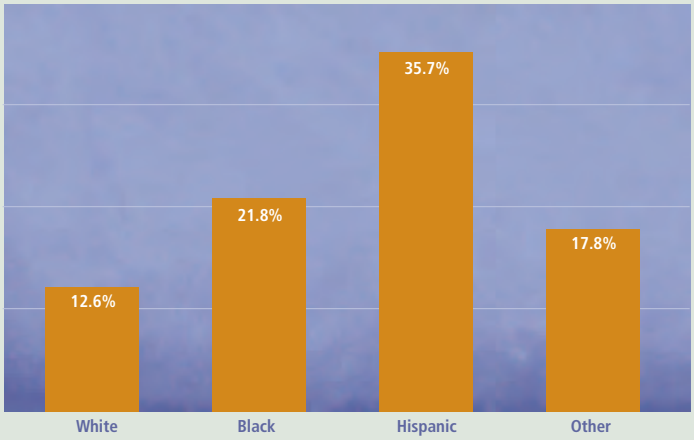
Limited access to pain care services is a key contributor to poorer pain treatment among minorities.

- Overall, minorities tend to be financially poorer than non-Hispanic whites.
- Socioeconomic factors can impede access to health insurance and primary health care services, and minorities are less likely to have access to pain treatment services than the general population.[3,4,7,16,17]
- Racial and ethnic minorities are at increased risk of having their pain complaints ignored by healthcare providers, thereby limiting their options for accessing appropriate pain treatment.[3,4,6,7]

According to the Robert Wood Johnson Foundation, 46 million Americans, including 9 million children, are living without health care coverage. More than eight out of 10 are from working families. The consequences of being uninsured are widely recognized and include: lack of access to health care, poor quality care, lost economic productivity, as well as financial burdens on individuals and society overall. As the minority population in the U.S. continues to grow, it becomes increasingly important to address the numbers of uninsured and underinsured among racial and ethnic groups.

Barriers also exist in patient access to pain medications. Research shows that physicians may be less likely to prescribe pain medications for minority populations [6,7,16,18] and pharmacies in neighborhoods with large minority populations often do not carry opioid medications.[3,4,12]

**PERCENTAGE UNINSURED AMONG THE NONELDERLY POPULATION BY RACE AND ETHNIC ORIGIN, 2006**




*Sources:* Employee Benefit Research Institute estimates from the March Current Population Survey, 2007 Supplement. Cover the Uninsured, www.covertheuninsured.org.

*"Inequities in access can contribute to and exacerbate existing disparities in health and quality of life, creating barriers to a strong and productive life."*

—The Commonwealth Fund

### More extensive research needed to close disparities gap

While national attention has become increasingly focused on health disparities, less attention has been given specifically to inequities in pain care.[19,20] However, the growing interest in health disparities in general provides pain treatment providers, researchers and advocates with an opportunity to raise awareness about disparities in pain management and the need for additional pain disparities research. Currently, the social impact of pain on patients, their families and communities is largely absent in most federal research plans.[3,4]

### Additional studies and a comprehensive pain research agenda are needed to:

- Understand the role of stereotypes and bias in doctor-patient interactions
- Improve training for healthcare providers
- Plan educational interventions for patients
- Understand the differences in patient behaviors that may contribute to pain care disparities
- Develop culturally sensitive pain assessment tools
- Raise consciousness about disparities in pain management and barriers to effective healthcare overall

## WEB RESOURCES

**CDC Office of Minority Health and Health Disparities**
http://www.cdc.gov/omhd/

**Cover the Uninsured: a Project of the Robert Wood Johnson Foundation**
http://covertheuninsured.org/

**American Pain Society: Racial and Ethnic Identifiers in Pain Management: The Importance to Research, Clinical Practice, and Public Health Policy**
http://www.ampainsoc.org/advocacy/ethnoracial.htm

**Agency for Healthcare Research and Quality: Addressing Racial and Ethnic Disparities in Health Care**
http://www.ahrq.gov/research/disparit.htm
http://www.ahrq.gov/qual/nhdr03/nhdrsum03.htm

## REFERENCES


1. *About Minority Health.* Retrieved October 12, 2008 from CDC Office of Minority Health. Web site: http://www.cdc.gov/omhd/AMH/AMH.htm
2. Bonham VL. (2001). Race, Ethnicity, and Pain Treatment: Striving to Understand the Causes and Solutions to the Disparities in Pain Treatment. *Journal of Law, Medicine & Ethics*, 129:52.
3. Green CR. (2003). The Unequal Burden of Pain: Confronting Racial and Ethnic Disparities in Pain. *Pain Medicine,* 4(3):277-294.
4. Green C. (2006). Disparities in Pain: Ethical Issues. *Pain Medicine*, 7(6):530-533.
5. Todd KH, Ducharme J, Choiniere M, Crandall CS, Fosnocht D, Homel P, et al. (2007). Pain in the Emergency Department: Results of the Pain and Emergency Medicine Initiative (PEMI) Multicenter Study. *The Journal of Pain*, 8(6):460-466.
6. Todd KH, et al. (1994) The effect of ethnicity on physician estimates of pain severity in patients with isolated extremity trauma. *JAMA*, 271(12):925-28.
7. Todd, Samaroo, Hoffman, et al. (1993) Ethnicity as a risk factor for inadequate emergency department analgesia. *JAMA*, 269(12):1537-39.
8. Paulson MR, Dekker AH, Aguilar-Gaxiola S. (2007). Eliminating Disparities in Pain Management. *J Am Osteopath Assoc*, 107(suppl_5), ES17-20.
9. Freeman HP, Payne R. (2000) Racial injustices in health care. *New Engl J Med*, 342(14):1045-1047.
10. Todd KH, Deaton C, D'Adamo AP, Goe L. (2000) Ethnicity and analgesic practice. *Ann Emerg Med*, 35(1):11-16.
11. Karpman, et al. (1997) Analgesia for emergency centers' orthopaedic patients: does an ethnic bias exist?, *Clinical Orthopaedics and Related Research,* 334:270-5.
12. Morrison R, et al. (2000) "We don't carry that" -- Failure of pharmacies in predominantly nonwhite neighborhoods to stock opioid analgesics. *N Engl J Med,* 342:1023-1026.
13. Cleeland CS, Gonin, R, Baez, L, Loehrer, P, & Pandya, KJ. (1997). Pain and Treatment of Pain in Minority Patients with Cancer: The Eastern Cooperative Oncology Group Minority Outpatient Pain Study. *Ann Intern Med*, 127(9):813-816.
14. *Pain: Hope through Research.* Retrieved October 12, 2008 from National Institute of Neurological Disorders and Stroke (NINDS). Web site: http://www.ninds.nih.gov/disorders/chronic_pain/detail_chronic_pain.htm
15. Dalton J. (1998). A call for standardizing the clinical rating of pain intensity using a 0 to 10 rating scale. *Cancer Nursing*, 21(11):46–49.
16. Lebovits A. (2005) The ethical implications of racial disparities in pain: Are some of us more equal? *Pain Med*; 6:3-4.
17. Payne R, et al. (2002) Quality of Life Concerns in Patients with Breast Cancer. *Cancer,* 97:311-317.
18. Ng, Dimsdale, Shragg, et al. (1996) Ethnic differences in analgesic consumption for postoperative pain. *Psychosomatic Medicine*, 58:125-9.
19. *Racial and Ethnic Identifiers in Pain Management: The Importance to Research, Clinical Practice, and Public Health Policy: A Position Statement from the American Pain Society.* Approved by the APS Board of Directors, October 22, 2004. Retrieved October 12, 2008 from American Pain Society. Web site: http://www.ampainsoc.org/advocacy/ethnoracial.htm
20. *Unequal Treatment - Confronting Racial and Ethnic Disparities in Healthcare* (2002). Retrieved October 10, 2008 from Institute of Medicine. Web site: http://www.iom.edu/?id=16740

Topic Brief

# CHRONIC PAIN AND OPIOID TREATMENT

Effective management of chronic pain often requires a step-wise trial of different treatment options, a team of healthcare providers and social support from family and friends. Healthcare providers may start with behavioral and non-pharmacological interventions (e.g., hot/cold therapy, physical therapy, relaxation techniques) when devising pain treatment plans. However, pain relievers, including prescription pain medicines (opioid analgesics), are often prescribed to help alleviate pain and improve function.

## Key Issues

- More than 76.5 million Americans suffer with pain.[1] The consequences of unmanaged chronic pain are devastating for patients. It is not uncommon for patients with intractable, debilitating pain—many of whom are often made to feel that the pain is "just in their heads"—to want to give up rather than living one more day in excruciating pain.
- For many patients, opioids are an integral part of a comprehensive pain management plan to help relieve pain, restore functioning and improve quality of life.[2,3]
- Unfortunately, patient access to these medications may be hindered by unbalanced state policies, persisting social stigma surrounding their use, as well as therapeutic switching and/or step therapies imposed by insurance companies.
- Unless a patient has a past or current personal or family history of substance abuse, the likelihood of addiction is low when opioids are taken as prescribed and under the guidance of a physician; however, they have the potential for misuse, abuse and diversion.
- Rising rates of prescription drug abuse and emergency room admissions related to prescription drug abuse, as well as an increase in the theft and illegal resale of prescription drugs, indicate that drug diversion is a growing problem nationwide.[4] The main source of drug diversion is unlikely the prescriber as was once assumed, but rather from theft by family, friends and workers in the home or from the sharing and selling of medications though often with good intentions.[5]
- Diverse players (e.g., lawmakers, educators, healthcare providers, the pharmaceutical industry, caregivers) must come together to address the dual public health crises of the undertreatment of pain and rising prescription drug abuse.[6]
- Alleviating pain remains a medical imperative—one that must be balanced with measures to address rising non-medical use of prescription drugs and to protect the public health.[6]

### Opioids 101

Opioids include morphine, oxycodone, oxymorphone, hydrocodone, hydromorphone, methadone, codeine and fentanyl. Opioids are classified in several ways, most commonly based on their origin and duration of effects.[7]

**Common classifications for opioids**[7,8]

| | | |
|---|---|---|
| **SOURCE** | **Natural or semisynthetic:** Contained in or slightly modified (semisynthetic) from chemicals found in poppy resin | Synthetic: Synthesized in the laboratory |
| **DURATION OF RESPONSE** | **Short-acting:** Provide quick-acting pain relief and are used primarily as "rescue medication," as in acute pain | Long-acting: Provide longer duration of pain relief and are most often used for stable, chronic pain |

One of the advantages of opioids is that they can be given in so many different ways. For example, they can be administered by mouth, rectal suppository, intravenous injection (IV), subcutaneously (under the skin), transdermally (in the form of a patch) or into a region around the spinal cord. Patches, IV injections and infusions are very important for patients who cannot swallow, or whose GI tracts are not working normally.[9]

Opioids are believed to work by binding to specific proteins (opioid receptors), which are found in specialized pain-controlling regions of the brain and spinal cord. When these compounds attach to certain opioid receptors, the electrical and chemical signals in these regions are altered, ultimately reducing pain.[7]

Because of their long history of use, the clinical profile of opioids has been very well characterized. Multiple clinical studies have shown that long-acting opioids, in particular, are effective in improving:

- Daily function
- Psychological health
- Overall health-related quality of life for patients with chronic pain[10]

However, some types of pain, such as pain caused by nerve compression or destruction, do not appear to be relieved by opioids.[8]

### Adverse Effects

Side effects of opioids result primarily from activation of opioid receptors outside and within the nervous system. Activation of opioid receptors in the gut, for example, may cause constipation, nausea and vomiting, and other gastrointestinal effects. Tolerance to nausea and vomiting usually develops within the first few days or weeks of therapy, but some patients are intolerant to opioids and experience severe adverse side effects.[8] Other side effects include drowsiness, mental clouding and, in some people, euphoria.[7] Recent research shows that genetic variations may influence opioid metabolism.

Depending on the amount taken, opioids can depress breathing. The risk of sedation and respiratory depression is heightened when opioids are taken with other sedating medications (e.g., antihistamines, benzodiazepines), reinforcing the need to carefully monitor patients. However, this effect is usually is not present after a patient has taken opioids regularly.

### Careful Monitoring of and Open Communication with Patients

Patients taking opioids must be carefully selected and monitored, and should speak openly with their healthcare provider about noticeable improvements in functioning, as well as side effects and other concerns (e.g., constipation, fears of addiction).

*The Four "A's"*

**Analgesia** – Is the pain relief clinically significant? Is there a reduction in the pain score (0-10)?

**Activity levels** – What is the patient's level of physical and psychosocial functioning? Has treatment made an improvement?

**Adverse effects** – Is the patient experiencing side effects from pain relievers? If so, are they tolerable?

**Aberrant behaviors** – Are there any behaviors of concern such as early refills or lost medication? Does the patient show signs of misuse, abuse or addiction? What is the plan of action?

*Source:* Passik & Weinreb, 1998; Passik & Portenoy, 1998

The American Pain Foundation's *Target Chronic Pain* materials help facilitate open dialogue between patients and their healthcare team, and give prescribers tools for selecting, monitoring and following patients. To access these resources, visit www.painfoundation.org and click on the Publications tab.

**Dual Public Health Crises: Balancing Medical Imperative to Relieve Suffering and Protect Public Safety**

Pain affects more Americans than diabetes, heart disease and cancer combined, and it is one of the leading causes of disability in the United States. Recognition of pain as a growing public health crisis has led to the establishment of specialized pain clinics, treatment guidelines for certain types of pain, as well as greater use of treatment strategies to effectively alleviate pain and improve functioning, including prescription pain medicines.

As the therapeutic use of opioids has increased to appropriately address pain, there has been a simultaneous and dramatic rise in non-medical use of prescription drugs.[11] When abused—that is, taken by someone other than the patient for whom the medication was prescribed, or taken in a manner or dosage other than what was prescribed—prescription medications can produce serious adverse health effects and can lead to addiction, overdose and even death.

People who abuse opioids typically do so for the euphoric effects (e.g., the "high"); however, most abusers are ***not*** patients who take opioids to manage pain.[12] Rather, they are often people within the social network of the patient. In fact, 71% of people abusing prescription pain relievers received them from a friend or family member without a prescription.[5] Prescription pain relievers are usually stolen from medicine cabinets, purchased or shared in schools, or simply given away.

**Picture of Prescription Drug Abuse in America**

- An estimated 2.2 million Americans abused pain medications for the first time in 2006.[12] The rate of new abuse of opioids has risen most dramatically among teenagers.
- Between 1992 and 2002, reported abuse by teenagers increased by 542%.[13]
- From 1999 to 2004, unintentional poisoning deaths associated with opioids and hallucinogens rose by 55%, and the increase has been attributable primarily to prescription pain relievers.[14]
- According to 2005 and 2006 National Surveys on Drug Use and Health, an annual national average of 6.2% of persons aged 12 or older had used a prescription psychotherapeutic drug non-medically in the 12 months leading up to the survey; an average of 9.1% of youths aged 12 to 17 were past year non-medical users of any prescription psychotherapeutic drug.[12]
- Nearly 600,000 emergency department visits involved non-medical use of prescription or over-the-counter (OTC) pharmaceuticals or dietary supplements. Opiates/opioid analgesics accounted for 33% of the non-medical visits. Anti-anxiety agents (sedatives and hypnotics) accounted for 34% of the non-medical visits.[4]

The growing prevalence of prescription drug abuse not only threatens the lives of abusers; concerns about misuse, abuse and diversion may also jeopardize effective pain management by impeding patient access to opioids. Fear of scrutiny by regulators or law enforcement, and specific action by some agencies, has had a "chilling effect" on the willingness of some doctors, nurse practitioners and physician assistants to prescribe opioids.[6,15]

Moreover, high profile reports of drug abuse, diversion and addiction, or of legal actions taken against prescribers have helped perpetuate a negative—and sometimes false—picture of chronic pain management.[6] Over time, these reports overshadow untold stories of people with pain—those whose lives have been shattered by unrelenting pain—who get needed pain relief from these medications. Understanding the difference between tolerance, physical dependence, abuse and addiction is also critical to telling the story (See page 31-32 for definitions). According to medical experts, use of the term "narcotic" in news reports may further reinforce the myths and misconceptions of this class of drugs, given the negative connotation.[6]

*"…[T]he attitude toward opioids has ranged from complete avoidance to widespread therapeutic use with minimal caution. These extremes have been driven by insufficient appreciation of risks by those at one end of the spectrum, and excessive fear of punitive regulatory scrutiny or exaggerated perceptions of addictive risk by those at the other. When opioids are prescribed for pain control in adequately evaluated, selected, and monitored patients, addiction is rare."*

— Perry Fine, Topics in Pain Management

## Strategies to Address Twin Public Health Crises

Systematic and targeted approaches are essential to address the growing prevalence and complexity of prescription drug abuse, while simultaneously ensuring that people with legitimate medical needs receive effective treatment.

*These approaches can generally be categorized as follows:*

- Legislative strategies to create balanced and consistent regulation and improve state-based prescription drug monitoring programs.
- Educational efforts to raise awareness about prescription drug abuse and its dangers among schools, families, healthcare providers, patients and potential abusers.
- Medical strategies to help identify and monitor patients who require opioid management, to include the incorporation of risk management into the treatment plan (e.g., treatment agreements, urine testing and monitoring, transition planning, collaborative practice with addiction medicine and behavioral health specialists).
- Pharmaceutical industry strategies to help prevent misuse, abuse and diversion by developing new tamper resistant packaging and/or formulations (e.g., tamper-resistant bottles, electromagnetic chips to track medication, new formulations that could resist or deter common methods of opioid abuse).

For additional recommendations, see the American Pain Foundation's report outlining critical barriers to appropriate opioid prescribing for pain management, *Provider Prescribing Patterns and Perceptions: Identifying Solutions to Build Consensus on Opioid Use in Pain Management.* This 16-page report calls for a more balanced perspective of the risks and benefits of these medications in practice and policy and summarizes key challenges and actionable solutions discussed by leading pain experts at a roundtable meeting hosted by APF.

### Making the Grade: Evaluation of State Policies

The Pain & Policy Studies Group (PPSG) report "Achieving Balance in State Pain Policy: A Progress Report" graded states on quality of its policies affecting pain treatment and centered on the balance between preventing abuse, trafficking and diversion of controlled substances and simultaneously ensuring the availability of these medications for legitimate medical purposes. PPSG researchers evaluated whether state pain policies and regulations enhance or impede pain management and assigned each state a grade from 'A' to 'F.'

**State Grades for 2008**

| State | 2008 Grade | State | 2008 Grade |
|---|---|---|---|
| Alabama | B+ | Montana | C+ |
| Alaska | C+ | Nebraska | B+ |
| Arizona | B+ | Nevada | C |
| Arkansas | B | New Hampshire | B |
| California | B | New Jersey | C+ |
| Colorado | B | New Mexico | B+ |
| Connecticut | B | New York | C |
| Delaware | C+ | North Carolina | B |
| District of Columbia | C+ | North Dakota | B |
| Florida | B | Ohio | B |
| Georgia | B | Oklahoma | C+ |
| Hawaii | B | Oregon | A |
| Idaho | B | Pennsylvania | C+ |
| Illinois | C | Rhode Island | B+ |
| Indiana | C+ | South Carolina | C+ |
| Iowa | B | South Dakota | B |
| Kansas | A | Tennessee | C |
| Kentucky | B | Texas | C |
| Louisiana | C | Utah | B+ |
| Maine | B+ | Vermont | B+ |
| Maryland | B | Virginia | A |
| Massachusetts | B+ | Washington | B+ |
| Michigan | A | West Virginia | B |
| Minnesota | B+ | Wisconsin | A |
| Mississippi | C+ | Wyoming | C+ |
| Missouri | C+ | | |

*Source:* The Pain & Policy Studies Group, http://www.painpolicy.wisc.edu/Achieving_Balance/PRC2008.pdf.

## At a Glance: Differentiating physical dependence, tolerance, abuse and addiction

Unfortunately, confusion between normal physiological responses to opioids (physical dependence and tolerance) and pathological phenomena such as addiction or abuse persist. Such misunderstandings not only reinforce the stigma surrounding legitimate medical use of these medicines, they also fuel fears of addiction and, in turn, may impinge on patient access to these medications. Although the use of opioids carries some risk of addiction, clinical studies have shown that the potential for addiction is low for the vast majority of patients using opioids for the long-term management of chronic pain.[17] As with any medication, there are risks, but these risks can be managed.

*"Universal agreement on definitions of addiction, physical dependence and tolerance is critical to the optimization of pain treatment and the management of addictive disorders."*

— Consensus document from the American Academy of Pain Medicine, the American Pain Society and the American Society of Addiction Medicine

**Physical dependence** is characterized by biological changes that lead to withdrawal symptoms (e.g., sweating, rapid heart rate, nausea, diarrhea, goosebumps, anxiety) when a medication is discontinued, and is not related to addiction. Physical dependence differs from psychological dependence, or the cravings for the euphoria caused by opioid abuse. Symptoms of physical dependence can often be ameliorated by gradually decreasing the dose of medication during discontinuation.[7]

**Tolerance** is a biological process in which a patient requires increasing amounts of a medication to achieve the same amount of pain relief. Dose escalations of opioid therapies are sometimes necessary and reflect a biological adaptation to the medication. Although the exact mechanisms are unclear, current research indicates that tolerance to opioid therapy develops from changes in opioid receptors on the surface of cells.[7] Thus, the need for higher doses of medication is not necessarily indicative of addiction.[3]

**Addiction** is a disease characterized by preoccupation with and compulsive use of a substance, despite physical or psychological harm to the person or others.[3] Behaviors suggestive of addiction may include: taking multiple doses together, frequent reports of lost or stolen prescriptions, and/or altering oral formulations of opioids.

**Abuse** is the intentional self-administration of a medication for a non-medical purpose, such as to obtain a high.[3] Both the intended patient and others have the potential to abuse prescription drugs; in fact, the majority of people who abuse opioids do not suffer from chronic pain.[12]

**Pseudo-addiction** describes patient behaviors that may occur when pain is undertreated. Patients with unrelieved pain may become focused on obtaining medications and may otherwise seem inappropriately "drug seeking," which may be misidentified as addiction by the patient's physician. Pseudo-addiction can be distinguished from true adduction in that this behavior ceases when pain is effectively treated.[3]

## MISUSE VS. ABUSE?

- **Medical Misuse**: Legitimate use of a valid personal prescription but using differently from provider's instruction, such as taking more frequently or higher than the recommended doses. Use may be unintentional and considered an educational issue.
- **Medical Abuse**: Valid personal prescription by using for reasons other than its intent, such as to alleviate emotional stress, sleep restoration/prevention, performance improvement, etc. Use may be unintentional and considered an educational issue.
- **Prescription Drug Misuse**: Intentional use of someone else's prescription medication for the purpose of alleviating symptoms that may be related to a health problem. The use may be appropriate to treat the problem but access to obtain this drug may be difficult/untimely or may have been provided from a well-intentioned family member or friend.
- **Prescription Drug Abuse**: Intentional use of a scheduled prescription medication to experiment, to get high or to create an altered state. Access to the source may be diversion from family, friends or obtained on the street. Inappropriate or alteration of drug delivery system, used in combination of other drugs or used to prevent withdrawal from other substances that are being abused are included in this definition.

*Source:* Carol J. Boyd PhD, MSN, RN; Director: Institute for Research on Women and Gender, Substance Abuse Research Center, University of Michigan

**Risk factors for opioid misuse include, but are not limited to:**[2,3,19]

- Personal or family history of prescription drug or alcohol abuse
- Cigarette smoking
- History of motor vehicle accidents
- Substance use disorder
- Major psychiatric disorder (e.g., bipolar disorder, major depression, personality disorder)
- Poor family support
- History of preadolescent sexual abuse

*NOTE:* Unless a patient has a past or current history of substance abuse, the potential for addiction is low when opioid medications are prescribed by a doctor and taken as directed. Those patients who suffer with chronic pain and addictive disease deserve the same quality of pain treatment as others, but may require greater resources in their care.

## WEB RESOURCES

**Opioid RX**
http://pain-topics.org/opioid_rx/#RiskManage

**Tufts Health Care Institute Program on Opioid Risk Management**
http://www.thci.org/opioid/

**Opioid Risk Management PainEDU**
http://www.painedu.org/soap.asp

**Emerging Solutions**
http://www.emergingsolutionsinpain.com/index.php?option=com_frontpage&Itemid=1

## REFERENCES


1. National Center for Health Statistics. Health, United States, 2006 With Chartbook on Trends in the Health of Americans. Hyattsville, MD. Available at http://www.cdc.gov/nchs/data/hus/hus06.pdf.
2. Fine, PG. Opioid Therapy as a Component of Chronic Pain Management: Pain Experts Weigh In on Key Principles to Optimize Treatment. *Topics in Pain Management.* May 2008;23(10):1-8.
3. Katz NP, Adams EF, Chilcoat H, Colucci RD, et al. Challenges in the development of prescription opioid abuse-deterrent formulations. *Clin J Pain.* 2007;23:648-660.
4. Substance Abuse and Mental Health Services Administration, Office of Applied Studies. Drug Abuse Warning Network, 2005: National Estimates of Drug-Related Emergency Department Visits. DAWN Series D-29, DHHS Publication No. (SMA) 07-4256, Rockville, MD, 2007. Available at http://dawninfo.samhsa.gov/files/DAWN-ED-2005-Web.pdf.
5. Substance Abuse and Mental Health Services Administration. (2008). *Results from the 2007 National Survey on Drug Use and Health: National Findings* (Office of Applied Studies, NSDUH Series H-34, DHHS Publication No. SMA 08-4343). Rockville, MD. See http://oas.samhsa.gov/nsduh/2k7nsduh/2k7Results.pdf.
6. American Pain Foundation. Provider Prescribing Patterns and Perceptions: Identifying Solutions to Build Consensus on Opioid Use in Pain Management—A Roundtable Discussion. April 2008. Available at www.painfoundation.org.
7. Schumacher MA, Basbaum AI, Way WL. Opioid analgesics & antagonists. In: Katzung, ed. Basic and clinical pharmacology. 10th ed. New York, NY: McGraw Hill, 2007;489-502.
8. McQuay H. Opioids in pain management. *The Lancet*. 1999;353:2229-2232.
9. American Pain Foundation. Treatment Options: A Guide for People Living with Pain. 2007. Available at www.painfoundation.org.
10. Furlan AD, Sandoval JA, Mailis-Gagnon AM, Tunks E. Opioids for chronic noncancer pain: a meta-analysis of effectiveness and side effects. *CMAJ.* 2006;11:1589-1594.
11. Kuehn BM. Opioid prescriptions soar: increase in legitimate use as well as abuse. *JAMA.* 2007;297:249-250.
12. Substance Abuse and Mental Health Services Administration. (2007). Results from the 2006 National Survey on Drug Use and Health: National Findings (Office of Applied Studies, NSDUH Series H-32, DHHS Publication No. SMA 07-4293). Rockville, MD.
13. Bollinger LC, Bush C, Califano JA, et al. Under the counter: the diversion and abuse of controlled prescription drugs in the U.S. The National Center on Addiction and Substance Abuse at Columbia University (CASA), July 2005.
14. Centers for Disease Control. Unintentional poisoning deaths – United States, 1999-2004. *MMWR.* February 9, 2007;56(05):93-96.
15. Lin JJ, Afandre D, Moore C. Physician attitudes toward opioid prescribing for patients with persistent noncancer pain. *Clin J Pain.* 2007;23:799-803.
16. Fine PG, Portenoy RK: Clinical Guide to Opioid Analgesia, 2nd Edition. New York: Vendome, 2007
17. Sinatra R. Opioid analgesics in primary care: challenges and new advances in the management 0f noncancer pain. *J Am Board Fam Med.* 2006;19:165-77.
18. Boyd C. Presentation at the NIDA/AMA Joint Meeting, Pain, Opioids, and Addiction: An Urgent Problem for Doctors and Patients, NIH, Bethesda, Maryland, March 5, 2007.



*"The purpose of life…is to be useful, to be honorable, to be compassionate, to make some difference…"*

*—Ralph Waldo Emerson*

# INTEGRATIVE MEDICINE: NON-DRUG TREATMENT OPTIONS FOR PAIN MANAGEMENT

Pain management continues to challenge healthcare providers and places added strain on an already fragmented health care system. The U.S. health system was built around acute illness; however, because of advances in modern medicine and increased longevity, many Americans are living longer and with one or more chronic conditions (for example, cancer, diabetes, heart disease and arthritis), which require careful coordination of care and symptom management.

While pain is a symptom of many chronic diseases and is expected after many surgical procedures, persistent pain should not be viewed simply as a symptom. According to experts, the pain itself becomes a disease when the origin of the pain signals fails to shut off due to damage of the pain alarm system, leaving the person with persisting pain.

Whatever the cause, chronic pain transcends the physical hurting. Persistent pain interferes with daily life and relationships, and takes a tremendous toll on a person's mind, body and spirit. It's no surprise that pain and associated problems (e.g., medication side effects, depression and anxiety, limited mobility) are best managed using a combination of treatments tailored to each patient. This is referred to as a "multi-modal" or integrative approach.

**Multi-modal Therapeutic Strategies for Managing Pain and Related Disability**




**Integrative medicine** combines conventional medicine with complementary healing techniques, such as massage, yoga and acupuncture, to address the specific needs. Because an interdisciplinary approach to pain management is patient-centered, patients learn how to manage and cope with pain by playing an active role in their treatment plan.

Integrative medicine combines treatments from conventional medicine and complementary and alternative therapies for which there is some high-quality evidence of safety and effectiveness.[1] Being able to deliver integrated medicine, which incorporates proven CAM therapies into "mainstream" care, is increasingly important to consumers and healthcare providers.[4]

## Benefits of Combined Treatment Modalities

While medications remain an integral part of pain management plans, non-drug therapies may be used to supplement and enhance the effectiveness of current pain medications. These strategies also offer additional options for those patients at greater risk for, or who are intolerant of, medication side effects.

Moreover, a growing body of research reinforces the benefits of interventions that address the psychosocial aspects of pain, especially given recent evidence of a biological link between the regions of the brain involved with depression and pain regulation. People with pain often suffer from depression, which can affect a patient's thinking, concentration and behavior, and increase pain sensitivity and severity.

Effective pain management may also require lifestyle changes that are supportive of patient mobility and independence.[2] For example, to improve daily functioning, specific therapies may be suggested to increase muscle strength and flexibility, enhance sleep and reduce fatigue, and assist patients in performing usual activities and work-related tasks.

As with the management of other chronic illnesses, patients with chronic pain need to play an active role in their care and incorporate non-drug options and other lifestyle changes (e.g., exercise, proper nutrition) over the long-term.

## Patients Seek Complementary Treatments

In their quest for better pain relief, patients are increasingly turning to non-drug approaches to help ease their discomfort and give them a sense of empowerment and control. There are a wide variety of non-drug therapies available to treat pain and related disability including:

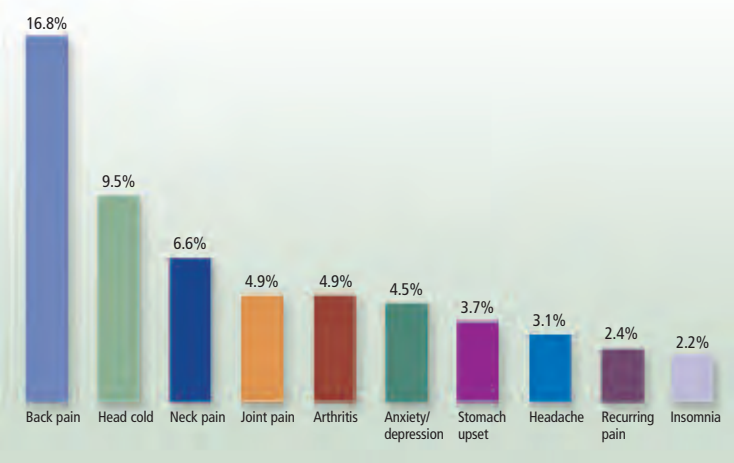
- *Psychosocial interventions* – cognitive behavioral therapy, stress management
- *Rehabilitation techniques* – exercise, heat or cold therapy, physical therapy
- *Complementary and alternative medicine* – meditation, acupuncture, hypnotherapy, yoga, aromatherapy, massage, touch therapy

Not surprisingly, pain conditions are among those most likely to prompt patients to turn to complementary and alternative medicine (CAM) therapies. These practices also give patients a greater sense of control, so they no longer feel that they are solely dependent on a single pill or procedure.

### COMMON NON-DRUG OPTIONS FOR PAIN RELIEF

- Stress management techniques (e.g., meditation, deep breathing and relaxation exercises)
- Massage
- Application of heat or cold, including heating pads or ice packs
- Acupuncture
- Visualization
- Physical therapy, including stretching or exercise
- Hypnotherapy
- Psychological and spiritual counseling
- Biofeedback
- Transcutaneous electrical nerve stimulation, also known as TENS

**Diseases/Conditions for Which CAM is Used Most Often**




Source: NCCAM, *The Uses of CAM in the United States.*

### What is CAM?

CAM includes a diverse group of healing systems, practices and products that are not part of conventional medicine. Examples of CAM therapies include acupuncture, massage, meditation, hypnosis, yoga and herbal therapies. These approaches are increasingly used to help manage pain and related issues (e.g., depression, anxiety, fatigue) and enhance patients' quality of life. NCCAM, one of 27 institutes and centers designated by the National Institutes of Health, is the lead agency for scientific research on CAM and groups these therapies into four areas.

| CAM DOMAINS DEFINED | |
|---|---|
| **Mind-body medicine** | Uses a variety of techniques designed to enhance the mind's ability to affect the body's function and symptoms. Examples include meditation, hypnosis, guided imagery, prayer, as well as art or music therapy. |
| **Biologically based practices** | Use substances found in nature, such as herbs, special diets or vitamins. Some examples include dietary supplements or herbal products (e.g., garlic, ginger, Kava Kava). |
| **Manipulative and body-based practices** | Based on manipulation or movement of one or more parts of the body. Examples include massage and chiropractic or osteopathic manipulation. |
| **Energy medicine** | Involves the use of energy fields, such as magnetic fields or biofields (energy that some believe surround and run through the body). Examples include qi gong, Reiki and therapeutic touch. |

Many CAM practices are gentle methods that tend to have fewer side effects, which is part of their appeal to patients. Patients also use these therapies to help alleviate the associated stress, depression and insomnia that can accompany and worsen pain sensations.

Some CAM practices, such as acupuncture, massage and chiropractic care require the practitioner to be licensed. It's important for patients to research and find a CAM practitioner who is certified, willing and equipped to coordinate with other members of the patient's health team, and has experience working with patients with chronic pain.

*When tailored to the individual patient, non-drug approaches to pain management can help:*

- Allow patients to take an active role in managing their pain, thereby, improving patient satisfaction
- Address the physical, emotional and spiritual needs of patients
- Reduce pain and manage related symptoms (e.g., pain and anxiety, depression, insomnia, fatigue)
- Enhance the effectiveness and minimize adverse effects of medications
- Reduce health care costs by reducing doctor visits and reliance on medications
- Improve functioning and the ability to perform activities of daily living
- Enhance wellness and quality of life

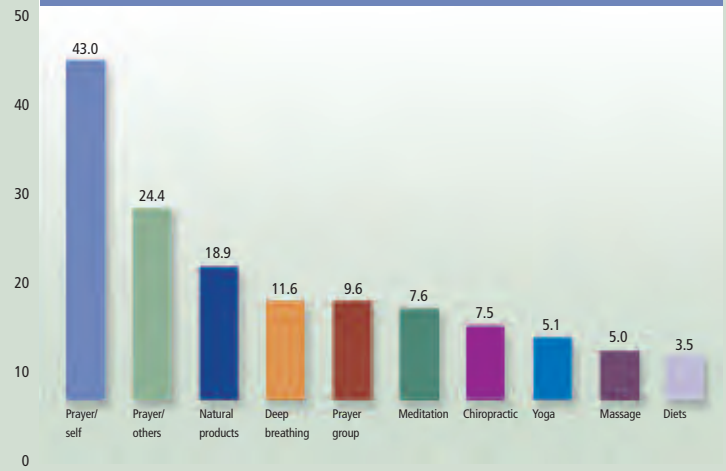
**More and more Americans are turning to CAM to help manage and treat various health problems, including pain and distress.**

- An estimated 36% of American adults use some form of CAM, and this percentage jumps to 62% if prayer for health reasons and megavitamin therapy are included.[3]
- Americans spend at least $34-47 billion on CAM therapies, exceeding out of pocket expenses for all U.S. hospitalizations. CAM is expected to grow by 15% each year.[3]
- People report using CAM because these methods mirror their personal beliefs, values and philosophical orientations toward life.[4]
- Many people use CAM to help relieve back pain, joint pain, severe headache and pain associated with migraines, dental and jaw pain and for a variety of other reasons.[4]

## Barriers to Fully Integrating CAM

Despite CAM's growing popularity, there are barriers to its widespread use. According to CAM experts, these include:

- Limited scientific evidence about the safety and effectiveness of certain therapies. Studies are underway to research specific CAM practices for pain management.
- Lack of professional training in CAM and integrative medicine and limited resources to coordinate services.
- Restricted health insurance coverage. Many CAM therapies are not yet covered by health insurance carriers and are, therefore, only available to patients on an outpatient fee-for-service basis. Insurers tend to restrict reimbursement to "medically necessary" therapies and without the data to back up their effectiveness, these practices are not covered.




*Source:* Barnes P, Powell-Griner E, McFann K, Nahin R, CDC Advance Data Report #343, *Complemenary and Alternative Medicine Use Among Adults*; United States, 2002. May 27, 2004.

- Lack of education (on the part of consumers and providers) about the appropriate use of CAM therapies and how best to integrate them with standard pain treatments.
- Misperceptions about CAM therapies as "elusive, nonsensical options."

*Source:* American Pain Foundation, *Pain Community News*, Spring 2008.

## Paying for non-drug therapies

The majority of CAM treatments are not currently covered by traditional insurance plans, largely due to the absence of scientific evidence proving the effectiveness of some CAM therapies. When coverage for CAM is offered, it is generally limited to more common therapies such as acupuncture and massage. Most people must pay for CAM services out-of-pocket; however, consumer interest has prompted more insurance companies to consider CAM coverage.

A study in Washington State, where private health insurers are legally required to cover licensed CAM providers, found that a significant number of people were utilizing CAM insurance benefits with only a modest effect on insurance expenditures.[5]

Given the high cost and low insurance coverage of many CAM therapies, it is important that patients, especially those that are no longer able to work, have access to low cost, at-home therapies that provide effective pain relief. These may include heat and cold therapies, relaxation techniques and exercise.

People living with chronic pain are increasingly turning to CAM to help alleviate their suffering and improve their quality of life. The addition of these therapies often results in better pain relief and fewer side effects. However, more research is needed to prove the effectiveness of certain therapies and increase the likelihood that they will be covered by conventional insurance providers and offered as an option to all patients living with pain.

With nearly half of all consumers concerned about the safety of their health care,[6] the use of CAM and other non-drug treatments for pain management is expected to grow as non-drug therapies are proven safe and effective and adopted into routine health care.[7]

**For a snapshot of recent research on select CAM therapies, see the Spring 2008 issue of *Pain Community News* at www.painfoundation.org.**

### REFERENCES


1. *What is CAM?* Retrieved October 10, 2008 from National Center for Complementary and Alternative Medicine (NCCAM). Web site: http://nccam.nih.gov/health/whatiscam/
2. Ahmad, M, Goucke, C.R. (2002) Management strategies for the treatment of neuropathic pain in the elderly. *Drugs Aging*, 19(12):929-4.
3. Barnes, P., Powell-Griner, E., McFann, K., Nahin, R. *CDC Advance Data Report #343*. Complementary and Alternative Medicine Use Among Adults: United States, 2002. May 27, 2004.
4. *Treatment Options: A Guide for People Living with Pain.* Retrieved October 10, 2008 from American Pain Foundation. Web site: http://www.painfoundation.org/Publications/TreatmentOptions2006.pdf
5. Lafferty, W.E., Tyree, P.T., et al. (2006) Insurance Coverage and Subsequent Utilization of Complementary and Alternative Medical (CAM) Providers. *Am J Manag Care*, 12(7):397–404.
6. Henry J. Kaiser Family Foundation (November 17, 2004). "Five years after IOM report on medical errors, nearly half of all consumers worry about the safety of their health care" Press release. Retrieved October 12, 2008 from http://www.kff.org/kaiserpolls/pomr111704nr.cfm
7. *The Pain Community News: Spring 2008*, 8(2). Retrieved October 10, 2008 from American Pain Foundation. Web site: http://www.painfoundation.org/Publications/PCN08spring.pdf


### WEB RESOURCES

**American Academy of Pain Management**
www.aapainmanage.org

**American Pain Foundation Treatment Options**
www.painfoundation.org/Publications/TreatmentOptions2006.pdf

**National Center for Complementary and Alternative Medicine (NCCAM)**
www.nccam.nih.gov

**The Office of Cancer Complementary and Alternative Medicine**
www.cancer.gov/cam

# Pain A to Z

## Common Pain Terms and Syndromes

*Hundreds of pain syndromes or disorders make up the spectrum of pain. There are the most benign, fleeting sensations of pain, such as a pin prick. There is the pain of childbirth, the pain of a heart attack, and the pain that sometimes follows amputation of a limb. There is also pain accompanying cancer and the pain that follows severe trauma, such as that associated with head and spinal cord injuries. A sampling of common pain terms and syndromes follows, listed alphabetically.*

**Acute Pain** occurs suddenly due to illness, injury or surgery. It has a short duration that subsides when the injured tissue heals.

**Arachnoiditis** is a condition in which one of the three membranes covering the brain and spinal cord, called the arachnoid membrane, becomes inflamed. A number of causes, including infection or trauma, can result in inflammation of this membrane. Arachnoiditis can produce disabling, progressive, and even permanent pain.

**Arthritis** is the most prevalent cause of chronic disability in the United States. Millions of Americans suffer from arthritic conditions such as osteoarthritis, rheumatoid arthritis, ankylosing spondylitis, and gout. These disorders are characterized by joint pain in the extremities. Many other inflammatory diseases affect the body's soft tissues, including tendonitis and bursitis.

**Back pain** has become the high price paid by our modern lifestyle and is a startlingly common cause of disability for many Americans, including both active and inactive people. Common types of back pain include:

- **Sciatica** — back pain that spreads to the leg (see below).
- **Degenerative or ruptured disc** — type of back pain associated with the discs of the spine, the soft, spongy padding between the vertebrae (bones) that form the spine. Discs protect the spine by absorbing shock, but they tend to degenerate over time and may sometimes rupture.
- **Spondylolisthesis** — back condition that occurs when one vertebra extends over another, causing pressure on nerves and therefore pain.
- **Radiculopathy** — damage to nerve roots is a serious condition that can be extremely painful.

Treatment for a damaged disc includes drugs such as painkillers, muscle relaxants, and steroids; exercise or rest, depending on the patient's condition; adequate support, such as a brace or better mattress and physical therapy. In some cases, surgery may be required to remove the damaged portion of the disc and return it to its previous condition, especially when it is pressing a nerve root. Surgical procedures include discectomy, laminectomy, or spinal fusion. Minimally invasive procedures (vertebroplasty), certain complementary and alternative therapies and implantable devices may also help certain patients.

**Breakthrough Pain** is intermittent worsening of pain that occurs spontaneously or in relation to a specific activity. The pain increases above the level of pain being treated with ongoing analgesics (pain medications).

**Burn pain** can be profound and poses an extreme challenge to the medical community. First-degree burns are the least severe; with third-degree burns, the skin is lost. Depending on the injury, pain accompanying burns can be excruciating, and even after the wound has healed patients may have chronic pain at the burn site.

**Cancer pain** can accompany the growth of a tumor, the treatment of cancer, or chronic problems related to cancer's permanent effects on the body. Fortunately, most cancer pain can be treated to help minimize discomfort and stress to the patient.

**Central pain syndrome** — see Traumatic Pain below.

**Chronic Pain** is pain that persists for long periods of time (usually >3 months). Failure to treat acute pain promptly and appropriately at the time of injury, during initial medical and surgical care, and at the time of transition to community-based care, contributes to the development of chronic pain syndromes. In chronic pain, pain signals may remain active in the nervous system for weeks, months or even years. Chronic pain has no value or benefit; it is a disease of the nervous system.

***Types of Chronic Pain:***

- **Intermittent Pain** - episodic and may occur in waves or patterns.
- **Persistent Pain** - lasts 12 or more hours every day for more than three months.

**Complex Regional Pain Syndrome,** or CRPS, is a chronic pain condition that typically affects one or more limbs. It is accompanied by burning pain and hypersensitivity to temperature. Often triggered by trauma or nerve damage, CRPS causes the skin of the affected area to become characteristically shiny.

***There are two types of CRPS:***

- **CRPS I** (formerly known as Reflex Sympathetic Dystrophy Syndrome, or RSDS) is frequently triggered by tissue injury, but with no underlying or identifiable nerve injury.

- **CRPS II** (formerly known as Causalgia) is characterized by the same symptoms, but these cases are clearly associated with a specific nerve injury.

The cause of CRPS is not well understood, but experts believe it is due to a malfunction of the autonomic nervous system following blunt trauma to an arm or leg, after surgical procedures or even from minor injuries such as a sprain or fracture. Nerves begin to misfire, repeatedly sending pain impulses to the brain. The resulting pain seems out of proportion to the severity of the injury.

**Deafferentation Pain**: pain due to alteration or damage to the central nervous system (central pain or neuropathic pain) or may be alteration of nervous system within larger nerves or nerve roots before entry into central nervous system.

**Fibromyalgia** is a chronic pain disorder characterized by widespread musculoskeletal pain that has lasted for at least three months. People with fibromyalgia report general tenderness and soreness, muscle stiffness, especially in the morning, as well as fatigue. Stress or lack of sleep can make the symptoms of fibromyalgia worse. An estimated 6 million Americans have fibromyalgia, most of them women.

**Headaches** affect millions of Americans. The three most common types of chronic headache are migraines, cluster headaches, and tension headaches. Each comes with its own telltale brand of pain.

- **Migraines** are characterized by throbbing pain and sometimes by other symptoms, such as nausea and visual disturbances. Migraines are more frequent in women than men. Stress can trigger a migraine headache, and migraines can also put the sufferer at risk for stroke.
- **Cluster** headaches are characterized by excruciating, piercing pain on one side of the head; they occur more frequently in men than women.
- **Tension headaches** are often described as a tight band around the head.

**Head and facial pain** can be agonizing, whether it results from dental problems or from disorders such as cranial neuralgia, in which one of the nerves in the face, head, or neck is inflamed. Another condition, **trigeminal neuralgia** (also called tic douloureux), affects the largest of the cranial nerves and is characterized by a stabbing, shooting pain.

**Muscle pain** can range from an aching muscle, spasm, or strain, to the severe spasticity that accompanies paralysis. Another disabling syndrome is **fibromyalgia**, a disorder characterized by fatigue, stiffness, joint tenderness, and widespread muscle pain. **Polymyositis**, **dermatomyositis**, and **inclusion body myositis** are painful disorders characterized by muscle inflammation. They may be caused by infection or autoimmune dysfunction and are sometimes associated with connective tissue disorders, such as lupus and rheumatoid arthritis.

**Myofascial pain syndromes** affect sensitive areas known as trigger points, located within the body's muscles. Myofascial pain syndromes are sometimes misdiagnosed and can be debilitating.

**Neuropathic Pain** – is a type of pain that results from damage to or dysfunction of the nerves in either the peripheral or central nervous system, rather than stimulation of pain receptors (as is the case of somatic and visceral pain). Neuropathic pain can occur in any part of the body and is frequently described as a hot, burning sensation, which can be devastating to the affected individual. It can result from diseases that affect nerves (such as diabetes) or from trauma, or, because chemotherapy drugs can affect nerves, it can be a consequence of cancer treatment. Among the many neuropathic pain conditions are:

- **Diabetic neuropathy**, which results from nerve damage secondary to vascular problems that occur with diabetes;
- **Reflex sympathetic dystrophy syndrome** (see below), which can follow injury;
- **Phantom limb** and **post-amputation pain**, which can result from the surgical removal of a limb;
- **Postherpetic neuralgia**, which can occur after an outbreak of shingles; and
- **Central pain syndrome**, which can result from trauma to the brain or spinal cord.

**Nociceptive pain** - caused by an injury that stimulates pain receptors. Pain receptors, located on the tips of nerve cells, recognize and react to an unpleasant stimulus (pressure, extreme temperatures [hot or cold], substances released by other cells) and send pain signals through the nervous system for recognition and response. This type of pain may be accompanied by inflammation. Infections, burns, cuts, a severe lack of oxygen in the blood, and stretching of or pressure within an organ, can injure tissues and cause nociceptive pain.

***Types of Nociceptive Pain:***

- **Somatic Pain** - caused by injury to skin, muscles, bone, joint, and connective tissues. Deep somatic pain is usually described as dull or aching, and localized in one area. Somatic pain from injury to the skin or the tissues just below it often is sharper and may have a burning or pricking quality.
- **Visceral Pain** - originates from ongoing injury to the internal organs or the tissues that support them. When the injured tissue is a hollow structure, like the intestine or the gall bladder, the pain often is poorly localized and feels like cramping. When the injured structure is not a hollow organ, the pain may be pressure-like, deep, and stabbing.

**Pain flares.** Pain that suddenly erupts or emerges with or without an aggravating event or activity.

**Peripheral Neuropathic Pain** due to vascular disease or injury-such as vasculitis or inflammation of blood vessels, coronary artery disease, and circulatory problems-all have the potential to cause pain. Vascular pain affects millions of Americans and occurs when communication between blood vessels and nerves is interrupted. Ruptures, spasms, constriction, or obstruction of blood vessels, as well as a condition called ischemia in which blood supply to organs, tissues, or limbs is cut off, can also result in pain.

**Reflex sympathetic dystrophy syndrome** — see Complex Regional Pain Syndrome.

**Repetitive stress injuries** are muscular conditions that result from repeated motions performed in the course of normal work or other daily activities. They include:

- writer's cramp, which affects musicians and writers and others,
- compression or entrapment neuropathies, including carpal tunnel syndrome, caused by chronic overextension of the wrist and
- tendonitis or tenosynovitis, affecting one or more tendons.

**Sciatica** is a painful condition caused by pressure on the sciatic nerve, the main nerve that branches off the spinal cord and continues down into the thighs, legs, ankles, and feet. Sciatica is characterized by pain in the buttocks and can be caused by a number of factors. Exertion, obesity, and poor posture can all cause pressure on the sciatic nerve. One common cause of sciatica is a herniated disc.

**Shingles and other painful disorders** affect the skin. Pain is a common symptom of many skin disorders, even the most common rashes. One of the most vexing neurological disorders is shingles or herpes zoster, an infection that often causes agonizing pain resistant to treatment. Prompt treatment with antiviral agents is important to arrest the infection, which if prolonged can result in an associated condition known as **postherpetic neuralgia**. Other painful disorders affecting the skin include:

- **Vasculitis**, or inflammation of blood vessels;
- Other infections, including **herpes simplex**;
- Skin **tumors** and **cysts**, and

Tumors associated with **neurofibromatosis**, a neurogenetic disorder.

**Somatic pain**—see Nociceptive Pain.

**Sports injuries** are common. Sprains, strains, bruises, dislocations, and fractures are all well-known words in the language of sports. Pain is another. In extreme cases, sports injuries can take the form of costly and painful spinal cord and head injuries, which cause severe suffering and disability.

**Spinal stenosis** refers to a narrowing of the canal surrounding the spinal cord. The condition occurs naturally with aging. Spinal stenosis causes weakness in the legs and leg pain usually felt while the person is standing up and often relieved by sitting down.

**Surgical pain** may require regional or general anesthesia during the procedure and medications to control discomfort following the operation. Control of pain associated with surgery includes presurgical preparation and careful monitoring of the patient during and after the procedure.

**Temporomandibular disorders** are conditions in which the temporomandibular joint (the jaw) is damaged and/or the muscles used for chewing and talking become stressed, causing pain. The condition may be the result of a number of factors, such as an injury to the jaw or joint misalignment, and may give rise to a variety of symptoms, most commonly pain in the jaw, face, and/or neck muscles. Physicians reach a diagnosis by listening to the patient's description of the symptoms and by performing a simple examination of the facial muscles and the temporomandibular joint.

**Traumatic pain** can occur after injuries in the home, at the workplace, during sports activities, or on the road. Any of these injuries can result in severe disability and pain. Some patients who have had an injury to the spinal cord experience intense pain ranging from tingling to burning and, commonly, both. Such patients are sensitive to hot and cold temperatures and touch. For these individuals, a touch can be perceived as intense burning, indicating abnormal signals relayed to and from the brain. This condition is called **central pain syndrome** or, if the damage is in the thalamus (the brain's center for processing bodily sensations), **thalamic pain syndrome**. It affects as many as 100,000 Americans with multiple sclerosis, Parkinson's disease, amputated limbs, spinal cord injuries, and stroke. Their pain is severe and is extremely difficult to treat effectively. A variety of medications, including analgesics, antidepressants, anticonvulsants, and electrical stimulation, are options available to central pain patients.

**Visceral pain –** see Nociceptive Pain.

*Sources*:

American Pain Foundation. *Treatment Options: A Guide for People Living with Pain*. Available at www.painfoundation.org.

"Pain: Hope Through Research," National Institutes of Neurological Disorders and Stroke. Publication date December 2001. NIH Publication No. 01-2406 . Last updated July 31, 2008.

# Pain Resources

American Pain Foundation
A United Voice of Hope and Power over Pain

**American Pain Foundation**
888-615-7246
www.painfoundation.org

## Key Publications

- *Pain Community News*, APF's quarterly newsletter
- *Pain Monitor*, APF's monthly e-news update
- *Treatment Options: A Guide for People Living with Pain*
- *Pain Resource Guide: Getting the Help You Need*
- *Targeting Chronic Pain Notebook* and companion provider resources
- APF Report, *Provider Prescribing Patterns and Perceptions: Identifying Solutions to Build Consensus on Opioid Use in Pain Management*
- Fact sheets on cancer pain, shingles/PHN, fibromyalgia, military/veterans and pain, among others
- Top 10 Tips Series, including:
  - Finding Quality Health Information Online
  - Exercising for Pain Relief
  - Making the Most of Your Medical Visits
  - Easing Pain Around the Holidays
  - Pain-free Tips for Travelers
  - Helpful Hints on the Road to Pain Relief
- Pain Care Bill of Rights

## Special Projects/Initiatives

- APF's grassroots **Power Over Pain Action Network** (POPAN) has 72 POPAN leaders in 36 states tirelessly working to help improve pain care, legislation related to pain care, healthcare access and medical practices.
- **Military and Veterans Pain Initiative**
- **Spotlight Series** on cancer pain, fibromyalgia and shingles
- **Pain & Creativity**
- **Yoga for Chronic Pain**
- ***Let's Talk Pain*** Coalition, www.letstalkpain.org, launched in partnership with the American Academy of Pain Management and the American Society for Pain Management Nursing

**For more information about APF's programs and services, see the 2007 Annual Report at http://www.painfoundation.org/About/2007AnnualReport.pdf**

**To subscribe to print or online publications, please visit www.painfoundation.org, or call Tina Register, Communications Manager, at (443) 690-4707 or tregister@painfoundation.org**

## Other Consumer Pain Associations

**American Chronic Pain Association**
800-533-3231
www.theacpa.org

**National Pain Foundation**
303-783-8899
www.nationalpainfoundation.org

## Condition-Specific Pain Organizations

The American Pain Foundation keeps an updated and searchable listing of condition-specific patient advocacy and professional organizations at www.painfoundation.org. These include such groups as the Amputee Coalition of America, Arthritis Foundation, the National Vulvodynia Association, National Fibromyalgia Association and the American Diabetes Association, among others.

## Professional Pain Associations

**Alliance of State Pain Initiatives**
608-262-0978
E-mail: aspi@mailplus.wisc.edu

**American Academy Hospice and Palliative Medicine**
847-375-4712
www.aahpm.org

**American Academy of Pain Management**
209-533-9744
www.aapainmanage.org

**American Academy of Pain Medicine**
847-375-4731
www.painmed.org

**American Pain Society**
847-375-4715
www.ampainsoc.org

**American Society of Addiction Medicine**
301-656-3920
www.asam.org

**American Society for Pain Management Nursing**
913-895-4606
www.aspmn.org

**National Hospice & Palliative Care Organization**
703-837-1500
www.nhpco.org

## Other organizations

**Pain Policy Studies**

**Pain and Policy Studies Group (PPSG)**
608-263-7662
www.painpolicy.wisc.edu

**Pain Law Studies**

**Pain and the Law**
617-262-4990
www.painandthelaw.org

**The Legal Side of Pain**
865-560-1945
www.legalsideofpain.com

**National Association of Attorneys General**
www.naag.org

**Drug Abuse/Addiction Groups**

**National Institute on Drug Abuse**
301-443-1124
www.nida.nih.gov

**Drug Enforcement Administration**
**Office of Diversion Control**
800-882-9539
www.deadiversion.usdoj.gov

**Partnership for a Drug-Free America**
212-922-1560
http://drugfreeamerica.com

**Substance Abuse and Mental Health Services Administration (SAMHSA)**
877-726-4727
www.samhsa.gov

**White House Office of National Drug Control Policy**
800-666-3332
www.whitehousedrugpolicy.gov

**Others**

**Center for Practical Bioethics**
800-344-3829
www.practicalbioethics.org

**Federation of State Medical Boards (FSMB)**
817-868-4000
www.fsmb.org

**National Family Caregivers Association**
301-942-6430
www.nfcacares.org



*"Pain is inevitable. Suffering is optional."*

*—Anonymous*



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# EXHIBIT F

Pain Medicine

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# Risk Factors for Serious Prescription Opioid-Related Toxicity or Overdose among Veterans Health Administration Patients

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## Abstract

**Objective. Prescription opioid use and deaths related to serious toxicity, including overdose, have increased dramatically in the United States since 1999. However, factors associated with serious opioid-related respiratory or central nervous system (CNS) depression or overdose in medical users are not well characterized. The objective of this study was to examine the factors associated with serious toxicity in medical users of prescription opioids.**

**Design. Retrospective, nested, case-control analysis of Veterans Health Administration (VHA) medical, pharmacy, and health care resource utilization administrative data.**

**Subjects. Patients dispensed an opioid by VHA between October 1, 2010 and September 30, 2012 (N = 8,987).**

**Methods. Cases (N = 817) experienced life-threatening opioid-related respiratory/CNS depression or overdose. Ten controls were randomly assigned to each case (N = 8,170). Logistic regression was used to examine associations with the outcome.**

**Results. The strongest associations were maximum prescribed daily morphine equivalent dose (MED) ≥ 100 mg (odds ratio [OR] = 4.1, 95% confidence interval [CI], 2.6–6.5), history of opioid dependence (OR = 3.9, 95% CI, 2.6–5.8), and hospitalization during the 6 months before the serious toxicity or overdose event (OR = 2.9, 95% CI, 2.3–3.6). Liver disease, extended-release or long-acting opioids, and daily MED of 20 mg or more were also significantly associated.**

**Conclusions. Substantial risk for serious opioid-related toxicity and overdose exists at even relatively low maximum prescribed daily MED, especially in patients already vulnerable due to underlying demographic factors, comorbid conditions, and concomitant use of CNS depressant medications or substances. Screening patients for risk, providing education, and coprescribing naloxone for those at elevated risk may be effective at reducing serious opioid-related respiratory/CNS depression and overdose in medical users of prescription opioids.**

**Key Words. Prescription; Opioid; Toxicity; Overdose; Risk; Predictors**



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## Introduction

Serious toxicity, including overdose, related to prescription opioid analgesics has increased dramatically in the United States since 1999 [1,2]. Opioids are central nervous system (CNS) depressants. Life-threatening opioid toxicity includes profound sedation/coma and severe respiratory depression that can result in death from respiratory arrest [3–5]. Prescription drug "overdose" is a type of serious toxicity in which the drug is used in amounts that exceed the individual's ability to tolerate the exposure, resulting in serious adverse effects. Serious opioid-related respiratory/CNS depression can occur unintentionally in patients using them for approved therapeutic indications ("medical users") and even at dosages in the recommended prescribing range. Medical users taking opioids as prescribed may experience circumstances that predispose them to opioid accumulation, prolonged duration of action or enhanced CNS, and consequent respiratory depression. Examples include certain comorbid conditions (e.g., impaired renal, hepatic, or respiratory function) and concomitant medications or substances (e.g., sedatives, alcohol). Opioid pain relievers were involved in nearly 17,000 deaths in 2010, representing a threefold increase since 1999 and three-fourths of all prescription drug poisoning deaths [1,2]. The alarming upward trajectory of fatal unintentional overdoses parallels increases of 29–80% in the use of prescription opioids for long-term management of chronic noncancer pain (2000–2005) in an estimated 9 million US adults per year currently [6–11]. Approximately 60% of overdoses occur in medical users of maximum prescribed daily morphine equivalent doses (MED) of 100 mg or more[1] or those who misuse opioid analgesics, typically prescribed by a single physician, to manage chronic pain [7]. The remaining 40% of overdoses occur in nonmedical users who abuse prescription opioids for recreational purposes, tend to receive prescriptions from multiple prescribers, and engage in diversion of prescription opioids to and from others [7,12,13].

The factors associated with *fatal* opioid-related overdose have been well characterized [2,7,12,14–21]. Patient-related factors include certain demographic characteristics and clinical comorbidities. Men have a higher opioid-related overdose death rate [12], but the percentage rise in deaths since 1999 is greater in women [15]. For prescription opioids, overdose death rates are highest in persons aged 45–54 years; non-Hispanic whites, American Indians and Alaskan Natives; rural and impoverished areas; and in the West and Southwest United States and the Appalachian states of Kentucky and West Virginia where the opioid analgesic prescribing rates are highest [2,8,12,18,19,22,23]. Geographic variations in overdose deaths reflect, in part, variations in opioid analgesic prescribing patterns, the number of physicians available, and state-regulated pain management policies rather than inherent patient differences [2,24]. Persons with a history of substance abuse, previous overdose, mental illness, and respiratory disease are significantly more likely to die of an opioid-related overdose [7,16,20,21,25,26]. Prescription drug-related factors significantly associated with fatal opioid-related overdose or serious toxicity include use of oxycodone, methadone, and extended-release formulations [7,12,16,18,27–29]; maximum prescribed daily MED exceeding 50 mg [21,25]; and concurrent use of other psychoactive CNS depressants (e.g., sedative–hypnotics, anxiolytics, alcohol) [1,20,26,30].

To date, most research on predictors of serious opioid-related toxicity or overdose has focused on fatal events, illicit opioids, nonmedical users of prescription opioids, or limited samples of medical users and used relatively small, geographically limited convenience samples. The objective of this study was to identify the factors independently most associated with overdose or life-threatening respiratory/CNS depression, including nonfatal events, among medical users of prescription opioids in a large, national, administrative health care database.

## Methods

### *Study Design*

A nested case-control design was used to examine factors associated with a diagnosis of serious opioid-related respiratory/CNS depression or overdose among Veterans Health Administration (VHA) patients who were dispensed an opioid by VHA. The study was exempt from Institutional Review Board review.

### *Study Setting and Data Source*

A retrospective analysis of deidentified national administrative health care data was conducted using VHA Medical SAS datasets from October 1, 2010 through September 30, 2012. These datasets contain data for VHA-provided health care that is utilized primarily by US military veterans and a small number of nonveterans (e.g., employees, eligible family members, research participants) and include inpatient, outpatient, laboratory, radiology, pharmacy, vital signs, vital status, and enrollment information.

### *Study Sample*

Study "cases" were defined as patients who satisfied the following criteria at any time between October 1, 2010 and September 30, 2012 (the "study period"): 1) were dispensed at least one opioid prescription by VHA (see Appendix I), identified by national drug code; and 2) had a claim for a serious opioid-related toxicity or overdose event based on International Classification of Disease, 9th Revision, Clinical Modification (ICD-9-CM) codes and Current Procedure Terminology (CPT) codes (Table 1) [7,21,31]. A serious opioid-related toxicity or overdose event was defined as follows: 1) a listed CNS or respiratory adverse effect code in addition to a listed poisoning event or external cause code occurring within ±1 day of

[1]Maximum prescribed daily morphine equivalent doses exceeding 200 mg are considered 'high-dose.' [55]



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**Table 1** Diagnostic codes for serious opioid-related toxicity including overdose

| ICD-9-CM Diagnosis Codes | Description |
|---|---|
| Poisoning codes | |
| 965.00 | Poisoning by opium (alkaloids), unspecified |
| 965.01 | Poisoning by heroin |
| 965.02 | Poisoning by methadone |
| 965.09 | Poisoning by other opiates and related narcotic |
| Adverse effect codes | |
| 518.81 | Acute respiratory failure |
| 518.82 | Other pulmonary insufficiency, not elsewhere classified |
| 780.0 | Alteration of consciousness |
| 786.03 | Apnea |
| 799.0 | Asphyxia and hypoxemia |
| CPT codes for mechanical ventilation or critical care | |
| 31500 | Intubation, endotracheal, emergency procedure |
| 94002 | Ventilation assist and management, initiation of pressure, or volume preset ventilators for assisted or controlled breathing; hospital inpatient/observation, each subsequent day |
| 94660 | CPAP ventilation; initiation and management |
| 94662 | CNAP ventilation; initiation and management |
| 99291 | Critical care, evaluation, and management of the critically ill or critically injured patient, first 30–74 minutes |
| External cause codes | |
| E850.0 | Accidental poisoning by heroin |
| E850.1 | Accidental poisoning by methadone |
| E850.2 | Accidental poisoning by other opiates and related narcotics |
| E935.0 | Adverse effects of heroin |
| E935.1 | Adverse effects of methadone |
| E935.2 | Adverse effects of other opioids and related narcotics |

CNAP = continuous negative airway pressure; CPAP = continuous positive airway pressure; ICD-9-CM, International Classification of Disease, 9th Revision, Clinical Modification.

the adverse effect; or 2) use of mechanical ventilation or critical care in addition to a listed poisoning event or external cause code occurring within ±1 day of the critical respiratory support. The first identified occurrence of opioid-related overdose or life-threatening respiratory/CNS depression ("index event") during the study period served as the "index date" for cases. All patients were required to have nonmissing age, sex, and race values in addition to continuous medical and pharmacy benefits in the 6 months before the index date (the "baseline period"). For cases, the follow-up period was calculated as the number of days after the end of the event until death, disenrollment, or the end of the study period.

For each case, 10 control patients were randomly selected and assigned from those who 1) were dispensed an opioid by VHA during the study period; 2) did not experience serious opioid-related toxicity or overdose as defined in the study; and 3) had complete data for age, sex, and race. The case index date was assigned to each of the 10 control patients, and the follow-up period for these controls was the number of days thereafter until death, disenrollment, or the end of the study period.

*Baseline Variables*

Baseline demographic variables included age group (18–34, 35–44, 45–54, 55–64, 65+ years), sex, race, marital status, body mass index, and the US Census region of the patient's VHA treatment center (Northeast, North Central, South, West, other). Baseline comorbidity measures included the Charlson Comorbidity Index score, calculated as the sum of assigned comorbidity category weights [32].

Other selected baseline comorbidities were stratified as pain-related and nonpain-related [33–36]. Pain-related comorbid conditions included low back disorders, other back/neck disorders, neuropathic disorders, fibromyalgia, headache/migraine, burns, traumatic injury, and motor vehicle accidents. Nonpain-related comorbidities included psychoactive substance use disorders including substance abuse and nonopioid substance dependence, tobacco use disorder, post-traumatic stress disorder, bipolar disorder, attention deficit hyperactivity disorder, schizophrenia, anxiety disorder, obsessive-compulsive–disorder, cardiovascular disease, endocarditis, viral and alcoholic hepatitis), pancreatitis, sexually transmitted



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disease, herpes simplex infection, skin infections/abscesses, sleep apnea, and obesity.

Additional baseline variables included the number of opioid prescriptions dispensed by VHA, opioid used (categorized by active ingredient, formulation [extended-release/long-acting vs short-acting], and route [oral, parenteral, transdermal, other]), and the maximum prescribed daily MED. For each opioid prescription dispensed during the baseline period, the product of the number of units dispensed and the opioid strength per unit (milligrams) was divided by the number of days supplied. The resulting opioid daily dose dispensed (milligrams per day) was then multiplied by a conversion factor derived from published sources to estimate the daily dose in morphine equivalents (MED) (see Table 2) [37–42]. The maximum prescribed daily MED during the baseline period was calculated for each patient by summing the daily MED for all opioid prescriptions dispensed to the patient during those 6 months. It reflects the maximum *prescribed* daily dose and not necessarily the actual amount *consumed*. Nonopioid medications also dispensed by VHA which can potentiate opioid-associated serious adverse effects, such as psychoactive drugs and nonopioid analgesics, were included as baseline variables [1,28]. Baseline health care utilization measures included the number of inpatient admissions and outpatient emergency department (ED), office, and pharmacy visits (Table 6).

*Outcome Variable*

The occurrence of serious opioid-related toxicity or overdose as defined by listed ICD-9-CM and CPT codes was the primary outcome variable (Table 1). All analyses were conducted at the patient level. For patients who experienced more than one episode of serious opioid-related respiratory/CNS depression or overdose during the study period, only the index event was evaluated.

*Statistical Analysis*

Baseline covariates and the outcome measure were summarized descriptively. Tests for normality were conducted, and medians and interquartile ranges (IQRs) were calculated for continuous variables that were not normally distributed. Frequencies and percentages were calculated for categorical variables. Student's *t*-tests or Wilcoxon Rank Sum tests were used, as appropriate, to examine differences in continuous variables of interest between cases and controls. Chi-squared tests of proportion were used to examine bivariate associations for categorical variables.

Multivariable analysis was performed using conditional logistic regression to examine factors potentially associated with the index event of serious opioid-related toxicity or overdose. The covariates included in the regression model were age, sex, race/ethnicity, marital status, US Census region, comorbidities, prescription opioid characteristics, the maximum prescribed daily MED, selected nonopioid prescription medications, and baseline health care resource utilization. Odds ratios (ORs) and corresponding 95% confidence intervals (CIs) and *P* values were calculated. *P* values less than 0.05 were considered statistically significant. One full, main effects, logistic regression model was run. Model discrimination was evaluated by the c-statistic which reflects the area under the receiver operating characteristic curve and ranges from 0.5 (no discrimination between cases and controls) to 1.0 (perfect discrimination) [41]. [43] Only the first (index) event of serious opioid-related toxicity or overdose was modeled for case patients who experienced more than one episode during the study period.

**Table 2** Prescription opioids and morphine equivalent conversion factors

| Opioid* | Morphine Equivalent Conversion Factor†,‡ (per mg of Opioid) |
|---|---|
| Short acting | |
| Meperidine hydrochloride | 0.1 |
| Codeine | 0.15 |
| Tramadol | 0.2 |
| Hydrocodone | 1.0 |
| Morphine sulfate | 1.0 |
| Oxycodone | 1.5 |
| Oxymorphone | 3.0 |
| Hydromorphone | 4.0 |
| Fentanyl citrate (transmucosal) | 0.13§ |
| Extended release/long-acting | |
| Morphine sulfate extended-release | 1.0 |
| Oxycodone hydrochloride controlled-release | 1.5 |
| Methadone | 3.0 |
| Fentanyl (transdermal) | 2.4¶, ** |

* Some drug products contained an opioid in combination with a nonopioid (e.g., acetaminophen, aspirin) (Appendix II). No MED was calculated for the two controls who used sublingual buprenorphine.

† Sources of morphine equivalent conversion factors: Von Korff [37] and Leppert and Luczak [69].

‡ For each opioid dispensed, the daily MED (mg per day) was calculated as follows (see text): (number of units dispensed × strength of unit [mg] × MED conversion factor)/number of days supply.

§ Converting transmucosal fentanyl to morphine equivalents assumes 50% bioavailability of transmucosal fentanyl and that 100 μg of transmucosal fentanyl is equivalent to 12.5–15 mg of morphine.

¶ Converting transdermal fentanyl to morphine equivalents assumes that each patch has a conversion factor of 2.4 and remains in place for 3 days. The daily MED (mg per day) was calculated as follows:
(number of patches dispensed × 3 days per patch × strength of patch [μg/h] × MED conversion factor)/number of days supply.

** Prescription Drug Monitoring Program Training and Technical Assistance Center (no specific author) [41].

MED = morphine equivalent dose.



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All analyses were conducted using SAS version 9.3 [44].

## Results

Sixteen patients were excluded from the analysis due to missing age, sex, or race data. We identified 921 patients with a claim of life-threatening opioid-related respiratory/CNS depression or overdose and at least 6 months of continuous medical and pharmacy benefits before the event, 817 of whom were also dispensed an opioid by VHA. Among these 817 cases, 16 experienced more than one episode during the study period. Among those who received an opioid prescription from VHA during the study period, 8,170 control patients without overdose were identified who met selection criteria (Figure 1).

### *Descriptive Analysis*

The median age was 62 years for both cases and controls (IQR, 10 and 16, respectively). As shown in Table 3, cases were more likely than controls to be non-Hispanic white, divorced, separated or widowed, and to reside in the western US Census region.

Compared with controls, patients with serious opioid-related toxicity or overdose were more likely to be diagnosed with other diseases and health conditions. The mean CCI score, which reflects general health status, was higher for cases than for controls (3.9 vs 1.7, $P < 0.0001$), indicating poorer overall health in the cases. As shown in Table 4, cases reported particularly significantly higher frequency during the 6-month baseline period of chronic pulmonary disease (e.g., emphysema, chronic bronchitis, asthma, pneumoconiosis, asbestosis); depression; skin ulcers; hypertension; malignancy; opioid dependence, substance abuse, nonopioid substance dependence, and tobacco use disorder; viral hepatitis; mental health disorders including anxiety, post traumatic stress, and bipolar disorders; cardiovascular disease; sleep apnea; back and neck disorders; neuropathic disorders; and traumatic injury (e.g., fracture, dislocation, contusion, laceration, wound).

VHA prescription drug dispensing data during the 6-month baseline period indicated that, overall, cases were prescribed opioids significantly more often than controls, in larger variety, with a higher proportion of extended-release or long-acting (ER/LA) formulations and with a higher mean maximum prescribed daily MED (Table 5). The mean number of opioid prescriptions dispensed in the baseline period was 6.8 among cases, compared with 2.5 among controls ($P < 0.0001$). Prescription opioid active ingredients varied significantly between cases and controls, with more hydrocodone, methadone, oxycodone, and morphine, but less tramadol, dispensed to cases than controls. Both ER/LA and short-acting formulations as well as oral opioids were prescribed to cases more often than to controls. The mean maximum prescribed MED was 122 mg per day in cases and 48 mg per day in controls, with significantly more cases receiving prescriptions for MED ≥50 mg per day and ≥100 mg per day. All selected nonopioid drugs were also prescribed to cases significantly more often than to controls (Table 5).

As shown in Table 6, cases had significantly greater health care resource utilization than controls during the baseline period, including outpatient office and ED visits, hospital-

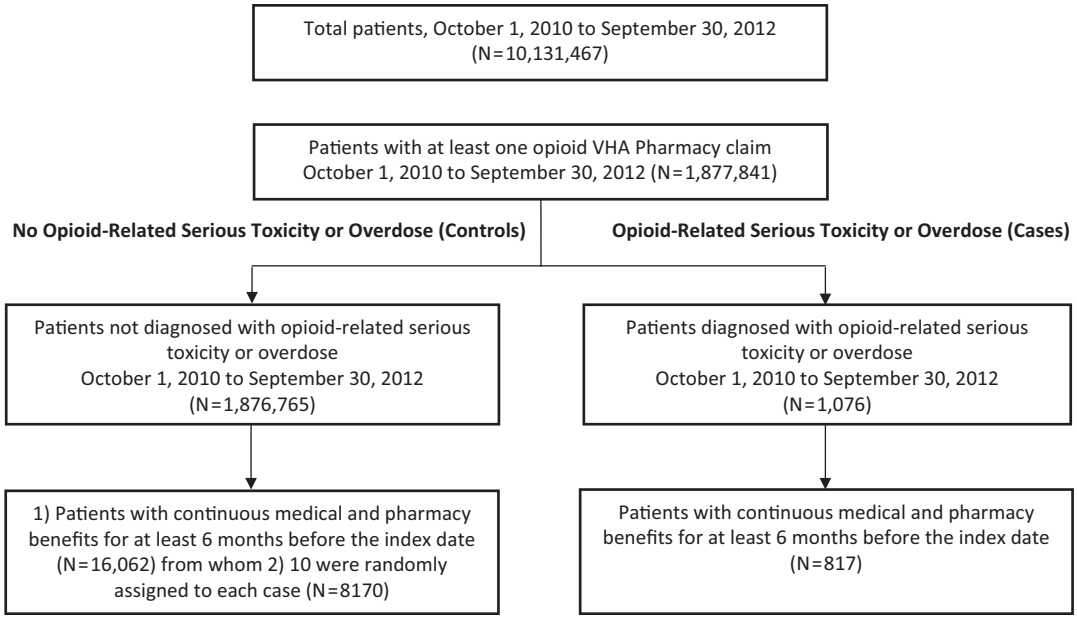



**Figure 1** Sample selection flowchart.



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**Table 3** Baseline demographic characteristics

| | Cases (N = 817) | | Controls (N = 8,170) | | |
|---|---|---|---|---|---|
| Characteristics | N | SD, % | N | SD, % | *P* |
| Age (years), median (IQR) | 62.0 | 10.0 | 62.0 | 16.0 | <0.001 |
| Age group (years) | | | | | |
| 18–34 | 27 | 3.3 | 565 | 6.9 | <0.001 |
| 35–44 | 31 | 3.8 | 619 | 7.6 | <0.001 |
| 45–54 | 115 | 14.1 | 1,240 | 15.2 | 0.402 |
| 55–64 | 377 | 46.1 | 2,672 | 32.7 | <0.001 |
| 65+ | 267 | 32.7 | 3,074 | 37.6 | 0.005 |
| Male | 753 | 92.2 | 7,528 | 92.1 | 0.980 |
| Race/ethnicity | | | | | |
| Non-Hispanic white | 555 | 67.9 | 4,546 | 55.6 | <0.001 |
| Non-Hispanic black | 83 | 10.2 | 1,300 | 15.9 | <0.001 |
| Hispanic | 32 | 3.9 | 431 | 5.3 | 0.094 |
| Other | 147 | 18.0 | 1,893 | 23.2 | 0.001 |
| Marital status | | | | | |
| Never married | 102 | 12.5 | 1,227 | 15.0 | 0.052 |
| Married | 351 | 43.0 | 4,246 | 52.0 | <0.001 |
| Separated | 20 | 2.5 | 41 | 0.5 | <0.001 |
| Divorced | 285 | 34.9 | 2,268 | 27.8 | <0.001 |
| Widowed | 59 | 7.2 | 388 | 4.8 | 0.002 |
| BMI ($kg/m^2$) | | | | | |
| Underweight (<18.5) | 29 | 3.6 | 72.0 | 0.9 | <0.001 |
| Normal (18.5–24.9) | 193 | 23.6 | 1,197 | 14.7 | <0.001 |
| Overweight (25.0–29.9) | 224 | 27.4 | 2,070 | 25.3 | 0.193 |
| Obese (≥30.0) | 306 | 37.5 | 2,667 | 32.6 | 0.005 |
| Missing | 65 | 8.0 | 2,164 | 26.5 | <0.001 |
| US Census region | | | | | |
| Northeast | 75 | 9.2 | 824 | 10.1 | 0.411 |
| North Central | 190 | 23.3 | 1,745 | 21.4 | 0.208 |
| South | 270 | 33.1 | 3,258 | 39.9 | <0.001 |
| West | 257 | 31.5 | 1,842 | 22.6 | <0.001 |
| Other | 25 | 3.1 | 501 | 6.1 | <0.001 |

BMI = body mass index; IQR = interquartile range; SD = standard deviation.

izations, and pharmacy visits. An ED visit occurred during the baseline period in 65% of cases compared with 21% of controls. Nearly half of the cases were hospitalized during the baseline period at least once compared with 9% of controls.

During the 2-year study period, 159/817 case patients died (19.5%) compared with 282/8,170 controls (3.5%). Twenty of the deaths in the case patients occurred during a VHA-treated episode of serious toxicity or overdose for an index event fatality rate of 2.4% (20/817).

*Multivariable Analysis*

The logistic regression model for the dichotomous outcome of serious opioid-related respiratory/CNS depression or overdose resulted in multiple, independent, statistically significant associations. To improve the estimate stability, the marital status categories "separated" (N = 20/817 cases) and "divorced" (N = 285/817 cases) were combined into one category. Endocarditis was not included in the final logistic regression as it was reported in only one case patient. The final model yielded a c-statistic of 0.89. As displayed in Figure 2, significant independent demographic predictors of serious opioid toxicity included ages 55–64 years and 65 and above, non-Hispanic white race, never married, widowed, and those receiving care in the western region of the United States.

Concomitant health conditions that were most strongly associated with the occurrence of serious opioid-related toxicity or overdose were opioid dependence, moderate or severe liver disease, skin ulcers, metastatic solid tumor, and pancreatitis. Other comorbidities significantly associated with the outcome included renal disease, bipolar disorder, traumatic injury chronic pulmonary disease, warfarin use, substance abuse, and sleep apnea (Figure 3).

Prescription opioids containing hydromorphone or oxycodone and those with ER/LA formulations were



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**Table 4** Baseline clinical characteristics

| | Cases (N = 817) | | Controls (N = 8,170) | | |
|---|---|---|---|---|---|
| Comorbidities | N | SD, % | N | SD, % | *P* |
| CCI score, mean (SD) | 3.9 | 3.3 | 1.7 | 2.0 | <0.001 |
| Individual CCI comorbidities | | | | | |
| Myocardial infarction | 28 | 3.4 | 105 | 1.3 | <0.001 |
| Congestive heart failure | 93 | 11.4 | 308 | 3.8 | <0.001 |
| Peripheral vascular disease | 71 | 8.7 | 353 | 4.3 | <0.001 |
| Cerebrovascular disease | 57 | 7.0 | 343 | 4.2 | <0.001 |
| Dementia | 5 | 0.6 | 32 | 0.4 | 0.348 |
| Chronic pulmonary disease | 291 | 35.6 | 1,047 | 12.8 | <0.001 |
| Rheumatologic disease | 6 | 0.7 | 96 | 1.2 | 0.257 |
| Peptic ulcer disease | 9 | 1.1 | 63 | 0.8 | 0.312 |
| Mild liver disease | 43 | 5.3 | 64 | 0.8 | <0.001 |
| Diabetes | 263 | 32.2 | 1,850 | 22.6 | <0.001 |
| Hypertension | 495 | 60.6 | 3,670 | 44.9 | <0.001 |
| Depression | 357 | 43.7 | 1,562 | 19.1 | <0.001 |
| Use of warfarin | 78 | 9.6 | 387 | 4.7 | <0.001 |
| Hemiplegia or paraplegia | 13 | 1.6 | 34 | 0.4 | <0.001 |
| Renal disease | 112 | 13.7 | 428 | 5.2 | <0.001 |
| Any malignancy, including leukemia and lymphoma | 147 | 18.0 | 646 | 8.0 | <0.001 |
| Diabetes with chronic complications | 92 | 11.3 | 432 | 5.3 | <0.001 |
| Skin ulcers | 122 | 14.9 | 302 | 3.7 | <0.001 |
| Moderate or severe liver disease | 28 | 3.4 | 19 | 0.2 | <0.001 |
| Metastatic solid tumor | 46 | 5.6 | 59 | 0.7 | <0.001 |
| HIV/AIDS | 11 | 1.4 | 42 | 0.5 | 0.003 |
| Other selected comorbidities | | | | | |
| Nonpain related | | | | | |
| Substance abuse and nonopioid substance dependence | 215 | 26.3 | 764 | 9.4 | <0.001 |
| Opioid dependence | 105 | 12.9 | 97 | 1.2 | <0.001 |
| Endocarditis | 1 | 0.1 | 9 | 0.1 | 0.920 |
| Viral hepatitis | 106 | 13.0 | 249 | 3.0 | <0.001 |
| Alcoholic hepatitis | 3 | 0.4 | 5 | 0.1 | 0.005 |
| Pancreatitis | 24 | 2.9 | 49 | 0.6 | <0.001 |
| Sexually transmitted disease | 12 | 1.5 | 69 | 0.8 | 0.072 |
| Herpes simplex infection | 7 | 0.9 | 45 | 0.6 | 0.272 |
| Skin infections/abscesses | 85 | 10.4 | 286 | 3.5 | <0.001 |
| Sleep apnea | 147 | 18.0 | 652 | 8.0 | <0.001 |
| Tobacco use disorder | 301 | 36.8 | 1,266 | 15.5 | <0.001 |
| PTSD | 221 | 27.1 | 1,119 | 13.7 | <0.001 |
| Bipolar disorder | 86 | 10.5 | 239 | 2.9 | <0.001 |
| ADHD | 7 | 0.9 | 58 | 0.7 | 0.637 |
| Schizophrenia | 36 | 4.4 | 114 | 1.4 | <0.001 |
| Anxiety disorder | 180 | 22.0 | 681 | 8.3 | <0.001 |
| OCD | 5 | 0.6 | 19 | 0.2 | 0.045 |
| Cardiovascular disease | 172 | 21.1 | 764 | 9.4 | <0.001 |
| Obesity | 150 | 18.4 | 1,072 | 13.1 | <0.001 |
| Pain related | | | | | |
| Low back disorders | 380 | 46.5 | 2,099 | 25.7 | <0.001 |
| Other back/neck disorders | 214 | 26.2 | 1,048 | 12.8 | <0.001 |
| Neuropathic disorders | 170 | 20.8 | 717 | 8.8 | <0.001 |
| Fibromyalgia | 34 | 4.2 | 157 | 1.9 | <0.001 |
| Headache/migraine | 88 | 10.8 | 427 | 5.2 | <0.001 |
| Burns | 4 | 0.5 | 16 | 0.2 | 0.089 |
| Traumatic injury | 212 | 26.0 | 869 | 10.6 | <0.001 |
| Motor vehicle accidents | 7 | 0.9 | 14 | 0.2 | <0.001 |

ADHD = attention deficit hyperactivity disorder; AIDS = acquired immunodeficiency syndrome; CCI = Charlson Comorbidity Index, 2008 updated (score); OCD = obsessive–compulsive disorder; PTSD = post-traumatic stress disorder; SD = standard deviation.



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**Table 5** Baseline prescription drugs dispensed by VHA

| | Cases (N = 817) | | Controls (N = 8,170) | | |
|---|---|---|---|---|---|
| Prescription Drug Use | N | SD, % | N | SD, % | *P* |
| Opioid use | 693 | 84.8 | 4,936 | 60.4 | <0.001 |
| By active ingredient | | | | | |
| Hydrocodone | 314 | 38.4 | 2,633 | 32.2 | <0.001 |
| Oxycodone | 305 | 37.3 | 876 | 10.7 | <0.001 |
| Buprenorphine | 0 | 0.0 | 2 | 0.0 | 0.655 |
| Tramadol | 114 | 14 | 1,428 | 17.5 | 0.011 |
| Codeine | 63 | 7.7 | 561 | 6.9 | 0.365 |
| Fentanyl | 49 | 6.0 | 44 | 0.5 | <0.001 |
| Morphine | 251 | 30.7 | 334 | 4.1 | <0.001 |
| Hydromorphone | 38 | 4.7 | 28 | 0.3 | <0.001 |
| Methadone | 107 | 13.1 | 139 | 1.7 | <0.001 |
| Oxymorphone | 1 | 0.1 | 1 | 0.0 | 0.044 |
| Other* | 2 | 0.2 | 4 | 0.1 | 0.039 |
| By formulation | | | | | |
| ER/LA‡ | 369 | 45.2 | 499 | 6.1 | <0.001 |
| Short acting‡ | 633 | 77.5 | 4,807 | 58.9 | <0.001 |
| Proportion of opioids = ER/LA† | 0.25 | 0.3 | <0.1 | 0.2 | <0.001 |
| By route | | | | | |
| Oral | 692 | 84.7 | 4,923 | 60.3 | <0.001 |
| Parenteral | 6 | 0.7 | 6 | 0.1 | <0.001 |
| Transdermal | 48 | 5.9 | 44 | 0.5 | <0.001 |
| Number of opioid prescriptions dispensed, mean (SD) | 6.8 | 5.9 | 2.5 | 3.4 | <0.001 |
| Number of unique opioid NDCs, mean (SD) | 2.4 | 1.9 | 0.9 | 1.1 | <0.001 |
| Maximum prescribed daily MED (mg), mean (SD) | 98.7 | 122.1 | 24.2 | 48.4 | <0.001 |
| 1–<20 | 35 | 4.3 | 1,331 | 16.3 | <0.001 |
| 20–<50 | 227 | 27.8 | 2,614 | 32.0 | 0.014 |
| 50–<100 | 163 | 20.0 | 718 | 8.8 | <0.001 |
| ≥100 | 268 | 32.8 | 273 | 3.3 | <0.001 |
| Selected nonopioid drugs | 747 | 91.4 | 5,905 | 72.3 | <0.001 |
| Benzodiazepines | 336 | 41.1 | 1,242 | 15.2 | <0.001 |
| Antidepressants | 565 | 69.2 | 2,886 | 35.3 | <0.001 |
| Nonopioid analgesics | 556 | 68.1 | 4,598 | 56.3 | <0.001 |
| Muscle relaxants | 226 | 27.7 | 1,288 | 15.8 | <0.001 |
| Other sedatives | 125 | 15.3 | 609 | 7.5 | <0.001 |
| Antipsychotics | 239 | 29.3 | 772 | 9.5 | <0.001 |
| Stimulants | 14 | 1.7 | 51 | 0.6 | <0.001 |

* Other opioids include meperidine and pentazocine/naloxone.
† Proportion of opioid prescriptions dispensed to a patient during baseline that contained an ER/LA formulation. Methadone is a long-acting opioid.
‡ Percentages exceed 100% due to prescription of both ER/LA and short-acting formulations in some patients.
ER/LA = extended release/long acting; MED = morphine equivalent dose; NDC = National drug code; SD = standard deviation; VHA = Veterans Health Administration.

significantly associated with increased risk of serious opioid-related toxicity or overdose. The likelihood of experiencing the outcome was related monotonically to increasing maximum prescribed daily MED of 20 mg or higher. Patients prescribed a maximum daily MED ≥100 mg during the baseline period were more than four times as likely to experience serious opioid-related toxicity or overdose compared with those prescribed MED of 1–<20 mg/day, whereas patients prescribed 50–<100 mg/day MED were 2.2 times as likely, and those prescribed 20–49 mg/day MED were 1.5 times as likely to experience life-threatening opioid-related respiratory/CNS depression or overdose (Figure 4). Coprescription of benzodiazepines, antidepressants, and antipsychotics in opioid users was significantly associated with experiencing serious toxicity or overdose.

Patients hospitalized for one or more days for any reason during the baseline period were nearly three times as likely to experience serious opioid-related toxicity compared with those who were not hospitalized.



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**Table 6** Baseline health care utilization

| All-Cause Health Care Utilization | Cases (N = 817) | | Controls (N = 8,170) | | |
|---|---|---|---|---|---|
| | N | SD, % | N | SD, % | *P* |
| Days of hospitalization, mean (SD) | 9.6 | 22.9 | 1.1 | 8.0 | <0.001 |
| Patients with ≥1 outpatient ED visit | 534 | 65.4 | 1,740 | 21.3 | <0.001 |
| Patients with ≥1 outpatient office visit | 792 | 96.9 | 7,333 | 89.8 | <0.001 |
| Patients with ≥1 inpatient hospitalization | 396 | 48.5 | 739 | 9.1 | <0.001 |
| Patients with ≥1 prescription fill | 800 | 97.9 | 7,561 | 92.6 | <0.001 |
| Outpatient ED visits per patient, mean (SD) | 2 | 2.6 | 0.4 | 1 | <0.001 |
| Outpatient office visits per patient, mean (SD) | 23 | 18.6 | 9.8 | 11.3 | <0.001 |
| Inpatient hospitalizations per patient, mean (SD) | 1 | 1.5 | 0.1 | 0.5 | <0.001 |
| Pharmacy visits per patient, mean (SD) | 24.6 | 15.0 | 12.9 | 10.4 | <0.001 |

ED = emergency department; SD = standard deviation.

Full regression results, including factors that were not statistically significantly related to the outcome in the logistic model are provided in Appendix II.

## Discussion

Our study produced a robust multivariable model that characterized the risk of life-threatening opioid-related respiratory/CNS depression or overdose in medical users of prescription opioids. Higher maximum prescribed daily MED, a history of opioid dependence, and hospitalization during the 6 months before the overdose or serious toxicity event were the factors most strongly associated with this outcome among an opioid-exposed cohort of predominantly US veterans. Consistent with published findings on prescription opioid overdose *deaths*, we found that certain demographic characteristics, comorbid conditions, and medication-related factors were associated with non-fatal prescription opioid-related serious toxicity or overdose as well [16,21,45]. Demographic variables previously identified as risk factors, and confirmed in the present study, included non-Hispanic white race, never married and widowed marital status, and residence in the Western United States [2,12,16,21,24,45,46]. These factors are likely to be proxies for underlying patient-related constructs, including genetic influences on drug metabolism; the social environment, such as isolation; the prescriber, including opioid-prescribing patterns; and the

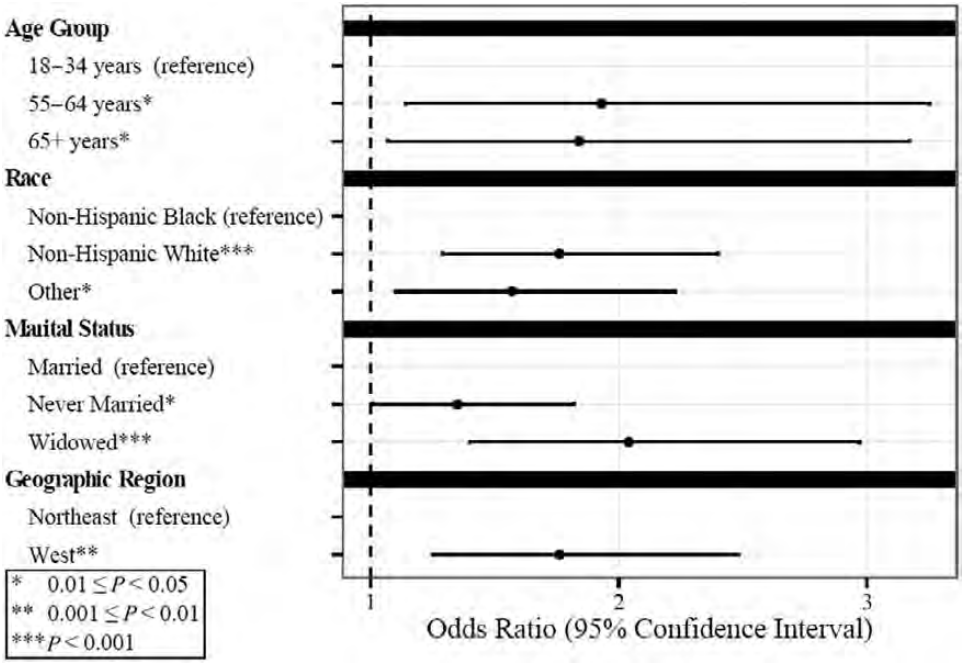



**Figure 2** Logistic regression results: significant demographic factors.



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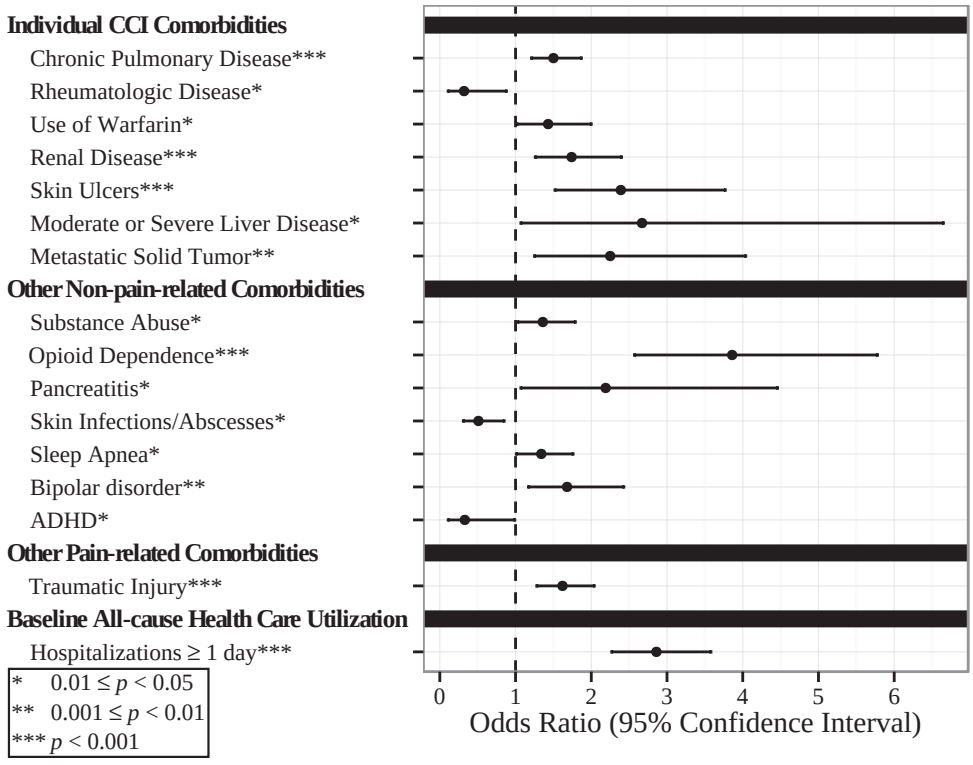



**Figure 3** Logistic regression results: significant comorbid conditions and health care utilization factors.

health care system, such as access to emergency care and other medical services [24,47–52].

Some of our findings differed from those of studies of fatal opioid overdose. In contrast to the typical occurrence of opioid overdose death in middle age (peaking at 45–54 years), most case patients in our study were aged 55 years or older [8,12]. This discrepancy likely reflects the older VHA population. The older age predominance also affected the pattern of comorbidity in our study population, with chronic diseases and cancer being prevalent. Physiologically older individuals have age-related impairment in the hepatic and renal ability to metabolize and excrete certain drugs and other substances and have a greater burden of disease and associated potentially interacting concomitant medications. Such individuals are biologically vulnerable to opioid accumulation and to experiencing toxicity even when using an opioid well within its recommended dosing range. The safe use of opioids long-term to manage chronic pain in elderly patients is particularly challenging [39,53–55].

Another reported treatment challenge observed in our study population was the strong association of serious respiratory/CNS depression or overdose with substance use disorders (dependence and abuse) and mental health disorders (bipolar disorder). Abuse of alcohol, illicit opioids, and other substances is more frequent among medical users of prescription opioids than in the general population or chronic pain patients not treated with opioids [11,33,56]. We observed that polypharmacy with psychoactive drugs commonly prescribed for mental health disorders, such as benzodiazepines, antidepressants, and antipsychotics, as well as mental illness itself, was involved in approximately one-half of overdose events [1,13,56,57]. The association between serious toxicity events among opioid users in this study and pharmacotherapy for mental health disorders such as depression and anxiety may be partially mediated by the substantially higher prevalence of substance use disorders [56].

We found certain opioid characteristics to be highly associated with the likelihood of experiencing opioid-related toxicity or overdose. Use of extended-release formulations and long-acting opioids was strongly associated with an increased likelihood of overdose events, as reported previously [27,30,58]. Methadone, a long-acting opioid, was also examined as an independent determinant due to its long half-life, variable pharmacokinetics, and disproportionate involvement in 30–40% of all opioid-related deaths despite accounting for only 5–19% of US opioids prescribed [7,12,18,27,59]. In contrast to other studies that focused exclusively on *fatal* overdose, methadone alone was not independently associated with serious respiratory/CNS depression or overdose events treated at VHA facilities, falling just short of the statistical significance threshold ($P = 0.08$). It is unclear whether this difference is due to the study sample's relatively low prevalence of



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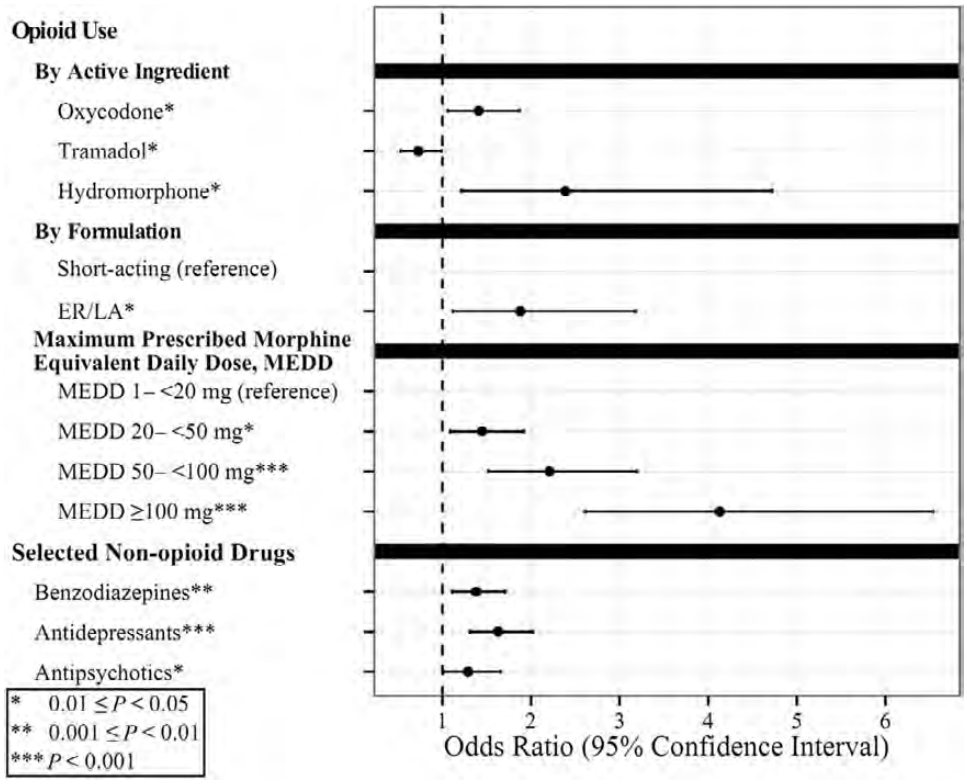



**Figure 4** Logistic regression results: significant prescription drug-related factors.

methadone dispensed and fatal outcomes or to other characteristic(s) of the study sample or model specification.

Our study confirmed the known dose-related toxicity of opioids. Importantly, maximum prescribed daily MED of as little as 20 mg was associated with serious fatal and nonfatal overdose and toxicity in opioid consumers overall. Previous research identified a significant risk of overdose death for daily MED of ≥20 or 50 mg in patients with chronic noncancer pain [21,25,60]. An increasing body of scientific evidence suggests that prescriber overreliance on, and inadequate proficiency using, opioid dose conversion factors or ratios in published equianalgesic dose tables to calculate MED is an important contributor in fatal or near-fatal opioid-related CNS/respiratory depression [27,38,61–64]. The numerous published equianalgesic tables that are widely available contain inconsistent and variable conversion ratios. To reduce the risk of unintentional serious toxicity when rotating or switching opioids, updated guidelines for the safe use of equianalgesic dose tables emphasize the need to consider the opioid conversion ratio or calculated equianalgesic dose in morphine equivalents as only an approximate starting point. The calculated MED must then be adjusted for each individual patient and clinical scenario by accounting for interindividual sources of variation that can alter opioid potency. Such sources of individual variation include demographic differences (age, sex, race, ethnicity), major organ impairment (liver, kidney, adrenal), potential interactions with concomitant nonopioid medications and substances (e.g., benzodiazepines, alcohol), the direction of the opioid switch, and incomplete cross tolerance between opioids, as well as differences in the likelihood of opioid-induced hyperalgesia and physical dependence. Some differences may be due to significant genetic variants in opioid receptors, metabolism, and transport in the nervous system [65,66]. The current guidelines are based on expert opinion and have not been validated for safety or efficacy [55,61,64,67].

Of note, medical use of tramadol in our study appeared to be protective against serious opioid-related overdose (OR 0.7, 95% CI, 0.5–1.0). Tramadol, a novel synthetic opioid analgesic with monamine reuptake inhibition contributing to its analgesic effect, has low mu opioid receptor binding affinity and is not currently regulated as a controlled substance at the federal level in the United States [68–71]. However, its US prescribing information contains warnings similar to all prescription opioids regarding the risk of CNS and respiratory depression, overdose, and death. This interesting study finding warrants further investigation.

Pain is a complex, multidimensional condition with a multiplicity of interacting and contributing influences [72]. Factors involved in the likelihood of serious opioid-related toxicity or overdose in individuals treated for painful conditions relate to the patient and their social environment,



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prescriber or other source of opioids, health care system, and the specific opioid and other exposures. Although pain is the most common reason a patient seeks medical care, current US data on the incidence, prevalence, and treatment of pain are not complete or consistent, partly because it is considered a symptom. Recent evidence indicates that approximately 80% of episodes of pain treated with opioids are short term [37]. However, an estimated 100–116 million US adults suffer chronic pain [53,72], and 3–4% of the adult population (9 million) are prescribed opioids each year to manage chronic noncancer pain [6–9,11]. Thus, the total population at risk of life-threatening opioid-related respiratory/CNS depression or overdose is substantial.

Our study confirms and extends findings from prior research that focused on fatal overdoses but did not differentiate between medical and nonmedical opioid users [13,16–18,21,60]. The relatively low frequency of fatal outcomes of serious opioid-related events in our VHA-treated cohort (2.4% over 2 years) suggests that the majority of such events in medical users is not fatal. However, nonfatal events do place a substantial burden on the health care system and patients [73,74].

### *Strengths and Limitations*

Major strengths of this study include the large, national patient population as well as the rich detail of VHA administrative data. The robust statistical model included variables that are readily available from medical and pharmacy claims data. In contrast to most previous research, we examined in a comprehensive and systematic fashion the determinants associated with nonfatal as well as fatal serious toxicity and overdose related to the medical use of prescription opioids. However, the study sample included all opioid-exposed patients and was stratified by neither therapeutic indication or acuity (e.g., acute vs chronic pain conditions; chronic pain related to cancer vs noncancer) nor by the duration of opioid treatment (short term vs long term), partly to avoid potential statistical challenges with the limited number of cases available. Our study was subject to many of the limitations commonly associated with observational studies using administrative data (e.g., limited ability to infer causality and limited access to information regarding actual medication consumption/adherence, other behavioral/social elements, and therapeutic indication, with the potential for residual confounding). In addition, while VHA provides a large, national population from which to sample, generalizability is limited as the population comprises primarily older, white men who receive most of their health care within a single, closed system.

Limitations in accuracy and completeness are inherent in administrative data and include missing data, coding errors, misclassification, and undiagnosed or undocumented comorbidities such as substance use disorders. While prescriptions dispensed within the VHA system are well documented, it is possible that patients in the study also consumed opioids and other medications or substances from unreported non-VHA sources, particularly in the case cohort which had a significantly higher prevalence of substance use disorders. In addition, the serious respiratory/CNS depression and overdose rate in this sample is likely an underestimate as we evaluated only cases that fulfilled a stringent case definition and came to medical attention within VHA.

### *Implications for Future Research*

Future studies should assess the generalizability of these findings to populations more representative of US medical users of prescription opioids, including wider age ranges and more women. With a larger dataset, selected interactions among risk factors should be evaluated, as well as the predictive utility of behavioral and other factors not routinely captured in administrative health care data (e.g., use of alcohol and other substances, other sources of opioids, therapeutic indication, social conditions, setting of the overdose or serious toxicity event, family history). Potential differences in risk factor profiles for overdose or life-threatening respiratory/CNS depression among those treated with opioids for acute vs chronic conditions, chronic noncancer pain vs chronic cancer pain, and short term vs long term should also be explored.

## Conclusions

The risk of life-threatening toxicity, including overdose, in medical users of prescription opioids is an alarming, escalating public health problem. Substantial risk exists when even relatively low daily MED of opioids is used in patients who are vulnerable due to sociodemographic factors, concomitant medical and psychiatric conditions, and simultaneous use of other medications or substances. Expert guidelines recommend screening all patients before initiating opioids for pain management to identify those at elevated risk for serious adverse outcomes [26,55,75]. An extensive literature review revealed several available instruments to screen for aberrant drug-related behaviors (abuse, addiction, diversion) [55,76], but no instruments that provide useful, real-time, evidence-based information to the prescriber regarding the risk of overdose or serious respiratory/CNS depression currently exist [55]. A public health imperative is the identification of medical users of prescription opioids who are at highest risk of life-threatening toxicity for whom additional precautions should be considered. These precautions include education of the patient and caregivers, increased caution in opioid selection and dose escalation, consultation with pain management specialists, and close monitoring for the emergence of opioid-related toxicity or known risk factors for this outcome [28,55,75]. Additional measures to reduce opioid-related morbidity and mortality may include enhanced training and compliance of health care providers with evidence-based best practices for prescribing opioids, such as considering coprescribing naloxone, particularly if delivery systems can be developed that are safe and more user friendly for nonmedical first responders than the current syringe or nasal atomizer-based systems. Naloxone is a rescue medication with more than three



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decades of proven effectiveness and safety in reversing life-threatening opioid-related respiratory/CNS depression or overdose [3,7,74,77–80].

The results of our study indicate that a statistically robust model based on administrative medical, pharmacy, and health care resource utilization data may help identify the demographic characteristics, comorbid conditions, concomitant medications, and opioid-related factors associated with increased risk of life-threatening toxicity and overdose. These factors help to identify the individuals most likely to benefit from preventive interventions. The development and widespread use of a risk profiling questionnaire based on these factors to guide patient treatment decisions would have the potential to significantly improve the balance between the analgesic benefit of opioid therapy and the risks of serious toxicity or overdose and other adverse outcomes, including abuse, diversion for nonmedical use, and iatrogenic addiction.

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**References**

1 Jones CM, Mack KA, Paulozzi LJ. Pharmaceutical overdose deaths, United States, 2010. JAMA 2013; 309(7):657–9.

2 Centers for Disease Control and Prevention. Vital signs: Overdoses of prescription opioid pain relievers—United States, 1999–2008. MMWR Morb Mortal Wkly Rep 2011;60(43):1487–92.

3 SAMHSA Opioid Overdose Prevention Toolkit: Information for Prescribers. Rockville, MD: Substance Abuse and Mental Health Services Administration, 2013. HHS Publication No. (SMA) 13-4742.

4 Jungquist CR, Karan S, Perlis ML. Risk factors for opioid-induced excessive respiratory depression. Pain Manag Nurs 2011;12(3):180–7.

5 Stephens E, Louden M, VanDe Voort J, et al. Opioid Toxicity: Medscape. 2012. Available at: http://emedicine.medscape.com/article/815784-overview (accessed December 2013); updated October 23, 2012, accessed December 8 2013.

6 Boudreau D, Von Korff M, Rutter CM, et al. Trends in long-term opioid therapy for chronic non-cancer pain. Pharmacoepidemiol Drug Saf 2009;18(12):1166–75.

7 CDC Grand Rounds. Prescription Drug Overdoses—A US Epidemic. MMWR Morb Mortal Wkly Rep 2012;61(1):10–3.

8 Manchikanti L, Fellows B, Ailinani H, Pampati V. Therapeutic use, abuse, and nonmedical use of opioids: A ten-year perspective. Pain Physician 2010;13(5):401–35.

9 Parsells Kelly J, Cook SF, Kaufman DW, et al. Prevalence and characteristics of opioid use in the US adult population. Pain 2008;138(3):507–13.

10 Sullivan MD, Edlund MJ, Fan MY, et al. Trends in use of opioids for non-cancer pain conditions 2000–2005 in commercial and Medicaid insurance plans: The TROUP study. Pain 2008;138(2):440–9.

11 Sullivan MD, Edlund MJ, Steffick D, Unutzer J. Regular use of prescribed opioids: Association with common psychiatric disorders. Pain 2005;119(1–3):95–103.

12 Warner M, Chen LH, Makuc DM, Anderson RN, Minino AM. Drug poisoning deaths in the United States, 1980–2008. NCHS Data Brief 2011;(81):1–8.

13 Paulozzi LJ, Kilbourne EM, Shah NG, et al. A history of being prescribed controlled substances and risk of drug overdose death. Pain Med 2012;13(1):87–95.

14 American Academy of Pain Medicine and the American Pain Society. The use of opioids for the treatment of chronic pain. A consensus statement from the American Academy of Pain Medicine and the American Pain Society. Clin J Pain 1997;13:6–8.

15 Centers for Disease Control and Prevention. Vital Signs. Overdoses of prescription opioid pain relievers and other drugs among women—United States, 1999–2010. Morb Mortal Wkly Rep 2013; 62(26):537–42.

16 Lanier WA, Johnson EM, Rolfs RT, Friedrichs MD, Grey TC. Risk factors for prescription opioid-related death, Utah, 2008–2009. Pain Med 2012;13(12):1580–9.

17 Madadi P, Hildebrandt D, Lauwers AE, Koren G. Characteristics of opioid-users whose death was related to opioid-toxicity: a population-based study in Ontario, Canada. PLoS One. 2013;8(4):e60600.

18 Paulozzi LJ, Logan JE, Hall AJ, et al. A comparison of drug overdose deaths involving methadone and other opioid analgesics in West Virginia. Addiction 2009; 104(9):1541–8.

19 Wunsch MJ, Nakamoto K, Behonick G, Massello W. Opioid deaths in rural Virginia: A description of the high prevalence of accidental fatalities involving prescribed medications. Am J Addict 2009;18(1):5–14.



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on 26 January 2018

20 Hall AJ, Logan JE, Toblin RL, et al. Patterns of abuse among unintentional pharmaceutical overdose fatalities. JAMA 2008;300(22):2613–20.

21 Bohnert AS, Valenstein M, Bair MJ, et al. Association between opioid prescribing patterns and opioid overdose-related deaths. JAMA 2011;305(13):1315–21.

22 Substance Abuse and Mental Health Services Administration. Results from the 2009 National Survey on Drug Use and Health: volume 1: summary of national findings. Rockville, MD: Substance Abuse and Mental Health Services Administration, 2010.

23 Coolen P, Best S. Overdose Deaths Involving Prescription Opioids Among Medicaid Enrollees—Washington, 2004–2007. Atlanta, Ga: Centers for Disease Control and Prevention, 2009 Contract No.: 42.

24 McDonald DC, Carlson K, Izrael D. Geographic variation in opioid prescribing in the U.S. J Pain 2012; 13(10):988–96.

25 Dunn KM, Saunders KW, Rutter CM, et al. Opioid prescriptions for chronic pain and overdose: A cohort study. Ann Intern Med 2010;152(2):85–92.

26 Joint Commission on Accreditation of Health Care Organizations. Safe use of opioids in hospitals: Joint Commission on Accreditation of Health Care Organizations. 2012. Available at: http://www.jointcommission.org/assets/1/18/SEA_49_opioids_8_2_12_final.pdf (accessed December 2013).

27 Webster LR, Cochella S, Dasgupta N, et al. An analysis of the root causes for opioid-related overdose deaths in the United States. Pain Med 2011;12(suppl 2):S26–35.

28 Nuckols TK, Anderson L, Popescu I, et al. Opioid prescribing: A systematic review and critical appraisal of guidelines for chronic pain. Ann Intern Med 2013; 160(1):38–47.

29 Braden JB, Russo J, Fan MY, et al. Emergency department visits among recipients of chronic opioid therapy. Arch Intern Med 2010;170(16):1425–32.

30 Morasco BJ, Duckart JP, Carr TP, Deyo RA, Dobscha SK. Clinical characteristics of veterans prescribed high doses of opioid medications for chronic non-cancer pain. Pain 2010;151(3):625–32.

31 Centers for Disease Control and Prevention. ICD-9-CM Official Guidelines for Coding and Reporting: Centers for Disease Control and Prevention. Available at: http://www.cdc.gov/nchs/data/icd9/icd9cm_guidelines_2011.pdf (accessed November 2013).

32 Charlson ME, Charlson RE, Peterson JC, et al. The Charlson comorbidity index is adapted to predict costs of chronic disease in primary care patients. J Clin Epidemiol 2008;61(12):1234–40.

33 Baser O, Xie L, Mardekian J, et al. Prevalence of Diagnosed Opioid Abuse and its Economic Burden in the Veterans Health Administration. Pain Pract 2013.

34 Fleming MF, Balousek SL, Klessig CL, Mundt MP, Brown DD. Substance use disorders in a primary care sample receiving daily opioid therapy. J Pain 2007; 8(7):573–82.

35 Walker JM, Farney RJ, Rhondeau SM, et al. Chronic opioid use is a risk factor for the development of central sleep apnea and ataxic breathing. J Clin Sleep Med 2007;3(5):455–61.

36 Xie L, Joshi AV, Schaaf D, et al. Differences in Healthcare Utilization and Associated Costs Between Patients Prescribed vs. Nonprescribed Opioids During an Inpatient or Emergency Department Visit. Pain Pract 2013.

37 Von Korff M, Saunders K, Ray GT, Boudreau D, Campbell D, Merrill J, et al. Defacto long-term opioid therapy for non-cancer pain. Clin J Pain 2008; 24(6):521–7.

38 Vieweg WV, Lipps WF, Fernandez A. Opioids and methadone equivalents for clinicians. Prim Care Companion J Clin Psychiatry 2005;7(3):86–8.

39 Miaskowski C, Bair M, Chou R, D'Arcy Y, Hartwick C, Huffman L, et al., Principles of Analgesic Use in the Treatment of Acute Pain and Cancer Pain. Sixth ed. Glenview, IL: American Pain Society; 2008. p. 19–38.

40 Fine PG, Portenoy RK. A Clinical Guide to Opioid Analgesia. Minneapolis: McGraw Hill; 2004.

41 Technical Assistance Guide No. 01-13: Calculating Daily Morphine Milligram Equivalents: Prescription Drug Monitoring Program Training and Technical Assistance Center. 2013. Available at: http://www.pdmpassist.org/pdf/BJA_performance_measure_aid_MME_conversion.pdf (accessed March 2014). updated Feb 28 2013.

42 Prescription Drug Monitoring Program Training and Technical Assistance Center. Technical Assistance Guide No. 02-13: Daily Morphine Milligram Equivalents Calculator and Guide.: Prescription Drug Monitoring Program Training and Technical Assistance Center. 2013. Available at: http://www.pdmpassist.org/pdf/bja_performance_measure_aid_mme_conversion_tool.pdf (accessed March 2014). updated May 1 2013.



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43 Cook NR. Use and misuse of the receiver operating characteristic curve in risk prediction. Circulation 2007;115(7):928–35.

44 SAS. 9.3. SAS Institute. Cary, NC. 2013.

45 Cerda M, Ransome Y, Keyes KM, et al. Prescription opioid mortality trends in New York City, 1990–2006: Examining the emergence of an epidemic. Drug Alcohol Depend 2013;132(1–2):53–62.

46 Johnson EM, Lanier WA, Merrill RM, et al. Unintentional prescription opioid-related overdose deaths: Description of decedents by next of kin or best contact, Utah, 2008–2009. J Gen Intern Med 2013;28(4):522–9.

47 Johnson JA. Ethnic differences in cardiovascular drug response: Potential contribution of pharmacogenetics. Circulation 2008;118(13):1383–93.

48 Joung IM, van de Mheen H, Stronks K, van Poppel FW, Mackenbach JP. Differences in self-reported morbidity by marital status and by living arrangement. Int J Epidemiol 1994;23(1):91–7.

49 Joung IM, van der Meer JB, Mackenbach JP. Marital status and health care utilization. Int J Epidemiol 1995;24(3):569–75.

50 Mor A, Ulrichsen SP, Svensson E, Berencsi K, Thomsen RW. Does marriage protect against hospitalization with pneumonia? A population-based case-control study. Clin Epidemiol 2013;5:397–405.

51 Aizer AA, Chen MH, McCarthy EP, et al. Marital status and survival in patients with cancer. J Clin Oncol 2013;31(31):3869–76.

52 Sammon JD, Morgan M, Djahangirian O, et al. Marital status: A gender-independent risk factor for poorer survival after radical cystectomy. BJU Int 2012;110(9):1301–9.

53 National Institute on Drug Abuse. Prescription Drugs: Abuse and Addiction. Older Adults. Available at: www.drugabuse.gov/publications/research-reports/prescription-drugs/trends-in-prescription-drug-abuse/older-adults (accessed October 2013).

54 American Geriatrics Society Panel on Pharmacological Management of Persistent Pain in Older P. Pharmacological management of persistent pain in older persons. J Am Geriatr Soc 2009;57(8):1331–46.

55 Chou R, Fanciullo GJ, Fine PG, et al. Clinical guidelines for the use of chronic opioid therapy in chronic noncancer pain. J Pain 2009;10(2):113–30.

56 Edlund MJ, Sullivan M, Steffick D, Harris KM, Wells KB. Do users of regularly prescribed opioids have higher rates of substance use problems than nonusers? Pain Med 2007;8(8):647–56.

57 Seal KH, Shi Y, Cohen G, et al. Association of mental health disorders with prescription opioids and high-risk opioid use in US veterans of Iraq and Afghanistan. JAMA 2012;307(9):940–7.

58 Food and Drug Administration. FDA Blueprint for Prescriber Education for Extended-Release and Long-Acting Opioid Analgesics. US Food and Drug Administration. 2013.

59 Centers for Disease Control and Prevention. Vital Signs: Risk for overdose from methadone used for pain relief—United States, 1999–2010. MMWR Morb Mortal Wkly Rep 2012;61(26):493–7.

60 Gomes T, Juurlink DN, Dhalla IA, et al. Trends in opioid use and dosing among socio-economically disadvantaged patients. Open Med 2011;5(1):e13–22.

61 Fine PG, Portenoy RK, Ad Hoc Expert Panel on Evidence R, Guidelines for Opioid R. Establishing "best practices" for opioid rotation: Conclusions of an expert panel. J Pain Symptom Manage 2009;38(3):418–25.

62 McNicol E. Opioid equianalgesic conversions. J Pain Palliat Care Pharmacother 2009;23(4):459.

63 Shaheen PE, Walsh D, Lasheen W, Davis MP, Lagman RL. Opioid equianalgesic tables: Are they all equally dangerous? J Pain Symptom Manage 2009;38(3):409–17.

64 Webster LR, Fine PG. Review and critique of opioid rotation practices and associated risks of toxicity. Pain Med 2012;13(4):562–70.

65 Kalvass JC, Olson ER, Cassidy MP, Selley DE, Pollack GM. Pharmacokinetics and pharmacodynamics of seven opioids in P-glycoprotein-competent mice: Assessment of unbound brain EC50,u and correlation of in vitro, preclinical, and clinical data. J Pharmacol Exp Ther 2007;323(1):346–55.

66 Liang DY, Liao G, Lighthall GK, Peltz G, Clark DJ. Genetic variants of the P-glycoprotein gene Abcb1b modulate opioid-induced hyperalgesia, tolerance and dependence. Pharmacogenet Genomics 2006;16(11):825–35.

67 Pereira J, Lawlor P, Vigano A, Dorgan M, Bruera E. Equianalgesic dose ratios for opioids. a critical review and proposals for long-term dosing. J Pain Symptom Manage 2001;22(2):672–87.



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68 US Veterans' Health Affairs Administration. Veterans Administration/Department of Defense Clinical Practice Guideline for Management of Opioid Therapy for Chronic Pain. 2010. Available at: http://www.va.gov/painmanagement/docs/cpg_opioidtherapy_fulltext.pdf (accessed March 2014).

69 Leppert W, Luczak J. The role of tramadol in cancer pain treatment—A review. Support Care Cancer 2005;13(1):5–17.

70 Drug Enforcement Administration. Tramadol, Drug and Chemical Evaluation. US Drug Enforcement Administration, Office of Diversion Control. August 29 2013. Available at: http://www.deadiversion.usdoj.gov/drug_chem_info/tramadol.pdf (accessed March 2014).

71 Tramadol Label. Daily Med: Current Medication Information. National Library of Medicine. Available at: http://dailymed.nlm.nih.gov/dailymed/lookup.cfm?setid=45f59e6f-1794-40a4-8f8b-3a9415924468 (accessed March 2014).

72 Institute of Medicine. Relieving Pain in America: A Blueprint for Transforming Prevention, Care, Education, and Research. Washington, DC: The National Academies Press; 2011.

73 Inocencio TJ, Carroll NV, Read EJ, Holdford DA. The economic burden of opioid-related poisoning in the United States. Pain Med 2013;14(10):1534–1547.

74 Albert S, Brason FW, Sanford CK, et al. Project Lazarus: Community-based overdose prevention in rural North Carolina. Pain Med 2011;12(suppl 2):S77–85.

75 Furlan AD, Reardon R, Weppler C. Opioids for chronic noncancer pain: A new Canadian practice guideline. CMAJ 2010;182(9):923–30.

76 Passik SD, Kirsh KL, Casper D. Addiction-related assessment tools and pain management: Instruments for Screening, treatment planning, and monitoring compliance. Pain Med 2008;9:S145–66.

77 Beletsky L, Rich JD, Walley AY. Prevention of fatal opioid overdose. JAMA 2012;308(18):1863–4.

78 Wheeler E, Davidson PJ, Jones TS, Irwin KS. Community-Based Opioid Overdose Prevention Programs Providing Naloxone—United States, 2010. Atlanta, Ga: Centers for Disease Control and Prevention; 2012.

79 Kim D, Irwin KS, Khoshnood K. Expanded access to naloxone: Options for critical response to the epidemic of opioid overdose mortality. Am J Public Health 2009;99(3):402–7.

80 American Medical Association. AMA Adopts New Policies at Annual Meeting: American Medical Association. 2012. Available at: https://www.ama-assn.org/ama/pub/news/news/2012-06-19-ama-adopts-new-policies.page (accessed December 2013).

## Appendices

*Appendix I*

Prescription opioid drug products

| Active Ingredient(s) by Generic Name |
|---|
| Alfentanil hydrochloride |
| Buprenorphine |
| Butorphanol tartrate |
| Codeine, acetaminophen |
| Codeine base |
| Codeine phosphate |
| Codeine phosphate, triprolidine, pseudoephedrine hydrochloride |
| Codeine phosphate, chlorpheniramine maleate |
| Codeine phosphate, guaifenesin, pseudophedrine |
| Codeine phosphate, pyrilamine maleate |
| Codeine phosphate, acetaminophen, gamma-aminobutyric acid |
| Codeine phosphate, brompheniramine maleate, pseudoephedrine hydrochloride |
| Codeine phosphate, brompheniramine maleate, phenylephrine hydrochloride |
| Codeine phosphate, butalbital, acetaminophen, caffeine |
| Codeine phosphate, butalbital, aspirin, caffeine |
| Codeine phosphate, carisoprodol, aspirin |



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*Appendix I Continued*

| Active Ingredient(s) by Generic Name |
|---|
| Codeine phosphate, chlorcyclizine hydrochloride |
| Codeine phosphate, dexbrompheniramine maleate, pseudoephedrine hydrochloride |
| Codeine phosphate, guaifenesin, pseudoephedrine hydrochloride |
| Codeine phosphate, phenylephrine hydrochloride |
| Codeine phosphate, phenylephrine hydrochloride, diphenhydramine hydrochloride |
| Codeine phosphate, phenylephrine hydrochloride, chlorcyclizine hydrochloride |
| Codeine phosphate, phenylephrine hydrochloride, chlorpheniramine maleate |
| Codeine phosphate, phenylephrine hydrochloride, pyrilamine maleate |
| Codeine phosphate, promethazine hydrochloride |
| Codeine phosphate, promethazine hydrochloride, phenylephrine hydrochloride |
| Codeine phosphate, pseudoephedrine hydrochloride, chlorcyclizine HCl |
| Codeine phosphate, pseudoephedrine hydrochloride, chlorpheniramine maleate |
| Codeine phosphate, pseudoephedrine hydrochloride |
| Codeine phosphate, pseudoephedrine hydrochloride, pyrilamine maleate |
| Codeine sulfate |
| Dihydrocodeine bitartrate, acetaminophen, caffeine |
| Dihydrocodeine bitartrate, brompheniramine maleate, phenylephrine hydrochloride |
| Dihydrocodeine bitartrate, brompheniramine maleate, pseudoephedrine hydrochloride |
| Dihydrocodeine bitartrate, guaifenesin |
| Dihydrocodeine bitartrate, phenylephrine hydrochloride, guaifenesin |
| Dihydrocodeine bitartrate, phenylephrine hydrochloride, pyrilamine maleate |
| Fentanyl |
| Fentanyl citrate, bupivacaine HCl |
| Hydrocodone bitartrate, acetaminophen |
| Hydrocodone bitartrate, homatropine methylbromide |
| Hydrocodone bitartrate, ibuprofen |
| Hydrocodone bitartrate, chlorpheniramine maleate, pseudoephedrine hydrochloride |
| Hydrocodone bitartrate, pseudoephedrine hydrochloride |
| Hydrocodone polistirex, chlorpheniramine polistirex |
| Hydrocodone, acetaminophen, gamma-aminobutyric acid |
| Hydromorphone hydrochloride |
| Levorphanol tartrate |
| Meperidine hydrochloride |
| Methadone hydrochloride |
| Morphine |
| Nalbuphine hydrochloride |
| Naloxone, buprenorphine |
| Oxycodone, acetaminophen |
| Oxycodone, aspirin |
| Oxycodone hydrochloride |
| Oxycodone hydrochloride, ibuprofen |
| Oxymorphone hydrochloride |
| Pentazocine hydrochloride, acetaminophen |
| Pentazocine hydrochloride, naloxone hydrochloride |
| Pentazocine lactate |
| Propoxyphene hydrochloride |
| Propoxyphene hydrochloride, acetaminophen |
| Propoxyphene napsylate |
| Propoxyphene napsylate, acetaminophen |
| Sufentanil citrate |
| Tapentadol |
| Tramadol hydrochloride |
| Tramadol hydrochloride, acetaminophen |
| Tramadol hydrochloride, gamma-aminobutyric acid |
| Tramadol hydrochloride, acetaminophen |



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*Appendix II*

Logistic regression results: serious opioid-related toxicity or overdose

| Covariate[†] | Odds Ratio | 95% CI | | *P* |
|---|---|---|---|---|
| Age group (years) | | | | |
| 18–34 | *Reference* | | | |
| 35–44 | 0.9 | 0.5 | 1.8 | 0.826 |
| –45–54 | 1.4 | 0.8 | 2.4 | 0.224 |
| 55–64 | 1.9 | 1.1 | 3.3 | 0.014 |
| 65+ | 1.8 | 1.1 | 3.2 | 0.028 |
| Male | 0.9 | 0.6 | 1.3 | 0.553 |
| Race | | | | |
| Non-Hispanic black | *Reference* | | | |
| Non-Hispanic white | 1.8 | 1.3 | 2.4 | <0.001 |
| Hispanic | 1.4 | 0.8 | 2.4 | 0.248 |
| Other | 1.6 | 1.1 | 2.2 | 0.013 |
| Marital status | | | | |
| Married (reference) | *Reference* | | | |
| Separated/divorced | 1.1 | 0.9 | 1.4 | 0.404 |
| Never married | 1.4 | 1.0 | 1.8 | 0.044 |
| Widowed | 2.0 | 1.4 | 3.0 | <0.001 |
| Geographic region | | | | |
| Northeast (reference) | *Reference* | | | |
| North central | 1.3 | 0.9 | 1.8 | 0.184 |
| South | 1.2 | 0.9 | 1.7 | 0.316 |
| West | 1.8 | 1.3 | 2.5 | 0.001 |
| Other | 0.7 | 0.4 | 1.2 | 0.154 |
| Comorbidity | | | | |
| Individual CCI comorbidities | | | | |
| Myocardial infarction | 0.8 | 0.4 | 1.4 | 0.345 |
| Congestive heart failure | 1.1 | 0.7 | 1.8 | 0.674 |
| Peripheral vascular disease | 1.1 | 0.8 | 1.7 | 0.502 |
| Cerebrovascular disease | 0.7 | 0.4 | 1.1 | 0.132 |
| Dementia | 1.0 | 0.3 | 3.1 | 0.977 |
| Chronic pulmonary disease | 1.5 | 1.2 | 1.9 | <0.001 |
| Rheumatologic disease | 0.3 | 0.1 | 0.9 | 0.027 |
| Peptic ulcer disease | 0.5 | 0.2 | 1.2 | 0.123 |
| Mild liver disease | 1.6 | 0.9 | 3.2 | 0.137 |
| Diabetes | 1.1 | 0.9 | 1.4 | 0.418 |
| Hypertension | 1.0 | 0.8 | 1.3 | 0.791 |
| Depression | 1.2 | 1.0 | 1.5 | 0.105 |
| Use of warfarin | 1.4 | 1.0 | 2.0 | 0.040 |
| Hemiplegia or paraplegia | 0.9 | 0.4 | 2.3 | 0.867 |
| Renal disease | 1.7 | 1.3 | 2.4 | 0.001 |
| Any malignancy, including leukemia and lymphoma | 1.3 | 1.0 | 1.7 | 0.086 |
| Diabetes with chronic complications | 1.0 | 0.7 | 1.4 | 0.769 |
| Skin ulcers | 2.4 | 1.5 | 3.8 | <0.001 |
| Moderate or severe liver disease | 2.7 | 1.1 | 6.7 | 0.036 |
| Metastatic solid tumor | 2.3 | 1.3 | 4.0 | 0.007 |
| HIV/AIDS | 2.0 | 0.8 | 4.8 | 0.120 |
| Other Selected comorbidities | | | | |
| Nonpain related | | | | |
| Substance abuse and nonopioid substance dependence | 1.4 | 1.0 | 1.8 | 0.031 |
| Opioid dependence | 3.9 | 2.6 | 5.8 | <0.001 |
| Viral hepatitis | 1.4 | 0.9 | 2.0 | 0.098 |
| Alcoholic hepatitis | 0.7 | 0.1 | 10.9 | 0.823 |
| Pancreatitis | 2.2 | 1.1 | 4.5 | 0.032 |
| Sexually transmitted disease | 1.4 | 0.6 | 3.1 | 0.458 |
| Herpes simplex infection | 0.8 | 0.3 | 2.2 | 0.639 |
| Skin infections/abscesses | 0.5 | 0.3 | 0.9 | 0.010 |
| Sleep apnea | 1.3 | 1.0 | 1.8 | 0.040 |
| Tobacco use disorder | 1.2 | 1.0 | 1.5 | 0.066 |
| PTSD | 1.0 | 0.8 | 1.3 | 0.985 |
| Bipolar disorder | 1.7 | 1.2 | 2.4 | 0.005 |



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*Appendix II Continued*

| Covariate[†] | Odds Ratio | 95% CI | | *P* |
|---|---|---|---|---|
| ADHD | 0.3 | 0.1 | 1.0 | 0.048 |
| Schizophrenia | 1.6 | 0.9 | 2.8 | 0.105 |
| Anxiety disorder | 1.1 | 0.9 | 1.5 | 0.384 |
| OCD | 0.6 | 0.1 | 2.8 | 0.552 |
| Cardiovascular disease | 1.3 | 0.8 | 2.0 | 0.243 |
| Obesity | 1.1 | 0.8 | 1.4 | 0.498 |
| Pain related | | | | |
| Low back disorders | 1.1 | 0.9 | 1.4 | 0.241 |
| Other back/neck disorders | 1.1 | 0.9 | 1.4 | 0.309 |
| Neuropathic disorders | 1.0 | 0.8 | 1.3 | 0.815 |
| Fibromyalgia | 1.2 | 0.7 | 2.0 | 0.574 |
| Headache/migraine | 1.2 | 0.8 | 1.7 | 0.322 |
| Burns | 0.8 | 0.2 | 3.8 | 0.758 |
| Traumatic injury | 1.6 | 1.3 | 2.0 | <0.001 |
| Motor vehicle accidents | 2.4 | 0.7 | 7.6 | 0.145 |
| Prescription drug use | | | | |
| Opioids | | | | |
| By active ingredient | | | | |
| Hydrocodone | 1.0 | 0.8 | 1.4 | 0.779 |
| Oxycodone | 1.4 | 1.1 | 1.9 | 0.017 |
| Tramadol | 0.7 | 0.5 | 1.0 | 0.043 |
| Codeine | 1.3 | 0.9 | 1.9 | 0.146 |
| Fentanyl | 0.8 | 0.1 | 6.9 | 0.813 |
| Morphine | 1.6 | 1.0 | 2.5 | 0.079 |
| Hydromorphone | 2.4 | 1.2 | 4.7 | 0.012 |
| Methadone | 1.6 | 1.0 | 2.7 | 0.079 |
| Oxymorphone | 0.3 | 0.0 | 5.4 | 0.377 |
| Other* | 1.7 | 0.1 | 52.5 | 0.775 |
| By formulation | | | | |
| Short acting | *Reference* | | | |
| ER/LA | 1.9 | 1.1 | 3.2 | 0.018 |
| By route | | | | |
| Oral | *Reference* | | | |
| Parenteral or transdermal | 2.3 | 0.3 | 18.7 | 0.433 |
| Number of opioid prescriptions dispensed, mean (SD) | 1.0 | 1.0 | 1.0 | 0.852 |
| Number of unique opioid NDCs, mean (SD) | 1.0 | 0.9 | 1.1 | 0.488 |
| Maximum prescribed daily morphine equivalent dose (MED, mg/day) | | | | |
| 1–<20 (reference) | *Reference* | | | |
| 20–<50 | 1.5 | 1.1 | 1.9 | 0.011 |
| 50–<100 | 2.2 | 1.5 | 3.2 | <0.001 |
| ≥100 | 4.1 | 2.6 | 6.5 | <0.001 |
| Nonopioid drugs of interest | | | | |
| Benzodiazepines | 1.4 | 1.1 | 1.7 | 0.004 |
| Antidepressants | 1.6 | 1.3 | 2.0 | <0.001 |
| Nonopioid analgesics | 0.9 | 0.7 | 1.2 | 0.557 |
| Muscle relaxants | 1.1 | 0.9 | 1.4 | 0.293 |
| Other sedatives | 1.1 | 0.8 | 1.5 | 0.521 |
| Antipsychotics | 1.3 | 1.0 | 1.7 | 0.045 |
| Stimulants | 1.9 | 0.8 | 4.6 | 0.179 |
| All-cause health care utilization during the preceding 6 months | | | | |
| Days of hospitalization | | | | |
| 0 | *Reference* | | | |
| ≥1 | 2.9 | 2.3 | 3.6 | <0.001 |

* Other opioids included meperidine and pentazocine/naloxone. Methadone is a long-acting opioid.

ADHD = attention deficit hyperactivity disorder; CCI = Charlson Comorbidity Index; CI = confidence interval; ER/LA = extended release or long acting; MED = morphine equivalent dose; NDC = National drug code; OCD = obsessive–compulsive disorder; PTSD = post-traumatic stress disorder; SD = standard deviation.

[†]Covariates with frequencies less than 10 or which prevented model convergence were not included in the full model.



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# EXHIBIT G

**STATE OF TEXAS**

**COUNTY OF HOPKINS**

**Consider and take action to approve Resolution for approval of bringing suit on behalf of Hopkins County, Texas, vs. various drug manufacturers, developers, suppliers and others of a class of pharmaceutical class of drugs commonly referred to as opioids and approval of Professional Services Agreement for Special Counsel.**

WHEREAS, it reasonably appears from public information that there exists in Hopkins County an epidemic of opioid drug abuse created and/or fueled by misconduct of opioid manufacturers and distributors in their improper promotion and over-supply of those drugs, thus creating a public nuisance within Hopkins County; and

WHEREAS, Hopkins County is empowered by law to seek recompense for its expenditures to combat public nuisance; and

WHEREAS, enforcement of the law regarding public nuisance requires Hopkins County to institute civil legal proceedings requiring legal services, and the Hopkins County Commissioners Court finds as follows:

(1) there is a substantial need for the legal services;

(2) the legal services cannot be adequately performed by the attorneys and supporting personnel of Hopkins County or by the attorneys and supporting personnel of another governmental entity; and

(3) the legal services cannot reasonably be obtained from attorneys in private practice under a contract providing only for the payment of hourly fees, without regard to the outcome of the matter, because of the nature of the matter for which the services will be obtained and because Hopkins County does not have appropriated funds available to pay the estimated amounts required under a contract providing only for the payment of hourly fees; and

WHEREAS, Hopkins County is empowered to employ Special Counsel to prosecute lawful reimbursement of monies spent by Hopkins County to combat the opioid epidemic in Hopkins County and has selected the legal team of Simon Greenstone Panatier Bartlett, P.C. and Martin Walker, P.C. to serve as Special Counsel.

THEREFORE, BE IT HEREBY RESOLVED that:

1. The Commissioner's Court approves Resolution to bring suit on behalf of Hopkins County, Texas, versus various drug manufacturers, developers, suppliers and others of a class of pharmaceutical class of drugs commonly referred to as opioids and approval of Professional Services Agreement for Special Counsel.

2. The Commissioners Court selects the legal team of Simon Greenstone Panatier Bartlett, P.C. and Martin Walker, P.C. to serve as Special Counsel and approves and adopts the terms and conditions of employment of such counsel as set out in the attached Professional Services Agreement. Special Counsel will work under the oversight and approval of the Hopkins County District Attorney or County Attorney or her designee.

3. The Commissioners Court authorizes the District Attorney or County Attorney or her designee and Special Counsel to file such claims and litigation as Special Counsel deems necessary against various drug manufactures, developers, suppliers and others of a class of pharmaceutical class of drugs commonly referred to as opioids

4. The Hopkins County Judge or his designee is authorized to execute on behalf of Hopkins County an agreement with Special Counsel (hereinafter the "Agreement") containing terms and provisions substantially similar to those contained in the attached agreement because the Court finds that there is a substantial need for the legal services of Special Counsel which cannot adequately be performed by attorneys and supporting personnel of Hopkins County or another public agency, nor can the legal services reasonably be obtained from attorneys in private practice under a contract providing only for payment of hourly fees without regard to the outcome of the matter because of the nature of the representation.

5. All fees to be paid to Special Counsel are contingent upon the recovery of the penalties, attorneys' fees and costs as provided for in the Agreement and shall be paid only from such recovery and no money shall be due or paid from the General Fund or any special fund under the Agreement.

6. All Hopkins County officials and employees are authorized to do any and all things necessary or convenient to accomplish the purposes of this order.

THIS RESOLUTION WAS ADOPTED this 13 day of November, 2017, by Commissioners' Court of Hopkins County, Texas.

County Judge

Attested: Debbie Shirley
County Clerk

COMMISSIONERS COURT OF HOPKINS COUNTY, TEXAS

# EXHIBIT H

## RETENTION AGREEMENT

WHEREAS, Hopkins County has determined that claims should be made against Purdue Pharma, L.P., Purdue Pharma, Inc., The Purdue Frederick Company, Inc., Teva Pharmaceutical Industries USA, Ltd., Cephalon, Inc., Johnson & Johnson, Janssen Pharmaceuticals, Inc., Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc., Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc., Endo Health Solutions Inc., Endo Pharmaceuticals, Inc., Allergen, PLC f/k/a Actavis, PLC, Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc., Watson Laboratories, Inc., Actavis, LLC, and Actavis Pharma, Inc. f/k/a Watson Pharma, Inc., McKesson Corporation, AmerisourceBergen Corporation, and any other entities which have engaged in violations of the Texas Controlled Substances Act and other violations of law in the fraudulent marketing and sales of certain highly addictive, opiate-derived painkillers for purposes for which they are neither safe nor effective; and



WHEREAS, Hopkins County has further determined that it is in the best interests of the County and its citizens that the County retain attorneys with signification litigation experience; and

WHEREAS, Simon Greenstone Panatier Bartlett, P.C. and Martin Walker, P.C. are experienced at such litigation and consented to represent Hopkins County in association with the Attorney General, respecting the claims and pursuant to the terms and conditions hereof.



[redacted]

[redacted]

DATED this the 21 day of September, 2017.

HOPKINS COUNTY

BY: _____
[NAME], Hopkins County Judge
Robert Newsom

SIMON GREENSTONE
PANATIER BARTLETT, P.C.

BY: _____
Jeffrey Simon, Shareholder

MARTIN WALKER, P.C.

BY: _____
Jack Walker, Shareholder; or
Reid Martin, Shareholder

# Addendum to Retention Agreement

[redacted]

[redacted]

HOPKINS COUNTY

BY: _____
Robert Newsom,
Hopkins County Judge

SIMON GREENSTONE
PANATIER BARTLETT, P.C.

BY: _____
Jeffrey Simon, Shareholder

MARTIN WALKER, P.C.

BY: _____

Reid Martin, Shareholder; or
Jack Walker, Shareholder

APPROVED BY:

OFFICE OF THE TEXAS COMPTROLLER OF PUBLIC ACCOUNTS:

By: Deputy Comptroller or his designee Date: 11-29-17

# APPENDIX G

# VAN ZANDT PETITION

Van Zandt County - District Clerk

Filed 1/2/2018 10:25 AM
Karen L. Wilson
District Clerk
Van Zandt County, Texas

Stormy Canady

**CAUSE NO.** 17-00302

| | | |
|---|---|---|
| **COUNTY OF VAN ZANDT,** | § | **IN THE DISTRICT COURT** |
| | § | |
| *Plaintiff,* | § | |
| | § | |
| vs. | § | 294 **JUDICIAL DISTRICT** |
| | § | |
| **PURDUE PHARMA, L.P.;** | § | |
| **PURDUE PHARMA, INC.;** | § | |
| **THE PURDUE FREDERICK COMPANY;** | § | |
| **JOHNSON & JOHNSON;** | § | **VAN ZANDT COUNTY, TEXAS** |
| **JANSSEN PHARMACEUTICALS, INC.;** | § | |
| **ORTHO-MCNEIL-JANSSEN PHARMACEUTICALS, INC.** n/k/a JANSSEN PHARMACEUTICALS, INC.; | § | |
| **JANSSEN PHARMACEUTICA, INC.** n/k/a JANSSEN PHARMACEUTICALS, INC.; | § | |
| **ENDO HEALTH SOLUTIONS INC.;** | § | |
| **ENDO PHARMACEUTICALS, INC.;** | § | |
| **ABBVIE INC.;** | § | |
| **KNOLL PHARMACEUTICAL COMPANY,** a wholly-owned subsidiary of ABBVIE INC.; | § | |
| **ALLERGAN PLC** f/k/a ACTAVIS PLC; | § | |
| **ALLERGAN FINANCE LLC** f/k/a ACTAVIS, INC. f/k/a WATSON PHARMACEUTICALS, INC.; | § | |
| **WATSON LABORATORIES, INC.;** | § | |
| **ACTAVIS LLC;** | § | |
| **ACTAVIS PHARMA, INC.** f/k/a WATSON PHARMA, INC.; | § | |
| **MCKESSON CORPORATION;** | § | |
| **CARDINAL HEALTH, INC.;** | § | |
| **AMERISOURCEBERGEN CORPORATION**; and | § | |
| **DOES 1 – 100, INCLUSIVE,** | § | |
| | § | |
| *Defendants.* | § | |

## PLAINTIFF'S ORIGINAL PETITION AND JURY DEMAND

**TO THE HONORABLE JUDGE OF SAID COURT:**

Plaintiff, the County of Van Zandt, Texas, by and through the undersigned attorneys, (hereinafter "Van Zandt County" or "County"), on behalf of the District Attorney for Van Zandt County, against Defendants Purdue Pharma, L.P., Purdue Pharma, Inc., The Purdue Frederick Company, Johnson & Johnson, Janssen Pharmaceuticals, Inc., Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc., Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc., Endo Health Solutions Inc., Endo Pharmaceuticals, Inc., Abbvie Inc., Knoll Pharmaceutical Company, a wholly-owned subsidiary of Abbvie Inc., Allergan PLC f/k/a Actavis PLC, Allergan Finance, LLC f/k/a Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc., Watson Laboratories, Inc., Actavis LLC, Actavis Pharma, Inc. f/k/a Watson Pharma, Inc., McKesson Corporation, Cardinal Health, Inc., AmerisourceBergen Corporation, and Does 1 – 100, alleges as follows:

## I. INTRODUCTION

1. The United States is in the midst of an opioid epidemic caused by Defendants' fraudulent marketing and sales of prescription opioids ("opioids") that has resulted in addiction, criminal activity, and loss of life. The opioid crisis has been described as "the AIDS epidemic of our generation, but even worse."[1] On October 26, 2017, President Donald Trump "declared a nationwide public health emergency to combat the opioid crisis."[2]

2. In 2016 alone, health care providers wrote more than 289 million prescriptions for

[1] David Wright, "Christie on Opioids: "This is the AIDS Epidemic of Our Generation, but even Worse," (Oct. 27, 2017), http://www.cnn.com/2017/10/27/politics/chris-christie-opioid-commission-aids-cnntv/index.html.
[2] Dan Merica, "What Trump's Opioid Announcement Means – and Doesn't Mean," (Oct. 26, 2017), http://www.cnn.com/2017/10/26/politics/national-health-emergency-national-disaster/index.html.

opioids, enough for *every adult in the United States* to have more than one bottle of pills.[3] Americans "consume 85% of all the opioids in the world" and are "the most medicated country in the world . . . ."[4]

3. Unfortunately, using opioids too often leads to addiction and overdose from opioids. In 2014, almost 2 million Americans were addicted to opioids.[5] That same year, more people died from drug overdoses than in any other year, and most overdose deaths involved an opioid. The Texas Legislature has found "that deaths resulting from the use of opioids and other controlled substances constitute a public health crisis."[6] In 2015, Texas "had the second highest total healthcare costs from opioid abuse in the nation ($1.96 billion) . . . ."[7]

4. In fact, accidental drug overdose deaths, of which reportedly at least two-thirds are opioid overdoses, are the leading cause of death for Americans under the age of 50. Accidental drug overdose deaths, predominantly from opioids, exceed the number of deaths caused by cars or guns.

5. The economic burden caused by opioid abuse in the United States is at least $78.5 billion,[8] including lost productivity and increased social services, health insurance costs, increased criminal justice presence and strain on judicial resources, and substance abuse treatment and rehabilitation.

6. This epidemic did not occur by chance. Defendants manufacture, market, distribute,

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[3] *Prevalence of Opioid Misuse*, BupPractice (Sept. 7, 2017), https://www.buppractice.com/node/15576.
[4] David Wright, "Christie on Opioids: "This is the AIDS Epidemic of Our Generation, but even Worse," (Oct. 27, 2017), http://www.cnn.com/2017/10/27/politics/chris-christie-opioid-commission-aids-cnntv/index.html.
[5] Substance Abuse and Mental Health Services Administration, National Survey on Drug Use and Health, 2014.
[6] Opinion of the Attorney General of Texas, KP-0168 (Oct. 4, 2017), *citing* Act of May 26, 2017, 85th Leg., R.S., ch. 534, §3, 2017 Tex. Sess. Law Serv. 1467, 1468.
[7] Kerry Craig, "Opioid Addiction Results in one Woman's Daily Struggle," Oct. 7, 2017, https://www.ssnewstelegram.com/news/opioid-addiction-results-in-one-woman-s-daily-struggle/article_bded4eoa-ab80-11e7-a252-d3f304e26628.html.
[8] *See CDC Foundation's New Business Pulse Focuses on Opioid Overdose Epidemic*, Centers for Disease Control and Prevention (Mar. 15, 2017), https://www.cdc.gov/media/releases/2017/a0315-business-pulse-opioids.html.

and sell prescription opioids, including, but not limited to, brand-name drugs like OxyContin, Vicodin, Opana, Percocet, Percodan, Duragesic, Ultram, Ultracet, and generics like oxycodone, oxymorphone, hydromorphone, hydrocodone, fentanyl, and tramadol, which are powerful narcotics.

7. Historically, opioids were considered too addictive and debilitating for treating non-cancer chronic pain,[9] such as back pain, migraines, and arthritis, and were used only to treat short-term acute pain or for palliative or end-of-life care.

8. By the late 1990s or early 2000s, however, each Manufacturing Defendant began a marketing scheme to persuade doctors and patients that opioids can and should be used ubiquitously and perpetually to treat moderate, non-cancer chronic pain. Each Manufacturing Defendant spent large sums of money to promote the benefits of opioids for non-cancer moderate pain while trivializing, or even denying, their risks. The Manufacturing Defendants' promotional messages deviated substantially from any approved labeling of the drugs and caused prescribing physicians and consuming patients to underappreciate the health risks, and to overestimate the benefits, of opioids.

9. Contrary to the language of their drugs' labels, Defendants falsely and misleadingly, in their marketing: (1) downplayed the serious risk of addiction; (2) promoted and exaggerated the concept of "pseudoaddiction" thereby advocating that the signs of addiction should be treated with more opioids; (3) exaggerated the effectiveness of screening tools in preventing addiction; (4) claimed that opioid dependence and withdrawal are easily managed; (5) denied the risks of higher opioid dosages; and (6) exaggerated the effectiveness of "abuse-deterrent" opioid formulations to prevent abuse and addiction.

[9] "Chronic pain" means non-cancer pain lasting three months or longer.

10. Defendants disseminated these falsehoods through ads and/or their sales representatives and physicians who supported Defendants' message. Sales representatives, working at Defendants' behest, promoted highly addictive opioids through souvenirs and toys including, but not limited to, opioid brand-bearing stuffed plush toys, dolls, coffee cups, fanny packs, water bottles, notepads, pens, refrigerator magnets, clocks, letter openers, rulers, daytime planners, bags, puzzles, posters, hand-held calculators, clipboards, highlighters, flashlights, key chains, clothing, reflex mallets, and mock-ups of the United States Constitution.

11. Defendants also used third parties they controlled by: (a) funding, assisting, encouraging, and directing doctors, known as "key opinion leaders" ("KOLs") and (b) funding, assisting, directing, and encouraging seemingly neutral and credible professional societies and patient advocacy groups (referred to hereinafter as "Front Groups").

12. Defendants worked with KOLs and Front Groups to taint the sources that doctors and patients relied on for ostensibly "neutral" guidance, such as treatment guidelines, Continuing Medical Education ("CME") programs, medical conferences and seminars, and scientific articles. After their individual and concerted efforts, Defendants convinced doctors that instead of being addictive and unsafe for long-term use in most circumstances, opioids were *required* in the compassionate treatment of chronic pain.

13. The Distributor Defendants were not standing by idly while Marketing Defendants were peddling their opioids to physicians and consumers. Cardinal, AmerisourceBergen, and McKesson ("Distributor Defendants") are three of the largest opioid distributors in the United States. Distributor Defendants purchased opioids from Manufacturing Defendants herein and sold them to pharmacies servicing consumers in Van Zandt County.

14. Despite the alarming and suspicious rise in the ordering of opioids by retailers in

Van Zandt County, Distributor Defendants did nothing. The Manufacturing Defendants and Distributor Defendants worked hand and glove to glut the U.S. and Van Zandt County with more opioids than would be consumed for therapeutic purposes. Each Defendant disregarded its legal duty to report suspicious opioid prescriptions, and each Defendant financially benefitted from the other Defendants (both Manufacturing and Distributor Defendants), disregarding their individual duties to report.

15. Essentially each Defendant ignored science and consumer health for profits. Defendants' efforts were so successful that opioids are now the most prescribed class of drugs generating $11 billion in revenue for drug companies in 2014 alone. Even after Purdue reached a $600 million federal settlement in 2007, the settlement failed to impact what is a "$13-billion-a-year opioid industry."[10]

16. As a direct and foreseeable consequence of Defendants' misrepresentations regarding the safety and efficacy of using opioids for chronic pain, Van Zandt County has spent, and continues to spend, large sums combatting the public health crisis created by Defendants' negligent and fraudulent marketing campaigns.

17. For example, thousands of prescriptions were written for opioids in Van Zandt County in 2012[11] and in 2014 there were multiple deaths reported from drug overdoses.[12] A substantial number of those overdose deaths were a result, in whole or in part, of opioid ingestion. Upon information and belief, in each year from 2013-2017, there were multiple deaths in Van Zandt County caused in whole or in part from ingestion of prescription opioids. Defendants'

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[10] Rebecca L. Haffajee, J.D., Ph.D., M.P.H., and Michelle M. Mello, J.D., Ph.D., *Drug Companies' Liability for the Opioid Epidemic*, N. Engl. J. Med. at 2305, (Dec. 14, 2017).
[11] https://www.cdc.gov/drugoverdose/maps/rxrate-maps.html; https://www.cdc.gov/nchs/data-visualization/drug-poisoning-mortality.
[12] https://www.cdc.gov/nchs/data-visualization/drug-poisoning-mortality.

marketing misconduct, as well as Defendants' efforts to sell more prescription opioids that can be consumed therapeutically, were natural and foreseeable causes of overdose deaths and injuries in Van Zandt County.

18. As a direct and foreseeable consequence of Defendants' conduct described regarding prescription opioids, Van Zandt County has committed and continues to commit resources to provide and pay for health care, law enforcement, social services, public assistance, pharmaceutical care and other services necessary for its residents.

## II. RULE 47 STATEMENT OF MONETARY RELIEF SOUGHT

19. Per Rule 47 of the Texas Rules of Civil Procedure, the County states that although the full measure of its damages is still being calculated, its damages caused by Defendants' acts and omissions exceed $1,000,000 but are believed to be less than $50,000,000. Accordingly, at this time in the litigation, Van Zandt County states that it is seeking monetary relief for an amount greater than $1,000,000 and less than $50,000,000, the rightful and just amount to be determined by the jury.

## III. VENUE AND JURISDICTION

20. Venue is proper in Van Zandt County because all or a substantial part of the events or omissions giving rise to this claim occurred in Van Zandt County. TEX. CIV. PRAC. & REM. CODE §15.002(a)(2). This Court has subject-matter jurisdiction over this matter because Plaintiff's damages are in excess of the minimal jurisdictional limits of this Court. TEX. GOVT. CODE §24.007(b).

21. This Court has specific jurisdiction over all Defendants as their activities were directed toward Texas, and injuries complained of herein resulted from their activities. *Guardian Royal Exchange Assur., Ltd. v. English China Clays, P.L.C.*, 815 S.W.2d 223, 227 (Tex. 1991). Each Defendant has a substantial connection with Texas and the requisite minimum contacts with Texas necessary to constitutionally permit the Court to exercise jurisdiction. *See id.* at 226.

## IV. PARTIES

### A. Plaintiff

22. This action is brought for and on behalf of Van Zandt County, which provides a wide range of services on behalf of its residents, including services for families and children, public health, public assistance, law enforcement, and emergency care.

### B. Defendants

23. PURDUE PHARMA L.P. is a limited partnership organized under the laws of Delaware, and may be served through its registered agent for service of process, The Prentice-Hall Corporation System, Inc., 251 Little Falls Drive, Wilmington, DE 19808. PURDUE PHARMA L.P. is, through its ownership structure, a Texas resident. PURDUE PHARMA, INC. is a New York corporation with its principal place of business in Stamford, Connecticut, and may be served through its registered agent for service of process, Corporation Service Company, 80 State Street, Albany, NY 12207. THE PURDUE FREDERICK COMPANY is a Delaware corporation with its principal place of business in Stamford, Connecticut, and may be served through its registered agent for service of process, The Prentice-Hall Corporation System, Inc., 251 Little Falls Drive, Wilmington, DE 19808 (collectively, "Purdue").

24. Purdue manufactures, promotes, sells, and distributes opioids in the U.S. and Van Zandt County. Purdue's opioid drug, OxyContin, is among the most addictive and abused prescription drugs in the history of America. Purdue promotes opioids throughout the United States and in Van Zandt County.

25. JANSSEN PHARMACEUTICALS, INC. is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey, and may be served through its registered agent for service of process, CT Corporation System, 1999 Bryan Street, Suite 900, Dallas, TX

75201. JANSSEN PHARMACEUTICALS, INC. is a wholly owned subsidiary of JOHNSON & JOHNSON (J&J), a New Jersey corporation with its principal place of business in New Brunswick, New Jersey, and may be served through its registered agent for service of process, Attention: Legal Department, One Johnson & Johnson Plaza, New Brunswick, NJ 08933. ORTHO-MCNEIL-JANSSEN PHARMACEUTICALS, INC., now known as JANSSEN PHARMACEUTICALS, INC., is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey. JANSSEN PHARMACEUTICA INC., now known as JANSSEN PHARMACEUTICALS, INC., is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey. J&J is the only company that owns more than 10% of Janssen Pharmaceuticals' stock, and corresponds with the FDA regarding Janssen's products. Upon information and belief, J&J controls the sale and development of Janssen Pharmaceuticals' drugs and Janssen's profits inure to J&J's benefit. (Janssen Pharmaceuticals, Inc., Ortho-McNeil-Janssen Pharmaceuticals, Inc., Janssen Pharmaceutica, Inc., and J&J are referred to as "Janssen").

26. Janssen manufactures, promotes, sells, and distributes opioids in the U.S. and in Van Zandt County.

27. ENDO HEALTH SOLUTIONS INC. is a Delaware corporation with its principal place of business in Malvern, Pennsylvania, and may be served through its registered agent for service of process, The Corporation Trust Company, Corporation Trust Center, 1209 Orange St., Wilmington, DE 19801. ENDO PHARMACEUTICALS, INC. is a wholly- owned subsidiary of Endo Health Solutions Inc. and is a Delaware corporation with its principal place of business in Malvern, Pennsylvania, and may be served through its registered agent for service of process, CT Corporation System, 1999 Bryan Street, Suite 900, Dallas, TX 75201. (Endo Health Solutions Inc. and Endo Pharmaceuticals, Inc. are referred to as "Endo").

28. Endo develops, markets, and sells opioid drugs in the U.S. and in Van Zandt County. Endo also manufactures and sells generic opioids in the U.S. and Van Zandt County, by itself and through its subsidiary, Qualitest Pharmaceuticals, Inc.

29. ABBVIE INC. ("Abbvie") is a Delaware corporation with its principal place of business in North Chicago, Illinois, and may be served through its registered agent for service of process, CT Corporation System, 208 S. LaSalle Street, Suite 814, Chicago, IL 60604. KNOLL PHARMACEUTICAL COMPANY ("Knoll") has been a wholly-owned subsidiary of Abbvie from January 1, 2013. KNOLL PHARMACEUTICAL COMPANY is a New Jersey corporation with its principal place of business in Parsippany, New Jersey, and may be served through its registered agent for service of process, CT Corporation System, 208 S. LaSalle Street, Suite 814, Chicago, IL 60604.

30. Knoll irresponsibly marketed narcotics, such as Vicodin, through whimsical toys and souvenirs and did so to boost the sales of opioids. Taking advantage of the fact that Vicodin was not regulated as a Schedule II controlled substance for many years, and the fact physicians and consumers did not fully appreciate the highly addictive nature of Vicodin, Knoll advertised Vicodin with tag lines such as "The Highest Potency Pain Relief You Can Still Phone In." This tag line came as part and parcel of souvenirs like a "Vicodin" fanny pack and water bottle, both bearing the name of Vicodin, the opioid Knoll was promoting. This irresponsible marketing of a narcotic drug caused doctors and patients to believe Vicodin was safer than it really was, to the detriment of people in Van Zandt County.

31. Abbvie began manufacturing, developing, promoting, marketing, and selling the opioid drug, Vicodin, in the U.S. and in Van Zandt County beginning January 1, 2013. On information and belief, it continues to do so at the time of filing this pleading.

32. ALLERGAN PLC is a public limited company incorporated in Ireland with its principal place of business in Dublin, Ireland. ACTAVIS PLC acquired Allergan plc in March 2015, and the combined company changed its name to Allergan plc in January 2013. Before that, WATSON PHARMACEUTICALS, INC. acquired ACTAVIS, INC. in October 2012, and the combined company changed its name to ALLERGAN FINANCE, LLC as of October 2013. ALLERGAN FINANCE, LLC is a Nevada Corporation with its principal place of business in Parsippany, New Jersey, and may be served through its registered agent for service of process, The Corporation Trust Company of Nevada, 701 S. Carson St., Suite 200, Carson City, NV 89701. WATSON LABORATORIES, INC. is a Nevada corporation with its principal place of business in Corona, California, and is a wholly-owned subsidiary of Allergan plc (f/k/a Actavis, Inc., f/k/a Watson Pharmaceuticals, Inc.), and may be served through its registered agent for service of process, Corporate Creations Network, Inc., 8275 South Eastern Ave., #200, Las Vegas, NV 89123. ACTAVIS PHARMA, INC. (f/k/a Actavis, Inc.) is a Delaware corporation with its principal place of business in New Jersey and was formerly known as WATSON PHARMA, INC, and may be served through its registered agent for service of process, CT Corporation System, 1999 Bryan Street, Suite 900, Dallas, TX 75201. ACTAVIS LLC is a Delaware limited liability company with its principal place of business in Parsippany, New Jersey, and may be served through its registered agent for service of process, Corporate Creations Network, Inc., 3411 Silverside Rd., Tatnall Building, Suite 104, Wilmington, DE 19810. Each of these Defendants is owned by Allergan plc, which uses them to market and sell its drugs in the United States.

33. Upon information and belief, Allergan plc exercises control over these marketing and sales efforts and profits from the sale of Allergan/Actavis products ultimately inure to its benefit. (Allergan plc, Actavis plc, Actavis, Inc., Allergan Finance, LLC, Actavis LLC, Actavis

Pharma, Inc., Watson Pharmaceuticals, Inc., Watson Pharma, Inc., and Watson Laboratories, Inc. are referred to as "Actavis"). Actavis acquired the rights to Kadian from King Pharmaceuticals, Inc. on December 30, 2008 and began marketing Kadian in 2009.

34. Actavis manufactures, promotes, sells, and distributes opioids in the U.S. and in Van Zandt County.

35. MCKESSON CORPORATION ("McKesson") is a Delaware corporation with its principal place of business in San Francisco, California, and may be served through its registered agent for service of process, CSC - Lawyers Incorporating Service, 211 E. 7th Street, Suite 620, Austin, TX 78701. Upon information and belief, McKesson is a pharmaceutical distributor licensed to do business in Texas. McKesson distributes pharmaceuticals to retail pharmacies and institutional providers in all 50 states, including Texas and Van Zandt County.

36. CARDINAL HEALTH, INC. ("Cardinal") is an Ohio Corporation with its principal place of business in Dublin, Ohio, and may be served through its registered agent for service of process, CT Corporation System, 4400 Easton Commons, Suite 125, Columbus, OH 43219. Cardinal does substantial business in Texas and, upon information and belief, Cardinal is a pharmaceutical distributor licensed to do business in Texas. Cardinal distributes pharmaceuticals to retail pharmacies and institutional providers to customers in all 50 states, including Texas and Van Zandt County.

37. AMERISOURCEBERGEN DRUG CORPORATION ("Amerisource") is a Delaware Corporation with its principal place of business in Chesterbrook, Pennsylvania, and may be served through its registered agent for service of process, The Corporation Trust Company, Corporation Trust Center, 1209 Orange St., Wilmington, DE 19801. Amerisource does substantial business in Texas and, upon information and belief, Amerisource is a pharmaceutical distributor

licensed to do business in Texas. Amerisource distributes pharmaceuticals to retail pharmacies and institutional providers to customers in all 50 states, including Texas and Van Zandt County.

38. The County lacks information sufficient to specifically identify the true names or capacities, whether individual, corporate or otherwise, of Defendants sued herein under the fictitious names DOES 1 through 100 inclusive. The County will amend this Petition to show their true names and capacities if and when they are ascertained. Van Zandt County is informed and believes, and on such information and belief alleges, that each of the Defendants named as a DOE has engaged in conduct that contributed to cause events and occurrences alleged in this Petition and, as such, shares liability for at least some part of the relief sought herein.

## V. FACTUAL ALLEGATIONS

39. Before the 1990s, generally accepted standards of medical practice dictated that opioids should be used only for short-term acute pain – pain relating to recovery from surgery or for cancer or palliative (end-of-life) care. Using opioids for chronic pain was discouraged or even prohibited because there was a lack of evidence that opioids improved patients' ability to overcome pain and function. Instead the evidence demonstrated that patients developed tolerance to opioids over time, which increased the risk of addiction and other side effects.

40. Defendants dramatically changed doctors' views regarding opioids through a well-funded deceptive marketing scheme. Each Defendant used direct marketing and unbranded advertising disseminated by seemingly independent third parties to spread false and deceptive statements about the risks and benefits of long-term opioid use.

### A. Defendants Used Multiple Avenues To Disseminate their False and Deceptive Statements about Opioids.

41. Defendants spread their false and deceptive statements by (1) marketing their branded opioids directly to doctors treating patients residing in Van Zandt County and the Van

Zandt County patients themselves and (2) deploying so-called unbiased and independent third parties to Van Zandt County.

**1. Defendants Spread and Continue to Spread Their False and Deceptive Statements Through Direct Marketing of Their Branded Opioids.**

42. Defendants' direct marketing of opioids generally proceeded on two tracks. First, each Defendant conducted advertising campaigns touting the purported benefits of their branded drugs. For example, Defendants spent more than $14 million on medical journal advertising of opioids in 2011, nearly triple what they spent in 2001, including $8.3 million by Purdue, $4.9 million by Janssen, and $1.1 million by Endo.

43. A number of Defendants' branded ads deceptively portrayed the benefits of opioids for chronic pain. For example, Endo distributed and made available on its website, www.opana.com, a pamphlet promoting Opana ER with photographs depicting patients with physically demanding jobs like a construction worker and chef, implying that the drug would provide long-term pain-relief and functional improvement. Purdue also ran a series of ads, called "pain vignettes," for OxyContin in 2012 in medical journals. These ads featured chronic pain patients and recommended OxyContin for each. One ad described a "54-year-old writer with osteoarthritis of the hands" and implied that OxyContin would help the writer work more effectively. Pursuant to a settlement agreement, Endo and Purdue agreed in late 2015 and 2016 to halt these misleading representations in New York, but they may continue to disseminate them in Texas.

44. Second, each Defendant promoted the use of opioids for chronic pain through "detailers" – sales representatives who visited individual doctors and medical staff in their offices – and small-group speaker programs. Defendants devoted massive resources to direct sales contacts with doctors. In 2014 alone, Defendants spent $168 million on detailing branded opioids

to doctors, including $108 million by Purdue, $34 million by Janssen, $10 million by Endo, and $2 million by Actavis. This amount is twice as much as Defendants spent on detailing in 2000.

45. Defendants also identified doctors to serve, for payment, on their speakers' bureaus and to attend programs with speakers and meals paid for by Defendants. These speaker programs provided: (1) an incentive for doctors to prescribe a particular opioid (so they might be selected to promote the drug); (2) recognition and compensation for the doctors selected as speakers; and (3) an opportunity to promote the drug through the speaker to his or her peers. These speakers gave the false impression that they were providing unbiased and medically accurate presentations when they were, in fact, presenting a script prepared by Defendants. On information and belief, these presentations conveyed misleading information, omitted material information, and failed to correct Defendants' prior misrepresentations about the risks and benefits of opioids.

46. Defendants employed the same marketing plans, strategies, and messages in and around Van Zandt County, Texas as they did nationwide. Across the pharmaceutical industry, "core message" development is funded and overseen on a national basis by corporate headquarters. This comprehensive approach ensures that Defendants' messages are accurately and consistently delivered across marketing channels and in each sales territory. Defendants consider this high level of coordination and uniformity crucial to successfully marketing their drugs.

**2. Defendants Used a Diverse Group of Seemingly Independent Third Parties to Spread False and Deceptive Statements about the Risks and Benefits of Opioids.**

47. Defendants also deceptively marketed opioids in and around Van Zandt County through unbranded advertising – *i.e.*, advertising that promotes opioid use generally but does not name a specific opioid. This advertising was ostensibly created and disseminated by independent third parties. But by funding, directing, reviewing, editing, and distributing this unbranded

advertising, Defendants controlled the deceptive messages disseminated by these third parties and acted in concert with them to falsely and misleadingly promote opioids for treating chronic pain.

48. Unbranded advertising also avoided regulatory scrutiny because Defendants did not have to submit it to the FDA, and therefore it was not reviewed by the FDA.

49. Defendants' deceptive unbranded marketing often contradicted their branded materials reviewed by the FDA. For example, Endo's unbranded advertising contradicted its concurrent, branded advertising for Opana ER:

| **Pain: Opioid Therapy (Unbranded)** | **Opana ER Advertisement (Branded)** |
|---|---|
| "People who take opioids **as prescribed usually do not become addicted.**" | "All patients treated with opioids require careful monitoring for signs of abuse and addiction, since **use of opioid analgesic products carries the risk of addiction even under appropriate medical use.**" |

**a. Key Opinion Leaders (KOLs)**

50. Defendants spoke through a small circle of doctors who, upon information and belief, were selected, funded, and elevated by Defendants because their public positions supported using opioids to treat chronic pain. These doctors became known as "key opinion leaders" or "KOLs."

51. Defendants paid KOLs to serve as consultants or on their advisory boards and to give talks or present CMEs, and their support helped these KOLs become respected industry experts. As they rose to prominence, these KOLs touted the benefits of opioids to treat chronic pain, repaying Defendants by advancing their marketing goals. KOLs' professional reputations became dependent on continuing to promote a pro-opioid message, even in activities that were not directly funded by Defendants.

52. KOLs have written, consulted on, edited, and lent their names to books and articles,

and given speeches and CMEs supportive of chronic opioid therapy. Defendants created opportunities for KOLs to participate in research studies Defendants suggested or chose and then cited and promoted favorable studies or articles by their KOLs. By contrast, Defendants did not support, acknowledge, or disseminate publications of doctors unsupportive or critical of chronic opioid therapy.

53. Defendants' KOLs also served on committees that developed treatment guidelines that strongly encourage using opioids to treat chronic pain, and on the boards of pro-opioid advocacy groups and professional societies that develop, select, and present CMEs. Defendants were able to direct and exert control over each of these activities through their KOLs.

54. Pro-opioid doctors are one of the most important avenues that Defendants use to spread their false and deceptive statements about the risks and benefits of long-term opioid use. Defendants know that doctors rely heavily and less critically on their peers for guidance, and KOLs provide the false appearance of unbiased and reliable support for chronic opioid therapy.

55. Defendants utilized many KOLs, including many of the same ones. Two of the most prominent are described below.

**1. Russell Portenoy**

56. Dr. Russell Portenoy, former Chairman of the Department of Pain Medicine and Palliative Care at Beth Israel Medical Center in New York, is one example of a KOL who Defendants identified and promoted to further their marketing campaign. Dr. Portenoy received research support, consulting fees, and honoraria from Endo, Janssen, and Purdue (among others), and was a paid consultant to Purdue.

57. Dr. Portenoy was instrumental in opening the door for the regular use of opioids to treat chronic pain. He served on the American Pain Society ("APS")/American Academy of Pain

Medicine ("AAPM") Guidelines Committees, which endorsed the use of opioids to treat chronic pain, first in 1997 and again in 2009. He was also a member of the board of the American Pain Foundation ("APF"), an advocacy organization almost entirely funded by Defendants.

58. Dr. Portenoy also made frequent media appearances promoting opioids. He appeared on *Good Morning America* in 2010 to discuss using opioids long-term to treat chronic pain. On this widely-watched program, broadcast in Texas and across the country, Dr. Portenoy claimed: "Addiction, when treating pain, is distinctly uncommon. If a person does not have a history, a personal history, of substance abuse, and does not have a history in the family of substance abuse, and does not have a very major psychiatric disorder, most doctors can feel very assured that that person is not going to become addicted."[13]

59. Dr. Portenoy later admitted that he "gave innumerable lectures in the late 1980s and '90s about addiction that weren't true."[14] These lectures falsely claimed that less than 1% of patients would become addicted to opioids. According to Dr. Portenoy, because the primary goal was to "destigmatize" opioids, he and other doctors promoting them overstated their benefits and glossed over their risks. Dr. Portenoy also conceded that "[d]ata about the effectiveness of opioids does not exist."[15] Portenoy candidly stated: "Did I teach about pain management, specifically about opioid therapy, in a way that reflects misinformation? Well...I guess I did."[16]

**2. Lynn Webster**

60. Another KOL, Dr. Lynn Webster, was the co-founder and Chief Medical Director of Lifetree Clinical Research, an otherwise unknown pain clinic in Salt Lake City, Utah. Dr.

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[13] Good Morning America television broadcast, ABC News (Aug. 30, 2010).

[14] Thomas Catan & Evan Perez, *A Pain-Drug Champion Has Second Thoughts*, WALL ST. J., Dec. 17, 2012.

[15] *Id.*

[16] *Id.*

Webster was President in 2013 and is a current board member of AAPM, a Front Group that ardently supports chronic opioid therapy. He is a Senior Editor of *Pain Medicine*, the same journal that published Endo special advertising supplements touting Opana ER. Dr. Webster authored numerous CMEs sponsored by Endo and Purdue while he was receiving significant funding from Defendants.

61. In 2011, Dr. Webster presented a program via webinar sponsored by Purdue titled, *Managing Patient's Opioid Use: Balancing the Need and the Risk.* Dr. Webster recommended using risk screening tools, such as urine testing and patient agreements as a way to prevent "overuse of prescriptions" and "overdose deaths," which was available to and was intended to reach doctors treating Van Zandt County residents.

62. Dr. Webster also was a leading proponent of the concept of "pseudoaddiction," the notion that addictive behaviors should be seen not as warnings, but as indications of undertreated pain. In Dr. Webster's description, the only way to differentiate the two was to *increase* a patient's dose of opioids. As he and his co-author wrote in a book entitled *Avoiding Opioid Abuse While Managing Pain* (2007), a book that is still available online, when faced with signs of aberrant behavior, increasing the dose "in most cases . . . should be the clinician's first response." Endo distributed this book to doctors. Years later, Dr. Webster reversed himself, acknowledging that "[pseudoaddiction] obviously became too much of an excuse to give patients more medication."[17]

**b. Front Groups**

63. Defendants entered into arrangements with seemingly unbiased and independent patient and professional organizations to promote opioids for treating chronic pain. Under Defendants' direction and control, these "Front Groups" generated treatment guidelines, unbranded

[17] John Fauber & Ellen Gabler, *Networking Fuels Painkiller Boom*, MILWAUKEE WISC. J. SENTINEL (Feb. 19, 2012).

materials, and programs that favored chronic opioid therapy. They also assisted Defendants by responding to negative articles, by advocating against regulatory changes that would limit prescribing opioids in accordance with the scientific evidence, and by conducting outreach to vulnerable patient populations targeted by Defendants.

64. These Front Groups depended on Defendants for funding and, in some cases, for survival. Defendants also exercised control over programs and materials created by these groups by collaborating on, editing, and approving their content, and by funding their dissemination. In doing so, Defendants made sure these Groups would generate only the messages Defendants wanted to distribute. Even so, the Front Groups held themselves out as independent and as serving the needs of their members – whether patients suffering from pain or doctors treating those patients.

65. Defendants Endo, Janssen, and Purdue utilized many Front Groups, including many of the same ones. Several of the most prominent are described below, but there are many others, including the American Pain Society ("APS"), American Geriatrics Society ("AGS"), the Federation of State Medical Boards ("FSMB"), American Chronic Pain Association ("ACPA"), American Society of Pain Education ("ASPE"), National Pain Foundation ("NPF") and Pain & Policy Studies Group ("PPSG").

**1. American Pain Foundation ("APF")**

66. The most prominent of Defendants' Front Groups was APF, which received more than $10 million in funding from opioid manufacturers from 2007 until it closed its doors in May 2012. Endo alone provided more than half that funding; Purdue was next at $1.7 million.

67. In 2009 and 2010, more than 80% of APF's operating budget came from pharmaceutical industry sources. Including industry grants for specific projects, APF received about $2.3 million from industry sources out of total income of about $2.85 million in 2009; its budget for 2010 projected receipts of roughly $2.9 million from drug companies, out of total

income of about $3.5 million. By 2011, APF was entirely dependent on incoming grants from defendants Purdue, Endo, and others to avoid using its line of credit. As one of its board members, Russell Portenoy, explained the lack of funding diversity was one of the biggest problems at APF.

68. APF issued education guides for patients, reporters, and policymakers that touted the benefits of opioids for chronic pain and trivialized their risks, particularly the risk of addiction. APF also engaged in a significant multimedia campaign – through radio, television, and the internet – to educate patients about their "right" to pain treatment, namely opioids. All of the programs and materials were available nationally and were intended to reach patients and consumers in Van Zandt County.

**2. American Academy of Pain Medicine ("AAPM")**

69. The American Academy of Pain Medicine, with the assistance, prompting, involvement, and funding of Defendants, issued treatment guidelines and sponsored and hosted medical education programs essential to Defendants' deceptive marketing of chronic opioid therapy.

70. AAPM received over $2.2 million in funding since 2009 from opioid manufacturers. AAPM maintained a corporate relations council, whose members paid $25,000 per year (on top of other funding) to participate. The benefits included allowing members to present educational programs at off-site dinner symposia in connection with AAPM's marquee event – its annual meeting held in Palm Springs, California, or other resort locations. AAPM describes the annual event as an "exclusive venue" for offering education programs to doctors. Membership in the corporate relations council also allows drug company executives and marketing staff to meet with AAPM executive committee members in small settings. Defendants Endo, Purdue, and Actavis were members of the council and presented deceptive programs to doctors who attended

this annual event.

71. AAPM is viewed internally by Endo as "industry friendly," with Endo advisors and speakers among its active members. Endo attended AAPM conferences, funded its CMEs, and distributed its publications. The conferences sponsored by AAPM heavily emphasized sessions on opioids – 37 out of roughly 40 at one conference alone. AAPM's presidents have included top industry-supported KOLs Perry Fine, Russell Portenoy, and Lynn Webster. Dr. Webster was even elected president of AAPM while under a DEA investigation. Another past AAPM president, Dr. Scott Fishman, stated that he would place the organization "at the forefront" of teaching that "the risks of addiction are . . . small and can be managed."[18]

72. AAPM's staff understood they and their industry funders were engaged in a common task. Defendants were able to influence AAPM through both their significant and regular funding and the leadership of pro-opioid KOLs within the organization.

73. In 1997, AAPM and the American Pain Society jointly issued a consensus statement, *The Use of Opioids for the Treatment of Chronic Pain*, which endorsed opioids to treat chronic pain and claimed there was a low risk that patients would become addicted to opioids. The co-author of the statement, Dr. Haddox, was at the time a paid speaker for Purdue. Dr. Portenoy was the sole consultant. The consensus statement remained on AAPM's website until 2011, and was taken down from AAPM's website only after a doctor complained, though it still lingers on the internet elsewhere.

74. AAPM and APS issued their own guidelines in 2009 ("AAPM/APS Guidelines") and continued to recommend using opioids to treat chronic pain. Fourteen of the 21 panel members

[18] Interview by Paula Moyer with Scott M. Fishman, M.D., Professor of Anesthesiology and Pain Medicine, Chief of the Division of Pain Medicine, Univ. of Cal., Davis (2005), http://www.medscape.org/viewarticle/500829.

who drafted the AAPM/APS Guidelines, including KOLs Dr. Portenoy and Dr. Perry Fine of the University of Utah, received support from Janssen, Endo, and Purdue.

75. The 2009 Guidelines promote opioids as "safe and effective" for treating chronic pain, despite acknowledging limited evidence, and conclude that the risk of addiction is manageable for patients regardless of past abuse histories. One panel member, Dr. Joel Saper, Clinical Professor of Neurology at Michigan State University and founder of the Michigan Headache & Neurological Institute, resigned from the panel because he was concerned the 2009 Guidelines were influenced by contributions that drug companies, including Defendants, made to the sponsoring organizations and committee members. These AAPM/APS Guidelines have been a particularly effective channel of deception and have influenced not only treating physicians, but also the body of scientific evidence on opioids. The Guidelines have been cited 732 times in academic literature, were disseminated in and around Van Zandt County during the relevant time period, are still available online, and were reprinted in the *Journal of Pain.*

**B. Defendants' Marketing Scheme Misrepresented the Risks and Benefits of Opioids.**

76. To convince doctors treating residents in Van Zandt County and Van Zandt County patients that opioids can and should be used to treat chronic pain, Defendants had to convince them that long-term opioid use is both safe and effective. Knowing they could do so only by deceiving those doctors and patients about the risks and benefits of long-term opioid use, Defendants made claims that were not supported by, or were contrary to, the scientific evidence. Even though pronouncements by and guidance from the FDA and the CDC based on that evidence confirm that their claims were false and deceptive, Defendants have not corrected them, or instructed their KOLs or Front Groups to correct them, and continue to spread them today.

## C. Defendants Falsely Trivialized or Failed to Disclose the Known Risks of Long-Term Opioid Use.

77. To convince doctors and patients that opioids are safe, Defendants deceptively trivialized and failed to disclose the risks of long-term opioid use, particularly the risk of addiction, through a series of misrepresentations that have been conclusively debunked by the FDA and CDC. These misrepresentations – which are described below – reinforced each other and created the dangerously misleading impression that: (1) starting patients on opioids was low-risk because most patients would not become addicted, and because those who were at greatest risk of addiction could be readily identified and managed; (2) patients who displayed signs of addiction probably were not addicted and, in any event, could easily be weaned from the drugs; (3) the use of higher opioid doses, which many patients need to sustain pain relief as they develop tolerance to the drugs, do not pose special risks; and (4) abuse-deterrent opioids both prevent overdose and are inherently less addictive. Defendants have not only failed to correct these misrepresentations, they continue to make them today.

78. ***First***, Defendants falsely claimed the risk of addiction is low and unlikely to develop when opioids are prescribed, as opposed to obtained illicitly; and failed to disclose the greater risk of addiction with prolonged use of opioids. For example:

a. Actavis's predecessor caused a patient education brochure to be distributed in 2007 claiming opioid addiction is possible, but "less likely if you have never had an addiction problem." Upon information and belief, based on Actavis's acquisition of its predecessor's marketing materials along with the rights to Kadian, Actavis continued to use this brochure in 2009 and beyond;

b. Purdue sponsored APF's *Treatment Options: A Guide for People Living with Pain* (2007), which instructed that addiction is rare and limited to extreme cases of unauthorized dose escalations, obtaining duplicative opioid prescriptions from multiple sources, or theft. This publication is still available online;

c. Endo sponsored a website, Painknowledge.com, which claimed in 2009 that "[p]eople who take opioids as prescribed usually do not become addicted." Another Endo website, PainAction.com, stated "Did you know? Most chronic pain patients do not become addicted to the opioid medications that are prescribed for them;"

d. Endo distributed a pamphlet with the Endo logo entitled *Living with Someone with Chronic Pain*, which stated that: "Most health care providers who treat people with pain agree that most people do not develop an addiction problem." A similar statement appeared on the Endo website, www.opana.com;

e. Janssen reviewed, edited, approved, and distributed a patient education guide entitled *Finding Relief: Pain Management for Older Adults* (2009), which described as "myth" the claim that opioids are addictive, and asserted as fact that "[m]any studies show that opioids are rarely addictive when used properly for the management of chronic pain;"

f. Janssen currently runs a website, Prescriberesponsibly.com (last updated July 2, 2015), which claims that concerns about opioid addiction are "overestimated;"

g. Purdue sponsored APF's *A Policymaker's Guide to Understanding Pain & Its Management* – which claims that less than 1% of children prescribed opioids will become addicted and that pain is undertreated due to "misconceptions about opioid addiction[]." This publication is still available online; and

h. Detailers for Purdue, Endo, and Janssen in and around Van Zandt County minimized or omitted any discussion with doctors of the risk of addiction; misrepresented the potential for opioid abuse with purportedly abuse-deterrent formulations; and routinely did not correct the misrepresentations noted above.

79. These claims contradict longstanding scientific evidence, as the FDA and CDC have conclusively declared. As noted in the 2016 CDC Guideline endorsed by the FDA, there is "extensive evidence" of the "possible harms of opioids (including opioid use disorder [an alternative term for opioid addiction])."[19] The guideline points out that "[o]pioid pain medication use presents serious risks, including...opioid use disorder" and that "continuing opioid therapy for

[19] *CDC Guideline for Prescribing Opioids for Chronic Pain – United States 2016*, Centers for Disease Control and Prevention (Mar. 18, 2016).

3 months substantially increases risk for opioid use disorder."[20]

80. The FDA further exposed the falsity of Defendants' claims about the low risk of addiction when it announced changes to the labels for ER/LA opioids in 2013 and for IR opioids in 2016. In its announcements, the FDA discussed the risks related to opioid use and that IR opioids are associated with "persistent abuse, addiction, overdose mortality, and risk of NOWS [neonatal opioid withdrawal syndrome]."[21]

81. According to the FDA, because of the risks associated with long-term opioid use, including "the serious risk of addiction, abuse, misuse, overdose, and death,"[22] opioids should be "reserved for pain severe enough to require opioid treatment and for which alternative treatment options (e.g., non-opioid analgesics or opioid combination products, as appropriate) are inadequate or not tolerated."[23]

82. The warnings on Defendants' own FDA-approved drug labels caution that opioids "exposes users to risks of addiction, abuse and misuse, which can lead to overdose and death"[24] and that addiction "can occur in patients appropriately prescribed"[25] opioids.

83. ***Second***, Defendants falsely instructed doctors and patients that signs of addiction are actually signs of undertreated pain and should be treated by prescribing more opioids. Defendants called this phenomenon "pseudoaddiction" – a term coined by Dr. David Haddox, who went to work for Purdue, and popularized by Dr. Russell Portenoy, a KOL for Endo, Janssen, and Purdue – and claimed that pseudoaddiction is substantiated by scientific evidence. For example:

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[20] *Id.*
[21] *FDA Announcement of Enhanced Warnings for Immediate-Release Opioid Pain Medications Related to Risks of Misuse, Abuse, Addiction, Overdose and Death*, Federal Drug Administration (Mar. 22, 2016).
[22] *Id.*
[23] *Id.*
[24] *See, e.g.*, OxyContin label and insert at *OxyContin.com.*
[25] *Id.*

a. Purdue sponsored *Responsible Opioid Prescribing* (2007), which taught that behaviors such as "requesting drugs by name," "demanding or manipulative behavior," seeing more than one doctor to obtain opioids, and hoarding, are all signs of pseudoaddiction, rather than true addiction. Responsible Opioid Prescribing remains for sale online. The 2012 edition continues to teach that pseudoaddiction is real;

b. Janssen sponsored, funded, and edited the *Let's Talk Pain* website, which in 2009 stated: "pseudoaddiction . . . refers to patient behaviors that may occur when pain is under-treated . . . . Pseudoaddiction is different from true addiction because such behaviors can be resolved with effective pain management;"

c. Endo sponsored a National Initiative on Pain Control (NIPC) CME program in 2009 titled *Chronic Opioid Therapy: Understanding Risk While Maximizing Analgesia*, which promoted pseudoaddiction by teaching that a patient's aberrant behavior was the result of untreated pain. Endo substantially controlled NIPC by funding NIPC projects; developing, specifying, and reviewing content; and distributing NIPC materials;

d. Purdue published a pamphlet in 2011 entitled *Providing Relief, Preventing Abuse*, which described pseudoaddiction as a concept that "emerged in the literature" to describe the inaccurate interpretation of [drug-seeking behaviors] in patients who have pain that has not been effectively treated;" and

e. Purdue sponsored a CME program entitled *Path of the Patient, Managing Chronic Pain in Younger Adults at Risk for Abuse*. In a role play, a chronic pain patient with a history of drug abuse tells his doctor that he is taking twice as many hydrocodone pills as directed. The narrator notes that because of pseudoaddiction, the doctor should not assume the patient is addicted even if he persistently asks for a specific drug, seems desperate, hoards medicine, or "overindulges in unapproved escalating doses." The doctor treats this patient by prescribing a high-dose, long-acting opioid.

84. The 2016 CDC Guideline rejects the concept of pseudoaddiction. The Guideline nowhere recommends that opioid dosages be increased if a patient is not experiencing pain relief. To the contrary, the Guideline explains that "[p]atients who do not experience clinically meaningful pain relief early in treatment...are unlikely to experience pain relief with longer-term

use,"[26] and that physicians should "reassess[] pain and function within 1 month"[27] in order to decide whether to "minimize risks of long-term opioid use by discontinuing opioids"[28] because the patient is "not receiving a clear benefit."[29]

85. ***Third,*** Defendants falsely instructed doctors and patients that addiction risk screening tools, patient contracts, urine drug screens, and similar strategies allow them to reliably identify and safely prescribe opioids to patients predisposed to addiction. These misrepresentations were especially insidious because Defendants aimed them at general practitioners and family doctors who lack the time and expertise to closely manage higher-risk patients. Defendants' misrepresentations made these doctors feel more comfortable prescribing opioids to their patients, and patients more comfortable starting opioid therapy for chronic pain. For example:

a. Endo paid for a 2007 supplement in the *Journal of Family Practice* written by a doctor who became a member of Endo's speakers' bureau in 2010. The supplement, entitled *Pain Management Dilemmas in Primary Care: Use of Opioids*, emphasized the effectiveness of screening tools, claiming that patients at high risk of addiction could safely receive chronic opioid therapy using a "maximally structured approach" involving toxicology screens and pill counts;

b. Purdue sponsored a 2011 webinar, *Managing Patient's Opioid Use: Balancing the Need and Risk*, which claimed that screening tools, urine tests, and patient agreements prevent "overuse of prescriptions" and "overdose deaths;" and

c. As recently as 2015, Purdue has represented in scientific conferences that "bad apple" patients – and not opioids – are the source of the addiction crisis and that once those "bad apples" are identified, doctors can safely prescribe opioids without causing addiction.

86. Once again, the 2016 CDC Guideline confirms these representations are false. The Guideline notes that there are no studies assessing the effectiveness of risk mitigation strategies –

[26] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[27] *Id.*
[28] *Id.*
[29] *Id.*

such as screening tools, patient contracts, urine drug testing, or pill counts – widely believed by doctors to detect and deter outcomes related to addiction and overdose.[30] As a result, the Guideline recognizes that doctors should not overestimate the risk screening tools for classifying patients as high or low risk for opioid addiction because they are insufficient to rule out the risks of long-term opioid therapy.[31]

87. ***Fourth***, to underplay the risk and impact of addiction and make doctors feel more comfortable starting patients on opioids, Defendants falsely claimed that opioid dependence can easily be addressed by tapering and that opioid withdrawal is not a problem thereby failing to disclose the increased difficulty of stopping opioids after long-term use.

88. For example, a CME sponsored by Endo, entitled *Persistent Pain in the Older Adult*, claimed that withdrawal symptoms can be avoided by tapering a patient's opioid dose by 10%-20% for 10 days. And Purdue sponsored APF's *A Policymaker's Guide to Understanding Pain & Its Management*, which claimed that "[s]ymptoms of physical dependence can often be ameliorated by gradually decreasing the dose of medication during discontinuation."

89. Defendants deceptively minimized the significant symptoms of opioid withdrawal, which, as explained in the 2016 CDC Guideline, include drug cravings, anxiety, insomnia, abdominal pain, vomiting, diarrhea, sweating, tremor, tachycardia (rapid heartbeat), spontaneous abortion and premature labor in pregnant women, and the unmasking of anxiety, depression, and addiction – and grossly understated the difficulty of tapering, particularly after long-term opioid use.

90. Yet the 2016 CDC Guideline recognizes that the duration of opioid use and the dosage of opioids prescribed should be limited to "minimize the need to taper opioids to prevent

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[30] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[31] *See id.*

distressing or unpleasant withdrawal symptoms,"[32] because "physical dependence on opioids is an expected physiologic response in patients exposed to opioids for more than a few days."[33] The Guideline further states that "tapering opioids can be especially challenging after years on high dosages because of physical and psychological dependence"[34] and highlights the difficulties, including the need to carefully identify "a taper slow enough to minimize symptoms and signs of opioid withdrawal"[35] and pausing and restarting tapers depending on the patient's response.

91. The CDC also acknowledges the lack of any "high-quality studies comparing the effectiveness of different tapering protocols for use when opioid dosage is reduced or opioids are discontinued."[36]

92. ***Fifth***, Defendants falsely claimed that doctors and patients could increase opioid dosages indefinitely without added risk and failed to disclose the greater risks to patients at higher dosages. The ability to escalate dosages was critical to Defendants' efforts to market opioids for long-term use to treat chronic pain because, absent this misrepresentation, doctors would have abandoned treatment when patients built up tolerance and lower dosages did not provide pain relief. For example:

a. Actavis's predecessor created a patient brochure for Kadian in 2007 that stated, "Over time, your body may become tolerant of your current dose. You may require a dose adjustment to get the right amount of pain relief. This is not addiction." Upon information and belief, based on Actavis's acquisition of its predecessor's marketing materials along with the rights to Kadian, Actavis continued to use these materials in 2009 and beyond;

b. Purdue sponsored *APF's Treatment Options: A Guide for People Living with Pain* (2007), which claims that some patients "need" a larger dose of an opioid, regardless of the dose currently prescribed. The guide stated that opioids have "no ceiling dose" and are therefore the most appropriate

[32] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[33] *Id.*
[34] *Id.*
[35] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[36] *Id.*

treatment for severe pain. This guide is still available for sale online;

c. Endo sponsored a website, painknowledge.com, which claimed in 2009 that opioid dosages may be increased until "you are on the right dose of medication for your pain;"

d. Endo distributed a pamphlet edited by a KOL entitled *Understanding Your Pain: Taking Oral Opioid Analgesics*, which was available during the time period of this Complaint on Endo's website. In Q&A format, it asked "If I take the opioid now, will it work later when I really need it?" The response is, "The dose can be increased. . . . You won't 'run out' of pain relief;"

e. Janssen sponsored a patient education guide entitled *Finding Relief: Pain Management for Older Adults* (2009), which was distributed by its sales force. This guide listed dosage limitations as "disadvantages" of other pain medicines but omitted any discussion of risks of increased opioid dosages;

f. Purdue's In the Face of Pain website promotes the notion that if a patient's doctor does not prescribe what, in the patient's view, is a sufficient dosage of opioids, he or she should find another doctor who will;

g. Purdue sponsored APF's *A Policymaker's Guide to Understanding Pain & Its Management*, which taught that dosage escalations are "sometimes necessary," even unlimited ones, but did not disclose the risks from high opioid dosages. This publication is still available online;

h. Purdue sponsored a CME entitled *Overview of Management Options* that is still available for CME credit. The CME was edited by a KOL and taught that NSAIDs and other drugs, but not opioids, are unsafe at high dosages; and

i. Purdue presented a 2015 paper at the College on the Problems of Drug Dependence, the "the oldest and largest organization in the US dedicated to advancing a scientific approach to substance use and addictive disorders," challenging the correlation between opioid dosage and overdose.

93. These claims conflict with the scientific evidence, as confirmed by the FDA and CDC. As the CDC explains in its 2016 Guideline, the "[b]enefits of high-dose opioids for chronic pain are not established"[37] while the "risks for serious harms related to opioid therapy increase at

[37] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*

higher opioid dosage."[38]

94. More specifically, the CDC explains that "there is now an established body of scientific evidence showing that overdose risk is increased at higher opioid dosages."[39] Similarly, there is an "increased risk for opioid use disorder, respiratory depression, and death at higher dosages."[40] That is why the CDC advises doctors to avoid increasing dosages above 90 morphine milligram equivalents per day.

95. The 2016 CDC Guideline reinforces earlier findings announced by the FDA. In 2013, the FDA acknowledged that available data suggested that increasing the opioid dosage likewise increased certain adverse events. For example, the FDA noted that studies suggest a positive association between high-dose opioid use and overdoses.

96. ***Finally***, Defendants' deceptive marketing of the so-called abuse-deterrent properties of some of their opioids has created false impressions that these opioids can curb addiction and abuse.

97. More specifically, Defendants have made misleading claims about the ability of their so-called abuse-deterrent opioid formulations to deter addiction and overdose. For example, Endo's advertisements for the 2012 reformulation of Opana ER claimed that it was designed to be crush resistant in a way that suggested it was more difficult to misuse the product. This claim was false.

98. The FDA warned in a 2013 letter that there was no evidence Endo's design would provide a reduction in oral, intranasal or intravenous use.[41] Moreover, Endo's own studies, which it failed to disclose, showed that Opana ER could still be ground and chewed.

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[38] *Id.*
[39] *Id.*
[40] *Id.*
[41] *See FDA Statement: Original Opana ER Relisting Determination* (May 10, 2013).

99. In a 2016 settlement with the State of New York, Endo agreed not to make statements in New York that Opana ER was designed to be or is crush resistant. The State found those statements false and deceptive because there was no difference in the ability to extract the narcotic from Opana ER.

100. Similarly, the 2016 CDC Guideline states that no studies support the notion that "abuse-deterrent technologies [are] a risk mitigation strategy for deterring or preventing abuse,"[42] noting that the technologies – even when they work – "do not prevent opioid abuse through oral intake, the most common route of opioid abuse, and can still be abused by non-oral routes."[43]

101. These numerous, long-standing misrepresentations of the risks of long-term opioid use spread by Defendants successfully convinced doctors and patients to discount those risks.

**D. Defendants Grossly Overstated the Benefits of Chronic Opioid Therapy.**

102. To convince doctors and patients that opioids should be used to treat chronic pain, Defendants had to persuade them that there was a significant benefit to long-term opioid use. But as the 2016 CDC Guideline makes clear, there is "insufficient evidence to determine the long-term benefits of opioid therapy for chronic pain."[44]

103. In fact, the CDC found no evidence showing "a long-term benefit of opioids in pain and function versus no opioids for chronic pain with outcomes examined at least 1 year later (with most placebo-controlled randomized trials $\leq$ 6 weeks in duration)"[45] and that other treatments were more or equally beneficial and less harmful than long-term opioid use. The FDA, too, has recognized the lack of evidence to support long-term opioid use.

104. In 2013, the FDA stated that it was unaware of any studies demonstrating the

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[42] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[43] *Id.*
[44] *Id.*
[45] *Id.*

safety and efficacy of opioids for long-term use.[46] Despite this lack of studies, Defendants falsely and misleadingly touted the benefits of long-term opioid use and suggested that these benefits were supported by scientific evidence. Not only have Defendants failed to correct these false and deceptive claims, they continue to make them today. For example:

a. Actavis distributed an advertisement that claimed that the use of Kadian to treat chronic pain would allow patients to return to work, relieve "stress on your body and your mental health," and help patients enjoy their lives;

b. Endo distributed advertisements that claimed that the use of Opana ER for chronic pain would allow patients to perform demanding tasks like construction work or work as a chef and portrayed seemingly healthy, unimpaired subjects;

c. Janssen sponsored and edited a patient education guide entitled *Finding Relief: Pain Management for Older Adults* (2009) – which states as "a fact" that "opioids may make it easier for people to live normally." The guide lists expected functional improvements from opioid use, including sleeping through the night, returning to work, recreation, sex, walking, and climbing stairs;

d. Purdue ran a series of advertisements for OxyContin in 2012 in medical journals entitled "pain vignettes," which were case studies featuring patients with pain conditions persisting over several months and recommending OxyContin for them. The ads implied that OxyContin improves patients' function;

e. *Responsible Opioid Prescribing* (2007), sponsored and distributed by Endo and Purdue, taught that relief of pain by opioids, by itself, improved patients' function. The book remains for sale online;

f. Purdue sponsored APF's *Treatment Options: A Guide for People Living with Pain* (2007), which counseled patients that opioids "give [pain patients] a quality of life we deserve." The guide was available online until APF shut its doors in 2012;

g. Endo's NIPC website *painknowledge.com* claimed in 2009 that with opioids, "your level of function should improve; you may find you are now able to participate in activities of daily living, such as work and hobbies, that you were not able to enjoy when your pain was worse." Elsewhere, the website touted improved quality of life (as well as "improved function") as

[46] Letter from Janet Woodcock, M.D., Dir., Ctr. For Drug Eval. & Res., to Andrew Kolodny, M.D., Pres. Physicians for Responsible Opioid Prescribing, Re Docket No. FDA-2012-P-0818 (Sept. 10, 2013).

benefits of opioid therapy. The grant request that Endo approved for this project specifically indicated NIPC's intent to make misleading claims about function, and Endo closely tracked visits to the site;

h. Endo was the sole sponsor, through NIPC, of a series of CMEs titled *Persistent Pain in the Older Patient*, which claimed that chronic opioid therapy has been "shown to reduce pain and improve depressive symptoms and cognitive functioning." The CME was disseminated via webcast;

i. Janssen sponsored, funded, and edited a website, *Let's Talk Pain*, in 2009, which featured an interview edited by Janssen claiming that opioids allowed a patient to "continue to function." This video is still available today on YouTube;

j. Purdue sponsored the development and distribution of APF's *A Policymaker's Guide to Understanding Pain & Its Management*, which claimed that "multiple clinical studies" have shown that opioids are effective in improving daily function, psychological health, and health-related quality of life for chronic pain patients." The Policymaker's Guide was originally published in 2011 and is still available online today; and

k. Purdue's, Endo's, and Janssen's sales representatives have conveyed and continue to convey the message that opioids will improve patient function.

105. These claims find no support in the scientific literature. Most recently, the 2016 CDC Guideline, approved by the FDA, concluded, "There is no good evidence that opioids improve pain or function with long-term use"[47] and "complete relief of pain is unlikely."[48] (Emphasis added.) The CDC reinforced this conclusion throughout its 2016 Guideline:

a. "No evidence shows a long-term benefit of opioids in pain and function versus no opioids for chronic pain with outcomes examined at least 1 year later . . ."[49]

b. "Although opioids can reduce pain during short-term use, the clinical evidence review found insufficient evidence to determine whether pain relief is sustained and whether function or quality of life improves with long-term opioid therapy;"[50] and

c. "[E]vidence is limited or insufficient for improved pain or function with

[47] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*
[48] *Id.*
[49] *Id.*
[50] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*

> long-term use of opioids for several chronic pain conditions for which opioids are commonly prescribed, such as low back pain, headache, and fibromyalgia."[51]

106. The CDC also noted that the risks of addiction and death "can cause distress and inability to fulfill major role obligations."[52] As a matter of common sense (and medical evidence), drugs that can kill patients or commit them to a life of addiction or recovery do not improve their function and quality of life.

107. The 2016 CDC Guideline was not the first time a federal agency repudiated Defendants' claim that opioids improved function and quality of life. In 2010, the FDA warned Actavis that "[w]e are not aware of substantial evidence or substantial clinical experience demonstrating that the magnitude of the effect of the drug [Kadian] has in alleviating pain, taken together with any drug-related side effects patients may experience...results in any overall positive impact on a patient's work, physical and mental functioning, daily activities, or enjoyment of life."[53]

108. Defendants also falsely emphasized or exaggerated the risks of competing products like NSAIDs so that doctors and patients would look to opioids first for treating chronic pain. Once again, Defendants' misrepresentations contravene pronouncements by and guidance from the FDA and CDC based on the scientific evidence.

109. Consequently, the FDA changed the labels for ER/LA opioids in 2013 and IR opioids in 2016 to state that opioids should be used only as a last resort where alternative treatments like non-opioid drugs are inadequate. And the 2016 CDC Guideline states that NSAIDs,

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[51] *Id.*

[52] *Id.*

[53] Warning Letter from Thomas Abrams, Dir., FDA Div. of Mktg., Adver., & Commc'ns, to Doug Boothe, CEO, Actavis Elizabeth LLC (Feb. 18, 2010), http://www.fda.gov/Drugs/GuidanceComplianceRegulatoryInformation/EnforcementActivitiesbyFDA/WarningLettersandNoticeofViolationLetterstoPharmaceuticalCcompanies/ucm259240.htm.

not opioids, should be the first-line treatment for chronic pain, particularly arthritis and lower back pain.

110. In addition, Purdue misleadingly promoted OxyContin as unique among opioids in providing 12 continuous hours of pain relief with one dose. In fact, OxyContin does not last for 12 hours – a fact that Purdue has known at all times relevant to this action.

111. According to Purdue's own research, OxyContin wears off in under six hours in one quarter of patients and in under 10 hours in more than half. The reason is that OxyContin tablets release approximately 40% of their active medicine immediately, after which release tapers. Although the patient experiences a powerful initial response, there is little or no pain relief at the end of the dosing period because less medicine is released.

112. This phenomenon is known as "end of dose" failure, and the FDA found in 2008 that a substantial number of chronic pain patients taking OxyContin experience it.

113. This "end of dose" failure not only renders Purdue's promise of 12 hours of relief false and deceptive, it also makes OxyContin more dangerous because the declining pain relief patients experience toward the end of each dosing period drives them to take more OxyContin before the next dosing period begins, quickly increasing the amount of drug they are taking and spurring growing dependence.

114. Purdue's competitors were aware of this problem. For example, Endo ran advertisements for Opana ER referring to "real" 12-hour dosing. Nevertheless, Purdue falsely promoted OxyContin as if it were effective for a full 12 hours. Indeed, Purdue's sales representatives continue to tell doctors in and around Van Zandt County that OxyContin lasts a full 12 hours.

**E. Defendants also engaged in Other Unlawful, Unfair, and Fraudulent Misconduct.**

115. Defendants herein participated in illicit and unlawful prescribing of its drugs. For example, Purdue did not report illegal prescribing of OxyContin until years after law enforcement shut down a Los Angeles clinic that prescribed more than 1.1 million OxyContin tablets. In doing so, Purdue protected its own profits at the expense of public health and safety.

116. The State of New York found that Endo failed to require sales representatives to report signs of addiction, diversion, and inappropriate prescribing; paid bonuses to sales representatives for detailing prescribers who were subsequently arrested or convicted for illegal prescribing; and failed to prevent sales representatives from visiting prescribers whose suspicious conduct had caused them to be placed on a no-call list.

**F. Defendants Targeted Susceptible Prescribers and Vulnerable Patient Populations.**

117. As a part of their deceptive marketing scheme, Defendants identified and targeted susceptible prescribers and vulnerable patient populations in the U.S. and in and around Van Zandt County. For example, Defendants focused their deceptive marketing on primary care doctors, who were more likely to treat chronic pain patients and prescribe opioids, but were less likely to be educated about treating pain and the risks and benefits of opioids.

118. Defendants also targeted vulnerable patient populations like the elderly and veterans, who tend to suffer from chronic pain. Defendants targeted these vulnerable patients even though the risks of long-term opioid use were significantly greater for them.

119. For example, the 2016 CDC Guideline observes that existing evidence shows that elderly patients taking opioids suffer from elevated fall and fracture risks, greater risk of hospitalization, and increased vulnerability to adverse drug effects and interactions. The Guideline therefore concludes that there are "special risks of long-term opioid use for elderly patients" and

recommends that doctors use "additional caution and increased monitoring" to minimize the risks of opioid use in elderly patients.

120. The same is true for veterans, who are more likely to use anti-anxiety drugs (benzodiazepines) for post-traumatic stress disorder, which interact dangerously with opioids.

**G. Although Defendants knew that their Marketing of Opioids was False and Deceptive, they Fraudulently Concealed their Misconduct.**

121. Defendants, both individually and collectively, made, promoted, and profited from their misrepresentations about the risks and benefits of opioids for chronic pain even though they knew their misrepresentations were false and deceptive. The history of opioids, as well as research and clinical experience over the last 20 years, established that opioids were highly addictive and responsible for a long list of very serious adverse outcomes.

122. Not only did the FDA and other regulators warn Defendants, but Defendants had access to scientific studies, detailed prescription data, and reports of adverse events, including reports of addiction, hospitalization, and deaths – all of which made clear the harms from long-term opioid use, including the suffering from addiction, overdoses, and death in alarming numbers in patients using opioids.

123. More recently, the FDA and CDC have issued pronouncements based on the medical evidence that conclusively expose the known falsity of Defendants' misrepresentations, and Endo and Purdue have recently entered agreements prohibiting them from making some of the same misrepresentations described herein in New York.

124. Specifically, three current and former executives from Purdue plead guilty in 2007 to criminal charges that they misled regulators, doctors, and patients about OxyContin's risk of

addiction.[54] In pleading guilty to misbranding charges, Purdue admitted it had fraudulently marketed OxyContin as a drug less prone to addiction and as having fewer side effects than other opioids.[55] In reality, unlike other opioids, OxyContin contained pure oxycodone without any other ingredients, which made it a powerful narcotic despite its time-release design that Purdue touted as ameliorating its addictive potential.[56]

125. Moreover, Defendants took steps to avoid detection of and to fraudulently conceal their deceptive marketing and unlawful, unfair, and fraudulent conduct. For example, Defendants disguised their own role in the deceptive marketing of chronic opioid therapy by funding and working through third parties like Front Groups and KOLs.

126. Finally, Defendants manipulated their promotional materials and the scientific literature to make it appear that these items were accurate, truthful, and supported by objective evidence when they were not.

127. Thus, Defendants successfully concealed from the medical community and patients facts sufficient to arouse suspicion of the claims Van Zandt County now asserts. Van Zandt County did not know of the existence or scope of Defendants' industry-wide fraud and could not have acquired such knowledge earlier through the exercise of reasonable diligence.

## H. By Increasing Opioid Prescriptions and Use, Defendants' Deceptive Marketing Scheme has fueled the Opioid Epidemic and Devastated Van Zandt County Communities.

128. Defendants' misrepresentations deceived doctors and patients about the risks and benefits of long-term opioid use. Studies reveal that many doctors and patients are unaware of or do not understand the risks or benefits of opioids. Indeed, patients often report that they were not

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[54] *See* Barry Meier, "In Guilty Plea, OxyContin Maker to Pay $600 Million," (May 10, 2007), http://www.nytimes.com/2007/05/10/business/11drug-web.html.
[55] *See id.*
[56] *See id.*

warned they might become addicted to opioids prescribed to them. As reported in January 2016, a 2015 survey of more than 1,000 opioid patients found that 4 out of 10 were not told opioids were potentially addictive.[57]

129. Defendants' deceptive marketing scheme caused and continues to cause doctors in and around Van Zandt County to prescribe opioids for chronic pain conditions such as back pain, headaches, arthritis, and fibromyalgia. Absent Defendants' deceptive marketing scheme, these doctors would not have been able to over prescribe opioids or become embroiled in pill mills that negatively impacted residents of Van Zandt County.

130. While Defendants may claim the federal government authorized the amount of annual prescription opioids sold, they know in truth that several Defendants have successfully used their organized money and influence to render the federal government's enforcement agency, the Drug Enforcement Administration, virtually powerless to interrupt the over-supply of prescription opioid drugs.

131. Defendants' deceptive marketing scheme also caused and continues to cause patients to purchase and use opioids for their chronic pain believing they are safe and effective. Absent Defendants' deceptive marketing scheme, fewer patients would be using opioids long-term to treat chronic pain, and those patients using opioids would be using less of them.

132. Defendants' deceptive marketing has caused and continues to cause the prescribing and use of opioids to explode. Indeed, this dramatic increase in opioid prescriptions and use corresponds with the dramatic increase in Defendants' spending on their deceptive marketing scheme. Defendants' spending on opioid marketing totaled approximately $91 million in 2000.

[57] Hazelden Betty Ford Foundation, *Missed Questions, Missed Opportunities* (Jan. 27, 2016), *available at* http://www.hazeldenbettyford.org/about-us/news-and-media/pressrelease/doctors-missing-questions-that-could-prevent-opioid-addiction.

By 2011, that spending had tripled to $288 million.

133. The escalating number of opioid prescriptions written by doctors who were deceived by Defendants' deceptive marketing scheme is the cause of a correspondingly dramatic increase in opioid addiction, overdose, and death throughout the U.S. and Van Zandt County.

134. Scientific evidence demonstrates a strong correlation between opioid prescriptions and becoming addicted to opioids. In a 2016 report, the CDC explained that prescribing opioids has quadrupled since 1999, which has resulted in a parallel increase in opioid overdoses.[58] Indeed, there has been a two-third increase in overdose deaths from using opioids since 2000.[59] For these reasons, the CDC concluded that efforts to rein in the prescribing of opioids for chronic pain are critical "to reverse the cycle of opioid pain medication misuse that contributes to the opioid overdose epidemic."[60]

135. Due to the increase in opioid overdoses, first responders such as police officers, have been and will continue to be in the position to assist people experiencing opioid-related overdoses.[61] In 2016, "over 1,200 law enforcement departments nationwide carried naloxone in an effort to prevent opioid-related deaths."[62]

136. Defendants' deceptive marketing scheme has also detrimentally impacted children in Van Zandt County. Overprescribing opioids for chronic pain has made the drugs more accessible

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[58] CDC. National Vital Statistics System, Mortality. CDC WONDER. Atlanta, GA: US Department of Health and Human Services, CDC; 2016. https://wonder.cdc.gov/; Rudd RA, Seth P, David F, Scholl L. Increases in Drug and Opioid-Involved Overdose Deaths — United States, 2010–2015. MMWR Morb Mortal Wkly Rep. ePub: 16 December 2016.

[59] *National Vital Statistics System*, Mortality file and appearing *Center for Disease Control and Prevention* Morbidity and Mortality Weekly Report, January 1, 2006 / 64(50); 1378-82, Increases in Drug and Opioid Deaths – United States, 2000-2014.

[60] *CDC Guideline for Prescribing Opioids for Chronic Pain, supra; see also* Rudd RA, Seth P, David F, Scholl L. Increases in Drug and Opioid-Involved Overdose Deaths — United States, 2010–2015. MMWR Morb Mortal Wkly Rep. ePub: 16 December 2016.

[61] Opinion of the Attorney General of Texas, KP-0168 (Oct. 4, 2017).

[62] *Id. citing* http://www.nchrc.org/law-enforcement/us-law-enforcement-who-carry-naloxone/.

to school-aged children, who come into contact with opioids after they have been prescribed to friends or relatives in the same household.

137. Defendants' conduct has adversely affected Van Zandt County's child protection agencies in the number of children in foster care driven by parental drug addiction. Children with parents addicted to drugs tend to stay in foster care longer, and they often enter the system having experienced significant trauma, which makes these cases more expensive for counties like Van Zandt County.

138. Opioid addiction is one of the primary reasons that Van Zandt County residents seek treatment for substance dependence. A significant number of admissions for drug addiction were associated with a primary diagnosis of opiate addiction or dependence.

139. Defendants' creation, through false and deceptive advertising and other unlawful and unfair conduct, of a virtually limitless opioid market has significantly harmed Van Zandt County communities. Defendants' success in extending the market for opioids to new patients and chronic pain conditions has created an abundance of drugs available for non-medical and criminal use and fueled a new wave of addiction and injury. It has been estimated that 60% of the opioids to which people are addicted come, directly or indirectly, through doctors' prescriptions.[63]

140. Law enforcement agencies have increasingly associated prescription drug addiction with violent and property crimes. Despite strict federal regulation of prescription drugs, local law enforcement agencies are faced with increasing diversion from legitimate sources for illicit purposes, including doctor shopping, forged prescriptions, falsified pharmacy records, and employees who steal from their place of employment. The opioid epidemic has prompted a

[63] Nathaniel P. Katz, *Prescription Opioid Abuse: Challenges and Opportunities for Payers*, Am. J. Managed Care (Apr. 19 2013), at 5 ("The most common source of abused [opioids] is, directly or indirectly, by prescription."), http://www.ajmc.com/publications/issue/2013/2013-1-vol19-n4/Prescription-Opioid-Abuse-Challenges-and-Opportunities-for-Payers.

growing trend of crimes against pharmacies including robbery and burglary. This ongoing diversion of prescription narcotics creates a lucrative marketplace.

141. The rise in opioid addiction caused by Defendants' deceptive marketing scheme has also resulted in an explosion in heroin use. For example, heroin use has more than doubled in the past decade among adults aged 18 to 25 years.[64] Moreover, heroin-related overdoses in the United States has more than quadrupled since 2010.[65]

142. The costs and consequences of opioid addiction are staggering. For example, in 2007, the cost of healthcare due to opioid addiction and dependence was estimated at 25 billion, the cost of criminal justice was estimated at 5.1 billion, and the cost of lost workplace productivity was estimated at 25.6 billion.

143. Consequently, prescription opioid addiction and overdose have an enormous impact on the health and safety of individuals, as well as communities at large, because the consequences of this epidemic reach far beyond the addicted individual.

144. Some of the repercussions for residents of Van Zandt County include job loss, loss of custody of children, physical and mental health problems, homelessness and incarceration, which results in instability in communities often already in economic crisis and contributes to increased demand on community services such as hospitals, courts, child services, treatment centers, and law enforcement.

145. Defendants knew and should have known about these harms that their deceptive marketing has caused and continues to cause and will cause in the future. Defendants closely monitored their sales and the habits of prescribing doctors. Their sales representatives, who visited

[64] Centers for Disease Control and Prevention. Vital Signs: Today's Heroin Epidemic – More People at Risk, Multiple Drugs Abused. (https://www.cdc.gov/vitalsigns/heroin/index.html). MMWR 2015.
[65] https://www.cdc.gov/vitalsigns/heroin/index.html

doctors and attended CMEs, knew which doctors were receiving their messages and how they were responding.

146. Defendants also had access to and carefully watched government and other data that tracked the explosive rise in opioid use, addiction, injury, and death. Defendants not only knew, but intended that their misrepresentations would persuade doctors to prescribe and encourage patients to use their opioids for chronic pain.

147. Defendants' actions are neither permitted nor excused by the fact that their drug labels may have allowed, or did not exclude, the use of opioids for chronic pain. FDA approval of opioids for certain uses did not give Defendants license to misrepresent the risks and benefits of opioids. Indeed, Defendants' misrepresentations were directly contrary to pronouncements by, and guidance from, the FDA based on the medical evidence and their own labels.

148. Nor is Defendants' causal role broken by the involvement of doctors. Defendants' marketing efforts were ubiquitous and highly persuasive. Their deceptive messages tainted virtually every source doctors could rely on for information and prevented them from making informed treatment decisions. Defendants also hijacked what doctors wanted to believe – namely, that opioids represented a means of relieving their patients' suffering and of practicing medicine more compassionately.

149. Defendants' actions and omissions were each a cause-in-fact of Van Zandt County's past and future damages. Defendants' wrongful conduct caused injuries to Van Zandt County in the past, continues to cause injuries to Van Zandt County, and will continue to cause injuries to Van Zandt County in the future. Future damages include, but are not limited to, additional resources for counseling and medication assisted treatment of addicts, medical treatment for overdoses, life skills training for adolescents, increased law enforcement, and additional

resources to treat the psychological effects of opioids and the underlying conditions that make people susceptible to opioid addiction.

## I. Defendants' Fraudulent Marketing Has Led To Record Profits.

150. While using opioids has taken an enormous toll on Van Zandt County and its residents, Defendants have realized blockbuster profits. In 2014 alone, opioids generated $11 billion in revenue for drug companies like Defendants. Indeed, financial information indicates that each Defendant experienced a material increase in sales, revenue, and profits from the false and deceptive advertising and other unlawful and unfair conduct described above.

## VI. FIRST CAUSE OF ACTION: PUBLIC NUISANCE AGAINST ALL DEFENDANTS

151. Van Zandt County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

152. Defendants knowingly encouraged doctors in and around Van Zandt County to prescribe, and residents to use, highly addictive opioids for chronic pain even though Defendants knew using opioids had a high risk of addiction and reduced quality of life.

153. By doing so, Defendants purposefully interfered with Van Zandt County's public health, public safety, public peace, public comfort, and public convenience.

154. Defendants, individually and in concert with each other, have contributed to and/or assisted in creating and maintaining a condition that is harmful to the health and safety of Van Zandt County residents and/or unreasonably interferes with the peace and comfortable enjoyment of life in violation of Texas law.

155. The public nuisance created by Defendants' actions is substantial and unreasonable – it has caused and continues to cause significant harm to the community – and the harm inflicted outweighs any offsetting benefit.

156. The staggering rates of opioid use resulting from Defendants' marketing efforts have caused, and continues to cause, harm to the community including, but not limited to:

a. Upwards of 30% of all adults use opioids. These high rates of use have led to unnecessary opioid addiction, overdose, injuries, and deaths;

b. Children have been exposed to opioids prescribed to family members or others resulting in injury, addiction, and death. Easy access to prescription opioids has made opioids a recreational drug of choice among Van Zandt County teenagers; opioid use among teenagers is only outpaced by marijuana use. Even infants have been born addicted to opioids due to prenatal exposure causing severe withdrawal symptoms and lasting developmental impacts;

c. Residents of Van Zandt County, who have never taken opioids, have endured both the emotional and financial costs of caring for loved ones addicted to or injured by opioids and the loss of companionship, wages, or other support from family members who have used, become addicted to, overdosed on, or been killed by opioids;

d. More broadly, opioid use and addiction have driven Van Zandt County residents' health care costs higher;

e. Employers have lost the value of productive and healthy employees who have suffered from adverse consequences from opioid use;

f. Defendants' success in extending the market for opioids to new patients and chronic conditions has created an abundance of drugs available for criminal use and fueled a new wave of addiction and injury. Defendants' scheme created both ends of a new secondary market for opioids – providing both the supply of narcotics to sell and the demand of addicts to buy them;

g. This demand has created additional illicit markets in other opiates, particularly heroin. The low cost of heroin has led some of those who initially become addicted to prescription opioids to migrate to cheaper heroin, fueling a new heroin epidemic in the process;

h. Diverting opioids into secondary, criminal markets and increasing the number of individuals who are addicted to opioids has increased the demands on emergency services and law enforcement in Van Zandt County;

i. All of Defendants' actions have caused significant harm to the community – in lives lost; addictions endured; the creation of an illicit drug market and all its concomitant crime and costs; unrealized economic productivity; and broken families and homes;

j. These harms have taxed the human, medical, public health, law enforcement, and financial resources of Van Zandt County; and

k. Defendants' interference with the comfortable enjoyment of life of a substantial number of people is entirely unreasonable because there is limited social utility to opioid use and any potential value is outweighed by the gravity of harm inflicted by Defendants' actions.

157. Defendants knew, or should have known, that promoting opioid use would create a public nuisance in the following ways:

a. Defendants have engaged in massive production, promotion, and distribution of opioids for use by the citizens of Van Zandt County;

b. Defendants' actions created and expanded the market for opioids, promoting its wide use for pain management;

c. Defendants misrepresented the benefits of opioids for chronic pain and fraudulently concealed, misrepresented, and omitted the serious adverse effects of opioids, including the addictive nature of the drugs; and

d. Defendants knew or should have known that their promotion would lead to addiction and other adverse consequences that the larger community would suffer as a result.

158. Defendants' actions were, at the least, a substantial factor in doctors and patients not accurately assessing and weighing the risks and benefits of opioids for chronic pain thereby causing opioids to become widely available and used in Van Zandt County.

159. Without Defendants' actions, opioid use would not have become so widespread and the enormous public health hazard of opioid addiction would not have existed and could have been averted.

160. The health and safety of the citizens of Van Zandt County, including those who use, have used, or will use opioids, as well as those affected by opioid users, is a matter of great public interest and legitimate concern to Van Zandt County's citizens and residents.

161. The public nuisance created, perpetuated, and maintained by Defendants can be

abated and further reoccurrence of such harm and inconvenience can be prevented.

162. Defendants' conduct has affected and continues to affect a considerable number of people within Van Zandt County and is likely to continue to cause significant harm to patients who take opioids, their families, and the community at large.

163. Each Defendant created or assisted in creating the opioid epidemic, and each Defendant is jointly and severally liable for its abatement. Furthermore, each Defendant should be enjoined from continuing to create, perpetuate, or maintain said public nuisance in Van Zandt County.

## VII. SECOND CAUSE OF ACTION: COMMON LAW FRAUD AGAINST ALL DEFENDANTS

164. Van Zandt County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

165. At all relevant and material times, Defendants expressly and/or impliedly warranted that opioids were safe, of merchantable quality, and fit for use.

166. Defendants' superior knowledge and expertise, its relationship of trust and confidence with doctors and the public, its specific knowledge regarding the risks and dangers of opioids, and its intentional dissemination of promotional and marketing information about opioids for the purpose of maximizing sales, each gave rise to the affirmative duty to meaningfully disclose and provide all material information about the risks and harms associated with opioids.

167. At all times herein mentioned, Defendants, individually and acting through their employees and agents, and in concert with each other, fraudulently represented to physicians who Defendants knew would justifiably rely on Defendants' representations that opioids were safe and effective for treating chronic pain.

168. Defendants' false representations were fraudulently made, with the intent or

purpose that healthcare providers and patients would justifiably rely upon them, leading to the prescription, administration, filling, purchasing, and consumption of opioids in Van Zandt County.

169. Defendants' deliberate misrepresentations and/or concealment, suppression, and omission of material facts as alleged herein include, but are not limited to:

a. Making false and misleading claims regarding the known risks of the addictive nature of opioids and suppressing, failing to disclose, and mischaracterizing the addictive nature of opioids and in concomitant costs, such as overdoses, deaths, and heroin addiction;

b. Making false and misleading written and oral statements that opioids are more effective than traditional pain killers for chronic pain, or effective at all and/or omitting material information showing that opioids are no more effective than other non-addictive drugs for chronic pain;

c. Issuing false and misleading warnings and/or failing to issue adequate warnings concerning the risks and dangers of using opioids;

d. Making false and misleading claims downplaying the risk of addiction when using opioids and/or setting forth guidelines that would purportedly identify addictive behavior; and

e. Making false and misleading misrepresentations concerning the safety, efficacy and benefits of opioids without full and adequate disclosure of the underlying facts which rendered such statements false and misleading.

170. Defendants willfully, wantonly, and recklessly disregarded their duty to provide truthful representations regarding the safety and risk of opioids.

171. Defendants made these misrepresentations with the intent that the healthcare community and patients located wherever these opioid drugs were sold or consumed would rely upon them.

172. Defendants' misrepresentations were made with the intent of defrauding and deceiving the medical community and consumers to induce and encourage the sale of opioids.

173. Defendants' fraudulent representations evidence their callous, reckless, willful, and depraved indifference to the health, safety, and welfare of consumers living in Van Zandt County.

174. Defendants omitted, misrepresented, suppressed and concealed material facts

concerning the dangers and risk of injuries associated with the use of opioids, as well as the fact that the product was unreasonably dangerous.

175. Defendants' purpose was willfully blind to, ignored, downplayed, avoided, and/or otherwise understated the serious nature of the risks associated with the use of opioids.

176. The treating medical community and consumers in Van Zandt County did not know that Defendants' representations were false and/or misleading and justifiably relied on them.

177. Defendants had sole access to material facts concerning the dangers and unreasonable risks of opioids, which they intentionally concealed.

178. As a direct and proximate result of Defendants' fraudulent misrepresentations and intentional concealment of facts, upon which the medical community and consumers in Van Zandt County reasonably relied, Van Zandt County suffered actual and punitive damages.

## VIII. THIRD CAUSE OF ACTION: NEGLIGENCE AGAINST MANUFACTURING AND DISTRIBUTING DEFENDANTS

179. Van Zandt County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

180. Manufacturing Defendants have a duty to exercise reasonable care in marketing its opioids to physicians treating residents of Van Zandt County and Van Zandt County residents. Manufacturing Defendants have breached their duty by knowingly and fraudulently misrepresenting the benefits of, and downplaying the risks of, opioids for chronic pain.

181. Manufacturing Defendants have used deceitful marketing ploys, KOLs, Front Groups, and other schemes to increase profits at the cost of public health causing an opioid epidemic. Manufacturing Defendants have acted willfully, wantonly, and maliciously.

182. Likewise, Distributor Defendants have a duty to exercise ordinary care in distributing opioids. Distributor Defendants have breached their duty by failing to prevent or reduce the distribution of opioids, or to report the increase in the distribution and/or sale of opioids.

183. Distributor Defendants have intentionally failed to prevent or reduce the distribution of opioids, or to report any increases in the sale of opioids, so that they could increase profits and receive rebates or kick-backs from Manufacturing Defendants. Distributor Defendants have acted willfully, wantonly, and maliciously.

184. As a proximate result, Manufacturing and Distributor Defendants and its agents have caused Van Zandt County to incur excessive costs to treat the opioid epidemic in its county, including but not limited to increased costs of social services, health systems, law enforcement, judicial system, and treatment facilities.

185. Van Zandt County and its residents are therefore entitled to actual and punitive damages.

## IX. FOURTH CAUSE OF ACTION: GROSS NEGLIGENCE AGAINST MANUFACTURING AND DISTRIBUTING DEFENDANTS

186. Van Zandt County incorporates the allegations within all prior paragraphs within this Complaint as if they were fully set forth herein.

187. Defendants' marketing scheme to optimize profits by misrepresenting and falsely touting opioids as the panacea to chronic pain was done intentionally.

188. Defendants' hiring of KOLs, Front Groups, and others to spread its fraudulent message that opioids were useful and beneficial for chronic pain was grossly negligent and done with conscious indifference or reckless disregard for the safety of others.

189. Each Defendant's actions and omissions as described herein, singularly or in combination with each other, was malicious resulting in damages and injuries to Van Zandt County and its residents.

190. At every stage, Defendants knew or should have known that their conduct would create an unreasonable risk of physical harm to others, including Van Zandt County and its residents,

and should be held liable in punitive and exemplary damages to Van Zandt County.

## X. FIFTH CAUSE OF ACTION: TEXAS CONTROLLED SUBSTANCES ACT ("TCSA") AGAINST DISTRIBUTOR DEFENDANTS

191. Van Zandt County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

192. Distributor Defendants have knowingly distributed, delivered, administered, or dispensed a controlled substance in violation of the Texas Controlled Substances Act §481.128(a)(1) by deceiving practitioners into prescribing, dispensing, delivering, or administering a controlled substance, or causing a controlled substance to be administered when there is no valid medical purpose. Tex. Health & Safety Code §481.071.

193. As alleged herein, each Distributor Defendant, at all times relevant to this Complaint, violated the Texas Controlled Substance Act by making deceptive representations about using opioids to treat chronic pain. Each Distributor Defendant also omitted or concealed material facts and failed to correct prior misrepresentations and omissions about the risks and benefits of opioids. Each Distributor Defendant's omissions rendered even their seemingly truthful statements about opioids deceptive.

194. Distributor Defendants' deceptive representations and concealments were reasonably calculated to deceive practitioners treating Van Zandt County residents into prescribing opioids without any valid medical purpose, and Distributor Defendants continue to do so to this day.

195. As a direct and proximate cause of Distributor Defendants' conduct, Van Zandt County should be awarded civil penalties pursuant to the Texas Controlled Substances Act.

## XI. SIXTH CAUSE OF ACTION: UNJUST ENRICHMENT AGAINST ALL DEFENDANTS

196. Van Zandt County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

197. As an expected and intended result of their conscious wrongdoing as set forth in this Complaint, Defendants have profited and benefited from opioid purchases made by Van Zandt County and its residents.

198. When Van Zandt County and its residents purchased opioids, they expected that Defendants had provided necessary and accurate information regarding those risks. Instead, Defendants had misrepresented the material facts regarding the risks and benefits of opioids.

199. Defendants have been unjustly enriched at the expense of Van Zandt County, and Van Zandt County is therefore entitled to damages to be determined by the jury.

## PRAYER FOR RELIEF

WHEREFORE, Plaintiff respectfully prays:

a. That the acts alleged herein be adjudged and decreed to be unlawful and that the Court enter a judgment declaring them to be so;

b. That Defendants be enjoined from, directly or indirectly through KOLs, Front Groups or other third parties, continuing to misrepresent the risks and benefits of the use of opioids for chronic pain, and from continuing to violate Texas law;

c. That Plaintiff recover all measures of damages, including punitive and exemplary damages, allowable under the law, and that judgment be entered against Defendants in favor of Plaintiff;

d. That Plaintiff recover restitution on behalf of Van Zandt County consumers who paid for opioids for chronic pain;

e. That Plaintiff recover the costs and expenses of suit, pre- and post-judgment interest, and reasonable attorneys' fees as provided by law; and

f. That Defendants be ordered to abate the public nuisance that they created in

in violation of Texas common law.

Date: December 29, 2017

Respectfully Submitted,

**Van Zandt County District Attorney's Office**

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# APPENDIX H

## TRAVIS PETITION

2/5/2018 3:00 PM
Velva L. Price
District Clerk
Travis County
D-1-GN-18-000625
Carrisa Escalante

CAUSE NO. D-1-GN-18-000625

| | | |
|---|---|---|
| **COUNTY OF TRAVIS,** | § | **IN THE DISTRICT COURT** |
| | § | |
| *Plaintiff,* | § | |
| | § | |
| vs. | § | **261ST JUDICIAL DISTRICT** |
| | § | |
| **PURDUE PHARMA L.P.;** | § | |
| **PURDUE PHARMA INC.;** | § | |
| **THE PURDUE FREDERICK COMPANY;** | § | |
| **JOHNSON & JOHNSON;** | § | **TRAVIS COUNTY, TEXAS** |
| **JANSSEN PHARMACEUTICALS, INC.;** | § | |
| **ORTHO-MCNEIL-JANSSEN PHARMACEUTICALS, INC.** n/k/a JANSSEN PHARMACEUTICALS, INC.; | § | |
| **JANSSEN PHARMACEUTICA, INC.** n/k/a JANSSEN PHARMACEUTICALS, INC.; | § | |
| **ENDO HEALTH SOLUTIONS INC.;** | § | |
| **ENDO PHARMACEUTICALS, INC.;** | § | |
| **ABBVIE INC.;** | § | |
| **KNOLL PHARMACEUTICAL COMPANY,** a wholly-owned subsidiary of ABBVIE INC.; | § | |
| **ALLERGAN PLC** f/k/a ACTAVIS PLC; | § | |
| **ALLERGAN FINANCE LLC** f/k/a ACTAVIS, INC. f/k/a WATSON PHARMACEUTICALS, INC.; | § | |
| **WATSON LABORATORIES, INC.;** | § | |
| **ACTAVIS LLC;** | § | |
| **ACTAVIS PHARMA, INC.** f/k/a WATSON PHARMA, INC.; | § | |
| **MCKESSON CORPORATION;** | § | |
| **CARDINAL HEALTH, INC.;** | § | |
| **AMERISOURCEBERGEN DRUG CORPORATION**; and | § | |
| **DOES 1 – 100, INCLUSIVE,** | § | |
| | § | |
| *Defendants.* | § | |

## PLAINTIFF'S ORIGINAL PETITION AND JURY DEMAND

**TO THE HONORABLE JUDGE OF SAID COURT:**

Plaintiff, the County of Travis, Texas, by and through the undersigned attorneys, (hereinafter "Travis County" or "County") against Defendants Purdue Pharma L.P., Purdue Pharma Inc., The Purdue Frederick Company, Johnson & Johnson, Janssen Pharmaceuticals, Inc., Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc., Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc., Endo Health Solutions Inc., Endo Pharmaceuticals, Inc., Abbvie Inc., Knoll Pharmaceutical Company, a wholly-owned subsidiary of Abbvie Inc., Allergan PLC f/k/a Actavis PLC, Allergan Finance, LLC f/k/a Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc., Watson Laboratories, Inc., Actavis LLC, Actavis Pharma, Inc. f/k/a Watson Pharma, Inc., McKesson Corporation, Cardinal Health, Inc., AmerisourceBergen Drug Corporation, and Does 1 – 100, alleges as follows:

## I. DISCOVERY CONTROL PLAN

1. Travis County intends to conduct discovery under Level 3 of Texas Rule of Civil Procedure 190.4 and affirmatively pleads that this suit is not governed by the expedited-actions process in Texas Rule of Civil Procedure 169 because Travis County seeks monetary relief over $100,000.

## II. INTRODUCTION

2. The United States is in the midst of an opioid epidemic caused by Defendants' fraudulent marketing and sales of prescription opioids ("opioids") that has resulted in addiction, criminal activity, and loss of life. The opioid crisis has been described as "the AIDS epidemic of our generation, but even worse."[1] On October 26, 2017, President Donald Trump "declared a

[1] David Wright, "Christie on Opioids: "This is the AIDS Epidemic of Our Generation, but even Worse," (Oct. 27, 2017), http://www.cnn.com/2017/10/27/politics/chris-christie-opioid-commission-aids-cnntv/index.html.

nationwide public health emergency to combat the opioid crisis."[2]

3. In 2016 alone, health care providers wrote more than 289 million prescriptions for opioids, enough for *every adult in the United States* to have more than one bottle of pills.[3] Americans "consume 85% of all the opioids in the world" and are "the most medicated country in the world . . . ."[4]

4. Unfortunately, using opioids too often leads to addiction and overdose from opioids. In 2014, almost 2 million Americans were addicted to opioids.[5] That same year, more people died from drug overdoses than in any other year, and most overdose deaths involved an opioid. The Texas Legislature has found "that deaths resulting from the use of opioids and other controlled substances constitute a public health crisis."[6] In 2015, Texas "had the second highest total healthcare costs from opioid abuse in the nation ($1.96 billion) . . . ."[7]

5. In fact, accidental drug overdose deaths, of which reportedly at least two-thirds are opioid overdoses, are the leading cause of death for Americans under the age of 50. Accidental drug overdose deaths, predominantly from opioids, exceed the number of deaths caused by cars or guns.

6. The economic burden caused by opioid abuse in the United States is at least $78.5

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2 Dan Merica, "What Trump's Opioid Announcement Means – and Doesn't Mean," (Oct. 26, 2017), http://www.cnn.com/2017/10/26/politics/national-health-emergency-national-disaster/index.html.

3 *Prevalence of Opioid Misuse*, BupPractice (Sept. 7, 2017), https://www.buppractice.com/node/15576.

4 David Wright, "Christie on Opioids: "This is the AIDS Epidemic of Our Generation, but even Worse," (Oct. 27, 2017), http://www.cnn.com/2017/10/27/politics/chris-christie-opioid-commission-aids-cnntv/index.html.

5 Substance Abuse and Mental Health Services Administration, National Survey on Drug Use and Health, 2014.

6 Opinion of the Attorney General of Texas, KP-0168 (Oct. 4, 2017), *citing* Act of May 26, 2017, 85th Leg., R.S., ch. 534, §3, 2017 Tex. Sess. Law Serv. 1467, 1468.

7 Kerry Craig, "Opioid Addiction Results in one Woman's Daily Struggle," Oct. 7, 2017, https://www.ssnewstelegram.com/news/opioid-addiction-results-in-one-woman-s-daily-struggle/article_bded4eoa-ab80-11e7-a252-d3f304e26628.html.

billion,[8] including lost productivity and increased social services, health insurance costs, increased criminal justice presence and strain on judicial resources, and substance abuse treatment and rehabilitation.

7. This epidemic did not occur by chance. Defendants manufacture, market, distribute, and sell prescription opioids, including, but not limited to, brand-name drugs like OxyContin, Vicodin, Opana, Percocet, Percodan, Duragesic, Ultram, Ultracet, and generics like oxycodone, oxymorphone, hydromorphone, hydrocodone, fentanyl, and tramadol, which are powerful narcotics.

8. Historically, opioids were considered too addictive and debilitating for treating non-cancer chronic pain,[9] such as back pain, migraines, and arthritis, and were used only to treat short-term acute pain or for palliative or end-of-life care.

9. By the late 1990s or early 2000s, however, each Manufacturing Defendant[10] began a marketing scheme to persuade doctors and patients that opioids can and should be used ubiquitously and perpetually to treat moderate, non-cancer chronic pain. Each Manufacturing Defendant spent large sums of money to promote the benefits of opioids for non-cancer moderate pain while trivializing, or even denying, their risks. The Manufacturing Defendants' promotional messages deviated substantially from any approved labeling of the drugs and caused prescribing physicians and consuming patients to underappreciate the health risks, and to overestimate the

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[8] *See CDC Foundation's New Business Pulse Focuses on Opioid Overdose Epidemic*, Centers for Disease Control and Prevention (Mar. 15, 2017), https://www.cdc.gov/media/releases/2017/a0315-business-pulse-opioids.html.

[9] "Chronic pain" means non-cancer pain lasting three months or longer.

[10] The "Manufacturing Defendants" are Purdue Pharma L.P., Purdue Pharma Inc., The Purdue Frederick Company, Johnson & Johnson, Janssen Pharmaceuticals, Inc., Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc., Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc., Endo Health Solutions Inc., Endo Pharmaceuticals, Inc., Abbvie Inc., Knoll Pharmaceutical Company, a wholly-owned subsidiary of Abbvie Inc., Allergan PLC f/k/a Actavis PLC, Allergan Finance, LLC f/k/a Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc., Watson Laboratories, Inc., Actavis LLC, Actavis Pharma, Inc. f/k/a Watson Pharma, Inc.

benefits, of opioids.

10. Contrary to the language of their drugs' labels, Defendants falsely and misleadingly, in their marketing: (1) downplayed the serious risk of addiction; (2) promoted and exaggerated the concept of "pseudoaddiction" thereby advocating that the signs of addiction should be treated with more opioids; (3) exaggerated the effectiveness of screening tools in preventing addiction; (4) claimed that opioid dependence and withdrawal are easily managed; (5) denied the risks of higher opioid dosages; and (6) exaggerated the effectiveness of "abuse-deterrent" opioid formulations to prevent abuse and addiction.

11. Defendants disseminated these falsehoods through ads and/or their sales representatives and physicians who supported Defendants' message. Sales representatives, working at Defendants' behest, promoted highly addictive opioids through souvenirs and toys including, but not limited to, opioid brand-bearing stuffed plush toys, dolls, coffee cups, fanny packs, water bottles, notepads, pens, refrigerator magnets, clocks, letter openers, rulers, daytime planners, bags, puzzles, posters, hand-held calculators, clipboards, highlighters, flashlights, key chains, clothing, reflex mallets, and mock-ups of the United States Constitution.

12. Defendants also used third parties they controlled by: (a) funding, assisting, encouraging, and directing doctors, known as "key opinion leaders" ("KOLs") and (b) funding, assisting, directing, and encouraging seemingly neutral and credible professional societies and patient advocacy groups (referred to hereinafter as "Front Groups").

13. Defendants worked with KOLs and Front Groups to taint the sources that doctors and patients relied on for ostensibly "neutral" guidance, such as treatment guidelines, Continuing Medical Education ("CME") programs, medical conferences and seminars, and scientific articles. After their individual and concerted efforts, Defendants convinced doctors that instead of being

addictive and unsafe for long-term use in most circumstances, opioids were *required* in the compassionate treatment of chronic pain.

14. The Distributor Defendants were not standing by idly while Manufacturing Defendants were peddling their opioids to physicians and consumers. Cardinal, AmerisourceBergen, and McKesson ("Distributor Defendants") are three of the largest opioid distributors in the United States. Distributor Defendants purchased opioids from Manufacturing Defendants herein and sold them to pharmacies servicing consumers in Travis County.

15. Despite the rise in the ordering of opioids by retailers in Travis County, Distributor Defendants did nothing. The Manufacturing Defendants and Distributor Defendants worked hand and glove to flood the U.S. and, upon information and belief, Travis County with more opioids than would be consumed for therapeutic purposes. Each Defendant disregarded its legal duty to report suspicious opioid prescriptions, and each Defendant financially benefitted from the other Defendant's (both Manufacturing and Distributor Defendants), disregarding their individual duties to report.

16. Essentially each Defendant ignored science and consumer health for profits. Defendants' efforts were so successful that opioids are now the most prescribed class of drugs generating $11 billion in revenue for drug companies in 2014 alone. Even after Defendant Purdue reached a $600 million federal settlement in 2007, the settlement failed to impact what is a "$13-billion-a-year opioid industry."[11]

17. Upon information and belief, as a direct and foreseeable consequence of Defendants' misrepresentations regarding the safety and efficacy of using opioids for chronic pain, Travis County has spent and continues to spend large sums combatting the public health

[11] Rebecca L. Haffajee, J.D., Ph.D., M.P.H., and Michelle M. Mello, J.D., Ph.D., *Drug Companies' Liability for the Opioid Epidemic*, N. Engl. J. Med. at 2305, (Dec. 14, 2017).

crisis created by Defendants' negligent and fraudulent marketing campaign.

18. For example, scores of prescriptions were written for opioids in Travis County in 2012[12] and in 2012 there were multiple deaths reported from drug overdoses.[13] A substantial number of those overdose deaths were a result, in whole or in part, of opioid ingestion. In each year from 2013-2017, there were multiple deaths in Travis County caused in whole or in part from ingestion of prescription opioids. Defendants' marketing misconduct, as well as Defendants' efforts to sell more prescription opioids that can be consumed therapeutically, were natural and foreseeable causes of overdose deaths and injuries in Travis County.

19. Upon information and belief, as a direct and foreseeable consequence of Defendants' conduct described regarding prescription opioids, Travis County has committed and continues to commit resources to provide and pay for health care, law enforcement, social services, public assistance, pharmaceutical care and other services necessary for its residents.

## III. RULE 47 STATEMENT OF MONETARY RELIEF SOUGHT

20. Per Rule 47 of the Texas Rules of Civil Procedure, the County states that although the full measure of its damages is still being calculated, its damages caused by Defendants' acts and omissions exceed $1,000,000 but are believed to be less than $100,000,000. Accordingly, at this time in the litigation, Travis County states that it is seeking monetary relief for an amount greater than $1,000,000 and less than $100,000,000, the rightful and just amount to be determined by the jury.

## IV. VENUE AND JURISDICTION

21. Venue is proper in Travis County because all or a substantial part of the events or

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[12] https://www.cdc.gov/drugoverdose/maps/rxrate-maps.html; https://www.cdc.gov/nchs/data-visualization/drug-poisoning-mortality.

[13] https://www.cdc.gov/nchs/data-visualization/drug-poisoning-mortality.

omissions giving rise to this claim occurred in Travis County. TEX. CIV. PRAC. & REM. CODE §15.002(a)(2). This Court has subject-matter jurisdiction over this matter because Plaintiff's damages are in excess of the minimal jurisdictional limits of this Court. TEX. GOVT. CODE §24.007(b).

22. This Court has general jurisdiction over Purdue Pharma L.P. as it is a Texas citizen. This Court also has specific jurisdiction over all Defendants as their activities were directed toward Texas, and injuries complained of herein resulted from their activities. *Guardian Royal Exchange Assur., Ltd. v. English China Clays, P.L.C.*, 815 S.W.2d 223, 227 (Tex. 1991). Each Defendant has a substantial connection with Texas and the requisite minimum contacts with Texas necessary to constitutionally permit the Court to exercise jurisdiction. *See id.* at 226.

## V. PARTIES

### A. Plaintiff

23. This action is brought for and on behalf of Travis County, which provides a wide range of services on behalf of its residents, including services for families and children, public health, public assistance, law enforcement, and emergency care.

### B. Defendants

24. PURDUE PHARMA L.P. is a limited partnership organized under the laws of Delaware, and may be served through its registered agent for service of process, The Prentice-Hall Corporation System, Inc., 251 Little Falls Drive, Wilmington, DE 19808. PURDUE PHARMA L.P. is, through its ownership structure, a Texas resident. PURDUE PHARMA INC. is a New York corporation with its principal place of business in Stamford, Connecticut, and may be served through its registered agent for service of process, Corporation Service Company, 80 State Street, Albany, NY 12207. Furthermore, Purdue Pharma L.P. is a Texas citizen. THE

PURDUE FREDERICK COMPANY is a Delaware corporation with its principal place of business in Stamford, Connecticut, and may be served through its registered agent for service of process, The Prentice-Hall Corporation System, Inc., 251 Little Falls Drive, Wilmington, DE 19808 (collectively, "Purdue").

25. Purdue manufactures, promotes, sells, and distributes opioids in the U.S. and Travis County. Purdue's opioid drug, OxyContin, is among the most addictive and abused prescription drugs in the history of America. Purdue promotes opioids throughout the United States and in Travis County.

26. JANSSEN PHARMACEUTICALS, INC. is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey, and may be served through its registered agent for service of process, CT Corporation System, 1999 Bryan Street, Suite 900, Dallas, TX 75201. JANSSEN PHARMACEUTICALS, INC. is a wholly owned subsidiary of JOHNSON & JOHNSON (J&J), a New Jersey corporation with its principal place of business in New Brunswick, New Jersey, and may be served through its registered agent for service of process, Attention: Legal Department, One Johnson & Johnson Plaza, New Brunswick, NJ 08933. ORTHO-MCNEIL-JANSSEN PHARMACEUTICALS, INC., now known as JANSSEN PHARMACEUTICALS, INC., is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey. JANSSEN PHARMACEUTICA INC., now known as JANSSEN PHARMACEUTICALS, INC., is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey. J&J is the only company that owns more than 10% of Janssen Pharmaceuticals' stock, and corresponds with the FDA regarding Janssen's products. Upon information and belief, J&J controls the sale and development of Janssen Pharmaceuticals' drugs and Janssen's profits inure to J&J's benefit. (Janssen Pharmaceuticals, Inc., Ortho-McNeil-

Janssen Pharmaceuticals, Inc., Janssen Pharmaceutica, Inc., and J&J are referred to as "Janssen").

27. Janssen manufactures, promotes, sells, and distributes opioids in the U.S. and in Travis County.

28. ENDO HEALTH SOLUTIONS INC. is a Delaware corporation with its principal place of business in Malvern, Pennsylvania, and may be served through its registered agent for service of process, The Corporation Trust Company, Corporation Trust Center, 1209 Orange St., Wilmington, DE 19801. ENDO PHARMACEUTICALS, INC. is a wholly-owned subsidiary of Endo Health Solutions Inc. and is a Delaware corporation with its principal place of business in Malvern, Pennsylvania, and may be served through its registered agent for service of process, CT Corporation System, 1999 Bryan Street, Suite 900, Dallas, TX 75201. (Endo Health Solutions Inc. and Endo Pharmaceuticals, Inc. are referred to as "Endo").

29. Endo develops, markets, and sells opioid drugs in the U.S. and in Travis County. Endo also manufactures and sells generic opioids in the U.S. and Travis County, by itself and through its subsidiary, Qualitest Pharmaceuticals, Inc.

30. ABBVIE INC. ("Abbvie") is a Delaware corporation with its principal place of business in North Chicago, Illinois, and may be served through its registered agent for service of process, CT Corporation System, 208 S. LaSalle Street, Suite 814, Chicago, IL 60604. KNOLL PHARMACEUTICAL COMPANY ("Knoll") has been a wholly-owned subsidiary of Abbvie from January 1, 2013. KNOLL PHARMACEUTICAL COMPANY is a New Jersey corporation with its principal place of business in Parsippany, New Jersey, and may be served through its registered agent for service of process, CT Corporation System, 208 S. LaSalle Street, Suite 814, Chicago, IL 60604.

31. Knoll irresponsibly marketed narcotics, such as Vicodin, through whimsical toys and souvenirs and did so to boost the sales of opioids. Taking advantage of the fact that Vicodin was not regulated as a Schedule II controlled substance for many years, and the fact physicians and consumers did not fully appreciate the highly addictive nature of Vicodin, Knoll advertised Vicodin with tag lines such as "The Highest Potency Pain Relief You Can Still Phone In." This tag line came as part and parcel of souvenirs like a "Vicodin" fanny pack and water bottle, both bearing the name of Vicodin, the opioid Knoll was promoting. This irresponsible marketing of a narcotic drug caused doctors and patients to believe Vicodin was safer than it really was, to the detriment of people in Travis County.

32. Abbvie began manufacturing, developing, promoting, marketing, and selling the opioid drug, Vicodin, in the U.S. and in Travis County beginning January 1, 2013. On information and belief, it continues to do so at the time of filing this pleading.

33. ALLERGAN PLC is a public limited company incorporated in Ireland with its principal place of business in Dublin, Ireland. ACTAVIS PLC acquired Allergan plc in March 2015, and the combined company changed its name to Allergan plc in January 2013. Before that, WATSON PHARMACEUTICALS, INC. acquired ACTAVIS, INC. in October 2012, and the combined company changed its name to ALLERGAN FINANCE, LLC as of October 2013. ALLERGAN FINANCE, LLC is a Nevada Corporation with its principal place of business in Parsippany, New Jersey, and may be served through its registered agent for service of process, The Corporation Trust Company of Nevada, 701 S. Carson St., Suite 200, Carson City, NV 89701. WATSON LABORATORIES, INC. is a Nevada corporation with its principal place of business in Corona, California, and is a wholly-owned subsidiary of Allergan plc (f/k/a Actavis, Inc., f/k/a Watson Pharmaceuticals, Inc.), and may be served through its registered agent for

service of process, Corporate Creations Network, Inc., 8275 South Eastern Ave., #200, Las Vegas, NV 89123. ACTAVIS PHARMA, INC. (f/k/a Actavis, Inc.) is a Delaware corporation with its principal place of business in New Jersey and was formerly known as WATSON PHARMA, INC, and may be served through its registered agent for service of process, CT Corporation System, 1999 Bryan Street, Suite 900, Dallas, TX 75201. ACTAVIS LLC is a Delaware limited liability company with its principal place of business in Parsippany, New Jersey, and may be served through its registered agent for service of process, Corporate Creations Network, Inc., 3411 Silverside Rd., Tatnall Building, Suite 104, Wilmington, DE 19810. Each of these Defendants is owned by Allergan plc, which uses them to market and sell its drugs in the United States.

34. Upon information and belief, Allergan plc exercises control over these marketing and sales efforts and profits from the sale of Allergan/Actavis products ultimately inure to its benefit. (Allergan plc, Actavis plc, Actavis, Inc., Allergan Finance, LLC, Actavis LLC, Actavis Pharma, Inc., Watson Pharmaceuticals, Inc., Watson Pharma, Inc., and Watson Laboratories, Inc. are referred to as "Actavis"). Actavis acquired the rights to Kadian from King Pharmaceuticals, Inc. on December 30, 2008 and began marketing Kadian in 2009.

35. Actavis manufactures, promotes, sells, and distributes opioids in the U.S. and in Travis County.

36. MCKESSON CORPORATION ("McKesson") is a Delaware corporation with its principal place of business in San Francisco, California, and may be served through its registered agent for service of process, CSC - Lawyers Incorporating Service, 211 E. 7th Street, Suite 620, Austin, TX 78701. Upon information and belief, McKesson is a pharmaceutical distributor licensed to do business in Texas. McKesson distributes pharmaceuticals to retail pharmacies and

institutional providers in all 50 states, including Texas and Travis County.

37. CARDINAL HEALTH, INC. ("Cardinal") is an Ohio Corporation with its principal place of business in Dublin, Ohio, and may be served through its registered agent for service of process, CT Corporation System, 4400 Easton Commons, Suite 125, Columbus, OH 43219. Cardinal does substantial business in Texas and, upon information and belief, Cardinal is a pharmaceutical distributor licensed to do business in Texas. Cardinal distributes pharmaceuticals to retail pharmacies and institutional providers to customers in all 50 states, including Texas and Travis County.

38. AMERISOURCEBERGEN DRUG CORPORATION ("Amerisource") is a Delaware Corporation with its principal place of business in Chesterbrook, Pennsylvania, and may be served through its registered agent for service of process, The Corporation Trust Company, Corporation Trust Center, 1209 Orange St., Wilmington, DE 19801. Amerisource does substantial business in Texas and, upon information and belief, Amerisource is a pharmaceutical distributor licensed to do business in Texas. Amerisource distributes pharmaceuticals to retail pharmacies and institutional providers to customers in all 50 states, including Texas and Travis County.

39. The County lacks information sufficient to specifically identify the true names or capacities, whether individual, corporate or otherwise, of Defendants sued herein under the fictitious names DOES 1 through 100 inclusive. The County will amend this Petition to show their true names and capacities if and when they are ascertained. Travis County is informed and believes, and on such information and belief alleges, that each of the Defendants named as a DOE has engaged in conduct that contributed to cause events and occurrences alleged in this Petition and, as such, shares liability for at least some part of the relief sought herein.

## VI. FACTUAL ALLEGATIONS

40. Before the 1990s, generally accepted standards of medical practice dictated that opioids should be used only for short-term acute pain – pain relating to recovery from surgery or for cancer or palliative (end-of-life) care. Using opioids for chronic pain was discouraged or even prohibited because there was a lack of evidence that opioids improved patients' ability to overcome pain and function. Instead the evidence demonstrated that patients developed tolerance to opioids over time, which increased the risk of addiction and other side effects.

41. Defendants dramatically changed doctors' views regarding opioids through a well-funded deceptive marketing scheme. Each Defendant used direct marketing and unbranded advertising disseminated by seemingly independent third parties to spread false and deceptive statements about the risks and benefits of long-term opioid use.

### A. Defendants Used Multiple Avenues To Disseminate their False and Deceptive Statements about Opioids.

42. Upon information and belief, Defendants spread their false and deceptive statements by (1) marketing their branded opioids directly to doctors treating patients residing in Travis County and the Travis County patients themselves and (2) deploying so-called unbiased and independent third parties to Travis County.

#### 1. Defendants Spread and Continue to Spread Their False and Deceptive Statements Through Direct Marketing of Their Branded Opioids.

43. Defendants' direct marketing of opioids generally proceeded on two tracks. First, each Defendant conducted advertising campaigns touting the purported benefits of their branded drugs. For example, Defendants spent more than $14 million on medical journal advertising of opioids in 2011, nearly triple what they spent in 2001, including $8.3 million by Purdue, $4.9 million by Janssen, and $1.1 million by Endo.

44. A number of Defendants' branded ads deceptively portrayed the benefits of opioids for chronic pain. For example, Endo distributed and made available on its website, www.opana.com, a pamphlet promoting Opana ER with photographs depicting patients with physically demanding jobs like a construction worker and chef, implying that the drug would provide long-term pain-relief and functional improvement. Purdue also ran a series of ads, called "pain vignettes," for OxyContin in 2012 in medical journals. These ads featured chronic pain patients and recommended OxyContin for each. One ad described a "54-year-old writer with osteoarthritis of the hands" and implied that OxyContin would help the writer work more effectively. Pursuant to a settlement agreement, Endo and Purdue agreed in late 2015 and 2016 to halt these misleading representations in New York, but they may continue to disseminate them in Texas.

45. Second, each Defendant promoted the use of opioids for chronic pain through "detailers" – sales representatives who visited individual doctors and medical staff in their offices – and small-group speaker programs. Defendants devoted massive resources to direct sales contacts with doctors. In 2014 alone, Defendants spent $168 million on detailing branded opioids to doctors, including $108 million by Purdue, $34 million by Janssen, $10 million by Endo, and $2 million by Actavis. This amount is twice as much as Defendants spent on detailing in 2000.

46. Defendants also identified doctors to serve, for payment, on their speakers' bureaus and to attend programs with speakers and meals paid for by Defendants. These speaker programs provided: (1) an incentive for doctors to prescribe a particular opioid (so they might be selected to promote the drug); (2) recognition and compensation for the doctors selected as speakers; and (3) an opportunity to promote the drug through the speaker to his or her peers. These speakers gave the false impression that they were providing unbiased and medically

accurate presentations when they were, in fact, presenting a script prepared by Defendants. On information and belief, these presentations conveyed misleading information, omitted material information, and failed to correct Defendants' prior misrepresentations about the risks and benefits of opioids.

47. Upon information and belief, Defendants employed the same marketing plans, strategies, and messages in and around Travis County, Texas as they did nationwide. Across the pharmaceutical industry, "core message" development is funded and overseen on a national basis by corporate headquarters. This comprehensive approach ensures that Defendants' messages are accurately and consistently delivered across marketing channels and in each sales territory. Defendants consider this high level of coordination and uniformity crucial to successfully marketing their drugs.

**2. Defendants Used a Diverse Group of Seemingly Independent Third Parties to Spread False and Deceptive Statements about the Risks and Benefits of Opioids.**

48. Upon information and belief, Defendants also deceptively marketed opioids in and around Travis County through unbranded advertising – *i.e.*, advertising that promotes opioid use generally but does not name a specific opioid. This advertising was ostensibly created and disseminated by independent third parties. But by funding, directing, reviewing, editing, and distributing this unbranded advertising, Defendants controlled the deceptive messages disseminated by these third parties and acted in concert with them to falsely and misleadingly promote opioids for treating chronic pain.

49. Unbranded advertising also avoided regulatory scrutiny because Defendants did not have to submit it to the FDA, and therefore it was not reviewed by the FDA.

50. Defendants' deceptive unbranded marketing often contradicted their branded

materials reviewed by the FDA. For example, Endo's unbranded advertising contradicted its concurrent, branded advertising for Opana ER:

| Pain: Opioid Therapy (Unbranded) | Opana ER Advertisement (Branded) |
|---|---|
| "People who take opioids **as prescribed usually do not become addicted**." | "All patients treated with opioids require careful monitoring for signs of abuse and addiction, since **use of opioid analgesic products carries the risk of addiction even under appropriate medical use**." |

**a. Key Opinion Leaders (KOLs)**

51. Defendants spoke through a small circle of doctors who, upon information and belief, were selected, funded, and elevated by Defendants because their public positions supported using opioids to treat chronic pain. These doctors became known as "key opinion leaders" or "KOLs."

52. Defendants paid KOLs to serve as consultants or on their advisory boards and to give talks or present CMEs, and their support helped these KOLs become respected industry experts. As they rose to prominence, these KOLs touted the benefits of opioids to treat chronic pain, repaying Defendants by advancing their marketing goals. KOLs' professional reputations became dependent on continuing to promote a pro-opioid message, even in activities that were not directly funded by Defendants.

53. KOLs have written, consulted on, edited, and lent their names to books and articles, and given speeches and CMEs supportive of chronic opioid therapy. Defendants created opportunities for KOLs to participate in research studies Defendants suggested or chose and then cited and promoted favorable studies or articles by their KOLs. By contrast, Defendants did not support, acknowledge, or disseminate publications of doctors unsupportive or critical of chronic opioid therapy.

54. Defendants' KOLs also served on committees that developed treatment guidelines that strongly encourage using opioids to treat chronic pain, and on the boards of pro-opioid advocacy groups and professional societies that develop, select, and present CMEs. Defendants were able to direct and exert control over each of these activities through their KOLs.

55. Pro-opioid doctors are one of the most important avenues that Defendants use to spread their false and deceptive statements about the risks and benefits of long-term opioid use. Defendants know that doctors rely heavily and less critically on their peers for guidance, and KOLs provide the false appearance of unbiased and reliable support for chronic opioid therapy.

56. Defendants utilized many KOLs, including many of the same ones. Two of the most prominent are described below.

**1. Russell Portenoy**

57. Dr. Russell Portenoy, former Chairman of the Department of Pain Medicine and Palliative Care at Beth Israel Medical Center in New York, is one example of a KOL who Defendants identified and promoted to further their marketing campaign. Dr. Portenoy received research support, consulting fees, and honoraria from Endo, Janssen, and Purdue (among others), and was a paid consultant to Purdue.

58. Dr. Portenoy was instrumental in opening the door for the regular use of opioids to treat chronic pain. He served on the American Pain Society ("APS")/American Academy of Pain Medicine ("AAPM") Guidelines Committees, which endorsed the use of opioids to treat chronic pain, first in 1997 and again in 2009. He was also a member of the board of the American Pain Foundation ("APF"), an advocacy organization almost entirely funded by Defendants.

59. Dr. Portenoy also made frequent media appearances promoting opioids. He appeared on *Good Morning America* in 2010 to discuss using opioids long-term to treat chronic

pain. On this widely-watched program, broadcast in Texas and across the country, Dr. Portenoy claimed: "Addiction, when treating pain, is distinctly uncommon. If a person does not have a history, a personal history, of substance abuse, and does not have a history in the family of substance abuse, and does not have a very major psychiatric disorder, most doctors can feel very assured that that person is not going to become addicted."[14]

60. Dr. Portenoy later admitted that he "gave innumerable lectures in the late 1980s and '90s about addiction that weren't true."[15] These lectures falsely claimed that less than 1% of patients would become addicted to opioids. According to Dr. Portenoy, because the primary goal was to "destigmatize" opioids, he and other doctors promoting them overstated their benefits and glossed over their risks. Dr. Portenoy also conceded that "[d]ata about the effectiveness of opioids does not exist."[16] Portenoy candidly stated: "Did I teach about pain management, specifically about opioid therapy, in a way that reflects misinformation? Well...I guess I did."[17]

**2. Lynn Webster**

61. Another KOL, Dr. Lynn Webster, was the co-founder and Chief Medical Director of Lifetree Clinical Research, an otherwise unknown pain clinic in Salt Lake City, Utah. Dr. Webster was President in 2013 and is a current board member of AAPM, a Front Group that ardently supports chronic opioid therapy. He is a Senior Editor of *Pain Medicine*, the same journal that published Endo special advertising supplements touting Opana ER. Dr. Webster authored numerous CMEs sponsored by Endo and Purdue while he was receiving significant funding from Defendants.

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14 Good Morning America television broadcast, ABC News (Aug. 30, 2010).

15 Thomas Catan & Evan Perez, *A Pain-Drug Champion Has Second Thoughts*, WALL ST. J., Dec. 17, 2012.

16 *Id.*

17 *Id.*

62. In 2011, Dr. Webster presented a program via webinar sponsored by Purdue titled, *Managing Patient's Opioid Use: Balancing the Need and the Risk.* Dr. Webster recommended using risk screening tools, such as urine testing and patient agreements as a way to prevent "overuse of prescriptions" and "overdose deaths," which was available to and was, upon information and belief, intended to reach doctors treating Travis County residents.

63. Dr. Webster also was a leading proponent of the concept of "pseudoaddiction," the notion that addictive behaviors should be seen not as warnings, but as indications of undertreated pain. In Dr. Webster's description, the only way to differentiate the two was to *increase* a patient's dose of opioids. As he and his co-author wrote in a book entitled *Avoiding Opioid Abuse While Managing Pain* (2007), a book that is still available online, when faced with signs of aberrant behavior, increasing the dose "in most cases . . . should be the clinician's first response." Endo distributed this book to doctors. Years later, Dr. Webster reversed himself, acknowledging that "[pseudoaddiction] obviously became too much of an excuse to give patients more medication."[18]

**b. Front Groups**

64. Defendants entered into arrangements with seemingly unbiased and independent patient and professional organizations to promote opioids for treating chronic pain. Under Defendants' direction and control, these "Front Groups" generated treatment guidelines, unbranded materials, and programs that favored chronic opioid therapy. They also assisted Defendants by responding to negative articles, by advocating against regulatory changes that would limit prescribing opioids in accordance with the scientific evidence, and by conducting outreach to vulnerable patient populations targeted by Defendants.

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[18] John Fauber & Ellen Gabler, *Networking Fuels Painkiller Boom*, MILWAUKEE WISC. J. SENTINEL (Feb. 19, 2012).

65. These Front Groups depended on Defendants for funding and, in some cases, for survival. Defendants also exercised control over programs and materials created by these groups by collaborating on, editing, and approving their content, and by funding their dissemination. In doing so, Defendants made sure these Groups would generate only the messages Defendants wanted to distribute. Even so, the Front Groups held themselves out as independent and as serving the needs of their members – whether patients suffering from pain or doctors treating those patients.

66. Defendants Endo, Janssen, and Purdue utilized many Front Groups, including many of the same ones. Several of the most prominent are described below, but there are many others, including the American Pain Society ("APS"), American Geriatrics Society ("AGS"), the Federation of State Medical Boards ("FSMB"), American Chronic Pain Association ("ACPA"), American Society of Pain Education ("ASPE"), National Pain Foundation ("NPF") and Pain & Policy Studies Group ("PPSG").

**1. American Pain Foundation ("APF")**

67. The most prominent of Defendants' Front Groups was APF, which received more than $10 million in funding from opioid manufacturers from 2007 until it closed its doors in May 2012. Endo alone provided more than half that funding; Purdue was next at $1.7 million.

68. In 2009 and 2010, more than 80% of APF's operating budget came from pharmaceutical industry sources. Including industry grants for specific projects, APF received about $2.3 million from industry sources out of total income of about $2.85 million in 2009; its budget for 2010 projected receipts of roughly $2.9 million from drug companies, out of total income of about $3.5 million. By 2011, APF was entirely dependent on incoming grants from defendants Purdue, Endo, and others to avoid using its line of credit. As one of its board members, Russell Portenoy, explained the lack of funding diversity was one of the biggest

problems at APF.

69. APF issued education guides for patients, reporters, and policymakers that touted the benefits of opioids for chronic pain and trivialized their risks, particularly the risk of addiction. APF also engaged in a significant multimedia campaign – through radio, television, and the internet – to educate patients about their "right" to pain treatment, namely opioids. All of the programs and materials were available nationally and were, upon information and belief, intended to reach patients and consumers in Travis County.

**2. American Academy of Pain Medicine ("AAPM")**

70. The American Academy of Pain Medicine, with the assistance, prompting, involvement, and funding of Defendants, issued treatment guidelines and sponsored and hosted medical education programs essential to Defendants' deceptive marketing of chronic opioid therapy.

71. AAPM received over $2.2 million in funding since 2009 from opioid manufacturers. AAPM maintained a corporate relations council, whose members paid $25,000 per year (on top of other funding) to participate. The benefits included allowing members to present educational programs at off-site dinner symposia in connection with AAPM's marquee event – its annual meeting held in Palm Springs, California, or other resort locations. AAPM describes the annual event as an "exclusive venue" for offering education programs to doctors. Membership in the corporate relations council also allows drug company executives and marketing staff to meet with AAPM executive committee members in small settings. Defendants Endo, Purdue, and Actavis were members of the council and presented deceptive programs to doctors who attended this annual event.

72. AAPM is viewed internally by Endo as "industry friendly," with Endo advisors

and speakers among its active members. Endo attended AAPM conferences, funded its CMEs, and distributed its publications. The conferences sponsored by AAPM heavily emphasized sessions on opioids – 37 out of roughly 40 at one conference alone. AAPM's presidents have included top industry-supported KOLs Perry Fine, Russell Portenoy, and Lynn Webster. Dr. Webster was even elected president of AAPM while under a DEA investigation. Another past AAPM president, Dr. Scott Fishman, stated that he would place the organization "at the forefront" of teaching that "the risks of addiction are . . . small and can be managed."[19]

73. AAPM's staff understood they and their industry funders were engaged in a common task. Defendants were able to influence AAPM through both their significant and regular funding and the leadership of pro-opioid KOLs within the organization.

74. In 1997, AAPM and the American Pain Society jointly issued a consensus statement, *The Use of Opioids for the Treatment of Chronic Pain*, which endorsed opioids to treat chronic pain and claimed there was a low risk that patients would become addicted to opioids. The co-author of the statement, Dr. Haddox, was at the time a paid speaker for Purdue. Dr. Portenoy was the sole consultant. The consensus statement remained on AAPM's website until 2011, and was taken down from AAPM's website only after a doctor complained, though it still lingers on the internet elsewhere.

75. AAPM and APS issued their own guidelines in 2009 ("AAPM/APS Guidelines") and continued to recommend using opioids to treat chronic pain. Fourteen of the 21 panel members who drafted the AAPM/APS Guidelines, including KOLs Dr. Portenoy and Dr. Perry Fine of the University of Utah, received support from Janssen, Endo, and Purdue.

76. The 2009 Guidelines promote opioids as "safe and effective" for treating chronic

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[19] Interview by Paula Moyer with Scott M. Fishman, M.D., Professor of Anesthesiology and Pain Medicine, Chief of the Division of Pain Medicine, Univ. of Cal., Davis (2005), http://www.medscape.org/viewarticle/500829.

pain, despite acknowledging limited evidence, and conclude that the risk of addiction is manageable for patients regardless of past abuse histories. One panel member, Dr. Joel Saper, Clinical Professor of Neurology at Michigan State University and founder of the Michigan Headache & Neurological Institute, resigned from the panel because he was concerned the 2009 Guidelines were influenced by contributions that drug companies, including Defendants, made to the sponsoring organizations and committee members. These AAPM/APS Guidelines have been a particularly effective channel of deception and have influenced not only treating physicians, but also the body of scientific evidence on opioids. The Guidelines have been cited 732 times in academic literature, were, upon information and belief, disseminated in and around Travis County during the relevant time period, are still available online, and were reprinted in the *Journal of Pain.*

**B. Defendants' Marketing Scheme Misrepresented the Risks and Benefits of Opioids.**

77. Upon information and belief, to convince doctors treating residents in Travis County and Travis County patients that opioids can and should be used to treat chronic pain, Defendants had to convince them that long-term opioid use is both safe and effective. Knowing they could do so only by deceiving those doctors and patients about the risks and benefits of long-term opioid use, Defendants made claims that were not supported by, or were contrary to, the scientific evidence. Even though pronouncements by and guidance from the FDA and the CDC based on that evidence confirm that their claims were false and deceptive, Defendants have not corrected them, or instructed their KOLs or Front Groups to correct them, and continue to spread them today.

**C. Defendants Falsely Trivialized or Failed to Disclose the Known Risks of Long-Term Opioid Use.**

78. To convince doctors and patients that opioids are safe, Defendants deceptively

trivialized and failed to disclose the risks of long-term opioid use, particularly the risk of addiction, through a series of misrepresentations that have been conclusively debunked by the FDA and CDC. These misrepresentations – which are described below – reinforced each other and created the dangerously misleading impression that: (1) starting patients on opioids was low-risk because most patients would not become addicted, and because those who were at greatest risk of addiction could be readily identified and managed; (2) patients who displayed signs of addiction probably were not addicted and, in any event, could easily be weaned from the drugs; (3) the use of higher opioid doses, which many patients need to sustain pain relief as they develop tolerance to the drugs, do not pose special risks; and (4) abuse-deterrent opioids both prevent overdose and are inherently less addictive. Defendants have not only failed to correct these misrepresentations, they continue to make them today.

79. ***First***, Defendants falsely claimed the risk of addiction is low and unlikely to develop when opioids are prescribed, as opposed to obtained illicitly; and failed to disclose the greater risk of addiction with prolonged use of opioids. For example:

a. Actavis's predecessor caused a patient education brochure to be distributed in 2007 claiming opioid addiction is possible, but "less likely if you have never had an addiction problem." Upon information and belief, based on Actavis's acquisition of its predecessor's marketing materials along with the rights to Kadian, Actavis continued to use this brochure in 2009 and beyond;

b. Purdue sponsored APF's *Treatment Options: A Guide for People Living with Pain* (2007), which instructed that addiction is rare and limited to extreme cases of unauthorized dose escalations, obtaining duplicative opioid prescriptions from multiple sources, or theft. This publication is still available online;

c. Endo sponsored a website, Painknowledge.com, which claimed in 2009 that "[p]eople who take opioids as prescribed usually do not become addicted." Another Endo website, PainAction.com, stated "Did you know? Most chronic pain patients do not become addicted to the opioid medications that are prescribed for them;"

d. Endo distributed a pamphlet with the Endo logo entitled *Living with Someone with Chronic Pain*, which stated that: "Most health care providers who treat people with pain agree that most people do not develop an addiction problem." A similar statement appeared on the Endo website, www.opana.com;

e. Janssen reviewed, edited, approved, and distributed a patient education guide entitled *Finding Relief: Pain Management for Older Adults* (2009), which described as "myth" the claim that opioids are addictive, and asserted as fact that "[m]any studies show that opioids are rarely addictive when used properly for the management of chronic pain;"

f. Janssen currently runs a website, Prescriberesponsibly.com (last updated July 2, 2015), which claims that concerns about opioid addiction are "overestimated;"

g. Purdue sponsored APF's *A Policymaker's Guide to Understanding Pain & Its Management* – which claims that less than 1% of children prescribed opioids will become addicted and that pain is undertreated due to "misconceptions about opioid addiction[]." This publication is still available online; and

h. Upon information and belief, detailers for Purdue, Endo, and Janssen in and around Travis County minimized or omitted any discussion with doctors of the risk of addiction; misrepresented the potential for opioid abuse with purportedly abuse-deterrent formulations; and routinely did not correct the misrepresentations noted above.

80. These claims contradict longstanding scientific evidence, as the FDA and CDC have conclusively declared. As noted in the 2016 CDC Guideline endorsed by the FDA, there is "extensive evidence" of the "possible harms of opioids (including opioid use disorder [an alternative term for opioid addiction])."[20] The guideline points out that "[o]pioid pain medication use presents serious risks, including...opioid use disorder" and that "continuing opioid therapy for 3 months substantially increases risk for opioid use disorder."[21]

81. The FDA further exposed the falsity of Defendants' claims about the low risk of

[20] *CDC Guideline for Prescribing Opioids for Chronic Pain – United States 2016*, Centers for Disease Control and Prevention (Mar. 18, 2016).

[21] *Id.*

addiction when it announced changes to the labels for ER/LA opioids in 2013 and for IR opioids in 2016. In its announcements, the FDA discussed the risks related to opioid use and that IR opioids are associated with "persistent abuse, addiction, overdose mortality, and risk of NOWS [neonatal opioid withdrawal syndrome]."[22]

82. According to the FDA, because of the risks associated with long-term opioid use, including "the serious risk of addiction, abuse, misuse, overdose, and death,"[23] opioids should be "reserved for pain severe enough to require opioid treatment and for which alternative treatment options (e.g., non-opioid analgesics or opioid combination products, as appropriate) are inadequate or not tolerated."[24]

83. The warnings on Defendants' own FDA-approved drug labels caution that opioids "exposes users to risks of addiction, abuse and misuse, which can lead to overdose and death"[25] and that addiction "can occur in patients appropriately prescribed"[26] opioids.

84. ***Second***, Defendants falsely instructed doctors and patients that signs of addiction are actually signs of undertreated pain and should be treated by prescribing more opioids. Defendants called this phenomenon "pseudoaddiction" – a term coined by Dr. David Haddox, who went to work for Purdue, and popularized by Dr. Russell Portenoy, a KOL for Endo, Janssen, and Purdue – and claimed that pseudoaddiction is substantiated by scientific evidence. For example:

a. Purdue sponsored *Responsible Opioid Prescribing* (2007), which taught

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22 *FDA Announcement of Enhanced Warnings for Immediate-Release Opioid Pain Medications Related to Risks of Misuse, Abuse, Addiction, Overdose and Death*, Federal Drug Administration (Mar. 22, 2016).

23 *Id.*

24 *Id.*

25 *See, e.g.*, OxyContin label and insert at *OxyContin.com.*

26 *Id.*

that behaviors such as "requesting drugs by name," "demanding or manipulative behavior," seeing more than one doctor to obtain opioids, and hoarding, are all signs of pseudoaddiction, rather than true addiction. Responsible Opioid Prescribing remains for sale online. The 2012 edition continues to teach that pseudoaddiction is real;

b. Janssen sponsored, funded, and edited the *Let's Talk Pain* website, which in 2009 stated: "pseudoaddiction . . . refers to patient behaviors that may occur when pain is under-treated . . . . Pseudoaddiction is different from true addiction because such behaviors can be resolved with effective pain management;"

c. Endo sponsored a National Initiative on Pain Control (NIPC) CME program in 2009 titled *Chronic Opioid Therapy: Understanding Risk While Maximizing Analgesia*, which promoted pseudoaddiction by teaching that a patient's aberrant behavior was the result of untreated pain. Endo substantially controlled NIPC by funding NIPC projects; developing, specifying, and reviewing content; and distributing NIPC materials;

d. Purdue published a pamphlet in 2011 entitled *Providing Relief, Preventing Abuse*, which described pseudoaddiction as a concept that "emerged in the literature" to describe the inaccurate interpretation of [drug-seeking behaviors] in patients who have pain that has not been effectively treated;" and

e. Purdue sponsored a CME program entitled *Path of the Patient, Managing Chronic Pain in Younger Adults at Risk for Abuse*. In a role play, a chronic pain patient with a history of drug abuse tells his doctor that he is taking twice as many hydrocodone pills as directed. The narrator notes that because of pseudoaddiction, the doctor should not assume the patient is addicted even if he persistently asks for a specific drug, seems desperate, hoards medicine, or "overindulges in unapproved escalating doses." The doctor treats this patient by prescribing a high-dose, long-acting opioid.

85. The 2016 CDC Guideline rejects the concept of pseudoaddiction. The Guideline nowhere recommends that opioid dosages be increased if a patient is not experiencing pain relief. To the contrary, the Guideline explains that "[p]atients who do not experience clinically meaningful pain relief early in treatment...are unlikely to experience pain relief with longer-term

use,"[27] and that physicians should "reassess[] pain and function within 1 month"[28] in order to decide whether to "minimize risks of long-term opioid use by discontinuing opioids"[29] because the patient is "not receiving a clear benefit."[30]

86. ***Third,*** Defendants falsely instructed doctors and patients that addiction risk screening tools, patient contracts, urine drug screens, and similar strategies allow them to reliably identify and safely prescribe opioids to patients predisposed to addiction. These misrepresentations were especially insidious because Defendants aimed them at general practitioners and family doctors who lack the time and expertise to closely manage higher-risk patients. Defendants' misrepresentations made these doctors feel more comfortable prescribing opioids to their patients, and patients more comfortable starting opioid therapy for chronic pain. For example:

a. Endo paid for a 2007 supplement in the *Journal of Family Practice* written by a doctor who became a member of Endo's speakers' bureau in 2010. The supplement, entitled *Pain Management Dilemmas in Primary Care: Use of Opioids*, emphasized the effectiveness of screening tools, claiming that patients at high risk of addiction could safely receive chronic opioid therapy using a "maximally structured approach" involving toxicology screens and pill counts;

b. Purdue sponsored a 2011 webinar, *Managing Patient's Opioid Use: Balancing the Need and Risk*, which claimed that screening tools, urine tests, and patient agreements prevent "overuse of prescriptions" and "overdose deaths;" and

c. As recently as 2015, Purdue has represented in scientific conferences that "bad apple" patients – and not opioids – are the source of the addiction crisis and that once those "bad apples" are identified, doctors can safely prescribe opioids without causing addiction.

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27 *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*

28 *Id.*

29 *Id.*

30 *Id.*

87. Once again, the 2016 CDC Guideline confirms these representations are false. The Guideline notes that there are no studies assessing the effectiveness of risk mitigation strategies – such as screening tools, patient contracts, urine drug testing, or pill counts – widely believed by doctors to detect and deter outcomes related to addiction and overdose.[31] As a result, the Guideline recognizes that doctors should not overestimate the risk screening tools for classifying patients as high or low risk for opioid addiction because they are insufficient to rule out the risks of long-term opioid therapy.[32]

88. ***Fourth***, to underplay the risk and impact of addiction and make doctors feel more comfortable starting patients on opioids, Defendants falsely claimed that opioid dependence can easily be addressed by tapering and that opioid withdrawal is not a problem thereby failing to disclose the increased difficulty of stopping opioids after long-term use.

89. For example, a CME sponsored by Endo, entitled *Persistent Pain in the Older Adult*, claimed that withdrawal symptoms can be avoided by tapering a patient's opioid dose by 10%-20% for 10 days. And Purdue sponsored APF's *A Policymaker's Guide to Understanding Pain & Its Management*, which claimed that "[s]ymptoms of physical dependence can often be ameliorated by gradually decreasing the dose of medication during discontinuation."

90. Defendants deceptively minimized the significant symptoms of opioid withdrawal, which, as explained in the 2016 CDC Guideline, include drug cravings, anxiety, insomnia, abdominal pain, vomiting, diarrhea, sweating, tremor, tachycardia (rapid heartbeat), spontaneous abortion and premature labor in pregnant women, and the unmasking of anxiety, depression, and addiction – and grossly understated the difficulty of tapering, particularly after long-term opioid use.

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31 *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*

32 *See id.*

91. Yet the 2016 CDC Guideline recognizes that the duration of opioid use and the dosage of opioids prescribed should be limited to "minimize the need to taper opioids to prevent distressing or unpleasant withdrawal symptoms,"[33] because "physical dependence on opioids is an expected physiologic response in patients exposed to opioids for more than a few days."[34] The Guideline further states that "tapering opioids can be especially challenging after years on high dosages because of physical and psychological dependence"[35] and highlights the difficulties, including the need to carefully identify "a taper slow enough to minimize symptoms and signs of opioid withdrawal"[36] and pausing and restarting tapers depending on the patient's response.

92. The CDC also acknowledges the lack of any "high-quality studies comparing the effectiveness of different tapering protocols for use when opioid dosage is reduced or opioids are discontinued."[37]

93. ***Fifth***, Defendants falsely claimed that doctors and patients could increase opioid dosages indefinitely without added risk and failed to disclose the greater risks to patients at higher dosages. The ability to escalate dosages was critical to Defendants' efforts to market opioids for long-term use to treat chronic pain because, absent this misrepresentation, doctors would have abandoned treatment when patients built up tolerance and lower dosages did not provide pain relief. For example:

a. Actavis's predecessor created a patient brochure for Kadian in 2007 that stated, "Over time, your body may become tolerant of your current dose. You may require a dose adjustment to get the right amount of pain relief.

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33 *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*

34 *Id.*

35 *Id.*

36 *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*

37 *Id.*

This is not addiction." Upon information and belief, based on Actavis's acquisition of its predecessor's marketing materials along with the rights to Kadian, Actavis continued to use these materials in 2009 and beyond;

b. Purdue sponsored *APF's Treatment Options: A Guide for People Living with Pain* (2007), which claims that some patients "need" a larger dose of an opioid, regardless of the dose currently prescribed. The guide stated that opioids have "no ceiling dose" and are therefore the most appropriate treatment for severe pain. This guide is still available for sale online;

c. Endo sponsored a website, painknowledge.com, which claimed in 2009 that opioid dosages may be increased until "you are on the right dose of medication for your pain;"

d. Endo distributed a pamphlet edited by a KOL entitled *Understanding Your Pain: Taking Oral Opioid Analgesics*, which was available during the time period of this Complaint on Endo's website. In Q&A format, it asked "If I take the opioid now, will it work later when I really need it?" The response is, "The dose can be increased. . . . You won't 'run out' of pain relief;"

e. Janssen sponsored a patient education guide entitled *Finding Relief: Pain Management for Older Adults* (2009), which was distributed by its sales force. This guide listed dosage limitations as "disadvantages" of other pain medicines but omitted any discussion of risks of increased opioid dosages;

f. Purdue's In the Face of Pain website promotes the notion that if a patient's doctor does not prescribe what, in the patient's view, is a sufficient dosage of opioids, he or she should find another doctor who will;

g. Purdue sponsored APF's *A Policymaker's Guide to Understanding Pain & Its Management*, which taught that dosage escalations are "sometimes necessary," even unlimited ones, but did not disclose the risks from high opioid dosages. This publication is still available online;

h. Purdue sponsored a CME entitled *Overview of Management Options* that is still available for CME credit. The CME was edited by a KOL and taught that NSAIDs and other drugs, but not opioids, are unsafe at high dosages; and

i. Purdue presented a 2015 paper at the College on the Problems of Drug Dependence, the "the oldest and largest organization in the US dedicated to advancing a scientific approach to substance use and addictive disorders," challenging the correlation between opioid dosage and overdose.

94. These claims conflict with the scientific evidence, as confirmed by the FDA and CDC. As the CDC explains in its 2016 Guideline, the "[b]enefits of high-dose opioids for chronic pain are not established"[38] while the "risks for serious harms related to opioid therapy increase at higher opioid dosage."[39]

95. More specifically, the CDC explains that "there is now an established body of scientific evidence showing that overdose risk is increased at higher opioid dosages."[40] Similarly, there is an "increased risk for opioid use disorder, respiratory depression, and death at higher dosages."[41] That is why the CDC advises doctors to avoid increasing dosages above 90 morphine milligram equivalents per day.

96. The 2016 CDC Guideline reinforces earlier findings announced by the FDA. In 2013, the FDA acknowledged that available data suggested that increasing the opioid dosage likewise increased certain adverse events. For example, the FDA noted that studies suggest a positive association between high-dose opioid use and overdoses.

97. ***Finally***, Defendants' deceptive marketing of the so-called abuse-deterrent properties of some of their opioids has created false impressions that these opioids can curb addiction and abuse.

98. More specifically, Defendants have made misleading claims about the ability of their so-called abuse-deterrent opioid formulations to deter addiction and overdose. For example, Endo's advertisements for the 2012 reformulation of Opana ER claimed that it was designed to be crush resistant in a way that suggested it was more difficult to misuse the product. This claim

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38 *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*

39 *Id.*

40 *Id.*

41 *Id.*

was false.

99. The FDA warned in a 2013 letter that there was no evidence Endo's design would provide a reduction in oral, intranasal or intravenous use.[42] Moreover, Endo's own studies, which it failed to disclose, showed that Opana ER could still be ground and chewed.

100. In a 2016 settlement with the State of New York, Endo agreed not to make statements in New York that Opana ER was designed to be or is crush resistant. The State found those statements false and deceptive because there was no difference in the ability to extract the narcotic from Opana ER.

101. Similarly, the 2016 CDC Guideline states that no studies support the notion that "abuse-deterrent technologies [are] a risk mitigation strategy for deterring or preventing abuse,"[43] noting that the technologies – even when they work – "do not prevent opioid abuse through oral intake, the most common route of opioid abuse, and can still be abused by non-oral routes."[44]

102. These numerous, long-standing misrepresentations of the risks of long-term opioid use spread by Defendants successfully convinced doctors and patients to discount those risks.

**D. Defendants Grossly Overstated the Benefits of Chronic Opioid Therapy.**

103. To convince doctors and patients that opioids should be used to treat chronic pain, Defendants had to persuade them that there was a significant benefit to long-term opioid use. But as the 2016 CDC Guideline makes clear, there is "insufficient evidence to determine the long-term benefits of opioid therapy for chronic pain."[45]

104. In fact, the CDC found no evidence showing "a long-term benefit of opioids in

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42 *See FDA Statement: Original Opana ER Relisting Determination* (May 10, 2013).

43 *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*

44 *Id.*

45 *Id.*

pain and function versus no opioids for chronic pain with outcomes examined at least 1 year later (with most placebo-controlled randomized trials ≤ 6 weeks in duration)"[46] and that other treatments were more or equally beneficial and less harmful than long-term opioid use. The FDA, too, has recognized the lack of evidence to support long-term opioid use.

105. In 2013, the FDA stated that it was unaware of any studies demonstrating the safety and efficacy of opioids for long-term use.[47] Despite this lack of studies, Defendants falsely and misleadingly touted the benefits of long-term opioid use and suggested that these benefits were supported by scientific evidence. Not only have Defendants failed to correct these false and deceptive claims, they continue to make them today. For example:

a. Actavis distributed an advertisement that claimed that the use of Kadian to treat chronic pain would allow patients to return to work, relieve "stress on your body and your mental health," and help patients enjoy their lives;

b. Endo distributed advertisements that claimed that the use of Opana ER for chronic pain would allow patients to perform demanding tasks like construction work or work as a chef and portrayed seemingly healthy, unimpaired subjects;

c. Janssen sponsored and edited a patient education guide entitled *Finding Relief: Pain Management for Older Adults* (2009) – which states as "a fact" that "opioids may make it easier for people to live normally." The guide lists expected functional improvements from opioid use, including sleeping through the night, returning to work, recreation, sex, walking, and climbing stairs;

d. Purdue ran a series of advertisements for OxyContin in 2012 in medical journals entitled "pain vignettes," which were case studies featuring patients with pain conditions persisting over several months and recommending OxyContin for them. The ads implied that OxyContin improves patients' function;

e. *Responsible Opioid Prescribing* (2007), sponsored and distributed by Endo and Purdue, taught that relief of pain by opioids, by itself, improved

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46 *Id.*

47 Letter from Janet Woodcock, M.D., Dir., Ctr. For Drug Eval. & Res., to Andrew Kolodny, M.D., Pres. Physicians for Responsible Opioid Prescribing, Re Docket No. FDA-2012-P-0818 (Sept. 10, 2013).

patients' function. The book remains for sale online;

f. Purdue sponsored APF's *Treatment Options: A Guide for People Living with Pain* (2007), which counseled patients that opioids "give [pain patients] a quality of life we deserve." The guide was available online until APF shut its doors in 2012;

g. Endo's NIPC website *painknowledge.com* claimed in 2009 that with opioids, "your level of function should improve; you may find you are now able to participate in activities of daily living, such as work and hobbies, that you were not able to enjoy when your pain was worse." Elsewhere, the website touted improved quality of life (as well as "improved function") as benefits of opioid therapy. The grant request that Endo approved for this project specifically indicated NIPC's intent to make misleading claims about function, and Endo closely tracked visits to the site;

h. Endo was the sole sponsor, through NIPC, of a series of CMEs titled *Persistent Pain in the Older Patient*, which claimed that chronic opioid therapy has been "shown to reduce pain and improve depressive symptoms and cognitive functioning." The CME was disseminated via webcast;

i. Janssen sponsored, funded, and edited a website, *Let's Talk Pain*, in 2009, which featured an interview edited by Janssen claiming that opioids allowed a patient to "continue to function." This video is still available today on YouTube;

j. Purdue sponsored the development and distribution of APF's *A Policymaker's Guide to Understanding Pain & Its Management*, which claimed that "multiple clinical studies" have shown that opioids are effective in improving daily function, psychological health, and health-related quality of life for chronic pain patients." The Policymaker's Guide was originally published in 2011 and is still available online today; and

k. Purdue's, Endo's, and Janssen's sales representatives have conveyed and continue to convey the message that opioids will improve patient function.

106. These claims find no support in the scientific literature. Most recently, the 2016 CDC Guideline, approved by the FDA, concluded, "There is no good evidence that opioids improve pain or function with long-term use"[48] and "complete relief of pain is unlikely."[49]

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[48] *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*

(Emphasis added.) The CDC reinforced this conclusion throughout its 2016 Guideline:

a. "No evidence shows a long-term benefit of opioids in pain and function versus no opioids for chronic pain with outcomes examined at least 1 year later . . ."[50]

b. "Although opioids can reduce pain during short-term use, the clinical evidence review found insufficient evidence to determine whether pain relief is sustained and whether function or quality of life improves with long-term opioid therapy;"[51] and

c. "[E]vidence is limited or insufficient for improved pain or function with long-term use of opioids for several chronic pain conditions for which opioids are commonly prescribed, such as low back pain, headache, and fibromyalgia."[52]

107. The CDC also noted that the risks of addiction and death "can cause distress and inability to fulfill major role obligations."[53] As a matter of common sense (and medical evidence), drugs that can kill patients or commit them to a life of addiction or recovery do not improve their function and quality of life.

108. The 2016 CDC Guideline was not the first time a federal agency repudiated Defendants' claim that opioids improved function and quality of life. In 2010, the FDA warned Actavis that "[w]e are not aware of substantial evidence or substantial clinical experience demonstrating that the magnitude of the effect of the drug [Kadian] has in alleviating pain, taken together with any drug-related side effects patients may experience...results in any overall positive impact on a patient's work, physical and mental functioning, daily activities, or

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49 *Id.*

50 *Id.*

51 *CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.*

52 *Id.*

53 *Id.*

enjoyment of life."[54]

109. Defendants also falsely emphasized or exaggerated the risks of competing products like NSAIDs so that doctors and patients would look to opioids first for treating chronic pain. Once again, Defendants' misrepresentations contravene pronouncements by and guidance from the FDA and CDC based on the scientific evidence.

110. Consequently, the FDA changed the labels for ER/LA opioids in 2013 and IR opioids in 2016 to state that opioids should be used only as a last resort where alternative treatments like non-opioid drugs are inadequate. And the 2016 CDC Guideline states that NSAIDs, not opioids, should be the first-line treatment for chronic pain, particularly arthritis and lower back pain.

111. In addition, Purdue misleadingly promoted OxyContin as unique among opioids in providing 12 continuous hours of pain relief with one dose. In fact, OxyContin does not last for 12 hours – a fact that Purdue has known at all times relevant to this action.

112. According to Purdue's own research, OxyContin wears off in under six hours in one quarter of patients and in under 10 hours in more than half. The reason is that OxyContin tablets release approximately 40% of their active medicine immediately, after which release tapers. Although the patient experiences a powerful initial response, there is little or no pain relief at the end of the dosing period because less medicine is released.

113. This phenomenon is known as "end of dose" failure, and the FDA found in 2008 that a substantial number of chronic pain patients taking OxyContin experience it.

114. This "end of dose" failure not only renders Purdue's promise of 12 hours of relief

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[54] Warning Letter from Thomas Abrams, Dir., FDA Div. of Mktg., Adver., & Commc'ns, to Doug Boothe, CEO, Actavis Elizabeth LLC (Feb. 18, 2010), http://www.fda.gov/Drugs/GuidanceComplianceRegulatoryInformation/EnforcementActivitiesbyFDA/WarningLettersandNoticeofViolationLetterstoPharmaceuticalCcompanies/ucm259240.htm.

false and deceptive, it also makes OxyContin more dangerous because the declining pain relief patients experience toward the end of each dosing period drives them to take more OxyContin before the next dosing period begins, quickly increasing the amount of drug they are taking and spurring growing dependence.

115. Purdue's competitors were aware of this problem. For example, Endo ran advertisements for Opana ER referring to "real" 12-hour dosing. Nevertheless, Purdue falsely promoted OxyContin as if it were effective for a full 12 hours. Upon information and belief, Purdue's sales representatives continue to tell doctors in and around Travis County that OxyContin lasts a full 12 hours.

**E. Defendants also engaged in Other Unlawful, Unfair, and Fraudulent Misconduct.**

116. Defendants herein participated in illicit and unlawful prescribing of its drugs. For example, Purdue did not report illegal prescribing of OxyContin until years after law enforcement shut down a Los Angeles clinic that prescribed more than 1.1 million OxyContin tablets. In doing so, Purdue protected its own profits at the expense of public health and safety.

117. The State of New York found that Endo failed to require sales representatives to report signs of addiction, diversion, and inappropriate prescribing; paid bonuses to sales representatives for detailing prescribers who were subsequently arrested or convicted for illegal prescribing; and failed to prevent sales representatives from visiting prescribers whose suspicious conduct had caused them to be placed on a no-call list.

**F. Defendants Targeted Susceptible Prescribers and Vulnerable Patient Populations.**

118. As a part of their deceptive marketing scheme, Defendants identified and targeted susceptible prescribers and vulnerable patient populations in the U.S. and, upon information and belief, in and around Travis County. For example, Defendants focused their deceptive marketing

on primary care doctors, who were more likely to treat chronic pain patients and prescribe opioids, but were less likely to be educated about treating pain and the risks and benefits of opioids.

119. Defendants also targeted vulnerable patient populations like the elderly and veterans, who tend to suffer from chronic pain. Defendants targeted these vulnerable patients even though the risks of long-term opioid use were significantly greater for them.

120. For example, the 2016 CDC Guideline observes that existing evidence shows that elderly patients taking opioids suffer from elevated fall and fracture risks, greater risk of hospitalization, and increased vulnerability to adverse drug effects and interactions. The Guideline therefore concludes that there are "special risks of long-term opioid use for elderly patients" and recommends that doctors use "additional caution and increased monitoring" to minimize the risks of opioid use in elderly patients.

121. The same is true for veterans, who are more likely to use anti-anxiety drugs (benzodiazepines) for post-traumatic stress disorder, which interact dangerously with opioids.

## G. Although Defendants knew that their Marketing of Opioids was False and Deceptive, they Fraudulently Concealed their Misconduct.

122. Defendants, both individually and collectively, made, promoted, and profited from their misrepresentations about the risks and benefits of opioids for chronic pain even though they knew their misrepresentations were false and deceptive. The history of opioids, as well as research and clinical experience over the last 20 years, established that opioids were highly addictive and responsible for a long list of very serious adverse outcomes.

123. Not only did the FDA and other regulators warn Defendants, but Defendants had access to scientific studies, detailed prescription data, and reports of adverse events, including reports of addiction, hospitalization, and deaths – all of which made clear the harms from long-

term opioid use, including the suffering from addiction, overdoses, and death in alarming numbers in patients using opioids.

124. More recently, the FDA and CDC have issued pronouncements based on the medical evidence that conclusively expose the known falsity of Defendants' misrepresentations, and Endo and Purdue have recently entered agreements prohibiting them from making some of the same misrepresentations described herein in New York.

125. Specifically, three current and former executives from Purdue plead guilty in 2007 to criminal charges that they misled regulators, doctors, and patients about OxyContin's risk of addiction.[55] In pleading guilty to misbranding charges, Purdue admitted it had fraudulently marketed OxyContin as a drug less prone to addiction and as having fewer side effects than other opioids.[56] In reality, unlike other opioids, OxyContin contained pure oxycodone without any other ingredients, which made it a powerful narcotic despite its time-release design that Purdue touted as ameliorating its addictive potential.[57]

126. Moreover, Defendants took steps to avoid detection of and to fraudulently conceal their deceptive marketing and unlawful, unfair, and fraudulent conduct. For example, Defendants disguised their own role in the deceptive marketing of chronic opioid therapy by funding and working through third parties like Front Groups and KOLs.

127. Finally, Defendants manipulated their promotional materials and the scientific literature to make it appear that these items were accurate, truthful, and supported by objective evidence when they were not.

128. Thus, Defendants successfully concealed from the medical community and

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55 *See* Barry Meier, "In Guilty Plea, OxyContin Maker to Pay $600 Million," (May 10, 2007), http://www.nytimes.com/2007/05/10/business/11drug-web.html.

56 *See id.*

57 *See id.*

patients facts sufficient to arouse suspicion of the claims Travis County now asserts. Travis County did not know of the existence or scope of Defendants' industry-wide fraud and could not have acquired such knowledge earlier through the exercise of reasonable diligence.

## H. By Increasing Opioid Prescriptions and Use, Defendants' Deceptive Marketing Scheme has fueled the Opioid Epidemic and Devastated Travis County Communities.

129. Defendants' misrepresentations deceived doctors and patients about the risks and benefits of long-term opioid use. Studies reveal that many doctors and patients are unaware of or do not understand the risks or benefits of opioids. Indeed, patients often report that they were not warned they might become addicted to opioids prescribed to them. As reported in January 2016, a 2015 survey of more than 1,000 opioid patients found that 4 out of 10 were not told opioids were potentially addictive.[58]

130. Upon information and belief, Defendants' deceptive marketing scheme caused and continues to cause doctors in and around Travis County to prescribe opioids for chronic pain conditions such as back pain, headaches, arthritis, and fibromyalgia. Absent Defendants' deceptive marketing scheme, these doctors would not have been able to over prescribe opioids or become embroiled in pill mills that negatively impacted residents of Travis County.

131. For example, upon information and belief, Defendants' deceptive marketing scheme allowed doctors located in or near Travis County, Texas to promote, overprescribe, and financially benefit from prescribing opioids. Indeed, some doctors "knowingly or intentionally manufactured, distributed, dispensed, or possessed with the intent to manufacture, distribute, or dispense a controlled substance, including opioids such as OxyContin, Hydrocodone, and

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[58] Hazelden Betty Ford Foundation, *Missed Questions, Missed Opportunities* (Jan. 27, 2016), *available at* http://www.hazeldenbettyford.org/about-us/news-and-media/pressrelease/doctors-missing-questions-that-could-prevent-opioid-addiction.

Vicodin in violation of the Texas Controlled Substances Act in Tex. Health & Safety Code §481.001et seq.

132. If the manufacturing and distributing Defendants were not over-supplying opioids, then such physicians could not devise schemes to prescribe opioids without a legitimate purpose as a means to flood the open market with opioids, such as OxyContin, Hydrocodone, and Vicodin.

133. While Defendants may claim the federal government authorized the amount of annual prescription opioids sold, they know in truth that several Defendants have successfully used their organized money and influence to render the federal government's enforcement agency, the Drug Enforcement Administration, virtually powerless to interrupt the over-supply of prescription opioid drugs.

134. Defendants' deceptive marketing scheme also caused and continues to cause patients to purchase and use opioids for their chronic pain believing they are safe and effective. Absent Defendants' deceptive marketing scheme, fewer patients would be using opioids long-term to treat chronic pain, and those patients using opioids would be using less of them.

135. Defendants' deceptive marketing has caused and continues to cause the prescribing and use of opioids to explode. Indeed, this dramatic increase in opioid prescriptions and use corresponds with the dramatic increase in Defendants' spending on their deceptive marketing scheme. Defendants' spending on opioid marketing totaled approximately $91 million in 2000. By 2011, that spending had tripled to $288 million.

136. The escalating number of opioid prescriptions written by doctors who were deceived by Defendants' deceptive marketing scheme is the cause of a correspondingly dramatic increase in opioid addiction, overdose, and death throughout the U.S. and Travis County.

137. Scientific evidence demonstrates a strong correlation between opioid prescriptions

and becoming addicted to opioids. In a 2016 report, the CDC explained that prescribing opioids has quadrupled since 1999, which has resulted in a parallel increase in opioid overdoses.[59] Indeed, there has been a two-third increase in overdose deaths from using opioids since 2000.[60] For these reasons, the CDC concluded that efforts to rein in the prescribing of opioids for chronic pain are critical "to reverse the cycle of opioid pain medication misuse that contributes to the opioid overdose epidemic."[61]

138. Due to the increase in opioid overdoses, first responders such as police officers, have been and will continue to be in the position to assist people experiencing opioid-related overdoses.[62] In 2016, "over 1,200 law enforcement departments nationwide carried naloxone in an effort to prevent opioid-related deaths."[63]

139. Upon information and belief, Defendants' deceptive marketing scheme has also detrimentally impacted children in Travis County. Overprescribing opioids for chronic pain has made the drugs more accessible to school-aged children, who come into contact with opioids after they have been prescribed to friends or relatives in the same household.

140. Upon information and belief, Defendants' conduct has adversely affected Travis County's child protection agencies in the number of children in foster care driven by parental drug addiction. Children with parents addicted to drugs tend to stay in foster care longer, and

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[59] CDC. National Vital Statistics System, Mortality. CDC WONDER. Atlanta, GA: US Department of Health and Human Services, CDC; 2016. https://wonder.cdc.gov/; Rudd RA, Seth P, David F, Scholl L. Increases in Drug and Opioid-Involved Overdose Deaths — United States, 2010–2015. MMWR Morb Mortal Wkly Rep. ePub: 16 December 2016.

[60] *National Vital Statistics System*, Mortality file and appearing *Center for Disease Control and Prevention* Morbidity and Mortality Weekly Report, January 1, 2006 / 64(50); 1378-82, Increases in Drug and Opioid Deaths – United States, 2000-2014.

[61] *CDC Guideline for Prescribing Opioids for Chronic Pain, supra; see also* Rudd RA, Seth P, David F, Scholl L. Increases in Drug and Opioid-Involved Overdose Deaths — United States, 2010–2015. MMWR Morb Mortal Wkly Rep. ePub: 16 December 2016.

[62] Opinion of the Attorney General of Texas, KP-0168 (Oct. 4, 2017).

[63] *Id. citing* http://www.nchrc.org/law-enforcement/us-law-enforcement-who-carry-naloxone/.

they often enter the system having experienced significant trauma, which makes these cases more expensive for counties like Travis County.

141. Upon information and belief, opioid addiction is a significant reason that Travis County residents seek treatment for substance dependence. A significant number of admissions for drug addiction were associated with a primary diagnosis of opiate addiction or dependence.

142. Upon information and belief, Defendants' creation, through false and deceptive advertising and other unlawful and unfair conduct, of a virtually limitless opioid market has significantly harmed Travis County communities. Defendants' success in extending the market for opioids to new patients and chronic pain conditions has created an abundance of drugs available for non-medical and criminal use and fueled a new wave of addiction and injury. It has been estimated that 60% of the opioids to which people are addicted come, directly or indirectly, through doctors' prescriptions.[64]

143. Law enforcement agencies have increasingly associated prescription drug addiction with violent and property crimes. Despite strict federal regulation of prescription drugs, local law enforcement agencies are faced with increasing diversion from legitimate sources for illicit purposes, including doctor shopping, forged prescriptions, falsified pharmacy records, and employees who steal from their place of employment. The opioid epidemic has prompted a growing trend of crimes against pharmacies including robbery and burglary. This ongoing diversion of prescription narcotics creates a lucrative marketplace.

144. The rise in opioid addiction caused by Defendants' deceptive marketing scheme has also resulted in an explosion in heroin use. For example, heroin use has more than doubled

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[64] Nathaniel P. Katz, *Prescription Opioid Abuse: Challenges and Opportunities for Payers*, Am. J. Managed Care (Apr. 19 2013), at 5 ("The most common source of abused [opioids] is, directly or indirectly, by prescription."), http://www.ajmc.com/publications/issue/2013/2013-1-vol19-n4/Prescription-Opioid-Abuse-Challenges-and-Opportunities-for-Payers.

in the past decade among adults aged 18 to 25 years.[65] Moreover, heroin-related overdoses in the United States has more than quadrupled since 2010.[66]

145. The costs and consequences of opioid addiction are staggering. For example, in 2007, the cost of healthcare due to opioid addiction and dependence was estimated at $25 billion, the cost of criminal justice was estimated at $5.1 billion, and the cost of lost workplace productivity was estimated at $25.6 billion.

146. Consequently, prescription opioid addiction and overdose have an enormous impact on the health and safety of individuals, as well as communities at large, because the consequences of this epidemic reach far beyond the addicted individual.

147. Upon information and belief, some of the repercussions for residents of Travis County include job loss, loss of custody of children, physical and mental health problems, homelessness and incarceration, which results in instability in communities often already in economic crisis and contributes to increased demand on community services such as hospitals, courts, child services, treatment centers, and law enforcement.

148. Defendants knew and should have known about these harms that their deceptive marketing has caused and continues to cause and will cause in the future. Defendants closely monitored their sales and the habits of prescribing doctors. Their sales representatives, who visited doctors and attended CMEs, knew which doctors were receiving their messages and how they were responding.

149. Defendants also had access to and carefully watched government and other data that tracked the explosive rise in opioid use, addiction, injury, and death. Defendants not only

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[65] Centers for Disease Control and Prevention. Vital Signs: Today's Heroin Epidemic – More People at Risk, Multiple Drugs Abused. (https://www.cdc.gov/vitalsigns/heroin/index.html). MMWR 2015.

[66] https://www.cdc.gov/vitalsigns/heroin/index.html

knew, but intended that their misrepresentations would persuade doctors to prescribe and encourage patients to use their opioids for chronic pain.

150. Defendants' actions are neither permitted nor excused by the fact that their drug labels may have allowed, or did not exclude, the use of opioids for chronic pain. FDA approval of opioids for certain uses did not give Defendants license to misrepresent the risks and benefits of opioids. Indeed, Defendants' misrepresentations were directly contrary to pronouncements by, and guidance from, the FDA based on the medical evidence and their own labels.

151. Nor is Defendants' causal role broken by the involvement of doctors. Defendants' marketing efforts were ubiquitous and highly persuasive. Their deceptive messages tainted virtually every source doctors could rely on for information and prevented them from making informed treatment decisions. Defendants also hijacked what doctors wanted to believe – namely, that opioids represented a means of relieving their patients' suffering and of practicing medicine more compassionately.

152. Upon information and belief, Defendants' actions and omissions were each a cause-in-fact of Travis County's past and future damages. Upon information and belief, Defendants' wrongful conduct caused injuries to Travis County in the past, continues to cause injuries to Travis County, and will continue to cause injuries to Travis County in the future. Future damages include, but are not limited to, additional resources for counseling and medication assisted treatment of addicts, medical treatment for overdoses, life skills training for adolescents, increased law enforcement, and additional resources to treat the psychological effects of opioids and the underlying conditions that make people susceptible to opioid addiction.

## I. Defendants' Fraudulent Marketing Has Led To Record Profits.

153. While using opioids has taken a toll on Travis County and its residents,

Defendants have realized blockbuster profits. In 2014 alone, opioids generated $11 billion in revenue for drug companies like Defendants. Indeed, financial information indicates that each Defendant experienced a material increase in sales, revenue, and profits from the false and deceptive advertising and other unlawful and unfair conduct described above.

## VII. FIRST CAUSE OF ACTION: PUBLIC NUISANCE AGAINST ALL DEFENDANTS

154. Travis County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

155. Upon information and belief, Defendants knowingly encouraged doctors in and around Travis County to prescribe, and residents to use, highly addictive opioids for chronic pain even though Defendants knew using opioids had a high risk of addiction and reduced quality of life.

156. Upon information and belief, by doing so, Defendants purposefully interfered with Travis County's public health, public safety, public peace, public comfort, and public convenience.

157. Upon information and belief, Defendants, individually and in concert with each other, have contributed to and/or assisted in creating and maintaining a condition that is harmful to the health and safety of Travis County residents and/or unreasonably interferes with the peace and comfortable enjoyment of life in violation of Texas law.

158. The public nuisance created by Defendants' actions is substantial and unreasonable – it has caused and continues to cause significant harm to the community – and the harm inflicted outweighs any offsetting benefit.

159. The staggering rates of opioid use resulting from Defendants' marketing efforts have caused, and continues to cause, harm to the community including, but not limited to:

a. Upwards of 30% of all adults use opioids. These high rates of use have led to unnecessary opioid addiction, overdose, injuries, and deaths;

b. Children have been exposed to opioids prescribed to family members or others resulting in injury, addiction, and death. Upon information and belief, easy access to prescription opioids has made opioids a recreational drug of choice among Travis County teenagers; opioid use among teenagers is only outpaced by marijuana use. Even infants have been born addicted to opioids due to prenatal exposure causing severe withdrawal symptoms and lasting developmental impacts;

c. Upon information and belief, residents of Travis County, who have never taken opioids, have endured both the emotional and financial costs of caring for loved ones addicted to or injured by opioids and the loss of companionship, wages, or other support from family members who have used, become addicted to, overdosed on, or been killed by opioids;

d. Upon information and belief, more broadly, opioid use and addiction have driven Travis County residents' health care costs higher;

e. Employers have lost the value of productive and healthy employees who have suffered from adverse consequences from opioid use;

f. Defendants' success in extending the market for opioids to new patients and chronic conditions has created an abundance of drugs available for criminal use and fueled a new wave of addiction and injury. Defendants' scheme created both ends of a new secondary market for opioids – providing both the supply of narcotics to sell and the demand of addicts to buy them;

g. This demand has created additional illicit markets in other opiates, particularly heroin. The low cost of heroin has led some of those who initially become addicted to prescription opioids to migrate to cheaper heroin, fueling a new heroin epidemic in the process;

h. Upon information and belief, diverting opioids into secondary, criminal markets and increasing the number of individuals who are addicted to opioids has increased the demands on emergency services and law enforcement in Travis County;

i. All of Defendants' actions have caused significant harm to the community – in lives lost; addictions endured; the creation of an illicit drug market and all its concomitant crime and costs; unrealized economic productivity; and broken families and homes;

j. Upon information and belief, these harms have taxed the human, medical,

public health, law enforcement, and financial resources of Travis County; and

k. Defendants' interference with the comfortable enjoyment of life of a substantial number of people is entirely unreasonable because there is limited social utility to opioid use and any potential value is outweighed by the gravity of harm inflicted by Defendants' actions.

160. Defendants knew, or should have known, that promoting opioid use would create a public nuisance in the following ways:

a. Upon information and belief, Defendants have engaged in massive production, promotion, and distribution of opioids for use by the citizens of Travis County;

b. Defendants' actions created and expanded the market for opioids, promoting its wide use for pain management;

c. Defendants misrepresented the benefits of opioids for chronic pain and fraudulently concealed, misrepresented, and omitted the serious adverse effects of opioids, including the addictive nature of the drugs; and

d. Defendants knew or should have known that their promotion would lead to addiction and other adverse consequences that the larger community would suffer as a result.

161. Upon information and belief, Defendants' actions were, at the least, a substantial factor in doctors and patients not accurately assessing and weighing the risks and benefits of opioids for chronic pain thereby causing opioids to become widely available and used in Travis County.

162. Without Defendants' actions, opioid use would not have become so widespread and the enormous public health hazard of opioid addiction would not have existed and could have been averted.

163. Upon information and belief, The health and safety of the citizens of Travis County, including those who use, have used, or will use opioids, as well as those affected by opioid users, is a matter of great public interest and legitimate concern to Travis County's

citizens and residents.

164. The public nuisance created, perpetuated, and maintained by Defendants can be abated and further reoccurrence of such harm and inconvenience can be prevented.

165. Upon information and belief, Defendants' conduct has affected and continues to affect a considerable number of people within Travis County and is likely to continue to cause significant harm to patients who take opioids, their families, and the community at large.

166. Each Defendant created or assisted in creating the opioid epidemic, and each Defendant is jointly and severally liable for its abatement. Furthermore, each Defendant should be enjoined from continuing to create, perpetuate, or maintain said public nuisance in Travis County.

## VIII. SECOND CAUSE OF ACTION: COMMON LAW FRAUD AGAINST ALL DEFENDANTS

167. Travis County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

168. At all relevant and material times, Defendants expressly and/or impliedly warranted that opioids were safe, of merchantable quality, and fit for use.

169. Defendants' superior knowledge and expertise, their relationship of trust and confidence with doctors and the public, their specific knowledge regarding the risks and dangers of opioids, and their intentional dissemination of promotional and marketing information about opioids for the purpose of maximizing sales, each gave rise to the affirmative duty to meaningfully disclose and provide all material information about the risks and harms associated with opioids.

170. At all times herein mentioned, Defendants, individually and acting through their employees and agents, and in concert with each other, fraudulently represented to physicians

who Defendants knew would justifiably rely on Defendants' representations that opioids were safe and effective for treating chronic pain.

171. Upon information and belief, Defendants' false representations were fraudulently made, with the intent or purpose that healthcare providers and patients would justifiably rely upon them, leading to the prescription, administration, filling, purchasing, and consumption of opioids in Travis County.

172. Defendants' deliberate misrepresentations and/or concealment, suppression, and omission of material facts as alleged herein include, but are not limited to:

a. Making false and misleading claims regarding the known risks of the addictive nature of opioids and suppressing, failing to disclose, and mischaracterizing the addictive nature of opioids and in concomitant costs, such as overdoses, deaths, and heroin addiction;

b. Making false and misleading written and oral statements that opioids are more effective than traditional pain killers for chronic pain, or effective at all and/or omitting material information showing that opioids are no more effective than other non-addictive drugs for chronic pain;

c. Issuing false and misleading warnings and/or failing to issue adequate warnings concerning the risks and dangers of using opioids;

d. Making false and misleading claims downplaying the risk of addiction when using opioids and/or setting forth guidelines that would purportedly identify addictive behavior; and

e. Making false and misleading misrepresentations concerning the safety, efficacy and benefits of opioids without full and adequate disclosure of the underlying facts which rendered such statements false and misleading.

173. Defendants willfully, wantonly, and recklessly disregarded their duty to provide truthful representations regarding the safety and risk of opioids.

174. Defendants made these misrepresentations with the intent that the healthcare community and patients located wherever these opioid drugs were sold or consumed would rely upon them.

175. Defendants' misrepresentations were made with the intent of defrauding and

deceiving the medical community and consumers to induce and encourage the sale of opioids.

176. Upon information and belief, Defendants' fraudulent representations evidence their callous, reckless, willful, and depraved indifference to the health, safety, and welfare of consumers living in Travis County.

177. Defendants omitted, misrepresented, suppressed and concealed material facts concerning the dangers and risk of injuries associated with the use of opioids, as well as the fact that the product was unreasonably dangerous.

178. Upon information and belief, Defendants' purpose was willfully blind to, ignored, downplayed, avoided, and/or otherwise understated the serious nature of the risks associated with the use of opioids.

179. Upon information and belief, the treating medical community and consumers in Travis County did not know that Defendants' representations were false and/or misleading and justifiably relied on them.

180. Defendants had sole access to material facts concerning the dangers and unreasonable risks of opioids, which they intentionally concealed.

181. Upon information and belief, as a direct and proximate result of Defendants' fraudulent misrepresentations and intentional concealment of facts, upon which the medical community and consumers in Travis County reasonably relied, Travis County suffered actual and punitive damages.

## IX. THIRD CAUSE OF ACTION: NEGLIGENCE AGAINST ALL DEFENDANTS

182. Travis County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

183. Manufacturing Defendants have a duty to exercise reasonable care in marketing their opioids to physicians treating residents of Travis County and Travis County residents. Manufacturing Defendants have breached their duty by knowingly and fraudulently misrepresenting the benefits of, and downplaying the risks of, opioids for chronic pain.

184. Manufacturing Defendants have used deceitful marketing ploys, KOLs, Front Groups, and other schemes to increase profits at the cost of public health causing an opioid epidemic. Manufacturing Defendants have acted willfully, wantonly, and maliciously.

185. Likewise, Distributor Defendants have a duty to exercise ordinary care in distributing opioids. Distributor Defendants have breached their duty by failing to prevent or reduce the distribution of opioids, or to report the increase in the distribution and/or sale of opioids.

186. Distributor Defendants have intentionally failed to prevent or reduce the distribution of opioids, or to report any increases in the sale of opioids, so that they could increase profits and receive rebates or kick-backs from Manufacturing Defendants. Distributor Defendants have acted willfully, wantonly, and maliciously.

187. Upon information and belief, as a proximate result, Manufacturing and Distributor Defendants and their agents have caused Travis County to incur excessive costs to treat the opioid epidemic in its county, including but not limited to increased costs of social services, health systems, law enforcement, judicial system, and treatment facilities.

188. Travis County and its residents are therefore entitled to actual and punitive damages.

## X. FOURTH CAUSE OF ACTION: GROSS NEGLIGENCE AGAINST ALL DEFENDANTS

189. Travis County incorporates the allegations within all prior paragraphs within this Complaint as if they were fully set forth herein.

190. Defendants' marketing scheme to optimize profits by misrepresenting and falsely touting opioids as the panacea to chronic pain was done intentionally.

191. Defendants' hiring of KOLs, Front Groups, and others to spread their fraudulent

message that opioids were useful and beneficial for chronic pain was grossly negligent and done with conscious indifference or reckless disregard for the safety of others.

192. Each Defendant's actions and omissions as described herein, singularly or in combination with each other, was malicious resulting in damages and injuries to Travis County and its residents.

193. Upon information and belief, at every stage, Defendants knew or should have known that their conduct would create an unreasonable risk of physical harm to others, including Travis County and its residents, and should be held liable in punitive and exemplary damages to Travis County.

## XI. FIFTH CAUSE OF ACTION: TEXAS CONTROLLED SUBSTANCES ACT ("TCSA") AGAINST DISTRIBUTOR DEFENDANTS

194. Travis County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

195. Distributor Defendants have knowingly distributed, delivered, administered, or dispensed a controlled substance in violation of the Texas Controlled Substances Act §481.128(a)(1) by deceiving practitioners into prescribing, dispensing, delivering, or administering a controlled substance, or causing a controlled substance to be administered when there is no valid medical purpose. Tex. Health & Safety Code §481.071.

196. As alleged herein, each Distributor Defendant, at all times relevant to this Complaint, violated the Texas Controlled Substance Act by making deceptive representations about using opioids to treat chronic pain. Each Distributor Defendant also omitted or concealed material facts and failed to correct prior misrepresentations and omissions about the risks and benefits of opioids. Each Distributor Defendant's omissions rendered even their seemingly

truthful statements about opioids deceptive.

197. Upon information and belief, Distributor Defendants' deceptive representations and concealments were reasonably calculated to deceive practitioners treating Travis County residents into prescribing opioids without any valid medical purpose, and Distributor Defendants continue to do so to this day.

198. As a direct and proximate cause of Distributor Defendants' deceptive conduct, Travis County should be awarded civil penalties pursuant to the Texas Controlled Substances Act.

## XII. SIXTH CAUSE OF ACTION: UNJUST ENRICHMENT AGAINST ALL DEFENDANTS

199. Travis County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Complaint as though fully alleged herein.

200. Upon information and belief, as an expected and intended result of their conscious wrongdoing as set forth in this Complaint, Defendants have profited and benefited from opioid purchases made by Travis County and its residents.

201. Upon information and belief, when Travis County and its residents purchased opioids, they expected that Defendants had provided necessary and accurate information regarding those risks. Instead, Defendants had misrepresented the material facts regarding the risks and benefits of opioids.

202. Upon information and belief, Defendants have been unjustly enriched at the expense of Travis County, and Travis County is therefore entitled to damages to be determined by the jury.

## XIII. RULE 193.7 NOTICE

203. Pursuant to Rule 193.7 of the Texas Rules of Civil Procedure, Plaintiff hereby gives actual notice to Defendants that any and all documents produced by any Defendant may be

used at any pretrial proceeding and/or at trial of this matter without the necessity of authenticating the documents.

## PRAYER FOR RELIEF

WHEREFORE, Plaintiff respectfully prays:

a. That the acts alleged herein be adjudged and decreed to be unlawful and that the Court enter a judgment declaring them to be so;

b. That Defendants be enjoined from, directly or indirectly through KOLs, Front Groups or other third parties, continuing to misrepresent the risks and benefits of the use of opioids for chronic pain, and from continuing to violate Texas law;

c. That Plaintiff recover all measures of damages, including punitive and exemplary damages, allowable under the law, and that judgment be entered against Defendants in favor of Plaintiff;

d. That Plaintiff recover restitution on behalf of Travis County consumers who paid for opioids for chronic pain;

e. That Plaintiff recover the costs and expenses of suit, pre- and post-judgment interest, and reasonable attorneys' fees as provided by law; and

f. That Defendants be ordered to abate the public nuisance that they created in in violation of Texas common law.

Date: February 5, 2018

Respectfully Submitted,

**THE LANIER LAW FIRM**

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# APPENDIX I

## MAVERICK PETITION

**Electronically Filed at**
**10/27/2017 1:13 PM**
**Leopoldo Vielma, District Clerk**
**Maverick County, Texas**
**By: Monica Gonzalez, Deputy**

**Cause NO.** 17-10-34909-MCVAJA

MAVERICK COUNTY, TEXAS

Plaintiff,

v.

PURDUE PHARMA L.P.
c/o The Prentice Hall Corporation
2711 Centerville Road
Wilmington, DE 19808

and

PURDUE PHARMA INC.
c/o The Prentice Hall Corporation
2711 Centerville Road
Wilmington, DE 19808

and

THE PURDUE FREDERICK COMPANY, INC.
c/o The Prentice Hall Corporation
2711 Centerville Road
Wilmington, DE 19808

and

TEVA PHARMACEUTICALS USA, INC.
c/o Corporate Creations Network Inc.
2985 Gordy Parkway, 1st Floor
Cobb, Marietta GA 30066

and

CEPHALON, INC.
c/o Corporate Creations Network Inc.
2985 Gordy Parkway, 1st Floor
Cobb, Marietta GA 30066

and

JOHNSON & JOHNSON
c/o CT Corp System
1st National Bank Tower
Atlanta, GA 30303

IN THE DISTRICT COURT:

365TH
_____ JUDICIAL DISTRICT

MAVERICK COUNTY, TEXAS

**PLAINTIFFS ORIGINAL PETITION**

**(JURY DEMAND)**

**Electronically Filed at**
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**Leopoldo Vielma, District Clerk**
**Maverick County, Texas**
**By: Monica Gonzalez, Deputy**

and

JANSSEN PHARMACEUTICALS, INC.
c/o CT Corporation System
289 S Culver Street
Lawrenceville, GA 30046-4805

and

ORTHO-MCNEIL-JANSSEN
PHARMACEUTICALS, INC. N/K/A JANSSEN
PHARMACEUTICALS, INC.
c/o CT Corporation System
289 S Culver Street
Lawrenceville, GA 30046-4805

and

JANSSEN PHARMACEUTICA, INC. N/K/A
JANSSEN PHARMACEUTICALS, INC.
c/o CT Corporation System
289 S Culver Street
Lawrenceville, GA 30046-4805

and

ENDO PHARMACEUTICALS, INC.
c/o CT Corporation System
1201 Peachtree Street, NE
Atlanta, GA 30361

and

ALLERGAN PLC F/K/A ACTAVIS PLC
c/o Corporate Creations Network, Inc.
2985 Gordy Pkwy, 1st Floor
Marietta, GA 30006

and

ACTAVIS, INC. F/K/A WATSON
PHARMACEUTICALS, INC.
c/o Corporate Creations Network, Inc.
2985 Gordy Pkwy, 1st Floor
Marietta, GA 30006

and

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WATSON LABORATORIES, INC.
c/o Corporate Creations Network, Inc.
2985 Gordy Pkwy, 1st Floor
Marietta, GA 30006

and

INSYS THERAPEUTICS, INC.
c/o Corporation Trust Center
Wilmington, DE 19801

and

ACTAVIS LLC
c/o Corporate Creations Network, Inc.
2985 Gordy Pkwy, 1st Floor
Marietta, GA 30006

and

ACTAVIS PHARMA, INC. F/K/A WATSON PHARMA, INC.
c/o Corporate Creations Network, Inc.
2985 Gordy Pkwy, 1st Floor
Marietta, GA 30006

and

ENDO HEALTH SOLUTIONS INC.
c/o CT Corporation System
1201 Peachtree Street, NE
Atlanta, GA 30361

and

MCKESSON CORPORATION
c/o Corporation Service Company
40 Technology Parkway South, #300
Norcross, GA 30092

and

WALSH DISTRIBUTION, LLC
5005 North State Line Avenue
Texarkana, TX 75503-2956

and

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**Maverick County, Texas**
**By: Monica Gonzalez, Deputy**

CARDINAL HEALTH, INC.
c/o CT Corporation System
389 Culver Street
Lawrenceville, GA 30046

and

AMERISOURCEBERGEN CORPORATION
c/o CT Corporation System
289 S. Culver St
Lawrenceville, GA 30046

and

RUSSELL PORTENOY
39 Broadway, 1st Floor
New York, NY 10006

and

PERRY FINE
615 Arapeen Way, Suite 155
Salt Lake City, UT 84132

and

SCOTT FISHMAN
2221 Stockton Blvd.
Sacramento, CA 95817

and

LYNN WEBSTER
PRA Health Sciences
3838 S 700 E #202
Salt Lake City, UT 84106

Defendants.

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**By: Monica Gonzalez, Deputy**

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Plaintiff, the County of Maverick, Texas("Plaintiff," "County," or "Maverick County"), by and through its attorneys, for its Complaint against Defendants Purdue Pharma L.P.; Purdue Pharma Inc.; the Purdue Frederick Company, Inc.; Teva Pharmaceuticals USA, Inc.; Cephalon, Inc.; Johnson & Johnson; Janssen Pharmaceuticals, Inc.; Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc.; Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc.; Endo Health Solutions Inc.; Endo Pharmaceuticals, Inc.; Allergan plc f/k/a Actavis plc; Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc.; Watson Laboratories, Inc.; Actavis LLC; and Actavis Pharma, Inc. f/k/a Watson Pharma, Inc.; Insys Therapeutics, Inc. (collectively, "Manufacturers," "Manufacturer Defendants," or "Defendants"); McKesson Corporation; Cardinal Health Inc.; Amerisource Drug Corporation; Walsh Distribution, LLC (collectively, "Distributors," "Distributor Defendants," or "Defendants") Russell Portenoy; Perry Fine; Scott Fishman; Lynn Webster; (collectively, "Individual," "Individual Defendants," or "Defendants ("Clinic," "Clinic Defendant," or "Defendants") (collectively, "Defendants") alleges as follows:

## INTRODUCTION

1. This case is about one thing: corporate greed. Defendants put their desire for profits above the health and well-being of Maverick County consumers at the cost of Plaintiff.

2. Maverick County spends millions of dollars each year to provide and pay for health care, services, pharmaceutical care and other necessary services and programs on behalf of residents of its County whom are indigent or otherwise eligible for services, including payments through services such as Medicaid for prescription opium painkillers ("opioids") which are manufactured, marketed, promoted, sold, and/or distributed by the Defendants.

3. Maverick County also provides a wide range of other services to its residents, including law enforcement, services for families and children, and public assistance.

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4. In recent years, the County of Maverick has been forced to expend exorbitant amounts of money, described further below, due to what is commonly referred to as the "opioid epidemic" and as a direct result of the actions of Defendants.

5. Plaintiff is responsible for partially funding a medical insurance plan for its employees, including the costs of prescription drugs, including opioids.

6. Addiction is a spectrum of substance use disorders that range from misuse and abuse of drugs to addiction.[1] Throughout this Complaint, "addiction" refers to the entire range of substance abuse disorders. Individuals suffer negative consequences wherever they fall on the substance use disorder spectrum.

7. Defendants knew that opioids were effective treatments for short-term post-surgical and trauma-related pain, and for palliative (end-of-life) care. Yet they also knew–and had known for years–that opioids were addictive and subject to abuse, particularly when used long-term for chronic non-cancer pain (pain lasting three months or longer, hereinafter referred to as "chronic pain"), and should there not be used except as a last-resort.

8. Defendants knew that, barring exceptional circumstances, opioids were too addictive and too debilitating for long-term use for chronic non-cancer pain lasting three months or longer.

9. Defendants further knew–and had known for years–that with prolonged use, the effectiveness of opioids wanes, requiring increases in doses and markedly increasing the risk of significant side effects and addiction.[2, 3]

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[1] Diagnostic and Statistical Manual of Mental Disorders (5th ed. 2013) ("DSM-V").

[2] *See, e.g.*, Russell K. Portenoy, *Opioid Therapy for Chronic Nonmalignant Pain: Current Status*, 1 Progress in Pain Res. & Mgmt. 247 (1994).

[3] The authoritative *Diagnostic and Statistical Manual of Mental Disorders*, (5th ed. 2013) ("DSM-V") classifies addiction as a spectrum of "substance use disorders" that ranges from misuse and abuse of drugs to addiction. Patients suffer negative consequences wherever they fall on the substance use disorder continuum. Throughout this Complaint, "addiction" refers to this range of substance use disorders.

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10. Defendants also knew that controlled studies of the safety and efficacy of opioids were limited to short-term use (not longer than 90 days), and in managed settings (*e.g.*, hospitals), where the risk of addiction and other adverse outcomes was much less significant.

11. Indeed, the U.S. Food and Drug Administration ("FDA") has expressly recognized that there have been no long-term studies demonstrating the safety and efficacy of opioids for long-term use.[4]

12. Prescription opioids, which include well-known brand-name drugs like OxyContin and Percocet, and generics like oxycodone and hydrocodone, are narcotics. They are derived from or possess properties similar to opium and heroin, which is why they are regulated as controlled substances.[5] Like heroin, prescription opioids work by binding to receptors on the spinal cord and in the brain, dampening the perception of pain. Opioids also can create a euphoric high, which can make them addictive. At certain doses, opioids can slow the user's breathing, causing respiratory depression and death.

13. In order to expand the market for opioids and realize blockbuster profits, Defendants needed to create a sea of change in the medical and public perception that would permit the use of opioids not just for acute and palliative care, but also for long periods of time to treat more common aches and pains, like lower back pain, arthritis, and headaches.

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[4] Letter from Janet Woodcock, M.D., Dir., Ctr. for Drug Eval. & Res., to Andrew Kolodny, M.D., Pres. Physicians for Responsible Opioid Prescribing, Re Docket No. FDA-2012-P-0818 (Sept. 10, 2013).

[5] Since passage of the Controlled Substances Act ("CSA") in 1970, opioids have been regulated as controlled substances. As controlled substances, they are categorized in five schedules, ranked in order of their potential for abuse, with Schedule I being the most dangerous. The CSA imposes a hierarchy of restrictions on prescribing and dispensing drugs based on their medicinal value, likelihood of addiction or abuse, and safety. Opioids generally had been categorized as Schedule II or Schedule III drugs. Schedule II drugs have a high potential for abuse, have a currently accepted medical use, and may lead to severe psychological or physical dependence. Schedule III drugs are deemed to have a lower potential for abuse, but their abuse still may lead to moderate or low physical dependence or high psychological dependence.

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14. Defendants, through a sophisticated and highly deceptive and unfair marketing campaign that began in the late 1990s, deepened around 2006, and continues to the present, set out to, and did, reverse the popular and medical understanding of opioids. Chronic opioid therapy—the prescribing of opioids to treat chronic pain long-term—is now commonplace.

15. To accomplish this reversal, Defendants spent hundreds of millions of dollars: (a) developing and disseminating seemingly truthful scientific and educational materials and advertising that misrepresented the risks, benefits, and superiority of opioids long-term use to treat chronic pain (b) deploying sales representatives who visited doctors and other prescribers and delivered misleading messages about the use of opioids (c) recruiting prescribing physicians as paid speakers as a means to secure those physicians' future "brand loyalty" and extend their reach to all physicians; (d) funding, assisting, encouraging, and directing certain doctors, known as "key opinion leaders" ("KOLs"), not only to deliver scripted talks, but also to draft misleading studies, present continuing medical education programs ("CMEs") that were deceptive and lacked balance, and serve on the boards and committees of professional societies and patient advocacy groups that delivered messages and developed guidelines supporting chronic opioid therapy; and (e) funding, assisting, directing, and encouraging seemingly neutral and credible professional societies and patient advocacy groups (referred to hereinafter as "Front Groups") that developed educational materials and treatment guidelines that were then distributed by Defendants, which urged doctors to prescribe, and patients to use, opioids long-term to treat chronic pain.

16. These efforts, executed, developed, supported, and directed by Defendants, were designed not to present a fair view of how and when opioids could be safely and effectively used, but rather to convince doctors, patients, and others that the benefits of using opioids to treat chronic pain outweighed the risks and that opioids could be used safely by most patients. Defendants and the third parties whom they recruited and supported, all profited handsomely through their dissemination of the

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deceptive information. KOLs and Front Groups saw their stature in the medical community elevated dramatically due to Defendants' funding, and Defendants saw an equally dramatic rise in their revenues.

17. Working individually, with, and through these Front Groups and KOLs, Defendants pioneered a new and far broader market for their potent and highly addictive drugs— the chronic pain market. Defendants persuaded doctors, patients, and others that what they had long understood—that opioids are addictive drugs and unsafe in most circumstances for long-term use— was untrue, and to the contrary, that the compassionate treatment of pain *required* opioids. Ignoring the limitations and cautions in their own drugs' labels, Defendants: (a) overstated the benefits of chronic opioid therapy, promised improvement in patients' function and quality of life, and failed to disclose the lack of evidence supporting long-term use; (b) trivialized or obscured their serious risks and adverse outcomes, including the risk of addiction, overdose, and death; (c) overstated their superiority compared with other treatments, such as other non-opioid analgesics, physical therapy, and other alternatives; and (d) mischaracterized the difficulty of withdrawal from opioids and the prevalence of withdrawal symptoms. There was, and is, no reliable scientific evidence to support Defendants' marketing claims, and there was, and is, a wealth of scientific evidence that these claims are simply false. Defendants also deceptively and unfairly marketed the drugs for indications and benefits that were outside of the drugs' labels and not supported by substantial evidence.

18. Even Defendants' KOLs initially were very cautious about whether opioids were appropriate to treat chronic pain. Some of these same KOLs have since recanted their pro-opioid marketing messages and acknowledged that Defendants' marketing went too far. Yet despite the voices of renowned pain specialists, researchers, and physicians who have sounded the alarm on the overprescribing of opioids to treat chronic pain, Defendants continue to disseminate their misleading and unfair marketing claims to this day.

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19. Defendants' efforts were wildly successful in expanding opioid abuse. The United States is now awash in opioids. In 2012, health care providers wrote 259 million prescriptions for opioid painkillers— enough to medicate every adult in America around the clock for a month. Twenty percent of all doctors' visits in 2010 resulted in the prescription of an opioid, nearly double the rate in 2000. Opioids—once a niche drug—are now the most prescribed class of drugs—more than blood pressure, cholesterol, or anxiety drugs. While Americans represent only 4.6% of the world's population, they consume 80% of the opioids supplied around the world and 99% of the global hydrocodone supply.

20. Together, opioids generated $8 billion in revenue for drug companies in 2012. Of that amount, $3.1 billion went to Purdue for its OxyContin sales. By 2015, sales of opioids grew further to approximately $9.6 billion.[6]

21. It was Defendants' marketing—and not any medical breakthrough—that rationalized prescribing opioids for chronic pain and opened the floodgates of opioid use and abuse. The result has been catastrophic.

22. Indeed, the National Institutes of Health "NIH" not only recognizes the opioid abuse problem, but also identifies Defendants' "aggressive marketing" as a major cause: "Several factors are likely to have contributed to the severity of the current prescription drug abuse problem. They include drastic increases in the number of prescriptions written and dispensed, greater social acceptability for using medications for different purposes, and *aggressive marketing by pharmaceutical companies*."[7] As shown herein, the "drastic increases in the number of prescriptions written and dispensed" and the "greater social acceptability for using medications for different purposes" are not really independent causative factors but are in fact the direct result of "the aggressive marketing by pharmaceutical companies."

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[6] D. Crow, *Drugmakers hooked on $10bn opioid habit*, Financial Times (August 10, 2016).
[7] America's Addiction to Opioids: Heroin and Prescription Drug Abuse. Available at http://www.drugabuse.gov/about-nida/legislative-activities/testimony-to-congress/2015/americas-addiction-to-opioids-heroin-prescription-drug-abuse#_ftn2 (accessed August 18, 2017) (emphasis added).

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23. According to the U.S. Centers for Disease Control and Prevention ("CDC"), the nation has been swept up in an opioid-induced "public health epidemic." [8] According to the CDC, prescription opioid use contributed to 16,651 overdose deaths nationally in 2010; 16,917 in 2011; and 16,007 in 2012. One Defendant's own 2010 internal data shows that it knew that the use of prescription opioids gave rise to 40% of drug-related emergency department visits in 2010 and 40% of drug poisoning deaths in 2008, and that the trend of opioid poisonings was increasing from 1999-2008. For every death, more than 30 individuals are treated in emergency rooms.

24. Resident of Maverick County continue to die from the use of opioid pain medications at an alarming rate. The rate of overdose deaths in Maverick County have continued to climb since 2004.

25. Opioids have become the most serious drug problem for the Maverick County Law Enforcement. The commission of criminal acts to obtain opioids is an inevitable consequence of opioid addiction.

26. Due to the continued rise of the opioid epidemic and deaths, Maverick County has taken steps and will continue to take steps to fight the use of opioids and save lives.

27. In sum, Maverick County has experienced economic costs directly related to the opioid epidemic, including Medicaid costs, law enforcement, judicial, foster care, Narcan costs, loss of productivity, and various other costs directly caused by the actions of the defendants.

28. But even these alarming statistics do not fully communicate the toll of prescription opioid abuse on patients and their families.

29. The dramatic increase in opioid prescriptions to treat common chronic pain conditions has resulted in a population of addicts who seek drugs from doctors. When turned down by one

[8] CDC, *Examining the Growing Problems of Prescription Drug and Heroin Abuse* (Apr. 29, 2014), http://www.cdc.gov/washington/testimony/2014/t20140429.htm (accessed May 30, 2017)

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physician, many of these addicts deploy increasingly desperate tactics—including doctor-shopping, use of aliases, and criminal means—to satisfy their cravings.

30. Efforts by doctors to reverse course for a chronic pain patient already on opioids long-term include managing the physical suffering and psychological distress a patient endures while withdrawing from the drugs. This process is often thwarted by a secondary criminal market well-stocked by a pipeline of drugs that is diverted to supply them. Even though they never would have prescribed opioids in the first place, many doctors feel compelled to continue prescribing opioids to patients who have become dependent on them.

31. According to the CDC, more than 12 million Americans age 12 or older have used prescription painkillers without a prescription in 2010[9], and adolescents are abusing opioids in alarming numbers.

32. Opioid abuse has not displaced heroin, but rather triggered a resurgence in its use, imposing additional burdens on the County and local agencies that address heroin use and addiction. According to the CDC, the percentage of heroin users who also use opioid pain relievers rose from 20.7% in 2002-2004 to 45.2% in 2011-2013. In Maverick County in 2015, there were 141 heroin-related emergency department admissions.[10] Heroin produces a very similar high to prescription opioids, but is often cheaper. While a single opioid pill may cost $10-$15 on the street, users can obtain a bag of heroin, with multiple highs, for the same price. It is hard to imagine the powerful pull that would cause a law-abiding, middle-aged person who started on prescription opioids for a back injury to turn to buying, snorting, or injecting heroin, but that is the dark side of opioid abuse and addiction.

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[9] CDC, *Presxcription Painkiller Overdoses in the US* (Nov. 2011), https://www.cdc.gov/vitalsigns/painkilleroverdoses/ (accessed May 30, 2017).

[10] http://www.newsday.com/long-island/Maverick/Maverick-issues-warning-about-dangers-of-synthetic-opioid-carfentanil-1.13156187 (accessed May 30, 2017).

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33. Dr. Robert DuPont, former director of the National Institute on Drug Abuse, opines that opioids are more destructive than crack cocaine:

> [Opioid abuse] is building more slowly, but it's much larger. And the potential[] for death, in particular, [is] way beyond anything we saw then. . . . [F]or pain medicine, a one-day dose can be sold on the black market for $100. And a single dose can [be] lethal to a non-patient. There is no other medicine that has those characteristics. And if you think about that combination and the millions of people who are using these medicines, you get some idea of the exposure of the society to the prescription drug problem.[11]

34. The inability of opioid users to continue to obtain opioid pain medications directly caused the resurgence of heroin. Around the same time that Prescription Drug Monitoring Programs were created and increased legislation regulating these drugs, a dramatically increased supply of heroin from Afghanistan, Mexico and Central Mexico was seen. This was no coincidence but simply filling a new demand. Officials estimate that Mexico's opium production rose an estimated 50% in 2014, while the poppy fields in Afghanistan expanded by over a third in the year prior. Global opium poppy production reached a peak not seen since the 1930s, and shows no sign of stopping due to the rapidly intensifying demand for heroin.[12]

35. Due to its located, Atlanta has been referred to a "heroin distribution hub", placing an enormous burden on the government, law enforcement, and the community.[13]

36. Countless County residents suffer from chronic pain, which takes an enormous toll on their health, their lives, and their families. These residents deserve both appropriate care and the ability

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[11] Transcript, *Use and Abuse of Prescription Painkillers*, The Diane Rehm Show (Apr. 21, 2011), http://thedianerehmshow.org/shows/2011-04-21/use-and-abuse-prescription-painkillers/transcript (accessed May 30, 2017).

[12] Katharine Q. Seelye (2015). The Numbers Behind America's Heroin Epidemic: A guide to the drug's spread and impact. New York Times, October 30, 2015.

[13] http://news.wabe.org/post/Maverick-county-calls-heroin-fatalities-emerging-epidemic

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to make decisions based on accurate and complete information about treatment risks and benefits. But Defendants' deceptive and unfair marketing practices deprived County residents and their doctors of the ability to make informed medical decisions and, instead, caused important, sometimes life-or-death decisions to be made based not on science, but on hype. Defendants deprived patients, their doctors, and health care payors of the chance to exercise informed judgment and subjected them to enormous costs and suffering.

37. Defendants' actions are not permitted or excused by the fact that their labels (with the exception of Cephalon's labels for Fentora and Actiq) may have allowed, or did not exclude, the use of opioids for chronic non-cancer pain. The FDA's approval did not give Defendants license to misrepresent the risks, benefits, or superiority of opioids. Indeed, what makes Defendants' efforts particularly nefarious—and dangerous—is that, unlike other prescription drugs marketed unlawfully in the past, opioids are highly addictive controlled substances. Defendants deceptively and unfairly engaged a patient base that—physically and psychologically—could not turn away from their drugs, many of whom were not helped by the drugs or were profoundly damaged by them.

38. Nor is Defendants' causal role broken by the involvement of doctors. Defendants' marketing efforts were both ubiquitous and highly persuasive; their deceptive messages tainted virtually every source doctors could rely on for information and prevented them from making informed treatment decisions. Defendants targeted not only pain specialists, but also primary care physicians (PCPs), nurse practitioners, physician assistants, and other non-pain specialists who were even less likely to be able to assess the companies' misleading statements. Defendants were also able to callously manipulate what doctors wanted to believe—namely, that opioids represented a means of relieving their patients' suffering and of practicing medicine more compassionately.

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39. By 2014, nearly two million Americans were either abusing opioid medications or were dependent on opioids.[14] According to the CDC, opioids have created a "public health epidemic" as of 2016.[15]

40. Defendants' marketing campaign has been extremely harmful and has cost American lives – including lives of residents of the County of Maverick. Deaths from prescription opioids have quadrupled since 1999. From 2000 to 2014 nearly 500,000 people died from such overdoses; seventy-eight Americans die everyday from opioid overdoses.[16]

41. It is estimated that, in 2012, 2.1 million people in the United States suffered from substance use disorders related to prescription opioid pain relievers.[17] Between 30% and 40% of long-term users of opioids experience problems with opioid use disorders.[18]

42. The National Institutes of Health ("NIH") not only recognizes the opioid abuse problem, but also identifies Defendants' "aggressive marketing" as a major cause: "Several factors are likely to have contributed to the severity of the current prescription drug abuse problem. They include drastic increases in the number of prescriptions written and dispensed, greater social acceptability for using medications for different purposes, and *aggressive marketing by pharmaceutical companies.*"[19] As shown below, the "drastic increases in the number of prescriptions written and dispensed" and the

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[14] CDC, Injury Prevention & Control: Opioid Overdose, Prescription Opioids, Addiction and Overdose. Available at http://www.cdc.gov/drugoverdose/opioids/prescribed.html (accessed May 30, 2017).

[15] CDC, *Examining the Growing Problems of Prescription Drug and Heroin Abuse*, (Apr. 29, 2014), http://www.cdc.gov/washington/testimony/2014/ts0140429.htm (accessed May 30, 2017).

[16] CDC, Injury Prevention & Control: Opioid Overdose, Understanding the Epidemic.

[17] Substance Abuse and Mental Health Services Administration, *Results from the 2012 National Survey on Drug Use and Health: Summary of National Findings*, NSDUH Series H- 46, HHS Publication No. (SMA) 13-4795. Rockville, MD: Substance Abuse and Mental Health Services Administration, 2013.

[18] J. Boscarino et al., Risk factors for drug dependence among out-patients on opioid therapy in a large US health-care system, 105(10) Addiction 1776 ( 2010); J. Boscarino et al., Prevalence of Prescription Opioid-Use Disorder Among Chronic Pain Patients: Comparison of the DSM-5 vs. DSM-4 Diagnostic Criteria, 30(3) Journal of Addictive Diseases 185 (2011).

[19] America's Addiction to Opioids: Heroin and Prescription Drug Abuse. Available at http://www.drugabuse.gov/about-nida/legislative-activities/testimony-to- congress/2015/americas-addiction-to-opioids-heroin-prescription-drug-abuse#_ftn2 (accessed May 30, 2017) (emphasis added).

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"greater social acceptability for using medications for different purposes " are not really independent causative factors but are in fact the direct result of "the aggressive marketing by pharmaceutical companies."

43. The rising numbers of persons addicted to opioids have led not only to an increase in health care costs to the County, and specifically the Plaintiff, but also a major increase in issues such as drug abuse, diversion,[20] and crimes related to obtaining opioid medications. The County of Maverick has been severely and negatively impacted due to the fraudulent misrepresentations and omissions by Defendants regarding the use and risk related to opioids. In fact, upon information and belief, Defendants have been and continue to be aware of the high levels of diversion of their product.

44. As a direct and foreseeable consequence of Defendants' wrongful conduct, Plaintiff has been required to spend millions of dollars each year in its efforts to combat the public nuisance created by Defendants' deceptive marketing campaign. Plaintiff has incurred and continues to incur costs related to opioid addiction and abuse, including, but not limited to, health care costs, criminal justice and victimization costs, social costs, and lost productivity costs. Defendants' misrepresentations regarding the safety and efficacy of long-term opioid use proximately caused injury to Plaintiff and its residents.

## JURISDICTION AND VENUE

45. This Court has jurisdiction over this action in that Defendants, by and through their authorized agents, servants and employees, regularly transacted business in Texas, manufactured, supplied, and distributed opioids in Texas and further through their acts and omissions tortuously caused injuries in Texas by engaging in a persistent course of conduct in Texas which violated Texas law. Defendants derived substantial revenue as the result of the opioids which were distributed to Texas physicians, patients, and others and later consumed by persons then residing in Texas.

[20] The CDC defines using or obtaining opioids illegally as "diversion."

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46. Venue is proper in Maverick County in that the Defendants committed tortious acts, omissions, and/or injuries in Maverick County. Further, Defendants transacted substantial business in Maverick County.

47. This action is non-removable because there is incomplete diversity of residents and no substantial federal question is presented.

## PARTIES

### A. Plaintiff.

48. Maverick County is a municipality located in the State of Texas with a population of approximately 60,000 residents. Plaintiff provides a wide range of services on behalf of its residents, including but not limited to services for families and children, public health, public assistance, law enforcement, and emergency care. As mentioned above, Plaintiff also partially funds a health insurance plan for its employees.

### B. Defendants.

49. Purdue Pharma L.P. is a limited partnership organized under the laws of Delaware with its principal place of business in Stamford Connecticut. Purdue Pharma Inc. is a New York corporation with its principal place of business in Stamford, Connecticut, and The Purdue Frederick Company, Inc. is a New York corporation with its principal place of business in Stamford, Connecticut (collectively, "Purdue").

50. Purdue is primarily engaged in the manufacture, promotion, sale, and distribution of opioids nationally and in Maverick County, including the following:

a. OxyContin (oxycodone hydrochloride extended release) is a Schedule II opioid agonist[21] tablet first approved in 1995 and indicated for the "management of

[21] An opioid agonist is a drug that activates certain opioid receptors in the brain. An antagonist, by contrast, blocks the receptor and can also be used in pain relief or to counter the effect of an opioid

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pain severe enough to require daily, around-the-clock, long-term opioid treatment and for which alternative treatment options are inadequate." Prior to April 2014,[22] OxyContin was indicated for the "management of moderate to severe pain when a continuous, around-the- clock opioid analgesic is needed for an extended period of time."

b. MS Contin (morphine sulfate extended release) is a Schedule II opioid agonist tablet first approved in 1987 and indicated for the "management of pain severe enough to require daily, around-the-clock, long-term opioid treatment and for which alternative treatment options are inadequate." Prior to April 2014, MS Contin was indicated for the "management of moderate to severe pain when a continuous, around-the-clock opioid analgesic is needed for an extended period of time."

c. Dilaudid (hydromorphone hydrochloride) is a Schedule II opioid agonist first approved in 1984 (injection) and 1992 (oral solution and tablet) and indicated for the "management of pain in patients where an opioid analgesic is appropriate."

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overdose.

[22] The labels for OxyContin and other long-acting opioids were amended in response to a 2012 citizens' petition by doctors. The changes were intended to clarify the existing obligation to "make an individualized assessment of patient needs." The petitioners also successfully urged that the revised labels heighten the requirements for boxed label warnings related to addiction, abuse, and misuse by changing "Monitor for signs of misuse, abuse, and addiction" to "[Drug name] exposes users to risks of addiction, abuse, and misuse, which can lead to overdose and death." Letter from Bob Rappaport, Dir. Ctr. for Drug Evaluations & Res., *Labeling Supplement and PMR [Post-Marketing Research] Required* (Sept. 10, 2013), http://www.fda.gov/downloads/Drugs/DrugSafety/InformationbyDrugClass/UCM367697.pdf (accessed May 30, 2017).

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d. Dilaudid-HP (hydromorphone hydrochloride) is a Schedule II opioid agonist injection first approved in 1984 and indicated for the "relief of moderate-to-severe pain in opioid-tolerant patients who require larger than usual doses of opioids to provide adequate pain relief."

e. Butrans (buprenorphine) is a Schedule III opioid partial agonist transdermal patch first approved in 2010 and indicated for the "management of pain severe enough to require daily, around- the-clock, long-term opioid treatment and for which alternative treatment options are inadequate." Prior to April 2014, Butrans was indicated for the "management of moderate to severe pain when a continuous, around-the-clock opioid analgesic is needed for an extended period of time."

f. Hysingla ER (hydrocodone bitrate) is a Schedule II opioid agonist tablet first approved in 2014 and indicated for the management of pain severe enough to require daily, around-the-clock, long-term opioid treatment and for which alternative treatment options are inadequate.

g. Targiniq ER (oxycodone hydrochloride and naloxone hydrochloride) is a Schedule II combination product of oxycodone, an opioid agonist, and naloxone, an opioid antagonist, first approved in 2014 and indicated for the management of pain severe enough to require daily, around- the-clock, long-term opioid treatment and for which alternative treatment options are inadequate.

51. OxyContin is Purdue's largest-selling opioid. Since 2009, Purdue's national annual sales of OxyContin have fluctuated between $2.47 billion and $2.99 billion, up four-fold from 2006 sales of

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$800 million. OxyContin constitutes roughly 30% of the entire market for analgesic drugs (*i.e.*, painkillers).

52. In 2007, Purdue settled criminal and civil charges against it for misbranding OxyContin and agreed to pay the United States $635 million—at the time one of the largest settlements with a drug company for marketing misconduct.[23] Pursuant to its settlement, Purdue operated under a Corporate Integrity Agreement with the Office of Inspector General of the U.S. Department of Health and Human Services, which required the company, *inter alia*, to ensure that its marketing was fair and accurate, and to monitor and report on its compliance with the Agreement.

53. Defendant Teva Pharmaceuticals USA, Inc. ("Teva USA") is a Delaware corporation with its principal place of business in North Whales, Pennsylvania. Teva USA is a wholly owned subsidiary of Teva Pharmaceutical Industries, Ltd. ("Teva Ltd."), an Israeli corporation.

54. Defendant Cephalon, Inc. is a Delaware corporation with its principal place of business in Frazer, Pennsylvania. In 2011, Teva Ltd. acquired Cephalon, Inc.

55. Teva USA and Cephalon, Inc. work together closely to market, manufacture, distribute and sell Cephalon products in the United States. Teva USA conducts Teva Ltd.'s sales and marketing activities for Cephalon in the United States and has done so since Teva Ltd.'s October 2011 acquisition of Cephalon. Teva USA holds out Actiq and Fentora as Teva products to the public. Teva USA sells all former Cephalon branded products through its "specialty medicines" division. The FDA approved prescribing information and medication guide, which is distributed with Cephalon opioids marketed and sold in Maverick County, discloses that the guide was submitted by Teva USA, and directs physicians to contact Teva USA to report adverse events. (Teva USA and Cephalon, Inc. collectively are referred to herein as "Cephalon.")

[23] https://oig.hhs.gov/publications/docs/press/2007/SemiannualRelfall2007E.pdf (accessed May 30, 2017).

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56. Cephalon has been in the business of manufacturing, selling, and distributing the following opioids, nationally and in Maverick County:

a. Actiq (fentanyl citrate) is a Schedule II opioid agonist lozenge (lollipop) first approved in 1998 and indicated for the "management of breakthrough cancer pain in patients 16 years of age and older who are already receiving and who are tolerant to opioid therapy for their underlying persistent cancer pain."

b. Fentora (fentanyl citrate) is a Schedule II opioid agonist buccal tablet (similar to plugs of smokeless tobacco) first approved in 2006 and indicated for the "management of breakthrough pain in cancer patients 18 years of age and older who are already receiving and who are tolerant to around-the-clock opioid therapy for their underlying persistent cancer pain."

57. In November 1998, the FDA granted restricted marketing approval for Actiq, limiting its lawful promotion to cancer patients experiencing pain. The FDA specified that Actiq should not be marketed for off-label uses, stating that the drug must be prescribed solely to cancer patients. In 2008, Cephalon pleaded guilty to a criminal violation of the Federal Food, Drug and Cosmetic Act for its misleading promotion of Actiq and two other drugs, and agreed to pay $425 million in fines, damages, and penalties.

58. The FDA requested that Endo remove Cephalon's Opana ER from the market in June 2017. The FDA relied on postmarketing data in reaching its conclusion based on the concern that the benefits of the drug may no longer outweigh its risk of abuse.[24]

59. Teva USA was in the business of selling generic opioids, including a generic form of OxyContin from 2005 through 2009 nationally and within the County of Maverick.

[24] FDA requests removal of OPANA ER for risks related to abuse. Available at: https://www.fda.gov/newsevents/newsroom/pressannouncements/ucm562401.htm (accessed August 17, 2017).

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60. On September 29, 2008, Cephalon entered into a five-year Corporate Integrity Agreement with the Office of Inspector General of the U.S. Department of Health and Human Services.[25] The agreement, *inter alia*, required Cephalon to send doctors a letter advising them of the settlement terms and gave them a means to report questionable conduct of its sales representatives; disclose payments to doctors on its web site; and regularly certify that the company has an effective compliance program.

61. Janssen Pharmaceuticals, Inc. is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey, and is a wholly owned subsidiary of Johnson & Johnson, a New Jersey corporation with its principal place of business in New Brunswick, New Jersey. Janssen Pharmaceuticals, Inc. was formerly known as ("f/k/a") Ortho-McNeil- Janssen Pharmaceuticals, Inc., which in turn was formerly known as Janssen Pharmaceutica Inc. Defendant Ortho-Mcneil-Janssen Pharmaceuticals, Inc., now known as Janssen Pharmaceuticals, Inc., is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey. Janssen Pharmaceutica, Inc., now known as Janssen Pharmaceuticals, Inc., is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey. Johnson & Johnson is the only company that owns more than 10% of Janssen Pharmaceuticals, Inc.'s stock, and it corresponds with the FDA regarding Janssen's products. Upon information and belief, Johnson & Johnson controls the sale and development of Janssen Pharmaceutical's drugs, and Janssen Pharmaceuticals, Inc.'s profits inure to Johnson & Johnson's benefit. (Janssen Pharmaceuticals, Inc., Ortho-McNeil-Janssen Pharmaceuticals, Inc., Janssen Pharmaceutica, Inc., and Johnson & Johnson collectively are referred to herein as "Janssen.")

62. Janssen manufactures, sells, and distributes a range of medical devices and pharmaceutical drugs in Maverick County and the rest of the nation, including Duragesic (fentanyl),

[25] https://www.justice.gov/archive/opa/pr/2008/September/08-civ-860.html (accessed May 30, 2017).

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which is a Schedule II opioid agonist transdermal patch first approved in 1990 and indicated for the "management of pain in opioid-tolerant patients, severe enough to require daily, around-the-clock, long-term opioid treatment and for which alternative treatment options are inadequate."

63. Until January 2015, Janssen also developed, marketed, and sold Nucynta and Nucynta ER:

a. Nucynta ER (tapentadol extended release) is a Schedule II opioid agonist tablet first approved in 2011 and indicated for the "management of pain severe enough to require daily, around-the-clock, long-term opioid treatment and for which alternative treatment options are inadequate." Prior to April 2014, Nucynta ER was indicated for the "management of moderate to severe chronic pain in adults [and] neuropathic pain associated with diabetic peripheral neuropathy (DPN) in adults." The DPN indication was added in August 2012.

b. Nucynta (tapentadol) is a Schedule II opioid agonist tablet and oral solution first approved in 2008 and indicated for the "relief of moderate to severe acute pain in patients 18 years of age or older."

64. Together, Nucynta and Nucynta ER accounted for $172 million in sales in 2014.[26] Prior to 2009, Duragesic accounted for at least $1 billion in annual sales.

65. Endo Health Solutions Inc. is a Delaware corporation with its principal place of business in Malvern, Pennsylvania. Endo Pharmaceuticals, Inc. is a wholly-owned subsidiary of Endo Health Solutions Inc. and is a Delaware corporation with its principal place of business in Malvern,

[26] http://www.prnewswire.com/news-releases/depomed-announces-closing-of-acquisition-of-us-rights-to-nucynta-tapentadol-nucynta-er-tapentadol-extended-release-tablets-and-nucynta-tapentadol-oral-solution-from-janssen-pharmaceuticals-inc-for-105-billion-300060453.html (accessed May 30, 2017)

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Pennsylvania. (Endo Health Solutions Inc. and Endo Pharmaceuticals, Inc. collectively are referred to herein as "Endo.")

66. Endo develops, markets, and sells prescription drugs, including the following opioids, in Maverick County and nationally:

a. Opana ER (oxymorphone hydrochloride extended release) is a Schedule II opioid agonist tablet first approved in 2006 and indicated for the "management of pain severe enough to require daily, around-the-clock, long-term opioid treatment and for which alternative treatment options are inadequate." Prior to April 2014, Opana ER was indicated for the "relief of moderate to severe pain in patients requiring continuous, around-the-clock opioid treatment for an extended period of time." On June 8, 2017, the FDA requested that Endo Pharmaceuticals remove its opioid medication, reformulated Opana ER (oxymorphone hydrochloride), from the market.[27]

b. Opana (oxymorphone hydrochloride) is a Schedule II opioid agonist tablet first approved in 2006 and indicated for the "relief of moderate to severe acute pain where the use of an opioid is appropriate."

c. Percodan (oxycodone hydrochloride and aspirin) is a Schedule II opioid agonist tablet first approved in 1950 and first marketed by Endo in 2004 and indicated for the "management of moderate to moderately severe pain."

[27] *FDA Requests Removal of Opana ER for Risks Related to Abuse.* https://www.fda.gov/NewsEvents/Newsroom/PressAnnouncements/ucm562401.htm.

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d. Percocet (oxycodone hydrochloride and acetaminophen) is a Schedule II opioid agonist tablet first approved in 1999 and first marketed by Endo in 2006 and indicated for the "relief of moderate to moderately severe pain."[28]

67. Opioids made up roughly $403 million of Endo's overall revenues of $3 billion in 2012. Opana ER yielded revenue of $1.15 billion from 2010 to 2013, and alone accounted for 10% of Endo's total revenue in 2012. Endo also manufactures and sells generic opioids nationally and in Maverick County, both itself and through its subsidiary, Qualitest Pharmaceuticals, Inc., including generic oxycodone, oxymorphone, hydromorphone, and hydrocodone products.

68. Allergan plc is a public limited company incorporated in Ireland with its principal place of business in Dublin, Ireland. Actavis plc acquired Allergan plc in March 2015, and the combined company changed its name to Allergan plc in March 2015. Prior to that, Watson Pharmaceuticals, Inc. acquired Actavis, Inc. in October 2012; the combined company changed its name to Actavis, Inc. in January 2013 and then to Actavis plc in October 2013. Watson Laboratories, Inc. is a Nevada corporation with its principal place of business in Corona, California, and is a wholly owned subsidiary of Allergan plc (f/k/a Actavis, Inc., f/k/a Watson Pharmaceuticals, Inc.). Actavis Pharma, Inc. (f/k/a Actavis, Inc.) is a Delaware corporation with its principal place of business in New Jersey, and was formerly known as Watson Pharma, Inc. Actavis LLC is a Delaware limited liability company with its principal place of business in Parsippany, New Jersey. Each of these defendants is owned by Allergan plc, which uses them to market and sell its drugs in the United States. Upon information and belief, Allergan plc exercises control over these marketing and sales efforts, and profits from the sale of Allergan/Actavis products ultimately inure to its benefit. (Allergan plc, Actavis plc, Actavis, Inc.,

[28] In addition, Endo marketed Zydone (hydrocodone bitartrate and acetaminophen), a Schedule III opioid agonist tablet indicated for the "relief of moderate to moderately severe pain," from 1998 through 2013. The FDA's website indicates this product is currently discontinued, but it appears on Endo's own website.

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Actavis LLC, Actavis Pharma, Inc., Watson Pharmaceuticals, Inc., Watson Pharma, Inc., and Watson Laboratories, Inc. hereinafter collectively are referred to as "Actavis.")

69. Actavis engages in the business of marketing and selling opioids in Maverick County and across the country, including the branded drugs Kadian and Norco, a generic version of Kadian, and generic versions of Duragesic and Opana. Kadian (morphine sulfate extended release) is a Schedule II opioid agonist capsule first approved in 1996 and indicated for the "management of pain severe enough to require daily, around-the-clock, long-term opioid treatment and for which alternative treatment options are inadequate." Prior to April 2014, Kadian was indicated for the "management of moderate to severe pain when a continuous, around-the-clock opioid analgesic is needed for an extended period of time." Actavis acquired the rights to Kadian from King Pharmaceuticals, Inc., on December 30, 2008 and began marketing Kadian in 2009.

70. Insys Therapeutics, Inc. ("Insys") is a Delaware corporation with its principal place of business in Chandler, Arizona.

71. Insys develops, markets, and sells prescription drugs, including Subsys, a sublingal spray of fentanyl, in Maverick County and nationally.

72. Defendant McKesson Corporation ("McKesson") is a Delaware corporation with its principal place of business in San Francisco, California.

73. Defendant McKesson had a net income in excess of $1.5 Billion in 2015.

74. Defendant McKesson distributes pharmaceuticals to retail pharmacies and institutional providers to customers in all 50 states, including Texas and the County of Maverick.

75. Upon information and belief, defendant McKesson is a pharmaceutical distributor licensed to do business in Texas

76. Defendant McKesson is the largest pharmaceutical distributor in North America.

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77. Upon information and belief, McKesson delivers one-third of all pharmaceuticals used in North America.

78. Defendant McKesson does substantial business in the State of Texas and Maverick County.

79. Defendant Walsh Distribution, LLC ("Walsh") is a Texas corporation with its principal place of business in Texarkana, Texas.

80. Defendant Walsh is a wholly-owned subsidiary of co-defendant McKesson.

81. Defendant Walsh distributes pharmaceuticals, including opioids, to retail pharmacies and institutional providers through the State of Texas and the County of Maverick.

82. Defendant Walsh does substantial business in the State of Texas and Maverick County.

83. Defendant Cardinal Health Inc. ("Cardinal") is an Ohio Corporation with its principal place of business in Dublin, Ohio.

84. Defendant Cardinal distributes pharmaceuticals to retail pharmacies and institutional providers to customers in all 50 states, including Texas and Maverick County.

85. Upon information and belief, defendant Cardinal is a pharmaceutical distributor licensed to do business in Texas State.

86. Defendant Cardinal does substantial business in the State of Texas and Maverick County.

87. Upon information and belief, defendant Cardinal is one of the largest distributors of opioid pain medications in Maverick Country, including Texas State and the County of Maverick.

88. Upon information and belief, Defendant AmerisourceBergen Drug Corporation ("Amerisource") is a Delaware Corporation with its principal place of business in Chesterbrook, Pennsylvania.

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89. Defendant Amerisource does substantial business in the State of Texas and Maverick County.

90. Upon information and belief, defendant Amerisource is a pharmaceutical distributor licensed to do business in Texas.

91. Defendant Amerisource distributes pharmaceuticals to retail pharmacies and institutional providers to customers in all 50 states, including Texas and in Maverick County.

92. Upon information and belief, defendant Amerisource is one of the largest distributors of opioid pain medications in the Country, including the state of Texas.

93. Defendant

94. The Distributor Defendants are the largest opioid distributors in Maverick County.

95. The Distributor Defendants purchased opioids from manufacturers, such as the named defendants herein, and sold them to pharmacies throughout Maverick County.

96. The Distributor Defendants played an integral role in the chain of opioids being distributed throughout Maverick County.

97. Pursuant to Texas State Law, the defendants, including the distributor defendants, were required to report suspicious orders of opioid pharmaceuticals to the DEA and appropriate State agencies.

98. This number only represents opioids distributed directly to retailers.

99. The Distributor Defendants were each on notice that the controlled substances they distributed or prescribed were the kinds that were susceptible to diversion for illegal purposes, abused, overused, and otherwise sought for illegal, unhealthy and problematic purposes.

100. The Distributor Defendants were each on notice that there was an alarming and suspicious rise in distributing opioids to retailers within Maverick County during this time period.

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101. As entities involved in the distribution of opioid medications, Defendants were engaged in abnormally and/or inherently dangerous activity and had a duty of care under Texas Law.

102. The Distributor Defendants had a duty to notice suspicious or alarming orders of opioid pharmaceuticals and to report suspicious orders to the proper authorities and governing bodies including the Texas Department of Health.

103. The Distributor Defendants knew or should have known that they were supplying vast amounts of dangerous drugs in Maverick County that were already facing abuse, diversion, misuse, and other problems associated with the opioid epidemic.

104. The Distributor Defendants failed in their duty to take any action to prevent or reduce the distribution of these drugs.

105. The Distributor Defendants were in a unique position and had a duty to inspect, report, or otherwise limit the flow of these drugs to Maverick County.

106. The Distributor Defendants, in the interest of their own massive profits, intentionally failed in this duty.

107. The Distributor Defendants have displayed a continuing pattern of failing to submit suspicious order reports.

108. In 2008, McKesson paid a $13.25 million fine to settle similar claims regarding suspicious orders from internet pharmacies.[29]

109. Despite these prior penalties, McKesson's pattern of failing to report suspicious orders continued for many years.

[29] http://www.wvgazettemail.com/news-health/20161218/suspicious-drug-order-rules-never-enforced-by-state (accessed May 30, 2017).

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110. According to the DEA, McKesson "supplied various U.S. pharmacies an increasing amount of oxycodone and hydrocodone pills" during the time in question, and "frequently misused products that are part of the current opioid epidemic."[30]

111. On January 17, 2017, the DEA announced that McKesson had agreed to pay a record $150 million fine and suspend the sale of controlled substances from distribution centers in several states.[31]

112. In 2008, defendant Cardinal paid a $34 million penalty to resolve allegations that it failed to report suspicious opioid orders.[32]

113. Despite this past penalty, in 2017, it was announced that defendant Cardinal agreed to a $44 million fine to "resolve allegations that it failed to alert the Drug Enforcement Agency to suspicious orders of powerful narcotics by pharmacies in Florida, Maryland, and New York.[33]

114. Defendant AmeriSource faced a criminal inquiry "into its oversight of painkiller sales" in 2012.[34] They have paid out fines for similar claims to the state of West Virginia. Despite the charges, fines, and penalties brought against the Distributor Defendants in the past, they continued to fail to report suspicious orders or prevent the flow of prescription opioids, including into Maverick County.

115. The Distributor Defendants are also members of the Healthcare Distribution Management Association ("HDMA"). The HDMA created "Industry Compliance Guidelines" which

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[30] https://www.justice.gov/opa/pr/mckesson-agrees-pay-record-150-million-settlement-failure-report-suspicious-orders (accessed May 30, 2017).
[31] *Id.*
[32] https://www.justice.gov/usao-wdwa/pr/united-states-reaches-34-million-settlement-cardinal-health-civil-penalties-under-0 (access May 30, 2017).
[33] https://www.washingtonpost.com/national/health-science/cardinal-health-fined-44-million-for-opioid-reporting-violations/2017/01/11/4f217c44-d82c-11e6-9a36-1d296534b31e_story.html?utm_term=.7049c4431465 (accessed on May 30, 2017).
[34] http://www.nytimes.com/2013/06/12/business/walgreen-to-pay-80-million-settlement-over-painkiller-sales.html (accessed on May 30, 2017).

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stressed the critical role of each member of the supply chain in distributing controlled substances. The HDMA guidelines provided that "[a]t the center of a sophisticated supply chain, Distributors are uniquely situated to perform due diligence in order to help support the security of controlled substances they deliver to their customers."

116. Between the years in question, including 2007 through 2016, the Distributor Defendants have shipped millions of doses of highly addictive controlled opioid pain killers into Maverick County.

117. Pursuant to the Texas Controlled Substances Act the Defendants are required to:

> … keep a complete and accurate record of all controlled substances on hand, received manufactured, sold, dispensed, or otherwise disposed of and shall maintain such records and inventories in conformance with the record-keeping and inventory requirements of federal law and with any rules issued by State Board of Pharmacy.

118. According to the Drug Enforcement Administration Automation of Reports and Consolidated Orders System ("ARCOS"), from 2007 to 2016 pharmaceutical distributors, including Defendants, distributed at least 8,366,216 grams of opioid pharmaceuticals to retail providers such as pharmacies located within Maverick County zip codes.

119. This number only represents opioids distributed directly to retailers.

120. This number shows an alarming and suspicious rise in the ordering of opioid pain medications by retailers throughout Maverick County.

121. Many of these orders should have been stopped, or at the very least, investigated as potential suspicious orders.

122. The sheer volume of the increase in opioid pain medications, including OxyCodone, being distributed to retailers, should have put the Distributor Defendants on notice to investigate and report such orders.

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123. The Distributor Defendants delivered an excessive and unreasonable amount of opioid pain medications to retailers in Maverick County.

124. Upon information and belief, the Distributor Defendants did not refuse to ship or supply any opioid medications to any pharmacy in Maverick County from 2007 to the present.

125. The Defendant Distributors knew or should have known that they were distributing levels of opioid medications that far exceeded the legitimate needs of Maverick County.

126. The Defendant Distributors also paid their sales force bonuses and commissions on the sale of most or all of the highly addictive opioid pain medications within Maverick County.

127. The Distributor Defendants made substantial profits from the opioids sold in Maverick County..

128. By the actions and inactions described above, the Distributor Defendants showed a reckless disregard for the safety of the residents of Maverick County.

129. By the actions and inactions described above, the Distributor Defendants caused great harm to the County of Maverick.

130. Russell Portenoy, M.D., is an individual residing in New York. Defendant Dr. Portenoy is a physician licensed to practice medicine in the state of New York. Dr. Portenoy was instrumental in promoting opioids for sale and distribution nationally and in Maverick County.

131. Perry Fine, M.D., is an individual residing in Utah. Dr. Fine was instrumental in promoting opioids for sale and distribution nationally and in Maverick County.

132. Scott Fishman, M.D., is an individual residing in California. Dr. Fishman was instrumental in promoting opioids for sale and distribution nationally and in Maverick County.

133. Lynn Webster, M.D., is an individual residing in Utah. Dr. Webster was instrumental in promoting opioids for sale and distribution nationally and in Maverick County.

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## FACTS RELEVANT TO ALL CAUSES OF ACTION

### A. Background on Pain Medicine.

#### 1. Safe and Effective Treatment of Chronic Pain Centers on Informed Risk Management.

134. The practice of medicine centers on informed risk management. Prescribers must weigh the potential risks and benefits of each treatment option, as well as the risk of non-treatment.

135. Accordingly, the safe and effective treatment of chronic pain requires that a physician be able to weigh the relative risks of prescribing opioids against both (a) the relative benefits that may be expected during the course of opioid treatment and (b) the risks and benefits of alternatives.

136. This bedrock principle of full disclosure is particularly important in the context of chronic opioid therapy because of the risk that patients will become physically and psychologically dependent on the drugs, finding it difficult to manage or terminate their use.

137. The FDA-approved drug labels on each of Defendants' opioids do not attempt to advise physicians how to maximize the benefits and minimize the risks for patients on long-term chronic opioid therapy. The labels contain no dosing cap above which it would be unsafe for any doctor to prescribe to any patient. Nor do any of the labels provide a duration limit, after which the risks to a patient might increase. Thus, doctors and patients rely more heavily on educational materials such as treatment guidelines, CMEs, and scientific and patient education articles and websites to inform their treatment decisions.

#### 2. Opioid Use Is Associated with Known and Substantial Risks.

138. Opium has been recognized as a tool to relieve pain for millennia; so has the magnitude of its potential for abuse, addiction and its dangers. Opioids are related to illegal drugs like opium and heroin. In fact, types of fentanyl, a widely-distributed opioid in the United States, have now been made illegal in China.

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139. During the Civil War, opioids, then known as "tinctures of laudanum," gained popularity among doctors and pharmacists for their ability to reduce anxiety and relieve pain – particularly on the battlefield – and they were popularly used in a wide variety of commercial products ranging from pain elixirs to cough suppressants and beverages. By 1900, an estimated 300,000 people were addicted to opioids in the United States.[35] Many doctors prescribed opioids solely to avoid patients' withdrawal. Both the numbers of opioid addicts and the difficulty in weaning patients from opioids made clear their highly addictive nature.

140. Due to concerns about their addictive properties, opioids have been regulated at the federal level as controlled substances by the U.S. Drug Enforcement Administration ("DEA") since 1970. The labels for scheduled opioid drugs carry black box warnings of potential addiction and "[s]erious, life-threatening, or fatal respiratory depression," as the result of an excessive dose.

141. Studies and articles from the 1970s and 1980s also made the reasons to avoid opioids clear. Scientists observed negative outcomes from long-term opioid therapy in pain management programs; opioids' mixed record in reducing pain long-term and failure to improve patients' function; greater pain complaints as most patients developed tolerance to opioids; opioid patients' diminished ability to perform basic tasks; their inability to make use of complementary treatments like physical therapy due to the side effects of opioids; and addiction. Leading authorities discouraged, and even prohibited, the use of opioid therapy for chronic pain.

142. Discontinuing opioids after more than just a few weeks of therapy will cause most patients to experience withdrawal symptoms. These withdrawal symptoms include: severe anxiety, nausea, vomiting, headaches, agitation, insomnia, tremors, hallucinations, delirium, pain, and other

[35] Substance Abuse and Mental Health Services Administration, Medication-Assisted Treatment for Opioid Addiction in Opioid Treatment Programs, Treatment Improvement Protocol (TIP Services), No. 43 (2005).

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serious symptoms, which may persist for months after a complete withdrawal from opioids, depending on how long the patient had been using opioids.

143. When under the continuous influence of opioids over time, patients grow tolerant to their analgesic effects. As tolerance increases, a patient typically requires progressively higher doses to obtain the same levels of pain reduction to which he or she has become accustomed – up to and including doses that are "frighteningly high."[36] At higher doses, the effects of withdrawal are more substantial, thus leaving a patient at a much higher risk of addiction. A patient can take the opioids at the continuously escalating dosages to match pain tolerance and still overdose at recommended levels.

144. Dr. Andrew Kolodny, Chief Medical Officer for Phoenix House, a national addiction treatment program, has explained the effect of opioids as akin to "hijack[ing] the brain's reward system," which in turn convinces a user that "the drug is needed to stay alive."[37] A patient's fear of the unpleasant effects of discontinuing opioids combined with the negative reinforcement during a period of actual withdrawal can drive a patient to seek further opioid treatment—even where ineffective or detrimental to quality of life—simply to avoid the deeply unpleasant effects of withdrawal.

145. Patients that receive high doses of opioids as part of long-term opioid therapy are three to nine times more likely to suffer an overdose from opioid-related causes than those on low doses. As compared to available alternative pain remedies, scholars have suggested that tolerance to the respiratory depressive effects of opioids develops at a slower rate than tolerance to analgesic effects. Accordingly, the practice of continuously escalating doses to match pain tolerance can, in fact, lead to an overdose even when opioids are taken as recommended.

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[36] M. Katz, Long-term Opioid Treatment of Nonmalignant Pain: A Believer Loses His Faith, 170(16) Archives of Internal Med. 1422 (2010).

[37] David Montero, *Actor's Death Sows Doubt Among O.C.'s Recovering Opioid Addicts*, The Orange Cnty. Reg. (Feb. 3, 2014), http://www.ocregister.com/articles/heroin-600148-shaffer-hoffman.html (accessed May 30, 2017).

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146. Further, "a potential side effect from chronic use [of opioids] can be abuse and addiction . . . . [i]n fact, correct use and abuse of these agents are not polar opposites—they are complex, inter-related phenomena."[38] It is very difficult to tell whether a patient is physically dependent, psychologically dependent, or addicted. Drug-seeking behaviors, which are signs of addiction, will exist and emerge when opioids are suddenly not available, the dose is no longer effective, or tapering of a dose is undertaken too quickly.

147. Studies have shown that between 30% and 40% of long-term users of opioids experience problems with opioid use disorders.[39]

148. Each of these risks and adverse effects—dependence, tolerance, and addiction—is fully disclosed in the labels for each of Defendants' opioids (though, as described below, not in Defendants' marketing).[40] Prior to Defendants' deceptive marketing scheme, each of these risks was well-recognized by doctors and seen as a reason to use opioids to treat chronic pain sparingly and only after other treatments had failed.

149. Opioids vary by duration. Long-acting opioids, such as Purdue's OxyContin and MS Contin, Janssen's Nucynta ER and Duragesic, Endo's Opana ER, and Actavis's Kadian, are designed to be taken once or twice daily and are purported to provide continuous opioid therapy for, in general, 12 hours. Short-acting opioids, such as Cephalon's Actiq and Fentora, are designed to be taken in addition to long-acting opioids to address "episodic pain" and provide fast-acting, supplemental opioid therapy lasting approximately 4 to 6 hours.

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[38] Wilson M. Compton & Nora D. Volkow, *Major Increases in Opioid Analgesic Abuse in the United States: Concerns and Strategies*, 81(2) Drug & Alcohol Dependence 103, 106 (2006).

[39] Joseph A. Boscarino et al., *Risk factors for drug daependence among out-patients on opioid therapy in a large US health-care system*, 105(10) Addiction 1776 ( 2010); Joseph A. Boscarino et al., *Prevalence of Prescription Opioid-Use Disorder Among Chronic Pain Patients: Comparison of the DSM-5 vs. DSM-4 Diagnostic Criteria*, 30(3) Journal of Addictive Diseases 185 (2011).

[40] For example, Purdue's OxyContin label (October 5, 2011) states: "Physical dependence and tolerance are not unusual during chronic opioid therapy."

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150. Defendants promoted the idea that pain should be treated by taking long-acting opioids continuously and supplementing them with short-acting, rapid- onset opioids for episodic pain.

151. Defendant Purdue was aware that its drug OxyContin did not provide pain relief for up to 12 hours. Purdue was also aware of the risk that patients would then take additional pain medications, beyond what was prescribed, to make of up for that gap in time. Despite this knowledge, Purdue continued to market OxyContin as lasting for 12 hours.

152. While it was once thought that long-acting opioids would not be as susceptible to abuse and addiction as short-acting ones, this view has been discredited. OxyContin's label now states, as do all labels of Schedule II long-acting opioids, that the drug "exposes users to risks of addiction, abuse, and misuse, which can lead to overdose and death." The FDA has required extended release and long-acting opioids to adopt "Risk Evaluation Mitigation Strateg[ies]" on the basis that they present "a serious public health crisis of addiction, overdose, and death."[41]

153. In 2013, in response to a petition to restrict the labels of long-acting opioid products, the FDA noted the "grave risks" of opioids, "the most well-known of which include addiction, overdose, and even death."[42] The FDA further warned that "[e]ven proper use of opioids under medical supervision can result in life-threatening respiratory depression, coma, and death."[43] The FDA required that—going forward—opioid makers of long-acting formulations clearly communicate these risks in their labels. Thus, the FDA confirmed what had previously been accepted practice in the treatment of pain— that the adverse outcomes from opioid use include "addiction, unintentional

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[41] FDA, *Risk Evaluation and Mitigation Strategy (REMS) for Extended-Release and Long-Acting Opioids* (last updated Oct. 9, 2014), http://www.fda.gov/Drugs/DrugSafety/InformationbyDrugClass/ucm163647.htm (accessed May 30, 2017).

[42] Letter from Janet Woodcock, M.D., Dir., Ctr. for Drug Eval. & Res., to Andrew Kolodny, M.D., Pres. Physicians for Responsible Opioid Prescribing, Re Docket No. FDA-2012-P-0818 (Sept. 10, 2013).

[43] *Id.*

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overdose, and death" and that long-acting or extended release opioids "should be used ***only when alternative treatments are inadequate***."[44]

154. Notably, in reaching its conclusion, the FDA did not rely on new or otherwise previously unavailable scientific studies regarding the properties or effects of opioids.

155. The FDA-approved labels on each of Defendant's opioids do not attempt to advise physicians on how to maximize the benefits and minimize the risks for patients on long term opioid therapy. The labels contain no dosage cap above which it would be unsafe to prescribe to any patient. Nor do they provide a duration limit. Doctors and patients rely heavily on education materials, such as treatment guidelines, CMEs, and scientific and patient education articles and websites, to inform their treatment decisions.

156. On July 25, 2012, the Physician For Responsible Opioid Prescribing ("PROP"), a non-profit organization made up of doctors and other health care professionals, petitioned the FDA to change the labeling of opioid medications. The petition was signed by thirty-seven physicians located nationwide. In its letter to the FDA, the group stated that "an increasing body of medical literature suggests that long term-use of opioids may be neither safe nor effective for many patients, especially when prescribed in high doses."[45]

157. In its petition, PROP also stated that "many clinicians are under the false impression that chronic opioid therapy (COT) is an evidence-based treatment for chronic non-cancer pain" and that "these misconceptions lead to overprescribing and high dose prescribing." It was also their opinion that "the current label on opioid analgesics does not comply with [FDA law]".

158. As the basis for its petition, PROP provided "Statements of Scientific Basis for Petition" which provided a list of detailed reports and studies proving the risks of opioid medications,

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[44] *Id.* at 7 (emphasis in original).
[45] July 25, 2012 letter from PROP to FDA, accessed at http://www.citizen.org/documents/2048.pdf on May 30, 2017.

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the high risk of addiction, the exaggerated and false benefits, and further medically backed reasons to change the labelling of opioid medications to reduce prescribing.

159. In 2013, in response to a petition to require manufacturers to strengthen warnings on the labels of long-acting opioid products, the FDA warned of the "grave risks" of opioids, including "addiction, overdose, and even death." The FDA further warned, "[e]ven proper use of opioids under medical supervision can result in life- threatening respiratory depression, coma, and death." Because of those grave risks, the FDA said that long-acting or extended release opioids "should be used only when alternative treatments are inadequate."[46] The FDA required that – going forward – opioid makers of long-acting formulations clearly communicate these risks on their labels.

160. In 2016, the FDA expanded its warnings for immediate-release opioid pain medications, requiring similar changes to the labeling of immediate-release for opioid pain medications as it had for extended release opioids in 2013. The FDA also required several additional safety-labeling changes across all prescription opioid products to include additional information on the risk of these medications.[47]

161. The facts on which the FDA relied in 2013 and 2016 were well known to Defendants for many years since they began marketing these drugs.

**3. Long-Term Opioid Use Benefits Are Unproven and Contradicted.**

162. Despite the fact that opioids are now routinely prescribed, there has never been evidence of their safety and efficacy for long-term use.

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[46] Letter from Janet Woodcock, M.D., Dir., Ctr. For Drug Eval. & Res., to Andrew Kolodny, M.D., Pres. *Physicians for Responsible Opioid Prescribing,* Re Docket No. FDA-2012-P-0818 (Sept. 10, 2013) (emphasis in original).

[47] FDA announces enhanced warnings for immediate-release opioid pain medications related to risks of misuse, abuse, addiction, overdose and death. Available at http://www.fda.gov/newsevents/newsroom/pressannouncements/ucm491739.htm (accessed May 30, 2017).

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163. Defendants have always been aware of these gaps in knowledge. While promoting opioids to treat chronic pain, Defendants have failed to disclose the lack of evidence to support their long-term use and have failed to disclose the contradictory evidence that chronic opioid therapy actually makes patients sicker.

164. There are no controlled studies of the use of opioids beyond 16 weeks, and no evidence that opioids improve patients' pain and function long-term. The first random, placebo-controlled studies appeared in the 1990s, and revealed evidence only for short-term efficacy and only in a minority of patients.[48]

165. A 2004 report reviewed 213 randomized, controlled trials of treatments for cancer pain and showed that, while opioids had short-term efficacy, the data was insufficient to establish long-term effectiveness. Subsequent reviews of the use of opioids for cancer and non-cancer pain consistently note the lack of data to assess long-term outcomes. For example, a 2007 systematic review of opioids for back pain concluded that opioids have limited, if any, efficacy for back pain and that evidence did not allow judgments regarding long-term use. Similarly, a 2011 systematic review of studies for non-cancer pain found that evidence of long-term efficacy is poor. One year later, a similar review reported poor evidence of long-term efficacy for morphine, tramadol, and oxycodone, and fair evidence for transdermal fentanyl (approved only for use for cancer pain).

166. On the contrary, evidence exists to show that opioid drugs are not effective to treat chronic pain, and may worsen patients' health. A 2006 study-of-studies found that opioids as a class did not demonstrate improvement in functional outcomes over other non-addicting treatments. Most notably, it stated: "For functional outcomes, the other analgesics were significantly more effective than

[48] Nathaniel Katz, *Opioids: After Thousands of Years, Still Getting to Know You*, 23(4) Clin J. Pain 303 (2007); Roger Chou et al., *Research Gaps on Use of Opioids for Chronic Noncancer Pain*, 10(2) J. Pain 147 (2009).

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were opioids."[49] Another review of evidence relating to the use of opioids for chronic pain found that up to 22.9% of patients in opioid trials dropped out before the study began because of the intolerable effects of opioids, and that the evidence of pain relief over time was weak.

167. Endo's own research shows that patients taking opioids, as opposed to other prescription pain medicines, report higher rates of obesity (30% to 39%); insomnia (9% to 22%); and self-described fair or poor health (24% to 34%).

168. Increasing duration of opioid use is strongly associated with an increasing prevalence of mental health conditions (depression, anxiety, post-traumatic stress disorder, or substance abuse), increased psychological distress, and greater health care utilization.

169. As a pain specialist noted in an article titled *Are We Making Pain Patients Worse?*, "[O]pioids may work acceptably well for a while, but over the long term, function generally declines, as does general health, mental health, and social functioning. Over time, even high doses of potent opioids often fail to control pain, and these patients are unable to function normally."[50]

170. This is true both generally and for specific pain-related conditions. Studies of the use of opioids long-term for chronic lower back pain have been unable to demonstrate an improvement in patients' function. Conversely, research consistently shows that long-term opioid therapy for patients who have lower back injuries does not help patients return to work or to physical activity. This is due partly to addiction and other side effects.

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[49] Andrea D. Furlan et al., *Opioids for chronic noncancer pain: a meta-analysis of effectiveness and side effects*, 174(11) Can. Med. Ass'n J. 1589 (2006). This same study revealed that efficacy studies do not typically include data on opioid addiction. In many cases, patients who may be more prone to addiction are pre-screened out of the study pool. This does not reflect how doctors actually prescribe the drugs, because even patients who have past or active substance use disorders tend to receive higher doses of opioids. Karen H. Seal, *Association of Mental Health Disorders With Prescription Opioids and High-Risk Opioids in US Veterans of Iraq and Afghanistan*, 307(9) J. Am. Med. Ass'n 940 (2012).

[50] Andrea Rubenstein, *Are we making pain patients worse?*, Sonoma Medicine (Fall 2009).

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171. As many as 30% of patients who suffer from migraines have been prescribed opioids to treat their headaches. Users of opioids had the highest increase in the number of headache days per month, scored significantly higher on the Migraine Disability Assessment (MIDAS), and had higher rates of depression, compared to non-opioid users. A survey by the National Headache Foundation found that migraine patients who used opioids were more likely to experience sleepiness, confusion, and rebound headaches, and reported a lower quality of life than patients taking other medications.

172. The lack of evidence for the efficacy of opioid use long-term has been well-documented nationally in the context of workers' compensation claims, where some of the most detailed data exists. Claims involving workers who take opioids are almost four times as likely to reach costs of over $100,000 than claims without opioids, as these patients suffer greater side effects and are slower to return to work. Even adjusting for injury severity and self-reported pain score, taking an opioid for more than seven days and receiving more than one opioid prescription increased the risk that the patient would be on work disability one year later. A prescription for opioids, as the first treatment for a workplace injury, doubled the average length of the claim.

**4. Defendants' Impact on the Perception and Prescribing of Opioids.**

173. Before Defendants began the marketing campaign complained of herein, generally accepted standards of medical practice dictated that opioids should only be used short-term, for instance, for acute pain, pain relating to recovery from surgery, or for cancer or palliative care. In those instances, the risks of addiction are low or of little significance.

174. In 1986, the World Health Organization ("WHO") published an "analgesic ladder" for the treatment of cancer pain.[51] The WHO recommended treatment with over-the- counter or prescription acetaminophen or non-steroidal anti-inflammatory drugs ("NSAIDs") first, and then the use of unscheduled or combination opioids, and then stronger (Schedule II or III) opioids if pain

[51] http://apps.who.int/iris/bitstream/10665/43944/1/9241561009_eng.pdf (accessed May 30, 2017)

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persisted. The WHO ladder pertained only to the treatment of cancer pain, and did not contemplate the use of narcotic opioids for chronic pain—because the use of opioids for chronic pain was not considered appropriate medical practice at the time.

175. Studies and articles from the 1970s and 1980s made the reasons to avoid opioids clear. Scientists observed negative outcomes from long-term opioid therapy in pain management programs: opioids' mixed record in reducing pain long-term and failure to improve patients' function; greater pain complaints as most patients developed tolerance to opioids; opioid patients' diminished ability to perform basic tasks; their inability to make use of complementary treatments like physical therapy due to the side effects of opioids; and addiction. Leading authorities discouraged, or even prohibited, the use of opioid therapy for chronic pain.

176. In 1986, Defendant Dr. Russell Portenoy, who later became Chairman of the Department of Pain Medicine and Palliative Care at Beth Israel Medical Center in New York, while at the same time serving as a top spokesperson for drug companies, published an article reporting that "[f]ew substantial gains in employment or social function could be attributed to the institution of opioid therapy."[52]

177. Writing in 1994, Dr. Portenoy described the prevailing attitudes regarding the dangers of long-term use of opioids:

> The traditional approach to chronic nonmalignant pain does not accept the long-term administration of opioid drugs. This perspective has been justified by the perceived likelihood of tolerance, which would attenuate any beneficial effects over time, and the potential for side effects, worsening disability, and addiction. According to conventional thinking, the initial response to an opioid drug may appear favorable, with partial analgesia and salutary mood changes, but adverse effects inevitably occur thereafter. It is assumed that the motivation to improve function will cease as mental clouding occurs and the belief takes hold that the drug can, by itself, return the patient to a normal life. *Serious management problems are anticipated, including difficulty in discontinuing a problematic*

[52] Russell K. Portenoy & Kathleen M. Foley, *Chronic Use of Opioid Analgesics in Non-Malignant Pain: Report of 38 cases*, 25(2) Pain 171 (1986).

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> *therapy and the development of drug seeking behavior induced by the desire to maintain analgesic effects, avoid withdrawal, and perpetuate reinforcing psychic effects. There is an implicit assumption that little separates these outcomes from the highly aberrant behaviors associated with addiction.*[53]

According to Portenoy, these problems could constitute "compelling reasons to reject long term opioid administration as a therapeutic strategy in all but the most desperate cases of chronic nonmalignant pain."[54]

178. For the reasons outlined by Dr. Portenoy, and in the words of one researcher from the Harvard Medical School, "it did not enter [doctors'] minds that there could be a significant number of chronic pain patients who were successfully managed with opioids."[55] Defendants changed that perception.

### B. Defendants Promoted Their Branded Products Through Direct Marketing to Prescribers and Consumers.

179. Defendants' direct marketing proceeded on two tracks, serving two related purposes. First, Defendants worked through branded and unbranded marketing to build confidence in long-term opioid use by overstating its benefits and downplaying its risks, thereby expanding the chronic pain market. In addition, Defendants worked through their own staffs of sales representatives, physician speakers whom those representatives recruited, and advertising in medical journals to claim their share of that broader market. Defendants directed all of this activity through carefully designed marketing plans that were based on extensive research into prescriber habits and the efficacy of particular sales approaches and messages.

#### 1. Defendants Relied Upon Branded Advertisements.

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[53] Russell K. Portenoy, *Opioid Therapy for Chronic Nonmalignant Pain: Current Status*, 1 Progress in Pain Res. & Mgmt. 247 (1994) (emphasis added).
[54] *Id.*
[55] Igor Kissin, *Long-term opioid treatment of chronic nonmalignant pain: unproven efficacy and neglected safety?*, 6 J. Pain Research 513, 514 (2013) (quoting Loeser JD, *Five crises in pain management*, 20(1) Pain Clinical Updates 1-4 (2012).

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180. Defendants engaged in widespread advertising campaigns touting the benefits of their branded drugs. Defendants published print advertisements in a broad array of medical journals, ranging from those aimed at specialists, such as the *Journal of Pain* and *Clinical Journal of Pain*, to journals with wider medical audiences, such as the *Journal of the American Medical Association*. Defendants' advertising budgets peaked in 2011, when they collectively spent more than $14 million on the medical journal advertising of opioids, nearly triple what they spent in 2001. The 2011 total includes $8.3 million by Purdue, $4.9 million by Janssen, and $1.1 million by Endo.[56]

181. A number of these branded advertisements deceptively portrayed the benefits of opioid therapy for chronic pain. As just one example, a 2005 Purdue advertisement for OxyContin that ran in the *Journal of Pain* touted the drug as an "around-the-clock analgesic . . . for an extended period of time." The advertisement featured a man and boy fishing and proclaimed that "There Can Be Life With Relief." This depiction falsely implied that OxyContin provides both effective long-term pain relief and functional improvement, claims that, as described below, are unsubstantiated and contradicted in medical literature.

**2. Defendants Relied Upon Their Sales Forces and Recruited Physician Speakers.**

182. Each Defendant promoted the use of opioids for chronic pain through "detailers"—sales representatives who visited individual physicians and their staff in their offices—and small group speaker programs. By establishing close relationships with doctors, Defendants' sales representatives were able to disseminate their misrepresentations in targeted, one-on-one settings that allowed them to differentiate their opioids and to address individual prescribers' concerns about prescribing opioids for

[56] In 2011, Actavis spent less than $100,000 on such advertising, and Cephalon spent nothing. These companies' medical journal advertising peaked earlier, with Actavis spending $11.7 million in 2005, and Cephalon spending about $2 million in each of 2007 and 2008.

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chronic pain. Representatives were trained on techniques to build these relationships, with Actavis even rolling out an "Own the Nurse" kit as a "door opener" to time with doctors.

183. Defendants developed sophisticated plans to select prescribers for sales visits based on their specialties and prescribing habits. In accordance with common industry practice, Defendants purchase and closely analyze prescription sales data from IMS Health. This data allows them to precisely track the rates of initial prescribing and renewal by individual doctors, which in turn allows them to target, tailor, and monitor the impact of their appeals.

184. Defendants, in particular, relied upon "influence mapping," *i.e.*., using decile rankings or similar breakdowns to identify the high-volume prescribers on whom detailing would have the greatest sales impact. Endo, for example, identified prescribers representing 30% of its nationwide sales volume and planned to visit these physicians three times per month. Defendants also closely monitored doctors' prescribing after a sales representative's visit to allow them to refine their planning and messaging and to evaluate and compensate their detailers.

185. Defendants' sales representatives have visited hundreds of thousands of doctors, including thousands of visits to Maverick County prescribers, and as described herein, spread misinformation regarding the risks, benefits, and superiority of opioids for the treatment of chronic pain. This misinformation includes deceptive and unfair claims regarding the risks of opioids for chronic pain, particularly the risks of addiction, withdrawal, and high doses, as well as the benefits.

186. Each Defendant carefully trained its sales representatives to deliver company-approved messages designed to generate prescriptions of that company's drugs specifically, and opioids in general. Pharmaceutical companies exactingly direct and monitor their sales representatives—through detailed action plans, trainings, tests, scripts, role-plays, supervisor tag-alongs, and other means—to ensure that individual detailers actually deliver the desired messages and do not veer off-script. Pharmaceutical companies likewise require their detailers to deploy sales aids reviewed, approved, and

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supplied by the company and forbid them to use, in industry parlance, "homemade bread"—*i.e.*, promotional materials not approved by the company's marketing and compliance departments. Sales representatives' adherence to their corporate training is typically included in their work agreements. Departing from their company's approved messaging can, and does, lead to severe consequences including termination of employment.

187. Besides carefully training their sales representatives, Defendants used surveys of physicians—conducted by third-party research firms—to assess how well their core messages came across to prescribers.

188. In addition to making sales calls, Defendants' detailers also identified doctors to serve, for payment, on Defendants' speakers' bureaus and to attend programs with speakers and meals paid for by Defendants. Defendants almost always selected physicians who were "product loyalists," as they were sure to be asked whether they prescribe the drug themselves. Endo, for instance, sought to use specialists in pain medicine—including high prescribers of its drugs—as local "thought leaders" to market Opana ER to primary care doctors. Such invitations are lucrative to the physicians selected for these bureaus; honorarium rates range from $800 to $2,000 per program, depending on the type of event, speaker training is typically compensated at $500 per hour.

189. These speaker programs and associated speaker trainings serve three purposes: they provide an incentive to doctors to prescribe, or increase their prescriptions of, a particular drug; a forum in which to further market to the speaker him or herself; and an opportunity to market to the speaker's peers. Defendants grade their speakers and future opportunities are based on speaking performance, post-program sales, and product usage. Defendants also track the prescribing of event attendees, with Endo noting that "physicians who came into our speaker programs wrote more prescriptions for Opana ER after attending than before." It would make little sense for Defendants to devote significant resources to programs that did not increase their sales.

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190. Like the sales representatives who select them, speakers are expected to stay "on message"—indeed, they agree in writing to follow the slide decks provided to them. Endo's speaker rules, for example, provide that "all slides must be presented in their entirety and without alterations . . . and in sequence." This is important because the FDA regards promotional talks as part of product labeling, and requires their submission for review. Speakers thus give the appearance of providing independent, unbiased presentations on opioids, when in fact they are presenting a script prepared by Defendants' marketing departments. Although these meal-based speaker events are more expensive to host, and typically have lower attendance than CMEs, they are subject to less professional scrutiny and thus afford Defendants greater freedom in the messages they present.

191. Defendants devoted massive resources to these direct sales contacts with prescribers. In 2014, Defendants collectively spent $168 million on detailing branded opioids to physicians nationwide. This figure includes $108 million spent by Purdue, $34 million by Janssen, $13 million by Cephalon, $10 million by Endo, and $2 million by Actavis. The total figure is more than double Defendants' collective spending on detailing in 2000. Detailers' role in Defendants' overall promotional efforts was also carefully calibrated; Endo, for example, found that devoting 61% of its marketing budget to sales representatives reflected an "[a]ppropriate combination of personal . . . and non-personal . . . selling initiatives."

192. Defendants have spent hundreds of millions of dollars promoting their opioids through their respective sales forces because they understand that detailers' sales pitches are effective. Numerous studies indicate that marketing can and does impact doctors' prescribing habits,[57] and face-

[57] *See, e.g.,* Puneet Manchanda & Pradeep K. Chintagunta, *Responsiveness of Physician Prescription Behavior to Salesforce Effort: An Individual Level Analysis*, 15 (2-3) Mktg. Letters 129 (2004) (detailing has a positive impact on prescriptions written); Ian Larkin, *Restrictions on Pharmaceutical Detailing Reduced Off-Label Prescribing of Antidepressants and Antipsychotics in Children*, 33(6) Health Affairs 1014 ( 2014) (finding academic medical centers that restricted direct promotion by pharmaceutical sales representatives resulted in a 34% decline in on-label use of promoted drugs); *see also* Art Van Zee, *The Promotion and*

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to-face detailing has the highest influence on intent to prescribe. Defendants could see this phenomenon at work not only in the aggregate, as their sales climbed with their promotional spending, but also at the level of individual prescribers whom they targeted for detailing, and who responded by prescribing more of Defendants' drugs.

**3. Defendants Directed These Promotional Efforts Through Detailed Marketing Plans.**

193. Defendants guided their efforts to expand opioid prescribing through comprehensive marketing and business plans for each drug. These documents, based on the companies' extensive market research, laid out ambitious plans to bring in new prescribers and increase overall prescribing of Defendants' opioids.

a. Targeting categories of prescribers

194. Defendants targeted, by zip codes and other local boundaries, individual health care providers for detailing. Defendants chose their targets based on the potential for persuading a provider to prescribe, ease of in-person access, and the likelihood of higher numbers of prescriptions at higher doses, with no correlation to demonstrated need or demand for opioid therapy, or to risk of abuse.

195. Collectively, Defendants' marketing plans evince dual strategies, which often operated parallel to one another. Defendants' sales representatives continued to focus their detailing efforts on pain specialists and anesthesiologists, the highest-volume prescribers of opioids and, as a group, more educated than other practitioners about opioids' risks and benefits. Seeking to develop market share and expand sales, however, Defendants also targeted increasing numbers and types of prescribers for marketing.

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*Marketing of OxyContin: Commercial Triumph, Public Health Tragedy*, 99(2) Am J. Pub. Health 221 (2009) (correlating an increase of OxyContin prescriptions from 670,000 annually in 1997 to 6.2 million in 2002 to a doubling of Purdue's sales force and trebling of annual sales calls).

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196. This expanded market of prescribers was, as a group, less informed about opioids and, as market research concluded, more susceptible to Defendants' marketing messages. These prescribers included nurse practitioners and physician assistants who, a 2012 Endo business plan noted, were "share acquisition" opportunities because they were "3x times more responsive than MDs to details" and wrote "96% of [their] prescriptions . . . without physician consult."

197. The expanded market also included internists and general practitioners who were low- to mid-volume prescribers. Actavis, for example, rolled out a plan in 2008 to move beyond "Kadian loyalists" to an "expanded audience" of "low morphine writers."

b. Increasing "direct to consumer" marketing

198. Defendants knew that physicians were more likely to prescribe their branded medications when patients asked for those medications. Endo's research, for example, found that such communications resulted in greater patient "brand loyalty," with longer durations of Opana ER therapy and fewer discontinuations. Defendants thus increasingly took their opioid sales campaigns directly to consumers, including through patient-focused "education and support" materials. These took the form of pamphlets, videos, or other publications that patients could view in their physician's office, as well as employer and workers' compensation plan initiatives to, as Endo put it, "[d]rive demand for access through the employer audience by highlighting cost of disease and productivity loss."

199. Defendants also knew that one of the largest obstacles to patients starting and remaining on their branded opioids—including by switching from a competitor's drug—was out- of-pocket cost. They recognized they could overcome this obstacle by providing patients financial assistance with their insurance co-payments, and each of the Defendants did so through vouchers and coupons distributed during detailing visits with prescribers. A 2008 Actavis business review, for example, highlighted co-pay assistance, good for up to $600 per patient per year, as a way to drive

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conversions to Kadian from competitor drugs like Avinza and MS Contin. In 2012, Janssen planned to distribute 1.5 million savings cards worth $25 each.

c. Differentiating each brand

200. Purdue's OxyContin was the clear market leader in prescription opioid therapy, with 30% of the market for analgesic drugs in 2012. However, by 2010, Defendants had begun facing increasing pushback from the medical community and regulators based on the growing problems of opioid addiction and abuse. Both market conditions prompted Defendants to pursue product differentiation strategies—particularly an emphasis on their products being less subject to diversion, abuse, and addiction—as a means of grabbing market share from Purdue and other competitors.

201. Endo, for example, tracked in detail prescriber "switching" from OxyContin to Opana ER. Actavis and Janssen did the same for switches to Kadian and Nucynta ER, respectively. Pressure to stand out among other drugs resulted in Defendants identifying marketing themes that thereafter were reflected in Defendants' deceptive and harmful messages to physicians and consumers. A 2008 Janssen plan emphasized "value" messaging in support of Nucynta ER, including claims of less dose escalation, lower toxicity, fewer withdrawal symptoms, and less dependence, and a 2009 Opana ER market research report focused on greater potency and lower abuse potential of Opana ER vis-à-vis OxyContin.

d. Moving beyond office visits

202. Defendants sought to reach additional prescribers by expanding beyond traditional sales calls and speaker events to new channels for their messages. For their sales forces, these included marketing to prescribers through voice mail, postcards, and email—so- called "e-detailing."

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Defendants also created new platforms for their speakers by implementing "peer to peer" programs such as teleconferences and webinars that were available to prescribers nationally. These programs allowed Defendants to use this more seemingly credible vehicle to market to, among other hard-to-reach audiences, prescribers at hospitals, academic centers, and other locations that limit or prohibit in-person detailing. Employing these new approaches, each Defendant relied heavily on speakers to promote its drugs.

**4. Defendants Marketed Opioids in Maverick County Using the Same Strategies and Messages They Employed Nationwide.**

203. Defendants employed the same marketing plans and strategies and deployed the same messages in Maverick County as they did nationwide.

204. Across the pharmaceutical industry, "core message" development is funded and overseen on a national basis by corporate headquarters. This comprehensive approach ensures that Defendants' messages are accurately and consistently delivered across marketing channels—including detailing visits, speaker events, and advertising—and in each sales territory. Defendants consider this high level of coordination and uniformity crucial to successfully marketing their drugs.

205. Defendants ensure marketing consistency nationwide through national and regional sales representative training; national training of local medical liaisons, the company employees who respond to physician inquiries; centralized speaker training; single sets of visual aids, speaker slide decks, and sales training materials; and nationally coordinated advertising. Defendants' sales representatives and physician speakers were required to stick to prescribed talking points, sales messages, and slide desks, and supervisors traveled with them periodically to check on both their performance and compliance.

206. As they did nationwide, Defendants extensively tracked the prescribing behavior of County-area health care providers and used that data to target their detailing and speaker- recruiting

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efforts. Top prescribers were profiled at the city, region, zip code, and sometimes facility levels, with information about their specialty, prescribing patterns (including product and dose), product loyalty and refill history. Providers' prescribing volume was ranked and sorted into deciles.

207. As described herein, misrepresentations and deceptions regarding the risks, benefits, and superiority of opioid use to treat chronic pain were part and parcel of Defendants' marketing campaigns in Maverick County.

### C. Defendants Used "Unbranded" Marketing to Evade Regulations and Consumer Protection Laws.

208. In addition to their direct marketing efforts, Defendants used unbranded, third- party marketing, which they deployed as part of their national marketing strategies for their branded drugs. Each Defendant executed these strategies through a network of third-party KOLs and Front Groups, with which it acted in concert by funding, assisting, encouraging, and directing their efforts. At the same time, Defendants exercised substantial control over the content of the messages third parties generated and disseminated, and distributed certain of those materials themselves. As with their other marketing strategies, Defendants' unbranded marketing created, and relied upon, an appearance of independence and credibility that was undeserved but central to its effectiveness. Unlike their direct promotional activities, Defendants' unbranded marketing allowed them to evade the oversight of federal regulators and gave them greater freedom to expand their deceptive messages.

#### 1. Regulations Governing Branded Promotion Require that it Be Truthful, Balanced, and Supported by Substantial Evidence.

209. Drug companies that make, market, and distribute opioids are subject to generally applicable rules requiring truthful marketing of prescription drugs. A drug company's branded marketing, which identifies and promotes a specific drug, must: (a) be consistent with its label and supported by substantial scientific evidence; (b) not include false or misleading statements or material

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omissions; and (c) fairly balance the drug's benefits and risks.[58] The regulatory framework governing the marketing of specific drugs reflects a public policy designed to ensure that drug companies, which are best suited to understand the properties and effects of their drugs, are responsible for providing prescribers with the information they need to accurately assess the risks and benefits of drugs for their patients.

210. Further, the Federal Food, Drug, and Cosmetic Act ("FDCA") prohibits the sale in interstate commerce of drugs that are "misbranded." A drug is "misbranded" if it lacks "adequate directions for use" or if the label is false or misleading "in any particular."[59] "Adequate directions for use" are directions "under which the layman can use a drug safely and for the purposes for which it is intended."[60] "Labeling" includes more than the drug's physical label; it also includes "all . . . other written, printed, or graphic matter . . . accompanying" the drug, including promotional material.[61] "The term "accompanying" is interpreted broadly to include promotional materials—posters, websites, brochures, books, and the like—disseminated by or on behalf of the manufacturer of the drug.[62] Thus, Defendants' promotional materials are part of their drugs' labels and are required to be accurate, balanced, and not misleading.

211. Labeling is misleading if it is not based on substantial evidence, if it materially misrepresents the benefits of the drug, or if it omits material information about or minimizes the frequency or severity of a product's risks. "The most serious risks set forth in a product's labeling are generally material to ***any*** presentation of efficacy." The FDA notes that "[b]ecause people expect to see risk information, there is no reason for them to imagine that the product has important risks that

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[58] 21 U.S.C. § 352(a); 21 C.F.R. §§ 1.21(a), 202.1(e)(3), 202.1(e)(6).
[59] 21 U.S.C. §§ 352.
[60] 21 C.F.R. § 201.5.
[61] 21 U.S.C. § 321(m).
[62] *See id.*

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have been omitted . . . especially if some risks are included."[63] Promotion that fails to present the most important risks of the drug as prominently as its benefits lacks fair balance and is therefore deceptive.

212. It is also illegal for drug companies to distribute materials that exclude contrary evidence or information about the drug's safety or efficacy or present conclusions that "clearly cannot be supported by the results of the study."[64] Further, drug companies must not make comparisons between their drugs and other drugs that represent or suggest that "a drug is safer or more effective than another drug in some particular when it has not been demonstrated to be safer or more effective in such particular by substantial evidence or substantial clinical experience."[65]

213. While the FDA must approve a drug's label, it is the drug company's responsibility to ensure that the material in its label is accurate and complete and is updated to reflect any new information.[66] Promotional materials also must be submitted to the FDA when they are first used or disseminated. The FDA does not have to approve these materials in advance; if, upon review, the FDA determines that materials marketing a drug are misleading, it can issue an untitled letter or warning letter. The FDA uses untitled letters for violations such as overstating the effectiveness of the drug or making claims without context or balanced information. Warning letters address promotions involving safety or health risks and indicate the FDA may take further enforcement action.

**2. Defendants Deployed Front Groups and Doctors to Disseminate Unbranded Information on Their Behalf.**

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[63] FDA, *Draft Guidance for Industry, Presenting Risk Information in Prescription Drug and Medical Device Promotion*, May 2009, at 14.

[64] 21 C.F.R. § 99.101(a)(4).

[65] 21 C.F.R. § 202.1(e)(6)(ii).

[66] *See* 21 C.F.R. § 201.56 (providing general requirements for prescription drug labeling); *see also Wyeth v. Levine*, 555 U.S. 555 (2009) (holding that a drug company bears responsibility for the content of its drug labels at all times); 21 C.F.R. § 314.70(c)(6) (iii)(A-C) (allowing manufacturers to make changes that "strengthen . . . a warning, precaution, or adverse reaction" or "strengthen a statement about drug abuse, dependence, psychological effect, or overdosage").

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214. Drug companies market both directly and indirectly, using third party validators (such as scientists, physicians, patient or professional organizations) that appear to be independent and therefore more credible. The FDA has made clear that its promotional requirements apply to both forms of marketing:

> FDA's regulation of prescription drug product promotion extends both to promotional activities that are carried out by the firm itself, and to promotion conducted on the firm's behalf.
>
> . . . .
>
> Therefore, a firm is responsible for the content generated by its employees or any agents acting on behalf of the firm who promote the firm's product. For example, if an employee or agent of a firm, such as a medical science liaison or paid speaker (e.g., a key opinion leader) acting on the firm's behalf, comments on a third- party site about the firm's product, the firm is responsible for the content its employee or agent provides. A firm is also responsible for the content on a blogger's site if the blogger is acting on behalf of the firm.[67]

215. In addition to being carried out directly or through third parties, drug companies' promotional activity can be branded or unbranded; unbranded marketing refers not to a specific drug, but more generally to a disease state or treatment. By using unbranded communications, drug companies can sidestep the extensive regulatory framework governing branded communications.

216. Defendants disseminated many of their false, misleading, imbalanced, and unsupported statements indirectly, through KOLs and Front Groups, and in unbranded marketing materials. These KOLs and Front Groups were important elements of Defendants' marketing plans, which specifically contemplated their use, because they seemed independent and therefore outside FDA oversight. Through unbranded materials, Defendants, with their own knowledge of the risks, benefits and advantages of opioids, presented information and instructions concerning opioids generally that were

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[67] FDA, *Draft Guidance for Industry on Fulfilling Regulatory Requirements for Postmarketing Submissions of Interactive Promotional Media for Prescription Human and Animal Drugs and Biologics*, January 2014, at 1, 4, http://www.fda.gov/downloads/drugs/guidancecomplianceregulatoryinformation/guidances/ucm381352.pdf (accessed May 30, 2017).

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contrary to, or at best, inconsistent with information and instructions listed on Defendants' branded marketing materials and drug labels. Defendants did so knowing that unbranded materials typically are not submitted to or reviewed by the FDA.

217. Even where such unbranded messages were channeled through third-party vehicles, Defendants adopted these messages as their own when they cited to, edited, approved, and distributed such materials knowing they were false, misleading, unsubstantiated, unbalanced, and incomplete. Unbranded brochures and other materials that are "disseminated by or on behalf of [the] manufacturer" constitute drug "labeling" that may not be false or misleading in any particular. *See* 21. C.F.R. 202.1(e)(7)(l)(2).[68] Defendants' sales representatives distributed third-party marketing material that was deceptive to Defendants' target audiences. Defendants are responsible for these materials.

218. Moreover, Defendants took an active role in guiding, reviewing, and approving many of the misleading statements issued by these third parties, ensuring that Defendants were consistently aware of their content. By funding, directing, editing, and distributing these materials, Defendants exercised control over their deceptive messages and acted in concert[69] with these third parties to fraudulently promote the use of opioids for the treatment of chronic pain.

219. For example, drug companies have been admonished for making functional claims in FDA-reviewed branded materials if there is no evidence for such claims. Thus, drug companies were put on notice that the FDA would not allow such claims in branded materials. Defendants instead

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[68] This regulation provides: "Brochures, booklets, mailing pieces, detailing pieces, file cards, bulletins, calendars, price lists, catalogs, house organs, letters, motion picture films, film strips, lantern slides, sound recordings, exhibits, literature, and reprints and similar pieces of printed, audio, or visual matter descriptive of a drug and the references published . . . containing drug information supplied by the manufacturer, packer, or distributor of the drug and which are disseminated by or on behalf of its manufacturer, packer, or distributor are hereby determined to be labeling, as defined in section 201(m) of the act." As labeling, such third party-created content distributed by a drug company may not be misleading and must meet the accuracy, substantiation, and fair balance requirements in the FDCA.

[69] As used in this Complaint, the allegation that Defendants "acted in concert" with third parties is intended to mean *both* that they conspired with these third parties to achieve some end and that they aided and abetted these third parties in the commission of acts necessary to achieve it.

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created and disseminated these same unsupported claims—that opioids allow patients to sleep, return to work, or walk more easily—through *unbranded* marketing materials.

220. The third-party publications Defendants assisted in creating and distributing did not include the warnings and instructions mandated by their FDA-required drug labels and consistent with the risks and benefits known to Defendants. For example, these publications either did not disclose the risks of addiction, abuse, misuse, and overdose, or affirmatively denied that patients faced a serious risk of addiction.

221. By acting through third parties, Defendants were able to both avoid FDA scrutiny and give the false appearance that the messages reflected the views of independent third parties. Later, Defendants would cite to these sources as "independent" corroboration of their own statements. As one physician adviser to Defendants noted, third-party documents not only had greater credibility, but broader distribution as doctors did not "push back" at having materials from, for example, the non-profit American Pain Foundation ("APF") on display in their offices, as they might with first party, drug company pieces. Nevertheless, the independence of these materials was a ruse—Defendants were in close contact with these third parties, paid for and were aware of the misleading information they were disseminating about the use of opioids to treat chronic pain, and regularly helped them to tailor and distribute their misleading, pro-opioid messaging.

222. As part of a strategic marketing scheme, Defendants spread and validated their deceptive messages through the following vehicles: (a) KOLs, who could be counted upon to write favorable journal articles and deliver supportive CMEs; (b) a body of biased and unsupported scientific literature; (c) treatment guidelines; (d) CMEs; (e) unbranded patient education materials; and (f) Front Group patient-advocacy and professional organizations, which exercised their influence both directly and through Defendant-controlled KOLs who served in leadership roles in those organizations.

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a. Defendants' Use of KOLs

223. Defendants cultivated a small circle of doctors who, upon information and belief, were selected and sponsored by Defendants solely because they favored the aggressive treatment of chronic pain with opioids. Defendants' support helped these doctors become respected industry experts. In return, these doctors repaid Defendants by touting the benefits of opioids to treat chronic pain.

224. Pro-opioid doctors have been at the hub of Defendants' promotional efforts, presenting the appearance of unbiased and reliable medical research supporting the broad use of opioid therapy for chronic pain. KOLs have written, consulted on, edited, and lent their names to books and articles, and given speeches and CMEs supportive of chronic opioid therapy. They have served on committees that developed treatment guidelines that strongly encourage the use of opioids to treat chronic pain (even while acknowledging the lack of evidence in support of that position) and on the boards of pro-opioid advocacy groups and professional societies that develop, select, and present CMEs. Defendants were able to exert control of each of these modalities through their KOLs.

225. In return, the KOLs' association with Defendants provided not only money, but prestige, recognition, research funding, and avenues to publish. This positioned them to exert even more influence in the medical community.

226. Although some KOLs initially may have advocated for more permissive opioid prescribing with honest intentions, Defendants cultivated and promoted only those KOLs who could be relied on to help broaden the chronic opioid therapy market. Defendants selected, funded, and elevated those doctors whose public positions were unequivocal and supportive of using opioids to

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treat chronic pain.[70] These doctors' professional reputations were then dependent on continuing to promote a pro-opioid message, even in activities that were not directly funded by the drug companies.

227. Defendants cited and promoted favorable studies or articles by these KOLs. By contrast, Defendants did not support, acknowledge, or disseminate the publications of doctors critical of the use of chronic opioid therapy. Indeed, one prominent KOL sponsored by Defendants, Russell Portenoy, stated that he was told by a drug company that research critical of opioids (and the doctors who published that research) would never obtain funding. Some KOLs have even gone on to become direct employees and executives of Defendants, like Dr. David Haddox, Purdue's Vice President of Risk Management, or Dr. Bradley Galer, Endo's former Chief Medical Officer.

228. Defendants provided substantial opportunities for KOLs to participate in research studies on topics Defendants suggested or chose, with the predictable effect of ensuring that many favorable studies appeared in the academic literature. As described by Dr. Portenoy, drug companies would approach him with a study that was well underway and ask if he would serve as the study's author. Dr. Portenoy regularly agreed.

229. Defendants also paid KOLs to serve as consultants or on their advisory boards and give talks or present CMEs, typically over meals or at conferences. Since 2000, Cephalon, for instance, has paid doctors more than $4.5 million for programs relating to its opioids.

230. These KOLs were carefully vetted to ensure that they were likely to remain on-message and supportive of a pharmaceutical industry agenda. One measure was a doctor's prior work for trusted Front Groups.

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[70] Opioid-makers were not the first to mask their deceptive marketing efforts in purported science. The tobacco industry also used KOLs in its effort to persuade the public and regulators that tobacco was not addictive or dangerous. For example, the tobacco companies funded a research program at Harvard and chose as its chief researcher a doctor who had expressed views in line with industry's views. He was dropped when he criticized low-tar cigarettes as potentially more dangerous, and later described himself as a pawn in the industry's campaign.

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231. Defendants kept close tabs on the content of the misleading materials published by these KOLs. In many instances, they also scripted what these KOLs said—as they did with all their recruited speakers. The KOLs knew, or deliberately ignored, the misleading way in which they portrayed the use of opioids to treat chronic pain to patients and prescribers, but they continued to publish those misstatements to benefit themselves and Defendants, all the while causing harm to County prescribers and patients.

i. *Defendant Russell Portenoy*

232. Defendant Dr. Russell Portenoy, former Chairman of the Department of Pain Medicine and Palliative Care at Beth Israel Medical Center in New York, is one example of a KOL whom Defendants identified and promoted to further their marketing campaign. Dr. Portenoy received research support, consulting fees, and honoraria from Cephalon, Endo, Janssen, and Purdue (among others), and was a paid consultant to Cephalon and Purdue.

233. Defendant Dr. Portenoy was instrumental in opening the door for the regular use of opioids to treat chronic pain. He served on the American Pain Society ("APS") / American Academy of Pain Medicine ("AAPM") Guidelines Committees, which endorsed the use of opioids to treat chronic pain, first in 1997 and again in 2009. He was also a member of the board of APF, an advocacy organization almost entirely funded by Defendants.

234. Defendant Dr. Portenoy also made frequent media appearances promoting opioids and spreading misrepresentations. He appeared on *Good Morning America* in 2010 to discuss the use of opioids long-term to treat chronic pain. On this widely watched program, broadcast in Maverick County and across the country, Dr. Portenoy claimed: "Addiction, when treating pain, is distinctly uncommon. If a person does not have a history, a personal history, of substance abuse, and does not

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have a history in the family of substance abuse, and does not have a very major psychiatric disorder, most doctors can feel very assured that that person is not going to become addicted."[71]

235. Defendant Dr. Portenoy has recently admitted that he "gave innumerable lectures in the late 1980s and '90s about addiction that weren't true." These lectures falsely claimed that fewer than 1% of patients would become addicted to opioids. According to Dr. Portenoy, because the primary goal was to "destigmatize" opioids, he and other doctors promoting them overstated their benefits and glossed over their risks. Dr. Portenoy also conceded that "[d]ata about the effectiveness of opioids does not exist."[72] Portenoy candidly stated: "Did I teach about pain management, specifically about opioid therapy, in a way that reflects misinformation? Well, . . . I guess I did."[73]

ii. *Defendant Lynn Webster*

236. Another KOL, Defendant Dr. Lynn Webster, was the co-founder and Chief Medical Director of Lifetree Clinical Research, an otherwise unknown pain clinic in Salt Lake City, Utah. Dr. Webster was President in 2013 and is a current board member of AAPM, a front group that ardently supports chronic opioid therapy.[74] He is a Senior Editor of *Pain Medicine*, the same journal that published Endo special advertising supplements touting Opana ER. Dr. Webster was the author of numerous CMEs sponsored by Cephalon, Endo, and Purdue. At the same time, Dr. Webster was receiving significant funding from Defendants (including nearly $2 million from Cephalon).

237. Dr. Webster had been under investigation for overprescribing by the DEA, which raided his clinic in 2010. More than 20 of Dr. Webster's former patients at the Lifetree Clinic have died of opioid overdoses. Ironically, Dr. Webster created and promoted the Opioid Risk Tool, a five question, one-minute screening tool relying on patient self-reports that purportedly allows doctors to

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[71] Good Morning America television broadcast, ABC News (Aug. 30, 2010).
[72] Thomas Catan & Evan Perez, *A Pain-Drug Champion Has Second Thoughts*, Wall St. J., Dec. 17, 2012.
[73] *Id.*
[74] Journal supplements are paid for by drug manufacturers and, although they may be designed to blend into the rest of the journal, are not peer-reviewed and constitute drug company advertising.

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manage the risk that their patients will become addicted to or abuse opioids. The claimed ability to pre-sort patients likely to become addicted is an important tool in giving doctors confidence to prescribe opioids long-term, and for this reason, references to screening appear in various industry-supported guidelines. Versions of Dr. Webster's Opioid Risk Tool appear on, or are linked to, websites run by Endo, Janssen, and Purdue. In 2011, Dr. Webster presented, via webinar, a program sponsored by Purdue titled, *Managing Patient's Opioid Use: Balancing the Need and the Risk*. Dr. Webster recommended use of risk screening tools, urine testing, and patient agreements to prevent "overuse of prescriptions" and "overdose deaths." This webinar was available to and was intended to reach County doctors.

238. Dr. Webster also was a leading proponent of the concept of "pseudoaddiction," the notion that addictive behaviors should be seen not as warnings, but as indications of undertreated pain. In Dr. Webster's description, the only way to differentiate the two was to *increase* a patient's dose of opioids. As he and his co-author wrote in a book entitled *Avoiding Opioid Abuse While Managing Pain* (2007), when faced with signs of aberrant behavior, increasing the dose "in most cases . . . should be the clinician's first response." Endo distributed this book to doctors. Years later, Dr. Webster reversed himself, acknowledging that "[pseudoaddiction] obviously became too much of an excuse to give patients more medication."[75]

b. <u>"Research" That Lacked Supporting Evidence</u>

239. Rather than find a way to actually test the safety and efficacy of opioids for long- term use, Defendants led people to believe that they already had. Defendants created a body of false, misleading, and unsupported medical and popular literature about opioids that (a) understated the risks and overstated the benefits of long-term use; (b) appeared to be the result of independent, objective research; and (c) was thus more likely to shape the perceptions of prescribers, patients and payors. This

[75] John Fauber & Ellen Gabler, *Networking Fuels Painkiller Boom*, Milwaukee Wisc. J. Sentinel (Feb. 19, 2012).

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literature was, in fact, marketing material focused on persuading doctors and consumers that the benefits of long-term opioid use outweighed the risks.

240. To accomplish this, Defendants—sometimes through third-party consultants and/or advocacy organizations—commissioned, edited, and arranged for the placement of favorable articles in academic journals. Defendants' internal documents reveal plans to submit research papers and "studies" to long lists of journals, including back-up options and last resort, "fast-track" application journals, that they could use if the pending paper was rejected everywhere else.

241. Defendants coordinated the timing and publication of manuscripts, abstracts, posters/oral presentations, and educational materials in peer-reviewed journals and other publications to support the launch and sales of their drugs. The plans for these materials did not originate in the departments within the Defendant organizations that were responsible for research, development or any other area that would have specialized knowledge about the drugs and their effects on patients, but in Defendants' marketing departments and with Defendants' marketing and public relations consultants. Defendants often relied on "data on file" or presented posters, neither of which are subject to peer review. They also published their articles not through a competitive process, but in paid journal supplements, which allowed Defendants to publish, in nationally circulated journals, studies supportive of their drugs.

242. Defendants also made sure that favorable articles were disseminated and cited widely in the medical literature, even where references distorted the significance or meaning of the underlying study. Most notably, Purdue promoted a 1980 reference in the well-respected *New England Journal of Medicine*: J. Porter & H. Jick, *Addiction Rare in Patients Treated with Narcotics*, 302(2) *New Eng. J. Med.* 123 (1980) ("Porter-Jick Letter"). It is cited 856 times in Google Scholar, and 86 times since 2010. It also

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appears as a reference in two CME programs in 2012 sponsored by Purdue and Endo.[76] Defendants and those acting on their behalf fail to reveal that this "article" is actually a letter-to-the-editor, not a peer-reviewed study (or any kind of study at all). The Porter-Jick Letter, reproduced in full below, describes a review of the charts of hospitalized patients who had received opioids. (Because it was a 1980 study, standards of care almost certainly would have limited opioids to acute or end-of-life situations, not chronic pain.)

**ADDICTION RARE IN PATIENTS TREATED WITH NARCOTICS**

*To the Editor:* Recently, we examined our current files to determine the incidence of narcotic addiction in 39,946 hospitalized medical patients[1] who were monitored consecutively. Although there were 11,882 patients who received at least one narcotic preparation, there were only four cases of reasonably well documented addiction in patients who had no history of addiction. The addiction was considered major in only one instance. The drugs implicated were meperidine in two patients,[2] Percodan in one, and hydromorphone in one. We conclude that despite widespread use of narcotic drugs in hospitals, the development of addiction is rare in medical patients with no history of addiction.

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1. Jick H, Miettinen OS, Shapiro S, Lewis GP, Siskind Y, Slone D. Comprehensive drug surveillance. JAMA. 1970; 213:1455-60.
2. Miller RR, Jick H. Clinical effects of meperidine in hospitalized medical patients. J Clin Pharmacol. 1978; 18:180-8.

243. The Porter-Jick Letter notes that, when these patients' records were reviewed, it found almost no references to signs of addiction, though there is no indication that caregivers were instructed to assess or document signs of addiction. None of these serious limitations is disclosed when Defendants, or those acting on their behalf, cite the Porter-Jick Letter, typically as the sole scientific support for the proposition that opioids are rarely addictive, even when taken long-term. In fact, Dr. Jick later complained that his letter had been distorted and misused.

[76] AAPM, Safe Opioid Prescribing Course, February 25-26, 2012, sponsored by Purdue and Endo; "Chronic Pain Management and Opioid Use," October 11, 2012, sponsored by Purdue. Each CME is available for online credit, including to prescribers in Maverick County.

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244. Defendants worked not only to create or elevate favorable studies in the literature, but to discredit or bury negative information. Defendants' studies and articles often targeted articles that contradicted Defendants' claims or raised concerns about chronic opioid therapy. In order to do so, Defendants—often with the help of third-party consultants—targeted a broad range of media to get their message out, including negative review articles, letters to the editor, commentaries, case-study reports, and newsletters.

245. Defendants' strategies—first, to plant and promote supportive literature and then, to cite the pro-opioid evidence in their promotional materials, while failing to disclose evidence that contradicts those claims—are in dereliction of their legal obligations. The strategies were intended to, and did, knowingly and intentionally distort the truth regarding the risks, benefits and superiority of opioids for chronic pain relief resulting in distorted prescribing patterns.

c. Treatment Guidelines

246. Treatment guidelines have been particularly important in securing acceptance for chronic opioid therapy. They are relied upon by doctors, especially the general practitioners and family doctors targeted by Defendants, who are otherwise not experts, nor trained, in the treatment of chronic pain. Treatment guidelines not only directly inform doctors' prescribing practices, but are cited throughout the scientific literature and referenced by third-party payors in determining whether they should cover treatments for specific indications. Furthermore, Endo's internal documents indicate that pharmaceutical sales representatives employed by Endo, Actavis, and Purdue discussed treatment guidelines with doctors during individual sales visits.

i. *FSMB*

247. The Federation of State Medical Boards ("FSMB") is a trade organization representing the various state medical boards in the United States. The state boards that comprise the FSMB

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membership have the power to license doctors, investigate complaints, and discipline physicians. The FSMB finances opioid- and pain-specific programs through grants from Defendants.

248. In 1998, the FSMB developed *Model Guidelines for the Use of Controlled Substances for the Treatment of Pain* ("FSMB Guidelines"), which FSMB admitted was produced "in collaboration with pharmaceutical companies."[77] The FSMB Guidelines taught not that opioids could be appropriate in limited cases or after other treatments had failed, but that opioids were "essential" for treatment of chronic pain, including as a first prescription option. The FSMB Guidelines failed to mention risks relating to respiratory depression and overdose, and they discussed addiction only in the sense that "inadequate understandings" of addiction can lead to "inadequate pain control."

249. A 2004 iteration of the FSMB Guidelines and the 2007 book adapted from the 2004 guidelines, *Responsible Opioid Prescribing*, also make these same claims. These guidelines were posted online and were available to and intended to reach County physicians.

250. The publication of *Responsible Opioid Prescribing* was backed largely by drug manufacturers, including Cephalon, Endo, and Purdue. The FSMB financed the distribution of *Responsible Opioid Prescribing* by its member boards by contracting with drug companies, including Endo and Cephalon, for bulk sales and distribution to sales representatives (for distribution to prescribing doctors).

251. In all, 163,131 copies of *Responsible Opioid Prescribing* were distributed to state medical boards (and through the boards, to practicing doctors), and the FSMB benefitted by earning approximately $250,000 in revenue and commissions from their sale. The FSMB website describes the book as the "leading continuing medication education (CME) activity for prescribers of opioid medications."

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252. Drug companies relied on FSMB guidelines to convey the message that "under-treatment of pain" would result in official discipline, but no discipline would result if opioids were prescribed as part of an ongoing patient relationship and prescription decisions were documented. FSMB turned doctors' fear of discipline on its head—doctors, who used to believe that they would be disciplined if their patients became addicted to opioids, were taught that they would be punished instead if they failed to prescribe opioids to their patients with pain.

253. FSMB, more recently, has moderated its stance. Although the 2012 revision of *Responsible Opioid Prescribing* continued to teach that "pseudoaddiction" is real and that opioid addiction risk can be managed through risk screening, it no longer recommended chronic opioid therapy as a first choice after the failure of over-the-counter medication and has heightened its addiction and risk warnings.

ii. *AAPM/APS Guidelines*

254. AAPM and the APS are professional medical societies, each of which received substantial funding from Defendants from 2009 to 2013 (with AAPM receiving over $2 million).

255. They issued a consensus statement in 1997, *The Use of Opioids for the Treatment of Chronic Pain*, which endorsed opioids to treat chronic pain and claimed that the risk that patients would become addicted to opioids was low.[78] The co-author of the statement, Dr. Haddox, was, at the time, a paid speaker for Purdue. Dr. Portenoy was the sole consultant. The consensus statement, which also formed the foundation of the FSMB Guidelines, remained on AAPM's website until 2011. The statement was taken down from AAPM's website only after a doctor complained, though it lingers on the internet elsewhere.[79]

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[78] Consensus statement, *The Use of Opioids for the Treatment of Chronic Pain*, APS & AAPM (1997), *available at* http://opi.areastematicas.com/generalidades/OPIOIDES.DOLORCRONICO.pdf (accessed May 30, 2017).

[79] *Id.*

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256. AAPM and APS issued their own guidelines in 2009 ("AAPM/APS Guidelines" or "Consensus Recommendation") and continued to recommend the use of opioids to treat chronic pain.[80] Fourteen of the 21 panel members who drafted the AAPM/APS Guidelines, including KOLs Dr. Portenoy and Dr. Perry Fine of the University of Utah, received support from Janssen, Cephalon, Endo, and Purdue.

257. The 2009 Guidelines promote opioids as "safe and effective" for treating chronic pain, despite acknowledging limited evidence, and conclude that the risk of addiction is manageable for patients regardless of past abuse histories. One panel member, Dr. Joel Saper, Clinical Professor of Neurology at Michigan State University and founder of the Michigan Headache & Neurological Institute, resigned from the panel because of his concerns that the 2009 Guidelines were influenced by contributions that drug companies, including Defendants, made to the sponsoring organizations and committee members. These AAPM/APS Guidelines have been a particularly effective channel of deception and have influenced not only treating physicians, but also the body of scientific evidence on opioids; the Guidelines have been cited 732 times in academic literature, were disseminated in Maverick County during the relevant time period, are still available online, and were reprinted in the *Journal of Pain*.

258. Defendants widely referenced and promoted the 2009 Guidelines without disclosing the acknowledged lack of evidence to support them.

iii. *American Geriatrics Society*

259. The American Geriatrics Society ("AGS"), a nonprofit organization serving health care professionals who work with the elderly, disseminated guidelines regarding the use of opioids for chronic pain in 2002 (*The Management of Persistent Pain in Older Persons*, hereinafter "2002 AGS

[80] Roger Chou et al., *Clinical Guidelines for the Use of Chronic Opioid Therapy in Chronic Noncancer Pain* , 10(2) The Journal of Pain: Official Journal of the American Pain Society 113-130 (2009)

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Guidelines") and 2009 (*Pharmacological Management of Persistent Pain in Older Persons*, hereinafter "2009 AGS Guidelines"). The 2009 AGS Guidelines included the following recommendations: "All patients with moderate to severe pain . . . should be considered for opioid therapy (low quality of evidence, strong recommendation)," and "the risks [of addiction] are exceedingly low in older patients with no current or past history of substance abuse."[81] These recommendations, which continue to appear on AGS's website, are not supported by any study or other reliable scientific evidence. Nevertheless, they have been cited 278 times in Google Scholar since their 2009 publication.

260. AGS contracted with Defendants Endo, Purdue, and Janssen to disseminate the 2009 Guidelines, and to sponsor CMEs based on them. These Defendants were aware of the content of the 2009 Guidelines when they agreed to provide funding for these projects. The 2009 Guidelines were first published online on July 2, 2009. AGS submitted grant requests to Defendants including Endo and Purdue beginning July 15, 2009. Internal AGS discussions in August 2009 reveal that it did not want to receive up-front funding from drug companies, which would suggest drug company influence, but would instead accept commercial support to disseminate the publication. However, by drafting the guidelines knowing that pharmaceutical company funding would be needed, and allowing these companies to determine whether to provide support only after they had approved the message, AGS ceded significant control to these companies. Endo, Janssen, and Purdue all agreed to provide support to distribute the guidelines.

261. According to one news report, AGS has received $344,000 in funding from opioid makers since 2009.[82] Five of 10 of the experts on the guidelines panel disclosed financial ties to Defendants, including serving as paid speakers and consultants, presenting CMEs sponsored by

[81] Pharmacological Management of Persistent Pain in Older Persons, 57 J. Am. Geriatrics Soc'y 1331, 1339, 1342 (2009), *available at* http://onlinelibrary.wiley.com/doi/10.1111/j.1526-4637.2009.00699.x/full (accessed May 30, 2017).

[82] John Fauber & Ellen Gabler, *Narcotic Painkiller Use Booming Among Elderly*, Milwaukee J. Sentinel, May 30, 2012.

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Defendants, receiving grants from Defendants, and investing in Defendants' stock. The Institute of Medicine recommends that, to ensure an unbiased result, fewer than 50% of the members of a guidelines committee should have financial relationships with drug companies.

iv. *Guidelines That Did Not Receive Defendants' Support*

262. The extent of Defendants' influence on treatment guidelines is demonstrated by the fact that independent guidelines—the authors of which did not accept drug company funding—reached very different conclusions. The 2012 *Guidelines for Responsible Opioid Prescribing in Chronic Non-Cancer Pain*, issued by the American Society of Interventional Pain Physicians ("ASIPP"), warned that "[t]he recent revelation that the pharmaceutical industry was involved in the development of opioid guidelines as well as the bias observed in the development of many of these guidelines illustrate that the model guidelines are not a model for curtailing controlled substance abuse and may, in fact, be facilitating it." ASIPP's Guidelines further advise that "therapeutic opioid use, specifically in high doses over long periods of time in chronic non-cancer pain starting with acute pain, not only lacks scientific evidence, but is in fact associated with serious health risks including multiple fatalities, and is based on emotional and political propaganda under the guise of improving the treatment of chronic pain." ASIPP recommends long-acting opioids in high doses only "in specific circumstances with severe intractable pain" and only when coupled with "continuous adherence monitoring, in well-selected populations, in conjunction with or after failure of other modalities of treatments with improvement in physical and functional status and minimal adverse effects."[83]

263. Similarly, the 2011 *Guidelines for the Chronic Use of Opioids*, issued by the American College of Occupational and Environmental Medicine, recommend against the "routine use of opioids in the management of patients with chronic pain," finding "at least moderate evidence that harms and

[83] Laxmaiah Manchikanti, et al., American Society of Interventional Pain Physicians (ASIPP) *Guidelines for Responsible Opioid Prescribing in Chronic Non-Cancer Pain: Part 1, Evidence Assessment*, 15 Pain Physician (Special Issue) S1-S66; *Part 2 – Guidance*, 15 Pain Physician (Special Issue) S67-S116 (2012).

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costs exceed benefits based on limited evidence," while conceding there may be patients for whom opioid therapy is appropriate.[84]

264. The *Clinical Guidelines on Management of Opioid Therapy for Chronic Pain*, issued by the U.S. Department of Veterans Affairs ("VA") and Department of Defense ("DOD") in 2010, notes that their review:

> revealed the lack of solid evidence based research on the efficacy of long-term opioid therapy. Almost all of the randomized trials of opioids for chronic non-cancer pain were short-term efficacy studies. Critical research gaps . . . include: lack of effectiveness studies on long-term benefits and harms of opioids . . .; insufficient evidence to draw strong conclusions about optimal approaches to risk stratification . . .; lack of evidence on the utility of informed consent and opioid management plans . . .; and treatment of patients with chronic non-cancer pain at higher risk for drug abuse or misuse.[85]

d. Continuing Medical Education

265. CMEs are ongoing professional education programs provided to doctors. Doctors are required to attend a certain number and, often, type of CME programs each year as a condition of their licensure. These programs are delivered in person, often in connection with professional organizations' conferences, online, or through written publications. Doctors rely on CMEs not only to satisfy licensing requirements, but to get information on new developments in medicine or to deepen their knowledge in specific areas of practice. Because CMEs are typically delivered by KOLs who are highly respected in their fields, and are thought to reflect these physicians' medical expertise, they can be especially influential with doctors.

266. The countless doctors and other health care professionals who participate in accredited CMEs constitute an enormously important audience for opioid reeducation. As one target, Defendants

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[84] *American College of Occupational and Environmental Medicine's Guidelines for the Chronic Use of Opioids*, (2011), *available at*: https://www.nhms.org/sites/default/files/Pdfs/ACOEM%202011-Chronic%20Pain%20Opioid%20.pdf (accessed May 30, 2017).

[85] Management of Opioid Therapy for Chronic Pain Working Group, VA/DoD Clinical Practice Guideline for Management of Opioid Therapy for Chronic Pain (May 2010), *available at* http://www.healthquality.va.gov/guidelines/Pain/cot/COT_312_Full-er.pdf (accessed May 30, 2017).

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aimed to reach general practitioners, whose broad area of focus and lack of specialized training in pain management made them particularly dependent upon CMEs and, as a result, especially susceptible to Defendants' deceptions.

267. In all, Defendants sponsored CMEs that were delivered thousands of times, promoting chronic opioid therapy and supporting and disseminating the deceptive and biased messages described in this Complaint. These CMEs, while often generically titled to relate to the treatment of chronic pain, focused on opioids to the exclusion of alternative treatments, inflated the benefits of opioids, and frequently omitted or downplayed their risks and adverse effects.

268. The American Medical Association ("AMA") has recognized that support from drug companies with a financial interest in the content being promoted "creates conditions in which external interests could influence the availability and/or content" of the programs and urges that "[w]hen possible, CME[s] should be provided without such support or the participation of individuals who have financial interests in the educational subject matter."[86]

269. Dozens of CMEs that were available to and attended or reviewed by County doctors during the relevant time period did not live up to the AMA's standards.

270. The influence of Defendants' funding on the content of these CMEs is clear. One study by a Georgetown University Medical Center professor compared the messages retained by medical students who reviewed an industry-funded CME article on opioids versus another group who reviewed a non-industry-funded CME article. The industry-funded CME did not mention opioid-related death once; the non-industry-funded CME mentioned opioid-related death 26 times. Students who read the industry-funded article more frequently noted the impression that opioids were underused in treating chronic pain. The "take-aways" of those reading the non- industry-funded CME

[86] Opinion 9.0115, *Financial Relationships with Industry in CME*, Am. Med. Ass'n (Nov. 2011), *available at* http://eo2.commpartners.com/users/ama/downloads/120328_Opinion_E-9_0115.pdf (accessed May 30, 2017).

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mentioned the risks of death and addiction much more frequently than the other group. Neither group could accurately identify whether the article they read was industry-funded, making clear the difficulty health care providers have in screening and accounting for source bias.[87]

271. By sponsoring CME programs presented by Front Groups like APF, AAPM, and others, Defendants could expect messages to be favorable to them, as these organizations were otherwise dependent on Defendants for other projects. The sponsoring organizations honored this principle by hiring pro-opioid KOLs to give talks that supported chronic opioid therapy. Defendant-driven content in these CMEs had a direct and immediate effect on prescribers' views on opioids. Producers of CMEs and Defendants measured the effects of CMEs on prescribers' views on opioids and their absorption of specific messages, confirming the strategic marketing purpose in supporting them.

e. Unbranded Patient Education

272. Pharmaceutical industry marketing experts see patient-focused advertising, including direct-to-consumer marketing, as particularly valuable in "increas[ing] market share . . . by bringing awareness to a particular disease that the drug treats."[88] Evidence also demonstrates that physicians are willing to acquiesce to patient demands for a particular drug— even for opioids and for conditions for which they are not generally recommended.[89] An Actavis marketing plan, for example, noted that "[d]irect-to-consumer marketing affects prescribing decisions." Recognizing this fact, Defendants put

[87] Adriane Fugh-Berman, *Marketing Messages in Industry-Funded CME*, PharmedOut (June 25, 2010), *available at* pharmedout.galacticrealms.com/Fugh-BermanPrescriptionforConflict6-25-10.pdf.

[88] Kanika Johar, *An Insider's Perspective: Defense of the Pharmaceutical Industry's Marketing Practices*, 76 Albany L. Rev. 299, 308 (2013).

[89] Prescribers often accede to patient requests. According to one study, nearly 20% of sciatica patients requesting oxycodone would receive a prescription for it, compared with 1% making no request. More than half of patients requesting a strong opioid received one. J.B. McKinlay et al., *Effects of Patient Medication Requests on Physician Prescribing Behavior*, 52(2) Med. Care 294 (2014).

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their relationships with Front Groups to work to engage in largely unbranded patient education about opioid treatment for chronic pain.

273. The drug companies expect that they will recoup their investment in direct-to-consumer advertisements by capturing at least some of any additional prescriptions that result from patients "asking their doctor" about drugs that can treat their pain. Doctors also may review direct-to-consumer materials sales representatives give them to distribute to patients.

f. Defendants' Use of Front Groups

274. As noted above, Defendants Cephalon, Endo, Janssen, and Purdue entered into arrangements with numerous organizations to promote opioids. These organizations depend upon Defendants for significant funding and, in some cases, for their survival. They were involved not only in generating materials and programs for doctors and patients that supported chronic opioid therapy, but also in assisting Defendants' marketing in other ways—for example, responding to negative articles and advocating against regulatory changes that would constrain opioid prescribing. They developed and disseminated pro-opioid treatment guidelines; conducted outreach to groups targeted by Defendants, such as veterans and the elderly; and developed and sponsored CMEs that focused exclusively on use of opioids to treat chronic pain. Defendants funded these Front Groups in order to ensure supportive messages from these seemingly neutral and credible third parties, and their funding did, in fact, ensure such supportive messages.

275. Several representative examples of such Front Groups are highlighted below, but there are others, too, such as APS, AGS, FSMB, American Chronic Pain Association ("ACPA"), AAPM, American Society of Pain Educators ("ASPE"), NPF, and PPSG.

i. *American Pain Foundation*

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276. The most prominent of Defendants' Front Groups was APF, which received more than $10 million in funding from opioid manufacturers from 2007 until it closed its doors in May 2012. Endo alone provided more than half of that funding; Purdue was next, at $1.7 million.

277. APF issued education guides for patients, reporters, and policymakers that touted the benefits of opioids for chronic pain and trivialized their risks, particularly the risk of addiction. APF also launched a campaign to promote opioids for returning veterans, which has contributed to high rates of addiction and other adverse outcomes—including death—among returning soldiers. APF also engaged in a significant multimedia campaign—through radio, television and the internet—to educate patients about their "right" to pain treatment, namely opioids. All of the programs and materials were available nationally and were intended to reach County residents.

278. In addition to Perry Fine, Russell Portenoy, and Scott Fishman, who served on APF's Board and reviewed its publications, another board member, Lisa Weiss, was an employee of a public relations firm that worked for both Purdue and APF.

279. In 2009 and 2010, more than 80% of APF's operating budget came from pharmaceutical industry sources. Including industry grants for specific projects, APF received about $2.3 million from industry sources out of total income of about $2.85 million in 2009; its budget for 2010 projected receipts of roughly $2.9 million from drug companies out of total income of about $3.5 million. By 2011, APF was entirely dependent on incoming grants from defendants Purdue, Cephalon, Endo, and others to avoid using its line of credit. As one of its board members, Russell Portenoy, explained, the lack of funding diversity was one of the biggest problems at APF.

280. APF held itself out as an independent patient advocacy organization. It often engaged in grassroots lobbying against various legislative initiatives that might limit opioid prescribing, and thus the profitability of its sponsors. It was often called upon to provide "patient representatives" for Defendants' promotional activities, including for Purdue's *Partners Against Pain* and Janssen's *Let's Talk*

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*Pain*. As laid out below, APF functioned largely as an advocate for the interests of Defendants, not patients. Indeed, as early as 2001, Purdue told APF that the basis of a grant was Purdue's desire to "strategically align its investments in nonprofit organizations that share [its] business interests."

281. In practice, APF operated in close collaboration with opioid makers. On several occasions, representatives of the drug companies, often at informal meetings at Front Group conferences, suggested activities and publications APF could pursue. APF then submitted grant proposals seeking to fund these activities and publications, knowing that drug companies would support projects conceived as a result of these communications.

282. APF assisted in other marketing projects for drug companies. One project funded by another drug company—*APF Reporter's Guide: Covering Pain and Its Management* (2008)[90]—recycled text that was originally created as part of the company's training document.

283. The same drug company made general grants, but even then, it directed how APF used them. In response to an APF request for funding to address a potentially damaging state Medicaid decision related to pain medications generally, the company representative responded, "I provided an advocacy grant to APF this year—this would be a very good issue on which to use some of that. How does that work?"

284. The close relationship between APF and the drug company was not unique, but in fact mirrors the relationships between APF and Defendants. APF's clear lack of independence—in its finances, management, and mission—and its willingness to allow Defendants to control its activities and messages, support an inference that each Defendant that worked with APF was able to exercise editorial control over its publications.

[90] https://assets.documentcloud.org/documents/277606/apf-reporters-guide.pdf (accessed May 30, 2017)

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285. Indeed, the U.S. Senate Finance Committee began looking into APF in May 2012 to determine the links, financial and otherwise, between the organization and the manufacturers of opioid painkillers. The investigation caused considerable damage to APF's credibility as an objective and neutral third party and Defendants stopped funding it. Within days of being targeted by Senate investigation, APF's board voted to dissolve the organization "due to irreparable economic circumstances." APF "cease[d] to exist, effective immediately."[91]

ii. *The American Academy of Pain Medicine*

286. The American Academy of Pain Medicine, with the assistance, prompting, involvement, and funding of Defendants, issued treatment guidelines and sponsored and hosted medical education programs essential to Defendants' deceptive marketing of chronic opioid therapy.

287. AAPM has received over $2.2 million in funding since 2009 from opioid manufacturers. AAPM maintains a corporate relations council, whose members pay $25,000 per year (on top of other funding) to participate. The benefits include allowing members to present educational programs at off-site dinner symposia in connection with AAPM's marquee event—its annual meeting held in Palm Springs, California, or other resort locations. AAPM describes the annual event as an "exclusive venue" for offering education programs to doctors.

288. Membership in the corporate relations council also allows drug company executives and marketing staff to meet with AAPM executive committee members in small settings. Defendants Endo, Purdue, Cephalon and Actavis were members of the council and presented deceptive programs to doctors who attended this annual event.

289. AAPM is viewed internally by Endo as "industry friendly," with Endo advisors and speakers among its active members. Endo attended AAPM conferences, funded its CMEs, and distributed its publications. The conferences sponsored by AAPM heavily emphasized sessions on

[91] http://www.painfoundation.org (last visited May 30, 2017).

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opioids—37 out of roughly 40 at one conference alone. AAPM's presidents have included top industry-supported KOLs Perry Fine, Russell Portenoy, and Lynn Webster. Dr. Webster was even elected president of AAPM while under a DEA investigation. Another past AAPM president, Dr. Scott Fishman, stated that he would place the organization "at the forefront" of teaching that "the risks of addiction are . . . small and can be managed."[92]

290. AAPM's staff understood that they and their industry funders were engaged in a common practice. Defendants were able to influence AAPM through both their significant and regular funding, and the leadership of pro-opioid KOLs within the organization.

**3. Defendants Acted in Concert with KOLs and Front Groups in the Creation, Promotion, and Control of Unbranded Marketing.**

291. Like cigarette manufacturers, which engaged in an industry-wide effort to misrepresent the safety and risks of smoking, Defendants worked with each other and with the Front Groups and KOLs they funded and directed to carry out a common scheme to deceptively present the risks, benefits, and superiority of opioids to treat chronic pain.

292. Defendants acted through and with the same network of Front Groups, funded the same KOLs, and often used the very same language and format to disseminate the same deceptive messages. These KOLs have worked reciprocally with Defendants to promote misleading messaging regarding the appropriate use of opioids to treat chronic pain. Although participants knew this information was false and misleading, these misstatements were nevertheless disseminated to Maverick County prescribers and patients.

293. One vehicle for their collective collaboration was Pain Care Forum ("PCF"). PCF began in 2004 as an APF project with the stated goals of offering "a setting where multiple

[92] Interview by Paula Moyer with Scott M. Fishman, M.D., Professor of Anesthesiology and Pain Medicine, Chief of the Division of Pain Medicine, Univ. of Cal., Davis (2005), http://www.medscape.org/viewarticle/500829 (accessed May 30, 2017).

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organizations can share information" and to "promote and support taking collaborative action regarding federal pain policy issues." APF President Will Rowe described the Forum as "a deliberate effort to positively merge the capacities of industry, professional associations, and patient organizations."

294. PCF is comprised of representatives from opioid manufacturers and distributors (including Cephalon, Endo, Janssen, and Purdue); doctors and nurses in the field of pain care; professional organizations (*e.g.*, American Academy of Pain Management, APS, and American Society of Pain Educators); patient advocacy groups (*e.g.*, APF and ACPA); and other like-minded organizations (*e.g.*, FSMB and Wisconsin Pain & Policy Studies Group), almost all of which received substantial funding from Defendants.

295. PCF, for example, developed and disseminated "consensus recommendations" for a Risk Evaluation and Mitigation Strategy ("REMS") for long-acting opioids that the FDA mandated in 2009 to communicate the risks of opioids to prescribers and patients.[93] This was critical as a REMS that went too far in narrowing the uses or benefits, or highlighting the risks of chronic opioid therapy, would deflate Defendants' marketing efforts. The recommendations—drafted by Will Rowe of APF—claimed that opioids were "essential" to the management of pain, and that the REMS "should acknowledge the importance of opioids in the management of pain and should not introduce new barriers."[94] Defendants worked with PCF members to limit the reach and manage the message of the REMS, which enabled them to maintain, and not undermine, their deceptive marketing of opioids for chronic pain.

[93] The FDA can require a drug maker to develop a REMS—which could entail (as in this case) an education requirement or distribution limitation—to manage serious risks associated with a drug.
[94] Defendants also agreed that short-acting opioids should also be included in REMS as not to disadvantage the long-acting, branded drugs.

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**4. Defendants Targeted Vulnerable and Lucrative Populations.**

a. The Elderly

296. Elderly patients taking opioids have been found to be exposed to elevated fracture risks, a greater risk for hospitalizations, and increased vulnerability to adverse drug effects and interactions, such as respiratory depression, which, as Defendants acknowledge in their labels (but not in their marketing), occurs more frequently in elderly patients. A 2010 paper in the Archives of Internal Medicine reported that elderly patients who used opioids had a significantly higher rate of death, heart attacks, and strokes than users of NSAIDs. Defendants' targeted marketing to the elderly and the absence of cautionary language in their promotional materials flies in the face of scientific evidence and their own labels, and creates a heightened risk of serious injury to elderly patients.

297. Defendants also promoted the notion—also without adequate scientific foundation—that the elderly are particularly unlikely to become addicted to opioids. AGS's 2009 Guidelines, for example, which Purdue, Endo, and Janssen publicized, described the risk of addiction as "exceedingly low in older patients with no current or past history of substance abuse." Yet, a 2010 study examining overdoses among long-term opioid users found that patients 65 or older were among those with the largest number of serious overdoses.

298. Defendants' efforts have paid off. Since 2007, prescriptions for the elderly have grown at twice the rate of prescriptions for adults between the ages of 40 and 59.

b. Veterans

299. Veterans, too, are suffering greatly from the effects of Defendants' targeted marketing. A 2008 survey showed that prescription drug abuse among military personnel doubled from 2002 to 2005, and then nearly tripled again over the next three years. In 2009, military doctors wrote 3.8

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million prescriptions for narcotic pain pills—four times as many as they did in 2001. Further, one-third of veterans prescribed opioids as of 2012 remained on take-home opioids for more than 90 days. Although many of these veterans are returning from service with traumatic injuries, the increase in opioid prescribing is disproportionate to the population and, in far too many cases, unsuited for their treatment. Among former service members receiving VA services nationally in a single year (2005), 1,013 had died of accidental drug overdoses—double the rate of the civilian population.

300. The County has a substantial population of veterans who must cope with the consequences of overprescribing opioids.

301. Opioids are particularly dangerous to veterans. According to a study published in the 2013 Journal of American Medicine, veterans returning from Iraq and Afghanistan who were prescribed opioids have a higher incidence of adverse clinical outcomes, such as overdoses and self-inflicted and accidental injuries; 40% of veterans with post-traumatic stress disorder received opioids and benzodiazepines (anti-anxiety drugs) that, when mixed with alcohol, can cause respiratory depression and death. According to a VA Office of Inspector General Report, despite the risks, 92.6% of veterans who were prescribed opioid drugs were also prescribed benzodiazepines.[95] Again, as with elderly patients, Defendants both purposefully sought to increase opioid prescribing to this vulnerable group and omitted from their promotional materials the known, serious risks opioids pose to them.

302. *Exit Wounds*, a 2009 publication sponsored by Purdue, distributed by APF with grants from Janssen and Endo, and written as a personal narrative of one veteran, describes opioids as "underused" and the "gold standard of pain medications" and fails to disclose the risk of addiction, overdose, or injury. It notes that opioid medications "increase a person's level of functioning" and that "[l]ong experience with opioids shows that people who are not predisposed to addiction are unlikely to become addicted to opioid pain medications." The book also asserts that "[d]enying a person opioid

[95] https://www.va.gov/oig/pubs/VAOIG-14-00895-163.pdf (accessed May 30, 2017)

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pain medication because he or she has a history of substance abuse or addiction is contrary to the model guidelines for prescribing opioids, published by the U.S. Federation of State Medical Boards." As laid out above, the FSMB itself received support from Defendants during the time it created and published its guidelines.

303. *Exit Wounds* minimizes the risks of chronic opioid therapy and does not disclose the risk that opioids may have fatal interactions with benzodiazepines, which were taken by a significant number of veterans.[96] It is not the unbiased narrative of a returning war veteran. It is pure marketing, sponsored by Purdue, Endo, and Janssen. The American Pain Foundation's name is prominently marked on the book's spine. Dr. Scott Fishman, then-chair of the APF, wrote the book's preface, which touted the APF as "an organization that raises public awareness, provides education, promotes research, and advocates for improved access to effective pain management – answering the unmet needs of our active military and veterans in pain."

304. Janssen, for example, supported the *Exit Wounds* marketing effort, advocacy of "improved access to effective pain management," and the book's insufficient disclosures, despite acknowledging on the label for its opioid Duragesic that its use with benzodiazepines "may cause respiratory depression, hypotension, and profound sedation or potentially result in coma." A similar warning is found on the labels of other Defendants' opioids.

305. The deceptive nature of *Exit Wounds* is obvious in comparing it to guidance on opioids published by the VA and DOD in 2010 and 2011. The VA's *Taking Opioids Responsibly* describes opioids as "dangerous." It cautions against taking extra doses and mentions the risk of overdose and the dangers of interactions with alcohol. The list of side effects from opioids includes decreased

[96] FDA guidance states that materials designed to target a particular audience should disclose risks particular to that audience. *See* FDA Notice, Guidance for Industry, "Brief Summary and Adequate Directions for Use: Disclosing Risk Information in Consumer-Directed Print Advertisements and Promotional Labeling for Prescription Drugs," August 6, 2015.

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hormones, sleep apnea, hyperalgesia, addiction, immune system changes, birth defects and death—none of which is disclosed in *Exit Wounds*.

**D. Why Defendants' Marketing Messages Are Misleading and Unfair**

306. Defendants' marketing of opioids for long-term use to treat chronic pain, both directly and with and through third parties, included information that was false, misleading, contrary to credible scientific evidence and their own labels, and lacked balance and substantiation. Their marketing materials omitted material information about the risks of opioids, and overstated their benefits. Moreover, Defendants inaccurately suggested that chronic opioid therapy was supported by evidence, and failed to disclose the lack of evidence in support of treating chronic pain with opioids.

307. There are seven primary misleading and unfounded representations. Defendants and the third parties with which they teamed:

- misrepresented that opioids improve function;
- concealed the link between long-term use of opioids and addiction;
- misrepresented that addiction risk can be managed;
- masked the signs of addiction by calling them "pseudoaddiction";
- falsely claimed withdrawal is easily managed;
- misrepresented or omitted the greater dangers from higher doses of opioids; and
- deceptively minimized the adverse effects of opioids and overstated the risks of NSAIDs.

308. In addition to these misstatements, Purdue purveyed an eighth deception that OxyContin provides a full 12 hours of pain relief.

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309. Exacerbating each of these misrepresentations and deceptions was the collective effort of Defendants and third parties to hide from the medical community the fact that the FDA "is not aware of adequate and well-controlled studies of opioid use longer than 12 weeks."[97]

**1. Defendants and Their Third-Party Allies Misrepresented that Opioids Improve Function**

310. Each of the following materials was created with the expectation that, by instructing patients and prescribers that opioids would improve patients' function and quality of life, patients would demand opioids and doctors would prescribe them. These claims also encouraged doctors to continue opioid therapy in the belief that failure to improve pain, function, or quality of life, could be overcome by increasing doses or prescribing supplemental short-acting opioids to take on an as-needed basis for breakthrough pain.

311. However, not only is there no evidence of improvement in long-term functioning, a 2006 study-of-studies found that "[f]or functional outcomes . . . other analgesics were significantly more effective than were opioids."[98] Studies of the use of opioids in chronic conditions for which they are commonly prescribed, such as low back pain, corroborate this conclusion and have failed to demonstrate an improvement in patients' function. Instead, research consistently shows that long-term opioid therapy for patients who have lower back injuries does not cause patients to return to work or physical activity.[99] Indeed, one Defendant's own internal marketing plans characterized functional

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[97] Letter from Janet Woodcock, M.D., Dir., Ctr. for Drug Eval. & Res., to Andrew Kolodny, M.D., Pres. Physicians for Responsible Opioid Prescribing, Re Docket No. FDA-2012-P-0818 (Sept. 10, 2013).

[98] Andrea D. Furlan et al., *Opioids for chronic noncancer pain: a meta-analysis of effectiveness and side effects*, 174(11) Can. Med. Ass'n J. 1589-1594 (2006). This study revealed that efficacy studies do not typically include data on opioid addiction, such that, if anything, the data overstate effectiveness.

[99] Moreover, users of opioids had the highest increase in the number of headache days per month, scored significantly higher on the Migraine Disability Assessment (MIDAS), and had higher rates of depression, compared to non-opioid users. They also were more likely to experience sleepiness, confusion, and rebound headaches, and reported a lower quality of life than patients taking other medications.

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improvement claims as "aspirational." Another acknowledged in 2012 that "[s]ignificant investment in clinical data [was] needed" to establish opioids' effect on mitigating quality of life issues, like social isolation.

312. The long-term use of opioids carries a host of serious side effects, including addiction, mental clouding and confusion, sleepiness, hyperalgesia, and immune-system and hormonal dysfunction that degrade, rather than improve, patients' ability to function. Defendants often omitted these adverse effects as well as certain risks of drug interactions from their publications.

313. Yet each of the following statements by Defendants, suggests that the long-term use of opioids improve patients' function and quality of life, and that scientific evidence supports this claim.

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| **Actavis** | a. Documents from a 2010 sales training indicate that Actavis trained its sales force to instruct prescribers that "**most** chronic benign pain patients do have **markedly improved ability to function** when maintained on chronic opioid therapy." (Emphasis added.)<br><br>b. Documents from a 2010 sales training indicate that Actavis trained its sales force that increasing and restoring function is an expected outcome of chronic Kadian therapy, including physical, social, vocational, and recreational function.<br><br>c. Actavis distributed a product advertisement that claimed that use of Kadian to treat chronic pain would allow patients to return to work, relieve "stress on your body and your mental health," and cause patients to enjoy their lives. The FDA warned Actavis that such claims were misleading, writing: "We are not aware of substantial evidence or substantial clinical experience demonstrating that the magnitude of the effect of the drug has in alleviating pain, taken together with any drug-related side effects patients may experience . . . results in any overall positive impact on a patient's work, physical and mental functioning, daily activities, or enjoyment of life."[100]<br><br>d. Actavis sales representatives told Maverick County prescribers that prescribing Actavis's opioids would improve their patients' ability to function and improve their quality of life. |

[100] Warning Letter from Thomas Abrams, Dir., FDA Div. of Mktg., Adver., & Commc'ns, to Doug Boothe, CEO, Actavis Elizabeth LLC (Feb. 18. 2010), *available at* http://www.fda.gov/Drugs/GuidanceComplianceRegulatoryInformation/EnforcementActivitiesbyFDA/WarningLettersandNoticeofViolationLetterstoPharmaceuticalCompanies/ucm259240.htm.

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| **Cephalon** | e. Cephalon sponsored the FSMB's Responsible Opioid Prescribing (2007), which taught that relief of pain itself improved patients' function. *Responsible Opioid Prescribing* explicitly describes functional improvement as the goal of a "long-term therapeutic treatment course." Cephalon also spent $150,000 to purchase copies of the book in bulk and distributed the book through its pain sales force to 10,000 prescribers and 5,000 pharmacists.<br><br>f. Cephalon sponsored the American Pain Foundation's *Treatment Options: A Guide for People Living with Pain* (2007), which taught patients that opioids, when used properly "give [pain patients] a quality of life we deserve." The *Treatment Options* guide notes that non-steroidal anti-inflammatory drugs have greater risks associated with prolonged duration of use, but there was no similar warning for opioids. APF distributed 17,200 copies in one year alone, according to its 2007 annual report. The publication is also currently available online.<br><br>g. Cephalon sponsored a CME written by key opinion leader Dr. Lynn Webster, titled *Optimizing Opioid Treatment for Breakthrough Pain*, which was offered online by Medscape, LLC from September 28, 2007, to December 15, 2008. The CME taught that Cephalon's Actiq and Fentora improve patients' quality of life and allow for more activities when taken in conjunction with long- acting opioids.<br><br>h. Cephalon sales representatives told Maverick County prescribers that opioids would increase patients' ability to function and improve their quality of life. |

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| **Endo** | i. Endo sponsored a website, painknowledge.com, through APF and NIPC, which, in 2009, claimed that with opioids, "your level of function should improve; you may find you are now able to participate in activities of daily living, such as work and hobbies, that you were not able to enjoy when your pain was worse." Endo continued to provide funding for this website through 2012, and closely tracked unique visitors to it.<br><br>j. A CME sponsored by Endo, titled *Persistent Pain in the Older Patient*, taught that chronic opioid therapy has been "shown to reduce pain and improve depressive symptoms and cognitive functioning."<br><br>k. Endo distributed handouts to prescribers that claimed that use of Opana ER to treat chronic pain would allow patients to perform work as a chef. This flyer also emphasized Opana ER's indication without including equally prominent disclosure of the "moderate to severe pain" qualification.[101]<br><br>l. Endo's sales force distributed FSMB's *Responsible Opioid Prescribing* (2007), which taught that relief of pain itself improved patients' function. *Responsible Opioid Prescribing* explicitly describes functional improvement as the goal of a "long-term therapeutic treatment course."<br><br>m. Endo provided grants to APF to distribute *Exit Wounds* to veterans, which taught that opioid medications "*increase* your level of functioning" (emphasis in the original). *Exit Wounds* also omits warnings of the risk of interactions between opioids and benzodiazepines, which would increase fatality risk. Benzodiazepines are frequently prescribed to veterans diagnosed with post-traumatic stress disorder.<br><br>n. Endo sales representatives told Maverick County prescribers that opioids would increase patients' ability to function and improve their quality of life by helping them become more physically active and return to work. |

[101] FDA regulations require that warnings or limitations be given equal prominence in disclosure, and failure to do so constitutes "misbranding" of the product. 21 C.F.R. § 202.1(e)(3); *see also* 21 U.S.C. §331(a).

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| **Janssen** | o. Janssen sponsored a patient education guide titled *Finding Relief: Pain Management for Older Adults* (2009), which its personnel reviewed and approved, and its sales force distributed. This guide features a man playing golf on the cover and lists examples of expected functional improvement from opioids, like sleeping through the night, returning to work, recreation, sex, walking, and climbing stairs. The guide states as a "fact" that "opioids may make it *easier* for people to live normally" (emphasis in the original). The myth/fact structure implies authoritative backing for the claims that does not exist. The targeting of older adults also ignored heightened opioid risks in this population.<br><br>p. Janssen sponsored, developed, and approved content of a website, *Let's Talk Pain* in 2009, acting in conjunction with the APF, AAPM, and ASPMN, whose participation in *Let's Talk Pain* Janssen financed and orchestrated. This website featured an interview, which was edited by Janssen personnel, claiming that opioids were what allowed a patient to "continue to function," inaccurately implying her experience would be representative.<br><br>q. Janssen provided grants to APF to distribute *Exit Wounds* to veterans, which taught that opioid medications "*increase* your level of functioning" (emphasis in the original). *Exit Wounds* also omits warnings of the risk of interactions between opioids and benzodiazepines, which would increase fatality risk. Benzodiazepines are frequently prescribed to veterans diagnosed with post-traumatic stress disorder.<br><br>r. Janssen sales representatives told Maverick County prescribers that opioids would<br>increase patients' ability to function and improve their quality of life by helping them become more physically active and return to work. |

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| **Purdue** | s. Purdue ran a series of advertisements for OxyContin in 2012 in medical journals titled "Pain vignettes," which were case studies featuring patients, each with pain conditions persisting over several months, recommending OxyContin for each. One such patient, "Paul," is described as a "54-year- old writer with osteoarthritis of the hands," and the vignettes imply that an OxyContin prescription will help him work more effectively.<br><br>t. Purdue sponsored APF's *A Policymaker's Guide to Understanding Pain & Its Management*, which inaccurately claimed that "multiple clinical studies" had shown that opioids are effective in improving daily function, psychological health, and health-related quality of life for chronic pain patients." The sole reference for the functional improvement claim noted the absence of long-term studies and actually stated: "For functional outcomes, the other analgesics were significantly more effective than were opioids." The *Policymaker's Guide* is still available online.<br><br>u. Purdue sponsored APF's *Treatment Options: A Guide for People Living with Pain* (2007), which counseled patients that opioids, when used properly, "give [pain patients] a quality of life we deserve." APF distributed 17,200 copies in one year alone, according to its 2007 annual report. The guide is currently available online.<br><br>v. Purdue sponsored APF's *Exit Wounds* (2009), which taught veterans that opioid medications "increase your level of functioning." *Exit Wounds* also omits warnings of the risk of interactions between opioids and benzodiazepines, which would increase fatality risk. Benzodiazepines are frequently prescribed to veterans diagnosed with post-traumatic stress disorder.<br><br>w. Purdue sponsored the FSMB's *Responsible Opioid Prescribing* (2007), which taught that relief of pain itself improved patients' function. *Responsible Opioid Prescribing* explicitly describes functional improvement as the goal of a "long-term therapeutic treatment course." Purdue also spent over $100,000 to support distribution of the book.<br><br>x. Purdue sales representatives told Maverick County prescribers that opioids would increase patients' ability to function and improve their quality of life. |

**2. Defendants and Their Third-Party Allies Concealed the Truth About the Risk of Addiction from Long-Term Opioid Use**

314. The fraudulent representation that opioids are rarely addictive is central to Defendants' scheme. To reach chronic pain patients Defendants, and the Front Groups and KOLs that they directed, assisted, and collaborated with, had to overcome doctors' legitimate fears that opioids would addict their patients. The risk of addiction is an extremely weighty risk—condemning patients to,

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among other things, dependence, compulsive use, haziness, a lifetime of battling relapse, and a dramatically heightened risk of serious injury or death. But for Defendants' campaign to convince doctors otherwise, finding benefits from opioid use for common chronic pain conditions sufficient to justify that risk would have, and previously had, posed a nearly insurmountable challenge.

315. Through their well-funded, comprehensive marketing efforts, Defendants and their KOLs and Front Groups were able to change prescriber perceptions despite the well-settled historical understanding and clear evidence that opioids taken long-term are often addictive. Defendants and their third-party partners: (a) brazenly maintained that the risk of addiction for patients who take opioids long-term was low; and (b) omitted the risk of addiction and abuse from the list of adverse outcomes associated with chronic opioid use, even though the frequency and magnitude of the risk—and Defendants' own labels—compelled disclosure.

316. Further, in addition to falsely claiming opioids had low addiction risk or omitting disclosure of the risk of addiction altogether, Defendants employed language that conveyed to prescribers that the drugs had lower potential for abuse and addiction. Further, in addition to making outright misrepresentations about the risk of addiction, or failing to disclose that serious risk at all, Defendants used code words that conveyed to prescribers that their opioid was less prone to abuse and addiction. For instance, sales representatives for Actavis, Endo, Janssen, and Purdue promoted their drugs as having "steady-state" properties with the intent and expectation that prescribers would understand this to mean that their drugs caused less of a rush or a feeling of euphoria, which can trigger abuse and addiction. Further, Endo actively promoted its reformulated Opana ER on the basis that it was "designed to be crush-resistant," suggesting both (a) that Endo had succeeded in making the drug harder to adulterate, and (b) that it was less addictive, in consequence. In fact, however, Endo knew that "the clinical significance of INTAC Technology or its impact on abuse/misuse has not been established for Opana ER" and that Opana ER could still be ground and cut into small pieces by those

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looking to abuse the drug. In the same vein, Janssen denied that Nucynta ER was an opioid and claimed that it was not addictive, and Purdue claimed that its opioids were not favored by addicts and did not produce a buzz, all of which falsely suggested that its opioids were less likely to be abused or addictive.

317. Each of the following was created with the expectation that, by instructing patients and prescribers that addiction rates are low and that addiction is unlikely when opioids are prescribed for pain, doctors would prescribe opioids to more patients. For example, one publication sponsored exclusively by Purdue—APF's 2011 *A Policymaker's Guide to Understanding Pain & Its Management*—claimed that opioids are not prescribed often enough because of "misconceptions about opioid addiction."[102]

318. Acting directly or with and through third parties, each of the Defendants claimed that the potential for addiction from its drugs was relatively small, or non-existent, even though there was no scientific evidence to support those claims, and the available research contradicted them. A recent literature survey found that while ranges of "problematic use" of opioids ranged from <1% to 81%,[103] abuse averages between 21% and 29% and addiction between 8% and 12%.[104] These estimates are well in line with Purdue's own studies, showing that between 8% and 13% of OxyContin patients became addicted, but on which Purdue chose not to rely, instead citing the Porter-Jick letter.

319. The FDA has found that 20% of opioid patients use two or more pharmacies, 26% obtain opioids from two or more prescribers, and 16.5% seek early refills—all potential "red flags" for

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[102] http://s3.documentcloud.org/documents/277603/apf-policymakers-guide.pdf (accessed May 30, 2017)

[103] Cited for the low end of that range was the 1980 Porter-Jick letter in the *New England Journal of Medicine*.

[104] Kevin Vowels et al., *Rates of opioid misuse, abuse, and addiction in chronic pain: a systematic review and data synthesis*, 156 PAIN 569-76 (April 2015).

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abuse or addiction.[105] The FDA in fact has ordered manufacturers of long-acting opioids to "[c]onduct one or more studies to provide quantitative estimates of the serious risks of misuse, abuse, addiction, overdose and death associated with long-term use of opioid analgesics for management of chronic pain," in recognition of the fact that it found "high rates of addiction" in the medical literature.[106]

320. Of course, the significant (and growing) incidence of abuse, misuse, and addiction to opioids is also powerful evidence that Defendants' statements regarding the low risk of addiction were, and are, untrue. This was well-known to Defendants who had access to sales data and reports, adverse event reports, federal abuse and addiction-related surveillance data, and other sources that demonstrated the widening epidemic of opioid abuse and addiction.

321. Acting directly or through and with third parties, each of the Defendants claimed that the potential for addiction even from long-term use of its drugs was relatively small, or non- existent, despite the fact that the contention was false and there was no scientific evidence to support it. Examples of these misrepresentations are laid out below:

[105] Len Paulozzi, M.D., "Abuse of Marketed Analgesics and Its Contribution to the National Problem of Drug Abuse," *available at* http://www.fda.gov/downloads/AdvisoryCommittees/CommitteesMeetingMaterials/Drugs/AnestheticAndLifeSupportDrugsAdvisoryCommittee/UCM233244.pdf (accessed May 30, 2017)

[106] September 10, 2013 letter from Bob Rappaport, M.D., to NDA applicants of ER/LA opioid analgesics*, available at* http://www.fda.gov/downloads/Drugs/DrugSafety/InformationbyDrugClass/UCM367697.pdf (accessed May 30, 2017).; Letter from Janet Woodcock, M.D., Dir., Ctr. for Drug Eval. & Res., to Andrew Kolodny, M.D., Pres. Physicians for Responsible Opioid Prescribing, Re Docket No. FDA-2012-P-0818 (Sept. 10, 2013).

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| **Actavis** | a. Documents from a 2010 sales training indicate that Actavis trained its sales force that long-acting opioids were less likely to produce addiction than short-acting opioids, although there is no evidence that either form of opioid is less addictive or that any opioids can be taken long-term without the risk of addiction.<br><br>b. Actavis had a patient education brochure distributed in 2007 that claimed addiction is possible, but it is "less likely if you have never had an addiction problem." Although the term "less likely" is not defined, the overall presentation suggests the risk is so low as not to be a worry.<br><br>c. Kadian sales representatives told Maverick County prescribers that Kadian was "steady state" and had extended release mechanisms, the implication of which was that it did not produce a rush or euphoric effect, and therefore was less addictive and less likely to be abused.<br><br>d. Kadian sales representatives told Maverick County prescribers that the contents of Kadian could not be dissolved in water if the capsule was opened, implying that Kadian was less likely to be abused—and thereby less addictive—than other opioids.<br><br>e. Kadian sales representatives omitted any discussion of addiction risks related to Actavis's drugs to County prescribers. |

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| **Cephalon** | f. Cephalon sponsored and facilitated the development of a guidebook, *Opioid Medications and REMS: A Patient's Guide*, which claims, among other things, that "patients without a history of abuse or a family history of abuse do not commonly become addicted to opioids."<br><br>g. Cephalon sponsored APF's *Treatment Options: A Guide for People Living with Pain* (2007), which taught that addiction is rare and limited to extreme cases of unauthorized dose escalations, obtaining opioids from multiple sources, or theft.<br><br>h. Cephalon sales representatives omitted any discussion of addiction risks related to Cephalon's drugs to County prescribers. |

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| **Endo** | i. Endo trained its sales force in 2012 that use of long-acting opioids resulted in increased patient compliance, without any supporting evidence.<br><br>j. Endo's advertisements for the 2012 reformulation of Opana ER claimed it was *designed to be crush resistant*, in a way that conveyed that it was less likely to be abused. This claim was false; the FDA warned in a May 10, 2013 letter that there was no evidence Endo's design "would provide a reduction in oral, intranasal or intravenous abuse" and Endo's "post-marketing data submitted are insufficient to support any conclusion about the overall or route-specific rates of abuse." Further, Endo instructed its sales representatives to repeat this claim about "design," with the intention of conveying Opana ER was less subject to abuse.<br><br>k. Endo sponsored a website, painknowledge.com, through APF and NIPC, which, in 2009, claimed that: "[p]eople who take opioids as prescribed usually do not become addicted." Although the term "usually" is not defined, the overall presentation suggests the risk is so low as not to be a concern. The language also implies that, as long as a prescription is given, opioid use will not become problematic. Endo continued to provide funding for this website through 2012, and closely tracked unique visitors to it.<br><br>l. Endo sponsored a website, PainAction.com, which stated "Did you know? Most chronic pain patients do not become addicted to the opioid medications that are prescribed for them."<br><br>m. Endo sponsored CMEs published by APF's NIPC, of which Endo was the sole funder, titled *Persistent Pain in the Older Adult* and *Persistent Pain in the Older Patient*. These CMEs claimed that opioids used by elderly patients present "possibly less potential for abuse than in younger patients[,]" which lacks evidentiary support and deceptively minimizes the risk of addiction for elderly patients.<br><br>n. Endo distributed an education pamphlet with the Endo logo titled *Living with Someone with Chronic Pain*, which inaccurately minimized the risk of addiction: "Most health care providers who treat people with pain agree that most people do not develop an addiction problem."<br><br>o. Endo distributed a patient education pamphlet edited by key opinion leader Dr. Russell Portenoy titled *Understanding Your Pain: Taking Oral Opioid Analgesics*. It claimed that "[a]ddicts take opioids for other reasons [than pain relief], such as unbearable emotional problems." This implies that pain patients prescribed opioids will not become addicted, which is unsupported and untrue. |

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| | p. Endo contracted with AGS to produce a CME promoting the 2009 guidelines for the Pharmacological Management of Persistent Pain in Older Persons. These guidelines falsely claim that "the risks [of addiction] are exceedingly low in older patients with no current or past history of substance abuse." None of the references in the guidelines corroborates the claim that elderly patients are less likely to become addicted to opioids, and there is no such evidence. Endo was aware of the AGS guidelines' content when it agreed to provide this funding, and AGS drafted the guidelines with the expectation it would seek drug company funding to promote them after their completion.<br><br>q. Endo sales representatives told Maverick County prescribers that its drugs were "steady state," the implications of which was that they did not produce a rush or euphoric effect, and therefore were less addictive and less likely to be abused.<br><br>r. Endo provided grants to APF to distribute Exit Wounds (2009) to veterans, which taught that "[l]ong experience with opioids shows that people who are not predisposed to addiction are very unlikely to become addicted to opioid pain medications." Although the term "very unlikely" is not defined, the overall presentation suggests that the risk is so low as not to be a concern.<br><br>s. Endo sales representatives omitted discussion of addiction risks related to Endo's drugs. |

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| **Janssen** | t. Janssen sponsored a patient education guide titled *Finding Relief: Pain Management for Older Adults* (2009), which its personnel reviewed and approved and which its sales force distributed. This guide described a "myth" that opioids are addictive, and asserts as fact that "[m]any studies show that opioids are *rarely* addictive when used properly for the management of chronic pain." Although the term "rarely" is not defined, the overall presentation suggests the risk is so low as not to be a concern. The language also implies that as long as a prescription is given, opioid use is not a problem.<br><br>u. Janssen contracted with AGS to produce a CME promoting the 2009 guidelines for the *Pharmacological Management of Persistent Pain in Older Persons*. These guidelines falsely claim that "the risks [of addiction] are exceedingly low in older patients with no current or past history of substance abuse." The study supporting this assertion does not analyze addiction rates by age and, as already noted, addiction remains a significant risk for elderly patients. Janssen was aware of the AGS guidelines' content when it agreed to provide this funding, and AGS drafted the guidelines with the expectation it would seek drug company funding to promote them after their completion. |

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| | v. Janssen provided grants to APF to distribute *Exit Wounds* (2009) to veterans, which taught that [l]ong experience with opioids shows that people who are not predisposed to addiction are very unlikely to become addicted to opioid pain medications." Although the term "very unlikely" is not defined, the overall presentation suggests the risk is so low as not to be a concern.<br><br>w. Janssen currently runs a website, *Prescriberesponsibly.com* (last modified July 2, 2015), which claims that concerns about opioid addiction are "overstated."<br><br>x. A June 2009 Nucynta Training module warns Janssen's sales force that physicians are reluctant to prescribe controlled substances like Nucynta, but this reluctance is unfounded because "the risks . . . are much smaller than commonly believed."<br><br>y. Janssen sales representatives told Maverick County prescribers that its drugs were "steady state," the implication of which was that they did not produce a rush or euphoric effect, and therefore were less addictive and less likely to be abused.<br><br>z. Janssen sales representatives told Maverick County prescribers that Nucynta and Nucynta ER were "not opioids," implying that the risks of addiction and other adverse outcomes associated with opioids were not applicable to Janssen's drugs. In truth, however, as set out in Nucynta's FDA-mandated label, Nucynta "contains tapentadol, an opioid agonist and Schedule II substance with abuse liability similar to other opioid agonists, legal or illicit."<br><br>aa. Janssen sales representatives falsely told prescribers that Duragesic had anti abuse properties when it had none.<br><br>bb. Janssen's sales representatives told Maverick County prescribers that Nucynta's unique properties eliminated the risk of addiction associated with the drug.<br><br>cc. Janssen sales representatives omitted discussion of addiction risks related to Janssen's drugs. |

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| **Purdue** | dd. Purdue published a prescriber and law enforcement education pamphlet in 2011 entitled *Providing Relief, Preventing Abuse*, which under the heading, "Indications of Possible Drug Abuse," shows pictures of the stigmata of injecting or snorting opioids—skin popping, track marks, and perforated nasal septa. In fact, opioid addicts who resort to these extremes are uncommon; the far more typical reality is patients who become dependent and addicted through oral use.[107] Thus, these misrepresentations wrongly reassure doctors that, as long as they do not observe those signs, they need not be concerned that their patients are abusing or addicted to opioids.<br><br>ee. Purdue sponsored APF's *A Policymaker's Guide to Understanding Pain & Its Management*, which inaccurately claimed that less than 1% of children prescribed opioids will become addicted. This publication is still available online. This publication also asserted that pain is undertreated due to "misconceptions about opioid addiction."<br><br>ff. Purdue sponsored APF's *Treatment Options: A Guide for People Living with Pain* (2007), which asserted that addiction is rare and limited to extreme cases of unauthorized dose escalations, obtaining opioids from multiple sources, or theft.<br><br>gg. A Purdue-funded study with a Purdue co-author claimed that "evidence that the risk of psychological dependence or addiction is low in the absence of a history of substance abuse."[108] The study relied only on the Porter-Jick letter to the editor concerning a chart review of hospitalized patients, not patients taking Purdue's long-acting, take-home opioid. Although the term "low" is not defined, the overall presentation suggests the risk is so low as not to be a concern.<br><br>hh. Purdue contracted with AGS to produce a CME promoting the 2009 guidelines for the *Pharmacological Management of Persistent Pain in Older Persons*. These guidelines falsely claim that "the risks [of addiction] are exceedingly low in older patients with no current or past history of substance abuse." None of the references in the guidelines corroborates the claim that elderly patients are less likely to become addicted to opioids and the claim is, in fact, untrue. Purdue was aware of the AGS guidelines' content when it agreed to provide this funding, and AGS drafted the guidelines with the expectation it would seek drug company funding to promote them after their completion. |

[107] Purdue itself submitted briefing materials in October 2010 to a meeting of the FDA's Joint Meeting of the Anesthetic and Life Support Drugs Advisory Committee and the Drug Safety and Risk Management Advisory Committee in which it stated that OxyContin was used non-medically by injection 4-17% of the time.

[108] C. Peter N. Watson et al., *Controlled-release oxycodone relieves neuropathic pain: a randomized controlled trial I painful diabetic neuropathy*, 105 Pain 71 (2003).

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| | ii. Purdue sponsored APF's *Exit Wounds* (2009), which counseled veterans that "[l]ong experience with opioids shows that people who are not predisposed to addiction are very unlikely to become addicted to opioid pain medications." Although the term "very unlikely" is not defined, the overall presentation suggests it is so low as not to be a worry.<br><br>jj. Purdue sales representatives told Maverick County prescribers that its drugs were "steady state," the implication of which was that they did not produce a rush or euphoric effect, and therefore were less addictive and less likely to be abused.<br><br>kk. Purdue sales representatives told Maverick County prescribers that Butrans has a lower abuse potential than other drugs because it was essentially tamper-proof and, after a certain point, patients no longer experience a "buzz" from increased dosage.<br><br>ll. Advertisements that Purdue sent to Maverick County prescribers stated that OxyContin ER was less likely to be favored by addicts, and, therefore, less likely to be abused or diverted, or result in addiction.<br><br>mm. Purdue sales representatives omitted discussion of addiction risk related to Purdue's drugs. |

322. In addition to denying or minimizing the risk of addiction and abuse generally, Defendants also falsely claimed that their particular drugs were safer, less addictive, and less likely to be abused or diverted than their competitors' or predecessor drugs. In making these claims, Defendants said or implied that because their drug had a "steady-state" and did not produce peaks and valleys, which cause drug-seeking behavior—either to obtain the high or avoid the low—it was less likely to be abused or addicting. Endo also asserted in particular that, because a reformulation of Opana ER was (or was designed to be) abuse-deterrent or tamper-resistant, patients were less likely to become addicted to it. Defendants had no evidence to support any of these claims, which, by FDA regulation, must be based on head-to-head trials;[109] the claims also were false and misleading in that they misrepresented the risks of both the particular drug and opioids as a class.

[109] *See Guidance for Industry*, "Abuse-Deterrent Opioids—Evaluation and Labeling," April 2015 (describing requirements for premarket and postmarket studies).

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323. Further, rather than honestly disclose the risk of addiction, Defendants, and the third parties they directed and assisted and whose materials they distributed, attempted to portray those who were concerned about addiction as unfairly denying treatment to needy patients. To increase pressure on doctors to prescribe chronic opioid therapy, Defendants turned the tables; it was doctors who fail to treat their patients' chronic pains with opioids—not doctors who cause their patients to become addicted to opioids—who are failing their patients (and subject to discipline). Defendants and their third-party allies claimed that purportedly overblown worries about addiction cause pain to be under-treated and opioids to be over-regulated and under-prescribed. This mantra of under-treated pain and under-used drugs reinforced Defendants' messages that the risks of addiction and abuse were not significant and were overblown.

324. For example, Janssen's website, *Let's Talk Pain*, warns in a video posted online that "strict regulatory control has made many physicians reluctant to prescribe opioids. The unfortunate casualty in all of this is the patient, who is often undertreated and forced to suffer in silence." The program goes on to say: "Because of the potential for abusive and/or addictive behavior, many healthcare professionals have been reluctant to prescribe opioids for their patients . . . . This prescribing environment is one of many barriers that may contribute to the undertreatment of pain, a serious problem in the United States."

325. In the same vein, a Purdue website called *In the Face of Pain* complains, under the heading of "Protecting Access," that, through at least mid-2013, policy governing the prescribing of opioids was "at odds with" best medical practices by "unduly restricting the amounts that can be prescribed and dispensed"; "restricting access to patients with pain who also have a history of substance abuse"; and "requiring special government-issued prescription forms only for the medications that are capable of relieving pain that is severe." This unsupported and untrue rhetoric

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aims to portray doctors who do not prescribe opioids as uncaring, converting their desire to relieve patients' suffering into a mandate to prescribe opioids.

### 3. Defendants and Their Third-Party Allies Misrepresented that Addiction Risk Can Be Avoided or Managed

326. To this day, defendants each continue to maintain that most patients can safely take opioids long-term for chronic pain without becoming addicted. Presumably only to explain why doctors encounter so many patients addicted to opioids, Defendants and their third-party allies have come to admit that some patients could become addicted, but that doctors can avoid or manage that risk by using screening tools or questionnaires. These tools, they say, identify those with higher addiction risks (stemming from personal or family histories of substance abuse, mental illness, or abuse) so that doctors can more closely monitor patients at greater risk of addiction.

327. There are three fundamental flaws in these assurances that doctors can identify and manage the risk of addiction. First, there is no reliable scientific evidence that screening works to accurately predict risk or reduce rates of addiction. Second, there is no reliable scientific evidence that high-risk or addicted patients can take opioids long-term without triggering addiction, even with enhanced monitoring and precautions. Third, there is no reliable scientific evidence that patients without these red flags are necessarily free of addiction risk.

328. Addiction is difficult to predict on a patient-by-patient basis, and there are no reliable, validated tools to do so. A recent Evidence Report by the Agency for Healthcare Research and Quality ("AHRQ"), which "systematically review[ed] the current evidence on long-term opioid therapy for chronic pain" identified "[n]o study" that had "evaluated the effectiveness of risk mitigation strategies, such as use of risk assessment instruments, opioid management plans, patient education, urine drug screening, prescription drug monitoring program data, monitoring instruments, more frequent monitoring intervals, pill counts, or abuse- deterrent formulations on outcomes related to overdose,

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addiction, abuse or misuse."[110] Furthermore, attempts to treat high-risk patients, such as those who have a documented predisposition to substance abuse, by resorting to patient contracts, more frequent refills, or urine drug screening are not proven to work in the real world, if busy doctors even in fact attempt them.

329. Most disturbingly, despite the widespread use of screening tools, patients with past substance use disorders—which every tool rates as a risk factor—receive, on average, higher doses of opioids.

330. Each Defendant claimed that the risk of addiction could be avoided or managed, claims that are deceptive and without scientific support:

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| **Actavis** | a. Documents from a 2010 sales training indicate that Actavis trained its sales force that prescribers can use risk screening tools to limit the development of addiction. |

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| **Cephalon** | b. Cephalon sponsored APF's *Treatment Options: A Guide for People Living with Pain* (2007), which taught patients that "opioid agreements" between doctors and patients can "ensure that you take the opioid as prescribed." |

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| **Endo** | c. Endo paid for a 2007 supplement[111] available for continuing education credit in the Journal of Family Practice. This publication, titled Pain Management Dilemmas in Primary Care: Use of Opioids, recommended screening patients using tools like the Opioid Risk Tool or the Screener and Opioid Assessment for Patients with Pain, and advised that patients at high risk of addiction could safely (e.g., without becoming addicted) receive chronic opioid therapy using a "maximally structured approach" involving toxicology screens and pill counts. |

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| **Purdue** | d. Purdue's unbranded website, In the Face of Pain (inthefaceofpain.com) states that policies that "restrict[] access to patients with pain who also have a history of substance abuse" and "requiring special government-issued prescription forms for the only medications that are capable of relieving pain that is severe" are "at odds with" best medical practices.[112] |

[110] *The Effectiveness and Risks of Long-term Opioid Treatment of Chronic Pain*, Agency for Healthcare Res. & Quality (September 19, 2014).
[111] The Medical Journal, *The Lancet* found that all of the supplement papers it received failed peer-review. Editorial, "The Perils of Journal and Supplement Publishing," 375 *The Lancet* 9712 (347) 2010.
[112] *See In the Face of Pain Fact Sheet: Protecting Access to Pain Treatment*, Purdue Pharma L.P. (Resources verified Mar. 2012),

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| | e. Purdue sponsored a 2012 CME program taught by a KOL titled Chronic Pain Management and Opioid Use: Easing Fears, Managing Risks, and Improving Outcomes. This presentation recommended that use of screening tools, more frequent refills, and switching opioids could treat a high-risk patient showing signs of potentially addictive behavior.<br><br>f. Purdue sponsored a 2011 webinar taught by Dr. Lynn Webster, titled Managing Patient's Opioid Use: Balancing the Need and Risk. This publication taught prescribers that screening tools, urine tests, and patient agreements have the effect of preventing "overuse of prescriptions" and "overdose deaths."<br><br>g. Purdue sales representatives told Maverick County prescribers that screening tools can be used to select patients appropriate for opioid therapy and to manage the risks of addiction. |

**4. Defendants and Their Third-Party Allies Created Confusion By Promoting the Misleading Term "Pseudoaddiction."**

331. Defendants and their third-party allies developed and disseminated each of the following misrepresentations with the intent and expectation that, by instructing patients and prescribers that signs of addiction are actually the product of untreated pain, doctors would prescribe opioids to more patients and continue to prescribing them, and patients would continue to use opioids despite signs that the patient was addicted. The concept of "pseudoaddiction" was coined by Dr. David Haddox, who went to work for Purdue, and popularized by Dr. Russell Portenoy, who consulted for Cephalon, Endo, Janssen, and Purdue. Much of the same language appears in other Defendants' treatment of this issue, highlighting the contrast between "undertreated pain" and "true addiction," as if patients could not experience both. As KOL Dr. Lynn Webster wrote:

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www.inthefaceofpain.com/content/uploads/2011/12/factsheet_ProtectingAccess.pdf (accessed May 30, 2017).

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"[Pseudoaddiction] obviously became too much of an excuse to give patients more medication.. . . It led us down a path that caused harm. It is already something we are debunking as a concept."[113]

332. Each of the publications and statements below falsely states or suggests that the concept of "pseudoaddiction" is substantiated by scientific evidence and accurately describes the condition of patients who only need, and should be treated with, more opioids:

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| **Actavis** | a. Documents from a 2010 sales training indicate that Actavis trained its sales force to instruct physicians that aberrant behaviors like self-escalation of doses constituted "pseudoaddiction." |

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| **Cephalon** | b. Cephalon sponsored FSMB's *Responsible Opioid Prescribing* (2007), which taught that behaviors such as "requesting drugs by name," "demanding or manipulative behavior," seeing more than one doctor to obtain opioids, and hoarding are all signs of "pseudoaddiction." Cephalon also spent $150,000 to purchase copies of the book in bulk and distributed it through its pain sales force to 10,000 prescribers and 5,000 pharmacists. |

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| **Endo** | c. Endo distributed copies of a book by KOL Dr. Lynn Webster entitled *Avoiding Opioid Abuse While Managing* Pain (2007). Endo's internal planning documents describe the purpose of distributing this book as to "[i]ncrease the breadth and depth of the Opana ER prescriber base." The book claims that when faced with signs of aberrant behavior, the doctor should regard it as "pseudoaddiction" and thus, increasing the dose *in most cases . . . should be the clinician's first response*." (emphasis added).<br><br>d. Endo spent $246,620 to buy copies of FSMB's *Responsible Opioid Prescribing* (2007), which was distributed by Endo's sales force. This book asserted that behaviors such as "requesting drugs by name," "demanding or manipulative behavior," seeing more than one doctor to obtain opioids, and hoarding, are all signs of "pseudoaddiction." |

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| **Janssen** | e. From 2009 to 2011 Janssen's website, *Let's Talk Pain*, stated that "pseudoaddiction . . . refers to patient behaviors that may occur when pain is under-treated" and that "[p]seudoaddiction is different from true addiction because such behaviors can be resolved with effective pain management." (emphasis added). |

[113] John Fauber & Ellen Gabler, *Networking Fuels Painkiller Boom*, Milwaukee Wisc. J. Sentinel (Feb.19, 2012).

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| **Purdue** | f. Purdue published a prescriber and law enforcement education pamphlet in 2011 entitled *Providing Relief, Preventing Abuse*, which described "pseudoaddiction" as a concept that "emerged in the literature to describe the inaccurate interpretation of [drug-seeking behaviors] in patients who have pain that has not been effectively treated."<br><br>g. Purdue distributed to physicians, at least as of November 2006, and posted on its unbranded website, *Partners Against Pain*, a pamphlet copyrighted 2005 and titled *Clinical Issues in Opioid Prescribing*. This pamphlet included a list of conduct, including "illicit drug use and deception" it defined as indicative of "pseudoaddiction" or untreated pain. It also states: "Pseudoaddiction is a term which has been used to describe patient behaviors that may occur when *pain is undertreated*. . . . Even such behaviors as illicit drug use and deception can occur in the patient's efforts to obtain relief. Pseudoaddiction can be *distinguished from true addiction* in that the behaviors resolve when the pain is effectively treated." (Emphasis added.)<br><br>h. Purdue sponsored FSMB's *Responsible Opioid Prescribing* (2007), which taught that behaviors such as "requesting drugs by name, "demanding or manipulative behavior," seeing more than one doctor to obtain opioids, and hoarding, are all signs of "pseudoaddiction." Purdue also spent over $100,000 to support distribution of the book.<br><br>i. Purdue sponsored APF's *A Policymaker's Guide to Understanding Pain & Its Management*, which states: "Pseudo-addiction describes patient behaviors that may occur when *pain is undertreated*. . . . Pseudo-addiction can be distinguished from true addiction in that this behavior ceases when pain is effectively treated." (Emphasis added.) |

**5. Defendants and Their Third-Party Allies Claimed Withdrawal is Simply Managed**

333. Defendants and their third-party allies promoted the false and misleading messages below with the intent and expectation that, by misrepresenting the difficulty of withdrawing from opioids, prescribers and patients would be more likely to start chronic opioid therapy and would fail to recognize the actual risk of addiction.

334. In an effort to underplay the risk and impact of addiction, Defendants and their third-party allies frequently claim that, while patients become "physically" dependent on opioids, physical dependence can be addressed by gradually tapering patients' doses to avoid the adverse effects of withdrawal. They fail to disclose the extremely difficult and painful effects that patients can experience

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when they are removed from opioids—effects that also make it less likely that patients will be able to stop using the drugs.

335. In reality, withdrawal is prevalent in patients after more than a few weeks of therapy. Common symptoms of withdrawal include: severe anxiety, nausea, vomiting, headaches, agitation, insomnia, tremors, hallucinations, delirium, and pain. Some symptoms may persist for months, or even years, after a complete withdrawal from opioids, depending on how long the patient had been using opioids. Withdrawal symptoms trigger a feedback loop that drives patients to seek opioids, contributing to addiction.

336. Each of the publications and statements below falsely states or suggests that withdrawal from opioids was not a problem and they should not be hesitant about prescribing or using opioids:

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| **Actavis** | a. Documents from a 2010 sales training indicate that Actavis trained its sales force that discontinuing opioid therapy can be handled "simply" and that it can be done at home. Actavis's sales representative training claimed opioid withdrawal would take only a week, even in addicted patients. |

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| **Endo** | b. A CME sponsored by Endo, titled *Persistent Pain in the Older Adult*, taught that withdrawal symptoms can be avoided entirely by tapering the dose by 10-20% per day for ten days. |

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| **Janssen** | c. A Janssen PowerPoint presentation used for training its sales representatives titled "Selling Nucynta ER" indicates that the "low incidence of withdrawal symptoms" is a "core message" for its sales force. This message is repeated in numerous Janssen training materials between 2009 and 2011. The studies supporting this claim did not describe withdrawal symptoms in patients taking Nucynta ER beyond 90 days or at high doses and would therefore not be representative of withdrawal symptoms in the chronic pain population. Patients on opioid therapy long-term and at high doses will have a harder time discontinuing the drugs and are more likely to experience withdrawal symptoms. In addition, in claiming a low rate of withdrawal symptoms, Janssen relied upon a study that only began tracking withdrawal symptoms in patients two to four days after discontinuing opioid use; Janssen knew or should have known that these symptoms peak earlier than that for most patients. Relying on data after that initial window painted a misleading picture of the likelihood and severity of withdrawal associated with chronic opioid therapy. Janssen also knew or should have known that the patients involved in the study were not on the drug long enough to develop rates of withdrawal symptoms comparable to rates of withdrawal suffered by patients who use opioids for chronic pain—the use for which Janssen promoted Nucynta ER.<br><br>d. Janssen sales representatives told Maverick County prescribers that patients on Janssen's drugs were less susceptible to withdrawal than those on other opioids. |

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| **Purdue** | e. Purdue sponsored *APF's A Policymaker's Guide to Understanding Pain & Its Management*, which taught that "Symptoms of physical dependence can often be ameliorated by gradually decreasing the dose of medication during discontinuation," but did not disclose the significant hardships that often accompany cessation of use.<br><br>f. Purdue sales representatives told Maverick County prescribers that the effects of withdrawal from opioid use can be successfully managed.<br><br>g. Purdue sales representatives told Maverick County prescribers that the potential for withdrawal on Butrans was low due to Butrans's low potency and its extended release mechanism. |

### 6. Defendants and Their Third-Party Allies Misrepresented that Increased Doses Pose No Significant Additional Risks

337. Each of the following misrepresentations was created with the intent and expectation that, by misrepresenting and failing to disclose the known risks of high dose opioids, prescribers and patients would be more likely to continue to prescribe and use opioids, even when they were not

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effective in reducing patients' pain, and not to discontinue opioids even when tolerance required them to reach even higher doses.

338. Defendants and their third-party allies claimed that patients and prescribers could increase doses of opioids indefinitely without added risk, even when pain was not decreasing or when doses had reached levels that were "frighteningly high," suggesting that patients would eventually reach a stable, effective dose. Each of Defendants' claims also omitted warnings of increased adverse effects that occur at higher doses, and misleadingly suggested that there was no greater risk to higher dose opioid therapy.

339. These claims are false. Patients receiving high doses of opioids as part of long-term opioid therapy are three to nine times more likely to suffer an overdose from opioid-related causes than those on low doses. As compared to available alternative pain remedies, scholars have suggested that tolerance to the respiratory depressive effects of opioids develops at a slower rate than tolerance to analgesic effects. Accordingly, the practice of continuously escalating doses to match pain tolerance can, in fact, lead to overdose even where opioids are taken as recommended. The FDA has itself acknowledged that available data suggest a relationship between increased doses and the risk of adverse effects. Moreover, it is harder for patients to terminate use of higher-dose opioids without severe withdrawal effects, which contributes to a cycle of continued use, even when the drugs provide no pain relief and are causing harm—the signs of addiction.

340. Each of the following claims suggests that high-dose opioid therapy is safe:

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| **Actavis** | a. Documents from a 2010 sales training indicate that Actavis trained its sales force that "individualization" of opioid therapy depended on increasing doses "until patient reports adequate analgesia" and to "set dose levels on [the] basis of patient need, not on [a] predetermined maximal dose." Actavis further counseled its sales representatives that the reasons some physicians had for not increasing doses indefinitely were simply a matter of physician "comfort level," which could be overcome or used as a tool to induce them to switch to Actavis's opioid, Kadian. |

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| **Cephalon** | b. Cephalon sponsored APF's *Treatment Options: A Guide for People Living with Pain* (2007), which claimed that some patients "need" a larger dose of their opioid, regardless of the dose currently prescribed.<br><br>c. Cephalon sponsored a CME written by KOL Dr. Lynn Webster, *Optimizing Opioid Treatment for Breakthrough Pain*, which was offered online by Medscape, LLC from September 28, 2007 through December 15, 2008. The CME taught that non-opioid analgesics and combination opioids that include aspirin and acetaminophen are less effective to treat breakthrough pain because of dose limitations.<br><br>d. Cephalon sales representatives assured Maverick County prescribers that opioids were safe, even at high doses. |

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| **Endo** | e. Endo sponsored a website, painknowledge.com, through APF and NIPC, which, in 2009, claimed that opioids may be increased until "you are on the right dose of medication for your pain," and once that occurred, further dose increases would not occur. Endo funded the site, which was a part of Endo's marketing plan, and tracked visitors to it.<br><br>f. Endo distributed a patient education pamphlet edited by KOL Dr. Russell Portenoy titled *Understanding Your Pain: Taking Oral Opioid Analgesics*. In Q&A format, it asked: "If I take the opioid now, will it work later when I really need it?" The response was: "The dose can be increased . . . . You won't 'run out' of pain relief." |

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| **Janssen** | g. Janssen sponsored a patient education guide entitled *Finding Relief: Pain Management for Older Adults* (2009), which its personnel reviewed and approved an its sales force distributed. This guide listed dose limitations as "disadvantages" of other pain medicines and omitted any discussion of risks of increased doses of opioids. The publication also falsely claimed that it is a "myth" that "opioid doses have to be bigger over time." |

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| **Purdue** | h. Purdue's *In the Face of Pain* website, along with initiatives of APF, promoted the notion that if a patient's doctor does not prescribe them what—in their view—is a sufficient dose of opioids, they should find another doctor who will. In so doing, Purdue exerted undue, unfair, and improper influence over prescribers who face pressure to accede to the resulting demands.<br><br>i. Purdue sponsored APF's A Policymaker's Guide to Understanding Pain & Its Management, which taught that dose escalations are "sometimes necessary," even indefinitely high ones. This suggested that high dose opioids are safe and appropriate and did not disclose the risks from high dose opioids. This publication is still available online. |

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| | j. Purdue sponsored APF's Treatment Options: A Guide for People Living with Pain (2007), which taught patients that opioids have "no ceiling dose" and are therefore the most appropriate treatment for severe pain. The guide also claimed that some patients "need" a larger dose of the drug, regardless of the dose currently prescribed. This language fails to disclose heightened risks at elevated doses.<br><br>k. Purdue sponsored a CME issued by the American Medical Association in 2003, 2007, 2010, and 2013. The CME, Overview of Management Options, was edited by KOL Dr. Russell Portenoy, among others, and taught that other drugs, but not opioids, are unsafe at high doses. The 2013 version is still available for CME credit.<br><br>l. Purdue sales representatives told Maverick County prescribers that opioids were just as effective for treating patients long-term and omitted any discussion that increased tolerance would require increasing, and increasingly dangerous, doses. |

**7. Defendants and Their Third-Party Allies Deceptively Omitted or Minimized Adverse Effects of Opioids and Overstated the Risks of Alternative Forms of Pain Treatment.**

341. Each of the following misrepresentations was created with the intent and expectation that, by omitting the known, serious risks of chronic opioid therapy, including the risks of addiction, abuse, overdose, and death, and emphasizing or exaggerating risks of competing products, prescribers and patients would be more likely to choose opioids. Defendants and their third-party allies routinely ignored the risks of chronic opioid therapy. These include (beyond the risks associated with misuse, abuse, and addiction): hyperalgesia, a "known serious risk associated with chronic opioid analgesic therapy in which the patient becomes more sensitive to certain painful stimuli over time;"[114] hormonal dysfunction; decline in immune function; mental clouding, confusion, and dizziness; increased falls and fractures in the elderly; neonatal abstinence syndrome (when an infant exposed to opioids prenatally withdraws from the drugs after birth); and potentially fatal interactions with alcohol or

[114] Letter from Janet Woodcock, M.D., Dir., Ctr. for Drug Eval. & Res., to Andrew Kolodny, M.D., Pres. Physicians for Responsible Opioid Prescribing, Re Docket No. FDA-2012-P-0818 (Sept. 10, 2013).

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benzodiazepines, which are used to treat post-traumatic stress disorder and anxiety (disorders frequently coexisting with chronic pain conditions).[115]

342. Despite these serious risks, Defendants asserted, or implied, that opioids were appropriate first-line treatments and safer than alternative treatments, including NSAIDs such as ibuprofen (Advil, Motrin) or naproxen (Aleve). While NSAIDs can pose significant gastrointestinal, renal, and cardiac risks, particularly for elderly patients, Defendants' exaggerated descriptions of those risks were deceptive in themselves, and also made their omissions regarding the risks of opioids all the more striking and misleading. Defendants and their third-party allies described over-the-counter NSAIDs as life-threatening and falsely asserted that they were responsible for 10,000-20,000 deaths annually (more than opioids), when in reality the number is closer to 3,200. This description of NSAIDs starkly contrasted with their representation of opioids, for which the listed risks were nausea, constipation, and sleepiness (but not addiction, overdose, or death). Compared with NSAIDs, opioids are responsible for roughly four times as many fatalities annually.

343. As with the preceding misrepresentations, Defendants' false and misleading claims regarding the comparative risks of NSAIDs and opioids had the effect of shifting the balance of opioids' risks and purported benefits. While opioid prescriptions have exploded over the past two decades, the use of NSAIDs has declined during that same time.

344. Each of the following reflects Defendants' deceptive claims and omissions about the risks of opioids, including in comparison to NSAIDs:

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| **Actavis** | a. Documents from a 2010 sales training indicate that Actavis trained its sales force that the ability to escalate doses during long-term opioid therapy, without hitting a dose ceiling, made opioid use safer than other forms of therapy that |

[115] Several of these risks do appear in the FDA-mandated warnings. *See, e.g.,* the August 13, 2015 OxyContin Label, Section 6.2, identifying adverse reactions including: "abuse, addiction … death, … hyperalgesia, hypogonadism . . . mood altered . . . overdose, palpitations (in the context of withdrawal), seizures, suicidal attempt, suicidal ideation, syndrome of inappropriate antidiuretic hormone secretion, and urticaria [hives]."

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| | had defined maximum doses, such as acetaminophen or NSAIDs<br><br>b. Actavis also trained physician-speakers that "maintenance therapy with opioids can be safer than long-term use of other analgesics," including NSAIDs, for older persons.<br><br>c. Kadian sales representatives told Maverick County prescribers that NSAIDs were more toxic than opioids. |

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| **Cephalon** | d. Cephalon sponsored APF's *Treatment Options: A Guide for People Living with Pain* (2007), which taught patients that opioids differ from NSAIDs in that they have "no ceiling dose" and are therefore the most appropriate treatment for severe pain. The publication attributed 10,000 to 20,000 deaths annually to NSAID overdose. *Treatment Options* also warned that risks of NSAIDs increase if "taken for more than a period of months," with no corresponding warning about opioids.<br><br>e. Cephalon sales representatives told County prescribers that NSAIDs were more toxic than Cephalon's opioids |

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| **Endo** | f. Endo distributed a "case study" to prescribers titled *Case Challenges in Pain Management: Opioid Therapy for Chronic Pain*. The study cites an example, meant to be representative, of a patient "with a massive upper gastrointestinal bleed believed to be related to his protracted use of NSAIDs" (over eight years), and recommends treating with opioids instead.<br><br>g. Endo sponsored a website, painknowledge.com, through APF and NIPC, which contained a flyer called *"Pain: Opioid Therapy."* This publication included a list of adverse effects from opioids that omitted significant adverse effects like hyperalgesia, immune and hormone dysfunction, cognitive impairment, tolerance, dependence, addiction, and death. Endo continued to provide funding for this website through 2012, and closely tracked unique visitors to it.<br><br>h. Endo provided grants to APF to distribute *Exit Wounds* (2009), which omitted warnings of the risk of interactions between opioids and benzodiazepines, which would increase fatality risk. *Exit Wounds* also contained a lengthy discussion of the dangers of using alcohol to treat chronic pain but did not disclose dangers of mixing alcohol and opioids.<br><br>i. Endo sales representatives told Maverick County prescribers that NSAIDs were more toxic than opioids. |

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| **Janssen** | j. Janssen sponsored a patient education guide titled *Finding Relief: Pain Management for Older Adults* (2009), which its personnel reviewed and approved and its sales force distributed. This publication described the advantages and disadvantages of NSAIDs on one page, and the "myths/facts" of opioids on the facing page. The disadvantages of NSAIDs are described as involving "stomach upset or bleeding," "kidney or liver damage if taken at high doses or for a long time," "adverse reactions in people with asthma," and "can increase the risk of heart attack and stroke." The only adverse effects of opioids listed are "upset stomach or sleepiness," which the brochure claims will go away, and constipation.<br><br>k. Janssen sponsored APF's *Exit Wounds* (2009), which omits warnings of the risk of interactions between opioids and benzodiazepines. Janssen's label for Duragesic, however, states that use with benzodiazepines "may cause respiratory depression, [low blood pressure], and profound sedation or potentially result in coma. *Exit Wounds* also contained a lengthy discussion of the dangers of using alcohol to treat chronic pain but did not disclose dangers of mixing alcohol and opioids.<br><br>l. Janssen sales representatives told Maverick County prescribers that Nucynta was not an opioid, making it a good choice for chronic pain patients who previously were unable to continue opioid therapy due to excessive side effects. This statement was misleading because Nucynta is, in fact, an opioid and has the same effects as other opioids. |

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| **Purdue** | m. Purdue sponsored APF's *Exit Wounds* (2009), which omits warnings of the risk of interactions between opioids and benzodiazepines, which would increase fatality risk. *Exit Wounds* also contained a lengthy discussion of the dangers of using alcohol to treat chronic pain but did not disclose dangers of mixing alcohol and opioids.<br><br>n. Purdue sponsored APF's Treatment Options: A Guide for People Living with Pain (2007), which advised patients that opioids differ from NSAIDs in that they have "no ceiling dose" and are therefore the most appropriate treatment for severe pain. The publication attributes 10,000 to 20,000 deaths annually to NSAID overdose. Treatment Options also warned that risks of NSAIDs increase if "taken for more than a period of months," with no corresponding warning about opioids.<br><br>o. Purdue sponsored a CME issued by the American Medical Association in 2003, 2007, 2010, and 2013; The 2013 version is still available for CME credit. The CME, Overview of Management Options, was edited by KOL Dr. Russell Portenoy, among others, and taught that NSAIDs and other drugs, but not opioids, are unsafe at high doses.<br><br>p. Purdue sales representatives told Maverick County prescribers that NSAIDs were more toxic than opioids. |

### 8. Purdue Misleadingly Promoted OxyContin as Providing 12 Hours of Relief

345. In addition to making the deceptive statements above, Purdue also dangerously misled doctors and patients about OxyContin's duration and onset of action.

346. Purdue promotes OxyContin as an extended-release opioid, but the oxycodone does not enter the body on a linear rate. OxyContin works by releasing a greater proportion of oxycodone into the body upon administration, and the release gradually tapers, as illustrated in the following chart, which was, upon information and belief, adapted from Purdue's own sales materials:[116]

[116] Jim Edwards, "*How Purdue Used Misleading Charts to Hide OxyContin's Addictive Power*," *CBSNews.com*, Sept. 28, 2011, http://www.cbsnews.com/news/how-purdue-used-misleading-charts-to- hide-oxycontins-addictive-power/ *(*accessed May 30, 2017). The 160 mg dose is no longer marketed. Purdue's promotional materials in the past displayed a logarithmic scale, which gave the misleading impression the concentration remained constant.

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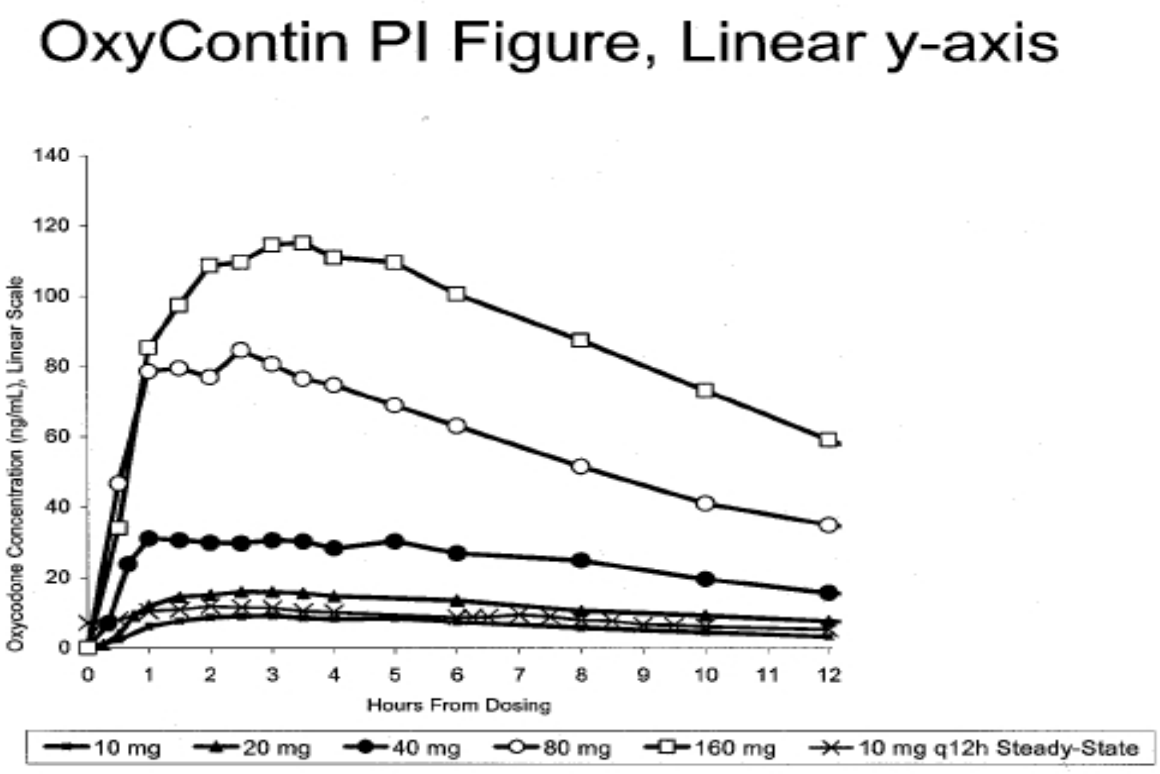



Figure 1

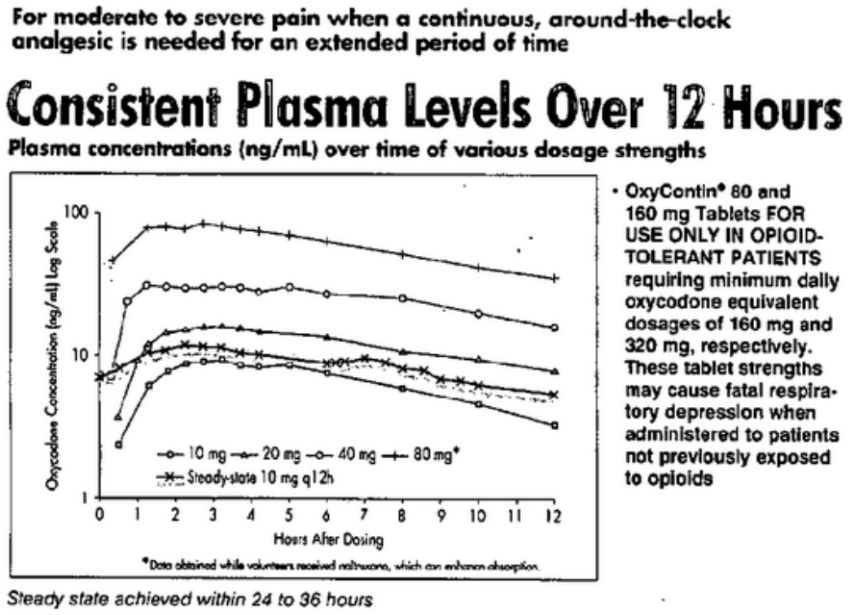
The reduced release of the drug over time means that the oxycodone no longer provides the same level of pain relief; as a result, in many patients, OxyContin does not last for the 12 hours for which Purdue promotes it—a fact that Purdue has known at all times relevant to this action.

347. OxyContin tablets provide an initial absorption of approximately 40% of the active medicine. This has a two-fold effect. First, the initial rush of nearly half of the powerful opioid—OxyContin is roughly twice as powerful as morphine—triggers a powerful psychological response. OxyContin thus behaves more like an immediate release opioid, which Purdue itself once claimed was more addicting in its original 1995 FDA-approved drug label. Second, the initial burst of oxycodone means that there is less of the drug at the end of the dosing period, which results in the drug not lasting for a full 12 hours and precipitates withdrawal symptoms in patients, a phenomenon known as "end of dose" failure. (The FDA found in 2008 that a "substantial number" of chronic pain patients

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will experience "end-of-dose failure" with OxyContin.) The combination of fast onset and end-of-dose failure makes OxyContin particularly addictive, even compared with other opioids.

348. Purdue nevertheless has falsely promoted OxyContin as if it were effective for a full 12 hours. Its advertising in 2000 included claims that OxyContin provides "Consistent Plasma Levels Over 12 Hours." That claim was accompanied by a chart depicting plasma levels on a logarithmic scale. The chart minimized the rate of end-of-dose failure by depicting 10 mg in a way that it appeared to be half of 100 mg in the table's y-axis. That chart, shown below, depicts the same information as the chart above, but does so in a way that makes the absorption rate appear more consistent:




349. More recently, other Purdue advertisements also emphasized "Q12h" (meaning twice-daily) dosing. These include an advertisement in the February 2005 *Journal of Pain* and 2006 *Clinical Journal of Pain* featuring an OxyContin logo with two pill cups, reinforcing the twice-a-day message. Other advertisements that ran in the 2005 and 2006 issues of the *Journal of Pain* depict a sample prescription for OxyContin, with "Q12h" handwritten for emphasis.

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350. The information that OxyContin did not provide pain relief for a full 12 hours was known to Purdue, and Purdue's competitors, but was not disclosed to general practitioners. Purdue's knowledge of some pain specialists' tendency to prescribe OxyContin three times per day instead of two (which would have compensated for end-of-dose failure) was set out in Purdue's internal documents as early as 1999 and is apparent from MEDWATCH Adverse Event reports for OxyContin.[117] Even Purdue's competitor, Endo, was aware of the problem; Endo attempted to position its Opana ER drug as offering "durable" pain relief, which Endo understood to suggest a contrast to OxyContin. Opana ER advisory board meetings featured pain specialists citing lack of 12-hour dosing as a disadvantage of OxyContin. Endo even ran advertisements for Opana ER referring to "real" 12-hour dosing.

351. Purdue's failure to disclose the prevalence of end-of-dose failure meant that prescribers in Maverick County were not informed of risks relating to addiction, and that they received the misleading message that OxyContin would be effective for treating chronic pain for the advertised duration. Furthermore, doctors would compensate by increasing the dose or prescribing "rescue" opioids, which had the same effect as increasing the amount of opioids prescribed to a patient.[118, 119]

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[117] MEDWATCH refers to the FDA's voluntary adverse event reporting system.

[118] Purdue's *Clinical Issues in Opioid Prescribing*, put out in 2005 under Purdue's unbranded *Partners Against Pain* banner, states that "it is recommended that a supplementary immediate-release medication be provided to treat exacerbations of pain that may occur with stable dosing." References to "rescue" medication appear in publications Purdue sponsored such as APF's *A Policymaker's Guide* (2011) and the 2013 CME *Overview of Pain Management Options*.

[119] The Connecticut Attorney General's office filed a citizens' petition with the FDA on January 27, 2004, requesting that the OxyContin label be amended with a warning not to prescribe the drug more than twice daily as a means of compensating for end-of-dose failure. The FDA denied this request on September 11, 2008. The FDA found that the state had failed to present sufficient evidence that more frequent dosing caused adverse outcomes, but the FDA did not challenge the Connecticut finding that end-of-dose failure of OxyContin was prevalent. Indeed, the FDA found that end-of-dose failure affected a "substantial" number of chronic pain patients prescribed OxyContin.

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**E. Each Defendant Engaged in Deceptive Marketing, Both Branded and Unbranded, that Targeted and Reached County Prescribers.**

352. Defendants—and the Front Groups and KOLs who depended on and worked alongside them—were able to affect a sea change in medical opinion in favor of accepting opioids as a medically necessary long-term treatment for chronic pain. As set forth below, each Defendant contributed to that result through a combination of both direct marketing efforts and third-party marketing efforts over which that Defendant exercised editorial control. These deceptive and misleading statements were directed to, and reached, County prescribers and patients, with the intent of distorting their views on the risks, benefits, and superiority of opioids for treatment of chronic pain.

353. Defendants engaged in their deceptive marketing campaign, both nationwide and in Maverick County, using a number of strategies. Defendants trained their sales forces and recruited physician speakers to deliver these deceptive messages and omissions, and they in turn conveyed them to prescribers. Defendants also broadly disseminated promotional messages and materials, both by delivering them personally to doctors during detailing visits and by mailing deceptive advertisements directly to prescribers. Because they are disseminated by Defendant drug manufacturers and relate to Defendants' drugs, these materials are considered "labeling" within the meaning of 21 C.F.R. § 1.3(a), which means Defendants are liable for their content.

354. As described below, the County has located a number of County-area prescribers who received Defendants' misrepresentations. Each of the misrepresentations received by these doctors constitutes an integral piece of a centrally directed marketing strategy to change medical perceptions regarding the use of opioids to treat chronic pain. Defendants were aware of each of these misrepresentations, and Defendants approved of them and oversaw their dissemination at the national, corporate level.

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**1. Acatvis**

355. As described below, Actavis promoted its branded opioid, Kadian, through a highly deceptive marketing campaign, carried out principally through its sales force and recruited physician speakers. As internal documents indicate, this campaign rested on a series of misrepresentations and omissions regarding the risks, benefits, and superiority of opioids, and indeed incorporated each of the types of deceptive messages. Based on the highly coordinated and uniform nature of Actavis's marketing, Actavis conveyed these deceptive messages to County prescribers. Actavis did so with the intent that County prescribers and/or consumers would rely on the messages in choosing to use opioids to treat chronic pain.[120]

a. Actavis' Deceptive Direct Marketing

356. To help devise its marketing strategy for Kadian, Actavis commissioned a report from one of its consultants in January 2005 about barriers to market entry. The report concluded that two major challenges facing opioid manufacturers in 2005 were (i) overcoming "concerns regarding the safety and tolerability" of opioids, and (ii) the fact that "physicians have been trained to evaluate the supporting data before changing their respective practice behavior." To address these challenges, the report advocated a "[p]ublication strategy based on placing in the literature key data that influence members of the target audience" with an "emphasis . . . on ensuring that the message is believable and relevant to the needs of the target audience." This would entail the creation of "effective copy points . . . backed by published references" and "developing and placing publications that demonstrate [the] efficacy [of opioids] and [their] safety/positive side effect profile." According to the report, this would allow physicians to "reach[] a mental agreement" and change their "practice behavior" without having first evaluated supporting data—of which Actavis (and other Defendants) had none.

[120] Actavis also sold various generic opioids, including Norco, which were widely prescribed in Maverick County and benefited from Actavis's overall promotion of opioids, but were not directly marketed by sales representatives.

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357. The consulting firm predicted that this manufactured body of literature "w[ould], in turn, provide greater support for the promotional message and add credibility to the brand's advocates" based on "either actual or ***perceived*** 'scientific exchange'" in relevant medical literature. (emphasis added). To this end, it planned for three manuscripts to be written during the first quarter of 2005. Of these, "[t]he neuropathic pain manuscript will provide evidence demonstrating KADIAN is as effective in patients with presumptive neuropathic pain as it is in those with other pain types"; "[t]he elderly subanalysis . . . will provide clinicians with evidence that KADIAN is efficacious and well tolerated in appropriately selected elderly patients" and will "be targeted to readers in the geriatrics specialty"; and "[t]he QDF/BID manuscript will . . . .call attention to the fact that KADIAN is the only sustained-release opioid to be labeled for [once or twice daily] use." In short, Actavis knew exactly what each study would show—and how that study would fit into its marketing plan—before it was published. Articles matching Actavis's descriptions later appeared in the *Journal of Pain* and the *Journal of the American Geriatrics Society*.

358. To ensure that messages based on this science reached individual physicians, Actavis deployed sales representatives, or detailers, to visit prescribers in Maverick County and across the country. At the peak of Actavis's promotional efforts in 2011, the company spent $6.7 million on detailing.

359. To track its detailers' progress, Actavis's sales and marketing department actively monitored the prescribing behavior of physicians. It tracked the Kadian prescribing activity of individual physicians, and assessed the success of its marketing efforts by tabulating how many Kadian prescriptions a prescriber wrote after he or she had been detailed. As described below, Kadian monitored numerous County physicians.

360. Actavis also planned to promote Kadian by giving presentations at conferences of organizations where it believed it could reach a high concentration of pain specialists. Its choice of

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conferences was also influenced by the host's past support of opioids. For example, Actavis documents show that Actavis presented papers concerning Kadian at an annual meeting of AGS because AGS's guidelines "support the use of opioids."

361. Actavis targeted prescribers using both its sales force and recruited physician speakers, as described below.

i. *Actavis' Deceptive Sales Training*

362. Actavis's sales representatives targeted physicians to deliver sales messages that were developed centrally and deployed uniformly across the country. These sales representatives were critical in delivering Actavis's marketing strategies and talking points to individual prescribers.

363. Actavis's strategy and pattern of deceptive marketing is evident in its internal training materials. A sales education module titled "Kadian Learning System" trained Actavis's sales representatives on the marketing messages—including deceptive claims about improved function, the risk of addiction, the false scientific concept of "pseudoaddiction," and opioid withdrawal—that sales representatives were directed and required, in turn, to pass on to prescribers, nationally and in Maverick County.

364. The sales training module, dated July 1, 2010, includes the misrepresentations documented in this Complaint, starting with its promise of improved function. The sales training instructed Actavis sales representatives that "most chronic benign pain patients do have markedly improved ability to function when maintained on chronic opioid therapy," when, in reality, available data demonstrate that patients on chronic opioid therapy are *less likely* to participate in daily activities like work. The sales training also misleadingly implied that the dose of prescription opioids could be escalated without consequence and omitted important facts about the increased risks of high dose opioids. First, Actavis taught its sales representatives, who would pass the message on to doctors, that pain patients would not develop tolerance to opioids, which would have required them to receive

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increasing doses: "Although tolerance and dependence do occur with long-term use of opioids, many studies have shown that tolerance is limited in most patients with [Chronic pain]." Second, Actavis instructed its sales personnel that opioid "[d]oses are titrated to pain relief, and so no ceiling dose can be given as to the recommended maximal dose." Actavis failed to explain to its sales representatives and, through them, to doctors, the greater risks associated with opioids at high doses.

365. Further, the 2010 sales training module highlighted the risks of alternate pain medications without providing a comparable discussion of the risks of opioids, painting the erroneous and misleading impression that opioids are safer. Specifically, the document claimed that "NSAIDs prolong the bleeding time by inhibiting blood platelets, which can contribute to bleeding complications" and "can have toxic effects on the kidney." Accordingly, Actavis coached its sales representatives that "[t]he potential toxicity of NSAIDs limits their dose and, to some extent, the duration of therapy" since "[t]hey should only be taken short term." By contrast, the corresponding section related to opioids neglects to include a *single* side effect or risk associated with the use of opioids, including from long-term use.

366. This sales training module also severely downplayed the main risk associated with Kadian and other opioids—addiction. It represented that "there is no evidence that simply taking opioids for a period of time will cause substance abuse or addiction" and, instead, "[i]t appears likely that most substance-abusing patients in pain management practices had an abuse problem before entering the practice." This falsely suggests that few patients would become addicted, that only those with a prior history of abuse are at risk of opioid addiction, and that doctors could screen for those patients and safely prescribe to others. To the contrary, opioid addiction affects a significant population of patients; while patients with a history of abuse may be more prone to addiction, all patients are at risk, and doctors may not be able to identify, or safely prescribe to, patients at greater risk.

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367. The sales training also noted that there were various "signs associated with substance abuse," including past history or family history of substance or alcohol abuse, frequent requests to change medication because of side effects or lack of efficacy, and a "social history of dysfunctional or high-risk behaviors including multiple arrests, multiple marriages, abusive relationships, etc." This is misleading, as noted above, because it implies that only patients with these kinds of behaviors and history become addicted to opioids.

368. Further, the sales training neglected to disclose that no risk-screening tools related to opioids have ever been scientifically validated. The AHRQ recently issued an Evidence Report that could identify "[n]o study" that had evaluated the effectiveness of various risk mitigation strategies—including the types of patient screening implied in Actavis's sales training—on outcomes related to overdose, addiction, abuse or misuse.

369. The sales training module also directed representatives to counsel doctors to be on the lookout for the signs of "[p]seudoaddiction," which were defined as "[b]ehaviors (that mimic addictive behaviors) exhibited by patients with inadequately treated pain." However, the concept of "pseudoaddiction" is unsubstantiated and meant to mislead doctors and patients about the risks and signs of addiction.

370. Finally, the 2010 national training materials trivialized the harms associated with opioid withdrawal by explaining that "[p]hysical dependence simply requires a tapered withdrawal should the opioid medication no longer be needed." This, however, overlooks the fact that the side effects associated with opiate withdrawal are severe and a serious concern for *any person* who wishes to discontinue long-term opioid therapy.

371. The Kadian Learning System module dates from July 2010, but Actavis sales representatives were passing deceptive messages on to prescribers even before then. A July 2010 "Dear Doctor" letter issued by the FDA indicated that "[b]etween June 2009 and February 2010, Actavis

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sales representatives distributed . . . promotional materials that . . . omitted and minimized serious risks associated with [Kadian]." Certain risks that were misrepresented included the risk of "[m]isuse, [a]buse, and [d]iversion of [o]pioids" and, specifically, the risk that "[o]pioid agonists have the potential for being abused and are sought by drug abusers and people with addiction disorders and are subject to criminal diversion." The FDA also took issue with an advertisement for misrepresenting Kadian's ability to help patients "live with less pain and get adequate rest with less medication," when the supporting study did not represent "substantial evidence or substantial clinical experience."

372. Actavis's documents also indicate that the company continued to deceptively market its drugs after 2010. Specifically, a September 2012 Kadian Marketing Update, and the "HCP Detail" aid contained therein, noted that Kadian's "steady state plasma levels" ensured that Kadian "produced higher trough concentrations and a smaller degree of peak-to-trough fluctuations" than other opioids.

373. Actavis also commissioned surveys of prescribers to ensure Kadian sales representatives were promoting the "steady-state" message. That same survey—paid for and reviewed by Actavis—found repeated instances of prescribers being told by sales representatives that Kadian had low potential of abuse or addiction. This survey also found that prescribers were influenced by Actavis's messaging. A number of Kadian prescribers stated that they prescribed Kadian because it was "without the addictive potential" and wouldn't "be posing high risk for addiction." As a result, Actavis's marketing documents celebrated a "perception" among doctors that Kadian had "low abuse potential".

374. Finally, the internal documents of another Defendant, Endo, indicate that pharmaceutical sales representatives employed by Endo, Actavis, and Purdue discussed the AAPM/APS Guidelines with doctors during detailing visits. These guidelines deceptively concluded that the risk of addiction is manageable for patients regardless of past abuse histories.

ii. *Actavis' Deceptive Speaking Training*

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375. Actavis also increasingly relied on speakers—physicians whom Actavis recruited to market opioids to their peers—to convey similar marketing messages. Actavis set a goal to train 100 new Kadian speakers in 2008 alone, with a plan to set up "power lunch teleconferences" connecting speakers to up to 500 participating sites nationwide. Actavis sales representatives, who were required to make a certain number of sales visits each day and week, saw the definition of sales call expanded to accommodate these changes; such calls now included physicians' "breakfast & lunch meetings with Kadian advocate/speaker."

376. A training program for Actavis speakers included training on many of the same messages found in the Kadian Learning System, as described below. The deceptive messages in Actavis's speakers' training are concerning for two reasons: (a) the doctors who participated in the training were, themselves, prescribing doctors, and the training was meant to increase their prescriptions of Kadian; and (b) these doctors were trained, paid, and directed to deliver these messages to other doctors who would write prescriptions of Kadian.

377. Consistent with the training for sales representatives, Actavis's speakers' training falsely minimized the risk of addiction posed by long-term opioid use. Actavis claimed, without scientific foundation, that "[o]pioids can be used with minimal risk in chronic pain patients without a history of abuse or addiction." The training also deceptively touted the effectiveness of "Risk Tools," such as the Opioid Risk Tool, in determining the "risk for developing aberrant behaviors" in patients being considered for chronic opioid therapy. In recommending the use of these screening tools, the speakers' training neglected to disclose that none of them had been scientifically validated.

378. The speakers' training also made reference to "pseudoaddiction" as a "[c]ondition characterized by behaviors, such as drug hoarding, that outwardly mimic addiction but are in fact driven by a desire for pain relief and usually signal undertreated pain." It then purported to assist doctors in identifying those behaviors that *actually* indicated a risk of addiction from those that did not.

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Behaviors it identified as "[m]ore suggestive of addiction" included "[p]rescription forgery," "[i]njecting oral formulations," and "[m]ultiple dose escalations or other nonadherence with therapy despite warnings." Identified as "[l]ess suggestive of addiction" were "[a]ggressive complaining about the need for more drugs," "[r]equesting specific drugs," "[d]rug hoarding during periods of reduced symptoms," and "[u]napproved use of the drug to treat another symptom." By portraying the risks in this manner, the speakers' training presentation deceptively gave doctors a false sense of security regarding the types of patients who can become addicted to opioids and the types of behaviors these patients exhibit.

379. The speakers' training downplayed the risks of opioids, while focusing on the risks of competing analgesics like NSAIDs. For example, it asserted that "Acetaminophen toxicity is a major health concern." The slide further warned that "Acetaminophen poisoning is the most common cause of acute liver failure in an evaluation of 662 US Subjects with acute liver failure between 1998-2003," and was titled "Opioids can be a safer option than other analgesics." However, in presenting the risks associated with opioids, the speakers' training focused on nausea, constipation, and sleepiness, and ignored the serious risks of hyperalgesia, hormonal dysfunction, decline in immune function, mental clouding, confusion, and dizziness; increased falls and fractures in the elderly, neonatal abstinence syndrome, and potentially fatal interactions with alcohol or benzodiazepines. As a result, the training exaggerated the risks of NSAIDs, both absolutely and relative to opioids, to make opioids appear to be a more attractive first-line treatment for chronic pain.

380. The speakers' training also misrepresented the risks associated with increased doses of opioids. For example, speakers were instructed to "[s]tart low and titrate until patient reports adequate analgesia" and to "[s]et dose levels on [the] basis of patient need, not on predetermined maximal dose." However, the speakers' training neglected to warn speakers (and speakers bureau attendees) that patients on high doses of opioids are more likely to suffer adverse events.

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b. Actavis's Deceptive Statements to Maverick County Prescribers and Patients

381. The misleading messages and training materials Actavis provided to its sales force and speakers were part of a broader strategy to convince prescribers to use opioids to treat their patients' pain, without complete and accurate information about the risks, benefits, and alternatives. This deception was national in scope and included Maverick County. Actavis's nationwide messages reached Maverick County prescribers in a number of ways. For example, they were carried into Maverick County by Actavis's sales representatives during detailing visits as well as made available to County patients and prescribers through websites and ads, including ads in prominent medical journals. They have also been delivered to County prescribers by Actavis's paid speakers, who were required by Actavis policy and by FDA regulations to stay true to Actavis's nationwide messaging.

382. Once trained, Actavis's sales representatives and speakers were directed to, and did, visit potential prescribers in Maverick County, as elsewhere, to deliver their deceptive messages. These contacts are demonstrated by Actavis's substantial effort in tracking the habits of individual County physicians prescribing Kadian, and by the direct evidence of Actavis detailing County prescribers.

383. Actavis tracked, in substantial detail, the prescribing behavior of Maverick County area physicians.

**2. Cephalon**

384. At the heart of Cephalon's deceptive promotional efforts was a concerted and sustained effort to expand the market for its branded opioids, Actiq and Fentora, far beyond their FDA-approved use in opioid-tolerant cancer patients. Trading on their rapid-onset formulation, Cephalon touted its opioids as the answer to "breakthrough pain"—a term its own KOL allies planted in the medical literature—whether cancer pain or not. Cephalon promoted this message through its sales force, paid physician speakers, advertisements, and CMEs, even after the FDA issued the company warnings and rejected an expanded drug indication.

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385. Even as it promoted Actiq and Fentora off-label, Cephalon also purveyed many of the deceptive messages described above. It did so both directly—through detailing visits and speaker programs—and through the publications and CMEs of its third-party partners. These messages included misleading claims about functional improvement, addiction risk, pseudoaddiction, and the safety of alternatives to opioids.

386. Based on the highly coordinated and uniform nature of Cephalon's marketing, Cephalon conveyed these deceptive messages to Maverick County prescribers. The materials that Cephalon generated in collaboration with third-parties were also distributed or made available in Maverick County. Cephalon distributed these messages, or facilitated their distribution, in Maverick County with the intent that Maverick County prescribers and/or consumers would rely on them in choosing to use opioids to treat chronic pain.

a. Cephalon's Deceptive Direct Marketing

387. Like the other Defendants, Cephalon directly engaged in misleading and deceptive marketing of its opioids through its sales force and branded advertisements. These messages were centrally formulated and intended to reach prescribers nationwide, including those practicing in the Maverick County area. Cephalon also spent the money necessary to aggressively promote its opioid drugs, setting aside $20 million to market Fentora in 2009 alone.

i. *Cephalon's Fraudulent Off-Label Marketing of Actiq and Fentora*

388. Chief among Cephalon's direct marketing efforts was its campaign to deceptively promote its opioids for off-label uses. Cephalon reaps significant revenue from selling its opioids for treatment of chronic non-cancer pain. However, neither of its two opioid drugs— Actiq or Fentora—is approved for this purpose. Instead, both have indications that are very clearly and narrowly defined to limit their use to a particular form of cancer pain. Despite this restriction, and in order to claim its piece of the broader chronic non-cancer pain market, Cephalon deceptively and unlawfully marketed

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Actiq and then Fentora for patients and uses for which they were not safe, effective, or allowed. This resulted in prescriptions written and paid and, grievously, caused patients to be injured and die. Cephalon's efforts to expand the market for its drugs beyond cancer pain extended to Maverick County prescribers.

a) Cephalon launched its fraudulent marketing scheme for Actiq

389. Cephalon's Actiq is a powerful opioid narcotic that is delivered to the bloodstream by a lollipop lozenge that dissolves slowly in the mouth. As described by one patient, Actiq "tastes like the most delicious candy you ever ate."[121]

390. Actiq is appropriately used only to treat "breakthrough" cancer pain that cannot be controlled by other medications. Breakthrough pain is a short-term flare of moderate-to- severe pain in patients with otherwise stable persistent pain. Actiq is a rapid-onset drug that takes effect within 10-15 minutes but lasts only a short time. It is also an extremely strong drug, considered to be at least 80 times more powerful than morphine. Fentanyl, a key ingredient in Actiq, has been linked to fatal respiratory complications in patients. Actiq is not safe in any dose for patients who are not opioid tolerant, meaning patients who have taken specific doses of opioids for a week or longer and whose systems have acclimated to the drugs.

391. In 1998, the FDA approved Actiq "***ONLY*** for the management of breakthrough cancer pain in patients with malignancies who are already receiving and who are tolerant to opioid therapy for their underlying persistent cancer pain."[122] (emphasis in FDA document). Because of Actiq's dangers, wider, off-label uses—as the FDA label makes clear—are not permitted:

> This product ***must not*** be used in opioid non-tolerant patients because life-threatening respiratory depression and death could occur at any dose in patients not on a chronic

[121] *See* John Carreyrou, *Narcotic 'Lollipop' Becomes Big Seller Despite FDA Curbs*, Wall St. J., Nov. 3, 2006.
[122] FDA Approval Letter for NDA 20-747 (Nov. 4, 1998) at 5, http://www.accessdata.fda.gov/drugsatfda_docs/appletter/1998/20747ltr.pdf (accessed May 30, 2017)

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> regimen of opioids. For this reason, ACTIQ is contraindicated in the management of acute or postoperative pain.[123]

392. Actiq and Fentora are thus intended to be used only in the care of cancer patients and only by oncologists and pain specialists who are knowledgeable of, and skilled in, the use of Schedule II opioids to treat cancer pain. Unlike other drugs, of which off-label uses are permitted but cannot be promoted by the drug maker, Actiq and Fentora are so potent that off- label use for opioid naïve patients is barred by the FDA, as their labels make clear.

393. Notwithstanding the drug's extreme potency and related dangers, and the FDA's explicit limitations, Cephalon actively promoted Actiq for chronic non-cancer pain—an unapproved, off-label use. Cephalon marketed Actiq as appropriate for the treatment of various conditions including back pain, headaches, pain associated with sports-related injuries, and other conditions not associated with cancer and for which it was not approved, appropriate, or safe.

394. Actiq's initial sales counted in the tens of millions of dollars, corresponding to its limited patient population. But by 2005, Actiq sales reached $412 million, making it Cephalon's second-highest selling drug. As a result of Cephalon's deceptive, unlawful marketing, sales exceeded $500 million by 2006.

b) October 1, 2006 – Cephalon fraudulently marked Actiq's successor drug, Fentora

395. Actiq was set to lose its patent protection in September 2006. To replace the revenue stream that would be lost once generic competitors came to market, Cephalon purchased a new opioid drug, Fentora, from Cima Labs and, in August 2005, submitted a New Drug Application ("NDA") to the FDA for approval. Like Actiq, Fentora is an extremely powerful and rapid-onset opioid. It is

[123] Actiq Drug Label, July 2011. The 1998 version does not substantively differ: "Because life-threatening hypoventilation could occur at any dose in patients not taking chronic opiates, *Actiq* is contra- indicated in the management of acute or postoperative pain. This product **must not** be used in opioid non-tolerant patients." (emphasis in original).

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administered by placing a tablet in the mouth until it disintegrates and is absorbed by the mucous membrane that lines the inside of the mouth.

396. On September 25, 2006, the FDA approved Fentora, like Actiq, only for the treatment of breakthrough cancer pain in cancer patients who were already tolerant to around- the-clock opioid therapy for their underlying persistent cancer pain. Fentora's unusually strong and detailed black box warning label—the most serious medication warning required by the FDA—makes clear that, among other things:

> Fatal respiratory depression has occurred in patients treated with FENTORA, including following use in opioid non-tolerant patients and improper dosing. The substitution of FENTORA for any other fentanyl product may result in fatal overdose.
>
> Due to the risk of respiratory depression, FENTORA is contraindicated in the management of acute or postoperative pain including headache/migraine and in opioid non-tolerant patients.[124]

397. When Cephalon launched Fentora on October 1, 2006, it picked up the playbook it had developed for Actiq and simply substituted in Fentora. Cephalon immediately shifted 100 general pain sales representatives from selling Actiq to selling Fentora to the very same physicians for uses that would necessarily and predictably be off-label. Cephalon's marketing of Actiq therefore "primed the market" for Fentora. Cephalon had trained numerous KOLs to lead promotional programs for Fentora, typically including off-label uses for the drug. Cephalon billed Fentora as a major advance that offered a significant upgrade in the treatment of breakthrough pain generally—not breakthrough cancer pain in particular—from Actiq. Cephalon also developed a plan in 2007 to target elderly chronic pain patients via a multi-city tour with stops at AARP events, YMCAs, and senior living facilities.

[124] Fentora Drug Label, February 2013, http://www.accessdata.fda.gov/drugsatfda_docs/label/2013/021947s008lbl.pdf (accessed May 30, 2017)

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398. On February 12, 2007, only four months after the launch, Cephalon CEO Frank Baldino told investors:

> [W]e've been extremely pleased to retain a substantial portion, roughly 75% of the ` rapid onset opioid market. We executed our transition strategy and the results in our pain franchise have been better than we expected. With the successful launch of FENTORA and the progress in label expansion program, we are well positioned to grow our pain franchise for many years to come.[125]

399. On May 1, 2007, just seven months after Fentora's launch, Cephalon's then-Executive Vice President for Worldwide Operations, Bob Roche, bragged to financial analysts that Fentora's reach would exceed even Actiq's. He described the company's successful and "aggressive" launch of Fentora that was persuading physicians to prescribe Fentora for ever broader uses. He identified two "major opportunities"—treating breakthrough cancer pain and:

> The other opportunity of course is the prospect for FENTORA outside of cancer pain, in indications such as breakthrough lower back pain and breakthrough neuropathic pain. . . .
> . . . .
> We believe that a huge opportunity still exists as physicians and patients recognize FENTORA as their first choice rapid onset opioid medication. . . . [opioids are] widely used in the treatment of. . . non-cancer patients . . . .
> . . . .
> Of all the patients taking chronic opioids, 32% of them take that medication to treat back pain, and 30% of them are taking their opioids to treat neuropathic pain. In contrast only 12% are taking them to treat cancer pain, 12%.
>
> We know from our own studies that breakthrough pain episodes experienced by these non-cancer sufferers respond very well to FENTORA. And for all these reasons, we are tremendously excited about the significant impact FENTORA can have on patient health and wellbeing and the exciting growth potential that it has for Cephalon.
>
> In summary, we have had a strong launch of FENTORA and continue to grow the product aggressively. Today, that growth is

[125] *See Cephalon Q4 2006 Earnings Call Transcript,* Seeking Alpha (February 12, 2007, 8:48 PM EST) at 5, http://seekingalpha.com/article/26813-cephalon-q4-2006-earnings-call-transcript (accessed May 30, 2017)

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> coming from the physicians and patient types that we have identified through our efforts in the field over the last seven years. In the future, with new and broader indications and a much bigger field force presence, the opportunity that FENTORA represents is enormous.[126]

c) September 2007 – Reports of death and serious side effects led the FDA to issue a public health warning for Fentora

400. On September 10, 2007, Cephalon sent letters to doctors warning of deaths and other "serious adverse events" connected with the use of Fentora, indicating that "[t]hese deaths occurred as a result of improper patient selection (e.g., use in opioid non-tolerant patients), improper dosing, and/or improper product substitution."[127] The warning did not mention Cephalon's deliberate role in the "improper patient selection."

401. Two weeks later, the FDA issued its own Public Health Advisory. The FDA emphasized, once again, that Fentora should be prescribed only for approved conditions and that dose guidelines should be carefully followed. The FDA Advisory made clear that several Fentora-related deaths had occurred in patients who were prescribed the drug for off-label uses. The FDA Advisory warned that Fentora should not be used for any off-label conditions, including migraines, post-operative pain, or pain due to injury, and that it should be given only to patients who have developed opioid tolerance. The Advisory reiterated that, because Fentora contains a much greater amount of fentanyl than other opiate painkillers, it is not a suitable substitute for other painkillers.[128]

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[126] *See Cephalon Q1 2007 Earnings Call Transcript,* Seeking Alpha (May 1, 2007, 8:48 PM EST) at 23, http://seekingalpha.com/article/34163-cephalon-q1-2007-earnings-call-transcript?page=1 (accessed May 30, 2017)

[127] Letter from Jeffrey M. Dayno, M.D., Vice President, Medical Services, Cephalon, Inc. to Healthcare Providers (Sept. 10, 2007), http://www.fda.gov/downloads/Safety/MedWatch/SafetyInformation/SafetyAlertsforHumanMed icalProducts/UCM154439.pdf (accessed May 30, 2017).

[128] FDA Public Health Advisory, *Important Information for the Safe Use of Fentora (fentanyl buccal tablets*) (Sept. 26, 2007), http://www.fda.gov/Drugs/DrugSafety/PostmarketDrugSafetyInformationforPatientsandProviders/ucm051273.htm (accessed May 30, 2017)

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402. Notwithstanding the regulatory scrutiny, Cephalon's off-label marketing continued. Cephalon's 2008 internal audit of its Sales & Marketing Compliance Programs concluded that marketing and tactical documents, as written, may be construed to promote off-label uses. The same report acknowledged that Cephalon lacked a process to confirm that speakers' program participants were following Cephalon's written, formal policies prohibiting off-label promotion, and that "non-compliant [Cephalon Speaker Programs] may be taking place." Moreover, the report acknowledged that Cephalon's "call universe" may include "inappropriate prescribers"— prescribers who had nothing to do with cancer pain.

d) May 6, 2008 – The FDA rejected Cephalon's request for expanded approval of Fentora

403. Cephalon filed a supplemental new drug application, ("sNDA"), asking the FDA to approve Fentora for the treatment of non-cancer breakthrough pain. Cephalon admitted that Fentora already had been heavily prescribed for non-cancer pain, but argued that such widespread use demonstrated why Fentora should be approved for these wider uses.[129] Cephalon's application also conceded that "[t]o date, no medication has been systematically evaluated in clinical studies or approved by the FDA for the management of [breakthrough pain] in patients with chronic persistent non-cancer-related pain." *Id.*

404. In response to Cephalon's application, the FDA presented data showing that 95% of all Fentora use was for treatment of non-cancer pain.[130] By a vote of 17-3, the relevant Advisory

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[129] *See Fentora CII: Advisory Committee Briefing Document,* U.S. FDA Anesthetic & Life Support Drugs Advisory Comm. & Drug Safety & Risk Mgmt. Advisory Comm. (May 6, 2008), http://www.fda.gov/ohrms/dockets/ac/08/briefing/2008-4356b2-02-Cephalon.pdf (accessed May 30, 2017)

[130] *See* Yoo Jung Chang & Lauren Lee, *Review of Fentora and Actiq Adverse Events from the Adverse Event Reporting System ("AERS") Database*, U.S. FDA Anesthetic & Life Support Drugs Advisory Comm. & Drug Safety & Risk Mgmt. Advisory Comm. (May 6, 2008), http://www.fda.gov/ohrms/dockets/ac/08/slides/2008-4356s2-02-FDAcorepresentations.ppt#289,1 (accessed May 30, 2017).

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Committee—a panel of outside experts—voted against recommending approval of Cephalon's sNDA for Fentora, citing the potential harm from broader use. On September 15, 2008, the FDA denied Cephalon's application and requested, in light of Fentora's already off- label use, that Cephalon implement and demonstrate the effectiveness of proposed enhancements to Fentora's Risk Management Program. In December 2008, the FDA followed that request with a formal request directing Cephalon to submit a Risk Evaluation and Mitigation Strategy for Fentora.

e) March 26, 2009 – the FDA's Division of Drug Marketing, Advertising and Communications ("DDMAC") warned Cephalon about its misleading advertising of Fentora

405. Undeterred by the rejection of its sNDA, Cephalon continued to use its general pain sales force to promote Fentora off-label to pain specialists as an upgrade of Actiq for the treatment of non-cancer breakthrough pain. Deceptively and especially dangerously, Cephalon also continued to promote Fentora for use by all cancer patients suffering breakthrough cancer pain, and not only those who were opioid tolerant.

406. On March 26, 2009, DDMAC issued a Warning Letter to Cephalon, telling Cephalon that its promotional materials for Fentora amounted to deceptive, off-label promotion of the drug.[131] Specifically, the Warning Letter asserted that a sponsored link on Google and other search engines for Fentora, which said "[l]earn about treating breakthrough pain in patients with cancer,"[132] was improper because it "misleadingly broaden[ed] the indication for Fentora by implying that any patient

[131] Letter from Michael Sauers, Regulatory Review Officer, Division of Drug Marketing, Advertising and Communications, to Carole S. Marchione, Senior Director and Group Leader, Regulatory Affairs (March 26, 2009), http://www.fda.gov/downloads/Drugs/GuidanceComplianceRegulatoryInformation/EnforcementActivitiesbyFDA/WarningLettersandNoticeofViolationLetterstoPharmaceuticalCompanies/UCM166238.pdf (accessed May 30, 2017).

[132] Screen shots of the sponsored link are available here: http://www.fda.gov/downloads/Drugs/GuidanceComplianceRegulatoryInformation/EnforcementActivitiesbyFDA/WarningLettersandNoticeofViolationLetterstoPharmaceuticalCompanies/UCM166240.pdf (accessed May 30, 2017).

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with cancer who requires treatment for breakthrough pain is a candidate for Fentora therapy . . . when this is not the case."

407. DDMAC emphasized that Fentora's label was limited to cancer patients with breakthrough pain **"*who are already receiving and who are tolerant to around-the-clock opioid therapy for their underlying persistent cancer pain.*"** (emphasis in original). DDMAC explained that the advertisement was "especially concerning given that Fentora **must not** be used in opioid non-tolerant patients because life-threatening hypoventilation and death could occur at any dose in patients not on a chronic regimen of opioids." (Emphasis in original). DDMAC also warned Cephalon that, based on a review of Cephalon-sponsored links for Fentora on internet search engines, the company's advertisements were "misleading because they make representations and/or suggestions about the efficacy of Fentora, but fail to communicate **any** risk information associated with the use" of the drug. (emphasis in original).

f) Cephalon continues to knowingly, deceptively, and illegally promote Fenotra for off-label uses

408. Cephalon's own market research studies confirm that its Fentora promotions were not focused on physicians who treat breakthrough cancer pain. Cephalon commissioned several market research studies to determine whether oncologists provided an "adequate" market potential for Fentora. These studies' central goal was to determine whether oncologists treat breakthrough cancer pain themselves, or whether they refer such patients to general pain specialists. The first study, completed in 2007, reported that 90% of oncologists diagnose and treat breakthrough cancer pain themselves, and do not refer their breakthrough cancer pain patients to pain specialists. The second study, completed in 2009, confirmed the results of the 2007 study, this time reporting that 88% of oncologists diagnose and treat breakthrough cancer pain themselves and rarely, if ever, refer those patients to general pain specialists. (One reason that general pain specialists typically do not treat

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oncological pain is that the presence of pain can, in itself, be an indicator of a change in the patient's underlying condition that should be monitored by the treating oncologist.)

409. Cephalon was well aware that physicians were prescribing Fentora for off-label uses.

410. Cephalon was also aware that its detailing had an impact on prescription rates.

411. In 2011, Cephalon wrote and copyrighted an article titled "2011 Special Report: An Integrated Risk Evaluation and Risk Mitigation Strategy for Fentanyl Buccal Tablet (FENTORA®) and Oral Transmucosal Fentanyl Citrate (ACTIQ®)" that was published in *Pain Medicine News*.[133] The article promoted Cephalon's drugs for off-label uses by stating that the "judicious use of opioids can facilitate effective and safe management of chronic pain" and noted that Fentora "has been shown to be effective in treatment of [break through pain] associated with multiple causes of pain," not just cancer.[134]

ii. *Cephalon's Misrepresentation of the Risks Associated with the Use of Opioids for the Long-Term Treatment of Chronic Pain*

412. Cephalon's conduct in marketing Actiq and Fentora for chronic non-cancer pain, despite their clear (and deadly) risks and unproved benefits, was an extension, and reaped the benefits, of Cephalon's generally deceptive promotion of opioids for chronic pain.

413. There is insufficient scientific evidence to corroborate a link between chronic opioid therapy and increased functionality. There is however, sufficient evidence to show increased risks of overdose and addiction .[135]

414. Along with deploying its sales representatives, Cephalon used speakers bureaus to help reach prescribers. The company viewed each treating physician as a vehicle to generate prescriptions

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[133] http://www.pharmacytimes.com/publications/issue/2012/january2012/r514-jan-12-rems (accessed May 30, 2017)
[134] *Id.*
[135] Thomas R. Frieden & Debra Houry, *Reducing the Risks of Relief – The CDC Opioid-Prescribing Guideline*, 347 New Eng. J. Med. 1501-04 (2016).

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– whether written by that physician directly or caused indirectly by his or her influence over other physicians.

415. Having determined that speakers were an effective way to reach prescribers, Cephalon set to work ensuring that its speakers would disseminate its misleading messages. Cephalon did not disclose to speakers that, even when these tools are applied, they are unable to control for the risk of addiction.

416. As with the other Defendants, Cephalon deployed the made-up concept of "pseudoaddiction" to encourage prescribers to address addictive behavior in the worst way possible—with more opioids.

417. Working with FSMB, Cephalon also trained its speakers to turn doctors' fear of discipline on its head—doctors, who believed that they would be disciplined if their patients became addicted to opioids, were taught instead that they would be punished if they failed to prescribe opioids to their patients with pain. Through this messaging, Cephalon aimed to normalize the prescribing of opioids for chronic pain and failed to acknowledge the serious risks of long-term opioid use and its inappropriateness as a front-line treatment for pain.

418. Finally, Cephalon also developed a guidebook called *Opioid Medications and REMS: A Patient's Guide*, which deceptively minimized the risks of addiction from the long- term use of opioids. Specifically, the guidebook claimed that "patients without a history of abuse or a family history of abuse do not commonly become addicted to opioids," which is dangerously false. Cephalon distributed the guidebook broadly, and it was available to, and intended to reach, prescribers in Maverick County.

419. The misleading messages and materials Cephalon provided to its sales force and its speakers were part of a broader strategy to convince prescribers to use opioids to treat their patients' pain, without complete and accurate information about the risks, benefits, and alternatives. This

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deception was national in scope and included Maverick County. Cephalon's nationwide messages have reached Maverick County prescribers in a number of ways. For example, they were delivered by Cephalon's sales representatives in detailing visits and made available to County patients and prescribers through websites and ads, including ads in prominent medical journals. They have also been delivered to County prescribers by Cephalon's paid speakers, who were required by Cephalon policy to stay true to the company's nationwide messaging.

b. Cephalon's Deceptive Third-Party Statements

420. Like the other Defendants, Cephalon also relied on third parties to disseminate its messages through deceptive publications and presentations. By funding, developing and reviewing the content, and distributing and facilitating the distribution of these messages, Cephalon exercised editorial control over them. Cephalon, in some instances, used its sales force to directly distribute certain publications by these Front Groups and KOLs, rendering those publications "labeling" within the meaning of § 21 C.F.R. § 1.3(a) and making Cephalon responsible for their contents. Cephalon also deployed its KOLs as speakers for talks and CMEs to selected groups of prescribers.

421. Cephalon's relationships with several such Front Groups and KOLs—and the misleading and deceptive publications and presentations those relationships generated—are described below.

i. *FSMB – Responsible Opioid Prescribing*

422. In 2007, for example, Cephalon sponsored and distributed through its sales representatives FSMB's *Responsible Opioid Prescribing*, which was drafted by Dr. Fishman. Dr. Fishman was frequently hired by a consulting Firm, Conrad & Associates LLC, to write pro-opioid marketing pieces disguised as science. Dr. Fishman's work was reviewed and approved by drug company representatives, and he felt compelled to draft pieces that he admits distorted the risks and benefits of chronic opioid therapy in order to meet the demands of his drug company sponsors.

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423. *Responsible Opioid Prescribing* was a signature piece of Dr. Fishman's work and contained a number of deceptive statements. This publication claimed that, because pain had a negative impact on a patient's ability to function, relieving pain—alone—would "reverse that effect and improve function." However, the truth is far more complicated; functional improvements made from increased pain relief can be offset by a number of problems, including addiction.

424. *Responsible Opioid Prescribing* also misrepresented the likelihood of addiction by mischaracterizing drug-seeking behavior as "pseudoaddiction." It explained that "requesting drugs by name," engaging in "demanding or manipulative behavior," seeing more than one doctor to obtain opioids, and hoarding were all signs of "pseudoaddiction" and are likely the effects of undertreated pain, rather than true addiction. There is no scientific evidence to support the concept of "pseudoaddiction," and any suggestion that addictive behavior masquerades as "pseudoaddiction" is false.

425. Cephalon spent $150,000 to purchase copies of *Responsible Opioid Prescribing* in bulk. It then used its sales force to distribute these copies to 10,000 prescribers and 5,000 pharmacists nationwide. These were available to, and intended to, reach prescribers and pharmacists in Maverick County.

ii. *APF – Treatment Options: A Guide for People Living with Pain*

426. Cephalon also exercised considerable control over the Front Group APF, which published and disseminated many of the most egregious falsehoods regarding chronic opioid therapy. Their relationship, and several of the APF publications, are described in detail below.

427. Documents indicate that Cephalon provided APF with substantial assistance in publishing deceptive information regarding the risks associated with the use of opioids for chronic pain. An April 3, 2008 Fentora Assessment Strategy Tactics Team Meeting presentation outlines Cephalon's strategy to prepare for a meeting at which the FDA Advisory Committee would consider

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expanding the indication of Fentora to include chronic, non-cancer pain. Cephalon prepared by "reaching out to third-party organizations, KOLs, and patients to provide context and, where appropriate, encourage related activity." First among the Front Groups listed was APF.

428. Cephalon was among the drug companies that worked with APF to "educate" the Institute of Medicine of the National Academies (IOM) on issues related to chronic opioid therapy. APF President Will Rowe circulated a document to Cephalon and other drug company personnel that contained key message points and suggested that they "[c]onsider using this document in your communications with the members of the IOM Committee." According to Rowe, recipients should "consider this a working document which you can add to or subtract from." Rowe also advised that, if recipients "have an ally on that Committee," they should "consider sharing this document with that person."

429. Cephalon personnel responded enthusiastically, with Cephalon's Associate Director for Alliance Development stating her belief that "the document does a good job of bringing together many important ideas." Cephalon reviewed and directed changes to this document, with the Cephalon Associate Director thanking Rowe "for incorporating the points we had raised." The close collaboration between Cephalon and APF on this project demonstrates their agreement to work collaboratively to promote the use of opioids as an appropriate treatment for chronic pain.

430. Cephalon's influence over APF's activities was so pervasive that APF's President, Will Rowe, even reached out to Defendants—including Cephalon—rather than his own staff, to identify potential authors to answer a 2011 article critical of opioids that had been published in the Archives of Internal Medicine.

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431. Starting in 2007, Cephalon sponsored APF's *Treatment Options: A Guide for People Living with Pain.*[136] It is rife with misrepresentations regarding the risks, benefits, and superiority of opioids.

432. For example, *Treatment Options* deceptively asserts that the long-term use of opioids to treat chronic pain could help patients function in their daily lives by stating that, when used properly, opioids "give [pain patients] a quality of life [they] deserve." There is no scientific evidence corroborating that statement, and such statements are, in fact, false. Available data demonstrate that patients on chronic opioid therapy are actually *less likely* to participate in life activities like work.

433. *Treatment Options* also claims that addiction is rare and is evident from patients' conduct in self-escalating their doses, seeking opioids from multiple doctors, or stealing the drugs. *Treatment Options* further minimizes the risk of addiction by claiming that it can be avoided through the use of screening tools, like "opioid agreements," which can "ensure [that patients] take the opioid as prescribed." Nowhere does *Treatment Options* explain to patients and prescribers that neither "opioid agreements" nor any other screening tools have been scientifically validated to decrease the risks of addiction, and the publication's assurances to the contrary are false and deceptive.

434. *Treatment Options* also promotes the use of opioids to treat chronic pain by painting a misleading picture of the risks of alternate treatments, most particularly NSAIDs. *Treatment Options* notes that NSAIDs can be dangerous at high doses, and attributes 10,000 to 20,000 deaths a year annually to NSAID overdose. According to *Treatment Options*, NSAIDs are different from opioids because opioids have "no ceiling dose," which is beneficial since some patients "need" larger doses of painkillers than they are currently prescribed. These claims misleadingly suggest that opioids are safe even at high doses and omit important information regarding the risks of high-dose opioids.

[136] https://assets.documentcloud.org/documents/277605/apf-treatmentoptions.pdf (accessed May 30, 2017)

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435. Additionally, *Treatment Options* warns that the risks associated with NSAID use increase if NSAIDs are "taken for more than a period of months," but deceptively omits any similar warning about the risks associated with the long-term use of opioids. This presentation paints a misleading picture of the risks and benefits of opioid compared with alternate treatments.

436. APF distributed 17,200 copies of *Treatment Options* in 2007 alone. It is currently available online and was intended to reach Maverick County prescribers and pharmacists.

iii. *Key Opinion Leaders and Misleading Science*

437. Cephalon also knew that its misleading messages would be more likely to be believed by prescribers if they were corroborated by seemingly neutral scientific support.

438. Employing these tactics, Cephalon caused the term "breakthrough pain"—a term it seeded in the medical literature—to be used in articles published by practitioners and clinicians it supported. With funding from Cephalon, for example, Dr. Portenoy wrote an article that purported to expand the definition of breakthrough cancer pain to non-cancer indications, vastly expanding the marketing potential of Cephalon's Fentora. The article was published in the nationally circulated *Journal of Pain* in 2006 and helped drive a surge in Fentora prescriptions.

439. The concept of "breakthrough pain" ultimately formed the sole basis for the central theme of promotional messages Cephalon cited to support the approval and marketing of Actiq and Fentora, rapid-acting opioids which begin to work very quickly but last only briefly. Neither of these drugs had a natural place in the treatment of chronic pain before Cephalon's marketing campaign changed medical practice. A recent literature survey of articles describing non-cancer breakthrough pain calls into question the validity of the concept, suggesting it is not a distinct pain condition but a hypothesis to justify greater dosing of opioids. In other words, Cephalon conjured the science of breakthrough pain in order to sell its drugs.

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440. As one scholar has pointed out, references to breakthrough pain in articles published on the MEDLINE bibliographic database spiked in 1998 and again in 2006.[137] These spikes coincide with FDA's approval of Actiq and Fentora.

iv. *Misleading Continuing Medical Education*

441. Cephalon developed sophisticated plans for the deployment of its KOLs, broken down by sub-type and specialty, to reach targeted groups of prescribers through CMEs. Cephalon used the CME programs it sponsored to deceptively portray the risks related to the use of opioids to treat chronic non-cancer pain and promote the off-label use of Actiq and Fentora.

442. In 2007 and 2008, Cephalon sponsored three CMEs that each positioned Actiq and Fentora as the only "rapid onset opioids" that would provide effective analgesia within the time period during which "breakthrough pain" was at its peak intensity. Although the CMEs used only the generic names of the drugs, the description of the active ingredient and means of administration means that a physician attending the CME knew it referred only to Actiq or Fentora.

443. The CMEs each taught attendees that there was no sound basis for the distinction between cancer and non-cancer "breakthrough pain," and one instructed patients that Actiq and Fentora were commonly used in non-cancer patients, thus effectively endorsing this use. Optimizing Opioid Treatment for Breakthrough Pain, offered online by Medscape, LLC from September 28, 2007, through December 15, 2008, was prepared by KOL Dr. Webster and M. Beth Dove. It recommends prescribing a "short-acting opioid" (e.g., morphine, hydromorphone, oxycodone) "when pain can be anticipated," or a rapid-onset opioid when it cannot. The only examples of rapid-onset opioids then on the market were oral transmucosal fentanyl citrate (i.e., Actiq) or fentanyl effervescent buccal tablet

[137] Adriane Fugh-Berman, *Marketing Messages in Industry-Funded CME*, PharmedOut , Georgetown U. Med. Ctr. (June 25, 2010), *available at* pharmedout.galacticrealms.com/Fugh BermanPrescriptionforConflict6-25-10.pdf (accessed May 30, 2017).

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(i.e., Fentora): "Both are indicated for treatment of [breakthrough pain] in opioid-tolerant cancer patients and are frequently prescribed to treat [breakthrough pain] in noncancer patients as well."

444. Optimizing Opioid Treatment for Breakthrough Pain not only deceptively promoted Cephalon's drugs for off-label use, but also misleadingly portrayed the risks, benefits, and superiority of opioids for the treatment of chronic pain. For example, the CME misrepresented that Actiq and Fentora would help patients regain functionality by advising that they improve patients' quality of life and allow for more activities when taken in conjunction with long-acting opioids. The CME also minimized the risks associated with increased opioid doses by explaining that NSAIDs were less effective than opioids for the treatment of breakthrough pain because of their dose limitations, without disclosing the heightened risk of adverse events on high-dose opioids.

445. Around the same time, Dr. Webster was receiving nearly $2 million in funding from Cephalon.

446. Optimizing Opioid Treatment for Breakthrough Pain was available online and was intended to reach County prescribers.

447. Cephalon similarly used an educational grant to sponsor the CME *Breakthrough Pain: Improving Recognition and Management*, which was offered online between March 31, 2008, and March 31, 2009, by Medscape, LLC. The direct result of Cephalon's funding was a purportedly educational document that echoed Cephalon's marketing messages. The CME deceptively omitted Actiq's and Fentora's tolerance limitations, cited examples of patients who experienced pain from accidents, not from cancer, and, like Cephalon's *Optimizing Opioid Treatment* CME, taught that Actiq and Fentora were the only products on the market that would take effect before the breakthrough pain episode subsided. This CME was available online and was intended to reach County prescribers.

448. Lastly, KOL Dr. Fine authored a CME, sponsored by Cephalon, titled *Opioid-Based Management of Persistent and Breakthrough Pain*, with KOLs Dr. Christine A. Miaskowski and Michael J.

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Brennan, M.D. Cephalon paid to have this CME published in a supplement of Pain Medicine News in 2009.[138] It instructed prescribers that "clinically, broad classification of pain syndromes as either cancer- or noncancer-related has limited utility," and recommended dispensing "rapid onset opioids" for "episodes that occur spontaneously" or unpredictably, including "oral transmucosal fentanyl," *i.e.*, Actiq, and "fentanyl buccal tablet," *i.e.*, Fentora, including in patients with chronic non-cancer pain. Dr. Miaskowski disclosed in 2009, in connection with the APS/AAPM Opioid Treatment Guidelines, that she served on Cephalon's speakers bureau.[139] Dr. Fine also received funding from Cephalon for consulting services.

449. *Opioid-Based Management of Persistent and Breakthrough Pain* was available to and was intended to reach County prescribers.

450. Cephalon's control over the content of these CMEs is apparent based on its advance knowledge of their content. A December 2005 Cephalon launch plan set forth key "supporting messages" to position Fentora for its product launch. Among them was the proposition that "15-minute onset of action addresses the unpredictable urgency of [breakthrough pain]." Years later, the same marketing messages reappeared in the Cephalon-sponsored CMEs described above. Echoing the Cephalon launch plan, Optimizing Opioid Treatment for Breakthrough Pain stated that "[t]he unpredictability of [breakthrough pain] will strongly influence the choice of treatment" and that Fentora "delivers an onset of analgesia that is similar to [Actiq] at ≤ 15 minutes." Similarly, Opioid-Based Management of Persistent and Breakthrough Pain defined "breakthrough pain" as "unpredictable," over a table describing both cancer and non-cancer "breakthrough pain."

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[138] https://www.yumpu.com/en/document/view/11409251/opioid-based-management-of-persistent-and-breakthrough-pain (accessed May 30, 2017).

[139] 14 of 21 panel members who drafted the AAPM/APS Guidelines received support from Janssen, Cephalon, Endo, and Purdue.

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451. Cephalon tracked the effectiveness of its deceptive marketing through third parties, demonstrating that Cephalon not only planned for, but depended upon, their activities as a key element of its marketing strategy. These programs were available to prescribers in Maverick County and, based on the uniform and nationwide character of Cephalon's marketing, featured the same deceptive messages described above.

c. Cephalon's Deceptive Third-Party Statements to County Prescribers and Patients

452. Cephalon used various measures to disseminate its deceptive statements regarding the risks of off-label use of Actiq and Fentora and the risks, benefits, and superiority of opioids to County patients and prescribers.

453. Cephalon's speakers regularly held talks for County prescribers. These talks followed the same deceptive talking points covered in Cephalon's speakers' training.

454. Cephalon also targeted County prescribers through the use of its sales force.

455. Given that Cephalon's own studies demonstrated that the overwhelming majority of oncologists diagnose and treat breakthrough cancer pain themselves, Cephalon knew the only purpose of representatives meeting with these prescribers was to promote off-label use. Based on the uniform and nationwide character of Cephalon's marketing, Cephalon's deceptive messages would have been disseminated to County prescribers by Cephalon's sales representatives during these events.

456. Sales representatives, and the misrepresentations on which they were trained, drove significant Fentora sales.

**3. Endo**

457. Endo promoted its opioids through the full array of marketing channels. The company deployed its sales representatives, paid physician speakers, journal supplements, and advertising in support of its branded opioids, principally Opana and Opana ER. Misleading claims about the purportedly lower abuse potential of Opana ER featured prominently in this campaign. Endo also

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made many other deceptive statements and omissions. These included deceptive messages about functional improvement, addiction risk, "pseudoaddiction," addiction screening tools, and the safety of alternatives to opioids.

458. At the same time, Endo also relied on third-party partners to promote the safety, efficacy, and superiority of opioids generally, through a combination of CMEs, websites, patient education pamphlets, and other publications. These materials echoed the misrepresentations described above, and also made deceptive statements about withdrawal symptoms and the safety of opioids at higher doses.

459. Through the highly coordinated and uniform nature of Endo's marketing, Endo conveyed these deceptive messages to County prescribers. The materials that Endo generated in collaboration with third-parties also were distributed or made available in Maverick County. Endo distributed these messages, or facilitated their distribution, in Maverick County with the intent that County prescribers and/or consumers would rely on them in choosing to use opioids to treat chronic pain.

a. Endo's Deceptive Direct Marketing

460. Like the other Defendants, Endo used deceptive direct marketing to increase the sales of its dangerous opioids. As set forth below, Endo conveyed these deceptive messages in training of its sales force and recruited speakers, who in turn conveyed them to physicians; in a misleading journal supplement; and in unbranded advertising.

i. *Endo's Sales Force and Deceptive Sales Training*

461. Endo's promotion of Opana ER relied heavily on in-person marketing, including to County prescribers. Endo had an aggressive detailing program. In the first quarter of 2010 alone, sales representatives made nearly 72,000 visits to prescribers nationwide to detail Opana ER. Between 2007

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and 2013, Endo spent between $3 million and $10 million each quarter to promote opioids through its sales force.

462. Endo's sales representatives, like those of the other Defendants, targeted physicians to deliver sales messages that were developed centrally and deployed uniformly across the country. These sales representatives were critical in transmitting Endo's marketing strategies and talking points to individual prescribers.

463. Endo specifically directed its sales force to target physicians who would prescribe its drugs to treat chronic pain. For example, an Opana Brand Tactical Plan dated August, 2007 aimed to increase "Opana ER business from [the Primary Care Physician] community" more than 45% by the end of that year. Indeed, Endo sought to develop strategies that would be most persuasive to primary care doctors—strategies that sought to influence the prescribing behavior of primary care physicians through the use of subject matter experts. A February 2011 Final Report on Opana ER Growth Trends, for example, predicted that Endo's planned "[u]se of Pain Specialists as local thought leaders should affect increased primary care adoption."

464. Endo trained its sales force to make a number of misrepresentations to physicians nationwide, including to physicians in Maverick County. Endo's sales representatives were trained to represent to these prescribers that Opana ER would help patients regain function they had lost to chronic pain; that Endo opioids had a lower potential for abuse because they were "designed to be crush resistant," despite the fact that "clinical significance of INTAC Technology or its impact on abuse/misuse ha[d] not been established for Opana ER;" and that drug seeking behavior was a sign of undertreated pain rather than addiction.

465. Endo knew that its marketing reached physicians repeatedly because it tracked their exposure. Internal Endo documents dated August 23, 2006 demonstrate that the following percentages of physicians would view an Endo journal insert (or paid supplement) at least 3 times in an 8 month

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period: 86% of neurologists; 86% of rheumatologists; 85% of oncologists; 85% of anesthesiologists; 70% of targeted primary care physicians; and 76% of OB/GYNs.

466. Endo was not only able to reach physicians through its marketing, but also successfully impart its marketing messages. The company found that its promotional materials tripled prescribers' ability to recall the sales message and doubled their willingness to prescribe Opana ER in the future. This was true of marketing that contained deceptions.

467. For example, according to internal Endo documents, up to 10% of physicians it detailed were able to recall, without assistance, the message that Opana ER had "Minimal/less abuse/misuse" potential than other drugs. The Endo message that prescribers retained was a plain misrepresentation: that use of Opana ER was unlikely to lead to abuse and addiction. Although Opana ER always has been classified under Schedule II as a drug with a "high potential for abuse", the largest single perceived advantage of Opana ER, according to a survey of 187 physicians who reported familiarity with the drug, was "perceived low abuse potential," cited by 15% of doctors as an advantage. Low abuse potential was among the deceptive messages that County prescribers received, and retained, from Endo sales representatives.

468. Endo's own internal documents acknowledged the misleading nature of these statements, conceding that "Opana ER has an abuse liability similar to other opioid analgesics as stated in the [FDA-mandated] box warning." A September 2012 Opana ER Business Plan similarly stated that Endo needed a significant investment in clinical data to support comparative effectiveness, scientific exchange, benefits and unmet need, while citing lack of "head-to-head data" as a barrier to greater share acquisition.

469. Nevertheless, Endo knew that its marketing was extremely effective in turning physicians into prescribers. Nationally, the physicians Endo targeted for in-person marketing represented approximately 84% of all prescribers of Opana ER in the first quarter of 2010.

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Endo also observed that the prescribers its sales representatives visited wrote nearly three times as many prescriptions per month for Opana ER as those physicians who were not targeted for Endo's marketing—7.4 prescriptions per month versus 2.5. The most heavily targeted prescribers wrote nearly 30 prescriptions per month. Internal Endo documents from May 2008 indicate that Endo expected that each of its sales representatives would generate 19.6 prescriptions per week by the end of 2008. As summarized by a February 2011 report on Opana ER growth trends, Endo's "[a]ggressive detailing [is] having an impact."

470. More broadly, Endo's sales trainings and marketing plans demonstrate that its sales force was trained to provide prescribers with misleading information regarding the risks of opioids when used to treat chronic pain. Foremost among these messages were misleading claims that the risks of addiction, diversion, and abuse associated with opioids—and Endo's products in particular—were low, and lower than other opioids.

a) Endo's Sales Force Deceptively Minimized the Risks of Addiction Associated with Chronic Opioid Therapy.

471. By way of illustration, Endo's Opana ER INTAC Technology Extended-Release Sell Sheet Implementation Guide, which instructs Endo sales personnel how to effectively "support key messages" related to the marketing of Opana ER, states that it is an "approved message" for sales representatives to stress that Opana ER was "designed to be crush resistant," even though this internal document conceded that "the clinical significance of INTAC Technology or its impact on abuse/misuse has not been established for Opana ER."

472. Other Endo documents acknowledged the limitations on Opana ER's INTAC technology, conceding that while Opana ER may be resistant to pulverization, it can still be "ground" and "cut into small pieces" by those looking to abuse the drug.

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473. Endo's claims about the crush-resistant design of Opana ER also made their way to the company's press releases. A January 2013 article in *Pain Medicine News*, based in part on an Endo press release, described Opana ER as "crush-resistant." This article was posted on the *Pain Medicine News* website, which was accessible to County patients and prescribers.

474. The only reason to promote the crush resistance of Opana ER was to persuade doctors that there was less risk of abuse, misuse, and diversion of the drug. The idea that Opana ER was less addictive than other drugs was the precise message that County prescribers took from Endo's marketing.

475. On May 10, 2013, the FDA warned Endo that there was no evidence that Opana ER's design "would provide a reduction in oral, intranasal, or intravenous abuse" and that the post-marketing data Endo had submitted to the FDA "are insufficient to support any conclusion about the overall or route-specific rates of abuse." Even though it was rebuked by the FDA, Endo continued to market Opana ER as having been ***designed*** to be crush resistant, knowing that this would (falsely) imply that Opana actually ***was*** crush resistant and that this crush-resistant quality would make Opana ER less likely to be abused.

476. Endo's sales training and the promotional materials distributed by its sales representatives also minimized the risk of addiction. Endo also circulated education materials that minimized the risk of addiction. For example, Endo circulated an education pamphlet with the Endo logo titled "Living with Someone with Chronic Pain," which implied, to persons providing care to chronic pain patients, that addiction was not a substantial concern by stating that "[m]ost health care providers who treat people with pain agree that most people do not develop an addiction problem." This pamphlet was downloadable from Endo's website and accessible to County prescribers.

477. Endo's sales training also misrepresented the risks of addiction associated with Endo's products by implying that Opana's prolonged absorption would make it less likely to lead to abuse.

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For example, a presentation titled "Deliver the Difference for the Opana Brand in POA II" sets out that one of the "[k]ey [m]essages" for the Endo sales force was that Opana ER provides "[s]table, steady-state plasma levels for true 12-hour dosing that lasts." Endo's sales representatives used this messaging to imply to County prescribers that Opana ER provided "steady state" pain relief, making Opana less likely to incite euphoria in patients and less likely to lead to addiction.

478. Endo further instructed its sales force to promote the misleading concept of "pseudoaddiction,"—i.e., that drug-seeking behavior was not cause for alarm, but merely a manifestation of undertreated pain. In a sales training document titled "Understanding the Primary Care MD and their use of Opioids," Endo noted that the "biggest concerns" among primary care physicians were "prescription drug abuse (84.2%), addiction (74.9%), adverse effects (68%), tolerance (60.7%), and medication interaction (32%)." In response to these concerns, Endo instructed its sales representatives to ask whether their customers were "confus[ing] 'pseudo-addiction' with 'drug-seekers'" and how confident they were that their health care providers "know these differences (Tolerance, Dependence, Addiction, Pseudo- Addiction . . .)."

b) Endo's Sales Force Deceptively Implied that Chronic Opioid Therapy Would Improve Patients' Ability to Function.

479. In addition to their deceptive messages regarding addiction, Endo's promotional materials and sales trainings also misleadingly claimed that patients using opioids for the long- term treatment of chronic pain would experience improvements in their daily function. In reality, long-term opioid use has not been shown to, and does not, improve patients' function, and, in fact, is often accompanied by serious side effects that degrade function. Endo's own internal documents acknowledged that claims about improved quality of life were unsubstantiated "off label claims."

480. Nevertheless, Endo distributed product advertisements that suggested that using Opana ER to treat chronic pain would allow patients to perform demanding tasks like work as a

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chef. One such advertisement states prominently on the front: "Janice is a 46-year-old chef with chronic low back pain. She needs a treatment option with true 12-hour dosing." The advertisement does not mention the "moderate to severe pain" qualification in Opana ER's indication, except in the fine print. These advertisements were mailed to prescribers and distributed by Endo's sales force in detailing visits, which would have included Endo representatives' visits to prescribers.

481. In a 2007 sales tool that was intended to be shown by Endo sales personnel to physicians during their detailing visits, Endo highlighted a hypothetical patient named "Bill," a 40-year-old construction worker who was reported to suffer from chronic low back pain. According to the sales tool, Opana ER will make it more likely that Bill can return to work and support his family.

482. Similarly, training materials for sales representatives from March 2009 ask whether it is true or false that "[t]he side effects of opioids prevent a person from functioning and can cause more suffering than the pain itself." The materials indicate that this is "[f]alse" because "[t]he overall effect of treatment with opioids is very favorable in most cases."

483. A sales training video dated March 8, 2012 that Endo produced and used to train its sales force makes the same types of claims. A patient named Jeffery explains in the video that he suffers from chronic pain and that "chronic pain [ . . .] reduces your functional level." Jeffery claims that after taking Opana ER, he "can go out and do things" like attend his son's basketball game and "[t]here's no substitute for that." This video was shown to Endo's sales force, which adopted its misleading messaging in its nationwide sales approach, including the approach it used in Maverick County.

484. Claims of improved functionality were central to Endo's marketing efforts for years. A 2012 Endo Business Plan lists ways to position Opana ER, and among them is the claim that Opana ER will help patients "[m]aintain[] normal functionality, sleep, [and] work/life/performance productivity" and have a positive "[e]ffect on social relationships." Indeed, that business plan describes

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the "Opana ER Vision" as "[t]o make the Opana franchise (Opana ER, Opana, Opana Injection) the choice that maximizes improvement in functionality and freedom from the burden of moderate-to-severe pain."

c) Endo's Sales Force Deceptively presented the Risks and Benefits of Opioids to Make Them Appear Safer Than Other Analgesics

485. Endo further misled patients and prescribers by downplaying the risks of opioids in comparison to other pain relievers. For example, in Maverick County and elsewhere, Endo distributed a presentation titled *Case Challenges in Pain Management: Opioid Therapy for Chronic Pain*. This study held out as a representative example one patient who had taken NSAIDs for more than eight years and, as a result, developed "a massive upper gastrointestinal bleed." The presentation recommended treating this patient with opioids instead. By focusing on the adverse side effects of NSAIDs, while omitting discussion of serious side effects associated with opioids, this presentation misleadingly portrayed the comparative risks and benefits of these drugs.

486. Endo distributed *Case Challenges in Pain Management: Opioid Therapy for Chronic Pain* to 116,000 prescribers in 2007, including primary care physicians.

ii. *Endo's Speakers Bureau Programs Deceptively Minimized the Risks of Addiction Associated with Chronic Opioid Therapy*

487. In addition to its sales representatives' visits to doctors, Endo also used deceptive science and speaker programs to spread its deceptive messages.

488. Endo leaned heavily on its speakers' bureau programs. In 2008 alone, Endo spent nearly $4 million to promote up to 1,000 speakers programs around the country. Endo contracted with a medical communications firm to operate its speakers bureau program, planning to hold a total of 500 "fee-for-service . . . peer-to-peer promotional programs" for Opana ER in just the second

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half of 2011, including dinners, lunches and breakfasts. These programs were attended by sales representatives, revealing their true purpose as marketing, rather than educational, events.

489. Endo's internal reporting stated that the "return on investment" turned positive 8-12 weeks after such programs. Endo measured that return on investment in numbers of prescriptions written by physicians who attended the events. One internal Endo document concluded: "[w]e looked at the data for [the] 2011 program and the results were absolutely clear: physicians who came into our speaker programs wrote more prescriptions for Opana ER after attending than they had before they participated. You can't argue with results like that."

490. These speakers bureau presentations included the very same misrepresentations Endo disseminated through its sales representatives. A 2012 speaker slide deck for Opana ER— on which Endo's recruited speakers were trained and to which they were required to adhere to in their presentations—misrepresented that the drug had low abuse potential, in addition to suggesting that as many as one-quarter of the adult population could be candidates for opioid therapy.

491. In addition, a 2013 training module directed speakers to instruct prescribers that "OPANA ER with INTAC is the only oxymorphone designed to be crush resistant" and advised that "[t]he only way for your patients to receive oxymorphone ER in a formulation designed to be crush resistant is to prescribe OPANA ER with INTAC." This was a key point in distinguishing Opana ER from competitor drugs. Although Endo mentioned that generic versions of oxymorphone were available, it instructed speakers to stress that "[t]he generics are not designed to be crush resistant." This was particularly deceptive given that Opana ER was not actually crush-resistant.

492. In 2009, Endo wrote a talk titled *The Role of Opana ER in the Management of Chronic Pain.* The talk included a slide titled "Use of Opioids is Recommended for Moderate to Severe Chronic Noncancer Pain," which cited the AAPM/APS Guidelines—and their accompanying misstatements

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regarding the likelihood of addiction (by claiming that addiction risks were manageable regardless of patients' past abuse histories) while omitting their disclaimer regarding the lack of supporting evidence in favor of that position. This dangerously misrepresented to doctors the force and utility of the 2009 Guidelines.

493. The misleading messages and materials Endo provided to its sales force and its speakers were part of a broader strategy to convince prescribers to use opioids to treat their patients' pain, irrespective of the risks, benefits, and alternatives. This deception was national in scope and included Maverick County. Endo's nationwide messages would have reached County prescribers in a number of ways. For example, they were carried into Maverick County by Endo's sales representatives during detailing visits as well as made available to County patients and prescribers through websites and ads. They also have been delivered to County prescribers by Endo's paid speakers, who were required by Endo policy and by FDA regulations to stay true to Endo's nationwide messaging.

iii. *Endo's Misleading Journal Supplement*

494. In 2007, Endo commissioned the writing, and paid for the publishing of a supplement available for CME credit in the Journal of Family Practice called Pain Management Dilemmas in Primary Care: Use of Opioids, and it deceptively minimized the risk of addiction by emphasizing the effectiveness of screening tools. Specifically, it recommended screening patients using tools like the Opioid Risk Tool or the Screener and Opioid Assessment for Patients with Pain. It also falsely claimed that, through the use of tools like toxicology screens, pill counts, and a "maximally structured approach," even patients at high risk of addiction could safely receive chronic opioid therapy. Endo distributed 96,000 copies of this CME nationwide, and it was available to, and was intended to, reach County prescribers.

iv. *Endo's Deceptive Unbranded Advertising*

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495. Endo also used unbranded advertisements to advance its goals. By electing to focus on unbranded marketing, Endo was able to make claims about the benefits of its opioids that the FDA would never allow in its branded materials. The chart below compares an Endo unbranded statement with one of Endo's FDA-regulated, branded statements:

| **Living with Someone with Chronic Pain (2009)(Unbranded)** | **Opana ER Advertisement (2011/2012/2013) (Branded)** |
|---|---|
| Patient education material created by Endo | Endo advertisement |
| "Most health care providers who treat people with pain agree that **most people do not develop an addiction problem**." | "[C]ontains oxymorphone, an opioid agonist and Schedule II controlled substance with an abuse liability similar to other opioid agonists, legal or illicit."<br><br>"All patients treated with opioids require careful monitoring for signs of abuse and addiction, since **use of opioid analgesic products carries the risk of addiction even under appropriate medical use**." |

b. Endo's Deceptive Third-Party Statements

496. Endo's efforts were not limited to directly making misrepresentations through its marketing materials, its speakers, and its sales force. Endo believed that support for patient advocacy and professional organizations would reinforce Endo's position as "the pain management company."

497. Prior to, but in contemplation of, the 2006 launch of Opana ER, Endo developed a "Public Stakeholder Strategy." Endo identified "tier one" advocates to assist in promoting the approval and acceptance of its new extended release opioid. Endo also intended to enlist the support of organizations that would be "favorable" to schedule II opioids from a sales perspective and that engaged in, or had the potential to advocate for, public policy. Endo sought to develop its

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relationships with these organizations through its funding. In 2008, Endo spent $1 million per year to attend conventions of these pro-opioid medical societies, including meetings of AAPM, APS, and the American Society of Pain Management Nursing ("ASPMN").

498. APF's ability to influence professional societies and other third parties is demonstrated by its approach to responding to a citizens' petition filed with the FDA by the Physicians for Responsible Opioid Prescribing (the "PROP Petition"). The PROP petition, filed by a group of prescribers who had become concerned with the rampant prescribing of opioids to treat chronic pain, asked the FDA to require dose and duration limitations on opioid use and to change the wording of the approved indication of various long-acting opioids to focus on the severity of the pain they are intended to treat.

499. The PROP Petition set off a flurry of activity at Endo. It was understood that Endo would respond to the petition but Endo personnel wondered "[s]hould we [ . . . ] consider filing a direct response to this [citizens' petition] or do you think we are better served by working through our professional society affiliations?" One Endo employee responded: "My sense is the societies are better placed to make a medical case than Endo." Endo's Director of Medical Science agreed that "a reply from an external source would be most impactful." These communications reflected Endo's absolute confidence that the professional societies would support its position.

i. *APF*

500. One of the societies with which Endo worked most closely was APF. Endo provided substantial assistance to, and exercised editorial control, over the deceptive and misleading messages that APF conveyed through its National Initiative on Pain Control ("NIPC"). Endo was one of APF's biggest financial supporters, providing more than half of the $10 million APF received from opioid manufacturers during its lifespan. Endo spent $1.1 million on the NIPC program in 2008

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alone, funding earmarked in part, for the creation of CME materials that were intended to be used repeatedly.

501. Endo's influence over APF's activities was so pervasive that APF President Will Rowe reached out to Defendants—including Endo—rather than his own staff, to identify potential authors to answer a 2011 article critical of opioids that had been published in the Archives of Internal Medicine. Personnel from Defendants Purdue, Endo, Janssen, and Cephalon worked with Rowe to formulate APF's response which was ultimately published.

502. Documents also indicate that Endo personnel were given advance notice of the materials APF planned to publish on its website and provided an opportunity to comment on the content of those materials before they were published. For example, in early July of 2009, APF's Director of Strategic Development wrote to Endo personnel to give them advance notice of content that APF planned to be "putting . . . up on the website but it's not up yet." The Endo employee assured the sender that she "w[ould] not forward it to anyone at all" and promised that she would "'double delete it' from [her] inbox." In response, APF's Director of Strategic Development replied internally with only four words: "And where's the money?"

503. At no time was Endo's relationship with APF closer than during its sponsorship of the NIPC. Before being taken over by APF, the NIPC was sponsored by Professional Postgraduate Services which the Accreditation Council for Continuing Medical Education determined to be a "commercial interest" and could no longer serve as a sponsor. In response, Endo reached out to APF. An August 2009 document titled "A Proposal for the American Pain Foundation to Assume Sponsorship of the National Initiative on Pain Control," pointed out that "[f]or the past 9 years, the NIPC has been supported by unrestricted annual grants from Endo Pharmaceuticals, Inc." According to this document, APF's sponsorship of the NIPC "[o]ffers the APF a likely opportunity to generate new revenue, as Endo has earmarked substantial funding: $1.2 million in net revenue for

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2010 to continue the NIPC." Further, sponsorship of the APF would "[p]rovide[] numerous synergies to disseminate patient education materials," including "[h]andouts to attendees at all live events to encourage physicians to drive their patients to a trusted source for pain education—the APF website."

504. A September 14, 2009 presentation to APF's board contained a materially similar discussion of NIPC sponsorship, emphasizing the financial benefit to APF from assuming the role of administering NIPC. The proposal "offer[ed] a solution to continue the development and implementation of the NIPC initiative as non-certified . . . yet independent education to physicians and healthcare professionals in the primary care setting, while providing the APF with a dependable, ongoing source of grant revenue." A number of benefits related to NIPC sponsorship were listed, but chief among them was "a likely opportunity [for APF] to generate new revenue, as Endo has earmarked substantial funding: $1.2 million in net revenue for 2010 to continue the NIPC."

505. Internal Endo scheduling documents indicate that "NIPC module curriculum development, web posting, and live regional interactive workshops" were Endo promotional tasks in 2010. Endo emails indicate that Endo personnel reviewed the content created by NIPC and provided feedback.

506. Behind the scenes, Endo exercised substantial control over NIPC's work. Endo exerted its control over NIPC by funding NIPC and APF projects; developing, specifying, and reviewing content; and taking a substantial role in the distribution of NIPC and APF materials, which in effect determined which messages were actually delivered to prescribers and consumers. As described below, Endo projected that it would be able to reach tens of thousands of prescribers nationwide through the distribution of NIPC materials.

507. From 2007 until at least 2011, Endo also meticulously tracked the distribution of NIPC materials, demonstrating Endo's commercial interest in, and access to, NIPC's reach. Endo

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knew exactly how many participants viewed NIPC webinars and workshops and visited its website, Painknowledge.com. Endo not only knew how many people viewed NIPC's content, but what their backgrounds were (e.g., primary care physicians or neurologists). Endo's access to and detailed understanding of the composition of the audience at these events demonstrates how deeply Endo was involved in NIPC's activities. Moreover, Endo tracked the activities of NIPC—ostensibly a third party—just as it tracked its own commercial activity.

508. Endo worked diligently to ensure that the NIPC materials it helped to develop would have the broadest possible distribution. Endo's 2008 to 2012 Opana Brand Tactical Plan indicates that it sought to reach 1,000 prescribers in 2008 through live NIPC events, and also to "[l]everage live programs via enduring materials and web posting." Endo also planned to disseminate NIPC's work by distributing two accredited newsletters to 60,000 doctors nationwide for continuing education credit and by sponsoring a series of 18 NIPC regional case-based interactive workshops. Endo had earmarked more than one million dollars for NIPC activities in 2008 alone.

509. In short, NIPC was a key piece of Endo's marketing strategy. Indeed, internal APF emails question whether it was worthwhile for APF to continue operating NIPC given that NIPC's work was producing far more financial benefits for Endo than for APF. Specifically, after Endo approved a $244,337.40 grant request to APF to fund a series of NIPC eNewsletters, APF personnel viewed it as "[g]reat news," but cautioned that "the more I think about this whole thing, [Endo's] making a lot of money on this with still pretty slender margins on [APF's] end." APF's commitment to NIPC's "educational" mission did not figure at all in APF's consideration of the value of its work, nor was Endo's motive or benefit in doubt.

a) Misleading Medical Education

510. NIPC distributed a series of eNewsletter CMEs focused on "key topic[s] surrounding the use of opioid therapy" sponsored by Endo. These newsletters were edited by KOL Dr. Fine and

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listed several industry-backed KOLs, including Dr. Webster, as individual authors. Endo estimated that roughly 60,000 prescribers viewed each one. These CMEs were available to, and would have been accessed by, County prescribers. Before-and-after surveys, summarized in the chart below, showed that prescriber comfort with prescribing opioids ranged from 27% to 62% before exposure to the CME, and from 76% to 92% afterwards:

| Topic | Comfort level *prior to reading the article* | Comfort level *after reading the article* |
|---|---|---|
| Patient Selection and Initiation of Opioid Therapy as a Component of Pain Treatment | 47% | 87% |
| Informed Consent and Management Plans to Optimize Opioid Therapy for Chronic Pain | 48% | 81% |
| Risk Stratification and Evaluation of High-Risk Behaviors for Chronic Opioid Therapy | 28% | 76% |
| Integration of Nonpharmacologic and Multidisciplinary Therapies Into the Opioid Treatment Plan | 42% | 85% |
| Addressing Patients' Concerns Associated With Chronic Pain Treatment and Opioid Use | 62% | 92% |
| Opioid Therapy in Patients With a History of Substance Use Disorders | 35% | 85% |
| Urine Drug Testing: An Underused Tool | 54% | 86% |
| Appropriate Documentation of Opioid Therapy: The Emergence of the 4As and Trust and Verify as the Paradigm | 44% | 86% |
| Opioid Rotation | 27% | 92% |
| Discontinuing Opioid Therapy: Developing and Implementing an "Exit Strategy" | 37% | 90% |

511. Endo documents made it clear that the persuasive power of NIPC speakers was directly proportional to their perceived objectivity. Accordingly, Endo personnel directed that, when giving Endo-sponsored talks, NIPC faculty would not appear to be "Endo Speakers." Nevertheless, the two parties understood that Endo and NIPC shared a common "mission to educate physicians" and working "through the APF . . . [wa]s a great way to work out . . .problems that could have been there without the APF's participation and support."

512. The materials made available on and through NIPC included misrepresentations. For example, Endo worked with NIPC to sponsor a series of CMEs titled *Persistent Pain in the Older Patient*

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and *Persistent Pain in the Older Adult*. These CMEs misrepresented the prevalence of addiction by stating that opioids have "possibly less potential for abuse" in elderly patients than in younger patients, even though there is no evidence to support such an assertion. Moreover, whereas withdrawal symptoms are always a factor in discontinuing long-term opioid therapy, *Persistent Pain in the Older Adult* also misleadingly indicated that such symptoms can be avoided entirely by tapering the patient's does by 10-20% per day for ten days. *Persistent Pain in the Older Patient*, for its part, made misleading claims that opioid therapy has been "shown to reduce pain and improve depressive symptoms and cognitive functioning." NIPC webcast these CMEs from its own website, where they were available to, and were intended to reach, County prescribers.

b) *Painknowledge.com*

513. Working with NIPC enabled Endo to make a number of misleading statements through the NIPC's website, *Painknowledge.com*. Endo tracked visitors to *PainKnowledge.com* and used *Painknowledge.com* to broadcast notifications about additional NIPC programming that Endo helped to create.

514. APF made a grant request to Endo to create an online opioid "tool-kit" for NIPC and to promote NIPC's website, *Painknowledge.com*. In so doing, APF made clear that it planned to disseminate Defendants' misleading messaging. The grant request expressly indicated APF's intent to make misleading claims about functionality, noting: "Some of these people [in chronic pain] may be potential candidates for opioid analgesics, which can improve pain, function, and quality of life." Endo provided $747,517 to fund the project.

515. True to APF's word, *Painknowledge.com* misrepresented that opioid therapy for chronic pain would lead to improvements in patients' ability to function. Specifically, in 2009 the website instructed patients and prescribers that, with opioids, a patient's "level of function should improve"

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and that patients "may find [they] are now able to participate in activities of daily living, such as work and hobbies, that [they] were not able to enjoy when [their] pain was worse."

516. *Painknowledge.com* also deceptively minimized the risk of addiction by claiming that "[p]eople who take opioids as prescribed usually do not become addicted." *Painknowledge.com* did not stop there. It deceptively portrayed opioids as safe at high doses and also misleadingly omitted serious risks, including the risks of addiction and death, from its description of the risks associated with the use of opioids to treat chronic pain.

517. Endo was the sole funder of Painknowledge.com, and it continued to provide that funding despite being aware of the website's misleading contents.

c) *Exit Wounds*

518. Finally, Endo also sponsored APF's publication and distribution of *Exit Wounds*, a publication aimed at veterans that also contained a number of misleading statements about the risks, benefits, and superiority of opioids to treat chronic pain. *Exit Wounds* was drafted by Derek Mcginnis." Derek Mcginnis was frequently hired by a consulting Firm, Conrad & Associates LLC, to write pro-opioid marketing pieces disguised as science. Derek Mcginnis's work was reviewed and approved by drug company representatives, and he felt compelled to draft pieces that he admits distorted the risks and benefits of chronic opioid therapy in order to meet the demands of his drug company sponsors.

519. *Exit Wounds* is a textbook example of Derek Mcginnis's authorship on drug companies' behalf. The book misrepresented the functional benefits of opioids by stating that opioid medications "*increase* your level of functioning" (emphasis in original).

520. *Exit Wounds* also misrepresented that the risk of addiction associated with the use of opioids to treat chronic pain was low. It claimed that "[l]ong experience with opioids shows that

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people who are not predisposed to addiction are very unlikely to become addicted to opioid pain medications."

521. Finally, *Exit Wounds* misrepresented the safety profile of using opioids to treat chronic pain by omitting key risks associated with their use. Specifically, it omitted warnings of the risk of interactions between opioids and benzodiazepines—a warning sufficiently important to be included on Endo's FDA-required labels. *Exit Wounds* also contained a lengthy discussion of the dangers of using alcohol to treat chronic pain but did not disclose dangers of mixing alcohol and opioids—a particular risk for veterans.

522. As outlined above, Endo exercised dominance over APF and the projects it undertook in an effort to promote the use of opioids to treat chronic pain. In addition, as outlined above, Derek Mcginnis's work was being reviewed and approved by drug company representatives, motivating him to draft pro-opioid propaganda masquerading as science. Combined, these factors gave Endo considerable influence over the work of Derek Mcginnis and over APF. Further, by paying to distribute *Exit Wounds*, Endo endorsed and approved its contents.

ii. *Other Front Groups: FSMB, AAPM, and AGS*

523. In addition to its involvement with APF, Endo worked closely with other third-party Front Groups and KOLs to disseminate deceptive messages regarding the risks, benefits, and superiority of opioids for the treatment of chronic pain. As with certain APF publications, Endo in some instances used its sales force to directly distribute certain publications by these Front Groups and KOLs, making those publications "labeling" within the meaning of 21 C.F.R.§ 1.3(a).

524. In 2007, Endo sponsored FSMB's *Responsible Opioid Prescribing*, which in various ways deceptively portrayed the risks, benefits, and superiority of opioids to treat chronic pain. *Responsible Opioid Prescribing* was drafted by "Dr. Fishman."

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525. Endo spent $246,620 to help FSMB distribute *Responsible Opioid Prescribing*. Endo approved this book for distribution by its sales force. Based on the uniform and nationwide character of Endo's marketing campaign, and the fact that Endo purchased these copies specifically to distribute them, these copies were distributed to physicians nationwide, including physicians in Maverick County.

526. In December 2009, Endo also contracted with AGS to create a CME to promote the 2009 guidelines titled the *Pharmacological Management of Persistent Pain in Older Persons* with a $44,850 donation. These guidelines misleadingly claimed that "the risks [of addiction] are exceedingly low in older patients with no current or past history of substance abuse," as the study supporting this assertion did not analyze addiction rates by age. They also stated, falsely, that "[a]ll patients with moderate to severe pain . . . should be considered for opioid therapy (low quality of evidence, strong recommendation)" when in reality, opioid therapy was only an appropriate treatment for a subset of those patients, as recognized by Endo's FDA-mandated labels.

527. AGS's grant request to Endo made explicit reference to the CME that Endo was funding. Endo thus knew full well what content it was paying to distribute, and was in a position to evaluate that content to ensure it was accurate, substantiated, and balanced before deciding whether or not to invest in it. After having sponsored the AGS CME, Endo's internal documents indicate that Endo's pharmaceutical sales representatives discussed the AGS guidelines with doctors during individual sales visits.

528. Endo also worked with AAPM, which it viewed internally as "Industry Friendly," with Endo advisors and speakers among its active members. Endo attended AAPM conferences, funded its CMEs, and distributed its publications.

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529. A talk written by Endo in 2009 and approved by Endo's Medical Affairs Review Committee,[140] titled *The Role of Opana ER in the Management of Chronic Pain,* includes a slide titled *Use of Opioids is Recommended for Moderate to Severe Chronic Noncancer Pain.* That slide cites the AAPM/APS Guidelines, which contain a number of misstatements and omits their disclaimer regarding the lack of supporting evidence. This talk dangerously misrepresented to doctors the force and utility of the 2009 Guidelines. Furthermore, Endo's internal documents indicate that pharmaceutical sales representatives employed by Endo, Actavis, and Purdue discussed treatment guidelines with doctors during individual sales visits.

iii. *Key Opinion Leaders and Misleading Science*

530. Endo also sought to promote opioids for the treatment of chronic pain through the use of key opinion leaders and biased, misleading science.

531. Endo's 2010 publication plan for Opana ER identified a corporate goal of making Opana ER the second-leading branded product for the treatment of moderate-to-severe chronic pain (after OxyContin). Endo sought to achieve that goal by providing "clinical evidence for the use of Opana ER in chronic low back pain and osteoarthritis," and subsequently successfully had articles on this topic published.[141]

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[140] Although they were given slightly different names by each Defendant, each Defendant employed a committee that could review and approve materials for distribution. These committees included representatives from all relevant departments within Defendants' organizations, including the legal, compliance, medical affairs, and marketing departments. The task of these review committees was to scrutinize the marketing materials Defendants planned to distribute and to ensure that those materials were scientifically accurate and legally sound. Tellingly, these committees were called to review only materials that created a potential compliance issue for the company, an implicit recognition by defendants that they ultimately would be responsible for the content under review.

[141] These studies suffered from the limitations common to the opioid literature—and worse. None of the comparison trials lasted longer than three weeks. Endo also commissioned a six-month, open label trial during which a full quarter of the patients failed to find a stable dose, and 17% of patients discontinued, citing intolerable effects. In open label trials, subjects know which drug they are taking; such trials are not as rigorous as double-blind, controlled studies in which neither the patients nor the examiners know which drugs the patients are taking.

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532. In the years that followed, Endo sponsored articles authored by Endo consultants and Endo employees, which argued that the metabolic pathways utilized by Opana ER, compared with other opioids, were less likely to result in drug interactions in elderly low back and osteoarthritis pain patients. In 2010, Endo directed its publication manager to reach out to a list of consultants conducting an ongoing Endo-funded study, to assess their willingness to respond to an article[142] that Endo believed emphasized the risk of death from opioids, "without [] fair balance."[143]

533. Endo's reliance on flawed, biased research is also evident in its 2012 marketing materials and strategic plans. A 2012 Opana ER slide deck for Endo's speakers bureaus—on which these recruited physician speakers were trained and to which they were required to adhere—misrepresented that the drug had low abuse potential and suggested that as many as one-quarter of the adult population could be candidates for opioid therapy. Although the FDA requires such speaker slide decks to reflect a "fair balance" of information on benefits and risks, Endo's slides reflected one-sided and deeply biased information. The presentation's 28 literature citations were largely to "data on file" with the company, posters, and research funded by, or otherwise connected to, Endo. Endo's speakers relayed the information in these slides to audiences that were unaware of the skewed science on which the information was based.

534. A 2012 Opana ER Strategic Platform Review suffered from similar defects. Only a small number of the endnote referenced in the document, which it cited to indicate "no gap" in scientific evidence for particular claims, were to national-level journals. Many were published in lesser or dated journals, and written or directly financially supported by opioid manufacturers. Where the strategy document did cite independent, peer-reviewed research, it did so out of context. For

[142] Susan Okie, *A Flood of Opioids, a Rising Tide of Deaths*, 363 New Engl. J. Med. 1981 (2010), finding that opioid overdose deaths and opioid prescriptions both increased by roughly 10-fold from 1990 to 2007.

[143] Endo did manage to get a letter written by three of those researchers, which was not published.

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example, it cited a 2008 review article on opioid efficacy for several claims, including that "treatment of chronic pain reduces pain and improves functionality," but it ignored the article's overall focus on the lack of consistent effectiveness of opioids in reducing pain and improving functional status.[144]

535. Notwithstanding Endo's reliance upon dubious or cherry-picked science, in an Opana ER brand strategy plan it internally acknowledged the continuing need for a significant investment in clinical data to support comparative effectiveness. Endo also cited a lack of "head-to-head data" as a barrier to greater share acquisition, and the "lack of differentiation data" as a challenge to addressing the "#1 Key Issue" of product differentiation. This acknowledged lack of support did not stop Endo from directing its sales representatives to tell prescribers that its drugs were less likely to be abused or be addictive than other opioids.

536. Endo also worked with various KOLs to disseminate various misleading statements about chronic opioid therapy. For example, Endo distributed a patient education pamphlet edited by KOL Dr. Russell Portenoy titled *Understanding your Pain: Taking Oral Opioid Analgesics*. This pamphlet deceptively minimized the risks of addiction by stating that "[a]ddicts take opioids for other reasons [than pain relief], such as unbearable emotional problems," implying that patients who are taking opioids for pain are not at risk of addiction.

537. *Understanding your Pain: Taking Oral Opioid Analgesics* also misleadingly omitted any description of the increased risks posed by higher doses of opioid medication. Instead, in a Q&A format, the pamphlet asked "[i]f I take the opioid now, will it work later when I really need it?" and responded that "[t]he dose can be increased... [y]ou won't 'run out' of pain relief."

538. Dr. Portenoy received research support, consulting fees, and honoraria from Endo for editing *Understanding Your Pain* and other projects.

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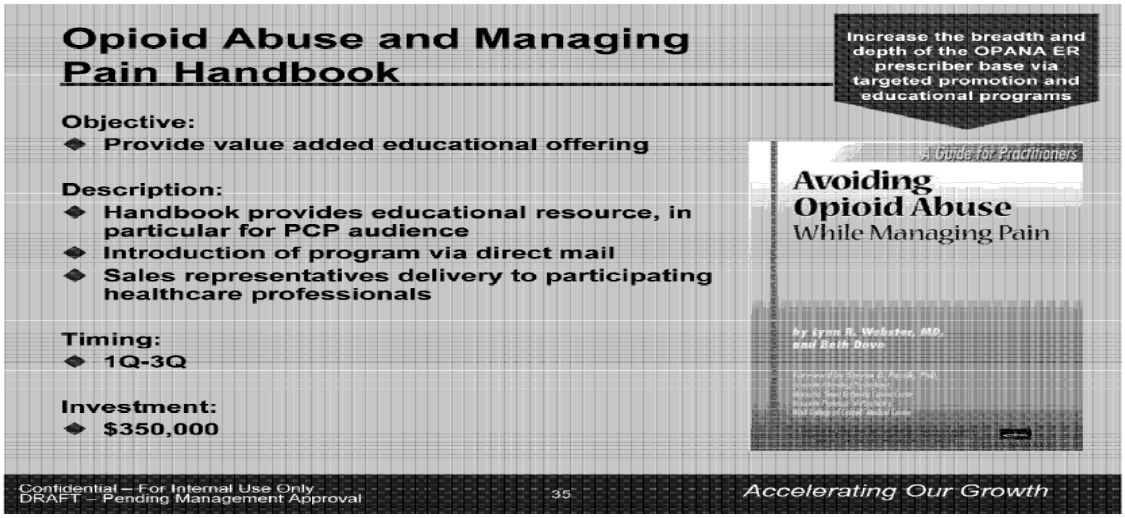
[144] Andrea M. Trescot et al., Opioids in the management of non-cancer pain: an Update of American Society of the Interventional Pain Physicians, Pain Physician 2008 Opioids Special Issue, 11:S5-S62.

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539. *Understanding Your Pain* was available on Endo's website during the time period of this Complaint and was intended to reach County prescribers.

540. Endo similarly distributed a book written by Dr. Lynn Webster titled Avoiding *Opioid Abuse While Managing Pain*, which stated that in the face of signs of aberrant behavior, increasing the dose "in most cases . . . should be the clinician's first response."

541. A slide from an Opana ER business plan contemplated distribution of the book as part of Endo's efforts to "[i]ncrease the breadth and depth of the OPANA ER prescriber base via targeted promotion and educational programs." The slide indicates that the book would be particularly effective "for [the] PCP audience" and instructed "[s]ales representatives [to] deliver[ the book] to participating health care professionals." The slide, shown below, demonstrates Endo's express incorporation of this book by a KOL into its marketing strategy:




542. Endo Documents indicate that, around 2007, the company purchased at least 50,000 copies of the book for distribution. Internal Endo documents Demonstrate that the book had been approved for distribution by Endo's sales force, and that Endo had fewer than 8,000 copies on hand

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in March of 2013. Based on the nationwide and uniform character of Endo's marketing, and the book's approval for distribution, this book was available to and was intended to reach prescribers.

c. Endo's Deceptive Statements to County Prescribers and Patients

543. Endo also directed the dissemination of the misstatements described above to County patients and prescribers, including through its sales force, speakers bureaus, CMEs, and the *Painknowledge.com* website.

544. Consistent with their training, Endo's sales representatives delivered all of these deceptive messages to County prescribers.

545. Endo also directed misleading marketing to County prescribers and patients through the APF/NIPC materials it sponsored, reviewed, and approved. For example, Endo hired a New York-based KOL to deliver a CME titled *Managing Persistent Pain in the Older Patient* on April 27, 2010. As described above, this CME misrepresented the prevalence of addiction in older patients and made misleading claims that chronic opioid therapy would improve patients' ability to function. An email invitation to the event and other NIPC programs was sent to "all healthcare professionals" in APF's database.

546. The significant response to *Painknowledge.com* also indicates that those websites were viewed by County prescribers, who were exposed to the site's misleading information regarding the effect of opioids on patients' ability to function and the deceptive portrayal of the risks of opioids. As of September 14, 2010, *Painknowledge.com* had 10,426 registrants, 86,881 visits, 60,010 visitors, and 364,241 page views. Upon information and belief, based on the site's nationwide availability, among the site's visitors were County patients and prescribers who were exposed to the site's misleading information regarding the effect of opioids on patients' ability to function and the deceptive portrayal of the risks of opioids.

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547. Endo knew that the harms from its deceptive marketing would be felt in Maverick County. It saw workers' compensation programs as a lucrative opportunity, and it promoted the use of opioids for chronic pain arising from work-related injuries, like chronic lower back pain. Endo developed plans to "[d]rive demand for access through the employer audience by highlighting cost of disease and productivity loss in those with pain; [with a] specific focus on high-risk employers and employees." In 2007, Endo planned to reach 5,000 workers' compensation carriers to ensure that Opana ER would be covered under disability insurance plans. Endo knew or should have known that claims for its opioids would be paid for by the County's workers' compensation program.

**4. Janssen**

548. Janssen promoted its branded opioids, including Duragesic, Nucynta, and Nucynta ER, through its sales representatives and a particularly active speakers program. Deceptive messages regarding low addiction risk and low prevalence of withdrawal symptoms were a foundation of this marketing campaign. Janssen also conveyed other misrepresentations including that its opioids could safely be prescribed at higher doses and were safer than alternatives such as NSAIDs.

549. Janssen supplemented these efforts with its own unbranded website, as well as third-party publications and a Front Group website, to promote opioids for the treatment of chronic pain. These materials likewise made deceptive claims about addiction risk, safety at higher doses, and the safety of alternative treatments. They also claimed that opioid treatment would result in functional improvement, and further masked the risk of addiction by promoting the concept of pseudoaddiction.

550. Based on the highly coordinated and uniform nature of Janssen's marketing, Janssen conveyed these deceptive messages to County prescribers. The materials that Janssen generated in collaboration with third-parties also were distributed or made available in Maverick County. Janssen distributed these messages, or facilitated their distribution, in Maverick County with the intent that

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County prescribers and/or consumers would rely on them in choosing to use opioids to treat chronic pain.

a. Janssen's Deceptive Direct Marketing

551. Janssen joined the other Defendants in propagating deceptive branded marketing that falsely minimized the risks and overstated the benefits associated with the long-term use of opioids to treat chronic pain. Like the other Defendants, Janssen sales representatives visited targeted physicians to deliver sales messages that were developed centrally and deployed identically across the country. These sales representatives were critical in transmitting Janssen's marketing strategies and talking points to individual prescribers. In 2011, at the peak of its effort to promote Nucynta ER, Janssen spent more than $90 million on detailing.

552. Janssen's designs to increase sales through deceptive marketing are apparent on the face of its marketing plans. For example, although Janssen knew that there was no credible scientific evidence establishing that addiction rates were low among patients who used opioids to treat chronic pain, its Nucynta Business Plans indicated that one of the "drivers" to sell more Nucynta among primary care physicians was the "[l]ow perceived addiction and/or abuse potential" associated with the drug. However, there is no evidence that Nucynta is any less addictive or prone to abuse than other opioids, or that the risk of addiction or abuse is low. Similarly, Janssen knew that there were severe symptoms associated with opioid withdrawal including, severe anxiety, nausea, vomiting, hallucinations, and delirium, but Janssen touted the ease with which patients could come off opioids.

i. *Janssen's Deceptive Sales Training*

553. Janssen's sales force was compensated based on the number of Nucynta prescriptions written in each sales representative's territory. Janssen encouraged these sales representatives to maximize sales of Nucynta and meet their sales targets by relying on the false and misleading statements described above.

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554. For example, Janssen's sales force was trained to trivialize addiction risk. A June 2009 Nucynta training module warns that physicians are reluctant to prescribe controlled substances like Nuycnta because of their fear of addicting patients, but this reluctance is unfounded because "the risks . . . are [actually] much smaller than commonly believed." Janssen also encouraged its sales force to misrepresent the prevalence of withdrawal symptoms associated with Nucynta. A Janssen sales training PowerPoint titled "Selling Nucynta ER and Nucynta" indicates that the "low incidence of opioid withdrawal symptoms" is a "core message" for its sales force. The message was touted at Janssen's Pain District Hub Meetings, in which Janssen periodically gathered its sales force personnel to discuss sales strategy.

555. This "core message" of a lack of withdrawal symptoms runs throughout Janssen's sales training materials. For example, Janssen's "Licensed to Sell" Facilitator's Guide instructs those conducting Janssen sales trainings to evaluate trainees, in part, on whether they remembered that "[w]ithdrawal symptoms after abrupt cessation of treatment with NUCYNTA ER were mild or moderate in nature, occurring in 11.8% and 2% of patients, respectively" and whether they were able to "accurately convey" this "core message." Janssen further claimed in 2008 that "low incidence of opioid withdrawal symptoms" was an advantage of the tapentadol molecule.

556. Similarly, a Nucynta Clinical Studies Facilitator's Guide instructs individuals training Janssen's sales representatives to ask trainees to describe a "key point"—that "83% of patients reported no withdrawal symptoms after abruptly stopping treatment without initiating alternative therapy"—"as though he/she is discussing it with a physician."

557. This misrepresentation regarding withdrawal was one of the key messages Janssen imparted to employees in the "Retail ST 101 Training" delivered to Nucynta sales representatives.

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558. Indeed, training modules between 2009 and 2011 instruct training attendees that "most patients [who discontinued taking Nucynta] experienced no withdrawal symptoms" and "[n]o patients experienced moderately severe or severe withdrawal symptoms."

559. During the very time Janssen was instructing its sales force to trivialize the risks of addiction and withdrawal associated with the use of Nucynta to treat chronic pain, it knew or should have known, that significant numbers of patients using opioids to treat chronic pain experienced issues with addiction. Janssen knew or should have known that its studies on withdrawal were flawed and created a misleading impression of the rate of withdrawal symptoms and, as a result, the risk of addiction.

560. The misleading messages and materials Janssen provided to its sales force were part of a broader strategy to convince prescribers to use opioids to treat their patients' pain, irrespective of the risks, benefits, and alternatives. This deception was national in scope and included Maverick County. Janssen's nationwide messages reached County prescribers in a number of ways, including through its sales force in detailing visits, as well as through websites and ads. They were also delivered to County prescribers by Janssen's paid speakers, who were required by Janssen policy and by FDA regulations to stay true to Janssen's nationwide messaging.

ii. *Janssen's Deceptive Speakers Bureau Programs*

561. Janssen did not stop at disseminating its misleading messages regarding chronic opioid therapy through its sales force. It also hired speakers to promote its drugs and trained them to make the very same misrepresentations made by its sales representatives.

562. Janssen's speakers worked from slide decks—which they were required to present—reflecting the deceptive information about the risks, benefits, and superiority of opioids outlined above. For example, a March 2011 speaker's presentation titled *A New Perspective For Moderate to Severe Acute Pain Relief: A Focus on the Balance of Efficacy and Tolerability* set out the following adverse events

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associated with use of Nucynta: nausea, vomiting, constipation, diarrhea, dizziness, headache, anxiety, restlessness, insomnia, myalgia, and bone pain. It completely omitted the risks of misuse, abuse, addiction, hyperalgesia, hormonal dysfunction, decline in immune function, mental clouding, confusion, and other known, serious risks associated with chronic opioid therapy. The presentation also minimized the risks of withdrawal by stating that "more than 82% of subjects treated with tapentadol IR reported no opioid withdrawal symptoms."

563. An August 2011 speaker presentation titled *New Perspectives in the Management of Moderate to Severe Chronic Pain* contained the same misleading discussion of the risks associated with chronic opioid therapy. It similarly minimized the risks of withdrawal by reporting that 86% of patients who stopped taking Nucynta ER "abruptly without initiating alternative opioid therapy" reported no withdrawal symptoms whatsoever. The same deceptive claims regarding risks of adverse events and withdrawal appeared in a July 2012 speaker's presentation titled *Powerful Pain Management: Proven Across Multiple Acute and Chronic Pain Models*.

564. These speakers presentations were part of Janssen's nationwide marketing efforts. Upon information and belief, a number of these events were available to and were intended to reach Maverick County prescribers.

iii. *Janssen's Deceptive Unbranded Advertising*

565. Janssen was aware that its branded advertisements and speakers programs would face regulatory scrutiny that would not apply to its unbranded materials, so Janssen also engaged in direct, unbranded marketing.

566. One such unbranded project was Janssen's creation and maintenance of *Prescriberesponsibly.com* (last updated July 2, 2015), a website aimed at prescribers and patients that claims that concerns about opioid addiction are "overstated." A disclaimer at the bottom of the website states

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that the "site is published by Janssen Pharmaceuticals, Inc., which is solely responsible for its content." This website was available to and intended to reach County prescribers and patients.

b. Janssen's Deceptive Third-Party Statements

567. Janssen's efforts were not limited to directly making misrepresentations through its sales force, speakers' bureau, and website. To avoid regulatory constraints and give its efforts an appearance of independence and objectivity, Janssen obscured its involvement in certain marketing activities by "collaborat[ing] with key patient advocacy organizations" to release misleading information about opioids.

i. *AAPM and AGS – Finding Relief: Pain Management for Older Adults*

568. Janssen worked with AAPM and AGS to create a patient education guide entitled *Finding Relief: Pain Management for Older Adults* (2009). In doing so, Janssen contracted with a medical publishing firm, Conrad & Associates, LLC. The content was drafted by a writer ("Medical Writer X") hired by Conrad & Associates and funded by Janssen. These materials were reviewed, in detail, by Janssen's medical-legal review team, which conducted detailed reviews and gave him editorial feedback on his drafts, which was adopted in the published version.

569. Medical Writer X understood, without being explicitly told, that since his work was funded and reviewed by Janssen, the materials he was writing should aim to promote the sale of more drugs by overcoming the reluctance to prescribe or use opioids to treat chronic pain. He knew that the publication was undertaken in connection with the launch of a new drug and was part of its promotional effort. Medical Writer X knew of the drug company's sponsorship of the publication, and he would go to the company's website to learn about the drug being promoted. He also knew that his clients—including Janssen—would be most satisfied with his work if he emphasized that: (a) even when used long-term, opioids are safe and the risk of addiction is low; (b) opioids are effective

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for chronic pain; and (c) opioids are under-prescribed because doctors are hesitant, confused, or face other barriers.[145]

570. *Finding Relief* is rife with the deceptive content. *Finding Relief* misrepresents that opioids increase function by featuring a man playing golf on the cover and listing examples of expected functional improvement from opioids, like sleeping through the night, returning to work, recreation, sex, walking, and climbing stairs. The guide states as a "fact" that "opioids may make it *easier* for people to live normally" (emphasis in the original). The functional claims contained in *Finding Relief* are textbook examples of Defendants' use of third parties to disseminate messages the FDA would not allow them to say themselves. Compare, e.g.:

| **Branded Advertisement That Triggers an FDA Warning Letter (2008)[146]** |
|---|
| Improvement in Daily Activities Includes:<br>• Walking on a flat surface<br>• Standing or sitting<br>• Climbing stairs<br>• Getting in and out of bed or bath<br>• Ability to perform domestic duties |

with:

[145] Medical Writer X now acknowledges that the lists of adverse effects from chronic opioid use in the publications he authored, which excluded respiratory depression, overdose, and death and minimized addiction, were, "ridiculous" and "prime examples" of leaving out facts that the pharmaceutical company sponsors and KOLs knew at the time were true. His writings repeatedly described the risk of addiction as low. Medical Writer X stated that he understood that the goal was to promote opioids and, as a result, discussing addiction would be "counterproductive."

[146] This advertisement drew an FDA Warning Letter dated March 24, 2008. Though the advertisement was by drug company King, it is used here to demonstrate the types of claims that the FDA regarded as unsupported.

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| **Seemingly Independent Publication: "Finding Relief: Pain Management for Older Adults" (Final Authority, Janssen 2009):** |
| --- |
| Your recovery will be measured by how well you reach functional goals such as<br>• Sleeping without waking from pain<br>• Walking more, or with less pain<br>• Climbing stairs with less pain<br>• Returning to work<br>• Enjoying recreational activities<br>• Having sex<br>• Sleeping in your own bed |

571. *Finding Relief* also trivialized the risks of addiction describing as a "myth" that opioids are addictive, and asserting as fact that "[m]any studies show that opioids are *rarely* addictive when used properly for the management of chronic pain."

572. *Finding Relief* further misrepresented that opioids were safe at high doses by listing dose limitations as "disadvantages" of other pain medicines and omitting any discussion of risks from increased doses of opioids. The publication also falsely claimed that it is a "myth" that "opioid doses have to be bigger over time."

573. Finally, *Finding Relief* deceptively overstated the risks associated with alternative forms of treatment. It juxtaposed the advantages and disadvantages of NSAIDs on one page, with the "myths/facts" of opioids on the facing page. The disadvantages of NSAIDs are described as involving "stomach upset or bleeding," "kidney or liver damage if taken at high doses or for a long time," "adverse reactions in people with asthma," and "increase[d] . . .risk of heart attack and stroke." Conversely, the only adverse effects of opioids listed by *Finding Relief* are "upset stomach or sleepiness," which the brochure claims will go away, and constipation. The guide never mentions addiction, overdose, abuse, or other serious side effects of opioids.

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574. Janssen was not merely a passive sponsor of *Finding Relief*. Instead, Janssen exercised control over its content and provided substantial assistance to AGS and AAPM to distribute it. A "Copy Review Approval Form" dated October 22, 2008 indicates that key personnel from Janssen's Advertising & Promotion, Legal, Health Care Compliance, Medical Affairs, Medical Communications, and Regulatory Departments reviewed and approved *Finding Relief*. All six Janssen personnel approving the publication checked the box on the approval form indicating that *Finding Relief* was "Approved With Changes." After the publication was modified at the behest of Janssen personnel, Janssen paid to have its sales force distribute 50,000 copies of *Finding Relief* throughout the nation. Thus, *Finding Relief* is considered labeling for Janssen's opioids within the meaning of 21 C.F.R. § 1.3(a).

575. AAPM purchased and distributed copies of *Finding Relief* to all of its members, including those who reside in Maverick County.

576. *Finding Relief's* author, Medical Writer X, later said it was clear, from his position at the intersection of science and marketing, that the money paid by drug companies to the KOLs and professional and patient organizations with which he worked, distorted the information provided to doctors and patients regarding opioids. The money behind these and many other "educational" efforts also, he believes, led to a widespread lack of skepticism on the part of leading physicians about the hazards of opioids. It also led these physicians to accept, without adequate scrutiny, published studies that, while being cited to support the safety of opioids, were, in fact, of such poor methodological quality that they would not normally be accepted as adequate scientific evidence.

ii. *AGS – Misleading Medical Education*

577. Janssen also worked with AGS on another project—AGS's CME promoting the 2009 guidelines for the *Pharmacological Management of Persistent Pain in Older Persons*. These guidelines falsely claimed that "the risks [of addiction] are exceedingly low in older patients with no current or past

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history of substance abuse" although the study supporting this assertion did not analyze addiction rates by age. They also stated falsely, that "[a]ll patients with moderate to severe pain . . . should be considered for opioid therapy (low quality of evidence, strong recommendation)." Based on Janssen's control over AGS's *Finding Relief*, Janssen also would have exercised control over this project as well.

iii. *APF*

578. Janssen also worked with APF to carry out its deceptive marketing campaign. Documents obtained from one of Janssen's public relations firms, Ketchum, indicate that Janssen and the firm enlisted APF as part of an effort to "draft media materials and execute [a] launch plan" for Janssen's drugs at an upcoming meeting of the AAPM. Janssen also drew on APF publications to corroborate claims in its own marketing materials and its sales training. Janssen personnel participated in a March 2011 call with APF's "Corporate Roundtable," in which they worked with APF and drug company personnel to develop strategies to promote chronic opioid therapy. APF personnel spoke with Janssen employees who "shar[ed] expertise from within their company for [a] public awareness campaign."

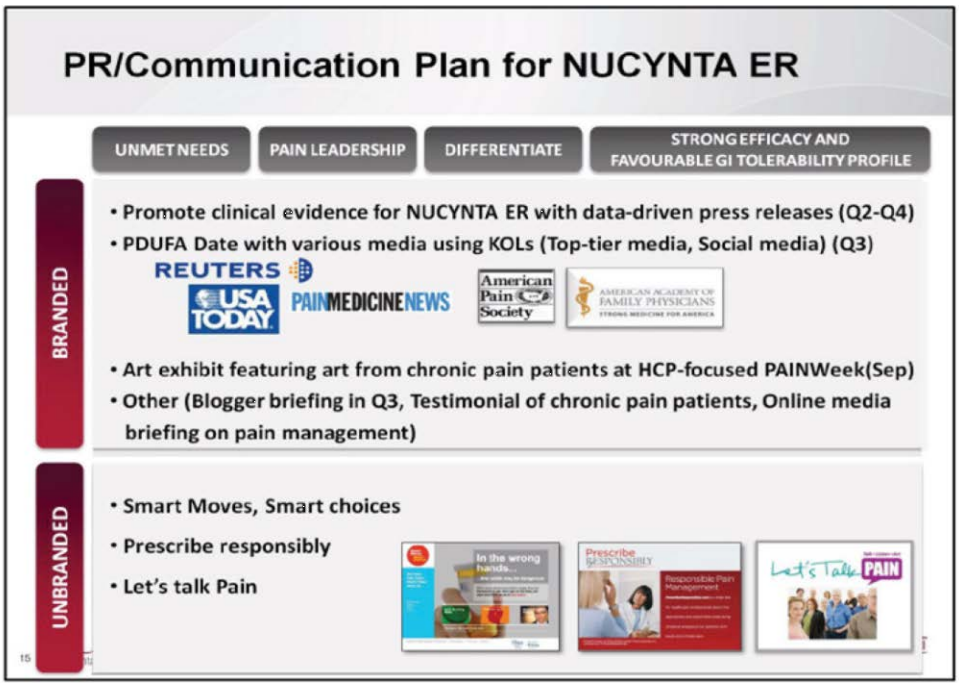
579. Their joint work on the "Corporate Roundtable" demonstrates the close collaboration between Janssen and APF in promoting opioids for the treatment of chronic pain. APF President Will Rowe also reached out to Defendants—including Janssen— rather than his own staff, to identify potential authors to answer a 2011 article critical of opioids that had been published in the Archives of Internal Medicine. Additional examples of APF's collaboration with Janssen are laid out below:

a) Let's Talk Pain

580. Most prominent among these efforts was the *Let's Talk Pain* website. Janssen sponsored *Let's Talk Pain* in 2009, acting in conjunction with APF, American Academy of Pain Management, and American Society of Pain Management Nursing. Janssen financed and orchestrated the participation of these groups in the website.

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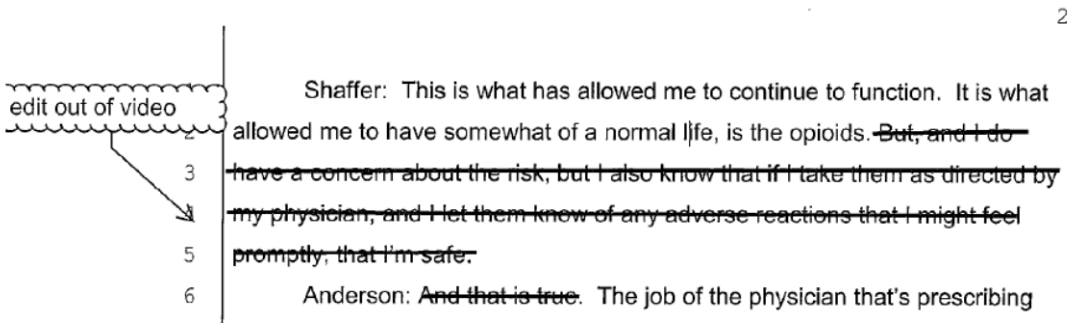
581. Janssen exercised substantial control over the content of the *Let's Talk Pain* website. Janssen's internal communications always referred to *Let's Talk Pain* as promoting tapentadol, the molecule it sold as Nucynta and Nucynta ER. Janssen regarded *Let's Talk Pain* and another website—*Prescriberesponsibly.com*— as integral parts of Nucynta's launch:




582. Janssen documents also reveal that Janssen personnel viewed APF and AAPM as "coalition members" in the fight to increase market share.

583. To this end, Janssen and APF entered into a partnership to "keep pain and the importance of responsible pain management top of mind" among prescribers and patients. They agreed to work to reach "target audiences" that included patients, pain management physicians, primary care physicians, and KOLs. One of the roles Janssen assumed in the process was to "[r]eview, provide counsel on, and approve materials." Janssen did in fact review and approve material for the *Let's Talk Pain* website, as evidenced by the following edits by a Janssen executive to the transcript of a video that was to appear on the site:

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584. The final version of the video on *Let's Talk Pain* omitted the stricken language above.

585. This review and approval authority extended to the *Let's Talk Pain* website. Emails between Janssen personnel and a consultant indicate that, even though the *Let's Talk Pain* website was hosted by APF, Janssen had approval rights over its content. Moreover, emails describing Janssen's review and approval rights related to *Let's Talk Pain* indicate that this right extended to "major changes and video additions."

586. As a 2009 Janssen memo conceded, "[t]he *Let's Talk Pain Coalition* is sponsored by PriCara, a Division of Ortho-McNeil-Janssen Pharmaceuticals, Inc." and "[t]he Coalition and Pricara **maintain editorial control of all *Let's Talk Pain* materials and publications**" (emphasis added).

587. A 2011 Consulting Agreement between Janssen and one of APF's employees, relating to the dissemination of national survey data, demonstrates the near-total control Janssen was empowered to exercise over APF in connection with the *Let's Talk Pain* website, including requiring APF to circulate and post Janssen's promotional content. The agreement required APF to "participate in status calls between Janssen, APF, AAPM, ASPMN, and Ketchum as requested by Janssen" and required APF to "respond to requests to schedule status calls **within 48 hours** of the request" (emphasis in original). APF also was required to "[r]eview and provide feedback to media materials, including a press release, pitch email, a key messages document, and social media messages, **within one week** of receipt" (emphasis in original).

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588. The agreement further required APF to provide a summary of the survey results in APF's PAIN MONITOR e-newsletter, post a link to the survey results on APF's Facebook page, send out tweets related to the survey, serve as a spokesperson available for media interviews, "[s]hare information with any media contacts with whom APF has existing relationships to promote the announcement of the national survey findings," identify at least two patient spokespersons to talk about the survey data, and include the survey results in "any future APF materials, as appropriate." Tellingly, "any ideas made or conceived by [APF] in connection with or during the performance" of the Agreement "shall be the property of, and belong to, [Janssen]."

589. Janssen also exercised its control over *Let's Talk Pain*. Janssen was able to update the *Let's Talk Pain* website to describe its corporate restructuring and Janssen personnel asserted their control over "video additions" by reviewing and editing the interview touting the functional benefits of opioids. Given its editorial control over the content of *Let's Talk Pain*, Janssen was, at all times, fully aware of—and fully involved in shaping—the website's content.[147]

590. Let's Talk Pain contained a number of misrepresentations.

591. For example, *Let's Talk Pain* misrepresented that the use of opioids for the treatment of chronic pain would lead to patients regaining functionality. Let's Talk Pain featured an interview claiming that opioids were what allowed a patient to "continue to function."

592. In 2009, *Let's Talk Pain* also promoted the concept of "pseudoaddiction," which it described as patient behaviors that may occur when pain is under-treated" but differs "from true addiction because such behaviors can be resolved with effective pain management" (emphasis added). *Let's Talk Pain* was available to, and was intended to, reach Maverick County patients and prescribers.

b) Exit Wounds

[147] It bears noting that Janssen does not publicly identify its role in creating *Let's Talk Pain*'s content. Instead, *Let's Talk Pain* represents that "coalition members" develop the content that appears on the website and lists Janssen as the only sponsor of that coalition.

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593. Janssen also engaged in other promotional projects with and through APF. One such project was the publication and distribution of *Exit Wounds*, which, as described above, deceptively portrayed the risks, benefits, and superiority of opioids to treat chronic pain. *Exit Wounds* was drafted by "Medical Writer X." It is fully representative of his work on behalf of drug companies.

594. Janssen gave APF substantial assistance in distributing *Exit Wounds* in Maverick County and throughout the nation by providing grant money and other resources.

c. Janssen's Deceptive Statements to Maverick County Prescribers and Patients

595. Janssen also directed the misstatements described above to Maverick County patients and prescribers, including through CMEs, its sales force, and recruited physician speakers.

i. *Janssen's Deceptive Medical Education Programs in Maverick County*

596. Janssen sponsored CMEs and talks attended by County prescribers.

ii. *Janssen's Deceptive Detailing Practices in Maverick County*

597. The experiences of specific prescribers confirm both that Janssen's national marketing campaign included the misrepresentations, and that the company disseminated these same misrepresentations to Maverick County prescribers and consumers. In particular, these prescriber accounts reflect that Janssen detailers claimed that Nucynta was "not an opioid" because it worked on an "alternate receptor";[148] claimed that Janssen's drugs would be less problematic for patients because they had anti-abuse properties and were "steady state"; claimed that patients on Janssen's drugs were less susceptible to withdrawal; omitted or minimized the risk of opioid addiction; claimed or implied that opioids were safer than NSAIDs; and overstated the benefits of opioids, including by making claims of improved function.

[148] The FDA-approved labels for both Nucynta and Nucynta ER describe the tapentadol molecule as an "opioid agonist and a Schedule II controlled substance that can be abused in a manner similar to other opioid agonists, legal or illicit."

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**5. Purdue**

598. Purdue promoted its branded opioids—principally, Oxycontin, Butrans, and Hysingla—and opioids generally in a campaign that consistently mischaracterized the risk of addiction and made deceptive claims about functional improvement. Purdue did this through its sales force, branded advertisements, promotional materials, and speakers, as well as a host of materials produced by its third-party partners, most prominently APF. Purdue's sales representatives and advertising also misleadingly implied that OxyContin provides a full 12 hours of pain relief, and its allied Front Groups and KOLs conveyed the additional deceptive messages about opioids' safety at higher doses, the safety of alternative therapies, and the effectiveness of addiction screening tools.

599. Based on the highly coordinated and uniform nature of Purdue's marketing, Purdue conveyed these deceptive messages to Maverick County prescribers. The materials that Purdue generated in collaboration with third parties also were distributed or made available in Maverick County. Purdue distributed these messages, or facilitated their distribution, in Maverick County with the intent that Maverick County prescribers and/or consumers would rely on them in choosing to use opioids to treat chronic pain.

a. Purdue's Deceptive Direct Marketing

600. Like the other Defendants, Purdue directly disseminated deceptive branded and unbranded marketing focused on minimizing the risks associated with the long-term use of opioids to treat chronic pain. Purdue directed these messages to prescribers and consumers through its sales force and branded advertisements.

601. Purdue engaged in in-person marketing to doctors in Maverick County. Purdue had 250 sales representatives in 2007, of whom 150 were devoted to promoting sales of OxyContin full time. Like the other Defendants' detailers, Purdue sales representatives visited targeted physicians to deliver sales messages that were developed centrally and deployed, identically, across the country.

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These sales representatives were critical in delivering Purdue's marketing strategies and talking points to individual prescribers.[149] Indeed, Endo's internal documents indicate that pharmaceutical sales representatives employed by Endo, Actavis, and Purdue discussed the AAPM/APS Guidelines, which as discussed above deceptively concluded that the risk of addiction is manageable for patients regardless of past abuse histories, with doctors during individual sales visits.

602. Purdue's spending on detailing reached its nadir in 2006 and 2007, as the company faced civil and criminal charges for misbranding OxyContin. Since settling those charges in 2007, however, Purdue has sharply increased its quarterly spending on promotion through its sales force, from under $5 million in 2007 to more than $30 million by the end of 2014.

603. Purdue also marketed its drugs through branded advertisements which relied on, among other deceptive tactics, misleading statements about the efficacy and onset of OxyContin. Purdue marketed its drug as effective for 12 hours while knowing that these claims were misleading because, for many patients, the pain relief lasted for as little as eight hours, leading to end-of-dose failure and withdrawal symptoms. This prompted doctors to prescribe, or patients to take, higher or more frequent doses of opioids, all of which increased the risk of abuse and addiction.

604. For example, a "Conversion and Titration Guide" submitted to the FDA and distributed to physicians by Purdue, prominently referred to "Q12h OxyContin Tablets," meaning that each tablet was intended to "offer . . . every-twelve-hour dosing." Other marketing materials directed at physicians and disseminated across the country in 2006 touted that OxyContin's "12-hour AcroContin Delivery System" was "designed to deliver oxycodone over 12 hours," which offered patients "life with Q12H relief." Those same marketing materials included a timeline graphic with

[149] But Purdue did not stop there. It also tracked around 1,800 doctors whose prescribing patterns demonstrated a probability that they were writing opioid prescriptions for addicts and drug dealers. Purdue kept the program secret for nine years and, when it finally did report information about these suspicious doctors to law enforcement authorities, it only did so with respect to 8% of them.

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little white paper pill cups at "8AM" and, further down the line, at "8PM" only. They also proclaimed that OxyContin provided "Consistent Plasma Levels Over 12 Hours" and set forth charts demonstrating absorption measured on a logarithmic scale, which fraudulently made it appear that levels of oxycodone in the bloodstream slowly taper over a 12-hour time period.

605. Purdue advertisements that ran in 2005 and 2006 issues of the *Journal of Pain* depicted a sample prescription for OxyContin with "Q12h" handwritten. Another advertisement Purdue ran in 2005 in the *Journal of Pain* touted OxyContin's "Q12h dosing convenience" and displayed two paper dosing cups, one labeled "8 am" and one labeled "8 pm," implying that OxyContin is effective for the 12-hour period between 8 a.m. and 8 p.m. Similar ads appeared in the March 2005 *Clinical Journal of Pain*.

606. Purdue continued to include prominent 12-hour dosing instructions in its branded advertising, such as in a 2012 Conversion and Titration Guide, which states: "Because each patient's treatment is personal / Individualize the dose / Q12h OxyContin Tablets."

607. As outlined above, however, these statements are misleading because they fail to make clear that a 12-hour dose does not equate to 12 hours of pain relief. Nevertheless, Purdue's direct marketing materials have misleadingly claimed OxyContin offers 12 hour "dosing convenience."

608. As described below, these deceptive statements regarding the efficacy of OxyContin were also carried into Maverick County by Purdue's detailers.

609. Purdue's direct marketing materials also misrepresented that opioids would help patients regain functionality and make it easier for them to conduct everyday tasks like walking, working, and exercising.

610. For example, in 2012, Purdue disseminated a mailer to doctors titled "Pain vignettes." These "vignettes" consisted of case studies describing patients with pain conditions that persisted over a span of several months. One such patient, "Paul," is described as a "54-year-old writer with

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osteoarthritis of the hands," and the vignettes imply that an OxyContin prescription will help him work. None of these ads, however, disclosed the truth—that there is no evidence that opioids improve patients' lives and ability to function and that there was substantial evidence to the contrary.

611. Some of the greatest weapons in Purdue's arsenal, however, were unbranded materials it directly funded and authored. These were in addition to the unbranded materials, described below, that Purdue channeled through third parties.

612. In 2011, Purdue published a prescriber and law enforcement education pamphlet titled *Providing Relief, Preventing Abuse*, which deceptively portrayed the signs—and therefore the prevalence—of addiction. However, Purdue knew, as described above, that OxyContin was used non-medically by injection less than less than 17% of the time. Yet, *Providing Relief, Preventing Abuse* prominently listed side effects of injection like skin popping and track marks as "Indications of Possible Drug Abuse"—downplaying much more prevalent signs of addiction associated with OxyContin use such as asking for early refills, making it seem as if addiction only occurs when opioids are taken illicitly.

613. *Providing Relief, Preventing Abuse* also deceptively camouflaged the risk of addiction by falsely supporting the idea that drug-seeking behavior could, in fact, be a sign of "pseudoaddiction" rather than addiction itself. Specifically, it noted that the concept of "pseudoaddiction" had "emerged in the literature" to describe "[drug-seeking behaviors] in patients who have pain that has not been effectively treated." Nowhere in *Providing Relief, Preventing Abuse* did Purdue disclose the lack of scientific evidence justifying the concept of "pseudoaddiction," or that the phrase itself had been coined by a Purdue vice president.

614. *Providing Relief, Preventing Abuse* was available nationally and was intended to reach Maverick County prescribers. As described below, the deceptive statements in *Providing Relief,*

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*Preventing Abuse* regarding addiction were the very same messages Purdue directed at Maverick County prescribers through its sales force.

615. Purdue also disseminated misrepresentations through two of its unbranded websites, *In the Face of Pain* and *Partners Against Pain*.

616. Consistent with Purdue's efforts to portray opioid treatment as "essential" for the proper treatment of chronic pain and label skepticism related to chronic opioid therapy as an "inadequate understanding" that leads to "inadequate pain control," *In the Face of Pain* criticized policies that limited access to opioids as being "at odds with best medical practices" and encouraged patients to be "persistent" in finding doctors who will treat their pain. This was meant to imply that patients should keep looking until they find a doctor willing to prescribe opioids.

617. *In the Face of Pain* was available nationally and was intended to reach Maverick County prescribers.

618. Purdue also used its unbranded website *Partners Against Pain* to promote the same deceptive messages regarding risk of addiction and delivered by its sales representatives. On this website, Purdue posted *Clinical Issues in Opioid Prescribing*, a pamphlet that was copyrighted in 2005. Purdue also distributed a hard-copy version of this pamphlet. *Clinical Issues in Opioid Prescribing* claimed that "illicit drug use and deception" were not indicia of addiction, but rather indications that a patient's pain was undertreated. The publication indicated that "[p]seudoaddiction can be distinguished from true addiction in that the behaviors resolve when the pain is effectively treated." In other words, Purdue suggested that when faced with drug-seeking behavior from their patients, doctors should prescribe more opioids—turning evidence of addiction into an excuse to sell and prescribe even more drugs.

619. Purdue's misleading messages and materials were part of a broader strategy to convince prescribers to use opioids to treat their patients' pain, irrespective of the risks, benefits, and

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alternatives. This deception was national in scope and included Maverick County. As described above, Purdue's nationwide messages would have reached County prescribers in a number of ways. For example, they were carried into Maverick County by Purdue's sales representatives during detailing visits as well as made available to Maverick County patients and prescribers through websites and ads, including ads in prominent medical journals. They would have also been delivered to Maverick County prescribers by Purdue's paid speakers, who were required by Purdue policy and by FDA regulations to stay true to Purdue's nationwide messaging.

b. Purdue's Deceptive Third-Party Statements

620. Purdue's efforts were not limited to making misrepresentations through its own sales force and its own branded and unbranded marketing materials. As described above, Purdue knew that regulatory constraints restricted what it could say about its drugs through direct marketing. For this reason, like the other Defendants, Purdue enlisted the help of third parties to release misleading information about opioids. The most prominent of these was APF.

i. *APF*

a) Purdue's Control of APF

621. Purdue exercised considerable control over APF, which published and disseminated many of the most blatant falsehoods regarding chronic opioid therapy. Their relationship, and several of the APF publications, is described in detail below.

622. Purdue exercised its dominance over APF over many projects and years. Purdue was APF's second-biggest donor, with donations totaling $1.7 million. Purdue informed APF that the grant money reflected Purdue's effort to "strategically align its investments in nonprofit organizations that share [its] business interests," making clear that Purdue's funding depended upon APF continuing to support Purdue's business interests. Indeed, Purdue personnel participated in a March 2011 call with APF's "Corporate Roundtable," where they suggested that APF "[s]end ambassadors

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to talk about pain within companies and hospitals." Thus, Purdue suggested what role APF could play that would complement its own marketing efforts. On that call, Purdue personnel also committed to provide APF with a list of "industry state advocates" who could help promote chronic opioid therapy, individuals and groups that, upon information and belief, APF reached out to. Purdue personnel remained in constant contact with their counterparts at APF.

623. This alignment of interests was expressed most forcefully in the fact that Purdue hired APF to provide consulting services on its marketing initiatives. Purdue and APF entered into a "Master Consulting Services" Agreement on September 14, 2011. That agreement gave Purdue substantial rights to control APF's work related to a specific promotional project. Moreover, based on the assignment of particular Purdue "contacts" for each project and APF's periodic reporting on their progress, the agreement enabled Purdue to be regularly aware of the misrepresentations APF was disseminating regarding the use of opioids to treat chronic pain in connection with that project. The agreement gave Purdue—but not APF—the right to end the project (and, thus, APF's funding) for any reason. This agreement demonstrates APF's lack of independence and its willingness to surrender to Purdue's control and commercial interests, which would have carried across all of APF's work.

624. Purdue used this agreement to conduct work with APF on the *Partners Against Pain* website. *Partners Against Pain* is a Purdue-branded site, and Purdue holds the copyright.

625. However, its ability to deploy APF on this project illustrates the degree of control Purdue exercised over APF. In 2011, it hired an APF employee to consult on the *Partners Against Pain* rollout, to orchestrate the media campaign associated with the launch of certain content on the website, and to make public appearances promoting the website along with a celebrity spokesperson. Purdue contemplated paying this consultant $7,500 in fees and expenses for 26 hours of work. Purdue would require this consultant to "to discuss and rehearse the delivery of [Purdue's] campaign

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messages" and Purdue committed that "[m]essage points will be provided to [the] Consultant in advance and discussed on [a planned] call." At all times, decisions regarding the final content on the *Partners Against Pain* website were "at the sole discretion of Purdue."

626. APF also volunteered to supply one of its staff (a medical doctor or a nurse practitioner) to assist Purdue as a consultant and spokesperson for the launch of one of Purdue's opioid-related projects, *Understanding & Coping with Lower Back Pain*, which appeared on *Partners Against Pain*. One of the consultants was APF's paid employee, Mickie Brown. The consultant's services would be provided in return for a $10,000 consulting fee for APF and $1,500 in honoraria for the spokesperson. All documents used by the consultant in her media appearances would be reviewed and approved by individuals working for Purdue. It was not until later that APF worried about "how Purdue sees this program fitting in with our [existing] grant request."

627. Given the financial and reputational incentives associated with assisting Purdue in this project and the direct contractual relationship and editorial oversight, APF personnel were acting under Purdue's control at all relevant times with respect to *Partners Against Pain*.

628. APF acquiesced to Purdue's frequent requests that APF provide "patient representatives" for *Partners against Pain*. Moreover, APF staff and board members and Front Groups ACPA and AAPM, among others (such as Dr. Webster), appear on *Inthefaceofpain.com* as "Voices of Hope"—"champions passionate about making a difference in the lives of people who live with pain" and providing "inspiration and encouragement" to pain patients. APF also contracted with Purdue for a project on back pain in which, among other things, it provided a patient representative who agreed to attend a Purdue-run "media training session."

629. According to an Assurance of Voluntary Compliance ("AVC") entered into between the New York Attorney General and Purdue Pharma on August 19, 2015, *Inthefaceofpain.com* received 251,648 page views between March 2014 and March 2015. With the exception of one document

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linked to the website, *Inthefaceofpain.com* makes no mention of opioid abuse or addiction. Purdue's copyright appears at the bottom of each page of the website, indicating its ownership and control of its content. There is no other indication that 11 of the individuals who provided testimonials on *Inthefaceofpain.com* received payments, according to the AVC, of $231,000 for their participation in speakers programs, advisory meetings and travel costs between 2008 and 2013. The New York Attorney General found Purdue's failure to disclose its financial connections with these individuals had the potential to mislead consumers.

630. Nowhere was Purdue's influence over APF so pronounced as it was with the APF's "Pain Care Forum" ("PCF"). PCF was and continues to be run not by APF, but by Defendant Purdue's in-house lobbyist, Burt Rosen. As described by a former drug company employee, Rosen exercised full control of PCF, telling them "what do do and how to do it." This control allowed him, in turn, to run APF as, in accordance with Rosen's thinking, "PCF was APF, which was Purdue." PCF meets regularly in-person and via teleconference, and shares information through an email listserv.

631. In 2011, APF and another third-party advocacy group, the Center for Practical Bioethics, were considering working together on a project. Having reviewed a draft document provided by the Center for Practical Bioethics, the APF employee cautioned that "this effort will be in cooperation with the efforts of the PCF" and acknowledged that "I know you have reservations about the PCF and pharma involvement, but I do believe working with them and keeping the lines of communications open is important." The Center for Practical Bioethics CEO responded by indicating some confusion about whom to speak with, asking "[i]s Burt Rosen the official leader" and reflecting what other sources have confirmed.

632. In 2007, the PCF Education Subgroup, consisting of drug companies Purdue and Alpharma, and Front Groups APF and ACPA (self-described as "industry-funded" groups),

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developed a plan to address a perceived "lack of coordination" among the industry and pro-opioid professional and patient organizations. PCF members agreed to develop simplified "key" messages" to use for public education purposes. Their messages were reflected in programs like NIPC's *Let's Talk Pain* (put together by Endo and APF), and Purdue's *In the Face of Pain*.

633. When the FDA required drug companies to fund CMEs related to opioid risks in accordance with its 2009 REMS, Purdue, along with these Front Groups, worked through the PCF to ensure that, although it was mandatory for drug companies to fund these CMEs, it would not be mandatory for prescribers to attend them. A survey was circulated among Defendants Endo, Janssen, and Purdue, which predicted that the rates of doctors who would prescribe opioids for chronic pain would fall by 13% if more than four hours of mandatory patient education were required in accordance with the REMS. With a push from PCF, acting under Purdue's direction, the CMEs were not made mandatory for prescribers.

634. APF showed its indebtedness to Purdue and its willingness to serve Purdue's corporate agenda when APF chairman Dr. James N. Campbell testifyied on the company's behalf at a July 2007 hearing before the Senate Judiciary Committee "evaluating the propriety and adequacy of the OxyContin criminal settlement."[150] Despite its ostensible role as a patient advocacy organization, APF was willing to overlook substantial evidence—resulting in the jailing of Purdue executives—that Purdue blatantly, despite its clear knowledge to the contrary, told physicians and patients that OxyContin was "rarely" addictive and less addictive than other opioids. Like Purdue, APF ignored

[150] *Evaluating the Propriety and Adequacy of the Oxycontin Criminal Settlement: Before the S. Comm. On the Judiciary*, 110th Cong. 46-50, 110-116 (2007) (statements of Dr. James Campbell, Chairman, APF), https://www.judiciary.senate.gov/imo/media/doc/Campbell%20Testimony%20073107.pdf (accessed May 30, 2017). Purdue was also able to exert control over APF through its relationships with APF's leadership. Purdue-sponsored KOLs Russell Portenoy and Scott Fishman chaired APF's board. Another APF board member, Perry Fine, also received consulting fees from Purdue. APF board member Lisa Weiss was an employee of a public relations firm that worked for both Purdue and APF. Weiss, in her dual capacity, helped vet the content of the Purdue-sponsored *Policymaker's Guide*, which is described below.

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the truth about opioids and parroted Purdue's deceptive messaging. Dr. Campbell testified on Purdue's behalf that addiction was a "rare problem" for chronic pain patients and asserted: "[T]he scientific evidence suggests that addiction to opioids prescribed by legitimate chronic non-cancer pain patients without prior histories of substance abuse using the medication as directed is rare. Furthermore, no causal effect has been demonstrated between the marketing of OxyContin and the abuse and diversion of the drug." There was, and is, no scientific support for those statements.

635. APF President Will Rowe reached out to Defendants—including Purdue—rather than his own staff, to identify potential authors to answer a 2011 article critical of opioids that had been published in the Archives of Internal Medicine..

636. Purdue's control over APF shaped, and was demonstrated by specific APF, pro-opioid publications. These publications had no basis in science and were driven (and can only be explained) by the commercial interest of pharmaceutical companies—Purdue chief among them.

b) *A Policymaker's Guide*

637. Purdue provided significant funding to and was involved with APF's creation and dissemination of *A Policymaker's Guide to Understanding Pain & Its Management*, originally published in 2011 and still available online. *A Policymaker's Guide to Understanding Pain & Its Management* misrepresented that there were studies showing that the use of opioids for the long-term treatment of chronic pain could improve patients' ability to function.

638. Specifically, *A Policymaker's Guide to Understanding Pain & Its Management* claimed that "multiple clinical studies" demonstrated that "opioids . . . are effective in improving [d]aily function, [p]sychological health [and] [o]verall health-related quality of life for people with chronic pain" and implied that these studies established that the use of opioids long-term led to functional improvement. The study cited in support of this claim specifically noted that there were no studies

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demonstrating the safety of opioids long-term and noted that "[f]or functional outcomes, the other [studied] analgesics were significantly more effective than were opioids."[151]

639. The *Policymaker's Guide* also misrepresented the risk of addiction. It claimed that pain had generally been "undertreated" due to "[m]isconceptions about opioid addiction" and that "less than 1% of children treated with opioids become addicted."

640. Moreover, the *Policymaker's Guide* attempted to distract doctors from their patients' drug-seeking behavior by labeling it as "pseudoaddiction," which, according to the guide, "describes patient behaviors that may occur when pain is undertreated." Like *Partners Against Pain*, *A Policymaker's Guide* noted that "[p]seudo-addiction can be distinguished from true addiction in that this behavior ceases when pain is effectively treated." The similarity between these messages regarding "pseudoaddiction" highlights the common, concerted effort behind Purdue's deceptive statements.

641. The *Policymaker's Guide* further misrepresented the safety of increasing doses of opioids and deceptively minimized the risk of withdrawal. For example, the *Policymaker's Guide* claimed that "[s]ymptoms of physical dependence" on opioids in long-term patients "can often be ameliorated by gradually decreasing the dose of medication during discontinuation" while omitting the significant hardship that often accompanies cessation of use. Similarly, the *Policymaker's Guide* taught that even indefinite dose escalations are "sometimes necessary" to reach adequate levels of pain relief while completely omitting the safety risks associated with increased doses.

642. Purdue provided substantial monetary assistance toward the creation and dissemination of the *Policymaker's Guide*, providing APF with $26,000 in grant money. APF ultimately disseminated *Policymaker's Guide* on behalf of Defendants, including Purdue. Purdue was not only kept abreast of

[151] Andrea D. Furlan *et al.*, *Opioids for chronic noncancer pain: a meta-analysis of effectiveness and side effects*, 174(11) Can. Med. Ass'n J. 1589 (2006).

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the content of the guide as it was being developed, but, based on the periodic reports APF provided to Purdue regarding its progress on the *Policymaker's Guide*, had editorial input of the contents.

643. The *Policymaker's Guide* was posted online and was available to, and intended to reach Maverick County prescribers and consumers. As described below, the deceptive statements in *Policymaker's Guide* regarding addiction and functionality were the very same messages Purdue directed at Maverick County through its own sales force.

c) *Treatment Options: A Guide for People Living with Pain*

644. Purdue's partnership with APF did not end with the *Policymaker's Guide*. Purdue also substantially assisted APF by sponsoring *Treatment Options: A Guide for People Living with Pain*, starting in 2007. Based on Purdue's control of other APF projects, Purdue also would have exercised control over *Treatment Options*.

645. *Treatment Options* is rife with misrepresentations regarding the safety and efficacy of opioids. For example, *Treatment Options* misrepresents that the long-term use of opioids to treat chronic pain could help patients function in their daily lives by stating that, when used properly, opioids "give [pain patients] a quality of life [they] deserve."

646. Further, as outlined above, *Treatment Options* claims that addiction is rare and that, when it does occur, it involves unauthorized dose escalations, patients who receive opioids from multiple doctors, or theft, painting a narrow and misleading portrait of opioid addiction.

647. *Treatment Options* also promotes the use of opioids to treat long-term chronic pain by denigrating alternate treatments, most particularly NSAIDs. *Treatment Options* notes that NSAIDs can be dangerous at high doses and inflates the number of deaths associated with NSAID use, distinguishing opioids as having less risk. According to *Treatment Options*, NSAIDs are different from opioids because opioids have "no ceiling dose." This lack of ceiling is considered to be beneficial as some patients "need" larger doses of painkillers than they are currently prescribed. *Treatment Options*

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warns that the risks associated with NSAID use increased if NSAIDs are "taken for more than a period of months," but deceptively omits any similar warning about the risks associated with the long-term use of opioids.

648. *Treatment Options* was posted online and remains online today. It was available to and intended to reach Maverick County prescribers and patients. As described below, the deceptive statements in *Treatment Options* regarding addiction and functionality echo the messages Purdue directed at Maverick County through its own sales force. Purdue also engaged in other promotional projects with and through APF. One such project was the publication and distribution of *Exit Wounds*, which, as described above, deceptively portrayed the risks, benefits, and superiority of opioids to treat chronic pain.

649. Purdue provided APF with substantial assistance in distributing *Exit Wounds* in Maverick County and throughout the nation by providing grant money and other resources.

ii. *Purdue's Work with Other Third Party Front Groups and KOLs*

650. Purdue also provided other third-party Front Groups with substantial assistance in issuing misleading statements regarding the risks, benefits, and superiority of opioids for the long-term treatment of chronic pain.

a) FSMB – *Responsible Opioid Prescribing*

651. In 2007, Purdue sponsored FSMB's *Responsible Opioid Prescribing*, which, as described above, deceptively portrayed the risks, benefits, and superiority of opioids to treat chronic pain. *Responsible Opioid Prescribing* also was drafted by Dr. Scott Fishman.

652. Purdue spent $150,000 to help FSMB distribute *Responsible Opioid Prescribing*. The book was distributed nationally, and was available to and intended to reach prescribers in Maverick County.

b) AGS – *Pharmacological Management of Persistent Pain in Older Persons*

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653. Along with Janssen, Purdue worked with the AGS on a CME to promote the 2009 guidelines for the *Pharmacological Management of Persistent Pain in Older Persons*. As discussed above, these guidelines falsely claimed that "the risks [of addiction] are exceedingly low in older patients with no current or past history of substance abuse" as the study supporting this assertion did not analyze addiction rates by age. They also stated, falsely, that "[a]ll patients with moderate to severe pain should be considered for opioid therapy (low quality of evidence, strong recommendation)."

654. Controversy surrounding earlier versions of AGS guidelines had taught AGS that accepting money directly from drug companies to fund the guidelines' development could lead to allegations of bias and "the appearance of conflict." Accordingly, AGS endeavored to eliminate "the root cause of that flack" by turning down commercial support to produce the 2009 Guidelines. Having determined that its veneer of independence would be tarnished if it accepted drug company money to create the content, AGS decided to develop the guidelines itself and turn to the drug companies for funding to *distribute* the pro-drug company content once it had been created. As explained by AGS personnel, it was AGS's "strategy that we will take commercial support to disseminate [the 2009 Guidelines] if such support is forthcoming." AGS knew that it would be difficult to find such support unless the report was viewed favorably by opioid makers.

655. AGS sought and obtained grants from Endo and Purdue to distribute *Pharmacological Management of Persistent Pain in Older Persons*. As a result, the publication was distributed nationally, and was available to and was intended to reach Maverick County prescribers. Indeed, internal documents of another Defendant, Endo, indicate that pharmaceutical sales representatives employed by Purdue discussed treatment guidelines that minimized the risk of addiction to opioids with doctors during individual sales visits.[152]

[152] As described above, Purdue also provided substantial support for the AAPM/APS guidelines. The 1997 AAPM and APS consensus statement *The Use of Opioids for the Treatment of Chronic Pain* was

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c) *Chronic Pain Management and Opioid Use: Easing Fears, Managing Risks, and Improving Outcomes*

656. Purdue sponsored a 2012 CME program called *Chronic Pain Management and Opioid Use: Easing Fears, Managing Risks, and Improving Outcomes*. The presentation deceptively instructed doctors that, through the use of screening tools, more frequent refills, and other techniques, high-risk patients showing signs of addictive behavior could be treated with opioids. This CME was presented at various locations in the United States and is available online today.

d) *Managing Patient's Opioid Use: Balancing the Need and Risk*

657. Purdue also sponsored a 2011 CME taught by KOL Lynn Webster via webinar titled *Managing Patient's Opioid Use: Balancing the Need and Risk*. This presentation also deceptively instructed prescribers that screening tools, patient agreements, and urine test prevented "overuse of prescriptions" and "overdose deaths." At the time, Dr. Webster was receiving significant funding from Purdue. Versions of Dr. Webster's Opioid Risk Tool appear on, or are linked to, websites run by Purdue (and other Defendants). The webinar was available to and was intended to reach Maverick County prescribers.

e) *Path of the Patient, Managing Chronic Pain in Younger Adults at Risk for Abuse*

658. Purdue also sponsored a CME program entitled *Path of the Patient, Managing Chronic Pain in Younger Adults at Risk for Abuse*. *Path of the Patient* was devoted entirely to the message of treating chronic pain with opioids. Although the program purported to instruct a treating physician how to manage chronic pain in younger adults at risk for abuse, it does no such thing.

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authored by one of its paid speakers, and 14 out of 21 panel members who drafted the AAPM/APS Guidelines received support from Defendants Janssen, Cephalon, Endo, and Purdue.

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659. This "educational" program, addressing treatment of a population known to be particularly susceptible to opioid addiction, presents none of the alternative treatment options available, only discussing treatment of chronic pain with opioids.

660. In a role-play in *Path of the Patient*, a patient who suffers from back pain tells his doctor that he is taking twice as many hydrocodone pills as directed. The doctor reports that the pharmacy called him because of the patient's early refills. The patient has a history of drug and alcohol abuse. Despite these facts, the narrator notes that, because of a condition known as "pseudoaddiction," the doctor should not assume his patient is addicted even if he persistently asks for a specific drug, seems desperate, hoards medicine, or "overindulges in unapproved escalating doses." The doctor in the role-play treats this patient by prescribing a high-dose, long-acting opioid. This CME was available online and was intended to reach County prescribers.

f) *Overview of Management Options*

661. Purdue also sponsored a CME titled *Overview of Management Options* issued by the American Medical Association in 2003, 2007, and 2013 (the latter of which is still available for CME credit). The CME was edited by KOL Russel Portenoy, among others. It deceptively instructs physicians that NSAIDs and other drugs, but not opioids, are unsafe at high doses. In reality, the data indicates that patients on high doses of opioids are more likely to experience adverse outcomes than patients on lower doses of the drugs. Dr. Portenoy received research support, consulting fees, and honoraria from Purdue (among others), and was a paid Purdue consultant. This CME was presented online in the United States and was available to Maverick County prescribers.

iii. *Purdue's Misleading Science*

662. Purdue also misrepresented the risks associated with long-term opioid use by promoting scientific studies in a deceptive way. In 1998, Purdue funded two articles by Dr. Lawrence Robbins, which showed that between 8% and 13% of the patients he studied became addicted to

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opioids—a troubling statistic for Purdue, whose market, and marketing, depended upon the claim that opioids were rarely addictive.[153] Purdue had these articles placed in headache-specific journals where they would be less likely to be encountered by pain specialists or general practitioners. The first of these articles has been cited a mere 16 times; the second does not even appear on Google scholar. Five years later, Purdue funded a study of OxyContin in diabetic neuropathy patients, which was published in 2003. Notwithstanding the fact that that Purdue-funded studies, testing Purdue's own drugs, had previously indicated that addiction rates were between 8% and 13%, Purdue's 2003 article reached back to the 1980 Porter-Jick Letter to support its claim that OxyContin was not commonly addictive. This article was placed in a prominent pain journal and has been cited 487 times.[154] While this article was drafted over a decade ago, it continues to be relied upon to further the misrepresentations that opioids are not addictive.

a) Purdue's Deceptive Statements to Maverick County Prescribers and Patients

663. Purdue directed the dissemination of the misstatements described above to Maverick County patients and prescribers through the Front Groups, KOLs, and publications described above, as well as through its sales force in Maverick County and through advertisements in prominent medical journals. The deceptive statements distributed through each of these channels reflect a common theme of misrepresenting the benefits of Purdue's opioids, unfairly portraying the risks of addiction associated with their use, and deceptively implying that they would improve patients' ability to function.

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[153] Lawrence Robbins, *Long-Acting Opioids for Severe Chronic Daily Headache*, 10(2) Headache Q. 135 (1999); Lawrence Robbins, *Works in Progress: Oxycodone CR, a Long-Acting Opioid, for Severe Chronic Daily Headache*, 19 Headache Q. 305 (1999).

[154] C. Peter N. Watson et al., *Controlled-release oxycodone relieves neuropathic pain: a randomized controlled trial I painful diabetic neuropathy*, 105 Pain 71 (2003).

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664. The deceptive message that OxyContin provided 12 hours of pain relief was not only available to, and intended to, reach Maverick County prescribers through nationally circulated advertising, but was also carried directly into the offices of Maverick County doctors by Purdue's sales representatives.

665. Likewise, the deceptive messages minimizing addiction were not only directed at Maverick County patients and prescribers through the publications circulated above, but were also disseminated directly by Purdue's sales force.

666. Purdue also used its sales force to disseminate misleading statements about the ability of opioids to improve functionality.

667. Purdue's national marketing campaign included the misrepresentations described above and the company disseminated these same misrepresentations to Maverick County prescribers and consumers. In particular, these prescriber accounts reflect that Purdue detailers omitted or minimized the risk of opioid addiction; claimed that Purdue's drugs would be less problematic for patients because they had extended release mechanisms, were tamper proof, and were "steady state"; claimed that OxyContin would provide 12 hours of pain relief; represented that screening tools could help manage the risk of addiction; minimized the symptoms of withdrawal; claimed or implied that opioids were safer than NSAIDs; and overstated the benefits of opioids, including by making claims of improved function.

668. A survey of a sample of physicians, who reported the messages that they retained from detailing visits and other promotional activity, documented that Purdue sales representatives from at least between 2008 and 2012, promoted OxyContin as being effective for a full 12 hours. Purdue sales representatives also promoted OxyContin as improving patients' sleep (an unsubstantiated functional improvement) to an orthopedic surgeon in 2006 and to a physicians' assistant in 2013. Purdue sales representatives also told internists that the reformulation of OxyContin prevented illegal

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drug use and that the formulation was 'less addicting," rather than being harder to adulterate. In 2011, Purdue sales representatives also claimed that the sustained-release property of OxyContin reduced patient "buzz," which is neither based on scientific evidence nor true.

669. The same survey indicated that Purdue sales representatives promoted its Schedule III opioid Butrans as having low or little abuse potential.

**6. Insys**

670. Insys was co-founded in 2002 by Dr. John Kapoor, a serial pharmaceutical industry entrepreneur "known for applying aggressive marketing tactics and sharp price increases on older drugs."[155]

671. In 2012, Insys received U.S. Food and Drug Administration approval for Subsys, a fentanyl sublingual spray product designed to treat breakthrough cancer pain. However, Insys encountered significant obstacles due to insurers employing a process known as prior authorization. Prior authorization prevents the over prescription and abuse of powerful and expensive drugs. The prior authorization process requires "additional approval from an insurer or its pharmacy benefit manager before dispensing…" and may also impose step therapy which requires beneficiaries to first use less expensive medications before moving on to a more expensive approach. [156]

672. Insys circumvented this process by forming a prior authorization unit, known at one point as the Insys Reimbursement Center ("IRC"), to facilitate the process using aggressive and likely

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[155] U.S. senate Homeland Security & Governmental Affairs Committee, *Insys Therapeutics and the Systemic Manipulation of Prior Authorization* (quoting *Fentanyl Billionaire Comes Under Fire as Death Toll Mounts From Prescription Opioids*, Wall Street Journal (Nov. 22, 2016) (www.wsj.com/articles/fentanylbillionaire-comes-under-fire-as-death-toll-mounts-from-prescription-opioids-1479830968)).

[156] Senate Permanent Subcommittee on Investigations, *Combatting the Opioid Epidemic: A Review of Anti-Abuse Efforts in Medicare and Private Health Insurance Systems*; see also Department of Health and Human Services, Centers for Medicare & Medicaid Services, *How Medicare Prescription Drug Plans & Medicare Advantage Plans with Prescription Drug Coverage Use Pharmacies, Formularies, & Common Coverage Rules*

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illegal marketing techniques. Insys published education articles that praised their products' non-addictive nature; and funded patient advocacy groups who unknowingly promoted Insys' agenda of raising the profile of pain so that drugs could be prescribed to treat it. Furthermore, Insys' former sales representatives, motivated by corporate greed, paid off medical practitioners to prescribe Subsys in spite of any medical need.[157] Insys employees were pressured internally and received significant monetary incentives to increase the rate of prescription approvals.[158]

673. According to a federal indictment and ongoing congressional investigation by Sen. Claire McCaskill, IRC employees pretended to be with doctors' offices and falsified medical histories of patients. The report, acquired by McCaskill's investigators, includes transcripts and an audio recording of employees implementing these techniques in order to obtain authorization from insurers and pharmacy benefit managers. The transcript reveals an Insys employee pretending to call on behalf of a doctor and inaccurately describes the patient's medical history.[159] For example, Insys employees would create the impression that the patient had cancer, without explicitly saying so, because cancer was a requirement for prior clearance to prescribe Subsys. Insys was warned by a consultant that it lacked needed policies for governing such activities, but the executives failed to implement corrective internal procedures.

674. In a class action law suit against Insys, it was revealed that management "was ware that only about 10% of prescriptions approved through the Prior Authorization Department were for cancer patients," and an Oregon Department of Justice Investigation found that 78% of

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[157] Lopez, Linette. "It's been a brutal week for the most shameless company in the opioid crisis- and it's about to get worse," *Business Insider,* http://www.businessinsider.com/opioid-addiction-drugmaker-insys-arrests-justice-department-action-2017-7

[158] Boyd, Roddy. *Murder Incorporated: Insys Therapeutics. Part 1.* Southern Investigative Reporting Foundation. http://sirf-online.org/2015/12/03/murder-incorporated-the-insys-therapeutics-story/; see also Indictment. *United States v. Babich, et al.*, D. Mass. (No. 1;16 CR 10343).

[159] U.S. Senate Homeland Security & Governmental Affairs Committee, *Fueling an Epidemic: Insys Therapeutics and the Systematic Manipulation of Prior Authorization, see p. 7-10.*

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preauthorization forms submitted by Insys on behalf of Oregon patients were for off-label uses. [160] Physicians are allowed to prescribe medications for indications outside of FDA guidelines if they see fit, but it is illegal for pharmaceutical companies to market a drug for off-label use.

675. In 2008, biopharmaceutical company Cephalon settled with the U.S. Government for 425 million in a suit against the company that alleged it marketed drugs for unapproved uses (off-label). The FDA approved the drug only for opioid tolerant cancer patients. According to the Oregon settlement and class-action lawsuit, at least three employees involved in sales and/or marketing at Cephalon had moved over to Insys Therapeutics.[161]

676. Additionally, Insys created a "legal speaker program" which turned out to be a scam. The Justice Department commented on the program and stated:

> The Speaker Programs, which were typically held at high-end restaurants, were ostensibly designed to gather licensed healthcare professionals who had the capacity to prescribe Subsys and educate them about the drug. In truth, the events were usually just a gathering of friends and co-workers, most of whom did not have the ability to prescribe Subsys, and no educational component took place. "Speakers" were paid a fee that ranged from $1,000 to several thousand dollars for attending these dinners. At times, the sign-in sheets for the Speaker Programs were forged so as to make it appear that the programs had an appropriate audience of healthcare professionals.

677. Insys paid hundreds of thousands of dollars to doctors in exchange for prescribing Subsys and three top prescribers have already been convicted of taking bribes.

678. Fentanyl products are considered to be the most potent and dangerous opioids on the market and up to 50 times more powerful than heroine.[162]

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[160] Gusovsky, Dina. The Pain Killer: *A drug Company Putting Profits Above Patients,* CNBC (https://www.cnbc.com/2015/11/04/the-deadly-drug-appeal-of-insys-pharmaceuticals.html)
[161] *Id.*
[162] U.S. Department of Justice. Drug Enforcement Administration. *A Real Threat to Law Enforcement: Fentanyl.* https://www.dea.gov/druginfo/DEA%20Targets%20Fentanyl%20%20A%20Real%20Threat%20to%20Law%20Enforcement%20(2016).pdf

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679. In an internal presentation dated 2012 and entitles, "2013 SUBSYS Brand Plan," Insys identified one of six "key strategic imperatives" as "Mitigate Prior Authorization barriers."[163] On a later slide, the company identified several tasks associated with this effort, including "Build internal [prior authorization] assistance infrastructure," "Establish an internal 1-800 reimbursement assistance hotline," and "Educate field force on [prior authorization] process and facilitation."[164]

680. Additional materials produced by Insys to the minority staff suggest, however, that Insys did not match these efforts with sufficient compliance processes to prevent fraud and was internally aware of the danger of problematic practices. Specifically, on February 18, 2014, Compliance Implementation Services (CIS)—a healthcare consultant—issued a draft report to Insys titled, "Insys Call Note, Email, & IRC Verbatim Data Audit Report."[165] The introduction to the report explained that "CIS was approached by INSYS' legal representative … on behalf of the Board of Directors for Insys to request that CIS support in review of certain communications with Health Care Professionals (HCPs) and INSYS employees, and report how there were being documented."[166] Insys had expressed concerns "with respect to communications with HCPs by INSYS employees being professional in nature and in alignment with INSYS approved topics regarding off or on-label promotion of an INSYS product, and general adherence to INSYS documentation requirements."[167] An additional concern "stemmed from the lack of monitoring of commercial activities where these types of interactions could occur."[168]

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[163] U.S. senate Homeland Security & Governmental Affairs Committee, *Insys Therapeutics and the Systemic Manipulation of Prior Authorization* (quoting Insys Therapeutics, Inc., *2013 Subsys Brand Plan, 2012 Assessment* (2012) (INSYS_HSGAC_00007472)).
[164] *Id.* at INSYS_HSGAC_00007765.
[165] U.S. senate Homeland Security & Governmental Affairs Committee, *Insys Therapeutics and the Systemic Manipulation of Prior Authorization* (quoting Compliance Implementation Services, Insys Call Note, Email & IRC Verbatim Data Audit Report (Feb. 18, 2014) (INSYS_HSGAC_00007763)).
[166] *Id.* at INSYS_HSGAC_00007765.
[167] *Id.*
[168] *Id.*

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681. Given these issues, Insys requested that CIS review—in part—"the general communications from the INSYS Reimbursement Center (IRC) to HCPs, their office staff or representatives, as well as health insurance carriers … to ensure they were appropriate in nature with respect to specific uses of SUBSYS, INSYS' commercially marketed product."[169]

682. According to the findings CIS issued, Insys lacked formal policies governing the actions of its prior authorization unit. For example, "[n]o formal and approved policy on appropriate communications between IRC employees and HCPs, their staff, [health care insurers (HCIs)], or patients exists…that governs the support function of obtaining a prior authorization for the use of SUBSYS."[170]

683. In addition, the report noted that "there were also gaps in formally approved foundational policies, procedures, and [standard operating procedures] with respect to required processes specifically within the IRC."[171]

684. In fact, "[t]he majority of managerial directives, changes to controlled documents or templates, as well as updates or revisions to processes were not formally approved, documented, and disseminated for use, and were sent informally via email blast."[172]

685. Although four informal standard operating procedures existed with regarded to IRC functions, these documents "lacked a formal review and approval" and failed to "outline appropriately the actions performed within the IRC."[173]

686. The report also explains that Insys lacked procedures for auditing interactions between IRC employees and outside entities. According to CIS, "no formal, documented, or detailed processes by which IRC representatives' calls via telephone were audited for proper communication

[169] *Id.*
[170] *Id.* at INSYS_HSGAC_00007770.
[171] *Id.* at INSYS_HSGAC_00007768.
[172] *Id.* at INSYS_HSGAC_00007771.
[173] *Id.* at INSYS_HSGAC_00007770.

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with HCPs or HCIs in any fashion [existed] other than random physical review of a call in a very informal and sporadic manner."[174]

687. More broadly, the report notes that "no formal and documented auditing and monitoring or quality control policy, process, or function exists between IRC employee communications and HCPs, HCP staff, HCIs, or patients."[175]

688. At the end of the report, CIS provided a number of recommendations concerning IRC activities. First, CIS suggested that IRC management "formally draft and obtain proper review and approval of an IRC specific policy detailing the appropriate communications that should occur while performing the IRC associate job functions and interacting with HCPs."[176]

689. Similarly, IRC management was urged to formally draft IRC-specific standard operating procedures "specific to each job function within the IRC," accompanied by "adequate training and understanding of these processes."[177] To ensure compliance with IRC standards, Insys was also directed to create an electronic system to allow management "to monitor both live and anonymously IRC employee communications both incoming and outgoing."[178] Finally, CIS recommended that Insys institute a formal process for revising and updating "IRC documentation used for patient and HCP data."[179]

690. The CIS report concluded by noting, in part, that a review of ten conversations between IRC employees and healthcare providers, office staff, and insurance carriers revealed "that

[174] *Id.* at INSYS_HSGAC_00007769.
[175] *Id.* at INSYS_HSGAC_00007771.
[176] *Id.* at INSYS_HSGAC_00007770.
[177] *Id.* at INSYS_HSGAC_00007771.
[178] *Id.*
[179] *Id.*

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all IRC staff was professional in communication, and in no instance was inaccurate or off-label usage of SUBSYS communicated."[180]

691. Yet within a year of this conclusion, according to the recording transcribed below, an Insys IRC employee appears to have misled a PBM representative regarding the IRC employee's affiliation and the diagnosis applicable to Sarah Fuller. The alleged result, in that case, was death due to inappropriate and excessive Subsys prescriptions.

692. One former Insys sales representative described the motto of this approach to patients as "Start them high and hope they don't die."[181]

### F. The Result of Defendants' Fraudulent Scheme

693. Through their direct promotional efforts, along with those of the third-party Front Groups and KOLs they assisted and controlled, and whose seemingly objective materials they distributed, Defendants accomplished exactly what they set out to do: change the institutional and public perception of the risk-benefit assessments and standard of care for treating patients with chronic pain. As a result, Maverick County doctors began prescribing opioids long-term to treat chronic pain—something most would never have considered prior to Defendants' campaign.

694. But for the misleading information disseminated by Defendants, doctors would not, in most instances, have prescribed opioids as medically necessary or reasonably required to address chronic pain.

#### 1. Defendants' Fraudulent and Deceptive Marketing of Opioids Directly Caused Harm to Maverick County.

695. In the first instance, the County was damaged directly, through its payments of false claims for chronic opioid therapy by (a) partially funding a medical insurance plan for its employees and (b) its workers' compensation program.

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[180] *Id.* at INSYS_HSGAC_00007772.

[181] Amended Class Action Complaint, *Larson v. Insys Therapeutics Inc.* (D. Ariz. Oct. 27, 2014.)

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696. Defendants' marketing of opioids caused health care providers to prescribe, and the County, through partially funding a medical insurance plan for its employees and its workers' compensation program, to pay for prescriptions of opioids to treat chronic pain. Because of Defendants' unbranded marketing, health care providers wrote and the County paid for prescriptions of opioids for chronic pain that were filled not only with their drugs, but with opioids sold by other manufacturers. All of these prescriptions were caused by Defendants' fraudulent marketing and therefore all of them constitute false claims. Because, as laid out below, the County is obligated to cover medically necessary and reasonably required care, it had no choice but to pay for these false and fraudulent claims.

697. The fact that the County would pay for these ineligible prescriptions was both the foreseeable and intended consequence of Defendants' fraudulent marketing scheme. Defendants set out to change the medical and general consensus supporting chronic opioid therapy with the intention of encouraging doctors to prescribe, and government payors such as Maverick County, to pay for long-term prescriptions of opioids to treat chronic pain despite the absence of genuine evidence supporting chronic opioid therapy and the contrary evidence regarding the significant risks and limited benefits from long-term use of opioids.

a. Increase in Opioid Prescribing Nationally

698. Defendants' scheme to change the medical consensus regarding opioid therapy for chronic pain was greatly successful. During the year 2000, outpatient retail pharmacies filled 174 million prescriptions for opioids nationwide, rising to 257 million in 2009.[182]

699. Opioid prescriptions increased even as the percentage of patients visiting doctors for pain remained constant. A study of 7.8 million doctor visits between 2000 and 2010 found that

[182] Office of National Drug Control Policy, *2011 Prescription Drug Abuse Prevention Plan*, Whitehouse.gov, (no longer available on whitehouse.gov), https://obamawhitehouse.archives.gov/ondcp/prescription-drug-abuse1 (accessed May 30, 2017).

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opioid prescriptions increased from 11.3% to 19.6% of visits, as NSAID and acetaminophen prescriptions fell from 38% to 29%, driven primarily by the decline of NSAID use.[183]

700. Approximately 20% of the population between the ages of 30 and 44 and nearly 30% of the population over 45 have used opioids. Indeed, "[o]pioids are the most common means of treatment for chronic pain."[184] From 1980 to 2000, opioid prescriptions for chronic pain visits doubled. This resulted not from an epidemic of pain, but an epidemic of prescribing. A study of 7.8 million doctor visits found that prescribing for pain increased by 73% between 2000 and 2010—even though the number of office visits in which patients complained of pain did not change and prescribing of non-opioid pain medications ***decreased***. For back pain alone—one of the most common chronic pain conditions—the percentage of patients prescribed opioids increased from 19% to 29% between 1999 and 2010, even as the use of NSAIDs or acetaminophen declined and referrals to physical therapy remained steady—and climbing.

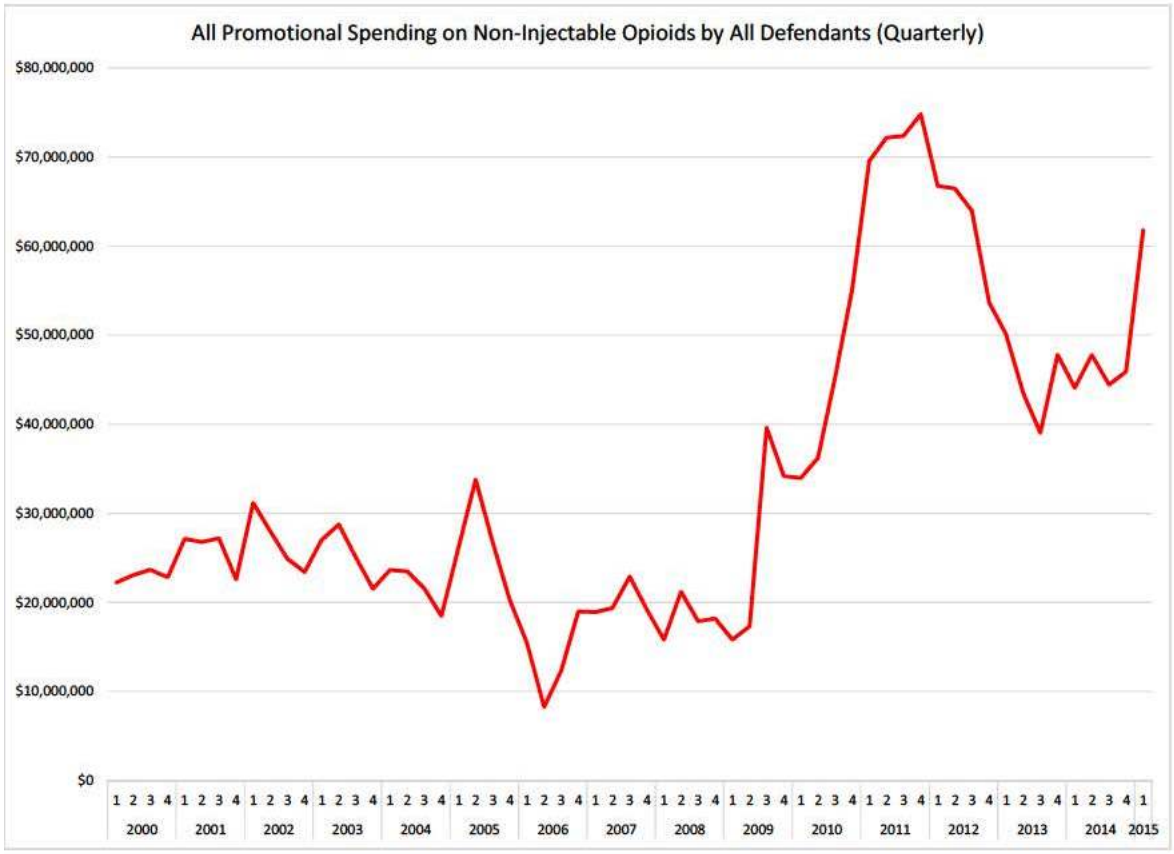
701. This increase corresponds with, and was caused by, Defendants' massive marketing push. As reflected in the chart below, according to data obtained from a marketing research company, Defendants' spending on marketing of opioids nationwide—including all of the drugs at issue here—stood at more than $20 million per quarter and $91 million annually in 2000. By 2011, that figure hit its peak of more than $70 million per quarter and $288 million annually, an increase of more than three-fold. By 2014, the figures dropped to roughly $45 million per quarter and $182 million annually, as Defendants confronted increasing concerns regarding opioid addiction, abuse, and diversion, and as Janssen, which accounted for most of the spending reduction, prepared to sell its U.S. rights to Nucynta and Nucynta ER. Even so, Defendants still spent double what they spent in 2000 on opioid marketing.

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[183] Matthew Daubresse et al., *Ambulatory* Diagnosis *and Treatment of Nonmalignant Pain in the United States, 2000-2010*, 51(10) Med. Care 870 (2013).

[184] Deborah Grady et al., *Opioids for Chronic Pain,* 171(16) Arch. Intern. Med. 1426 ( 2011).

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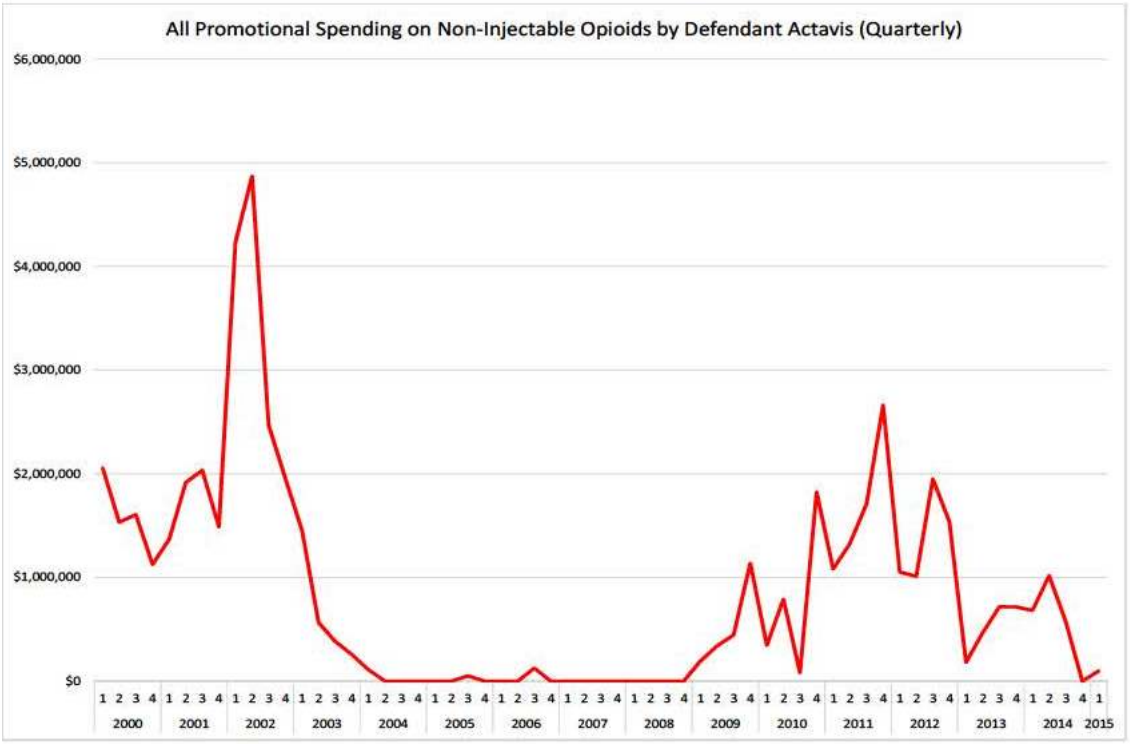



702. Defendants' opioid detailing visits to individual doctors made up the largest component of this spending, with total detailing expenditures more than doubling between 2000 and 2014 to $168 million annually.

703. Each Defendant's promotional spending reflects its participation in this marketing blitz. Between 2000 and 2011:

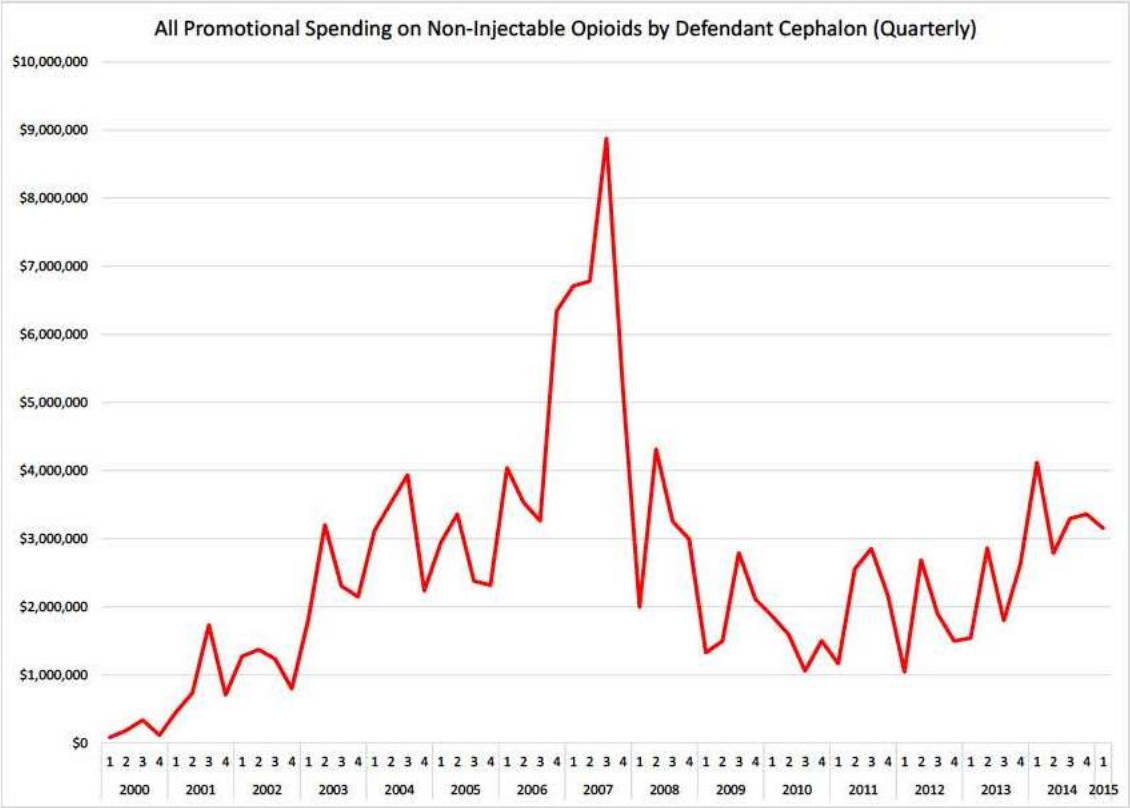
- Actavis's promotional spending, which was virtually nonexistent in the 2004-2008 period, began to sharply rise 2009. The third quarter of 2011 saw a peak of $3 million at one point in 2011 and nearly $7 million for the year, as shown below:

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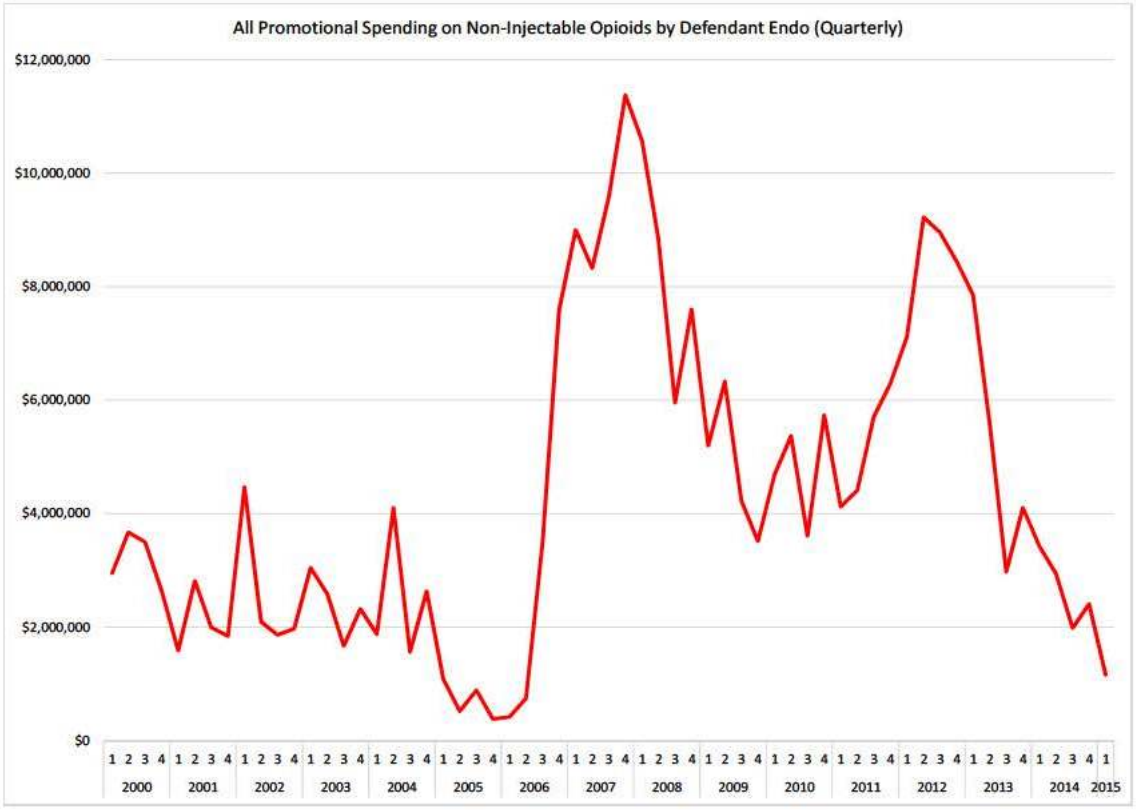

- Cephalon's quarterly spending steadily climbed from below $1 million in 2000 to more than $4 million in 2014 (and more than $13 million for the year), including a peak, coinciding with the launch of Fentora, of nearly $9 million half way through 2007 (and more than $27 million for the year), as shown below:

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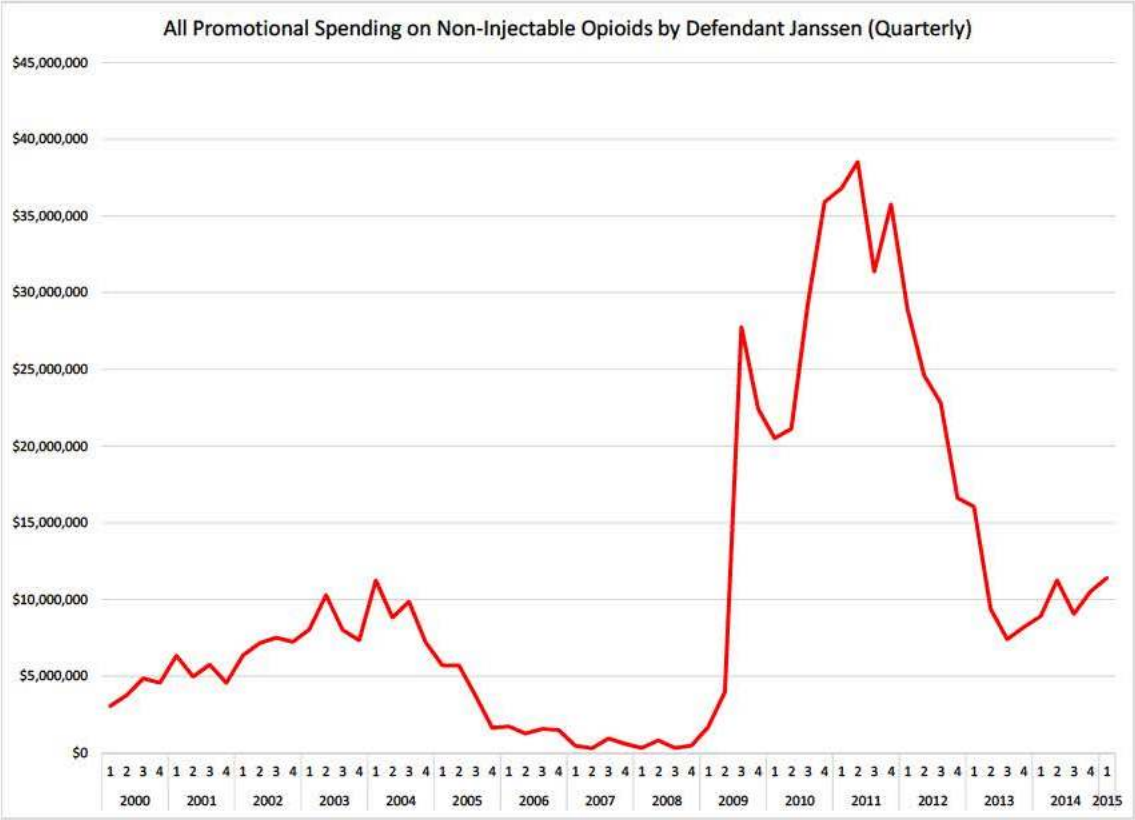

- Endo 's quarterly spending went from the $2 million to $4 million range from 2000 to 2004 to more than $10 million following the launch of Opana ER in mid-2006 (and more than $38 million for the year in 2007) and more than $8 million coinciding with the launch of a reformulated version in 2012 (and nearly $34 million for the year):

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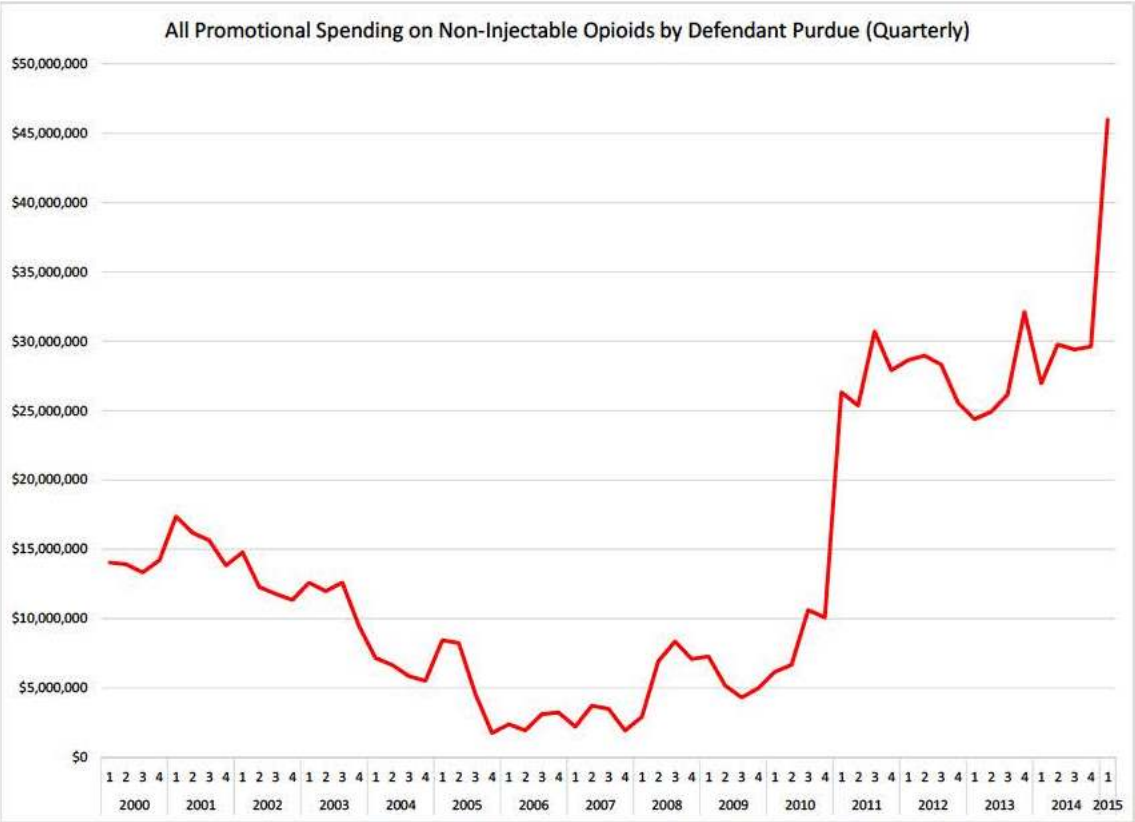

- Janssen's quarterly spending dramatically rose from less than $5 million in 2000 to more than $30 million in 2011, coinciding with the launch of Nucynta ER (with yearly spending at $142 million for 2011) as shown below:

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- Purdue's quarterly spending notably decreased from 2000 to 2007, as Purdue came under investigation by the Department of Justice, but then spiked to above $25 million in 2011 (for a total of $110 million that year), and continued to rise, as shown below:

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b. The County's Increased Spending on Opioids

704. As a direct and foreseeable consequence of Defendants' wrongful conduct, Plaintiff has been required to spend millions of dollars each year in its efforts to combat the public nuisance created by Defendants' deceptive marketing campaign. Plaintiff has incurred, and continues to incur, costs related to opioid addiction and abuse, including, but not limited to, health care costs, criminal justice and victimization costs, social costs, and lost productivity costs. Defendants' misrepresentations regarding the safety and efficacy of long-term opioid use proximately caused injury to Plaintiff and its residents.

i. *Defendants' Misrepresentations Were Material*

705. Defendants' misrepresentations were material to, and influenced, the County's decisions to pay claims for opioids for chronic pain (and, therefore, to bear its consequential costs in treating overdose, addiction, and other side effects of opioid use). In the first instance, the County

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would not have been presented with, or paid, claims for opioids that would not have been written but for Defendants' fraudulent and deceptive marketing. Second, the County has demonstrated that Defendants' marketing is material by taking further steps to ensure that the opioids are only prescribed and covered when medically necessary or reasonably required.

706. As laid out above, Defendants' misrepresentations related to the County's requirement that medical treatments be medically necessary or reasonably required – a condition of payment for any medical treatment under the County's health plans and workers' compensation program. But for Defendants' fraudulent and deceptive marketing, prescribers would have accurately understood the risks and benefits of opioids and would not have prescribed opioids where not medically necessary or reasonably required to treat chronic pain. Misrepresentations as to, for example, whether patients were likely to become addicted to the drug, would be able to resume life activities, and would experience long-term relief were not minor or insubstantial matters, but the core of prescribers' decision-making.

707. It is the County's practice not to pay claims that are not medically necessary or reasonably required. However, the County would not have known whether a prescriber had made an informed judgment that a particular claim for opioids was medically necessary or reasonably required, or, conversely had acted under the influence of Defendants' fraudulent and deceptive marketing. It is not clear from the face of a claim whether: (1) the patient suffered from cancer or another terminal condition, for example, where long-term prescribing was medically necessary or appropriate; or (2) the prescriber was exposed to Defendants' marketing materials, treatment guidelines, or education programs, or visited by a drug representative who engaged in affirmative misrepresentations or omissions, for example.

ii. *The County's Increased Costs Correlate with Defendants' Promotion*

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708. The County's spending in connection with opioids rose along with Defendants' spending to promote opioids. That spending was directly impacted by opioid use (and its consequences in abuse, addiction, and overdose) in Maverick County.

709. It is also distressing (and a sign of further problems ahead) that the drop in opioid prescribing beginning in 2014 has been accompanied by a corresponding increase in Defendants' promotional spending, which is headed towards a new high, despite evidence of the grave toll that opioids are taking on law enforcement, public health, and individual lives.

**2. Defendants' Fraudulent and Deceptive Marketing of Opioids Directly Caused Harm to Maverick County Consumers.**

a. Increased Opioid Use Has Led to an Increase in Opioid Abuse, Addiction, and Death

710. Nationally, the sharp increase in opioid use has led directly to a dramatic increase in opioid abuse, addiction, overdose, and death. Scientific evidence demonstrates a very strong correlation between therapeutic exposure to opioid analgesics, as measured by prescriptions filled, and opioid abuse. "Deaths from opioid overdose have risen steadily since 1990 in parallel with increasing prescription of these drugs."[185] Prescription opioid use contributed to 16,917 overdose deaths nationally in 2011—more than twice as many deaths as heroin and cocaine combined; drug poisonings now exceed motor vehicle accidents as a cause of death. More Americans have died from opioid overdoses than from participation in the Vietnam War.

711. Contrary to Defendants' misrepresentations, most of the illicit use stems from *prescribed* opioids; in 2011, 71% of people who abused prescription opioids got them through friends or relatives, not from drug dealers or the internet. According to the CDC, the 80% of opioid patients who take low-dose opioids from a single prescriber (in other words, who are not illicit users or "doctor-shoppers") account for 20% of all prescription drug overdoses.

[185] Deborah Grady et al., *Opioids for Chronic Pain,* 171(16) Arch. Intern. Med. 1426 ( 2011).

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712. Death statistics represent only the tip of the iceberg. According to 2009 data, for every overdose death that year, there were nine abuse treatment admissions, 30 emergency department visits for opioid abuse or misuse, 118 people with abuse or addiction problems, and 795 non-medical users. Nationally, there were more than 488,000 emergency room admissions for opioids other than heroin in 2008 (up from almost 173,000 in 2004).

713. Emergency room visits tied to opioid use likewise have sharply increased in Maverick County.

714. Widespread opioid use and abuse in Maverick County are problems even when they do not result in injury or death. Opioid addiction is affecting residents of all ages, ethnicities, and socio-economic backgrounds in the County. Many addicts start with a legal opioid prescription—chronic back pain, fibromyalgia, or even dental pain—and do not realize they are addicted until they cannot stop taking the drugs.

715. These glaring omissions, described consistently by counselors and patients, mirror and confirm Defendants' drug representatives' own widespread practice, as described above, of omitting any discussion of addiction from their sales presentations to physicians or in their "educational" materials.

b. Increased Opioid Use Has Increased Costs Related to Addiction Treatment

716. Maverick County has opioid treatment programs, Substance Alternative Clinics, that provide a comprehensive treatment program for persons addicted to heroin or other opioids.

717. In addition to intense counseling, many treatment programs prescribe additional drugs to treat opioid addiction. Nationally, in 2012, nearly 8 billion prescriptions of the two drugs commonly used to treat opioid addiction—buprenorphine/naloxone and naltrexone—were written and paid for. Studies estimate the total medical and prescription costs of opioid addiction and diversion to public and private healthcare payors to be $72.5 billion.

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c. Increased Opioid Use Has Fueled An Illegal Secondary Market for Narcotics and the Criminals Who Support It

718. Defendants' success in extending the market for opioids to new patients and chronic conditions has created an abundance of drugs available for criminal use and fueled a new wave of addiction, abuse, and injury. Defendants' scheme supplies both ends of the secondary market for opioids—producing both the inventory of narcotics to sell and the addicts to buy them. One researcher who has closely studied the public health consequences of opioids has found, not surprisingly, that a "substantial increase in the nonmedical use of opioids is a predictable adverse effect of substantial increases in the extent of prescriptive use."[186] It has been estimated that the majority of the opioids that are abused come, directly or indirectly, through doctors' prescriptions.

719. A significant black market in prescription opioids also has arisen, not only creating and supplying additional addicts, but fueling other criminal activities.

720. In addition, because heroin is cheaper than prescription painkillers, many prescription opioid addicts migrate to heroin. Self-reported heroin use nearly doubled between 2007 and 2012, from 373,000 to 669,000 individuals. In 2010, more than 3,000 people in the U.S. died from heroin overdoses, also nearly double the rate in 2006. Nearly 80% of those who used heroin in the past year had previously abused prescription opioids. Patients become addicted to opioids and then move on to heroin because these prescription drugs are roughly four times more expensive than heroin on the street. In the words of one federal DEA official, "Who would have ever thought in this country it would be cheaper to buy heroin than pills . . . [t]hat is the reality we're facing."[187]

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[186] G. Caleb Alexander et al., *Rethinking Opioid Prescribing to Protect Patient Safety and Public Health*, 308(18) JAMA 1865 (2012).

[187] Matt Pearce & Tina Susman, *Philip Seymour Hoffman's death calls attention to rise in heroin use*, L.A. Times, Feb. 3, 2014, http://articles.latimes.com/2014/feb/03/nation/la-na-heroin-surge-20140204 (accessed May 30, 2017).

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721. That reality holds true in Maverick County. According to addiction programs, a typical course sees addicts requesting more and more opioids from their doctors, who eventually cut them off. Many addicts then doctor-shop for additional prescriptions, and when that source runs out, turn to the streets to buy opioids illicitly. A significant number become heroin addicts. Addiction treatment programs, whose patient populations vary, reported rates of patients who had switched from prescription opioids to heroin ranging from half to 95%. Those addicts who do reach treatment centers often do so when their health, jobs, families and relationships reach the breaking point, or after turning to criminal activity such as prostitution and theft to sustain their addiction. Unfortunately, few are successful in getting and staying clean; repeated relapse is common.

**3. Defendants' Fraudulent Marketing Has Led to Record Profits**

722. While the use of opioids has taken an enormous toll on Maverick County and its residents, Defendants have gained blockbuster profits. In 2012, health care providers wrote 259 million prescriptions for opioid painkillers[188]—roughly one prescription per American adult. Opioids generated $8 billion in revenue for drug companies just in 2010.

723. Financial information—where available—indicates that Defendants each experienced a material increase in sales, revenue, and profits from the fraudulent, misleading, and unfair market activities laid out above. Purdue's OxyContin sales alone increased from $45 million in 1996 to $3.1 billion in 2010.

**4. Defendants Fraudulently Concealed Their Misrepresentations**

724. At all times relevant to this Complaint, Defendants took steps to avoid detection of, and fraudulently conceal, their deceptive marketing and conspiratorial behavior.

[188] Press Release, Center for Disease Control, Opioid painkiller prescribing varies widely among states: Where you live makes a difference (July 1, 2014), https://www.cdc.gov/media/releases/2014/p0701-opioid-painkiller.html (accessed May 30, 2017).

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725. First, and most prominently, Defendants disguised their own roles in the deceptive marketing of chronic opioid therapy by funding and working through patient advocacy and professional front organizations and KOLs. Defendants purposefully hid behind these individuals and organizations to avoid regulatory scrutiny and to prevent doctors and the public from discounting their messages.

726. While Defendants were listed as sponsors of many of the publications described in this Complaint, they never disclosed their role in shaping, editing, and exerting final approval over their content. Defendants exerted their considerable influence on these promotional and "educational" materials.

727. In addition to hiding their own role in generating the deceptive content, Defendants manipulated their promotional materials and the scientific literature to make it appear as if they were accurate, truthful, and supported by substantial scientific evidence. Defendants distorted the meaning or import of studies they cited and offered them as evidence for propositions they did not actually support. The true lack of support for Defendants' deceptive messages was not apparent to the medical professionals who relied upon them in making treatment decisions, nor could they have been detected by the County.

728. Thus, while the opioid epidemic was evident, Defendants, in furtherance of their respective marketing strategies, intentionally concealed their own role in causing it. Defendants successfully concealed from the medical community, patients, and health care payers facts sufficient to arouse suspicion of the existence of claims that the County now asserts. The County was not alerted to the existence and scope of Defendants industry-wide fraud and could not have acquired such knowledge earlier through the exercise of reasonable diligence.

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729. Through their public statements, marketing, and advertising, Defendants' deceptions deprived the County of actual or presumptive knowledge of facts sufficient to put them on notice of potential claims.

**G. Through Their Public Statements, Marketing, And Advertising, Defendants' Deceptions Deprived Plaintiff Of Actual Or Presumptive Knowledge Of Facts Sufficient To Put Them On Notice Of Potential Claims. Defendants Entered Into And Engaged In A Civil Conspiracy.**

730. Defendants entered into a conspiracy to engage in the wrongful conduct complained of herein, and intended to benefit both independently and jointly from their conspiratorial enterprise.

731. Defendants reached an agreement between themselves to set up, develop, and fund an unbranded promotion and marketing network to promote the use of opioids for the management of pain in order to mislead physicians, patients, and others through misrepresentations or omissions regarding the appropriate uses, risks and safety of opioids.

732. This network is interconnected and interrelated and relied upon Defendants' collective use of and reliance upon unbranded marketing materials, such as KOLs, scientific literature, CMEs, patient education materials, and Front Groups. These materials were developed and funded collectively by Defendants, and Defendants relied upon the materials to intentionally mislead consumers and medical providers of the appropriate uses, risks and safety of opioids.

733. By knowingly misrepresenting the appropriate uses, risks, and safety of opioids, Defendants committed overt acts in furtherance of their conspiracy.

**H. Defendants Flooded Plaintiff Counties with Suspiciously Large Amounts of Opioids.**

734. The Distributor Defendants are opioid distributors in Maverick County.

735. The Distributor Defendants purchased opioids from manufacturers, such as the named defendants herein, and sold them to pharmacies throughout Maverick County.

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## FIRST CAUSE OF ACTION

## PUBLIC NUISANCE
## (AGAINST ALL DEFENDANTS)

736. Plaintiff incorporates the allegations within all prior paragraphs within this Complaint as if they were fully set forth herein.

737. Defendants, individually and acting through their employees and agents, and in concert with each other, have intentionally, recklessly, or negligently engaged in conduct or omissions which endanger or injure the property, health, safety or comfort of a considerable number of persons in Maverick County by their production, promotion, and marketing of opioids for use by residents of Maverick County.

738. Defendants conduct and subsequent sale of its opioid products is not only unlawful, but has also resulted in substantial and unreasonable interference with the public health.

739. Defendants' conduct is not insubstantial or fleeting. Indeed, Defendants' unlawful conduct has so severely impacted public health on every geographic and demographic level that the public nuisance perpetrated by Defendants' conduct is commonly referred to as a "crisis" or an "epidemic." It has caused deaths, serious injuries, and a severe disruption of public peace, order and safety; it is ongoing, and it is producing permanent and long-lasting damage.

740. Defendants' conduct constitutes a public nuisance.

741. Defendants' conduct directly and proximately caused injury to Plaintiff and its residents.

742. The public nuisance created and maintained by the Defendants is a matter of great public concern and interest to the residents of Maverick County.

743. Plaintiff has been injured by reason of Defendants' creation of the public nuisance.

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744. Plaintiff is entitled to recover their damages caused by Defendants' creation of this public nuisance in an amount to be determined at trial, plus costs and attorneys' fees.

## SECOND CAUSE OF ACTION

## FRAUD
## (AGAINST ALL DEFENDANTS)

745. Plaintiff incorporates the allegations within all prior paragraphs within this Complaint as if they were fully set forth herein.

746. Defendants, individually and acting through their employees and agents, and in concert with each other, knowingly made material misrepresentations and omissions of facts to Plaintiff and its residents to induce them to purchase, administer, and consume opioids as set forth in detail above.

747. Defendants knew at the time that they made their misrepresentations and omissions that they were false.

748. Defendants intended that Plaintiff, physicians, patients, and/or others would rely on their misrepresentations and omissions.

749. Plaintiff, physicians, patients, and/or others reasonably relied upon Defendants' misrepresentations and omissions.

750. In the alternate, the Defendants recklessly disregarded the falsity of their representations regarding opioids.

751. By reason of their reliance on Defendants' misrepresentations and omissions of material fact Plaintiff, physicians, patients, and/or others suffered actual pecuniary damage.

752. Defendants' conduct was willful, wanton, and malicious and was directed at the public generally.

753. Plaintiff is entitled to recover damages caused by Defendants' fraud in an amount to be determined at trial.

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## THIRD CAUSE OF ACTION

## UNJUST ENRICHMENT
## (AGAINST ALL DEFENDANTS)

754. Plaintiff incorporates the allegations within all prior paragraphs within this Complaint as if they were fully set forth herein.

755. Defendants acted willfully, wantonly, and with conscious disregard of the rights of the Plaintiff.

756. As an expected and intended result of their conscious wrongdoing as set forth in this Complaint, Defendants have profited and benefited from opioid purchases made by Plaintiff.

757. In exchange for the opioid purchases, and at the time Plaintiff made these payments, Plaintiff expected that Defendants had provided all of the necessary and accurate information regarding those risks and had not misrepresented any material facts regarding those risks.

758. Defendants, through the wrongful conduct described above, have been unjustly enriched at the expense of Plaintiff.

759. In equity and good conscience, it would be unjust and inequitable to permit Defendants to enrich themselves at the expense of the Plaintiff and its residents.

760. By reason of the foregoing, Defendants must disgorge its unjustly acquired profits and other monetary benefits resulting from its unlawful conduct and provide restitution to the Plaintiff.

## FOURTH CAUSE OF ACTION

## NEGLIGENCE PER SE – VIOLATION OF THE TEXAS CONTROLLED SUBSTANCES ACT ("TCSA")
## (AGAINST ALL DEFENDANTS)

761. Plaintiff incorporates the allegations within all prior paragraphs within this Complaint as if they were fully set forth herein.

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762. Defendants have a duty to comply with the regulations of the Texas Controlled Substances Act ("TCSA"), including but not limited to §481.128

763. Defendants knowingly manufactured, distributed, marketed, sold, deliverer, administered, or dispensed controlled substances in violation of the TCSA.

764. Similarly, the TCSA §481.067 et seq., provides in part that "a person who is registered with the Federal Drug Enforcement Administration to manufacture, distribute, analyze, or dispense a controlled substance shall keep records and maintain inventories in compliance with recordkeeping and inventory requirements of federal law and with additional rules the board or director adopts." TCSA §481.067 (a). The TCSA defines a "person" as "an individual, corporation, government, business trust, estate, trust, partnership, association, or any other legal entity." TCSA § 481.002(33).

765. The TCSA § 481.128 provides for civil penalties in commercial matters if a "registrant or dispenser" violates the TCSA. This includes if a registrant "distributes, delivers, administers or dispenses a controlled substance in violation" of the TCSA or federal law, "refuses or fails to make, keep, or furnish a record, report, notification, order form, statement, invoice, or information required by this chapter", or "refuses or fails to maintain security required by this chapter or a rule adopted under this chapter." TCSA §§ 481.128(a)(1),(2),(9).

766. Failure to comply with the TCSA constitutes negligence per se.

767. Defendants failed to comply with the TCSA.

768. In the instant case, the TCSA requires that the Defendants know their customers, which includes, an awareness of the customer base, knowledge of the average prescriptions filled each day, the percentage of controlled substances compared to overall purchases, a description of how the dispenser fulfills its responsibility to ensure that prescriptions filled are for legitimate medical purposes, and identification of physicians and bogus centers for the alleged treatment of pain that are the dispenser's most frequent prescribers.

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769. Defendants have failed to diligently respond to the suspicious orders which Defendants have filled.

770. Defendants have failed to provide effective controls and procedures to guard against diversion of controlled substances in contravention of Texas law.

771. Defendants have willfully turned a blind eye towards the actual facts by regularly distributing large quantities of controlled substances to retailers and dispensers who are serving a customer base comprised of individuals who are themselves abusing and/or dealing prescription medications, many of whom are addicted and all of whom can reasonably be expected to become addicted.

772. Defendants negligently acted with others by dispensing controlled substances for illegitimate medical purposes, operating bogus pain clinics which do little more than provide prescriptions for controlled substances and thereby creating and continuing addictions to prescription medications in this state.

773. Defendants have, by their acts and omissions, proximately caused and substantially contributed to damages to Maverick County by violating Texas law, by creating conditions which contribute to the violations of Texas laws by others, and by their negligent and/or reckless disregard of the customs, standards and practices within their own industry.

774. Maverick County has suffered and will continue to suffer enormous damages as the proximate result of the failure by Defendants to comply with the TCSA.

775. Defendants' acts and omissions imposed an unreasonable risk of harm to others separately and/or combined with the negligent and/or criminal acts of third parties.

776. Defendants are in a class of a limited number of parties that can legally sell and distribute opioids, which places it in a position of great trust by the County.

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777. The trust placed in Defendants by Maverick County through the license to distribute opioids in Maverick County creates a duty on behalf of Defendants to prevent diversion of the medications it supplies to illegal purposes.

778. A negligent and/or intentional violation of this trust poses distinctive and significant dangers to the County and its residents from the diversion of opioids for non-legitimate medical purposes and addiction to the same by consumers.

779. Defendants were negligent in not acquiring and utilizing special knowledge and special skills that relate to the dangerous activity in order to prevent and/or ameliorate such distinctive and significant dangers.

780. Defendants are required to exercise a high degree of care and diligence to prevent injury to the public from the diversion of opioids during distribution.

781. Defendants breached their duty to exercise the degree of care, prudence, watchfulness, and vigilance commensurate to the dangers involved in the transaction of its business.

782. Defendants are in exclusive control of the management of the opioids it distributed to pharmacies and drug stores in Maverick County.

783. Maverick County is without fault and the injuries to the County and its residents would not have occurred in the ordinary course of events had Defendants used due care commensurate to the dangers involved in the distribution of opioids.

784. Plaintiff is within the class of persons the TCSA was intended to protect.

785. The harm that has occurred is the type of harm that the TCSA were intended to guard against.

786. Defendants breached this duty by failing to take any action to prevent or reduce the distribution of the opioids.

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787. As a direct and proximate result of Defendants' negligence per se, the County has suffered and continues to suffer injury, including but not limited to incurring excessive costs related to diagnosis, treatment, and cure of addiction or risk of addiction to opioids, bearing the massive costs of these illnesses and conditions by having to provide necessary resources for care, treatment facilities, and law enforcement services for County Residents and using County resources in relation to opioid use and abuse.

788. Defendants were negligent in failing to monitor and guard against third-party misconduct and participated and enabled such misconduct.

789. Defendants were negligent in failing to monitor against diversion of opioid pain medications.

790. Plaintiff is entitled to recover damages caused by Defendants' fraud in an amount to be determined at trial.

## SIXTH CAUSE OF ACTION
## NEGLIGENT MARKETING
## (AGAINST ALL DEFENDANTS)

791. Plaintiff incorporates the allegations within all prior paragraphs within this Complaint as if they were fully set forth herein.

792. Defendants had a duty to exercise reasonable care in the marketing of opioids.

793. Defendants were aware of the potentially dangerous situation involving opioids.

794. Defendants marketed opioids in an improper manner by:

a. overstating the benefits of chronic opioid therapy, promising improvement in patients' function and quality of life, and failing to disclose the lack of evidence supporting long-term use;

b. trivializing or obscuring opioids' serious risks and adverse outcomes, including the risk of addiction, overdose, and death;

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c. overstating opioids' superiority compared with other treatments, such as other non-opioid analgesics, physical therapy, and other alternatives;

d. mischaracterizing the difficulty of withdrawal from opioids and the prevalence of withdrawal symptoms;

e. marketing opioids for indications and benefits that were outside of the opioids' labels and not supported by substantial evidence.

795. It was Defendants' marketing—and not any medical breakthrough—that rationalized prescribing opioids for chronic pain and opened the floodgates of opioid use and abuse. The result has been catastrophic.

796. Defendants disseminated many of their false, misleading, imbalanced, and unsupported statements indirectly, through KOLs and Front Groups, and in unbranded marketing materials. These KOLs and Front Groups were important elements of Defendants' marketing plans, which specifically contemplated their use, because they seemed independent and therefore outside FDA oversight. Through unbranded materials, Defendants, with their own knowledge of the risks, benefits and advantages of opioids, presented information and instructions concerning opioids generally that were contrary to, or at best, inconsistent with information and instructions listed on Defendants' branded marketing materials and drug labels. Defendants did so knowing that unbranded materials typically are not submitted to or reviewed by the FDA.

797. Defendants also marketed opioids through the following vehicles: (a) KOLs, who could be counted upon to write favorable journal articles and deliver supportive CMEs; (b) a body of biased and unsupported scientific literature; (c) treatment guidelines; (d) CMEs; (e) unbranded patient education materials; and (f) Front Group patient-advocacy and professional organizations, which exercised their influence both directly and through Defendant-controlled KOLs who served in leadership roles in those organizations.

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798. Defendants knew or should have known that opioids were unreasonably dangerous and could cause addiction.

799. Defendants' marketing was a factor in physicians, patients, and others to prescribe or purchase opioids.

800. As a direct and proximate result of Defendants' negligence, the County has suffered and continues to suffer injury, including but not limited to incurring excessive costs related to diagnosis, treatment, and cure of addiction or risk of addiction to opioids, bearing the massive costs of these illnesses and conditions by having to provide necessary resources for care, treatment facilities, and law enforcement services for County Residents and using County resources in relation to opioid use and abuse.

801. Plaintiff is entitled to recover damages caused by Defendants' negligence in an amount to be determined at trial.

**SEVENTH CAUSE OF ACTION**
**NEGLIGENCE**
**(AGAINST ALL DEFENDANTS)**

802. Plaintiff incorporates the allegations within all prior paragraphs within this Complaint as if they were fully set forth herein.

803. Defendants have a duty to exercise reasonable care in the distribution, promotion, and marketing of opioids.

804. Defendants breached this duty by failing to take any action to prevent or reduce the distribution of the opioids.

805. As a proximate result, Defendants and its agents have caused Maverick County to incur excessive costs related to diagnosis, treatment, and cure of addiction or risk of addiction to opioids, the County has borne the massive costs of these illnesses and conditions by having to provide necessary

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resources for care, treatment facilities, and law enforcement services for County Residents and using County resources in relation to opioid use and abuse.

806. Plaintiff is entitled to recover damages caused by Defendants' negligence in an amount to be determined at trial.

## PRAYER FOR RELIEF

**WHEREFORE**, Plaintiff demands judgment against defendants, jointly and severally, as to all Causes of Action, awarding Plaintiff in amounts that exceed the jurisdiction of all lower Courts:

i. compensatory damages in an amount sufficient to fairly and completely compensate Plaintiff for all damages;

ii. treble damages, penalties, and costs

iii. Punitive Damages;

iv. attorneys' fees

v. interest, Costs and Disbursements;

vi. such and further relief as this Court may deem just and proper.

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Dated: October 19th, 2017
Maverick County, Texas

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# APPENDIX J

# HIDALGO PETITION

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Reviewed By: Alexis Bonilla

CAUSE NO. C-1349-18-C

| | | |
|---|---|---|
| COUNTY OF HIDALGO,<br><br>Plaintiff,<br><br>VS.<br><br>PURDUE PHARMA LP; PURDUE PHARMA INC; PURDUE FREDERICK COMPANY; TEVA PHARMACEUTICAL INDUSTRIES, LTD; TEV PHARMACEUTICALS USA, INC.; JANSSEN PHARMACEUTICA, INC., n/k/a JANSSEN PHARMACEUTICALS, INC.; ENDO HEALTH SOLUTIONS, INC.; ABBOTT LABORATORIES; KNOLL PHARMACEUTICAL COMPANY, a wholly-owned subsidiary Of ABBOTT LABORATORIES; ALLERGEN PLC, f/k/a ACTAVIS PLC, f/k/a ALLERGEN FINANCE LLC, f/k/a ACTAVIS, INC., f/k/a WATSON LABORATORIES, INC.; ACTAVIS LLC; ACTAVIS PHARMA, INC., f/k/a WATSON PHARMA, INC.; INSYS THERAPEUTICS, INC.; INSYS MANUFACTURING LLC, a wholly-owned Subsidiary of INSYS THERAPEUTICS, INC.; MALLINCKRODT PLC; MCKESSON CORPORATION; CARDINAL HEALTH, INC.; and AMERISOURCEBERGEN CORPORATION;<br><br>Defendants. | §<br>§<br>§<br>§

Pharma Inc., The Purdue Frederick Company, Johnson & Johnson, Janssen Pharmaceuticals, Inc., Janssen Pharmaceutica, Inc. n/k/a Janssen Pharmaceuticals, Inc., Ortho-McNeil-Janssen Pharmaceuticals, Inc. n/k/a Janssen Pharmaceuticals, Inc., Endo Health Solutions Inc., Endo Pharmaceuticals, Inc., Abbvie Inc., Knoll Pharmaceutical Company, a wholly-owned subsidiary of Abbvie Inc., Allergan PLC f/k/a Actavis PLC, Allergan Finance, LLC f/k/a Actavis, Inc. f/k/a Watson Pharmaceuticals, Inc., Watson Laboratories, Inc., Actavis LLC, Actavis Pharma, Inc. f/k/a Watson Pharma, Inc., Insys Therapeutics, Inc., Insys Manufacturing LLC, a wholly-owned subsidiary of Insys Therapeutics, Inc., Mallinckrodt PLC (the "Manufacturing Defendants"), McKesson Corporation, Cardinal Health, Inc., and AmerisourceBergen Drug Corporation ('the "Distributor Defendants").

## I. DISCOVERY CONTROL PLAN

1. Hidalgo County intends to conduct discovery under Level 3 of Texas Rule of Civil Procedure 190.4 and affirmatively pleads that this suit is not governed by the expedited action process in Texas Rule of Civil Procedure 169 because Hidalgo County seeks monetary relief over $100,000.

## II. INTRODUCTION

2. The United States is in the midst of an opioid epidemic caused by Defendants' fraudulent marketing, sales, and distribution of prescription opioids ("opioids") that has resulted in addiction, criminal activity, and loss of life. The opioid crisis has been described as "the AIDS epidemic of our generation, but even worse."[1] On October 26, 2017, President Donald Trump "declared a nationwide public health emergency to combat the opioid crisis."[2]

[1] David Wright, "Christie on Opioids: "This is the AIDS Epidemic of Our Generation, but even Worse," (Oct. 27, 2017) http://www.cnn.com/2017/10/27/politics/chris-christie-opioid-commission-aids-cnntv/index.html.
[2] Dan Merica, "What Trump's Opioid Announcement Means – and Doesn't Mean," (Oct. 26, 2017) http://www.cnn.com/2017/10/26/politics/national-health-emergency-national -disaster/index.html.

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3. In 2016 alone, health care providers wrote more than 289 million prescriptions for opioids, enough for every adult in the United States to have more than one bottle of pills.[3] Americans "consume 85% of all the opioids in the world" and are "the most medicated country in the world …."[4]

4. Unfortunately, using opioids too often leads to addiction and overdose from opioids. In 2014, almost 2 million Americans were addicted to opioids.[5] That same year more people died from drug overdoses than in any other year, and most overdose deaths involved an opioid. The Texas Legislature has found "that deaths resulting from the use of opioids and other controlled substances constitute a public health crisis."[6] In 2015, Texas had the second highest total healthcare costs from opioid abuse in the nation at $1.96 billion.

5. In fact, accidental drug overdose deaths, of which reportedly at least two-thirds are opioid overdoses, are the leading cause of death for Americans under the age of 50. Accidental drug overdose deaths, predominantly from opioids, exceed the number of deaths cause by cars or guns.

6. The economic burden caused by opioid abuse in the United States is at least $78.5 billion,[7] including lost productivity and increased social services, health insurance costs, increased criminal justice presence and strain on judicial resources, and substance abuse treatment and rehabilitation.

7. This epidemic did not occur by chance. Defendants manufacture, promote, market, distribute, and/or sell prescription opioids, including but not limited to brand-name drugs like Oxycontin, Vicodin, Opana, Percocet, Duragesic, Ultram, Ultracet, and Subsys, and generics like

[3] Prevalence of Opioid Misuse, BupPractice (Sept. 7, 2017), https://www.buppractice.com/node/15576.
[4] Substance Abuse and Mental Health Services Administration, National Survey on Drug Use and Health, 2014.
[5] Id.
[6] Opinion of the Attorney General of Texas, KP-0168 (Oct. 4, 2017), citing Act of May 26, 2017, 85th Leg., R.S., Ch. 534, §3, 2017 Tex. Sess. Law Serv. 1467, 1468.
[7] See CDC Foundation's New Business Pulse Focuses on Opioid Overdose Epidemic, Centers for Disease Control and Prevention (Mar. 15, 2017), https://www.cdc.gov/media/releases/2017/a-315-business-pulse-opioids.html.

oxycodone, oxymorphone, hydromorphone, hydrocodone, fentanyl, and tramadol, which are powerful narcotics.

8. Historically, opioids were considered too addictive and debilitating for treating non-cancer chronic pain – non-cancer pain that lasts for three months or longer – such as back pain, migraines, arthritis, or standard dental procedures and were used only to treat short-term acute pain or for palliative or end-of-life care.

9. By the late 1990s or early 2000s, however, the Manufacturing Defendants began a marketing scheme to persuade doctors and patients that opioids can and should be used ubiquitously and perpetually to treat moderate, non-cancer chronic pain.[8] The Manufacturing Defendants spent large sums of money to promote the benefits of opioids for non-cancer, moderate pain while trivializing, or even denying, their risks. The Manufacturing Defendants' promotional messages deviated substantially from any approved labeling of the drugs and caused prescribing physicians and consuming patients to underappreciate the health risks and to overestimate the benefits of opioids.

10. Contrary to the language of their drugs' labels, Defendants' marketing and promotional communications were false and misleadingly in many respects, including that they: (1) downplayed the serious risk of addiction; (2) promoted and exaggerated the concept of "pseudoaddiction," thereby advocating that the signs of addiction should be treated with more opioids; (3) exaggerated the effectiveness of screening tools in preventing addiction; (4) claimed that opioid dependence and withdrawal are easily managed; (5) denied the risks of higher opioid

[8] Insys did not enter the opioid market until 2012, when it manufactured and marketed Subsys (sublingual fentanyl intended solely for break-through cancer pain. In a superseding indictment on October 24, 2017, 7 Insys employees, former employees, former CEO and its founder were indicted in the United States District Court for the District of Massachusetts relating to alleged criminal conduct in the marketing and promotion of SUBSYS, including allegations of commercial bribery in Texas. See U.S.A. v. Michael L. Babich, et al., Cause No. 16-cr-10343-ADB.

dosages; (6) exaggerated the effectiveness of "abuse-deterrent" opioid formulations to prevent abuse and addiction; and, (7) in the case of Insys, engaged in fraudulent or negligent off-label promotion, misrepresentation of the diagnosis of patients on Subsys to insurance companies to obtain insurance coverage for Subsys, and engaged in commercial bribery of physicians.

11. Defendants disseminated these falsehoods through ads and/or their sales representatives and physicians who supported Defendants' message. Sales representatives, working at Defendants' behest, promoted highly addictive opioids through souvenirs and toys including, but not limited to, opioid brand-bearing stuffed plush toys, dolls, coffee cups, fanny packs, water bottles, notepads, pens, refrigerator magnets, clocks, letter openers, rulers, daytime planners, bags, puzzles, posters, hand-held calculators, clipboards, highlighters, flashlights, key chains, clothing, reflex mallets, and mock-ups of the United States Constitution.

12. Defendants also used third parties they controlled by: (1) funding, assisting, encouraging, and directing doctors, known as "key opinion leaders" ("KOLs"), and (2) funding, assisting, directing, and encouraging seemingly neutral and credible professional societies and patient advocacy groups (referred to hereinafter as "Front Groups").

13. Defendants worked with KOLs and Front Groups to taint the sources that doctors and patients relied on for ostensibly "neutral" guidance, such as treatment guidelines, continuing medical education ("CME") programs, medical conferences and seminars, and scientific articles. After their individual and concerted efforts, Defendants convinced doctors that, instead of being addictive and unsafe for long-term use in most circumstances, opioids were required in the compassionate treatment of chronic pain.

14. The Distributor Defendants were not standing by idly while the Manufacturing Defendants were peddling their opioids to physicians and consumers. Cardinal, AmerisourceBergen, and

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McKesson are three of the largest opioid distributors in the United States. The Distributor Defendants purchased opioids from the Manufacturing Defendants herein and sold them to pharmacies servicing consumers in Hidalgo County.

15. Despite the rise in the ordering of opioids by retailers in Hidalgo County, The Distributor Defendants did nothing. The Manufacturing Defendants and Distributor Defendants worked hand and glove to flood the U.S. and, upon information and belief, Hidalgo County, with more opioids than would be consumed for therapeutic purposes. Each Defendant disregarded its legal duty to report suspicious opioid prescriptions, and each Defendant financially benefitted from the other Defendant's (both Manufacturing and Distributor Defendants) disregarding their individual duties to report and stop suspicious orders.

16. Essentially each Defendant ignored science and consumer health for profits. Defendant's efforts were so successful that opioids are now the most prescribed class of drugs generating $11 billion in revenue for drug companies in 2014 alone. Even after Defendant Purdue reached a $600 million federal settlement in 2007, the settlement failed to impact what is a "$13-billion-a-year opioid industry."[9]

17. Upon information and belief, as a direct and foreseeable consequence of Defendants' misrepresentations regarding the safety and efficacy of using opioids for chronic pain, Hidalgo County has spent and continues to spend large sums combatting the public health crisis created by Defendants' negligence and fraudulent marketing campaign.

18. For example, scores of prescriptions were written for opioids in Hidalgo County from 2006-2016.[10] Many overdoses, deaths, hospital admissions, and public safety offenses related to

[9] R. L. Haffajee, J.D., Ph.D., M.P.H., and M. Mello, J.D., Ph.D., Drug Companies' Liability for the Opioid Epidemic, N.Eng. J. Med. (Dec. 14, 2017) at 2305.

[10] https://www.cdc.gov/drugoverdose/maps/rxrate-maps.html; https://www.cdc.gov/nchs/data-visualization/drug-poisning-mortality.

opioids occurred in this time period in and about Hidalgo County. Defendants' misconduct and efforts to ensure more prescription opioids than could be consumed therapeutically were available were natural and foreseeable causes of these events in and about Hidalgo County.

19. Upon information and belief, as a direct and foreseeable consequence of Defendants' conduct described herein regarding prescription opioids, Hidalgo County has committed and continues to commit resources to provide and pay for health care, law enforcement, social services, public assistance, pharmaceutical care, and other services necessary for its residents.

## III. RULE 47 STATEMENT OF MONETARY RELIEF SOUGHT

20. Pursuant to Rule 47 of the Texas Rules of Civil Procedure, the County states that, although the full measure of its damages is still being calculated, its damages cause by Defendants' acts and omissions exceed $1,000,000, but are believed to be less than $100,000,000. Accordingly, at this time in this litigation, the County states that it is seeking monetary relief for an amount greater than $1,000,000 and less than $100,000,000, their rightful and just amount to be determined by the Jury.

## IV. VENUE AND JURISDICTION

21. Venue is proper in Hidalgo County because all or a substantial part of the events or omissions giving rise to this claim occurred in Hidalgo County. Tex. Civ. Prac. & Rem. Code §15.002(a)(2).

22. This Court has subject-matter jurisdiction over this matter because Plaintiff's damages are in excess of the minimal jurisdictional limits of this Court. Tex. Civ. Prac. & Rem. Code §24.007(b).

23. This Court has general jurisdiction over Purdue Pharma L.P. as it is a Texas citizen. This Court has general jurisdiction over Insys Manufacturing LLC as it is a Texas citizen. This Court

also has specific jurisdiction over all Defendants as their activities were directed toward Texas, and the injuries complained of herein resulted from their activities. Guardian Royal Exchange Assur., Ltd. V. English China Clays, P.L.C., 815 S.W.2d 223, 227 (Tex. 1991). Each Defendant has a substantial connection with Texas and the requisite minimum contacts with Texas necessary to constitutionally permit the Court to exercise jurisdiction. *See id*. at 226.

## V. PARTIES

A. Plaintiff

24. This action is brought for an on behalf of Hidalgo County, which provides a wide range of services on behalf of its residents, including services for families and children, public health, public assistance, law enforcement, and emergency care.

B. Defendants

25. PURDUE PHARMA L.P. is a limited partnership organized under the laws of Delaware and may be served through its registered agent for service of process, The Prentice-Hall Corporation System, Inc., 251 Little Falls Drive, Wilmington, DE 19808. PURDUE PHARMA L.P. is, through its ownership structure, a Texas resident. PURDUE PHARMA INC. is a New York corporation with its principal place of business in Stamford, Connecticut and may be served through its registered agent for service of process, Corporation Service Company, 80 State Street, Albany, NY 12207. THE PURDUE FREDERICK COMPANY is a Delaware corporation with its principal place of business in Stamford, Connecticut and may be served through its registered agent for service of process, The Prentice-Hall Corporation System, Inc., 251 Little Falls Drive, Wilmington, DE 19808 (collectively "Purdue").

26. Purdue manufactures, promotes, sells, and distributes opioids in the U.S. and in Hidalgo County. Purdue's opioid drug, OxyContin, is among the most addictive and abused prescription drugs in the history of America.

27. JANSSEN PHARMACEUTICALS, INC. is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey and may be served through its registered agent for service of process, CT Corporation System, 1999 Bryan Street, Suite 900, Dallas, TX 75201. JANSSEN PHARMACEUTICALS, INC. is a wholly owned subsidiary of JOHNSON & JOHNSON (J&J), a New Jersey corporation with its principal place of business in New Brunswick, New Jersey and may be served through its registered agent for service of process, Attention: Legal Department, One Johnson & Johnson Plaza, New Brunswick, NJ 08933. ORTHO-MCNEIL-JANSSEN PHARMACEUTICALS, INC., now known as JANSSEN PHARMACEUTICALS, INC., is a Pennsylvania corporation with its principal place of business in Titusville, New Jersey. JANSSEN PHARMACEUTICA INC., now known as JANSSEN PHARMACEUTICALS, INC., is a Pennsylvania corporation with its principal place of business in Titusville, New jersey. J&J is the only company that owns more than 10% of JANSSEN PHARMACEUTICAL'S stock and corresponds with the FDA regarding JANSSEN PHARMACEUTICAL'S drugs, and JANSSEN'S profits inure to J&J's benefit. (collectively "Janssen").

28. Janssen manufactures, promotes, sells, and distributes opioids in the U.S. and in Hidalgo County.

29. ENDO HEALTH SOLUTIONS INC. is a Delaware corporation with its principal place of business in Malvern, Pennsylvania and may be served through its registered agent for service of process, The Corporation Trust Company, Corporation Trust Center, 1209 Orange St., Wilmington, DE 19801. ENDO PHARMACEUTICALS, INC. is a wholly-owned subsidiary of

ENDO HEALTH SOLUTIONS INC. and is a Delaware Corporation with its principal place of business in Malvern, Pennsylvania and may be served through its registered agent for service of process, CT Corporation System, 1999 Bryan Street, Suite 900, Dallas, TX 75201. (collectively "Endo").

30. Endo develops, markets, and sells opioid drugs in the U.S. and in Hidalgo County. Endo also manufactures and sells generic opioids in the U.S. and Hidalgo County, directly and through its subsidiary, Qualitest Pharmaceuticals, Inc.

31. ABBVIE INC. ("Abbvie") is a Delaware corporation with its principal place of business in North Chicago, Illinois and may be served through its registered agent for service of process, CT Corporation System, 208 S. LaSalle Street, Suite 814, Chicago, IL 60604. KNOLL PHARMACEUTICAL COMPANY ("Knoll") has been a wholly-owned subsidiary of Abbvie from January 1, 2013. KNOLL PHARMACEUTICAL COMPANY is a New Jersey corporation with its principal place of business in Parsippany, New Jersey and may be served through its registered agent for service of process, CT Corporation System, 208 LaSalle Street, Suite 814, Chicago, IL 60604.

32. Knoll irresponsibly marketed narcotics, such as Vicodin, through whimsical toys and souvenirs and did so to boost the sales of opioids. Taking advantage of the fact that Vicodin was not regulated as a Schedule II controlled substance for many years and the fact that physicians and consumers did not fully appreciate the highly addictive nature of Vicodin, Knoll advertised Vicodin with tag lines such as "The Highest Potency Pain Relief You Can Still Phone In." This tag line came as part and parcel of souvenirs like a "Vicodin" fanny pack and water bottle, both bearing the name of Vicodin, the opioid Knoll was promoting. This irresponsible marketing of a

narcotic drug caused doctors and patients to believe Vicodin was safer than it really was, to the detriment of people in Hidalgo County.

33. Abbvie began manufacturing, developing, promoting, marketing and selling the opioid drug, Vicodin, in the U.S. and in Hidalgo County beginning January 1, 2013. On information and belief, it continues to do so at the time of filing this pleading.

34. ALLERGAN PLC is a public limited company incorporated in Ireland with its principal place of business in Dublin, Ireland. ACTAVIS PLC acquired Allergan PLC in March of 2015, and the combined company changed its name to Allergan PLC in January 2013. Before that, WATSON PHARMACEUTICALS, INC. acquired ACTAVIS, INC. in October of 2012, and the combined company changed its name to ALLERGAN FINANCE, LLC as of October 2013. ALLERGAN FINANCE, LLC, is a Nevada Corporation with its principal place of business in Parsippany, New Jersey and may be served through its registered agent for service of process, The Corporation Trust Company of Nevada, 701 S. Carson St., Suite 200, Carson City, NV 89701. WATSON LABORATORIES, INC. is a Nevada corporation with its principal place of business in Corona, California and is a wholly-owned subsidiary of Allergan PLC f/k/a Actavis, Inc., f/k/a Watson Pharmaceuticals, Inc. and may be served through its registered agent for service of process, Corporate Creations Network, Inc., 8275 South Eastern Ave., #200, Las Vegas, NV 89123. ACTAVIS PHARMA, INC. f/k/a Actavis, Inc. is a Delaware corporation with its principal place of business in New Jersey and was formerly known as WATSON PHARMA, INC. and may be served through its registered agent for service of process, CT Corporation System, 1999 Bryan Street, Suite 900, Dallas, TX 75201. ACTAVIS LLC is a Delaware limited liability company with its principal place of business in Parsippany, New Jersey and may be served through its registered agent for service of process, Corporate Creations network, Inc., 3411 Silverside Rd., Tatnall

Building, Suite 104, Wilmington, DE 19810. Each of these Defendants is owned by Allergan PLC, which uses them to market and sell its drugs in the United States (collectively "Actavis").

35. Upon information and belief, Allergan PLC exercises control over these marketing and sales efforts and profits from the sale of Allergan/Actavis products, which ultimately inures to its benefit. Actavis acquired the rights to Kadian from King Pharmaceuticals, Inc. on December 30, 2008 and began marketing Kadian in 2009.

36. Actavis manufactures, promotes, sells, and distributes opioids in the U.S. and in Hidalgo County.

37. INSYS THERAPEUTICS, INC. is a Delaware corporation with its principal place of business in Chandler, Arizona. INSYS THERAPEUTICS, INC. can be served by serving its registered agent as follows: CT Corporation System, 1999 Bryan Street, Suite 900, Dallas, TX 75201. INSYS MANUFACTURING LLC is a duly formed Texas entity and resident and is a wholly owned subsidiary of INSYS THERAPEUTICS, INC. INSYS MANUFACTURING LLC's principal place of business is in Round Rock, Texas. INSYS MANUFACTURING LLC can be served by serving its registered agent as follows: CT Corporation System, 1999 Bryan Street, Suite 900, Dallas, TX 75201 (collectively "INSYS").

38. INSYS manufactures, promotes, sells and/or distributes opioids in the U.S. and in Hidalgo County.

39. MALLINCKRODT PLC ("Mallinckrodt") is an Irish public limited company with its corporate headquarters in Staines-Upon-Thames, Surrey, United Kingdom and maintains a U.S. headquarters in St. Louis, Missouri. MALLINCKRODT may be served by serving it registered agent CT Corporation System, 120 South Central Ave., Clayton, Missouri 63105.

40. Mallinckrodt manufactures, promotes, sells, and/or distributes opioids nationally and in Texas and Hidalgo County, including medications containing codeine, fentanyl, hydrocodone, morphine, and oxycodone. These opioid drugs are sold both directly by MALLINCKRODT and by third party drug distributors.

41. MCKESSON CORPORATION ("McKesson") is a Delaware corporation with its principal place of business in San Francisco, California and may be served through its registered agent for service of process, CSC-Lawyers Incorporating Service, 211 E. 7th Street, Suite 620, Austin, TX 78701. McKesson does substantial business in Texas, and upon information and belief, is a pharmaceutical distributor licensed to do business in Texas. McKesson distributes pharmaceuticals to retail pharmacies and institutional providers in all 50 states, including Texas and Hidalgo County.

42. CARDINAL HEALTH, INC. ("Cardinal") is an Ohio corporation with its principal place of business in Dublin, Ohio and may be served through its registered agent for service of process, CT Corporation System, 4400 Eastern Commons, Suite 125, Columbus, OH 43219. Cardinal does substantial business in Texas and, upon information and belief, Cardinal is a pharmaceutical distributor licensed to do business in Texas. Cardinal distributes pharmaceuticals to retail pharmacies and institutional providers to customers in all 50 states, including Texas and Hidalgo County.

43. AMERISOURCEBERGEN DRUG CORPORATION ("Amerisource") is a Delaware corporation with its principal place of business in Chesterbrook, Pennsylvania and may be served through its registered agent for service of process, The Corporation Trust Company, Corporation Trust Center 1209 Orange St., Wilmington, DE 19801. Amerisource does substantial business in Texas and, upon information and belief, Amerisource is a pharmaceutical distributor licensed to

do business in Texas. Amerisource distributes pharmaceuticals to retail pharmacies and institutional providers to customers in all 50 states, including Texas and Hidalgo County.

## VI. FACTUAL ALLEGATIONS

44. As a result of the opioid epidemic, oxycodone, hydrocodone, and fentanyl are among the most commonly diverted opioids in the United States. The Federal Drug Enforcement Agency's National Drug Threat Assessment for 2017 reported that more than 93.7 million prescriptions for hydrocodone were written in 2016, totaling more than 6.2 billion hydrocodone dosage units (pills) dispensed or sold in the U.S. More than 60 million prescriptions for oxycodone were written in 2016. Fentanyl was first synthesized in 1959 by Paul Janssen, of Janssen Pharmaceuticals, and entered the slow-release dermal patch pharmaceutical market in the 1990s. Fentanyl's use and diversion continues to grow in the U.S. market, and its illicit analogues are responsible for a growing number of opioid-related overdoses in the U.S.

45. The Northern and Southern borders to the U.S. continue to be common ports of entry for controlled prescription drugs diverted to the U. S. market. Diversion across the U.S. borders is yet another way in which prescription opioids are diverted into the U.S. market and counties like Hidalgo County, Texas.

46. This diversion of prescription opioids into the U.S. market from border countries is a loop-hole well known to manufacturers and distributors named in this petition. Unlimited distribution of opioids into border counties means that the opioid problem in the U.S. will continue, despite best efforts to curtail the diversion. Any decline in U.S. prescriptions is potentially meaningless without curtailing over-promotion and over-distribution immediately across the U.S. borders where countless prescription opiates enter into the U.S. undetected and available for diversion.

Countless units of the illicit analogues of fentanyl, derived from the Janssen formula, also continue to arrive through the U.S. border ports and are delivered through mail carriers.

47. Upon information and belief, the manufacturers, distributors, mail carriers, and potentially the DEA through export reports, maintain data and records that can identify the levels of prescription opioids that are sent by regulated U.S. entities into border counties or delivered illicitly into the U.S. Failure to eliminate the flow of opioids into border cities that are then potentially available for diversion through re-entry is necessary to ensure that the path for diversion of prescription opioids is fully closed.

48. Currently, no U.S. federal regulatory agency has any responsibility to ensure that diversion is not occurring on the U.S. borders for the purpose of creating a market for re-entry into the U.S. and diversion into counties like Hidalgo County, Texas. The control of prescription opioids that flow from the U.S. that are diverted into the U.S. across the border adds to the diversion of prescription opioids that is occurring within the U.S.

49. Before the 1990s, generally accepted standards of medical practice dictated that opioids should be used only for short-term acute pain – pain relating to recovery from major surgery or for cancer or palliative (end-of-life) care. Using opioids for chronic pain was discouraged or even prohibited because there was a lack of evidence that opioids improved patients' ability to overcome pain and function. Instead the evidence demonstrated that patients developed tolerance to opioids over time, which increased the risk of addiction and other side effects.

50. Defendants dramatically changed doctors' views regarding opioids through a well-funded deceptive marketing scheme. Each Defendant used direct marketing and unbranded advertising disseminated by seemingly independent third parties to spread false and deceptive statements about the risk and benefits of long-term opioid use.

A. Defendants Used Multiple Avenues to Disseminate Their False and Deceptive Statements About Opioids.

51. Upon information and belief, Defendants spread their false and deceptive statements by (1) marketing their branded opioids directly to doctors treating patients residing in Hidalgo County and the Hidalgo County patients themselves and (2) deploying so-called unbiased and independent third parties to Hidalgo County.

1. Defendants Spread and Continue to Spread Their False and Deceptive Statements Through Direct Marketing of Their Branded Opioids.

52. Defendants' direct marketing of opioids generally proceeded on two tracks. First, each Defendant conducted advertising campaigns touting the purported benefits of their branded drugs. For example, Defendants spent more than $14 million on medical journal advertising on opioids in 2011, nearly triple what they spent in 2001, including $8.3 million by Purdue, $4.9 million by Janssen and $1.1. million by Endo.

53. A number of Defendants' branded ads deceptively portrayed the benefits of opioids for chronic pain. For example, Endo distributed and made available on its website, www.opana.com, a pamphlet promoting Opana ER with photographs depicting patients with physically demanding jobs like a construction worker and chef, implying that the drug would provide long-term pain relief and functional improvement. Purdue also ran a series of ads, called "pain vignettes," for OxyContin in 2012 in medical journals. These ads featured chronic pain patients and recommended OxyContin for each. One ad described a "54-year-old-writer with osteoarthritis of the hands" and implied that OxyContin would help the writer work more effectively. Pursuant to a settlement agreement, Endo and Purdue agreed in late 2015 and 2016 to halt these misleading

representations in New York, but they may continue to disseminate them in Texas. Insys used pharmacy data acquired from third parties to track the success of competitor brands in their fentanyl spray market and ranked doctors from 1 to 10 according to the number of prescriptions written in the class (10 being the highest number of written prescriptions). Insys then targeted the high prescribing doctors to get them to prescribe Insys's opioid.[11]

54. Second, each Defendant promoted the use of opioids for chronic pain through "detailers" – sales representatives who visited individual doctors and medical staff in their offices – and small-group speaker programs. Defendants devoted massive resources to direct sales contacts with doctors. In 2014 alone, Defendants spent $168 million on detailing branded opioids to doctors, including $108 million by Purdue, $34 million by Janssen, $10 million by Endo, and $2 million by Actavis. This amount is twice as much as Defendants spent on detailing in 2000.[12]

55. Defendants also identified doctors to serve, for payment, on their speakers' bureaus and to attend programs with speakers and meals paid for by Defendants. These speaker programs provided: (1) an incentive for doctors to prescribe a particular opioid so they might be selected to promote the drug; (2) recognition and compensation for the doctors selected as speakers; and (3) an opportunity to promote the drug through the speaker to his or her peers. These speakers gave the false impression that they were providing unbiased and medically accurate presentations when they were, in fact, presenting a script prepared by Defendants. On information and belief, these presentations conveyed misleading information, omitted material information, and failed to correct Defendants' prior misrepresentations about the risks and benefits of opioids.

[11] The Federal indictment for Insys sets out allegations, among others, relating to the use of speaker fees to bribe high prescribing doctors to write prescriptions for Insys's Fentanyl Spray outside the usual course of their professional practice. See supra at note 8.

[12] In or about 2012, Insys engaged in speaker programs with physicians who were referred to as "Live Speaker Targets," and it was impressed upon representatives by the then CEO that "[t]hey do not need to be good speakers, they need to write a lot of … [prescriptions for the Fentanyl Spray]." See supra at note 8.

56. Upon information and belief, Defendants employed the same marketing plans, strategies, and message in and around Hidalgo County, Texas as they did nationwide. Across the pharmaceutical industry, "core message" development is funded and overseen on a national basis by corporate headquarters. This comprehensive approach ensures that Defendants' messages are consistently delivered across marketing channels and in each sales territory. Defendants consider this high level of coordination and uniformity crucial to successfully marketing their drugs.

2. Defendants Used a Diverse Group of Seemingly Independent Third Parties to Spread False and Deceptive Statements about the Risks and Benefits of Opioids.

57. Upon information and belief, Defendants also deceptively marketed opioids in and around Hidalgo County through unbranded advertising, i.e., advertising that promotes opioid use generally but does not name a specific opioid. This advertising was ostensibly created and disseminated by independent third parties. By funding, directing, reviewing, editing, and distributing this unbranded advertising, Defendants controlled the deceptive messages disseminated by these third parties and acted in concert with them to falsely and misleadingly promote opioids for treating chronic pain. Unbranded advertising also avoided regulatory scrutiny because Defendants did not have to submit it to the FDA; therefore, it was not reviewed by the FDA.

58. Defendants' deceptive unbranded marketing often contradicted their branded materials reviewed by the FDA. For example, Endo's unbranded advertising contradicted its concurrent, branded advertising for Opana ER:

| **Pain: Opioid Therapy (Unbranded)** | **Opana ER Advertisement (Branded)** |
|---|---|
| "People who take opioids **as prescribed usually do not become addicted."** | "All patients treated with opioids require careful monitoring for signs of abuse and addiction, since **use of opioid analgesic products carries the risk of addiction even under appropriate medical use.**" |

59. Defendants spoke through a small circle of doctors who, upon information and belief, were selected and funded by Defendants because their public positions supported using opioids to treat chronic pain. These doctors became known as "key opinion leaders" or "KOLs."

60. Defendants paid KOLs to serve as consultants or on their advisory boards and to give talks or present CMEs, and their support helped these KOLs become respected industry experts. As they rose to prominence, these KOLs touted the benefits of opioids to treat chronic pain, repaying Defendants by advancing their marketing goals. KOLs' professional reputations became dependent on continuing to promote a pro-opioid message, even in activities that were not directly funded by Defendants.

61. KOLs have written, consulted on, edited, and lent their names to books and articles and given speeches and CMEs supportive of chronic opioid therapy. Defendants created opportunities for KOLs to participate in research studies. Defendants suggested or chose and then cited and promoted favorable studies or articles by their KOLs. By contrast, Defendants did not support, acknowledge, or disseminate publications of doctors unsupportive or critical of chronic opioid therapy.

62. Defendants' KOLs also served on committees that developed treatment guidelines that strongly encourage using opioids to treat chronic pain and on the boards of pro-opioid advocacy groups and professional societies that develop, select, and present CMEs. Defendants were able to direct and exert control over each of these activities through their KOLs.

63. Pro-opioid doctors are one of the most important avenues that Defendants used to spread their false and deceptive statements about the risks and benefits of long-term opioid use. Defendants know that doctors rely heavily and less critically on their peers for guidance, and KOLs provide the false appearance of unbiased and reliable support for chronic opioid therapy.

64. Defendants utilized many KOLs, including many of the same ones. Two of the most prominent are described below.

(a) Key Opinion Leaders

65. Dr. Russel Portenoy, former Chairman of the Department of Pain Medicine and Palliative Care at Beth Israel Medical Center in New York, is one example of a KOL who Defendants identified and promoted to further their marketing campaign. Dr. Portenoy received research support, consulting fees, and honoraria from Endo, Janssen, and Purdue (among others), and was a paid consultant to Purdue.

66. Dr. Portenoy was instrumental in opening the door for the regular use of opioids to treat chronic pain. He served on the American Pain Society ("APS") and American Academy of Pain Medicine ("AAPM") Guidelines Committees, which endorsed the use of opioids to treat chronic pain, first in 1997 and again in 2009. He was also a member of the board of the American Pain Foundation ("APF"), an advocacy organization almost entirely funded by Defendants.

67. Dr. Portenoy also made frequent media appearances promoting opioids. He appeared on Good Morning America in 2010 to discuss using opioids long-term to treat chronic pain. On this widely-watched program, broadcast in Texas and across the country, Dr. Portenoy claimed: "Addiction, when treating pain, is distinctly uncommon. If a person does not have a history, a personal history, of substance abuse, and does not have a history in the family of substance abuse, and does not have a very major psychiatric disorder, most doctors can feel very assured that the person is not going to become addicted."[13]

[13] Good Morning America television broadcast, ABC News (Aug. 30, 2010).

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68. Dr. Portenoy later admitted that he "gave innumerable lectures in the late 1980s and 90s about addiction that weren't true."[14] These lectures falsely claimed that less than 1% of patients would become addicted to opioids. According to Dr. Portenoy, because the primary goal was to "destigmatize" opioids, he and other doctors promoting them overstated their benefits and glossed over their risks. Dr. Portenoy also conceded that "[d]ata about the effectiveness of opioids does not exist."[15] Dr. Portenoy candidly stated, "Did I teach about pain management, specifically about opioid therapy, in a way that reflects misinformation? Well … I guess I did."[16]

69. Another KOL, Dr. Lynn Webster, was the co-founder and Chief Medical Director of Lifetree Clinical Research, an otherwise unknown pain clinic in Salt Lake City, Utah. Dr. Webster was President in 2013 and is a current board member of AAPM, a Front Group that ardently supports chronic opioid therapy. He is a Senior Editor of Pain Medicine, the same journal that published Endo special advertising supplements touting Opana ER. Dr. Webster authored numerous CMEs sponsored by Endo and Purdue while he was receiving significant funding from Defendants.

70. In 2011, Dr. Webster presented a program via webinar sponsored by Purdue titled, "Managing Patient's Opioid Use: Balancing the Need and the Risk." Dr. Webster recommended using risk screening tools, such as urine testing and patient agreements, as a way to prevent "overuse of prescriptions" and "overdose deaths," which was available to and was, upon information and belief, intended to reach doctors treating Hidalgo County residents.

71. Dr. Webster also was a leading proponent of the concept of "pseudoaddiction," the notion that addictive behaviors should be seen not as warnings, but as indications of undertreated pain. In

[14] T. Catan & E. Perez, A Pain-Drug Champion Has Second Thoughts, Wall Street Journal (Dec. 17, 2012).
[15] Id.
[16] Id.

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Dr. Webster's description, the only way to differentiate the two was to increase the patient's dose of opioids. As he and his co-author wrote in a book that is still available online, when faced with signs of aberrant behavior, increasing the dose "in most cases … should be the clinician's first response." Endo distributed this book to doctors. Years later, Dr. Webster reversed himself, acknowledging that "[pseudoaddiction] obviously became too much of an excuse to give patients more medication."

(a) Front Groups

72. Defendants entered into arrangements with seemingly unbiased and independent patient and professional organizations to promote opioids for treating chronic pain. Under Defendants' direction and control, these "Front Groups" generated treatment guidelines, unbranded materials, and programs that favored chronic opioid therapy. They also assisted Defendants by responding to negative articles, by advocating against regulatory changes that would limit prescribing opioids in accordance with the scientific evidence, and by conducting outreach to vulnerable patient populations targeted by Defendants.

73. These Front Groups depended on Defendants for funding and, in some cases, for survival. Defendants also exercised control over programs and materials created by these groups by collaborating on, editing, and approving their content and by funding their dissemination. In doing so, Defendants made sure these Front Groups would generate only the messages Defendants wanted to distribute. Even so, the Front Groups held themselves out as independent and as serving the needs of their members – whether patients suffering from pain or doctors treating those patients.

74. Defendants Endo, Janssen, and Purdue utilized many Front Groups, including many of the same ones. Several of the most prominent are described below, but there are many others, including

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the American Pain Society ("APS"), American Geriatrics Society ("AGS"), the Federation of State Medical Boards ("FSMB"), American Chronic Pain Association ("ACPA"), American Society of Pain Education ("ASPE"), National Pain Foundation ("NPF"), and Pain and Policy Studies Group ("PPSG").

(i) American Pain Foundation ("APF")

The most prominent of Defendants' Front Groups was APF, which received more than $10 million in funding from opioid manufacturers from 2007 until it closed its doors in May of 2012. Endo alone provided more than half that funding; Purdue was next at $1.7 million.

In 2009 and 2010, more than 80% of APF's operating budget came from pharmaceutical industry sources. Including industry grants for specific projects, APF received about $2.3 million from industry sources out of a total income of about $2.85 million in 2009; its budget for 2010 projected receipts of roughly $2.9 million from drug companies out of total income of about $3.5 million. By 2011, APF was entirely dependent on incoming grants from defendants Purdue, Endo, and others to avoid using its line of credit. As one of its board members, Russell Portenoy, explained, the lack of funding diversity was one of the biggest problems at APF.

APF issued education guides for patients, reporters, and policymakers that touted the benefits of opioids for chronic pain and trivialized their risks, particularly the risk of addiction. APF also engaged in a significant multimedia campaign – through radio, television, and the internet – to educate patients about their "right" to pain treatment,

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namely opioids. All of the programs and materials were available nationally and were, upon information and belief, intended to reach patients and consumers in Hidalgo County.

(ii) American Academy of Pain Medicine ("AAPM")

The American Academy of Pain Medicine, with the assistance, prompting, involvement, and funding of Defendants, issued treatment guidelines and sponsored and hosted medical education programs essential to Defendants' deceptive marketing of chronic opioid therapy.

AAPM received over $2.2 million in funding since 2009 from opioid manufacturers. AAPM maintained a corporate relations council, whose members were paid $25,000 per year, on top of other funding, to participate. The benefits included allowing members to present educational programs at off-site dinner symposia in connection with AAPM's marquee event – its annual meeting held in Palm Springs, California or other resort locations. AAPM describes the annual event as an "exclusive venue" for offering education programs to doctors. Membership in the corporate relations council also allowed drug company executives and marketing staff to meet with AAPM executive committee members in small settings. Defendants Endo, Purdue, and Actavis were members of the council and presented deceptive programs to doctors who attended this annual event.

AAPM is viewed internally by Endo as "industry friendly" with Endo advisors and speakers among its active members. Endo attended AAPM conferences, funded its CMEs, and distributed its publications. The conferences sponsored by AAPM heavily emphasized sessions on opioids – 37 out of roughly 40 at one conference alone. AAPM's presidents

have included top industry-supported KOLs Perry Fine, Russel Portenoy, and Lynn Webster. Dr. Webster was even elected president of AAPM while under a DEA investigation. Another past AAPM president, Dr. Scott Fishman, stated that he would place the organization "at the forefront" of teaching that "the risks of addiction are … small and can be managed."[17]

AAPM's staff understood they and their industry funders were engaged in a common task. Defendants were able to influence AAPM through both their significant and regular funding and the leadership of pro-opioid KOLs within the organization.

In 1997, AAPM and the American Pain Society jointly issued a consensus statement, The Use of Opioids for the Treatment of Chronic Pain, which endorsed opioids to treat chronic pain and claimed there was a low risk that patients would become addicted to opioids. The co-author of the statement, Dr. Haddox, was at the time a paid speaker for Purdue. Dr. Portenoy was the sole consultant. The consensus statement remained on AAPM's website until 2011 and was taken down from AAPM's website only after a doctor complained, though it still lingers on the internet elsewhere.

AAPM and APS issued their own guidelines in 2009 ("AAPM/APS Guidelines") and continued to recommend using opioids to treat chronic pain. Fourteen of the 21 panel members who drafted the Guidelines (including KOLs Dr. Portenoy and Dr. Perry Fine of the University of Utah) received support from Janssen, Endo, and Purdue. The 2009 Guidelines promote opioids as "safe and effective" for treating chronic pain, despite acknowledging limited evidence, and conclude that the risk of addiction is manageable for patients regardless of past abuse histories. One panel member, Dr. Joel Saper, Clinical

[17] Interview by Paula Moyer with Scott M. Fishman, M.D., Professor of Anesthesiology and Pain Medicine, Chief of the Division of Pain Medicine, Univ. of Cal., Davis (2005), http://www.mescape.org/viewarticle/500829.

Professor of Neurology at Michigan State University and founder of the Michigan Headache & Neurological Institute, resigned from the panel because he was concerned the 2009 Guidelines were influenced by contributions that drug companies, including Defendants, made to the sponsoring organizations and committee members. These Guidelines have been a particularly effective channel of deception and have influenced not only treating physicians, but also the body of scientific evidence on opioids. The Guidelines have been cited 732 times in academic literature, were, upon information and belief, disseminated in and around Hidalgo County during the relevant time period, are still available online, and were reprinted in the Journal of Pain.

B. Defendants Marketing Scheme Misrepresented the Risks and Benefits of Opioids.

75. Upon information and belief, to convince doctors treating residents in Hidalgo County and Hidalgo County patients that opioids can and should be used to treat chronic pain, Defendants had to convince them that long-term opioid use is both safe and effective. Knowing they could do so only by deceiving those doctors and patients about the risks and benefits of long-term opioid use, Defendants made claims that were not supported by, or were contrary to, the scientific evidence. Even though pronouncements by and guidance from the FDA and the CDC based on that evidence confirm that their claims were false and deceptive, Defendants have not corrected them or instructed their KOLs or Front Groups to correct them and continue to spread them today.

C. Defendants Falsely Trivialized or Failed to Disclose the Known Risks of Long-Term Opioid Use.

76. To convince doctors and patients that opioids are safe, Defendants deceptively trivialized and failed to disclose the risks of long-term opioid use, particularly the risk of addiction, through a series of misrepresentations that have been conclusively debunked by the FDA and CDC. These misrepresentations – which are described below – reinforced each other and created the

dangerously misleading impression that: (1) starting patients on opioids was low-risk because most patients would not become addicted and because those who were at greatest risk of addiction could be readily identified and managed; (2) patients who displayed signs of addiction probably were not addicted and, in any event, could easily be weaned from the drugs; (3) the use of higher opioid doses, which many patients need to sustain pain relief as they develop tolerance to the drugs, do not pose special risks; and (4) abuse-deterrant opioids both prevent overdose and are inherently less addictive. Defendants have not only failed to correct these misrepresentations, they continue to make them today.

77. First, Defendants falsely claimed the risk of addiction is low and unlikely to develop when opioids are prescribed, as opposed to obtained illicitly, and failed to disclose the greater risk of addiction with prolonged use of opioids. For example:

(1) Actavis's predecessor caused a patient education brochure to be distributed in 2007 claiming opioid addiction is possible, but "less likely if you have never had an addiction problem." Upon information and belief, based on Actavis's acquisition of its predecessor's marketing materials along with the rights to Kadian, Actavis continued to use this brochure in 2009 and beyond;

(2) Purdue sponsored APF's Treatment Options: A Guide for People Living with Pain (2007), which instructed that addiction is rare and limited to extreme cases of unauthorized dose escalations, obtaining duplicative opioid prescriptions from multiple sources, or theft. This publication is still available online.;

(3) Endo sponsored a website, Painknowldge.com, which claimed in 2009 that "[p]eople who take opioids as prescribed usually do not become addicted." Another Endo website, PainAction.com, stated "Did you know? Most chronic

pain patients do not become addicted to the opioid medications that are prescribed for them;"

(4) Endo distributed a pamphlet with the Endo logo entitled Living with Someone with Chronic Pain, which stated that: "Most health care providers who treat people with pain agree that most people do not develop an addiction problem." A similar statement appeared on the Endo website, www.opana.com;

(5) Janssen reviewed, edited, approved, and distributed a patient education guide, entitled Finding Relief Pain Management for Older Adults (2009), which described as "myth" the claim that opioids are addictive and asserted as fact that "[m]any studies show that opioids are rarely addictive, when used properly for the management of chronic pain;"

(6) Janssen currently runs a website, Prescriberesponsibly.com (last updated July 2, 2015), which claims that concerns about opioid addiction are "overestimated;"

(7) Purdue sponsored APF's Policymaker's Guide to Understanding Pain & Its Management, which claims that less than 1% of children prescribed opioids will become addicted and that pain is undertreated due to "misconceptions about opioid addiction." This publication is still available online;

(8) Upon information and belief, detailers for Purdue, Endo, and Janssen in and around Hidalgo County minimized or omitted any discussion with doctors of the risk of addiction, misrepresented the potential for opioid abuse with purportedly abuse-deterrent formulations, and routinely did not correct the misrepresentations noted above;

(9) Insys's opioid, Subsys, was only indicated for breakthrough cancer pain. Yet Insys systematically misrepresented or omitted material information to agents of insurers and pharmacy benefit managers in order to get prior authorization for Subsys in patients off-label.

78. These claims contradict longstanding scientific evidence as the FDA and CDC have conclusively declared. As noted in the 2016 CDC Guideline endorsed by the FDA, there is "extensive evidence" of the "possible harms of opioids (including opioid use disorder [an alternative term for opioid addiction])."[18]

79. The FDA further exposed the falsity of Defendants' claims about the low risk of addiction when it announced changes to the labels for ER/LA opioids in 2013 and for opioids in 2016. In its announcements, the FDA discussed the risks related to opioid use and that instant release ("IR") opioids are associated with "persistent abuse, addiction, overdose mortality, and risk of NOWS [neonatal opioid withdrawal syndrome, now also referred to as NAS, neonatal abstinence syndrome]."[19]

80. According to the FDA, because of the risks associated with long-term opioid use, including "the serious risk of addiction, abuse, misuse, overdose, and death,"[20] opioids should be "reserved for pain severe enough to require opioid treatment and for which alternative treatment options (e.g., non-opioid analgesics or opioid combination products, as appropriate) are inadequate or not tolerated."[21]

[18] CDC Guidelines for Prescribing Opioids for Chronic Pain – United States 2016, Centers for Disease Control and Prevention (Mar. 18, 2016).
[19] FDA Announcement of Enhanced Warnings for Immediate-Release Opioid Pain Medications Related to Risks of Misuse, Abuse, Addiction, Overdose and Death, Federal Drug Administration (Mar. 22, 2016).
[20] Id.
[21] Id.

81. The warnings on Defendants' own FDA-approved drug labels caution that opioids "expose users to risks of addiction, abuse and misuse, which can lead to overdose and death"[22] and that addiction "can occur in patients appropriately prescribed"[23] opioids.

82. Second, Defendants falsely instructed doctors and patients that signs of addiction are actually signs of undertreated pain and should be treated by prescribing more opioids. Defendants called this phenomenon "pseudoaddiction" – a term coined by Dr. David Haddox, who went to work for Purdue, and popularized by Dr. Russell Portenoy, a KOL for Endo, Janssen, and Purdue – and claimed that pseudoaddiction is substantiated by scientific evidence. For example:

(1) Purdue sponsored Responsible Opioid Prescribing (2007), which taught that behaviors such as "requesting drugs by name," "demanding or manipulative behavior," seeking more than one doctor to obtain opioids, and hoarding, are all signs of pseudoaddiction, rather than true addiction. Responsible Opioid Prescribing remains for sale online. The 2012 edition continues to teach that pseudoaddiction is real;

(2) Janssen sponsored, funded, and edited the Let's Talk Pain website, which in 2009 stated: "pseudoaddiction … refers to patient behaviors that may occur when pain is under-treated … Pseudoaddiction is different from true addiction because such behaviors can be resolved with effective pain management;"

(3) Endo sponsored a National Initiative on Pain Control ("NIPC") CME program in 2009, titled Chronic Opioid Therapy: Understanding Risk While Maximizing Analgesia, which promoted pseudoaddiction by teaching that a patient's aberrant behavior was the result of untreated pain. Endo substantially controlled

[22] See, e.g., OxyContin label and insert at OxyContin.com.
[23] Id.

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NIPC by funding NIPC projects; developing, specifying, and reviewing content; and distributing NIPC materials;

(4) Purdue published a pamphlet in 2011 entitled Providing Relief, Preventing Abuse, which described pseudoaddiction as a concept that "emerged in the literature" to describe the inaccurate interpretation of [drug-seeking behaviors] in patients who have pain that has not been effectively treated;" and

(5) Purdue sponsored a CME program, entitled Path of the Patient, Managing Chronic Pain in Younger Adults at Risk for Abuse. In a role play, a chronic pain patient with a history of drug abuse tells his doctor that he is taking twice as many hydrocodone pills as directed. The narrator notes that because of pseudoaddiction, the doctor should not assume the patient is addicted even if he persistently asks for a specific drug, seems desperate, hoards medicine, or "overindulges in unapproved escalating doses." The doctor treats this patient by prescribing a high-dose, long-acting opioid.

83. The 2016 CDC Guideline rejects the concept of pseudoaddiction. The Guideline nowhere recommends that opioid dosages be increased if a patient is not experiencing pain relief. To the contrary, the Guideline explains that "[p]atients who do not experience clinically meaningful pain relief early in treatment … are unlikely to experience pain relief with longer-term use"[24] and that physicians should "reassess pain and function within 1 month" in order to decide whether to "minimize risks of long-term opioid use by discontinuing opioids" because the patient is "not receiving a clear benefit."[25]

[24] CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.
[25] Id.

84. Third, Defendants falsely instructed doctors and patients that addiction risk screening tools, patient contracts, urine drug screens, and similar strategies allow them to reliably identify and safely prescribe opioids to patients predisposed to addiction. These misrepresentations were especially insidious because Defendants aimed them at general practitioners and family doctors who lack the time and expertise to closely manage higher-risk patients. Defendants' misrepresentations made these doctors feel more comfortable prescribing opioids to their patients and patients more comfortable starting opioid therapy for chronic pain. For example:

(1) Endo paid for a 2007 supplement in the Journal of Family Prac written by a doctor who became a member of Endo's speaker's bureau in 2010. The supplement, entitled Pain Management Dilemmas in Primary Care: Use of Opioids, emphasized the effectiveness of screening tools, claiming that patients at high risk of addiction could safely receive chronic opioid therapy using a "maximally structure approach" involving toxicology screens and pill control;

(2) Purdue sponsored a 2011 webinar, Managing Patient's Opioid Use: Balancing the Need and Risk, which claimed that screening tools, urine tests, and patient agreements prevent "overuse of prescriptions" and "overdose deaths;" and

(3) As recently as 2015, Purdue has represented in scientific conferences that "bad apple" patients – and not opioids – are the source of the addiction crisis and that once those "bad apples" are identified, doctors can safely prescribe opioids without causing addiction.

85. Once again, the 2016 CDC Guideline confirms these representations are false. The Guideline notes that there are not studies assessing the effectiveness of risk mitigation strategies – such as screening tools, patient contracts, urine drug testing, or pill counts – widely believed by

doctors to detect and deter outcomes related to addiction and overdose.[26] As a result, the Guideline recognizes that doctors should not overestimate the risk screening tools for classifying patients as high or low risk for opioid addiction because they are insufficient to rule out the risks of long-term opioid therapy.[27]

86. Fourth, to underplay the risk and impact of addiction and make doctors feel more comfortable starting patients on opioids, Defendants falsely claimed that opioid dependence can easily be addressed by tapering and that opioid withdrawal is not a problem thereby failing to disclose the increased difficulty of stopping opioids after long-term use.

87. For example, a CME sponsored by Endo, entitled Persistent Pain in the Older Adults, claimed that withdrawal symptoms can be avoided by tapering a patient's opioid dose by 10%-20% for 10 days. Purdue sponsored APF's A Policymaker's Guide to Understanding Pain & Management, which claimed that "[s]ymptoms of physical dependence can often be ameliorated by gradually decreasing the dose of medication during discontinuation."

88. Defendants deceptively minimized the significant symptoms of opioid withdrawal, which, as explained in the 2016 CDC Guideline, include drug cravings, anxiety, insomnia, abdominal pain, vomiting, diarrhea, sweating, tremor, tachycardia (rapid heartbeat), spontaneous abortion, and premature labor in pregnant women and the unmasking of anxiety, depression, and addiction – and grossly understated the difficulty of tapering, particularly after long-term opioid use.

89. Yet the 2016 CDC Guideline recognizes that the duration of opioid use and the dosage of opioids prescribed should be limited to "minimize the need to taper opioids to prevent distressing or unpleasant withdrawal symptoms" because "physical dependence on opioids is an expected physiologic response in patients exposed to opioids for more than a few days." The Guideline

[26] CDC Guidelines for Prescribing Opioids for Chronic Pain, supra.
[27] Id.

further states that "tapering opioids can be especially challenging after years on high dosages because of physical and psychological dependence" and highlights the difficulties, including the need to carefully identify "a taper slow enough to minimize symptoms and signs of opioid withdrawal" and pausing and restarting tapers depending on the patient's response.

90. The CDC also acknowledges the lack of any "high-quality studies comparing the effectiveness of different tapering protocols for use when opioid dosage is reduced or opioids are discontinued."[28]

91. Fifth, Defendants falsely claimed that doctors and patients could increase opioid dosages indefinitely without added risk and failed to disclose the greater risks to patients at higher dosages. The ability to escalate dosages was critical to Defendants' efforts to market opioids for long-term use to treat chronic pain because, absent this misrepresentation, doctors would have abandoned treatment when patients built up tolerance and lower dosages did not provide pain relief. For example:

(1) Actavis's predecessor created a patient brochure for Kadian in 2007 that stated, "Over time, your body may become tolerant of your current dose. You may require dose adjustment to get the right amount of pain relief. This is not addiction." Upon information and belief, based on Actavis's acquisition of its predecessor's market materials along with the rights to Kadian, Actavis continued to use these materials in 2009 and beyond;

(2) Purdue sponsored APF's Treatment Options: A Guide for People Living with Pain (2007), which claims that some patients "need" a larger dose of an opioid regardless of the dose currently prescribed. The guide stated that opioids have

[28] Id.

"no ceiling dose" and are therefore the most appropriate treatment for severe pain. This guide is still available for sale online;

(3) Endo sponsored a website, painknowledge.com, which claimed in 2009 that opioid dosages may be increased until "you are on the right dose of medication for your pain;"

(4) Endo distributed a pamphlet edited by a KOL entitled Understanding Your Pain: Taking Oral Opioid Analgesics, which was available during the time period of this Compliant on Endo's website. In Q & A format, it asked "If I take the opioid now, will it work later when I really need it?" The response is, "The dose can be increased …. You won't 'run out' of pain relief;"

(5) Janssen sponsored a patient education guide entitled Finding Relief: Pain Management for Older Adults (2009), which was distributed by its sales forces. This guide listed dosage limitations as "disadvantages" of other pain medicines, but omitted any discussions of risks of increased opioid dosages;

(6) Purdue's In the Face of Pain website promotes the notion that if a patient's doctor does not prescribe what, in the patient's view, is a sufficient dosage of opioids, he or she should find another doctor who will;

(7) Purdue sponsored APF's A Policymaker's Guide to Understanding Pain & Its Management, which taught that dosage escalations are "sometimes necessary," even unlimited ones, but did not disclose the risks from high opioid dosages. This publication is still available online;

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(8) Purdue sponsored a CME, entitled Overview of Management Options, that is still available for CME credit. The CME was edited by a KOL and taught that NSAIDs and other drugs, but not opioids, are unsafe at high dosages; and

(9) Purdue presented a 2015 paper at the College on the Problems of Drug Dependence, the "oldest and largest organization in the US dedicated to advancing a scientific approach to substance use and addictive disorders," challenging the correlation between opioid dosage and overdose.

92. These claims conflict with the scientific evidence, as confirmed by the FDA and CDC. As the CDC explains in its 2016 Guideline, the "[b]enefits of high-dose opioids for chronic pain are not established" while the "risks for serious harms related to opioid therapy increase at higher opioid dosage."

93. More specifically, the CDC explains that "there is now an established body of scientific evidence showing that overdose risk is increased at higher opioid dosages." Similarly, there is an "increased risk for opioid use disorder, respiratory depression, and death at higher doses." That is why the CDC advises doctors to avoid increasing dosages above 90 morphine milligram equivalents per day.

94. The 2016 CDC Guideline reinforces earlier findings announced by the FDA. In 2013, the FDA acknowledged that available data suggested that increasing the opioid dosage likewise increased with certain advertisements. For example, the FDA noted that studies suggest a positive association between high-dose opioid use and overdoses.

95. Finally, Defendants' deceptive marketing of the so-called abuse-deterrent properties of some of their opioids created false impressions that these opioids can curb addiction and abuse.

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96. More specifically, Defendants made misleading claims about the ability of their so-called abuse-deterrent opioid formulations to deter addiction and overdose. For example, Endo's advertisements for the 2012 reformulation of Opana ER claimed that it was designed to be crush resistant in a way that suggested it was more difficult to misuse the product. This claim was false.

97. The FDA warned in a 2013 letter that there was no evidence Endo's design would provide a reduction in oral, intranasal or intravenous use.[29] Moreover, Endo's own studies, which it failed to disclose, showed that Opana ER could still be ground and chewed.

98. In a 2016 settlement with the State of New York, Endo agreed not to make statements in New York that Opana ER was designed to be or is crush resistant. The State found those statements false and deceptive because there was no difference in the ability to extract the narcotic from Opana ER.

99. Similarly, the 2016 CDC Guideline states that no studies support the notion that "abuse-deterrent technologies [are] a risk mitigation strategy for deterring or preventing abuse," noting that the technologies – even when they work – "do not prevent opioid abuse through oral intake, the most common route of opioid abuse, and can still be abused by non-oral routes."

100. These numerous, long-standing misrepresentations of the risks of long-term opioid use spread by Defendants successfully convinced doctors and patients to discount these risks.

D. Defendants Grossly Overstated the Benefits of Chronic Opioid Therapy.

101. To convince doctors and patients that opioids should be used to treat chronic pain, Defendants had to persuade them that there was a significant benefit to long-term opioid use. But, as the 2016 CDC Guideline makes clear, there is "insufficient evidence to determine the long-term benefits of opioid therapy for chronic pain."

[29] See FDA Statement: Original Opana ER Relisting Determination (May 10, 2013).

102. In fact, the CDC found no evidence showing "a long-term benefit of opioids in pain and function versus no opioids for chronic pain with outcomes examined at least 1 year later (with most placebo-controlled randomized trials … 6 weeks in duration)" and that other treatments were more or equally beneficial and less harmful than long-term opioid use. The FDA, too, has recognized the lack of evidence to support long-term opioid use.

103. In 2013, the FDA stated that it was unaware of any studies demonstrating the safety and efficacy of opioids for long-term use.[30] Despite this lack of studies, Defendants falsely and misleadingly touted the benefits of long-term use and suggested that these benefits were supported by scientific evidence. Not only have Defendants failed to correct these false and deceptive claims, they continue to make them today. For example:

(1) Actavis distributed an advertisement that claimed the use of Kadian to treat chronic pain would allow patients to return to work, relieve "stress on your body and your mental health," and help patients enjoy their lives;

(2) Endo distributed advertisements that claimed that the use of Opana ER for chronic pain would allow patients to perform demanding tasks like construction work or work as a chef and portrayed seemingly healthy, unimpaired subjects;

(3) Janssen sponsored and edited a patient education guide entitled Finding Relief: Pain Management for Older Adults (2009), which states as "a fact" that opioids may make it easier for people to live "normally." The guide lists expected functional improvements from opioid use, including sleeping through the night, returning to work, recreation, sex, walking, and climbing stairs;

[30] Letter from Janet Woodcock, M.D, Dir., Ctr. For Drug Eval. & Res., to Andrew Kolodny, M.D., President, Physicians for Responsible Opioid Prescribing, Re: Docket No. FDA-2012-P-0818 (Sept. 10, 2013).

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(4) Purdue ran a series of advertisements for OxyContin in 2012 in medical journals entitled "pain vignettes," which were case studies featuring patients with pain conditions persisting over several months and recommending OxyContin for them. The ads implied that OxyContin improves patients' function;

(5) Responsible Opioid Prescribing (2007), sponsored and distributed by Endo and Purdue, taught that relief of pain by opioids, by itself, improved patients' function. The book remains for sale online;

(6) Purdue sponsored APF's Treatment Options: A Guide for People Living with Pain (2007), which counseled patients that opioids "give [pain patients] a quality of life we deserve." The guide was available online until APF shut its doors in 2012.;

(7) Endo's NIPC website, painknowledge.com, claimed in 2009 that, with opioids, "your level of function should improve; you may find you are now able to participate in activities of daily living, such as work and hobbies, that you were not able to enjoy when your pain was worse." Elsewhere, the website touted improved quality of life as well as "improved function" as benefits of opioid therapy. The grant request that Endo approved for this project specifically indicated NIPC's intent to make misleading claims about function, and Endo closely tracked visits to the site;

(8) Endo was the sole sponsor, through NIPC, of a series of CMEs titled Persistent Pain in the Older Patient, which claimed that chronic opioid therapy has been "shown to reduce pain and improve depressive symptoms and cognitive functioning." The CME was disseminated via webcast;

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(9) Janssen sponsored, funded, and edited a website, Let's Talk Pain in the Older Patient, which claimed that chronic opioid therapy has been "shown to reduce pain and improve depressive symptoms and cognitive functioning." This video is still available today on YouTube;

(10) Purdue sponsored the development and distribution of APF's A Policymaker's Guide to Understanding Pain & its Management, which claimed that "multiple clinical studies" have shown that opioids are effective in improving daily function, psychological health, and health-related quality of life for chronic pain patients." The Policymaker's Guide was originally published in 2011 and is still available online today; and

(11) Purdue's, Endo's, and Janssen's sales representatives have conveyed and continue to convey the message that opioids will improve patient function.

104. These claims find no support in the scientific literature. Most recently, the 2016 CDC Guideline, approved by the FDA, concluded, "There is no good evidence that opioids improve pain or function with long-term use" and "complete relief of pain is unlikely." The CDC reinforced this conclusion throughout its 2016 Guideline by stating that: (a) no evidence shows a long-term benefit of opioids in pain and function versus no opioids for chronic pain with outcomes at least 1 year later; (b) although opioids can reduce pain during short-term use, the clinical evidence review found insufficient evidence to determine whether pain relief is sustained and whether function or quality of life improves with long-term opioid therapy; and (c) evidence is limited or insufficient for improved pain or function with long-term use of opioids for severe chronic pain conditions for which opioids are commonly prescribed, such as low back pain, headache, and fibromyalgia.

105. The CDC also noted that the risks of addiction and death "can cause distress and inability to fulfill major role obligations." As a matter of common sense and medical evidence, drugs that can kill patients or commit them to a life of addiction or recovery do not improve their function and quality of life.

106. The 2016 CDC Guideline was not the first time a federal agency repudiated Defendants' claims that opioids improved function and quality of life. In 2010, the FDA warned Actavis that "[w]e are not aware of substantial evidence or substantial clinical experience demonstrating that the magnitude of the effect of the drug [Kadian] has in alleviating pain, taken together with any drug-related side effects patients may experience … results in any overall positive impact on a patient's work, physical and mental functioning, daily activities, or enjoyment of life."[31]

107. Defendants also falsely emphasized or exaggerated the risks of competing products like NSAIDs so that doctors and patients would look to opioids first for treating chronic pain. Once again, Defendants' misrepresentations contravene pronouncements by and guidance from the FDA and CDC based on the scientific evidence.

108. Consequently, the FDA changed the labels for ER/LA opioids in 2013 and IR opioids in 2016 to state that opioids should be used only as a last resort when alternative treatments like non-opioid drugs are inadequate. And the 2016 CDC Guideline states that NSAIDs, not opioids, should be the first-line treatment for chronic pain, particularly arthritis and lower back pain.

109. In addition, Purdue misleadingly promoted OxyContin as unique among opioids in providing 12 continuous hours of pain relief with one dose. In fact, OxyContin does not last for 12 hours – a fact that Purdue has known at all times relevant to this action.

[31] Warning letter from Thomas Abrams, Dir., FDA Div. of Mktg., Adver., & Comm's, to Doug Boothe, CEO, Actavis LLC (Feb. 18, 2010), http://www.fda.gov/Drugs/GuidanceComplianceRegulatoryinformation/EnforcementActivitiesbyFDA/WarningLetterandNoticeofviolationLetterstoPharmaceuticalCcompanies/ucm259240.html.

110. According to Purdue's own research, OxyContin wears off in under six hours in one quarter of patients and in under 10 hours in more than half. The reason is that OxyContin tablets release approximately 40% of their active medicine immediately, after which release tapers. Although the patient experiences a powerful initial response, there is little or no pain relief at the end of the dosing period because less medicine is released.

111. This phenomenon is known as "end of dose" failure, and the FDA found in 2008 that a substantial number of chronic pain patients taking OxyContin experience it.

112. This "end of dose" failure not only renders Purdue's promise of 12 hours of relief false and deceptive, it also makes OxyContin more dangerous because the declining pain relief patients experience toward the end of each dosing period drives them to take more OxyContin before the next dosing period begins, quickly increasing the amount of drug they are taking and spurring growing dependence.

113. Purdue's competitors were aware of this problem. For example, Endo ran advertisements for Opana ER referring to "real" 12-hour dosing. Nevertheless, Purdue falsely promoted OxyContin as if it were effective for a full 12 hours. Upon information and belief, Purdue's sales representatives continue to tell doctors in and around Hidalgo County that OxyContin lasts a full 12 hours.

E. Defendants also Engaged in Other Unlawful, Unfair, and Fraudulent Misconduct.

114. Defendants herein participated in illicit and unlawful prescribing of opioids. For example, Purdue did not report illegal prescribing of OxyContin until years after law enforcement shut down a Los Angeles clinic that prescribed more than 1.1 million OxyContin tablets. In doing so, Purdue protected its own profits at the expense of public health and safety.

115. The State of New York found that Endo failed to require sales representatives to report signs of addiction, diversion, and inappropriate prescribing; paid bonuses to sales representatives for detailing prescribers who were subsequently arrested or convicted for illegal prescribing; and failed to prevent sales representative from visiting prescribers whose suspicious conduct had caused them to be place on a no-call list.

F. Defendants Targeted Susceptible Prescribers and Vulnerable Patient Populations.

116. As part of their deceptive marketing scheme, Defendants identified and targeted susceptible prescribers and vulnerable patient populations in the U.S. and, upon information and belief, in and around Hidalgo County. For example, Defendants focused their deceptive marketing on primary care doctors, who were more likely to treat chronic pain patients and prescribe opioids, but were less likely to be educated about treating pain and the risks and benefits of opioids.

117. Defendants also targeted vulnerable patient populations like the elderly and veterans who tend to suffer from chronic pain. Defendants targeted these vulnerable patients even though their risks of long-term opioid use were significantly greater.

118. For example, the 2016 CDC Guideline observes that existing evidence shows that elderly patients taking opioids suffer from elevated fall and fracture risks, greater risk of hospitalization, and increased vulnerability to adverse drug effects and interactions. The Guideline therefore concludes that there are "special risks of long-term opioid use for elderly patients" and recommends that doctors use "additional caution and increased monitoring" to minimize the risks of opioid use in elderly patient."

119. The same is true for veterans, who are more likely to use anti-anxiety drugs (benzodiazepines) for post-traumatic stress disorder, which interact dangerously with opioids.

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G. Although Defendants Knew that their Marketing of Opioids was False and Deceptive, and they Fraudulently Concealed their Misconduct.

120. Defendants, both individually and collectively, made, promoted, and profited from their misrepresentations about the risks and benefits of opioids for chronic pain even though they knew their misrepresentations were false and deceptive. The history of opioids, as well as research and clinical experience over the last 20 years, established that opioids were highly addictive and responsible for a long list of very serious adverse outcomes.

121. Not only did the FDA and other regulators warn Defendants, but Defendants had access to scientific studies, detailed prescription data, and reports of adverse events, including reports of addiction, hospitalization, and deaths – all of which made clear the harms from long-term opioid use, including the suffering from addiction, overdoses, and death in alarming numbers in patients using opioids.

122. More recently, the FDA and CDC have issued pronouncements based on the medical evidence that conclusively expose the known falsity of Defendants' misrepresentations, and Endo and Purdue have recently entered agreements prohibiting them from making some of the same misrepresentations described herein in New York.

123. Specifically, three current and former executives from Purdue plead guilty in 2007 to criminal charges that they misled regulators, doctors, and patients about OxyContin's risk of addiction.[32] In pleading guilty to misbranding charges, Purdue admitted it had fraudulently marketed OxyContin as a drug less-prone to addiction and as having fewer side effects than other opioids.[33] In reality, unlike other opioids, OxyContin contained pure oxycodone without any other

[32] See Barry Meier, "In Guilty Plea, OxyContin Maker to Pay $600 Million," (May 10, 2007), http://www.nytimes.com/2007/05/10/business/11drug-web.html.
[33] See Id.

ingredients, which made it a powerful narcotic despite its time-release design that Purdue touted as ameliorating its addictive potential.[34]

124. Moreover, Defendants took steps to avoid detection of and to fraudulently conceal their deceptive marketing and unlawful, unfair, and fraudulent conduct. For example, Defendants disguised their own role in the deceptive marketing of chronic opioid therapy by funding and working through third parties like Front Groups and KOLs.

125. Finally, Defendants manipulated their promotional materials and the scientific literature to make it appear that these items were accurate, truthful, and supported by objective evidence when they were not.

126. Thus, Defendants successfully concealed from the medical community and patients facts sufficient to arouse suspicion of the claims Hidalgo County now asserts. Hidalgo County did not know of the existence or scope of Defendants' industry-wide fraud and could not have acquired such knowledge earlier through the exercise of reasonable diligence.

H. By Increasing Opioid Prescriptions and Use Defendants' Deceptive Marketing Scheme has fueled the Opioid Epidemic and Devastated Hidalgo County Communities.

127. Defendants' misrepresentations deceived doctors and patients about the risks and benefits of long-term opioid use. Studies reveal that many doctors and patients are unaware of or do not understand the risks or benefits of opioids. Indeed, patients often report that they were not warned they might become addicted to opioids prescribed to them. As reported in January 2016, a 2015 survey of more than 1,000 opioid patients found that 4 out of 10 were not told opioids were potentially addictive.[35]

[34] See Id.
[35] Hazelden Betty Ford Foundation, Missed Questions, Missed Opportunities. (Jan. 27, 2016), available at http://www.hazeldenbettyford.org/about-us/news-and-media/pressrelease/doctors-missing-questions-that-could-prevent-opioid-addiction.

128. Upon information and belief, Defendants' deceptive marketing scheme caused and continues to cause doctors in and around Hidalgo County to prescribe opioids for chronic pain conditions such as back pain, headaches, arthritis, and fibromyalgia. Absent Defendants' deceptive marketing scheme, these doctors would not have been able to over prescribe opioids or become embroiled in pill mills that negatively impacted residents of Hidalgo County.

129. For example, upon information and belief, Defendants' deceptive marketing scheme allowed doctors located in or near Hidalgo County, Texas to promote, overprescribe, and financially benefit from prescribing opioids. Indeed, some doctors "knowingly or intentionally manufactured, distributed, dispensed, or possessed with the intent to manufacture, distribute, or dispense a controlled substance, including opioids such as OxyContin, Hydrocodone, Vicodin, or Fentanyl in violation of the Texas Controlled Substances Act in Tex. Health & Safety Code § 481.001 et seq.

130. If the manufacturing and distributing Defendants were not over-supplying opioids, then physicians could not devise schemes to prescribe opioids without a legitimate purpose as a means to flood the open market with opioids, such as OxyContin, Hydrocodone, Vicodin, and Fentanyl.

131. While Defendants may claim the federal government authorized the amount of annual prescription opioids sold, they know in truth that several Defendants have successfully used their organized money and influence to render the federal government's enforcement agency, the Drug Enforcement Administration, virtually powerless to interrupt the over-supply of prescription opioid drugs.

132. Defendants' deceptive marketing scheme also caused and continues to cause patients to purchase and use opioids for their chronic pain believing they are safe and effective. Absent

Defendants' deceptive marketing scheme, fewer patients would be using opioids long-term to treat chronic pain, and those patients using opioids would be using less of them.

133. Defendants' deceptive marketing has caused and continues to cause the prescribing and use of opioids to explode. Indeed, this dramatic increase in opioid prescriptions and use corresponds with the dramatic increase in Defendants' spending on their deceptive marketing scheme. Defendants' spending on opioid marketing totaled approximately $91 million in 2000. By 2011, that spending had tripled to $288 million.

134. The escalating number of opioid prescriptions written by doctors who were deceived by Defendants' deceptive marketing scheme is the cause of a correspondingly dramatic increase in opioid addiction, overdoses, or death through the U.S. and Hidalgo County.

135. Scientific evidence demonstrates a strong correlation between opioid prescriptions and becoming addicted to opioids. In a 2016 report, the CDC explained that prescribing opioids has quadrupled since 1999, which has resulted in a parallel increase in opioid overdose.[36] Indeed, there has been a two-third increase in overdose deaths from using opioids since 2000.[37] For these reasons, the CDC concluded that efforts to rein in the prescribing of opioids for chronic pain are critical "to reverse the cycle of opioid pain medication misuse that contributes to the opioid overdose epidemic."[38]

136. Due to the increase in opioid overdoses, first responders such as police officers have been and will continue to be in the position to assist people experiencing opioid-related overdoses.[39] In

[36] CDC, national Vital Statistics System, Mortality, CDCWONDER, (2016), https://wonder.cdc.gov/; Rudd, RA, Seth P, David F, Scholl L, Increases In Drug and Opioid-Involved Overdose Deaths-United States 2010-2015, MMWR Rep. (16 Dec. 2016).
[37] MMWR (1 Jan., 2016), Increases in Drug and Opioid Deaths - United States 2000-2014.
[38] CDC Guidelines for Prescribing Opioids for Chronic Pain, supra; see also supra at note 34.
[39] Opinion of the Attorney General of Texas, KP-0168 (Oct. 4, 2017).

2016, "over 1,200 law enforcement departments nationwide carried naloxone in an effort to prevent opioid-related deaths." [40]

137. Upon information and belief, Defendants' deceptive marketing scheme has also detrimentally impacted children in Hidalgo County. Overprescribing opioids for chronic pain has made the drugs more accessible to school-aged children, who come into contact with opioids after they have been prescribed to friends or relatives in the same household.

138. Upon information and belief, Defendants' conduct has adversely affected Hidalgo County's child protection agencies in the number of children in foster care driven by parental drug addiction. Children with parents addicted to drugs tend to stay in foster care longer, and they often enter the system having experienced significant trauma, which makes these cases more expensive for counties like Hidalgo County.

139. Upon information and belief, opioid addiction is a significant reason that Hidalgo County residents seek treatment for substance dependence. A significant number of admissions for drug addiction were associated with a primary diagnosis of opiate addiction or dependence.

140. Upon information and belief, Defendants' creation, through false and deceptive advertising and other unlawful and unfair conduct, of a virtually limitless opioid market has significantly harmed communities in Hidalgo County. Defendants' success in extending the market for opioids to new patients and chronic pain conditions has created an abundance of drugs available for non-medical and criminal use and fueled a new wave of addiction and injury. It has been estimated that 60%-80% of the opioids to which people are addicted come, directly or indirectly, through doctors' prescriptions.

[40] Id. (citing http://www.nchrc.org/law-enforcement/us-law-enforcement-who-carry-naloxone/)

141. Law enforcement agencies have increasingly associated prescription drug addiction with violent and property crimes. Despite strict federal regulation of prescription drugs, local law enforcement agencies are faced with increasing diversion from legitimate sources for illicit purposes, including doctor shopping, forged prescriptions, falsified pharmacy records, and employees who steal from their place of employment. The opioid epidemic has prompted a growing trend of crimes against pharmacies including robbery and burglary. This ongoing diversion of prescription narcotics creates a lucrative marketplace.

142. The rise in opioid addiction caused by Defendants' deceptive marketing schemes has also resulted in an explosion in heroin use. For example, heroin use has more than doubled in the past decade among adults aged 18 to 25 years.[41] Moreover, heroin-related overdoses in the United States has more than quadrupled since 2010.[42]

143. The costs and consequences of opioid addiction are staggering. For example, in 2007, the cost of healthcare due to opioid addiction and dependence was estimated at $25 billion, the cost of criminal justice was estimated at $5.1 billion, and the cost of lost workplace productivity was estimated at $25.6 billion.

144. Consequently, prescription opioid addiction and overdose have an enormous impact on the health and safety of individuals, as well as communities at large, because the consequences of this epidemic reach far beyond the addicted individual.

145. Upon information and belief, some of the repercussions for residents of Hidalgo County include job loss, loss of custody of children, physical and mental health problems, homelessness, and incarceration, which results in instability in communities often already in economic crisis and

[41] CDC, Vital Signs: Today's Heroin Epidemic – More People at Risk, Multiple Drugs Abused; https://www.cdc.gov/vitalsigns/heroin/index.html.
[42] Id.

contributes to increased demand on community services such as hospitals, courts, child services, treatment centers, and law enforcement.

146. Defendants knew and should have known about these harms that their deceptive marketing has caused and continues to cause and will cause in the future. Defendants closely monitored their sales and the habits of prescribing doctors. Their sales representatives, who visited doctors and attended CMEs, knew which doctors were receiving their messages and how they were responding.

147. Defendants also had access to and carefully watched government and other data that tracked the explosive rise in opioid use, addiction, injury, and death. Defendants not only knew, but intended, that their misrepresentations would persuade doctors to prescribe and encourage patients to use their opioids for chronic pain.

148. Defendants' actions are neither permitted nor excused by the fact that their drug labels may have allowed, or did not exclude, the use of opioids for chronic pain. FDA approval of opioids for certain uses did not give Defendants license to misrepresent the risks and benefits of opioids. Indeed, Defendants' misrepresentations were directly contrary to pronouncements by, and guidance from, the FDA based on the medical evidence and their own labels.

149. Nor is Defendants' causal role broken by the involvement of doctors. Defendants' marketing efforts were ubiquitous and highly persuasive. Their deceptive messages tainted virtually every source doctors could rely on for information and prevented them from making informed treatment decisions. Defendants also hijacked what doctors wanted to believe – namely, that opioids represented a means of relieving their patients' suffering and of practicing medicine more compassionately. Moreover, opioids are among the drugs that require a Medication Guide be provided directly to the patient that sets out in plain English the risks. It is the obligation of the manufacturers and those in the chain of distribution to ensure that the Medication Guide sets out

the risks of the opioid use in a way that the patient understands, regardless of what warnings the medical provider may or may not give. This obligation is non-delegable.

150. Upon information and belief, Defendants' actions and omissions were each a cause-in-fact of Hidalgo County's past and future damages. Upon information and belief, Defendants' wrongful conduct caused injuries to Hidalgo County in the past, continues to cause injuries to Hidalgo County, and will continue to cause injuries to Hidalgo County in the future. Such future damages include, but are not limited to, additional resources for getting people into appropriate treatment detoxification, counseling and medication assisted treatment of addicts, and continuing care in recovery programs related to substance use disorder, medical treatment for overdoses, life skills training for adolescents, increased law enforcement, and additional resources to treat the psychological effects of opioids and the underlying conditions that make people susceptible to opioid addiction.

I. Defendants' Fraudulent Marketing Has Led to Record Profits.

151. While using opioids has taken a toll on Hidalgo County and its residents, Defendants have realized blockbuster profits. In 2014 alone, opioids generated $11 billion in revenue for drug companies like Defendants. Indeed, financial information indicates that each Defendant experienced a material increase in sales, revenue, and profits from the false and deceptive advertising and other unlawful and unfair conduct described above.

## VII. FIRST CAUSE OF ACTION AGAINST ALL DEFENDANTS: PUBLIC NUISANCE

152. Hidalgo County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Petition as though fully set forth herein.

153. Upon information and belief, Defendants knowingly encouraged doctors in and around Hidalgo County to prescribe, and residents to use, highly addictive opioids for chronic pain, even though Defendants knew using opioids had a high risk of addiction and reduced quality of life.

154. Upon information and belief, by doing so, Defendants purposefully interfered with Hidalgo County's public health, public safety, public peace, public comfort, and public convenience.

155. Upon information and belief, Defendants, individually and in concert with each other, have contributed to and/or assisted in creating and maintaining a condition that is harmful to the health and safety of Hidalgo County residents and/or unreasonably interferes with the peace and comfortable enjoyment of life in violation of Texas law.

156. The public nuisance created by Defendants' actions is substantial and unreasonable – it has caused and continues to cause significant harm to the community – and the harm inflicted outweighs offsetting benefit. The staggering fact of opioid use resulting from Defendants' marketing efforts have caused, and continues to cause, harm to the community including, but not limited to:

(a) Upwards of 30% of all adults use opioids. These high rates of use have led to unnecessary opioid addiction, overdose, injuries, and deaths;

(b) Children have been exposed to opioids prescribed to family members or others resulting in injury, addiction, and death. Upon information and belief, easy access to prescription opioids has made opioids a recreational drug of choice among teenagers; opioid use among teenagers is only outpaced by marijuana use. Even infants have been born addicted to opioids due to prenatal exposure causing severe withdrawal symptoms and lasting developmental impacts;

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(c) Upon information and belief, residents of Hidalgo County who have never taken opioids have endured both the emotional and financial costs of caring for loved ones addicted to or injured by opioids and the loss of companionship, wages, or other support from family members who have used, become addicted to, overdose on, or been killed by opioids;

(d) Upon information and belief, more broadly, opioid use and addiction has driven Hidalgo County residents' health care costs higher;

(e) Employers have lost the value of productive and healthy employees who have suffered from adverse consequences from opioid use;

(f) Defendants' success in extending the market for opioids to new patients and chronic conditions has created an abundance of drugs available for criminal use and fueled a new wave of addiction and injury. Defendants' scheme created both the supply of narcotics to sell and the demand of addicts to buy them;

(g) This demand has created additional illicit markets in prescription opiates, including those that return to the U.S. and communities like Hidalgo County, and other opiates, particularly heroin. The low cost of these drugs has led some of those who initially become addicted to prescription opioids to migrate to cheaper opiates, fueling a new epidemic in the process;

(h) Upon information and belief, diverting opioids into secondary, criminal markets and increasing the number of individuals who are addicted to opioids has increased the demands on emergency services and law enforcement in Hidalgo County;

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(i) All of Defendants' actions have caused significant harm to the community – in lives lost; addictions endured; the creation of an illicit drug market; and all its concomitant crime and costs; unrealized economic productivity; and broken families and homes;

(j) Upon information and belief, these harms have taxed the human, medical, public health, law enforcement, and financial resources of Hidalgo County; and

(k) Defendants' interference with the comfortable enjoyment of life of a substantial number of people is entirely unreasonable because there is limited social utility to opioid use, and any potential value is outweighed by the gravity of harm inflicted by Defendants' actions.

158. Defendants knew, or should have known, that promoting opioid use would create a public nuisance in the following ways:

(a) Upon information and belief, Defendants have engaged in massive production, promotion, and distribution of opioids for use by the citizen of Hidalgo County;

(b) Defendants' actions created and expanded the market for opioids, promoting its wide use for pain management;

(c) Defendants knew or should have known that their promotion would lead to addiction and other adverse consequences that the larger community would suffer as a result.

159. Upon information and belief, Defendants' actions were, at the least, a substantial factor in doctors and patients not accurately assessing and weighing the risks and benefits of opioids for chronic pain therapy causing opioids to become widely available and used in Hidalgo County.

160. Without Defendants' actions, opioid use would not have become so widespread, and the enormous public health hazard of opioid addiction would not have existed and could have been averted.

161. Upon information and belief, the health and safety of the citizens of Hidalgo County, including those who use, have used, or will use opioids, as well as those affected by opioid users, is a matter of great public interest and legitimate concern to Hidalgo County's citizens and residents.

162. The public nuisance created, perpetuated, and maintained by Defendants can be abated and further reoccurrence of such harm and inconvenience can be prevented.

163. Upon information and belief, Defendants' conduct has affected and continues to affect a considerable number of people within Hidalgo County and is likely to continue to cause significant harm to patients who take opioids, their families, and the community at large.

164. Each Defendant created or assisted in creating the opioid epidemic, and each Defendant is jointly and severally liable for its abatement. Furthermore, each Defendant should be enjoined from continuing to create, perpetuate, or maintain said public nuisance in Hidalgo County.

## VIII. SECOND CAUSE OF ACTION AGAINST ALL DEFENDANTS: COMMON LAW FRAUD

165. Hidalgo County re-alleges and incorporates by reference each of the allegations in contained in the preceding paragraphs of this Petition as if fully set forth herein.

166. At all relevant and material times, Defendants expressly and/or impliedly warranted that opioids were safe, of merchantable quality, and fit for use.

167. Defendants' superior knowledge and expertise, their relationship of trust and confidence with doctors and the public, their specific knowledge regarding the risks and dangers of opioids, their intentional dissemination of promotional and marketing information about opioids, and the

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purpose of maximizing sales each gave rise to the affirmative duty to meaningfully disclose and provide all material information about the risks and harms associated with opioids.

168. At all times herein mentioned, Defendants, individually and acting through their employees and agents and in concert with each other, fraudulently represented to physicians, who Defendants knew would justifiably rely on Defendants' representations, that opioids were safe and effective for treating chronic pain.

169. Upon information and belief, Defendants' false representations were fraudulently made with the intent of purpose that healthcare providers and patients would justifiably rely upon them leading to the prescription, administration, filling, purchasing, and consumptions of opioids in Hidalgo County.

170. Defendants' deliberate misrepresentations and/or concealment, suppression, and omission of material facts as alleged herein include, but are not limited to:

(a) Making false and misleading claims regarding the known risks of the addictive nature of opioids and suppressing, failing to disclose, and mischaracterizing the addictive nature of opioids and in concomitant costs, such as overdoses, deaths, and heroin addictions;

(b) Making false and misleading written and oral statements that opioids are more effective than traditional pain killers for chronic pain or effective at all and/or omitting material information showing that opioids are not more effective than other non-addictive drugs for chronic pain;

(c) Issuing false and misleading warnings and/or failure to issue adequate warnings concerning the risks and dangers of using opioids;

(d) Making false and misleading claims downplaying the risk of addiction when using opioids and/or setting forth guidelines that would purportedly identify addictive behavior; and

(e) Making false and misleading misrepresentations concerning the safety, efficacy, and benefits of opioids without full and adequate disclosure of the underlying facts that rendered such statements false and misleading.

171. Defendants willfully, wantonly, and recklessly disregarded their duty to provide truthful representations regarding the safety and risks of opioids.

172. Defendants made these misrepresentations with the intent that the healthcare community and patients located wherever these opioid drugs were sold or consumed would rely upon them.

173. Defendants' misrepresentations were made with the intent of defrauding and deceiving the medical community to prescribe and consumers to take opioids.

174. Upon information and belief, Defendants' fraudulent representations evidence their callous, reckless, willful, and depraved indifference to the health, safety, and welfare of consumers living in Hidalgo County.

175. Defendants omitted, misrepresented, suppressed, and concealed material facts concerning the dangers and risk of injuries associated with the use of opioids as well as the fact that the product was unreasonably dangerous.

176. Upon information and belief, Defendants' purpose was willfully blind to, ignored, downplayed, avoided, and/or otherwise understated the serious nature of the risks associated with the use of opioids.

177. Upon information and belief, the treating medical community and consumers in Hidalgo County did not know that Defendants' representations were false and/or misleading and justifiably relied on them.

178. Defendants had sole access to material facts concerning the dangers and unreasonable risks of opioids, which they intentionally concealed.

179. Upon information and belief, as a direct and proximate result of Defendants' fraudulent misrepresentations and intentional concealment of facts upon which the medical community and consumers in Hidalgo County reasonably relied, Hidalgo County suffered actual and punitive damages.

## IX. THIRD CAUSE OF ACTION AGAINST ALL DEFENDANTS: NEGLIGENCE

180. Hidalgo County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Petition as if sully set forth herein.

181. Manufacturing Defendants have a duty to exercise reasonable care in marketing their opioids to physicians treating residents of Hidalgo County and Hidalgo County residents. Manufacturing Defendants have breached their duty by knowingly and fraudulently misrepresenting the benefits of, and downplaying the risks of, opioids for chronic pain.

182. Manufacturing Defendants have used deceitful marketing ploys, KOLs, Front Groups, and other schemes to increase profits at the cost of public health causing an opioid epidemic. Manufacturing Defendants have acted willfully, wantonly, and maliciously.

183. Likewise, Distributor Defendants have a duty to exercise ordinary care in distributing opioids. Distributor Defendants have breached their duty by failing to prevent or reduce the distribution of opioids or to report the increase in the distribution and/or sale of opioids.

184. Distributor Defendants intentionally failed to prevent or reduce the distribution of opioids or to report any increases in the sale of opioids so that they could increase profits and receive rebates or kick-backs from Manufacturing Defendants. Distributor Defendants have acted willfully, wantonly, and maliciously.

185. Upon information and belief, as a proximate result, Manufacturing and Distributor Defendants and their agents have caused Hidalgo County to incur excessive costs to treat the opioid epidemic in its County, including, but not limited to, increased costs of social services, health systems, law enforcement, judicial system, and treatment and recovery facilities.

186. Hidalgo County and its residents are therefore entitled to actual and punitive damages.

## X. FOURTH CAUSE OF ACTION AGAINST ALL DEFENDANTS: GROSS NEGLIGENCE

187. Hidalgo County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Petition as though fully set forth herein.

188. Defendants' marketing scheme to optimize profits by misrepresenting and falsely touting opioids as the panacea to chronic pain was done intentionally.

189. Defendants' hiring of KOLs, Front Groups, and others to spread their fraudulent message that opioids were useful and beneficial for chronic pain was grossly negligent and done with conscious indifference toward or reckless disregard for the safety of others.

190. Each Defendant's actions and omissions as described herein, singularly or in combination with each other, was malicious resulting in damages and injuries to Hidalgo County and its residents.

191. Upon information and belief, at every stage, Defendants knew or should have known that their conduct would create an unreasonable risk of physical harm to others, including Hidalgo

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County and its residents, and should be held liable in punitive and exemplary damages to Hidalgo County.

## XI. FIFTH CAUSE OF ACTION AGAINST DISTRIBUTOR DEFENDANTS: TEXAS CONTROLLED SUBSTANCES ACT ("TSCA')

192. Hidalgo County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Petition as if fully set forth herein.

193. Distributor Defendants have knowingly distributed, delivered, administered, or dispensed a controlled substance in violation of the Texas Controlled Substances Act § 481.128(a)(l) by deceiving practitioners into prescribing, dispensing, delivering, or administering a controlled substance or causing a controlled substance to be administered when there is no valid medical purpose. Tex. Health & Safety Code § 481.071.

194. As alleged herein, each Distributor Defendant, at all times relevant to this Petition, violated the Texas Controlled Substances Act by making deceptive representations about using opioids to treat chronic pain. Each Distributor Defendant also omitted or concealed material facts and failed to correct prior misrepresentations and omissions about the risks and benefits of opioids. Each Distributor Defendant's omissions rendered even their seemingly truthful statements about opioids deceptive.

195. Upon information and belief, Distributor Defendants' deceptive representations and concealments were reasonably calculated to deceive practitioners treating Hidalgo County residents into prescribing opioids without any valid medical purpose, and Distributor Defendants continue to do so to this day.

196. As a direct and proximate cause of Distributor Defendants' deceptive conduct, Hidalgo County should be awarded civil penalties pursuant to the Texas Controlled Substances Act.

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## XII. SIXTH CAUSE OF ACTION AGAINST ALL DEFENDANTS: UNJUST ENRICHMENT

197. Hidalgo County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Petition as if set forth fully herein.

198. Upon information and belief, as an expected and intended result of their conscious wrongdoing as set forth in this Petition, Defendants have profited and benefited from opioid purchases made by Hidalgo County and its residents.

199. Upon information and belief, when Hidalgo County and its residents purchased opioids, they expected that Defendants had provided necessary and accurate information regarding the material facts regarding the risks and benefits of opioids. Instead, Defendants had misrepresented those risks.

200. Upon information and belief, Defendants have been unjustly enriched at the expense of Hidalgo County and Hidalgo County is therefore entitled to damages to be determined by the jury.

## XIII. SEVENTH CAUSE OF ACTION AGAINST ALL DEFENDANTS: CIVIL CONSPIRACY

201. Hidalgo County re-alleges and incorporates by reference each of the allegations contained in the preceding paragraphs of this Petition as if set forth fully herein.

202. The conduct of all Defendants as described above was done in furtherance of a conspiracy. The joined Defendants had knowledge of, agreed to, and intended a common objective or course of action to be accomplished by unlawful means or for an unlawful purpose and committed one or more unlawful, overt acts in furtherance of the same that resulted in the damages that Plaintiff has sustained and continues to sustain on a regular basis. All of the Defendants performed acts to further the conspiracy and are jointly and severally liable for the damages, costs and expenses associated with their conduct.

203. More specifically, the Defendants coordinated their efforts to create a market for their opioid medications based on an industry-created misperception by the medical community and the public of the benefits and risks of these drugs. The specific actions taken by the Defendants have been described elsewhere in this pleading, but include the support and dissemination of medical and promotional information calculated to assure prescribers and patients of the safety of these drugs. This common behavior was illegal and tortious in many respects and was a direct cause of the propagation of both legal and illegal opioid drug use.

204. The conduct of the Defendants demonstrates the manner in which the objective of the civil conspiracy was accomplished. The healthcare providers and clinics prescribed and dispensed millions of doses of opioids under the auspices of a distribution system that purported to be "closed" and carefully monitored. The fact that the healthcare providers and clinics were empowered to make available to the public inconceivable quantities of these dangerous drugs can be explained only by concluding that the opioid drug makers and distributors knew and supported the behavior of these healthcare providers and clinics. With awareness – actual or constructive – of the conduct of the healthcare providers and clinics and their ilk, reasonable minds would necessarily conclude that the corporate Defendants were aware of and supported the wrongful conduct and accompanying harm attributable to the prescribing and dispensing by the healthcare providers and clinics.

205. The medical community and the public rely upon the integrity of prescription drug companies to advertise, promote, and market their products in conformity with both common law and statutory obligations. There is an overriding responsibility to promote these products in a manner that is truthful and that disclose important safety information. Similarly, when drug companies engage in indirect forms of communication, they have a concomitant obligation to

disseminate only accurate and honest product information. This responsibility applies to the myriad vehicles by which drug makers influence product use: marketing brochures for physicians and patients, TV commercials, FAQ'S, continuing medical education programs, web sites, and on and on. In all of these instances, the members of the opioid drug industry have both common law and regulatory restraints that govern their behavior. It is alleged that the corporate Defendants violated these rules and disseminated untrue, misleading, and erroneous information about opioids.

206. The joined Defendants are also guilty of acting in concert to profit from the opioid crisis. The drug manufacturers and distributors, as alleged herein, were responsible for maintaining and environment in which opioid drugs were available in massive quantities and were subject to significant rates of diversion to illicit uses. Plaintiff contends that the vast majority of healthcare providers who prescribed opioids, and clinics or pharmacies that dispenses them, were legitimate businesses.

207. It is alleged that one or more of the corporate Defendants had actual knowledge of the business operations of clinics and passively supported the clinics by failing to monitor, detect, investigate, and report suspicious orders of prescription opiates, in which case the corporate Defendants remain liable for the conduct of the clinics and their operators.

## XIV. RULE 193.7 NOTICE

208. Pursuant to Rule 193.7 of the Texas Rule of Civil Procedure, Plaintiff hereby gives actual notice to Defendants that any and all documents produced by any Defendants may be used at any pretrial proceeding and/or at trial of this matter without the necessity of authenticating the documents.

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PRAYER FOR RELIEF

WHEREFORE, Plaintiff, Hidalgo County, respectfully prays:

(a) That the acts alleged herein be adjudged and decreed to be unlawful and that the Court enter a judgment declaring them to be so;

(b) That the Defendants be enjoined from, directly or indirectly, through KOLs, Front Groups, or other third parties, continuing to misrepresent the risk and benefits of the use of opioids for chronic pain or off-label uses and from continuing to violate Texas law;

(c) That Plaintiff recover all measure of damages, including punitive and exemplary damages, allowable under the law and that judgment be entered against Defendants in favor of Plaintiff;

(d) That Plaintiff recover restitution on behalf of Hidalgo County consumers who paid for opioids for chronic pain;

(e) That Plaintiffs recover the costs and expenses of suit, pre-and post-judgment interest, and reasonable attorneys' fees as provided by law; and

(f) That Defendants be order to abate the public nuisance that they created in violation of Texas common law.

Dated: April 12, 2018

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Respectfully submitted,

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